Harmonia

Enterprise Architecture
Capability Map

Harmonia-EA-Capability-Map
Version 0.1

Definitive architectural reference catalogue

Current governed capability architecture
15 October 2026
`Harmonia-GovernedAccess`

Governed RAD architecture, authored explanation and definitive identifiers.

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1. Authority, identity and use

This publication answers: “What capability architecture does the governed RAD currently contain?” Version 0.1 represents the current governed Markdown architecture without extending its decomposition or adjudicating its unresolved relationships. The Markdown RAD remains architectural authority; this publication is its definitive LaTeX representation at the recorded baseline. The Architectural Axioms remain the highest design authority.

AX-01 and AX-04 preserve the business and health-information meaning of the model. AX-14 preserves distinctions between capability, behaviour, exposure, progression, relevance and status. AX-17 requires established, candidate and unresolved assertions to remain distinguishable. AX-18 prevents integration/coordination from conferring domain authority. This publication applies those constraints to representation; it makes no architectural change. The selected axiom text, metamodel and K9 decision are reproduced in the semantic-detail part.

1.1. Reading the model

Business Capabilities use only the explicitly established CT1 → CT2 → CT3 hierarchy. Regions are contextual groupings, not ancestors. Business Enabling Capabilities require CT1 → CT2 → CT3 → FT for a fully reconciled Feature, but the current RAD does not establish those BEC tier allocations or complete ancestry. This publication preserves established owners and the explicit partial Client Administration → Person Identity responsibility chain without manufacturing missing tiers. It reports zero *established* BEC CT1, CT2 or CT3 allocations; this does not mean that the 76 established BEC contexts do not exist.

Functions, Services and Processes retain their evidenced owners. They form a graph, not a mandatory Feature → Function → Service/Process tree. A Function need not have a Feature, Service or Process association. Enterprise Capabilities retain the fourteen governed entries and their actual descriptive responsibilities; no additional Enterprise decomposition is introduced.

Each element has a stable label, metadata and a link to exact RAD source detail. Every Capability has an authored Capability Overview; Features have authored overviews and behavioural entries preserve their particular governed descriptions and qualifications. Significant behavioural entries also have authored explanation. These explanations synthesise the element’s definition, established context and boundaries. They are interpretation for architectural review, not independent decisions, new architectural elements or a claim that every explanatory sentence was present verbatim in the RAD. A Canonical element can have newly authored prose. RAD-only architectural scope does not restrict explanation to source transcription.

The reviewable authored Markdown is held under `catalogue/`, with separate fingerprints in the manifest. It is not added to the eighteen governed RAD sources. Definitions and exact governed semantic detail remain distinguishable from those overviews. Reference links and containing/owned-element tables navigate the actual architecture rather than inferring containment from heading depth.

The semantic-detail part preserves the source definitions, boundaries, state-change statements, illustrative activities, examples, exclusions, notes, classifications, relationship tables and unresolved matters. Its source headings and retained diagram notation are not new containment assertions. The complete `manifest.json` records exact source lines and fingerprints; `source-coverage.json` records the transcribed blocks. Source references such as “business:152” mean source-key and Markdown line, not a new architectural identifier.

1.2. Definitive publication identity

Existing Feature identifiers (for example `FEAT-EM-01`) and Enterprise identifiers (`EC-01` through `EC-14`) are preserved exactly. Catalogue ordinals, BC-18-style source shorthand and CT syntax

examples are not allocated structural Canonical IDs merely by their occurrence in source prose. This authorised v0.1 allocation establishes Structural Canonical IDs for all 190 canonical Business Capabilities with complete, explicit ancestry. Existing `EC-01` through `EC-14` identifiers are retained for Enterprise Capabilities, to which no CT path applies.

For a Business Capability, the approved local code is its initial frozen `HCM-BC-` semantic token in uppercase, excluding that publication prefix. Each path component is typed `CT1-`, `CT2-` or `CT3-`; the full identifier concatenates the established ancestors and current element with dots. For example, Health Innovation Evaluation is identified by `CT1-HEALTH-RESEARCH-INNOVATION.CT2-HEALTH-INNOVATION.CT3-HEALTH-INNOVATION-EVALUATION`. The Reference Id supplied a deterministic initial allocation seed, not an existing Canonical ID or a display ordinal. Local codes are frozen after allocation and do not change merely because a name changes. Every ancestor prefix resolves to the corresponding current Capability. No local code is inferred from catalogue order.

For elements without an allocated identifier, the frozen Reference Id uses `HCM-`*`type-semantic-name-token`*. Type tokens are `BC`, `BEC`, `FN`, `SV` and `PR`. The initial token is the lowercase name with runs of non-alphanumeric characters replaced by a hyphen. This is a deterministic initial allocation rule only: **after v0.1, the manifest value must be reused verbatim, including after rename or relocation**. It is not regenerated from a future name, parent, tier, ordinal, chapter or build. A collision requires an explicitly recorded allocation, never a position-based suffix; this baseline has no collisions.

The Reference Id is a definitive publication identity, distinct from the metamodel's contextual structural Canonical ID. The metamodel requires a structural ID to change when responsibility context changes, retaining the former value as an alias. Keeping these two fields separate satisfies both that rule and publication identity stability. After an authorised structural relocation, the current structural path must change and the former path must be retained as a direct, globally unique, non-reusable ID alias. A name change alone does not change either identity. Ancestry-dependent structural IDs remain unresolved: all 512 canonical BEC/Feature/behaviour records require unresolved BEC tiers, and three Candidate concerns lack approved placement. Publication IDs remain usable independently.

Stable LaTeX label namespaces are `cap:bc-`, `cap:bec-`, `cap:ec-`, `feature:`, `function:`, `service:` and `process:`. The exact values are frozen in the manifest and are also preserved across relocation or rename. Source-detail labels are locators, not architectural element identities. Internal reference-candidate record keys are retained for navigation continuity only; they are neither Canonical IDs nor aliases.

1.3. Architectural status and uncertainty

Canonical means that the element's identity/responsibility is established by the cited governed RAD. It does not establish every possible property or relationship of that element. **Candidate** preserves a named provisional concern without approving its placement or decomposition. **Unresolved** qualifies a particular unestablished dimension or relationship. It is not a fifth Harmonia relevance classification. Source-specific provisional proposals remain provisional in their source prose; occurrence in a canonical document does not promote them to Canonical.

Health Service Governance is a Canonical CT1 with unresolved relevance and enabling derivation. The three assurance BECs are Canonical responsibilities with unresolved tiers, ancestry, view placement, relevance and Feature decomposition. All 137 Features are established FT identities with unresolved complete capability ancestry. The 69 enabling catalogue descriptions with unestablished standalone element identity are not counted as new Capabilities.

1.4. Reconciled publication counts

Architectural category	RAD / publication count
Business CT1 / CT2 / CT3	18 / 39 / 133
Canonical Business Capabilities	190

Named candidate BC concerns with unresolved placement	3 (separate from 190)
Established BEC CT1 / CT2 / CT3 assignments	0 / 0 / 0; tiers unresolved
Canonical BEC responsibility contexts	76
Features / Functions / Services / Processes	137 / 147 / 133 / 19
Enterprise Capabilities	14
Named enabling decomposition descriptions	73; 4 independently established, 69 unresolved identities
Definitive architectural-element publication records	719; 716 Canonical, 3 Candidate

Counts are of independently established identities or explicitly retained candidates, not source occurrences, descriptive bullets or possible future peers. The reference candidate capture was used to locate current Markdown evidence; its candidate standing supplies no architectural approval. Counts and identifiers are checked against the actual Markdown inventories and explicit parent assertions.

1.5. Preserved issues requiring later architectural review

The exception register summarises material description constraints. `review-exceptions.json` lists every unallocated structural ID and every identified material semantic limitation by frozen identity. No overview is absent, but the sharing-programme concern lacks enough governed scope for a full description and is explicitly recorded as constrained.

The full source qualifications remain controlling. In particular:

- BEC tier/ancestry reconciliation, 137 Feature frames and 69 description identities remain unresolved under K9; the five views do not resolve them.
- Health Information Exchange identifies distinct BC and BEC responsibilities. Their naming collision remains unresolved; they are not merged or renamed.
- Medication compounding/preparation (OI-BC-001), operational-readiness placement (OI-BC-002), maintenance membership and sharing-programme placement remain unadjudicated. Clinical Intervention's proposed three-child taxonomy remains provisional; Clinical Determination and Individual Care Planning have no adjudicated CT3 set.
- Operational Credential Management remains distinct from Clinical Qualification Management and its historical name alias. Name similarity does not establish Feature association.
- The existing care-team collaboration Function description mentions managing clinical handovers while the metamodel excludes that authority. Both source assertions are retained for review; this publication does not resolve the conflict.
- Process illustrations retain their stated scope and unresolved transitions, including Encounter ON_LEAVE and the disconnected stalled/failed Synthetic Task outcome. Assurance lifecycle detail and internal Service/Function mappings remain unestablished; no additional state or edge is inferred.
- The assurance Strategy source's opening reconciliation qualifies older BC-18 tier/relevance wording. Its Strategy stop boundary does not erase the later approved Domain03 assurance catalogue. B2.1's retained L1/L2 terminology does not allocate or override current CT classifications.

No legacy documentation or architectural material from the earlier candidate LaTeX publications was used. Version 0.2 derivation and reconciliation are outside this baseline.

2. Business Capabilities

The hierarchy below contains only explicit governed BC tier and parent assertions. Full definitions, decomposition assertions, boundaries, illustrative activities, care contexts, exclusions and stopping points are reproduced in the BC source detail. Relevance is not inherited. Regions are contextual groupings.

2.1. CT1: Individual Care Delivery

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-individual-care-delivery`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT1	Explicit source assertion.
Ancestry	CT1 root	Explicitly established.
Structural Canonical ID	`CT1-INDIVIDUAL-CARE-DELIVERY`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Harmonia-Relevant	Independent of status; never inherited.
Source	business:148 (p. 532); business:152 (p. 532); business:120 (p. 531)	Linked RAD detail; exact locators in manifest.
Enabled By	BEC: Health Information Access	Bounded support; not decomposition or runtime dependency.

Capability Overview

Individual Care Delivery is the organisation's ability to understand an individual's health and act on that understanding through purposeful care. Its six constituent abilities distinguish establishing findings, interpreting their clinical meaning, choosing a response, performing care, observing change and evaluating care against its objectives. These distinctions describe responsibilities and outcomes rather than a compulsory sequence: a consultation or continuing episode may revisit several of them.

The same ability can be exercised in a hospital, a general practice, a home or a virtual consultation. Setting and time pattern affect the circumstances of care without changing the responsibility represented by this capability.

Definition

Assess, clinically determine, plan, intervene, monitor and review individual care independently of setting, mode, time pattern or organisational arrangement.

Intent

Support clinically meaningful care whose intent, performance, observed effects and continuing appropriateness can be distinguished.

Boundary

Care access, appointments and operational coordination enable care but do not establish that it has been performed. Clinical judgement and the authority to deliver care remain with the responsible participants; possession or coordination of their information does not transfer that authority to Harmonia.

Harmonia Architectural Relevance

Harmonia supplies and connects information used in individual care. The approved Health Information Access support trace is bounded: records can inform care, but direct assessment can establish findings independently, and access to a proposed intervention does not authorise its performance.

Scope

Contained Capabilities

Capability	Reference Id	Tier
CT2: Individual Health Assessment (p. 5)	`HCM-BC-individual-health-assessment`	CT2
CT2: Clinical Determination (p. 8)	`HCM-BC-clinical-determination`	CT2
CT2: Individual Care Planning (p. 9)	`HCM-BC-individual-care-planning`	CT2
CT2: Clinical Intervention (p. 10)	`HCM-BC-clinical-intervention`	CT2
CT2: Individual Health Monitoring (p. 11)	`HCM-BC-individual-health-monitoring`	CT2
CT2: Individual Care Review (p. 13)	`HCM-BC-individual-care-review`	CT2

2.1.1. CT2: Individual Health Assessment

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-individual-health-assessment`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	CT1: Individual Care Delivery (p. 4)	Established BC responsibility path.
Structural Canonical ID	`CT1-INDIVIDUAL-CARE-DELIVERY.CT2-INDIVIDUAL-HEALTH-ASSESSMENT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:160 (p. 532); business:162 (p. 532); business:316 (p. 534); business:320 (p. 534); business:330 (p. 534)	Linked RAD detail; exact locators in manifest.
Enabled By	BEC: Health Information Access	Bounded support; not decomposition or runtime dependency.

Capability Overview

Assessment establishes what is currently known about an individual's health through history, examination and measurement. Within Individual Care Delivery it supplies assessed findings, including findings obtained directly from the person rather than retrieved from an existing record. Its three children distinguish ways of establishing knowledge, rather than professions, clinical specialties or locations.

Definition

The capability to establish an understanding of an individual's current health state through the collection, consideration and evaluation of relevant clinical, health and contextual information.

Intent

Give subsequent clinical interpretation a defensible account of the person's current state and relevant history.

Boundary

Finding an abnormal measurement is distinguishable from deciding what it means clinically. Diagnosis, prognosis, selection of care and performance of care belong to adjacent abilities.

Scope

Contained Capabilities

Capability	Reference Id	Tier
CT3: Health History Assessment (p. 6)	`HCM-BC-health-history-assessment`	CT3
CT3: Clinical Examination (p. 7)	`HCM-BC-clinical-examination`	CT3
CT3: Health Status Measurement (p. 7)	`HCM-BC-health-status-measurement`	CT3

2.1.1.1. CT3: Health History Assessment

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-history-assessment`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Individual Care Delivery (p. 4) → CT2: Individual Health Assessment (p. 5)	Established BC responsibility path.
Structural Canonical ID	`CT1-INDIVIDUAL-CARE-DELIVERY.CT2-INDIVIDUAL-HEALTH-ASSESSMENT.CT3-HEALTH-HISTORY-ASSESSMENT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:326 (p. 534); business:322 (p. 534); business:330 (p. 534)	Linked RAD detail; exact locators in manifest.

Capability Overview

Health History Assessment establishes clinically relevant understanding from a person's history and its surrounding context. Within assessment it contributes knowledge that a current examination or measurement alone may not reveal, while requiring consideration of the relevance of historical information to the present situation. Access to previous records can assist, but the responsibility is the assessment of history rather than retrieval itself.

Definition

Establish relevant health knowledge from the individual's history and applicable historical/contextual information.

Boundary

Historical information does not become a current clinical conclusion merely because it is available; interpretation of the assessed findings remains Clinical Determination.

2.1.1.2. CT3: Clinical Examination

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-examination`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Individual Care Delivery (p. 4) → CT2: Individual Health Assessment (p. 5)	Established BC responsibility path.
Structural Canonical ID	`CT1-INDIVIDUAL-CARE-DELIVERY.CT2-INDIVIDUAL-HEALTH-ASSESSMENT.CT3-CLINICAL-EXAMINATION`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:327 (p. 534); business:322 (p. 534); business:330 (p. 534)	Linked RAD detail; exact locators in manifest.

Capability Overview

Clinical Examination establishes findings through examination of the individual. It distinguishes directly acquired clinical knowledge from the historical account and the measurement-focused contribution of its assessment siblings. Its outcome is a set of examination findings that can inform clinical interpretation, irrespective of the setting or profession exercising the ability.

Definition

Establish health findings through clinical examination.

Boundary

An examination finding is not itself a diagnosis, care plan or performed treatment.

2.1.1.3. CT3: Health Status Measurement

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-status-measurement`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Individual Care Delivery (p. 4) → CT2: Individual Health Assessment (p. 5)	Established BC responsibility path.
Structural Canonical ID	`CT1-INDIVIDUAL-CARE-DELIVERY.CT2-INDIVIDUAL-HEALTH-ASSESSMENT.CT3-HEALTH-STATUS-MEASUREMENT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:328 (p. 534); business:322 (p. 534); business:330 (p. 534)	Linked RAD detail; exact locators in manifest.

Capability Overview

Health Status Measurement establishes observations or measurements of an individual's health. Its contribution to assessment is evidence about the measured state, with a purpose distinct from developing a view over time or deciding the clinical meaning of a result. Measurements can inform both present assessment and later monitoring without collapsing those abilities.

Definition

Establish individual health observations or measurements.

Boundary

Obtaining a value does not establish a temporal trend or the clinical significance of that value.

2.1.2. CT2: Clinical Determination

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-determination`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	CT1: Individual Care Delivery (p. 4)	Established BC responsibility path.
Structural Canonical ID	`CT1-INDIVIDUAL-CARE-DELIVERY.CT2-CLINICAL-DETERMINATION`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:186 (p. 532); business:188 (p. 532)	Linked RAD detail; exact locators in manifest.
Enabled By	BEC: Health Information Access	Bounded support; not decomposition or runtime dependency.

Capability Overview

Clinical Determination turns assessed findings into clinically interpreted understanding of a person's condition, risk or prognosis. It represents the professional responsibility for deciding what the findings mean in the individual's clinical context, including the significance of changes identified through monitoring. This is the interpretive responsibility within Individual Care Delivery, rather than another method of collecting evidence.

Definition

The capability to interpret assessed health information and establish clinically meaningful conclusions about an individual's health state, condition, risk or prognosis.

Intent

Establish clinical conclusions on which an intended response can be based.

Boundary

Determining clinical meaning does not itself select the care response. No CT3 taxonomy has been adjudicated; diagnosis and prognostic judgement illustrate this responsibility without becoming additional catalogue elements.

2.1.3. CT2: Individual Care Planning

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-individual-care-planning`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	CT1: Individual Care Delivery (p. 4)	Established BC responsibility path.
Structural Canonical ID	`CT1-INDIVIDUAL-CARE-DELIVERY.CT2-INDIVIDUAL-CARE-PLANNING`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:211 (p. 533); business:213 (p. 533)	Linked RAD detail; exact locators in manifest.
Enabled By	BEC: Health Information Access	Bounded support; not decomposition or runtime dependency.

Capability Overview

Individual Care Planning establishes the intended course of care in response to assessed and clinically interpreted health information. It connects clinical understanding to objectives, intended interventions and monitoring needs. Its outcome is an expression of what should happen, allowing intended care to remain distinguishable from requests, operational arrangements and actions subsequently performed.

Definition

The capability to establish the intended clinical response to an individual's assessed and clinically interpreted health state, including the objectives, interventions and monitoring required to progress their care.

Intent

Make the clinical response and its objectives explicit enough to guide care and later evaluate its effectiveness.

Boundary

Prescribing can express clinical intent without establishing dispensing or administration. Scheduling or allocating resources does not perform the planned intervention. No CT3 decomposition is established.

2.1.4. CT2: Clinical Intervention

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-intervention`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	CT1: Individual Care Delivery (p. 4)	Established BC responsibility path.
Structural Canonical ID	`CT1-INDIVIDUAL-CARE-DELIVERY.CT2-CLINICAL-INTERVENTION`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:237 (p. 533); business:239 (p. 533)	Linked RAD detail; exact locators in manifest.
Enabled By	BEC: Health Information Access	Bounded support; not decomposition or runtime dependency.

Capability Overview

Clinical Intervention is the ability to perform a clinical action under an established care intent. It marks the transition from intended care to care actually performed, whether the action seeks to alter, maintain or otherwise manage health. Its exercise can establish that treatment occurred even when the desired clinical result was not achieved.

Definition

The capability to perform clinical actions intended to alter, maintain or otherwise manage an individual's health state in accordance with an established care intent.

Intent

Put the intended clinical response into practice through appropriately authorised care.

Boundary

Orders and appointments express or arrange future activity; they are not evidence that the intervention occurred. Therapy, medication and procedural examples do not constitute an approved three-child taxonomy: that proposed decomposition remains provisional.

Additional Notes / Unresolved Matters

Intervention Taxonomy: DN-BC-010 retains the intervention candidate as provisional pending a coherent peer taxonomy.

2.1.5. CT2: Individual Health Monitoring

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-individual-health-monitoring`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	CT1: Individual Care Delivery (p. 4)	Established BC responsibility path.
Structural Canonical ID	`CT1-INDIVIDUAL-CARE-DELIVERY.CT2-INDIVIDUAL-HEALTH-MONITORING`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:263 (p. 533); business:265 (p. 533); business:332 (p. 534); business:336 (p. 534); business:345 (p. 535)	Linked RAD detail; exact locators in manifest.
Enabled By	BEC: Health Information Access	Bounded support; not decomposition or runtime dependency.

Capability Overview

Monitoring develops a view of an individual's health over time so that change, stability or deviation can be recognised. Health State Monitoring establishes the aggregate temporally situated view; Care Response Monitoring considers that view in relation to a particular condition and treatment. Together they distinguish observation over time from a single measurement and from the clinical judgement prompted by a trend.

Definition

The capability to observe and evaluate an individual's health state over time in order to identify clinically meaningful change, stability or deviation from an expected state or trajectory.

Intent

Make relevant trajectories visible while preserving what was observed and what still requires interpretation.

Boundary

Monitoring does not establish the clinical significance of a change or judge whether care achieved its objectives. Those responsibilities remain Clinical Determination and Individual Care Review respectively.

Scope

Contained Capabilities

Capability	Reference Id	Tier
CT3: Health State Monitoring (p. 12)	`HCM-BC-health-state-monitoring`	CT3
CT3: Care Response Monitoring (p. 12)	`HCM-BC-care-response-monitoring`	CT3

Additional Notes / Unresolved Matters

Semantic Relationship Representation: Canonical BC evidence/context/consumption semantics are preserved in Markdown; the current governed RADS relationship types do not establish their machine-readable encoding. No stronger runtime or structural relation is substituted.

2.1.5.1. CT3: Health State Monitoring

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-state-monitoring`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Individual Care Delivery (p. 4) → CT2: Individual Health Monitoring (p. 11)	Established BC responsibility path.
Structural Canonical ID	`CT1-INDIVIDUAL-CARE-DELIVERY.CT2-INDIVIDUAL-HEALTH-MONITORING.CT3-HEALTH-STATE-MONITORING`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:342 (p. 535); business:338 (p. 535); business:345 (p. 535)	Linked RAD detail; exact locators in manifest.

Capability Overview

Health State Monitoring brings applicable monitored data into an aggregate view situated in time. Its purpose is to make the individual's changing or stable health state intelligible across observations, rather than leaving each observation isolated. Within monitoring it provides the broader view that can also be considered in relation to a particular treatment or condition.

Definition

Establish an aggregate, temporally situated view of the individual's health from applicable monitored data.

Boundary

Aggregation is not clinical interpretation. Care Response Monitoring uses the established view rather than independently recreating it.

2.1.5.2. CT3: Care Response Monitoring

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-care-response-monitoring`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Individual Care Delivery (p. 4) → CT2: Individual Health Monitoring (p. 11)	Established BC responsibility path.
Structural Canonical ID	`CT1-INDIVIDUAL-CARE-DELIVERY.CT2-INDIVIDUAL-HEALTH-MONITORING.CT3-CARE-RESPONSE-MONITORING`	Allocated v0.1; structural responsibility identity.

Name	Value	Comment / Description
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:343 (p. 535); business:338 (p. 535); business:345 (p. 535)	Linked RAD detail; exact locators in manifest.

Capability Overview

Care Response Monitoring considers the monitored health-state view in relation to a particular problem or condition and the treatment applied. It makes a response trajectory visible in its treatment context, helping distinguish a general change in health from a change relevant to the care being followed. It consumes the aggregate view established through Health State Monitoring.

Definition

Scope that health-state view to a particular problem/condition and the treatment applied to it.

Boundary

Identifying the response does not judge treatment effectiveness against objectives or determine a replacement treatment.

2.1.6. CT2: Individual Care Review

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-individual-care-review`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	CT1: Individual Care Delivery (p. 4)	Established BC responsibility path.
Structural Canonical ID	`CT1-INDIVIDUAL-CARE-DELIVERY.CT2-INDIVIDUAL-CARE-REVIEW`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:289 (p. 534); business:291 (p. 534); business:347 (p. 535); business:351 (p. 535); business:360 (p. 535)	Linked RAD detail; exact locators in manifest.
Enabled By	BEC: Health Information Access	Bounded support; not decomposition or runtime dependency.

Capability Overview

Individual Care Review asks whether care is achieving its intended objectives and whether the existing care intent remains suitable. Its children separate effectiveness from appropriateness: an intervention may have an effect yet no longer be suitable to continue, or remain a reasonable choice despite an inadequate response. Review therefore evaluates the relationship between care, outcomes and intent rather than simply repeating assessment.

Definition

The capability to evaluate the progress and effectiveness of an individual's care against its intended objectives and establish the continuing appropriateness of the current care intent.

Intent

Establish whether continuing the current approach is justified and whether reconsideration is needed.

Boundary

Recognising a need to change care does not establish the replacement plan. Monitoring supplies relevant observations; planning establishes revised intent.

Scope

Contained Capabilities

Capability	Reference Id	Tier
CT3: Care Appropriateness Review (p. 14)	`HCM-BC-care-appropriateness-review`	CT3
CT3: Care Effectiveness Review (p. 15)	`HCM-BC-care-effectiveness-review`	CT3

2.1.6.1. CT3: Care Appropriateness Review

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-care-appropriateness-review`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Individual Care Delivery (p. 4) → CT2: Individual Care Review (p. 13)	Established BC responsibility path.
Structural Canonical ID	`CT1-INDIVIDUAL-CARE-DELIVERY.CT2-INDIVIDUAL-CARE-REVIEW.CT3-CARE-APPROPRIATENESS-REVIEW`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:357 (p. 535); business:353 (p. 535); business:360 (p. 535)	Linked RAD detail; exact locators in manifest.

Capability Overview

Care Appropriateness Review evaluates whether the current care intent remains suitable for the individual in their present context. Its question concerns continuing suitability rather than whether treatment has produced a measurable effect. That distinction permits a review to recognise that effective care may no longer be appropriate to continue.

Definition

Evaluate whether the current intended care remains suitable in the individual's current context.

Boundary

An appropriateness judgement can identify a need to reconsider care; establishing revised intent remains Individual Care Planning.

2.1.6.2. CT3: Care Effectiveness Review

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-care-effectiveness-review`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Individual Care Delivery (p. 4) → CT2: Individual Care Review (p. 13)	Established BC responsibility path.
Structural Canonical ID	`CT1-INDIVIDUAL-CARE-DELIVERY.CT2-INDIVIDUAL-CARE-REVIEW.CT3-CARE-EFFECTIVENESS-REVIEW`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:358 (p. 535); business:353 (p. 535); business:360 (p. 535)	Linked RAD detail; exact locators in manifest.

Capability Overview

Care Effectiveness Review evaluates delivered care against its intended goals and outcomes. It connects observed response to the reason the care was undertaken, making a judgement that monitoring alone does not supply. An effectiveness finding can inform review of appropriateness while preserving the possibility that a reasonable care choice has not achieved its objective.

Definition

Evaluate whether delivered care is achieving or has achieved its intended goals and outcomes.

Boundary

Observed improvement is not automatically proof that the intended objectives have been achieved or that continuing care is appropriate.

2.2. CT1: Care Access & Coordination

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-care-access-coordination`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT1	Explicit source assertion.
Ancestry	CT1 root	Explicitly established.
Structural Canonical ID	`CT1-CARE-ACCESS-COORDINATION`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Harmonia-Relevant	Independent of status; never inherited.
Source	business:362 (p. 535); business:366 (p. 535); business:120 (p. 531)	Linked RAD detail; exact locators in manifest.

Capability Overview

This capability makes care reachable and operationally coherent across participating services. It encompasses entering care, obtaining scheduled opportunities and usable capacity, coordinating participation and responsibility transitions, and arranging bounded support logistics. Its decomposition separates access decisions, resource use, continuity and supporting work so that movement or message delivery cannot stand in for acceptance of care responsibility.

Definition

Managing patient intake, referral triage, appointment scheduling, multidisciplinary care team coordination, cross-facility patient transfers, and care continuity across organisational boundaries.

Intent

Help a person's care requirements progress across organisational and operational boundaries with the relevant access and continuity context intact.

Boundary

Coordination does not determine clinical work or confer clinical authority. Corporate resource governance and general hospital logistics are outside the bounded care-support responsibility.

Harmonia Architectural Relevance

Referrals, encounters, appointments, operational state and communication provide material information for continuity. Harmonia's integration and coordination support does not make it the owner of clinical handover or the originating workforce and resource facts.

Scope

Contained Capabilities

Capability	Reference Id	Tier
CT2: Care Access Management (p. 16)	`HCM-BC-care-access-management`	CT2
CT2: Care Scheduling & Capacity Management (p. 19)	`HCM-BC-care-scheduling-capacity-management`	CT2
CT2: Care Coordination & Transition (p. 24)	`HCM-BC-care-coordination-transition`	CT2
CT2: Care Support Logistics Management (p. 27)	`HCM-BC-care-support-logistics-management`	CT2

2.2.1. CT2: Care Access Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-care-access-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	CT1: Care Access & Coordination (p. 15)	Established BC responsibility path.

Name	Value	Comment / Description
Structural Canonical ID	`CT1-CARE-ACCESS-COORDINATION.CT2-CARE-ACCESS-MANAGEMENT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:370 (p. 535); business:372 (p. 535); business:374 (p. 535); business:384 (p. 536)	Linked RAD detail; exact locators in manifest.

Capability Overview

Care Access Management establishes the circumstances in which an individual seeks or enters care. Intake creates the access context, referral requests another participant's involvement, and eligibility determines whether applicable access conditions are met. These are separate contributions to access: a person can be eligible without having a booking, and a received referral can remain undecided.

Definition

Establish and manage access to applicable care.

Intent

Make entry requirements and referral disposition intelligible without implying that care has already been accepted or delivered.

Boundary

Eligibility does not allocate care, and referral receipt does not establish an assumption of clinical responsibility.

Scope

Contained Capabilities

Capability	Reference Id	Tier
CT3: Care Intake Management (p. 17)	`HCM-BC-care-intake-management`	CT3
CT3: Care Referral Management (p. 18)	`HCM-BC-care-referral-management`	CT3
CT3: Care Eligibility Determination (p. 19)	`HCM-BC-care-eligibility-determination`	CT3

2.2.1.1. CT3: Care Intake Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-care-intake-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Care Access & Coordination (p. 15) → CT2: Care Access Management (p. 16)	Established BC responsibility path.
Structural Canonical ID	`CT1-CARE-ACCESS-COORDINATION.CT2-CARE-ACCESS-MANAGEMENT.CT3-CARE-INTAKE-MANAGEMENT`	Allocated v0.1; structural responsibility identity.

Name	Value	Comment / Description
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:380 (p. 535); business:376 (p. 535); business:384 (p. 536)	Linked RAD detail; exact locators in manifest.

Capability Overview

Care Intake Management establishes the access context through which a person seeks or enters care. It makes the entry circumstance manageable without assuming that every intake follows a referral or already satisfies eligibility conditions. Within Care Access Management, intake is the responsibility for entry context rather than a decision that all subsequent care arrangements are complete.

Definition

Establish and manage the intake context through which an individual seeks or enters care.

Boundary

An intake record does not establish acceptance of a referral, care responsibility or delivery of care.

2.2.1.2. CT3: Care Referral Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-care-referral-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Care Access & Coordination (p. 15) → CT2: Care Access Management (p. 16)	Established BC responsibility path.
Structural Canonical ID	`CT1-CARE-ACCESS-COORDINATION.CT2-CARE-ACCESS-MANAGEMENT.CT3-CARE-REFERRAL-MANAGEMENT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:381 (p. 535); business:376 (p. 535); business:384 (p. 536)	Linked RAD detail; exact locators in manifest.

Capability Overview

Care Referral Management concerns a request for another participant or service to assess, manage or assume applicable care responsibility. It preserves the distinction between the requester's intent and the recipient's disposition, allowing care coordination to recognise an unresolved referral rather than assume a successful handoff. Referral is therefore broader than sending a document.

Definition

Manage a request for another participant/service to assess, manage or assume applicable care responsibility.

Boundary

Submission or receipt does not establish acceptance, an appointment, delivered care or responsibility actually assumed.

2.2.1.3. CT3: Care Eligibility Determination

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-care-eligibility-determination`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Care Access & Coordination (p. 15) → CT2: Care Access Management (p. 16)	Established BC responsibility path.
Structural Canonical ID	`CT1-CARE-ACCESS-COORDINATION.CT2-CARE-ACCESS-MANAGEMENT.CT3-CARE-ELIGIBILITY-DETERMINATION`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:382 (p. 535); business:376 (p. 535); business:384 (p. 536)	Linked RAD detail; exact locators in manifest.

Capability Overview

Care Eligibility Determination establishes whether the conditions governing access to applicable care are met. It contributes a qualified access decision alongside intake and referral, so that an identifiable request can be distinguished from one that satisfies the relevant criteria. Its purpose is eligibility in the applicable context rather than clinical diagnosis or service allocation.

Definition

Determine whether applicable eligibility conditions for care access are satisfied.

Boundary

Eligibility alone does not reserve capacity, allocate a care place or deliver care.

2.2.2. CT2: Care Scheduling & Capacity Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-care-scheduling-capacity-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	CT1: Care Access & Coordination (p. 15)	Established BC responsibility path.
Structural Canonical ID	`CT1-CARE-ACCESS-COORDINATION.CT2-CARE-SCHEDULING-CAPACITY-MANAGEMENT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:386 (p. 536); business:388 (p. 536); business:390 (p. 536); business:403 (p. 536)	Linked RAD detail; exact locators in manifest.

Capability Overview

This capability connects care demand with scheduled opportunities, suitable care places and legitimately usable operational resources. Its children distinguish a booking, a place allocation, the readiness of that place, resource deployment, flow coordination and service-level usable capacity. An empty bed or an available practitioner is relevant evidence, but neither alone establishes that the service can accommodate care.

Definition

Manage scheduled opportunities, care places, usable operational capacity and care-demand resourcing.

Intent

Support care requirements with an operationally credible combination of time, place, resources and capacity.

Boundary

Deployment consumes roster, qualification and privilege context without changing their authoritative records. Operational capacity differs from strategic capacity planning and from the unresolved collective service-readiness concern.

Scope

Contained Capabilities

Capability	Reference Id	Tier
CT3: Care Appointment Management (p. 20)	`HCM-BC-care-appointment-management`	CT3
CT3: Care-Place Allocation (p. 21)	`HCM-BC-care-place-allocation`	CT3
CT3: Care-Place Readiness Management (p. 22)	`HCM-BC-care-place-readiness-management`	CT3
CT3: Care Resource Deployment (p. 22)	`HCM-BC-care-resource-deployment`	CT3
CT3: Operational Care Flow Coordination (p. 23)	`HCM-BC-operational-care-flow-coordination`	CT3
CT3: Operational Care Capacity Management (p. 24)	`HCM-BC-operational-care-capacity-management`	CT3

2.2.2.1. CT3: Care Appointment Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-care-appointment-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Care Access & Coordination (p. 15) → CT2: Care Scheduling & Capacity Management (p. 19)	Established BC responsibility path.
Structural Canonical ID	`CT1-CARE-ACCESS-COORDINATION.CT2-CARE-SCHEDULING-CAPACITY-MANAGEMENT.CT3-CARE-APPOINTMENT-MANAGEMENT`	Allocated v0.1; structural responsibility identity.

Name	Value	Comment / Description
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:396 (p. 536); business:392 (p. 536); business:403 (p. 536)	Linked RAD detail; exact locators in manifest.

Capability Overview

Care Appointment Management establishes a scheduled opportunity for care and maintains that opportunity as arrangements change. Within scheduling and capacity it supplies the temporal commitment around which participants can organise, while leaving the actual availability of places and resources distinguishable. A scheduled opportunity is useful coordination information even before attendance or care occurs.

Definition

Establish and manage a scheduled opportunity for care.

Boundary

A booking does not establish attendance, intervention performance or satisfaction of diagnostic-service-specific scheduling constraints.

2.2.2.2. CT3: Care-Place Allocation

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-care-place-allocation`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Care Access & Coordination (p. 15) → CT2: Care Scheduling & Capacity Management (p. 19)	Established BC responsibility path.
Structural Canonical ID	`CT1-CARE-ACCESS-COORDINATION.CT2-CARE-SCHEDULING-CAPACITY-MANAGEMENT.CT3-CARE-PLACE-ALLOCATION`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:397 (p. 536); business:392 (p. 536); business:403 (p. 536)	Linked RAD detail; exact locators in manifest.

Capability Overview

Care-Place Allocation associates a care requirement with an applicable care place. Its outcome is the allocation itself, distinct from defining the place or establishing its readiness. This separation supports reasoning about reserved, allocated and usable places without assuming that an apparently empty location is fit for the requirement.

Definition

Allocate an applicable care place to a care requirement.

Boundary

Allocation does not establish clinical suitability beyond the applicable context or make an unready place ready.

2.2.2.3. CT3: Care-Place Readiness Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-care-place-readiness-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Care Access & Coordination (p. 15) → CT2: Care Scheduling & Capacity Management (p. 19)	Established BC responsibility path.
Structural Canonical ID	`CT1-CARE-ACCESS-COORDINATION.CT2-CARE-SCHEDULING-CAPACITY-MANAGEMENT.CT3-CARE-PLACE-READINESS-MANAGEMENT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:398 (p. 536); business:392 (p. 536); business:403 (p. 536)	Linked RAD detail; exact locators in manifest.

Capability Overview

Care-Place Readiness Management establishes whether a particular place can be allocated or used in its operational context. It connects the place's condition with its intended use so that vacancy can be distinguished from readiness. This is a place-specific responsibility within scheduling and capacity, supporting credible allocation rather than merely counting physical spaces.

Definition

Establish and manage whether a particular care place is ready for allocation or use.

Boundary

Readiness of one place does not establish collective readiness or usable capacity of the whole service.

2.2.2.4. CT3: Care Resource Deployment

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-care-resource-deployment`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Care Access & Coordination (p. 15) → CT2: Care Scheduling & Capacity Management (p. 19)	Established BC responsibility path.
Structural Canonical ID	`CT1-CARE-ACCESS-COORDINATION.CT2-CARE-SCHEDULING-CAPACITY-MANAGEMENT.CT3-CARE-RESOURCE-DEPLOYMENT`	Allocated v0.1; structural responsibility identity.

Name	Value	Comment / Description
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:399 (p. 536); business:392 (p. 536); business:403 (p. 536)	Linked RAD detail; exact locators in manifest.

Capability Overview

Care Resource Deployment applies legitimately available resources to a current or planned care requirement. It considers coverage, role, qualification, engagement and privilege context to distinguish availability in principle from deployment to a particular need. Its contribution is the operational application of resources, without changing the authoritative facts that make their use legitimate.

Definition

Apply legitimately available and appropriately authorised resources to a current or planned operational care requirement, particularly using roster/on-call and service-coverage context.

Boundary

Deployment is neither roster authoring nor clinical work assignment and does not confer clinical authority or ownership of enterprise assets.

2.2.2.5. CT3: Operational Care Flow Coordination

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-operational-care-flow-coordination`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Care Access & Coordination (p. 15) → CT2: Care Scheduling & Capacity Management (p. 19)	Established BC responsibility path.
Structural Canonical ID	`CT1-CARE-ACCESS-COORDINATION.CT2-CARE-SCHEDULING-CAPACITY-MANAGEMENT.CT3-OPERATIONAL-CARE-FLOW-COORDINATION`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:400 (p. 536); business:392 (p. 536); business:403 (p. 536)	Linked RAD detail; exact locators in manifest.

Capability Overview

Operational Care Flow Coordination addresses progression of care flow within the delivery environment. It provides the coordinating responsibility around operational movement and impediments, rather than treating each scheduled opportunity or place allocation as an isolated fact. Its outcome concerns coherent flow under current conditions, not the clinical determination of what care should occur.

Definition

Coordinate operational care flow and its progression within the care-delivery environment.

Boundary

Coordinating flow does not transfer clinical work, workforce governance or general hospital-logistics authority.

2.2.2.6. CT3: Operational Care Capacity Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-operational-care-capacity-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Care Access & Coordination (p. 15) → CT2: Care Scheduling & Capacity Management (p. 19)	Established BC responsibility path.
Structural Canonical ID	`CT1-CARE-ACCESS-COORDINATION.CT2-CARE-SCHEDULING-CAPACITY-MANAGEMENT.CT3-OPERATIONAL-CARE-CAPACITY-MANAGEMENT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:401 (p. 536); business:392 (p. 536); business:403 (p. 536)	Linked RAD detail; exact locators in manifest.

Capability Overview

Operational Care Capacity Management establishes and manages the service's usable ability to accommodate care under current conditions. It considers capacity as a service-level concern rather than a simple count of staff, devices or vacant beds. This supports decisions grounded in what can actually be accommodated, while keeping current capacity distinct from plans for future provision.

Definition

Understand and manage the service's usable capacity to accommodate care in its current operational context.

Boundary

Resource availability, strategic capacity requirements and collective service readiness are distinct; the latter's placement remains unresolved.

2.2.3. CT2: Care Coordination & Transition

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-care-coordination-transition`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	CT1: Care Access & Coordination (p. 15)	Established BC responsibility path.
Structural Canonical ID	`CT1-CARE-ACCESS-COORDINATION.CT2-CARE-COORDINATION-TRANSITION`	Allocated v0.1; structural responsibility identity.

Name	Value	Comment / Description
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:405 (p. 536); business:407 (p. 536); business:409 (p. 536); business:419 (p. 536)	Linked RAD detail; exact locators in manifest.

Capability Overview

Care Coordination & Transition supports continuity by keeping participation, actual responsibility assumption and discharge progression distinct. Care teams may collaborate while responsibility is shared or changes, but physical transit and information exchange do not themselves establish who has accepted care. The capability therefore addresses continuity of accountable participation as well as the practical circumstances of departure.

Definition

Coordinate care participation, actual responsibility transition and discharge.

Intent

Avoid gaps in care continuity caused by treating a movement or communication as an accepted responsibility transition.

Boundary

Harmonia may assemble handover information without managing clinical handover. Arrival, discharge readiness and physical departure are not equivalent to responsibility acceptance.

Scope

Contained Capabilities

Capability	Reference Id	Tier
CT3: Care-Team Coordination (p. 25)	`HCM-BC-care-team-coordination`	CT3
CT3: Care Responsibility Transition (p. 26)	`HCM-BC-care-responsibility-transition`	CT3
CT3: Care Discharge Coordination (p. 27)	`HCM-BC-care-discharge-coordination`	CT3

2.2.3.1. CT3: Care-Team Coordination

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-care-team-coordination`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Care Access & Coordination (p. 15) → CT2: Care Coordination & Transition (p. 24)	Established BC responsibility path.
Structural Canonical ID	`CT1-CARE-ACCESS-COORDINATION.CT2-CARE-COORDINATION-TRANSITION.CT3-CARE-TEAM-COORDINATION`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.

Name	Value	Comment / Description
Source	business:415 (p. 536); business:411 (p. 536); business:419 (p. 536)	Linked RAD detail; exact locators in manifest.

Capability Overview

Care-Team Coordination maintains the participating team context and collaboration around care. It supports continuity by making participation intelligible across people and organisational boundaries, without assuming that membership itself assigns every clinical responsibility. Within Care Coordination & Transition it concerns collaboration around care rather than proof of a responsibility handoff.

Definition

Coordinate participating care-team context and collaboration around care.

Boundary

Harmonia's support for team information and collaboration does not confer management of clinical handover or clinical worklists.

2.2.3.2. CT3: Care Responsibility Transition

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-care-responsibility-transition`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Care Access & Coordination (p. 15) → CT2: Care Coordination & Transition (p. 24)	Established BC responsibility path.
Structural Canonical ID	`CT1-CARE-ACCESS-COORDINATION.CT2-CARE-COORDINATION-TRANSITION.CT3-CARE-RESPONSIBILITY-TRANSITION`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:416 (p. 536); business:411 (p. 536); business:419 (p. 536)	Linked RAD detail; exact locators in manifest.

Capability Overview

Care Responsibility Transition establishes who has actually assumed applicable care responsibility and coordinates that transition. Its purpose is to prevent physical movement or exchanged information from being mistaken for accountable acceptance. It gives care continuity a responsibility meaning that a transport checkpoint or delivered referral cannot establish alone.

Definition

Establish and coordinate who has actually assumed applicable care responsibility.

Boundary

Patient arrival, message receipt and departure do not by themselves demonstrate accepted responsibility.

2.2.3.3. CT3: Care Discharge Coordination

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-care-discharge-coordination`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Care Access & Coordination (p. 15) → CT2: Care Coordination & Transition (p. 24)	Established BC responsibility path.
Structural Canonical ID	`CT1-CARE-ACCESS-COORDINATION.CT2-CARE-COORDINATION-TRANSITION.CT3-CARE-DISCHARGE-COORDINATION`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:417 (p. 536); business:411 (p. 536); business:419 (p. 536)	Linked RAD detail; exact locators in manifest.

Capability Overview

Care Discharge Coordination brings applicable discharge requirements and progression into a coherent operational context. It supports the transition out of a care circumstance while keeping readiness, physical exit and continuing responsibility distinguishable. The capability concerns coordinating those requirements rather than deciding that every clinical condition for discharge has been met.

Definition

Coordinate applicable discharge requirements and progression.

Boundary

Discharge readiness and departure are not themselves evidence that another participant has accepted care responsibility.

2.2.4. CT2: Care Support Logistics Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-care-support-logistics-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	CT1: Care Access & Coordination (p. 15)	Established BC responsibility path.
Structural Canonical ID	`CT1-CARE-ACCESS-COORDINATION.CT2-CARE-SUPPORT-LOGISTICS-MANAGEMENT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:421 (p. 537); business:423 (p. 537); business:425 (p. 537); business:435 (p. 537)	Linked RAD detail; exact locators in manifest.

Capability Overview

Care Support Logistics Management coordinates the bounded physical and operational support needed to fulfil care. Patient transit, clinical-material movement and authorised support work

have different subjects and completion evidence, even when the same operational environment supports them. This capability provides their common care-support frame without absorbing every logistical activity of a healthcare enterprise.

Definition

Coordinate bounded operational support needed for care access and fulfilment.

Intent

Ensure necessary supporting activity can progress with the relevant patient, material and operational constraints visible.

Boundary

Assignment does not establish a worker's qualification or clinical authority. Enterprise supply chains, facilities and workforce governance retain their own responsibilities.

Scope

Contained Capabilities

Capability	Reference Id	Tier
CT3: Patient Movement Coordination (p. 28)	`HCM-BC-patient-movement-coordination`	CT3
CT3: Clinical Material Logistics Coordination (p. 29)	`HCM-BC-clinical-material-logistics-coordination`	CT3
CT3: Care Support Work Coordination (p. 30)	`HCM-BC-care-support-work-coordination`	CT3

2.2.4.1. CT3: Patient Movement Coordination

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-patient-movement-coordination`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Care Access & Coordination (p. 15) → CT2: Care Support Logistics Management (p. 27)	Established BC responsibility path.
Structural Canonical ID	`CT1-CARE-ACCESS-COORDINATION.CT2-CARE-SUPPORT-LOGISTICS-MANAGEMENT.CT3-PATIENT-MOVEMENT-COORDINATION`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:431 (p. 537); business:427 (p. 537); business:435 (p. 537)	Linked RAD detail; exact locators in manifest.

Capability Overview

Patient Movement Coordination concerns the person's physical transit and the mobility constraints relevant to it. Within care-support logistics it supports movement appropriate to the patient's circumstances without treating transport completion as a clinical transition. Its responsibility is useful wherever care requires a change of physical location.

Definition

Coordinate physical patient transit and applicable mobility constraints.

Boundary

Transport progression does not establish clinical handover, destination acceptance of care or authority over the clinical plan.

2.2.4.2. CT3: Clinical Material Logistics Coordination

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-material-logistics-coordination`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Care Access & Coordination (p. 15) → CT2: Care Support Logistics Management (p. 27)	Established BC responsibility path.
Structural Canonical ID	`CT1-CARE-ACCESS-COORDINATION.CT2-CARE-SUPPORT-LOGISTICS-MANAGEMENT.CT3-CLINICAL-MATERIAL-LOGISTICS-COORDINATION`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:432 (p. 537); business:427 (p. 537); business:435 (p. 537)	Linked RAD detail; exact locators in manifest.

Capability Overview

This capability coordinates clinically relevant material movement and the custody, availability or disposition needed for care or diagnostic fulfilment. It makes the material's operational journey intelligible while retaining why the material matters to the clinical activity. The responsibility differs from moving a patient and from managing the diagnostic meaning of information derived from a specimen.

Definition

Coordinate clinically relevant material movement, custody, availability or disposition needed for care or diagnostic fulfilment.

Boundary

Bounded clinical-material logistics does not establish a general procurement, supply-chain or enterprise inventory architecture.

2.2.4.3. CT3: Care Support Work Coordination

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-care-support-work-coordination`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Care Access & Coordination (p. 15) → CT2: Care Support Logistics Management (p. 27)	Established BC responsibility path.
Structural Canonical ID	`CT1-CARE-ACCESS-COORDINATION.CT2-CARE-SUPPORT-LOGISTICS-MANAGEMENT.CT3-CARE-SUPPORT-WORK-COORDINATION`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:433 (p. 537); business:427 (p. 537); business:435 (p. 537)	Linked RAD detail; exact locators in manifest.

Capability Overview

Care Support Work Coordination organises authorised operational support work around care: what needs doing, who will perform it and how progress, completion or escalation is coordinated. It gives supporting activity an explicit progression responsibility, rather than assuming that issuing a request means the work occurred. The work remains bounded by applicable authority and resource constraints.

Definition

Coordinate authorised operational support work: what needs doing, who performs it and how progression, completion or escalation is coordinated.

Boundary

Dispatch does not establish worker availability, qualification or privilege, and support coordination does not determine clinical work.

2.3. CT1: Health Rights, Advocacy & Participation

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-rights-advocacy-participation`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT1	Explicit source assertion.
Ancestry	CT1 root	Explicitly established.
Structural Canonical ID	`CT1-HEALTH-RIGHTS-ADVOCACY-PARTICIPATION`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Adjacent	Independent of status; never inherited.
Source	business:437 (p. 537); business:441 (p. 537); business:120 (p. 531)	Linked RAD detail; exact locators in manifest.

Capability Overview

This capability represents the organisation's responsibility to protect patients' rights and enable their voice in health decisions. Consent, advance care directives, advocacy and consumer participation concern the legitimacy and personal consequences of care, rather than merely the availability of clinical information. The governed model retains this broad enterprise responsibility without an established subordinate capability set.

Definition

Protecting patient rights, facilitating informed consent, managing advance care directives, supporting patient advocacy, and enabling consumer participation in health decisions.

Intent

Support informed participation and respect for the individual's rights in decisions affecting their health.

Boundary

Recording a directive or restricting access can support rights without performing advocacy or managing consumer participation.

Harmonia Architectural Relevance

The established Adjacent classification reflects the use and supply of consent and record-access evidence. It does not assign the underlying rights, advocacy or participation activity to Harmonia.

2.4. CT1: Diagnostic, Therapeutic & Clinical Support Services

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-diagnostic-therapeutic-clinical-support-services`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT1	Explicit source assertion.
Ancestry	CT1 root	Explicitly established.
Structural Canonical ID	`CT1-DIAGNOSTIC-THERAPEUTIC-CLINICAL-SUPPORT-SERVICES`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Harmonia-Relevant	Independent of status; never inherited.
Source	business:447 (p. 537); business:451 (p. 537); business:120 (p. 531)	Linked RAD detail; exact locators in manifest.

Capability Overview

Specialised services contribute evidence, professional interpretation, medication provision and therapeutic support to individual care. The established decomposition distinguishes diagnostic provision from medication and pharmacy services, while retaining wider therapeutic and allied-health scope as enterprise context. The catalogue deliberately explains the modelled branches without presenting them as an exhaustive inventory of clinical support.

Definition

The specialised clinical services that support direct patient care, including pathology and laboratory testing, diagnostic imaging (radiology), pharmacy and medication dispensing, and allied health therapies.

Intent

Provide the specialised contributions that treating participants need to assess and care for individuals.

Boundary

Producing diagnostic evidence, authorising a report, dispensing medication and administering treatment establish different outcomes and authorities.

Harmonia Architectural Relevance

Requests, specimen associations, reports and medication events require reliable correlation and exchange. Supporting those information flows does not make Harmonia the diagnostic practitioner, pharmacy or therapy provider.

Scope

Contained Capabilities

Capability	Reference Id	Tier
CT2: Diagnostic Support Provision (p. 32)	`HCM-BC-diagnostic-support-provision`	CT2
CT2: Medication & Pharmacy Services (p. 38)	`HCM-BC-medication-pharmacy-services`	CT2

2.4.1. CT2: Diagnostic Support Provision

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-diagnostic-support-provision`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	CT1: Diagnostic, Therapeutic & Clinical Support Services (p. 31)	Established BC responsibility path.
Structural Canonical ID	`CT1-DIAGNOSTIC-THERAPEUTIC-CLINICAL-SUPPORT-SERVICES.CT2-DIAGNOSTIC-SUPPORT-PROVISION`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:457 (p. 537); business:459 (p. 537); business:461 (p. 537); business:475 (p. 538)	Linked RAD detail; exact locators in manifest.

Capability Overview

Diagnostic Support Provision covers the distinct responsibilities through which a diagnostic service obtains evidence and makes its findings available. Diagnostic scheduling, service performance, specimen stewardship, data management, interpretation, report governance and report distribution each establish a different outcome. Their separation allows preliminary, amended or final information to retain its professional and evidentiary standing without treating delivery as authorisation.

Definition

Provide diagnostic services and manage their evidence, interpretation and governed reporting.

Intent

Make diagnostic evidence and professionally governed findings usable by the appropriate care participants.

Boundary

Diagnostic interpretation is limited to the service's professional scope; patient-level Clinical Determination considers the wider clinical context. The children do not prescribe a universal diagnostic workflow or establish a Diagnostic Order Management capability.

Scope

Contained Capabilities

Capability	Reference Id	Tier
CT3: Diagnostic Service Appointment Management (p. 33)	`HCM-BC-diagnostic-service-appointment-management`	CT3
CT3: Diagnostic Service Delivery (p. 34)	`HCM-BC-diagnostic-service-delivery`	CT3
CT3: Diagnostic Specimen Management (p. 35)	`HCM-BC-diagnostic-specimen-management`	CT3
CT3: Diagnostic Data Management (p. 35)	`HCM-BC-diagnostic-data-management`	CT3
CT3: Diagnostic Interpretation (p. 36)	`HCM-BC-diagnostic-interpretation`	CT3
CT3: Diagnostic Report Management (p. 37)	`HCM-BC-diagnostic-report-management`	CT3
CT3: Diagnostic Report Distribution (p. 37)	`HCM-BC-diagnostic-report-distribution`	CT3

2.4.1.1. CT3: Diagnostic Service Appointment Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-diagnostic-service-appointment-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Diagnostic, Therapeutic & Clinical Support Services (p. 31) → CT2: Diagnostic Support Provision (p. 32)	Established BC responsibility path.
Structural Canonical ID	`CT1-DIAGNOSTIC-THERAPEUTIC-CLINICAL-SUPPORT-SERVICES.CT2-DIAGNOSTIC-SUPPORT-PROVISION.CT3-DIAGNOSTIC-SERVICE-APPOINTMENT-MANAGEMENT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:467 (p. 538); business:463 (p. 537); business:475 (p. 538)	Linked RAD detail; exact locators in manifest.

Capability Overview

Diagnostic Service Appointment Management makes a scheduled diagnostic opportunity credible in the service's own context. Preparation, modality, urgency and duration can materially

affect whether an appointment is usable, so the responsibility is richer than recording a generic time slot. It supports service access while remaining separate from performance of the diagnostic activity.

Definition

Manage the diagnostic-service-specific scheduled opportunity, including preparation, modality, urgency, duration and other service-specific constraints.

Boundary

A diagnostic booking establishes neither evidence acquisition nor a report and is distinct from generic Care Appointment Management.

2.4.1.2. CT3: Diagnostic Service Delivery

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-diagnostic-service-delivery`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Diagnostic, Therapeutic & Clinical Support Services (p. 31) → CT2: Diagnostic Support Provision (p. 32)	Established BC responsibility path.
Structural Canonical ID	`CT1-DIAGNOSTIC-THERAPEUTIC-CLINICAL-SUPPORT-SERVICES.CT2-DIAGNOSTIC-SUPPORT-PROVISION.CT3-DIAGNOSTIC-SERVICE-DELIVERY`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:468 (p. 538); business:463 (p. 537); business:475 (p. 538)	Linked RAD detail; exact locators in manifest.

Capability Overview

Diagnostic Service Delivery performs the activity that obtains or produces diagnostic evidence. Within Diagnostic Support Provision it is the point at which the diagnostic procedure actually occurs, rather than being scheduled or requested. The evidence obtained can then be managed and interpreted under the distinct responsibilities that give it professional meaning.

Definition

Perform the diagnostic activity that obtains or produces diagnostic evidence.

Boundary

Performance does not itself establish interpretation, report authorisation or report delivery.

2.4.1.3. CT3: Diagnostic Specimen Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-diagnostic-specimen-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Diagnostic, Therapeutic & Clinical Support Services (p. 31) → CT2: Diagnostic Support Provision (p. 32)	Established BC responsibility path.
Structural Canonical ID	`CT1-DIAGNOSTIC-THERAPEUTIC-CLINICAL-SUPPORT-SERVICES.CT2-DIAGNOSTIC-SUPPORT-PROVISION.CT3-DIAGNOSTIC-SPECIMEN-MANAGEMENT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:469 (p. 538); business:463 (p. 537); business:475 (p. 538)	Linked RAD detail; exact locators in manifest.

Capability Overview

Diagnostic Specimen Management preserves the specimen context needed to trust its use: which specimen it is, whether it remains suitable, and how integrity and custody are maintained. It concerns the physical diagnostic subject across its applicable lifecycle rather than only the results generated from it. This distinction protects the relationship between evidence and the material from which it arose.

Definition

Maintain a specimen's identity, integrity, suitability, custody and applicable lifecycle context.

Boundary

Specimen stewardship is separate from diagnostic-data management and interpretation of results.

2.4.1.4. CT3: Diagnostic Data Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-diagnostic-data-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Diagnostic, Therapeutic & Clinical Support Services (p. 31) → CT2: Diagnostic Support Provision (p. 32)	Established BC responsibility path.
Structural Canonical ID	`CT1-DIAGNOSTIC-THERAPEUTIC-CLINICAL-SUPPORT-SERVICES.CT2-DIAGNOSTIC-SUPPORT-PROVISION.CT3-DIAGNOSTIC-DATA-MANAGEMENT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:470 (p. 538); business:463 (p. 537); business:475 (p. 538)	Linked RAD detail; exact locators in manifest.

Capability Overview

Diagnostic Data Management maintains acquired or produced diagnostic evidence as usable information. Its responsibility includes identity, association, integrity, organisation and lifecycle so that evidence can be found and understood in the right diagnostic context. It addresses the evidence record independently of the professional conclusion subsequently drawn from it.

Definition

Manage acquired or produced diagnostic evidence as information, including identity, association, integrity, organisation, availability and lifecycle.

Boundary

Managing diagnostic data neither interprets that data nor governs the authoritative expression of findings in a report.

2.4.1.5. CT3: Diagnostic Interpretation

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-diagnostic-interpretation`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Diagnostic, Therapeutic & Clinical Support Services (p. 31) → CT2: Diagnostic Support Provision (p. 32)	Established BC responsibility path.
Structural Canonical ID	`CT1-DIAGNOSTIC-THERAPEUTIC-CLINICAL-SUPPORT-SERVICES.CT2-DIAGNOSTIC-SUPPORT-PROVISION.CT3-DIAGNOSTIC-INTERPRETATION`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:471 (p. 538); business:463 (p. 537); business:475 (p. 538)	Linked RAD detail; exact locators in manifest.

Capability Overview

Diagnostic Interpretation establishes what diagnostic evidence demonstrates within the service's professional scope. It turns evidence into diagnostic findings without claiming the wider patient-level judgement that considers all clinical circumstances. Separating interpretation from acquisition and reporting makes the professional contribution clear even when one service performs all three.

Definition

Establish what diagnostic evidence demonstrates within the diagnostic service's professional scope.

Boundary

Patient-level Clinical Determination remains distinct, as do report authorisation and distribution.

2.4.1.6. CT3: Diagnostic Report Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-diagnostic-report-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Diagnostic, Therapeutic & Clinical Support Services (p. 31) → CT2: Diagnostic Support Provision (p. 32)	Established BC responsibility path.
Structural Canonical ID	`CT1-DIAGNOSTIC-THERAPEUTIC-CLINICAL-SUPPORT-SERVICES.CT2-DIAGNOSTIC-SUPPORT-PROVISION.CT3-DIAGNOSTIC-REPORT-MANAGEMENT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:472 (p. 538); business:463 (p. 537); business:475 (p. 538)	Linked RAD detail; exact locators in manifest.

Capability Overview

Diagnostic Report Management governs the authoritative expression of diagnostic findings. Creation, review, verification, amendment, finalisation and signing matter because they establish the standing of the report rather than merely its existence as data. The responsibility preserves a meaningful distinction between evidence, interpretation and the report that a service is prepared to authorise.

Definition

Establish and govern the authoritative expression of diagnostic findings, including applicable creation, review, amendment, verification, finalisation and authorisation/signing.

Boundary

Signing is behaviour within this capability, not another CT3. Sending a report does not establish its authorisation.

2.4.1.7. CT3: Diagnostic Report Distribution

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-diagnostic-report-distribution`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Diagnostic, Therapeutic & Clinical Support Services (p. 31) → CT2: Diagnostic Support Provision (p. 32)	Established BC responsibility path.
Structural Canonical ID	`CT1-DIAGNOSTIC-THERAPEUTIC-CLINICAL-SUPPORT-SERVICES.CT2-DIAGNOSTIC-SUPPORT-PROVISION.CT3-DIAGNOSTIC-REPORT-DISTRIBUTION`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:473 (p. 538); business:463 (p. 537); business:475 (p. 538)	Linked RAD detail; exact locators in manifest.

Capability Overview

Diagnostic Report Distribution provides an already established report to applicable authorised recipients or consuming contexts. Its purpose is to make the governed findings available where they are needed, while respecting access, privacy, security and communication controls. Distribution retains the report's standing rather than originating or silently reinterpreting its findings.

Definition

Provide an established report to applicable authorised recipients or consuming contexts under applicable access, privacy, security and communication controls.

Boundary

Delivery does not establish clinical review, report authorisation or a patient-level clinical decision.

2.4.2. CT2: Medication & Pharmacy Services

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-medication-pharmacy-services`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	CT1: Diagnostic, Therapeutic & Clinical Support Services (p. 31)	Established BC responsibility path.
Structural Canonical ID	`CT1-DIAGNOSTIC-THERAPEUTIC-CLINICAL-SUPPORT-SERVICES.CT2-MEDICATION-PHARMACY-SERVICES`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:477 (p. 538); business:479 (p. 538); business:481 (p. 538); business:492 (p. 538); business:1184 (p. 557)	Linked RAD detail; exact locators in manifest.

Capability Overview

This capability frames professional medication provision and medication-specific advice. Prescription dispensing, non-prescription provision, advice and dose administration aids answer different needs, even where exercised by one pharmacy. The decomposition keeps the authorised regimen, provision of medicine and actual administration separate so that a supply event is not mistaken for treatment performed.

Definition

Provide professional medication/pharmacy services under applicable controls.

Intent

Support safe, professionally controlled access to and use of medication within the established pharmacy responsibility.

Boundary

Advice does not confer prescribing authority, and dose aids do not establish a regimen. Whether specialised compounding/preparation warrants a separate capability remains unresolved under OI-BC-001.

Scope

Contained Capabilities

Capability	Reference Id	Tier
CT3: Prescription Medication Dispensing (p. 39)	`HCM-BC-prescription-medication-dispensing`	CT3
CT3: Non-Prescription Medication Provision (p. 40)	`HCM-BC-non-prescription-medication-provision`	CT3
CT3: Medication Advice (p. 40)	`HCM-BC-medication-advice`	CT3
CT3: Medication Dose Administration Aid Provision (p. 41)	`HCM-BC-medication-dose-administration-aid-provision`	CT3

Additional Notes / Unresolved Matters

Medication Compounding: OI-BC-001: a compounding/preparation CT3 is neither established nor rejected; do not infer subsumption.

2.4.2.1. CT3: Prescription Medication Dispensing

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-prescription-medication-dispensing`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Diagnostic, Therapeutic & Clinical Support Services (p. 31) → CT2: Medication & Pharmacy Services (p. 38)	Established BC responsibility path.
Structural Canonical ID	`CT1-DIAGNOSTIC-THERAPEUTIC-CLINICAL-SUPPORT-SERVICES.CT2-MEDICATION-PHARMACY-SERVICES.CT3-PRESCRIPTION-MEDICATION-DISPENSING`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:487 (p. 538); business:483 (p. 538); business:492 (p. 538)	Linked RAD detail; exact locators in manifest.

Capability Overview

Prescription Medication Dispensing is the professional preparation and provision of medication against an applicable authorised prescription or order. It fulfils a pharmacy responsibility grounded in the prescribed intent and dispensing requirements. The resulting dispensing event provides different evidence from the prescribing decision and from later administration to the individual.

Definition

Professionally prepare and provide medication according to an applicable authorised prescription/order and dispensing requirements.

Boundary

Dispensing does not establish that medication was administered; unresolved specialised compounding must not be silently assigned here.

2.4.2.2. CT3: Non-Prescription Medication Provision

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-non-prescription-medication-provision`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Diagnostic, Therapeutic & Clinical Support Services (p. 31) → CT2: Medication & Pharmacy Services (p. 38)	Established BC responsibility path.
Structural Canonical ID	`CT1-DIAGNOSTIC-THERAPEUTIC-CLINICAL-SUPPORT-SERVICES.CT2-MEDICATION-PHARMACY-SERVICES.CT3-NON-PRESCRIPTION-MEDICATION-PROVISION`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:488 (p. 538); business:483 (p. 538); business:492 (p. 538)	Linked RAD detail; exact locators in manifest.

Capability Overview

Non-Prescription Medication Provision makes medication available where the applicable conditions do not require a prescription. It remains a professional, regulated and safety-constrained responsibility rather than an unrestricted supply transaction. Its distinction from prescription dispensing preserves the different authority basis under which provision occurs.

Definition

Provide medication not requiring an applicable prescription, subject to professional, regulatory and safety controls.

Boundary

Provision neither establishes diagnosis nor proves use or administration of the medication.

2.4.2.3. CT3: Medication Advice

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-medication-advice`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Diagnostic, Therapeutic & Clinical Support Services (p. 31) → CT2: Medication & Pharmacy Services (p. 38)	Established BC responsibility path.
Structural Canonical ID	`CT1-DIAGNOSTIC-THERAPEUTIC-CLINICAL-SUPPORT-SERVICES.CT2-MEDICATION-PHARMACY-SERVICES.CT3-MEDICATION-ADVICE`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.

Name	Value	Comment / Description
Source	business:489 (p. 538); business:483 (p. 538); business:492 (p. 538)	Linked RAD detail; exact locators in manifest.

Capability Overview

Medication Advice provides professional guidance about medication use and its precautions, interactions, administration, adherence or handling. It supports understanding of how medication can be used safely without turning every advisory interaction into a prescribing decision. Within pharmacy services, the advice outcome is distinct from provision of the medicine itself.

Definition

Provide professional medication-specific advice about appropriate use, precautions, interactions, administration, adherence or safe handling.

Boundary

Advice does not confer authority to diagnose, prescribe or alter treatment; detailed scope remains subject to refinement.

2.4.2.4. CT3: Medication Dose Administration Aid Provision

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-medication-dose-administration-aid-provision`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Diagnostic, Therapeutic & Clinical Support Services (p. 31) → CT2: Medication & Pharmacy Services (p. 38)	Established BC responsibility path.
Structural Canonical ID	`CT1-DIAGNOSTIC-THERAPEUTIC-CLINICAL-SUPPORT-SERVICES.CT2-MEDICATION-PHARMACY-SERVICES.CT3-MEDICATION-DOSE-ADMINISTRATION-AID-PROVISION`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:490 (p. 538); business:483 (p. 538); business:492 (p. 538)	Linked RAD detail; exact locators in manifest.

Capability Overview

This capability prepares and supplies organised doses to support administration under an already established medication regimen. Its purpose is to make that regimen easier to follow in practice while preserving the difference between organising doses and choosing therapy. The business responsibility is not defined by any particular commercial packaging arrangement.

Definition

Prepare and provide organised medication doses supporting administration according to an already established regimen.

Boundary

Preparing a dose aid does not establish the regimen or demonstrate that a dose was administered.

2.5. CT1: Health Products & Clinical Technology

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-products-clinical-technology`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT1	Explicit source assertion.
Ancestry	CT1 root	Explicitly established.
Structural Canonical ID	`CT1-HEALTH-PRODUCTS-CLINICAL-TECHNOLOGY`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Harmonia-Relevant	Independent of status; never inherited.
Source	business:494 (p. 538); business:498 (p. 539); business:120 (p. 531)	Linked RAD detail; exact locators in manifest.

Capability Overview

This capability manages health products and clinical technology as business concerns in the care environment. Its established clinical-technology branch distinguishes knowledge of the device estate, functional state, deployment, user enablement, integration and lifecycle disposition. Product procurement and supply remain relevant context without a newly inferred Health Product Management decomposition.

Definition

Manage health products and clinical technology in the care environment, with distinct device inventory, functional/release, deployment, utilisation, integration and lifecycle responsibilities. Product procurement and supply context is retained without inferring further Health Product Management decomposition.

Intent

Keep the technology context needed for safe service activity and information exchange coherent and governed.

Boundary

Device-specific enablement, clinical privilege and use in care are different concerns. The seven established device children do not resolve the separate maintenance-membership discrepancy.

Harmonia Architectural Relevance

Integration provision and device-data integration materially affect exchanged information and its care context. Supply-chain, maintenance and clinical-use authority are not transferred to Harmonia.

Scope

Contained Capabilities

Capability	Reference Id	Tier
CT2: Clinical Technology Management (p. 43)	`HCM-BC-clinical-technology-management`	CT2

2.5.1. CT2: Clinical Technology Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-technology-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	CT1: Health Products & Clinical Technology (p. 42)	Established BC responsibility path.
Structural Canonical ID	`CT1-HEALTH-PRODUCTS-CLINICAL-TECHNOLOGY.CT2-CLINICAL-TECHNOLOGY-MANAGEMENT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:502 (p. 539); business:504 (p. 539); business:506 (p. 539); business:520 (p. 539)	Linked RAD detail; exact locators in manifest.

Capability Overview

Clinical Technology Management treats a device as more than a data endpoint. Its responsibilities distinguish what device exists, what functionality and release it has, where and to whom it is deployed, how authorised users are supported, whether exchange is provisioned, what data is integrated and whether it should remain in service. These distinctions preserve the business meaning of the device estate beyond the subset of information used by Harmonia.

Definition

Manage clinical technology as a coherent business concern rather than only the device information Harmonia happens to use.

Intent

Maintain intelligible device capability, deployment and lifecycle context for clinical service use.

Boundary

Provisioning integration does not demonstrate that data has flowed. Monitoring, restoring device condition and deciding lifecycle disposition remain distinct; maintenance's tier membership requires adjudication.

Scope

Contained Capabilities

Capability	Reference Id	Tier
CT3: Clinical Device Inventory Management (p. 44)	`HCM-BC-clinical-device-inventory-management`	CT3
CT3: Clinical Device Feature & Release Management (p. 45)	`HCM-BC-clinical-device-feature-release-management`	CT3
CT3: Clinical Device Loan & Deployment Management (p. 45)	`HCM-BC-clinical-device-loan-deployment-management`	CT3
CT3: Clinical Device Utilisation Support (p. 46)	`HCM-BC-clinical-device-utilisation-support`	CT3
CT3: Clinical Device Integration Provision (p. 47)	`HCM-BC-clinical-device-integration-provision`	CT3
CT3: Clinical Device Data Integration (p. 47)	`HCM-BC-clinical-device-data-integration`	CT3
CT3: Clinical Device Lifecycle Management (p. 48)	`HCM-BC-clinical-device-lifecycle-management`	CT3

2.5.1.1. CT3: Clinical Device Inventory Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-device-inventory-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Health Products & Clinical Technology (p. 42) → CT2: Clinical Technology Management (p. 43)	Established BC responsibility path.
Structural Canonical ID	`CT1-HEALTH-PRODUCTS-CLINICAL-TECHNOLOGY.CT2-CLINICAL-TECHNOLOGY-MANAGEMENT.CT3-CLINICAL-DEVICE-INVENTORY-MANAGEMENT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:512 (p. 539); business:508 (p. 539); business:520 (p. 539)	Linked RAD detail; exact locators in manifest.

Capability Overview

Clinical Device Inventory Management establishes the managed identity and inventory context of the device estate. Specification, ownership and custody make it possible to distinguish which devices exist from their current release, deployment or use. That stable asset context supports subsequent technology-management responsibilities without claiming their different outcomes.

Definition

Establish what devices exist and their managed asset identity, specification, ownership/custody and inventory context.

Boundary

Inventory presence does not establish readiness, functional release state or current assignment to a care requirement.

2.5.1.2. CT3: Clinical Device Feature & Release Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-device-feature-release-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Health Products & Clinical Technology (p. 42) → CT2: Clinical Technology Management (p. 43)	Established BC responsibility path.
Structural Canonical ID	`CT1-HEALTH-PRODUCTS-CLINICAL-TECHNOLOGY.CT2-CLINICAL-TECHNOLOGY-MANAGEMENT.CT3-CLINICAL-DEVICE-FEATURE-RELEASE-MANAGEMENT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:513 (p. 539); business:508 (p. 539); business:520 (p. 539)	Linked RAD detail; exact locators in manifest.

Capability Overview

This capability governs the functional state of a device, including applicable software, firmware, configuration and enabled characteristics. Two instances of the same model may therefore have materially different capabilities in service. The responsibility makes those differences intelligible without reducing device governance to physical inventory or location.

Definition

Manage functional/release state, including applicable software, firmware, configuration and enabled device characteristics.

Boundary

Release state is distinct from maintenance condition, deployment and professional authority to use the device.

2.5.1.3. CT3: Clinical Device Loan & Deployment Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-device-loan-deployment-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Health Products & Clinical Technology (p. 42) → CT2: Clinical Technology Management (p. 43)	Established BC responsibility path.
Structural Canonical ID	`CT1-HEALTH-PRODUCTS-CLINICAL-TECHNOLOGY.CT2-CLINICAL-TECHNOLOGY-MANAGEMENT.CT3-CLINICAL-DEVICE-LOAN-DEPLOYMENT-MANAGEMENT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:514 (p. 539); business:508 (p. 539); business:520 (p. 539)	Linked RAD detail; exact locators in manifest.

Capability Overview

Clinical Device Loan & Deployment Management manages where devices are placed and the issue, assignment, return or loan context under which they are available. It connects an identifiable device with its operational deployment rather than assuming ownership or inventory presence explains its current use. This supports an intelligible account of the device estate as placements change.

Definition

Manage device loan, issue, assignment, placement, return and deployment.

Boundary

Deployment is not device-specific user enablement, clinical use or evidence of clinical privilege.

2.5.1.4. CT3: Clinical Device Utilisation Support

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-device-utilisation-support`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Health Products & Clinical Technology (p. 42) → CT2: Clinical Technology Management (p. 43)	Established BC responsibility path.
Structural Canonical ID	`CT1-HEALTH-PRODUCTS-CLINICAL-TECHNOLOGY.CT2-CLINICAL-TECHNOLOGY-MANAGEMENT.CT3-CLINICAL-DEVICE-UTILISATION-SUPPORT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:515 (p. 539); business:508 (p. 539); business:520 (p. 539)	Linked RAD detail; exact locators in manifest.

Capability Overview

Clinical Device Utilisation Support helps otherwise appropriately authorised users use a particular device safely and effectively. It represents the device-specific enablement needed beyond general professional qualifications. Within technology management it addresses the practical support for use, without making that support the clinical action performed with the device.

Definition

Enable and support otherwise appropriately authorised users to use the particular device safely and effectively.

Boundary

Professional qualification or privilege alone does not establish device-specific enablement; enablement does not confer clinical authority.

2.5.1.5. CT3: Clinical Device Integration Provision

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-device-integration-provision`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Health Products & Clinical Technology (p. 42) → CT2: Clinical Technology Management (p. 43)	Established BC responsibility path.
Structural Canonical ID	`CT1-HEALTH-PRODUCTS-CLINICAL-TECHNOLOGY.CT2-CLINICAL-TECHNOLOGY-MANAGEMENT.CT3-CLINICAL-DEVICE-INTEGRATION-PROVISION`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:516 (p. 539); business:508 (p. 539); business:520 (p. 539)	Linked RAD detail; exact locators in manifest.

Capability Overview

Clinical Device Integration Provision establishes the device capability and configuration required to participate in applicable information exchanges. Its outcome is an integration-ready arrangement, which must remain distinguishable from data actually acquired or exchanged. This allows an architect to discuss whether connectivity has been enabled without assuming that a particular clinical feed has operated successfully.

Definition

Establish and maintain the device's capability and configuration to participate in applicable information exchanges.

Boundary

Provisioning does not demonstrate data integration, clinical interpretation or downstream authority for the data.

2.5.1.6. CT3: Clinical Device Data Integration

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-device-data-integration`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Health Products & Clinical Technology (p. 42) → CT2: Clinical Technology Management (p. 43)	Established BC responsibility path.
Structural Canonical ID	`CT1-HEALTH-PRODUCTS-CLINICAL-TECHNOLOGY.CT2-CLINICAL-TECHNOLOGY-MANAGEMENT.CT3-CLINICAL-DEVICE-DATA-INTEGRATION`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:517 (p. 539); business:508 (p. 539); business:520 (p. 539)	Linked RAD detail; exact locators in manifest.

Capability Overview

Clinical Device Data Integration acquires, exchanges or conveys device data using an established integration capability. It represents actual information participation rather than the prior configuration enabling it. The distinction makes it possible to preserve the device and care context of received data while retaining separate responsibility for its subsequent interpretation.

Definition

Acquire, exchange or convey device data through established integration capability.

Boundary

Data movement does not confer clinical meaning, originating authority or device-maintenance responsibility.

2.5.1.7. CT3: Clinical Device Lifecycle Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-device-lifecycle-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Health Products & Clinical Technology (p. 42) → CT2: Clinical Technology Management (p. 43)	Established BC responsibility path.
Structural Canonical ID	`CT1-HEALTH-PRODUCTS-CLINICAL-TECHNOLOGY.CT2-CLINICAL-TECHNOLOGY-MANAGEMENT.CT3-CLINICAL-DEVICE-LIFECYCLE-MANAGEMENT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:518 (p. 539); business:508 (p. 539); business:520 (p. 539)	Linked RAD detail; exact locators in manifest.

Capability Overview

Clinical Device Lifecycle Management governs whether devices enter, remain in or leave the managed estate. Introduction, review, replacement, withdrawal and retirement concern the device's overall service disposition. This gives technology management a longer-lived responsibility than the device's current loan, release or data-exchange session.

Definition

Govern introduction, continued service, review, replacement, withdrawal, decommissioning and retirement within the overall device estate.

Boundary

Lifecycle disposition differs from monitoring and restoring device condition; the maintenance-membership discrepancy remains unresolved.

2.6. CT1: Clinical Quality, Safety & Improvement

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-quality-safety-improvement`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT1	Explicit source assertion.
Ancestry	CT1 root	Explicitly established.
Structural Canonical ID	`CT1-CLINICAL-QUALITY-SAFETY-IMPROVEMENT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Harmonia-Relevant	Independent of status; never inherited.
Source	business:522 (p. 539); business:526 (p. 539); business:120 (p. 531)	Linked RAD detail; exact locators in manifest.

Capability Overview

This capability addresses operational clinical quality and safety, including concerns, incidents, response and improvement. Its established safety branch distinguishes risk identification, prospective mitigation and active response. Its quality branch distinguishes criteria, evidence, assessment and reporting. Together these make it possible to reason about clinical quality without equating an observation, a judgement and the action taken in response.

Definition

Operational clinical quality, safety and improvement: monitoring clinical outcomes; identifying quality and safety concerns; adverse event and incident management, reporting and response; infection control; operational quality and safety monitoring; continuous quality improvement; and coordination of operational responses to identified concerns.

Intent

Support recognition and management of clinical safety concerns and informed improvement of clinical quality.

Boundary

This is neither a generic enterprise incident container nor a replacement for macro-level Health Service Assurance. Its bounded decomposition does not exhaust clinical quality and safety practice.

Harmonia Architectural Relevance

Information about outcomes, risks, incidents and operational responses materially supports these responsibilities. Integrating that evidence does not assign clinical quality judgement or originating clinical authority to the platform.

Scope

Contained Capabilities

Capability	Reference Id	Tier
CT2: Clinical Safety Management (p. 50)	`HCM-BC-clinical-safety-management`	CT2
CT2: Clinical Quality Management (p. 53)	`HCM-BC-clinical-quality-management`	CT2

2.6.1. CT2: Clinical Safety Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-safety-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	CT1: Clinical Quality, Safety & Improvement (p. 49)	Established BC responsibility path.
Structural Canonical ID	`CT1-CLINICAL-QUALITY-SAFETY-IMPROVEMENT.CT2-CLINICAL-SAFETY-MANAGEMENT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:530 (p. 539); business:532 (p. 539); business:534 (p. 539); business:544 (p. 540)	Linked RAD detail; exact locators in manifest.

Capability Overview

Clinical Safety Management distinguishes recognising a safety risk from reducing future exposure and actively dealing with a developing or materialised concern. These responsibilities may use overlapping evidence, but their purposes differ: characterisation makes the risk intelligible, mitigation changes prospective risk, and response addresses the situation requiring action.

Definition

Manage the clinical-risk distinctions materially required for R1/R2.

Intent

Support proportionate safety action grounded in the nature of the identified clinical risk.

Boundary

Monitoring evidence is not automatically risk identification. Other capabilities can perform required actions without transferring their responsibilities; the three children are not a mandatory sequence or exhaustive safety taxonomy.

Scope

Contained Capabilities

Capability	Reference Id	Tier
CT3: Clinical Risk Identification (p. 51)	`HCM-BC-clinical-risk-identification`	CT3
CT3: Clinical Risk Mitigation Management (p. 51)	`HCM-BC-clinical-risk-mitigation-management`	CT3
CT3: Clinical Risk Response Management (p. 52)	`HCM-BC-clinical-risk-response-management`	CT3

2.6.1.1. CT3: Clinical Risk Identification

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-risk-identification`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Clinical Quality, Safety & Improvement (p. 49) → CT2: Clinical Safety Management (p. 50)	Established BC responsibility path.
Structural Canonical ID	`CT1-CLINICAL-QUALITY-SAFETY-IMPROVEMENT.CT2-CLINICAL-SAFETY-MANAGEMENT.CT3-CLINICAL-RISK-IDENTIFICATION`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:540 (p. 540); business:536 (p. 540); business:544 (p. 540)	Linked RAD detail; exact locators in manifest.

Capability Overview

Clinical Risk Identification recognises and characterises a safety risk from relevant evidence. It establishes the concern to which mitigation or active response may be directed, rather than assuming that every monitored deviation is already a clinical risk. Its value is the meaningful characterisation that makes proportionate safety consideration possible.

Definition

Recognise and characterise a clinical safety risk from applicable evidence.

Boundary

Evidence observation alone does not identify risk or establish a mitigation or response decision.

2.6.1.2. CT3: Clinical Risk Mitigation Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-risk-mitigation-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Clinical Quality, Safety & Improvement (p. 49) → CT2: Clinical Safety Management (p. 50)	Established BC responsibility path.
Structural Canonical ID	`CT1-CLINICAL-QUALITY-SAFETY-IMPROVEMENT.CT2-CLINICAL-SAFETY-MANAGEMENT.CT3-CLINICAL-RISK-MITIGATION-MANAGEMENT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:541 (p. 540); business:536 (p. 540); business:544 (p. 540)	Linked RAD detail; exact locators in manifest.

Capability Overview

Clinical Risk Mitigation Management addresses prospective measures intended to reduce an identified risk's likelihood, exposure or consequences. It gives prevention a distinct responsibility alongside risk characterisation and active response. Required measures can involve other capabilities without transferring the ownership of their actions or information to safety management.

Definition

Manage prospective/preventative measures reducing identified risk likelihood, exposure or consequence.

Boundary

Mitigation differs from containing or remediating a concern that has materialised or requires active intervention.

2.6.1.3. CT3: Clinical Risk Response Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-risk-response-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Clinical Quality, Safety & Improvement (p. 49) → CT2: Clinical Safety Management (p. 50)	Established BC responsibility path.
Structural Canonical ID	`CT1-CLINICAL-QUALITY-SAFETY-IMPROVEMENT.CT2-CLINICAL-SAFETY-MANAGEMENT.CT3-CLINICAL-RISK-RESPONSE-MANAGEMENT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:542 (p. 540); business:536 (p. 540); business:544 (p. 540)	Linked RAD detail; exact locators in manifest.

Capability Overview

Clinical Risk Response Management manages active intervention, containment or remediation where a safety risk is emerging, materialised or otherwise demands action. Its purpose is to address the developing situation rather than only reduce future exposure. The responsibility remains clinical-risk-specific while the actual actions may belong to other established abilities.

Definition

Manage active response, containment and remediation where risk has materialised, is emerging or requires active intervention.

Boundary

It is not a general enterprise incident-management container and does not acquire the clinical authority of responding participants.

2.6.2. CT2: Clinical Quality Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-quality-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	CT1: Clinical Quality, Safety & Improvement (p. 49)	Established BC responsibility path.
Structural Canonical ID	`CT1-CLINICAL-QUALITY-SAFETY-IMPROVEMENT.CT2-CLINICAL-QUALITY-MANAGEMENT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:546 (p. 540); business:548 (p. 540); business:550 (p. 540); business:561 (p. 540)	Linked RAD detail; exact locators in manifest.

Capability Overview

Clinical Quality Management supplies the criteria and evidence basis for evaluating clinical quality and communicating established findings. Its four children preserve the difference between the expected standard, the material considered, the assessment reached and the report of that assessment. This supports scrutiny of why a quality conclusion is justified rather than treating the presence of measurements as a judgement.

Definition

Manage the clinical-quality distinctions materially required for R1/R2.

Intent

Make clinical-quality conclusions understandable in relation to their criteria, evidence and assessment authority.

Boundary

Reporting does not originate the conclusion or perform subsequent improvement. Neither a separate monitoring provider nor generic Continuous Improvement placement is established by this decomposition.

Scope

Contained Capabilities

Capability	Reference Id	Tier
CT3: Clinical Quality Criteria Management (p. 54)	`HCM-BC-clinical-quality-criteria-management`	CT3
CT3: Clinical Quality Evidence Management (p. 54)	`HCM-BC-clinical-quality-evidence-management`	CT3
CT3: Clinical Quality Review & Assessment Management (p. 55)	`HCM-BC-clinical-quality-review-assessment-management`	CT3
CT3: Clinical Quality Reporting (p. 56)	`HCM-BC-clinical-quality-reporting`	CT3

2.6.2.1. CT3: Clinical Quality Criteria Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-quality-criteria-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Clinical Quality, Safety & Improvement (p. 49) → CT2: Clinical Quality Management (p. 53)	Established BC responsibility path.
Structural Canonical ID	`CT1-CLINICAL-QUALITY-SAFETY-IMPROVEMENT.CT2-CLINICAL-QUALITY-MANAGEMENT.CT3-CLINICAL-QUALITY-CRITERIA-MANAGEMENT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:556 (p. 540); business:552 (p. 540); business:561 (p. 540)	Linked RAD detail; exact locators in manifest.

Capability Overview

Clinical Quality Criteria Management establishes the measures, indicators and expectations used to consider clinical quality. It makes the basis of a later judgement explicit so that evidence can be assessed against an identifiable expectation rather than an unstated standard. Criteria are distinct from the measurements or findings produced when they are applied.

Definition

Establish and manage measures, indicators, expectations and criteria against which clinical quality is considered.

Boundary

Managing criteria does not originate the underlying evidence or establish a quality conclusion.

2.6.2.2. CT3: Clinical Quality Evidence Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-quality-evidence-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Clinical Quality, Safety & Improvement (p. 49) → CT2: Clinical Quality Management (p. 53)	Established BC responsibility path.
Structural Canonical ID	`CT1-CLINICAL-QUALITY-SAFETY-IMPROVEMENT.CT2-CLINICAL-QUALITY-MANAGEMENT.CT3-CLINICAL-QUALITY-EVIDENCE-MANAGEMENT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:557 (p. 540); business:552 (p. 540); business:561 (p. 540)	Linked RAD detail; exact locators in manifest.

Capability Overview

Clinical Quality Evidence Management maintains the body of material applicable to quality consideration. Its purpose is to retain evidence in a form and context that can support review without treating availability as proof of quality. The originating activity and its authority remain distinguishable from the later quality purpose for which its information is used.

Definition

Manage the body of applicable evidence used for clinical-quality consideration without owning its originating activity.

Boundary

Evidentiary use transfers neither source ownership nor authority to determine the activity being reviewed.

2.6.2.3. CT3: Clinical Quality Review & Assessment Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-quality-review-assessment-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Clinical Quality, Safety & Improvement (p. 49) → CT2: Clinical Quality Management (p. 53)	Established BC responsibility path.
Structural Canonical ID	`CT1-CLINICAL-QUALITY-SAFETY-IMPROVEMENT.CT2-CLINICAL-QUALITY-MANAGEMENT.CT3-CLINICAL-QUALITY-REVIEW-ASSESSMENT-MANAGEMENT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:558 (p. 540); business:552 (p. 540); business:561 (p. 540)	Linked RAD detail; exact locators in manifest.

Capability Overview

This capability evaluates quality evidence against applicable criteria and establishes a finding, assessment or conclusion. It is the judgement-bearing responsibility within Clinical Quality Management, distinct from maintaining the material and from communicating the result. Making that distinction supports scrutiny of the criterion and evidentiary basis for a quality assessment.

Definition

Review evidence against applicable criteria and establish a clinical-quality assessment, finding or conclusion.

Boundary

An assessment is not a report of itself or the subsequent improvement action it may inform.

2.6.2.4. CT3: Clinical Quality Reporting

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-quality-reporting`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Clinical Quality, Safety & Improvement (p. 49) → CT2: Clinical Quality Management (p. 53)	Established BC responsibility path.
Structural Canonical ID	`CT1-CLINICAL-QUALITY-SAFETY-IMPROVEMENT.CT2-CLINICAL-QUALITY-MANAGEMENT.CT3-CLINICAL-QUALITY-REPORTING`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:559 (p. 540); business:552 (p. 540); business:561 (p. 540)	Linked RAD detail; exact locators in manifest.

Capability Overview

Clinical Quality Reporting communicates established quality measures, assessments and findings to authorised consumers. It makes the outcome of quality consideration usable beyond the assessment context while retaining its basis and standing. Reporting therefore represents a communication responsibility, not another opportunity to originate or silently change the reported judgement.

Definition

Communicate established clinical-quality measures, assessments, findings and conclusions to authorised consumers.

Boundary

Communicating a quality concern does not perform the operational or clinical improvement response.

2.7. CT1: Community Health & Wellbeing

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-community-health-wellbeing`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT1	Explicit source assertion.
Ancestry	CT1 root	Explicitly established.
Structural Canonical ID	`CT1-COMMUNITY-HEALTH-WELLBEING`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Harmonia-Relevant	Independent of status; never inherited.
Source	business:563 (p. 540); business:567 (p. 540); business:120 (p. 531)	Linked RAD detail; exact locators in manifest.

Capability Overview

Community Health & Wellbeing frames organised activity supporting health beyond a single person's episode of care. The governed decomposition focuses on preventative health: identifying needs, defining programmes and cohorts, educating, observing programme operation and reporting. Wider outreach, environmental and social-determinant concerns remain enterprise context rather than additional modelled children.

Definition

Support community health and wellbeing, including the bounded preventative-health abilities required for R1/R2. Wider environmental health, outreach and social determinants remain enterprise context without inferred additional decomposition.

Intent

Support organised prevention with a clear relationship between the health problem, intended population and programme activity.

Boundary

Programme management does not acquire the clinical or appointment responsibilities needed to deliver a programme.

Harmonia Architectural Relevance

Programme and cohort distinctions materially shape the information, notifications and care enablement required. This supports the Relevant classification without assigning wider public-health operating authority to Harmonia.

Scope

Contained Capabilities

Capability	Reference Id	Tier
CT2: Preventative Health (p. 57)	`HCM-BC-preventative-health`	CT2

2.7.1. CT2: Preventative Health

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-preventative-health`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	CT1: Community Health & Wellbeing (p. 56)	Established BC responsibility path.
Structural Canonical ID	`CT1-COMMUNITY-HEALTH-WELLBEING.CT2-PREVENTATIVE-HEALTH`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:571 (p. 540); business:573 (p. 540); business:575 (p. 540); business:588 (p. 541)	Linked RAD detail; exact locators in manifest.

Capability Overview

Preventative Health organises a response to a health problem before adverse outcomes arise or worsen. The established children distinguish why attention is warranted, how an organised programme is managed, who it targets, the education provided, what programme operation shows and what is reported. This allows a screening or immunisation programme to be explained without creating a new capability for every programme type.

Definition

Address preventative-health needs through organised activity with sufficient R1/R2 distinctions.

Intent

Connect a preventative purpose with an identifiable target population and observable organised activity.

Boundary

Programme performance differs from population-health assessment and individual clinical monitoring. Unmodelled specialist responsibilities are not silently assigned to the existing children.

Scope

Contained Capabilities

Capability	Reference Id	Tier
CT3: Preventative Health Problem Identification (p. 58)	`HCM-BC-preventative-health-problem-identification`	CT3
CT3: Preventative Health Programme Management (p. 59)	`HCM-BC-preventative-health-programme-management`	CT3
CT3: Preventative Health Education Services (p. 60)	`HCM-BC-preventative-health-education-services`	CT3
CT3: Preventative Health Programme Monitoring (p. 60)	`HCM-BC-preventative-health-programme-monitoring`	CT3
CT3: Preventative Health Reporting (p. 61)	`HCM-BC-preventative-health-reporting`	CT3
CT3: Preventative Health Cohort Management (p. 62)	`HCM-BC-preventative-health-cohort-management`	CT3

2.7.1.1. CT3: Preventative Health Problem Identification

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-preventative-health-problem-identification`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Community Health & Wellbeing (p. 56) → CT2: Preventative Health (p. 57)	Established BC responsibility path.
Structural Canonical ID	`CT1-COMMUNITY-HEALTH-WELLBEING.CT2-PREVENTATIVE-HEALTH.CT3-PREVENTATIVE-HEALTH-PROBLEM-IDENTIFICATION`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.

Name	Value	Comment / Description
Source	business:581 (p. 541); business:577 (p. 541); business:588 (p. 541)	Linked RAD detail; exact locators in manifest.

Capability Overview

This capability establishes the preventative-health problem warranting organised attention. It gives an intended programme a reason grounded in the concern being addressed rather than assuming that a population statistic already defines the response. The identified problem can frame subsequent cohort and programme considerations without becoming an individual's clinical diagnosis.

Definition

Identify and characterise a preventative-health problem warranting organised attention.

Boundary

Population evidence informs problem identification but does not by itself establish a programme, cohort or individual care need.

2.7.1.2. CT3: Preventative Health Programme Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-preventative-health-programme-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Community Health & Wellbeing (p. 56) → CT2: Preventative Health (p. 57)	Established BC responsibility path.
Structural Canonical ID	`CT1-COMMUNITY-HEALTH-WELLBEING.CT2-PREVENTATIVE-HEALTH.CT3-PREVENTATIVE-HEALTH-PROGRAMME-MANAGEMENT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:582 (p. 541); business:577 (p. 541); business:588 (p. 541)	Linked RAD detail; exact locators in manifest.

Capability Overview

Preventative Health Programme Management establishes and manages an organised response to a preventative need or objective. It keeps the programme purpose and coordinated activity intelligible while allowing appointments, interventions and other delivery abilities to retain their own responsibilities. Screening and immunisation illustrate programmes rather than creating additional capability tiers.

Definition

Establish and manage an organised response to a preventative-health need/objective without absorbing its delivery capabilities.

Boundary

Managing the programme does not acquire the clinical or operational authority of the activities delivering it.

2.7.1.3. CT3: Preventative Health Education Services

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-preventative-health-education-services`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Community Health & Wellbeing (p. 56) → CT2: Preventative Health (p. 57)	Established BC responsibility path.
Structural Canonical ID	`CT1-COMMUNITY-HEALTH-WELLBEING.CT2-PREVENTATIVE-HEALTH.CT3-PREVENTATIVE-HEALTH-EDUCATION-SERVICES`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:583 (p. 541); business:577 (p. 541); business:588 (p. 541)	Linked RAD detail; exact locators in manifest.

Capability Overview

Preventative Health Education Services provides education intended to influence knowledge or behaviour so that adverse health outcomes can be prevented. Its purpose distinguishes it from general organisational communication: the educational contribution is directed to a preventative-health need. It can support organised programme activity without being equivalent to programme management or clinical intervention.

Definition

Provide education intended to prevent adverse health outcomes and influence preventative knowledge or behaviour.

Boundary

Communicating educational material does not establish individual clinical assessment, treatment or programme effectiveness.

2.7.1.4. CT3: Preventative Health Programme Monitoring

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-preventative-health-programme-monitoring`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Community Health & Wellbeing (p. 56) → CT2: Preventative Health (p. 57)	Established BC responsibility path.
Structural Canonical ID	`CT1-COMMUNITY-HEALTH-WELLBEING.CT2-PREVENTATIVE-HEALTH.CT3-PREVENTATIVE-HEALTH-PROGRAMME-MONITORING`	Allocated v0.1; structural responsibility identity.

Name	Value	Comment / Description
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:584 (p. 541); business:577 (p. 541); business:588 (p. 541)	Linked RAD detail; exact locators in manifest.

Capability Overview

This capability observes how a preventative programme is operating, including reach, participation, coverage and progress. It supplies an account of programme performance in the programme's own scope, rather than substituting population-health state for evidence of programme activity. That account can inform reporting or consideration of the programme without itself being an assurance conclusion.

Definition

Observe programme operation, reach, participation, coverage, progress and applicable performance.

Boundary

Programme monitoring is distinct from individual health monitoring, population assessment and independent assurance.

2.7.1.5. CT3: Preventative Health Reporting

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-preventative-health-reporting`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Community Health & Wellbeing (p. 56) → CT2: Preventative Health (p. 57)	Established BC responsibility path.
Structural Canonical ID	`CT1-COMMUNITY-HEALTH-WELLBEING.CT2-PREVENTATIVE-HEALTH.CT3-PREVENTATIVE-HEALTH-REPORTING`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:585 (p. 541); business:577 (p. 541); business:588 (p. 541)	Linked RAD detail; exact locators in manifest.

Capability Overview

Preventative Health Reporting communicates established information about preventative problems, programmes, cohorts, activity and outcomes. It brings these different subjects together for a reporting purpose while preserving their distinct meanings. A programme's reach and an observed population outcome can both be relevant without implying that one proves the other.

Definition

Report established preventative-health problems, programmes, cohorts, activity, performance and outcomes.

Boundary

Reporting does not originate the underlying problem, cohort or evaluation conclusion.

2.7.1.6. CT3: Preventative Health Cohort Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-preventative-health-cohort-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Community Health & Wellbeing (p. 56) → CT2: Preventative Health (p. 57)	Established BC responsibility path.
Structural Canonical ID	`CT1-COMMUNITY-HEALTH-WELLBEING.CT2-PREVENTATIVE-HEALTH.CT3-PREVENTATIVE-HEALTH-COHORT-MANAGEMENT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:586 (p. 541); business:577 (p. 541); business:588 (p. 541)	Linked RAD detail; exact locators in manifest.

Capability Overview

Preventative Health Cohort Management establishes who belongs to the population targeted for a preventative purpose. Inclusion, exclusion, stratification and entry or exit context make membership explainable rather than a list detached from its rationale. It provides a governed population context through which programme activity and reporting can be understood.

Definition

Establish and manage the target population, inclusion/exclusion criteria, membership, stratification and applicable entry/exit context.

Boundary

A population at risk is not automatically a defined cohort, and cohort membership is not an individual clinical determination.

2.8. CT1: Population Health & Health-System Planning

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-population-health-health-system-planning`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT1	Explicit source assertion.
Ancestry	CT1 root	Explicitly established.
Structural Canonical ID	`CT1-POPULATION-HEALTH-HEALTH-SYSTEM-PLANNING`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Harmonia-Relevant	Independent of status; never inherited.

Name	Value	Comment / Description
Source	business:590 (p. 541); business:594 (p. 541); business:120 (p. 531)	Linked RAD detail; exact locators in manifest.

Capability Overview

This capability connects an understanding of population health with consideration of future health-service needs. Surveillance addresses population state, significant events and notification; planning assesses service demand and required capacity and distribution. Population evidence can inform planning without predetermining its conclusions or becoming the operational allocation of resources.

Definition

Understand population health through assessment/surveillance and applicable event/notification management; assess health-service demand and plan required capacity and its distribution.

Intent

Support informed public-health awareness and a reasoned view of the services a population will require.

Boundary

Population conclusions differ from individual clinical findings, and planned capacity differs from capacity currently usable by a service.

Harmonia Architectural Relevance

Governed population information and formal notifications materially support health activity. Consuming planning context does not make the platform the health-system planner or owner of its resources.

Scope

Contained Capabilities

Capability	Reference Id	Tier
CT2: Population Health Surveillance (p. 63)	`HCM-BC-population-health-surveillance`	CT2
CT2: Health-System Planning (p. 68)	`HCM-BC-health-system-planning`	CT2

2.8.1. CT2: Population Health Surveillance

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-population-health-surveillance`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.

Name	Value	Comment / Description
Ancestry	CT1: Population Health & Health-System Planning (p. 62)	Established BC responsibility path.
Structural Canonical ID	`CT1-POPULATION-HEALTH-HEALTH-SYSTEM-PLANNING.CT2-POPULATION-HEALTH-SURVEILLANCE`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:598 (p. 541); business:600 (p. 541); business:602 (p. 541); business:614 (p. 542)	Linked RAD detail; exact locators in manifest.

Capability Overview

Population Health Surveillance establishes patterns and change across a defined population and manages the surveillance arrangements through which they are considered. Its children separate longitudinal assessment, surveillance management, event identification, formal notification and reporting. These distinctions matter because an observed pattern can be significant without being notifiable, and identifying a notifiable event does not demonstrate that the notification obligation has been discharged.

Definition

Understand population-health state and manage organised surveillance, significant events and applicable notification.

Intent

Make population-health concerns and applicable communication obligations visible in their population and temporal context.

Boundary

Notification management does not define the obligation or own the operational response. Surveillance evidence can inform other abilities without establishing their findings.

Scope

Contained Capabilities

Capability	Reference Id	Tier
CT3: Population Health Longitudinal Assessment (p. 65)	`HCM-BC-population-health-longitudinal-assessment`	CT3
CT3: Population Health Surveillance Management (p. 65)	`HCM-BC-population-health-surveillance-management`	CT3
CT3: Population Health Event Identification (p. 66)	`HCM-BC-population-health-event-identification`	CT3
CT3: Notifiable Health Event Management (p. 67)	`HCM-BC-notifiable-health-event-management`	CT3
CT3: Population Health Reporting (p. 67)	`HCM-BC-population-health-reporting`	CT3

Additional Notes / Unresolved Matters

Semantic Relationship Representation: Canonical BC evidence/context/consumption semantics are preserved in Markdown; the current governed RADS relationship types do not establish their machine-readable encoding. No stronger runtime or structural relation is substituted.

2.8.1.1. CT3: Population Health Longitudinal Assessment

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-population-health-longitudinal-assessment`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Population Health & Health-System Planning (p. 62) → CT2: Population Health Surveillance (p. 63)	Established BC responsibility path.
Structural Canonical ID	`CT1-POPULATION-HEALTH-HEALTH-SYSTEM-PLANNING.CT2-POPULATION-HEALTH-SURVEILLANCE.CT3-POPULATION-HEALTH-LONGITUDINAL-ASSESSMENT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:608 (p. 541); business:604 (p. 541); business:614 (p. 542)	Linked RAD detail; exact locators in manifest.

Capability Overview

This capability establishes population-health state, patterns and change over time from relevant evidence. It interprets information at a population scope rather than treating a collection of individual observations as a population conclusion by itself. Its longitudinal purpose makes the temporal and population basis important to the meaning of the assessment.

Definition

Establish population-health state, patterns and change over time from applicable evidence.

Boundary

Population assessment is not preventative-programme performance, an individual health conclusion or a service-demand decision.

2.8.1.2. CT3: Population Health Surveillance Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-population-health-surveillance-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Population Health & Health-System Planning (p. 62) → CT2: Population Health Surveillance (p. 63)	Established BC responsibility path.
Structural Canonical ID	`CT1-POPULATION-HEALTH-HEALTH-SYSTEM-PLANNING.CT2-POPULATION-HEALTH-SURVEILLANCE.CT3-POPULATION-HEALTH-SURVEILLANCE-MANAGEMENT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:609 (p. 541); business:604 (p. 541); business:614 (p. 542)	Linked RAD detail; exact locators in manifest.

Capability Overview

Population Health Surveillance Management establishes the purposes, populations and regimes under which surveillance operates. It concerns the organised basis for watching health concerns, rather than any particular event detected through that activity. Keeping the regime distinct from its findings makes it possible to understand why evidence is being collected and considered.

Definition

Establish and manage surveillance purposes, populations, concerns and regimes.

Boundary

Managing surveillance neither identifies every significant event nor determines the operational response to one.

2.8.1.3. CT3: Population Health Event Identification

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-population-health-event-identification`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Population Health & Health-System Planning (p. 62) → CT2: Population Health Surveillance (p. 63)	Established BC responsibility path.
Structural Canonical ID	`CT1-POPULATION-HEALTH-HEALTH-SYSTEM-PLANNING.CT2-POPULATION-HEALTH-SURVEILLANCE.CT3-POPULATION-HEALTH-EVENT-IDENTIFICATION`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:610 (p. 541); business:604 (p. 541); business:614 (p. 542)	Linked RAD detail; exact locators in manifest.

Capability Overview

Population Health Event Identification recognises and characterises a significant event or pattern from relevant evidence. It establishes the population-level concern rather than merely noting that observations changed. The resulting event context can inform notification or response decisions while retaining a separate responsibility for those decisions.

Definition

Identify and characterise significant population-health events/patterns from applicable evidence.

Boundary

An identified event is not automatically notifiable, and notifiability does not demonstrate that an obligation was discharged.

2.8.1.4. CT3: Notifiable Health Event Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-notifiable-health-event-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Population Health & Health-System Planning (p. 62) → CT2: Population Health Surveillance (p. 63)	Established BC responsibility path.
Structural Canonical ID	`CT1-POPULATION-HEALTH-HEALTH-SYSTEM-PLANNING.CT2-POPULATION-HEALTH-SURVEILLANCE.CT3-NOTIFIABLE-HEALTH-EVENT-MANAGEMENT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:611 (p. 541); business:604 (p. 541); business:614 (p. 542)	Linked RAD detail; exact locators in manifest.

Capability Overview

Notifiable Health Event Management addresses the formal communication responsibilities applicable to a health event. It manages applicability, required information, recipient and submission status, including correction or supplementation where relevant. Its purpose is to make the notification obligation and its discharge distinguishable from both event identification and ordinary reporting.

Definition

Manage formal notification applicability, required information, recipient, status, submission, correction/supplementation and applicable follow-up.

Boundary

Notification management does not define the governing obligation or own the operational response to the event.

2.8.1.5. CT3: Population Health Reporting

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-population-health-reporting`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Population Health & Health-System Planning (p. 62) → CT2: Population Health Surveillance (p. 63)	Established BC responsibility path.
Structural Canonical ID	`CT1-POPULATION-HEALTH-HEALTH-SYSTEM-PLANNING.CT2-POPULATION-HEALTH-SURVEILLANCE.CT3-POPULATION-HEALTH-REPORTING`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:612 (p. 541); business:604 (p. 541); business:614 (p. 542)	Linked RAD detail; exact locators in manifest.

Capability Overview

Population Health Reporting communicates established assessments, surveillance findings, events and trends. Its role is to make population understanding available with its scope intact, rather than replace the assessment or event-identification work underlying it. This supports use of the findings by other abilities without assuming what those abilities must conclude.

Definition

Communicate established population-health assessments, surveillance, events, trends and findings.

Boundary

General reporting is distinct from formal notification management and does not establish a response or planning decision.

2.8.2. CT2: Health-System Planning

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-system-planning`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	CT1: Population Health & Health-System Planning (p. 62)	Established BC responsibility path.
Structural Canonical ID	`CT1-POPULATION-HEALTH-HEALTH-SYSTEM-PLANNING.CT2-HEALTH-SYSTEM-PLANNING`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:616 (p. 542); business:618 (p. 542); business:620 (p. 542); business:630 (p. 542)	Linked RAD detail; exact locators in manifest.

Capability Overview

Health-System Planning considers the capacity and service footprint required to meet assessed demand. Demand Assessment establishes the basis and scale of need, Capacity Planning determines what is required, and Distribution Planning addresses where that capacity should sit across the service network. The decomposition preserves the difference between understanding demand and deciding a response to it.

Definition

Assess health-service demand and plan the capacity and footprint required to meet it.

Intent

Support a defensible relationship between expected demand and the planned scale and distribution of health services.

Boundary

Plans do not establish available resources or operational allocations. Detailed commissioning, capital, funding and workforce planning remain deliberately unmodelled.

Scope

Contained Capabilities

Capability	Reference Id	Tier
CT3: Health Service Demand Assessment (p. 69)	`HCM-BC-health-service-demand-assessment`	CT3
CT3: Health Service Capacity Planning (p. 70)	`HCM-BC-health-service-capacity-planning`	CT3
CT3: Health Service Distribution Planning (p. 70)	`HCM-BC-health-service-distribution-planning`	CT3

Additional Notes / Unresolved Matters

Semantic Relationship Representation: Canonical BC evidence/context/consumption semantics are preserved in Markdown; the current governed RADS relationship types do not establish their machine-readable encoding. No stronger runtime or structural relation is substituted.

2.8.2.1. CT3: Health Service Demand Assessment

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-service-demand-assessment`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Population Health & Health-System Planning (p. 62) → CT2: Health-System Planning (p. 68)	Established BC responsibility path.
Structural Canonical ID	`CT1-POPULATION-HEALTH-HEALTH-SYSTEM-PLANNING.CT2-HEALTH-SYSTEM-PLANNING.CT3-HEALTH-SERVICE-DEMAND-ASSESSMENT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:626 (p. 542); business:622 (p. 542); business:630 (p. 542)	Linked RAD detail; exact locators in manifest.

Capability Overview

Health Service Demand Assessment establishes a view of current or expected demand within a population, geography or service context. It translates relevant evidence into a demand judgement rather than assuming population-health state directly determines service requirements. Its assumptions and scope matter because capacity planning depends on what demand has actually been assessed.

Definition

Assess current/expected health-service demand across a population, geography, service domain or health-system context.

Boundary

System-level demand is distinct from a particular person's care requirement and does not itself determine planned capacity.

2.8.2.2. CT3: Health Service Capacity Planning

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-service-capacity-planning`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Population Health & Health-System Planning (p. 62) → CT2: Health-System Planning (p. 68)	Established BC responsibility path.
Structural Canonical ID	`CT1-POPULATION-HEALTH-HEALTH-SYSTEM-PLANNING.CT2-HEALTH-SYSTEM-PLANNING.CT3-HEALTH-SERVICE-CAPACITY-PLANNING`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:627 (p. 542); business:622 (p. 542); business:630 (p. 542)	Linked RAD detail; exact locators in manifest.

Capability Overview

Health Service Capacity Planning determines the strategic or tactical capacity required to address assessed demand. It considers what should be available, preserving a distinction between requirement and the resources currently in service. Within Health-System Planning it gives demand a capacity response without performing operational allocation.

Definition

Determine required strategic/tactical capacity to address assessed demand.

Boundary

Planned capacity is neither established resource availability nor the service's currently usable operational capacity.

2.8.2.3. CT3: Health Service Distribution Planning

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-service-distribution-planning`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Population Health & Health-System Planning (p. 62) → CT2: Health-System Planning (p. 68)	Established BC responsibility path.
Structural Canonical ID	`CT1-POPULATION-HEALTH-HEALTH-SYSTEM-PLANNING.CT2-HEALTH-SYSTEM-PLANNING.CT3-HEALTH-SERVICE-DISTRIBUTION-PLANNING`	Allocated v0.1; structural responsibility identity.

Name	Value	Comment / Description
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:628 (p. 542); business:622 (p. 542); business:630 (p. 542)	Linked RAD detail; exact locators in manifest.

Capability Overview

Health Service Distribution Planning determines how planned capacity should be distributed across a geographical, organisational or service-network footprint. It addresses the placement of the planned response rather than only its aggregate scale. This allows the planning context to distinguish enough total capacity from capacity positioned where it is needed.

Definition

Determine the distribution of planned capacity across a geographical, organisational, site or service-network footprint.

Boundary

A planned footprint does not establish recognised location records, identifiable offerings or actual resource deployment.

2.9. CT1: Health Information & Knowledge Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-information-knowledge-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT1	Explicit source assertion.
Ancestry	CT1 root	Explicitly established.
Structural Canonical ID	`CT1-HEALTH-INFORMATION-KNOWLEDGE-MANAGEMENT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Harmonia-Core	Independent of status; never inherited.
Source	business:636 (p. 542); business:640 (p. 542); business:120 (p. 531)	Linked RAD detail; exact locators in manifest.

Capability Overview

This capability governs both health information and reusable clinical knowledge while preserving their different meanings. Health Information Management concerns content, structures, lifecycle, discovery context, grouping, correlation and retrieval. Clinical Knowledge Management concerns identifying and formulating reusable knowledge and establishing its governed standing. A source clinical fact can contribute to knowledge without becoming a general clinical conclusion merely through reuse.

Definition

Manage governed health-information content, structures, lifecycle, catalogues, compartments, correlation and retrieval, alongside reusable clinical knowledge and its governed standing.

Intent

Keep information interpretable and usable across time and context while making the standing and limits of reusable knowledge explicit.

Boundary

Managing or preserving information does not originate its clinical facts. Governing structures and binding concepts does not acquire the authority of the terminology they reference.

Harmonia Architectural Relevance

These are substantive information-management responsibilities within Harmonia's assigned boundary. The Core classification remains bounded by information authority, active/durable-state separation and clinical decision authority.

Scope

Contained Capabilities

Capability	Reference Id	Tier
CT2: Health Information Management (p. 72)	`HCM-BC-health-information-management`	CT2
CT2: Clinical Knowledge Management (p. 78)	`HCM-BC-clinical-knowledge-management`	CT2

2.9.1. CT2: Health Information Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-information-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	CT1: Health Information & Knowledge Management (p. 71)	Established BC responsibility path.
Structural Canonical ID	`CT1-HEALTH-INFORMATION-KNOWLEDGE-MANAGEMENT.CT2-HEALTH-INFORMATION-MANAGEMENT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:644 (p. 542); business:646 (p. 542); business:648 (p. 542); business:662 (p. 543)	Linked RAD detail; exact locators in manifest.

Capability Overview

Health Information Management maintains health information as meaningful governed content rather than isolated persisted values. Its children distinguish the content itself from the structures that explain it, lifecycle and history, discovery aids, contextual groupings, relationships and retrieval. These distinctions let an architect ask whether information exists, what it means, which state applies and whether it may be obtained without conflating those questions.

Definition

Manage health information as governed, semantically meaningful content.

Intent

Preserve usable health information whose meaning, relationships and temporal standing can be understood.

Boundary

Discoverability is not permission, compartment membership is not an access decision, and correlation is not identity or clinical interpretation. Current state and immutable historical operational truth remain distinct.

Scope

Contained Capabilities

Capability	Reference Id	Tier
CT3: Health Information Data Management (p. 73)	`HCM-BC-health-information-data-management`	CT3
CT3: Health Information Structure Management (p. 74)	`HCM-BC-health-information-structure-management`	CT3
CT3: Health Information Lifecycle Management (p. 75)	`HCM-BC-health-information-lifecycle-management`	CT3
CT3: Health Information Catalogue and Index Management (p. 75)	`HCM-BC-health-information-catalogue-and-index-management`	CT3
CT3: Health Information Compartmentalisation (p. 76)	`HCM-BC-health-information-compartmentalisation`	CT3
CT3: Health Information Data Correlation (p. 77)	`HCM-BC-health-information-data-correlation`	CT3
CT3: Health Information Data Retrieval (p. 77)	`HCM-BC-health-information-data-retrieval`	CT3

Additional Notes / Unresolved Matters

Semantic Relationship Representation: Canonical BC evidence/context/consumption semantics are preserved in Markdown; the current governed RADS relationship types do not establish their machine-readable encoding. No stronger runtime or structural relation is substituted.

2.9.1.1. CT3: Health Information Data Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-information-data-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Health Information & Knowledge Management (p. 71) → CT2: Health Information Management (p. 72)	Established BC responsibility path.
Structural Canonical ID	`CT1-HEALTH-INFORMATION-KNOWLEDGE-MANAGEMENT.CT2-HEALTH-INFORMATION-MANAGEMENT.CT3-HEALTH-INFORMATION-DATA-MANAGEMENT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:654 (p. 542); business:650 (p. 542); business:662 (p. 543)	Linked RAD detail; exact locators in manifest.

Capability Overview

Health Information Data Management concerns the actual content maintained within applicable governed structures. It keeps the represented information distinct from definitions describing how that information should be organised and interpreted. Within Health Information Management, its responsibility is meaningful content rather than mere successful persistence of values.

Definition

Manage actual health-information content within applicable governed structures.

Boundary

Managing content does not originate its clinical facts or acquire the authority of its semantic references.

2.9.1.2. CT3: Health Information Structure Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-information-structure-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Health Information & Knowledge Management (p. 71) → CT2: Health Information Management (p. 72)	Established BC responsibility path.
Structural Canonical ID	`CT1-HEALTH-INFORMATION-KNOWLEDGE-MANAGEMENT.CT2-HEALTH-INFORMATION-MANAGEMENT.CT3-HEALTH-INFORMATION-STRUCTURE-MANAGEMENT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:655 (p. 542); business:650 (p. 542); business:662 (p. 543)	Linked RAD detail; exact locators in manifest.

Capability Overview

This capability manages the structures, element definitions, references and bindings that make content interpretable. It addresses how a represented item can have an unambiguous meaning without treating the structure as the clinical fact itself. Semantic bindings identify applicable governed references while preserving their external authority.

Definition

Manage structures, element/reference definitions, constraints and semantic bindings giving content unambiguous meaning.

Boundary

Structure management is not terminology governance and does not prescribe FHIR resources, physical schemas or storage technology.

2.9.1.3. CT3: Health Information Lifecycle Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-information-lifecycle-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Health Information & Knowledge Management (p. 71) → CT2: Health Information Management (p. 72)	Established BC responsibility path.
Structural Canonical ID	`CT1-HEALTH-INFORMATION-KNOWLEDGE-MANAGEMENT.CT2-HEALTH-INFORMATION-MANAGEMENT.CT3-HEALTH-INFORMATION-LIFECYCLE-MANAGEMENT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:656 (p. 542); business:650 (p. 542); business:662 (p. 543)	Linked RAD detail; exact locators in manifest.

Capability Overview

Health Information Lifecycle Management governs how information's state and temporal standing change through amendment, correction, supersession, retention and disposition. It allows current knowledge to change without retrospectively rewriting what was previously received, known or managed. That distinction supports interpretation of both the present representation and its historical operational meaning.

Definition

Govern state, amendment, correction, supersession, currency, retention and disposition while preserving provenance and historical meaning.

Boundary

Current authoritative state differs from historical truth; preserving either does not confer originating authority for the represented facts.

2.9.1.4. CT3: Health Information Catalogue and Index Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-information-catalogue-and-index-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Health Information & Knowledge Management (p. 71) → CT2: Health Information Management (p. 72)	Established BC responsibility path.
Structural Canonical ID	`CT1-HEALTH-INFORMATION-KNOWLEDGE-MANAGEMENT.CT2-HEALTH-INFORMATION-MANAGEMENT.CT3-HEALTH-INFORMATION-CATALOGUE-AND-INDEX-MANAGEMENT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:657 (p. 542); business:650 (p. 542); business:662 (p. 543)	Linked RAD detail; exact locators in manifest.

Capability Overview

This capability maintains the descriptive aids through which information existence and location can be established. It makes information discoverable without equating the catalogue entry with the content it describes. Within information management it answers where relevant information may be found before the separate question of obtaining it under an authorised context.

Definition

Manage descriptive catalogues/indexes through which information existence and location can be established.

Boundary

Discoverability does not authorise access, retrieve the content or communicate it to a recipient.

2.9.1.5. CT3: Health Information Compartmentalisation

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-information-compartmentalisation`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Health Information & Knowledge Management (p. 71) → CT2: Health Information Management (p. 72)	Established BC responsibility path.
Structural Canonical ID	`CT1-HEALTH-INFORMATION-KNOWLEDGE-MANAGEMENT.CT2-HEALTH-INFORMATION-MANAGEMENT.CT3-HEALTH-INFORMATION-COMPARTMENTALISATION`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:658 (p. 542); business:650 (p. 542); business:662 (p. 543)	Linked RAD detail; exact locators in manifest.

Capability Overview

Health Information Compartmentalisation establishes contextual associations and bounded groupings of information. It makes the relationship of information to a business context explicit, allowing groupings to carry meaning beyond physical storage location. That context can inform governed use while remaining distinct from sensitivity classification and a particular access decision.

Definition

Manage contextual association, partitioning and bounded information groupings.

Boundary

Compartment membership neither establishes permission nor replaces classification or terminology governance.

2.9.1.6. CT3: Health Information Data Correlation

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-information-data-correlation`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Health Information & Knowledge Management (p. 71) → CT2: Health Information Management (p. 72)	Established BC responsibility path.
Structural Canonical ID	`CT1-HEALTH-INFORMATION-KNOWLEDGE-MANAGEMENT.CT2-HEALTH-INFORMATION-MANAGEMENT.CT3-HEALTH-INFORMATION-DATA-CORRELATION`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:659 (p. 542); business:650 (p. 542); business:662 (p. 543)	Linked RAD detail; exact locators in manifest.

Capability Overview

Health Information Data Correlation establishes meaningful relationships between independently managed information items or sets. It helps information be understood together without requiring a merger of their identities, histories or originating authority. The responsibility is the association itself, rather than a clinical conclusion inferred from the associated content.

Definition

Establish meaningful relationships between independently managed information items/sets.

Boundary

Correlation does not establish person identity, clinical interpretation or ownership transfer between the related items.

2.9.1.7. CT3: Health Information Data Retrieval

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-information-data-retrieval`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Health Information & Knowledge Management (p. 71) → CT2: Health Information Management (p. 72)	Established BC responsibility path.
Structural Canonical ID	`CT1-HEALTH-INFORMATION-KNOWLEDGE-MANAGEMENT.CT2-HEALTH-INFORMATION-MANAGEMENT.CT3-HEALTH-INFORMATION-DATA-RETRIEVAL`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:660 (p. 542); business:650 (p. 542); business:662 (p. 543)	Linked RAD detail; exact locators in manifest.

Capability Overview

Health Information Data Retrieval obtains identified information under an applicable authorised request and context. It supplies the content whose existence may already be known through a catalogue or index. The distinction matters because locating an item, obtaining it and presenting it to a person each establish different outcomes.

Definition

Obtain identified information under an applicable authorised request/context.

Boundary

Retrieval does not itself grant permission, provide participant communication or confer authority over the retrieved facts.

2.9.2. CT2: Clinical Knowledge Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-knowledge-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	CT1: Health Information & Knowledge Management (p. 71)	Established BC responsibility path.
Structural Canonical ID	`CT1-HEALTH-INFORMATION-KNOWLEDGE-MANAGEMENT.CT2-CLINICAL-KNOWLEDGE-MANAGEMENT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:664 (p. 543); business:666 (p. 543); business:668 (p. 543); business:681 (p. 543)	Linked RAD detail; exact locators in manifest.

Capability Overview

Clinical Knowledge Management turns potentially useful source material into explicit reusable knowledge with an intelligible basis and governed standing. Context identification and extraction preserve where material came from; formulation expresses applicability and rationale; concept association supplies semantic context; review and publication govern its intended use. These responsibilities distinguish source information, formulated knowledge and approved knowledge without prescribing a compulsory workflow.

Definition

Identify, extract, formulate, associate, govern and share reusable clinical knowledge.

Intent

Make reusable clinical knowledge available with the qualifications and approval standing needed to judge its use.

Boundary

Publication does not establish applicability to an individual or a clinical decision. Concept association does not redefine terminology, and formulation does not constitute approval.

Scope

Contained Capabilities

Capability	Reference Id	Tier
CT3: Clinical Knowledge Context Identification (p. 79)	`HCM-BC-clinical-knowledge-context-identification`	CT3
CT3: Clinical Knowledge Context Extraction (p. 80)	`HCM-BC-clinical-knowledge-context-extraction`	CT3
CT3: Clinical Knowledge Information Formulation (p. 80)	`HCM-BC-clinical-knowledge-information-formulation`	CT3
CT3: Clinical Knowledge Concept Association (p. 81)	`HCM-BC-clinical-knowledge-concept-association`	CT3
CT3: Clinical Knowledge Review & Approval Management (p. 82)	`HCM-BC-clinical-knowledge-review-approval-management`	CT3
CT3: Clinical Knowledge Publication & Sharing (p. 83)	`HCM-BC-clinical-knowledge-publication-sharing`	CT3

Additional Notes / Unresolved Matters

Semantic Relationship Representation: Canonical BC evidence/context/consumption semantics are preserved in Markdown; the current governed RADS relationship types do not establish their machine-readable encoding. No stronger runtime or structural relation is substituted.

2.9.2.1. CT3: Clinical Knowledge Context Identification

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-knowledge-context-identification`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Health Information & Knowledge Management (p. 71) → CT2: Clinical Knowledge Management (p. 78)	Established BC responsibility path.
Structural Canonical ID	`CT1-HEALTH-INFORMATION-KNOWLEDGE-MANAGEMENT.CT2-CLINICAL-KNOWLEDGE-MANAGEMENT.CT3-CLINICAL-KNOWLEDGE-CONTEXT-IDENTIFICATION`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:674 (p. 543); business:670 (p. 543); business:681 (p. 543)	Linked RAD detail; exact locators in manifest.

Capability Overview

This capability identifies sources or contexts that may contain reusable clinical knowledge. It establishes where potentially useful material can be sought without assuming that all source information is already formulated knowledge. Within knowledge management it provides a contextual starting point rather than an approval or publication outcome.

Definition

Identify sources/contexts potentially containing reusable clinical knowledge.

Boundary

Identifying a source does not extract its material, establish reusable knowledge or determine its clinical applicability.

2.9.2.2. CT3: Clinical Knowledge Context Extraction

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-knowledge-context-extraction`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Health Information & Knowledge Management (p. 71) → CT2: Clinical Knowledge Management (p. 78)	Established BC responsibility path.
Structural Canonical ID	`CT1-HEALTH-INFORMATION-KNOWLEDGE-MANAGEMENT.CT2-CLINICAL-KNOWLEDGE-MANAGEMENT.CT3-CLINICAL-KNOWLEDGE-CONTEXT-EXTRACTION`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:675 (p. 543); business:670 (p. 543); business:681 (p. 543)	Linked RAD detail; exact locators in manifest.

Capability Overview

Clinical Knowledge Context Extraction selects relevant material while retaining its relationship to the source and context. Its purpose is to make potential knowledge material available without stripping away the circumstances needed to understand it. Extracted material remains distinguishable from the reusable knowledge subsequently formulated from it.

Definition

Extract relevant material while preserving its source/context relationship.

Boundary

Extraction does not establish approval or turn source information into an individual clinical conclusion.

2.9.2.3. CT3: Clinical Knowledge Information Formulation

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-knowledge-information-formulation`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.

Name	Value	Comment / Description
Ancestry	CT1: Health Information & Knowledge Management (p. 71) → CT2: Clinical Knowledge Management (p. 78)	Established BC responsibility path.
Structural Canonical ID	`CT1-HEALTH-INFORMATION-KNOWLEDGE-MANAGEMENT.CT2-CLINICAL-KNOWLEDGE-MANAGEMENT.CT3-CLINICAL-KNOWLEDGE-INFORMATION-FORMULATION`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:676 (p. 543); business:670 (p. 543); business:681 (p. 543)	Linked RAD detail; exact locators in manifest.

Capability Overview

This capability expresses reusable clinical knowledge with its applicability, qualifications, rationale and evidence relationships. It moves beyond extracted material by making the knowledge claim and its limits explicit. That formulation supports later review and use while retaining the distinction between a proposed knowledge expression and one approved for a governed purpose.

Definition

Formulate explicit reusable knowledge, including applicability, qualifications, rationale and evidence relationships.

Boundary

Formulation does not approve knowledge, govern terminology or define health-information structures.

2.9.2.4. CT3: Clinical Knowledge Concept Association

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-knowledge-concept-association`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Health Information & Knowledge Management (p. 71) → CT2: Clinical Knowledge Management (p. 78)	Established BC responsibility path.
Structural Canonical ID	`CT1-HEALTH-INFORMATION-KNOWLEDGE-MANAGEMENT.CT2-CLINICAL-KNOWLEDGE-MANAGEMENT.CT3-CLINICAL-KNOWLEDGE-CONCEPT-ASSOCIATION`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:677 (p. 543); business:670 (p. 543); business:681 (p. 543)	Linked RAD detail; exact locators in manifest.

Capability Overview

Clinical Knowledge Concept Association relates reusable knowledge to the governed clinical concepts relevant to its meaning. It makes the semantic context of the knowledge intelligible

without allowing local association to redefine the referenced concepts. The association assists interpretation while preserving the distinction between knowledge authority and concept authority.

Definition

Associate knowledge with applicable governed clinical concepts without acquiring concept authority.

Boundary

Associating knowledge with a terminology concept does not acquire terminology governance or establish approval of the knowledge.

2.9.2.5. CT3: Clinical Knowledge Review & Approval Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-knowledge-review-approval-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Health Information & Knowledge Management (p. 71) → CT2: Clinical Knowledge Management (p. 78)	Established BC responsibility path.
Structural Canonical ID	`CT1-HEALTH-INFORMATION-KNOWLEDGE-MANAGEMENT.CT2-CLINICAL-KNOWLEDGE-MANAGEMENT.CT3-CLINICAL-KNOWLEDGE-REVIEW-APPROVAL-MANAGEMENT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:678 (p. 543); business:670 (p. 543); business:681 (p. 543)	Linked RAD detail; exact locators in manifest.

Capability Overview

This capability manages the review and governed standing of reusable knowledge, including endorsement, rejection, conditions and history. It establishes how the knowledge may legitimately be treated for its intended use rather than assuming that formulation makes it trustworthy or approved. Temporal standing matters where a later review changes the conditions for use.

Definition

Manage review, endorsement, approval/rejection, conditions and applicable history for intended governed use.

Boundary

Approval does not establish applicability to a particular individual or replace the responsible clinical judgement.

2.9.2.6. CT3: Clinical Knowledge Publication & Sharing

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-knowledge-publication-sharing`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Health Information & Knowledge Management (p. 71) → CT2: Clinical Knowledge Management (p. 78)	Established BC responsibility path.
Structural Canonical ID	`CT1-HEALTH-INFORMATION-KNOWLEDGE-MANAGEMENT.CT2-CLINICAL-KNOWLEDGE-MANAGEMENT.CT3-CLINICAL-KNOWLEDGE-PUBLICATION-SHARING`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:679 (p. 543); business:670 (p. 543); business:681 (p. 543)	Linked RAD detail; exact locators in manifest.

Capability Overview

Clinical Knowledge Publication & Sharing makes approved knowledge available to applicable authorised consumers and contexts. It preserves the standing and limits under which knowledge was approved rather than reducing publication to unrestricted dissemination. Its contribution is availability for appropriate use, distinct from both approval and the individual clinical application that may follow.

Definition

Make approved knowledge available to applicable authorised consumers and use contexts.

Boundary

Published knowledge is not a patient-specific conclusion, decision or instruction merely because a clinician can access it.

2.10. CT1: Standards, Semantics & Reference Governance

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-standards-semantics-reference-governance`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT1	Explicit source assertion.
Ancestry	CT1 root	Explicitly established.
Structural Canonical ID	`CT1-STANDARDS-SEMANTICS-REFERENCE-GOVERNANCE`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Harmonia-Core	Independent of status; never inherited.
Source	business:683 (p. 543); business:687 (p. 544); business:120 (p. 531)	Linked RAD detail; exact locators in manifest.

Capability Overview

This capability establishes the organisational standing and appropriate use of models, standards and semantic artefacts. Standards governance addresses discovery, adoption, education, compliance and impacts; clinical-semantics governance addresses terminology, classifications and mappings. It provides the governed meaning within which information can be represented and exchanged, rather than merely maintaining technical schemas.

Definition

Govern organisational adoption, oversight and appropriate use of health-information models, standards, terminology, classifications and semantic mappings.

Intent

Support consistent, appropriate interpretation and use of health-information standards and semantic references.

Boundary

Using a terminology binding, mapping or classification does not transfer its governing authority. The established peer set must not be expanded simply to create a symmetrical taxonomy.

Harmonia Architectural Relevance

Semantic and conformance governance is a substantive platform responsibility. Its organisational meaning constrains technical representations; implementation products do not define that meaning.

Scope

Contained Capabilities

Capability	Reference Id	Tier
CT2: Health Information Standards Governance (p. 84)	`HCM-BC-health-information-standards-governance`	CT2
CT2: Clinical Semantics Governance (p. 90)	`HCM-BC-clinical-semantics-governance`	CT2

2.10.1. CT2: Health Information Standards Governance

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-information-standards-governance`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	CT1: Standards, Semantics & Reference Governance (p. 83)	Established BC responsibility path.
Structural Canonical ID	`CT1-STANDARDS-SEMANTICS-REFERENCE-GOVERNANCE.CT2-HEALTH-INFORMATION-STANDARDS-GOVERNANCE`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.

Name	Value	Comment / Description
Source	business:691 (p. 544); business:693 (p. 544); business:695 (p. 544); business:709 (p. 544)	Linked RAD detail; exact locators in manifest.

Capability Overview

This capability governs which information models and standards the organisation adopts and how their use is supported and evaluated. The model distinguishes discovering candidates, adopting models, educating users, addressing conformity and overseeing governance. Standards impact identification has a separate established place because its implications may extend into clinical practice, policy and obligations beyond information-system implementation.

Definition

Govern organisational adoption and use of health-information models and applicable standards.

Intent

Make organisational standards use deliberate and its consequences understandable.

Boundary

Oversight evaluates governance without performing every peer activity. Adoption includes its own model-impact consideration; the listed abilities do not establish a sequential adoption Process.

Scope

Contained Capabilities

Capability	Reference Id	Tier
CT3: Information Model Discovery (p. 85)	`HCM-BC-information-model-discovery`	CT3
CT3: Information Model Adoption (p. 86)	`HCM-BC-information-model-adoption`	CT3
CT3: Information Model Education (p. 87)	`HCM-BC-information-model-education`	CT3
CT3: Information Model Compliance (p. 87)	`HCM-BC-information-model-compliance`	CT3
CT3: Information Model Governance Oversight (p. 88)	`HCM-BC-information-model-governance-oversight`	CT3
CT3: Standards Adoption (p. 89)	`HCM-BC-standards-adoption`	CT3
CT3: Standards Impact Identification (p. 89)	`HCM-BC-standards-impact-identification`	CT3

2.10.1.1. CT3: Information Model Discovery

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-information-model-discovery`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Standards, Semantics & Reference Governance (p. 83) → CT2: Health Information Standards Governance (p. 84)	Established BC responsibility path.

Name	Value	Comment / Description
Structural Canonical ID	`CT1-STANDARDS-SEMANTICS-REFERENCE-GOVERNANCE .CT2-HEALTH-INFORMATION-STANDARDS-GOVERNANCE .CT3-INFORMATION-MODEL-DISCOVERY`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:701 (p. 544); business:697 (p. 544); business:709 (p. 544)	Linked RAD detail; exact locators in manifest.

Capability Overview

Information Model Discovery identifies candidate models relevant to the organisation's health-information needs. It supports informed standards governance by making applicable possibilities visible before adoption. Its business purpose is finding suitable semantic frames rather than selecting a software representation or assuming every discovered model should be used.

Definition

Identify information models relevant to the organisation's health-information needs.

Boundary

Discovery does not adopt a model, establish compliance or grant it organisational authority.

2.10.1.2. CT3: Information Model Adoption

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-information-model-adoption`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Standards, Semantics & Reference Governance (p. 83) → CT2: Health Information Standards Governance (p. 84)	Established BC responsibility path.
Structural Canonical ID	`CT1-STANDARDS-SEMANTICS-REFERENCE-GOVERNANCE .CT2-HEALTH-INFORMATION-STANDARDS-GOVERNANCE .CT3-INFORMATION-MODEL-ADOPTION`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:702 (p. 544); business:697 (p. 544); business:709 (p. 544)	Linked RAD detail; exact locators in manifest.

Capability Overview

Information Model Adoption establishes the organisation's use of an applicable information model, including consideration of the consequences of adopting it. It gives a discovered model an organisational standing distinct from mere availability. The intrinsic impact consideration helps adoption remain a business governance responsibility rather than a mechanical import of definitions.

Definition

Adopt applicable information models, including intrinsic assessment of adoption impacts.

Boundary

Adoption does not perform every education, compliance or governance-oversight responsibility associated with subsequent use.

2.10.1.3. CT3: Information Model Education

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-information-model-education`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Standards, Semantics & Reference Governance (p. 83) → CT2: Health Information Standards Governance (p. 84)	Established BC responsibility path.
Structural Canonical ID	`CT1-STANDARDS-SEMANTICS-REFERENCE-GOVERNANCE .CT2-HEALTH-INFORMATION-STANDARDS-GOVERNANCE .CT3-INFORMATION-MODEL-EDUCATION`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:703 (p. 544); business:697 (p. 544); business:709 (p. 544)	Linked RAD detail; exact locators in manifest.

Capability Overview

Information Model Education supports understanding and appropriate use of adopted models. It addresses the people and practice dimension of semantic consistency: a published model is not necessarily a model understood in the contexts where it is applied. Its responsibility therefore complements adoption and compliance without becoming either.

Definition

Support organisational understanding and appropriate use of adopted information models.

Boundary

Educational support neither changes the authoritative model nor establishes that actual use complies with it.

2.10.1.4. CT3: Information Model Compliance

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-information-model-compliance`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Standards, Semantics & Reference Governance (p. 83) → CT2: Health Information Standards Governance (p. 84)	Established BC responsibility path.
Structural Canonical ID	`CT1-STANDARDS-SEMANTICS-REFERENCE-GOVERNANCE .CT2-HEALTH-INFORMATION-STANDARDS-GOVERNANCE .CT3-INFORMATION-MODEL-COMPLIANCE`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.

Name	Value	Comment / Description
Source	business:704 (p. 544); business:697 (p. 544); business:709 (p. 544)	Linked RAD detail; exact locators in manifest.

Capability Overview

Information Model Compliance addresses conformity to applicable model requirements. It makes actual organisational use accountable to the adopted semantic frame rather than treating model availability or user education as proof of conformity. Within standards governance it concerns the requirements for use, not the separate task of discovering or adopting models.

Definition

Address organisational conformity to applicable information-model requirements.

Boundary

Conformity does not redefine the model or absorb governance oversight of the wider adoption arrangements.

2.10.1.5. CT3: Information Model Governance Oversight

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-information-model-governance-oversight`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Standards, Semantics & Reference Governance (p. 83) → CT2: Health Information Standards Governance (p. 84)	Established BC responsibility path.
Structural Canonical ID	`CT1-STANDARDS-SEMANTICS-REFERENCE-GOVERNANCE.CT2-HEALTH-INFORMATION-STANDARDS-GOVERNANCE.CT3-INFORMATION-MODEL-GOVERNANCE-OVERSIGHT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:705 (p. 544); business:697 (p. 544); business:709 (p. 544)	Linked RAD detail; exact locators in manifest.

Capability Overview

This capability evaluates the effectiveness and consistency of information-model governance itself. It asks whether discovery, adoption and use are being governed appropriately, rather than repeating each operational peer. Its purpose is coherent oversight of the arrangements that give models standing and guide their application.

Definition

Oversee and evaluate effectiveness, consistency and appropriate operation of information-model governance.

Boundary

Oversight does not itself perform discovery, adoption, education or compliance activities.

2.10.1.6. CT3: Standards Adoption

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-standards-adoption`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Standards, Semantics & Reference Governance (p. 83) → CT2: Health Information Standards Governance (p. 84)	Established BC responsibility path.
Structural Canonical ID	`CT1-STANDARDS-SEMANTICS-REFERENCE-GOVERNANCE .CT2-HEALTH-INFORMATION-STANDARDS-GOVERNANCE .CT3-STANDARDS-ADOPTION`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:706 (p. 544); business:697 (p. 544); business:709 (p. 544)	Linked RAD detail; exact locators in manifest.

Capability Overview

Standards Adoption establishes which applicable standards the organisation adopts. It gives standards an intentional place in organisational practice rather than allowing familiar products or conventions to define requirements by default. The business responsibility concerns adoption in context, including standards whose effects extend beyond information representation.

Definition

Adopt applicable standards within the organisation.

Boundary

Adoption does not establish a complete standards lifecycle taxonomy or perform the distinct identification of broader standards impacts.

2.10.1.7. CT3: Standards Impact Identification

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-standards-impact-identification`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Standards, Semantics & Reference Governance (p. 83) → CT2: Health Information Standards Governance (p. 84)	Established BC responsibility path.
Structural Canonical ID	`CT1-STANDARDS-SEMANTICS-REFERENCE-GOVERNANCE .CT2-HEALTH-INFORMATION-STANDARDS-GOVERNANCE .CT3-STANDARDS-IMPACT-IDENTIFICATION`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.

Name	Value	Comment / Description
Source	business:707 (p. 544); business:697 (p. 544); business:709 (p. 544)	Linked RAD detail; exact locators in manifest.

Capability Overview

Standards Impact Identification recognises the consequences of standards for relevant organisational activity. Impacts may concern clinical practice, medications, procedures, policy, safety, obligations or reporting, so the responsibility cannot be reduced to technical compatibility. It makes those consequences visible for the responsible capabilities to consider within their own authority.

Definition

Identify standards impacts across clinical/work practice, permitted medications, procedures, policy, safety, obligations and reporting as applicable.

Boundary

Identifying an impact neither adopts the standard nor acquires the operational authority needed to respond to it.

2.10.2. CT2: Clinical Semantics Governance

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-semantics-governance`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	CT1: Standards, Semantics & Reference Governance (p. 83)	Established BC responsibility path.
Structural Canonical ID	`CT1-STANDARDS-SEMANTICS-REFERENCE-GOVERNANCE.CT2-CLINICAL-SEMANTICS-GOVERNANCE`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:711 (p. 544); business:713 (p. 544); business:715 (p. 544); business:728 (p. 545)	Linked RAD detail; exact locators in manifest.

Capability Overview

Clinical Semantics Governance maintains the standing and appropriate use of terminology, classifications and semantic mappings. Terminology provides consistent meaning; classification concerns sensitivity and consequences of exposure or mishandling. The established children distinguish governing artefacts from overseeing actual use and permit controlled terminology localisation where adopted material does not meet legitimate needs.

Definition

Govern clinical meaning and appropriate organisational use of semantic artefacts.

Intent

Keep meaning and classification intelligible across organisational use without concealing semantic drift or local variation.

Boundary

Compartmentalisation concerns contextual relationships rather than classification or terminology. Classification Localisation is not an established peer; mappings must preserve the separate meanings and authorities they connect.

Scope

Contained Capabilities

Capability	Reference Id	Tier
CT3: Terminology Governance (p. 91)	`HCM-BC-terminology-governance`	CT3
CT3: Terminology Utilisation Oversight (p. 92)	`HCM-BC-terminology-utilisation-oversight`	CT3
CT3: Terminology Localisation (p. 92)	`HCM-BC-terminology-localisation`	CT3
CT3: Classification Governance (p. 93)	`HCM-BC-classification-governance`	CT3
CT3: Classification Utilisation Oversight (p. 94)	`HCM-BC-classification-utilisation-oversight`	CT3
CT3: Semantic Mapping Governance (p. 94)	`HCM-BC-semantic-mapping-governance`	CT3

2.10.2.1. CT3: Terminology Governance

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-terminology-governance`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Standards, Semantics & Reference Governance (p. 83) → CT2: Clinical Semantics Governance (p. 90)	Established BC responsibility path.
Structural Canonical ID	`CT1-STANDARDS-SEMANTICS-REFERENCE-GOVERNANCE.CT2-CLINICAL-SEMANTICS-GOVERNANCE.CT3-TERMINOLOGY-GOVERNANCE`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:721 (p. 544); business:717 (p. 544); business:728 (p. 545)	Linked RAD detail; exact locators in manifest.

Capability Overview

Terminology Governance maintains the lifecycle and standing of terminology and reference sets used by the organisation. Acceptance, release, updates and retirement require coordination with the appropriate authority or provider so that a code's meaning is not treated as timeless or locally self-defined. This supplies a governed semantic basis for information bindings and knowledge associations.

Definition

Manage governed terminology/reference-set lifecycle and standing, including acceptance, release, update, supersession/retirement and authority/provider coordination.

Boundary

Governing terminology differs from binding a structure to it or overseeing its actual use.

2.10.2.2. CT3: Terminology Utilisation Oversight

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-terminology-utilisation-oversight`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Standards, Semantics & Reference Governance (p. 83) → CT2: Clinical Semantics Governance (p. 90)	Established BC responsibility path.
Structural Canonical ID	`CT1-STANDARDS-SEMANTICS-REFERENCE-GOVERNANCE.CT2-CLINICAL-SEMANTICS-GOVERNANCE.CT3-TERMINOLOGY-UTILISATION-OVERSIGHT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:722 (p. 544); business:717 (p. 544); business:728 (p. 545)	Linked RAD detail; exact locators in manifest.

Capability Overview

Terminology Utilisation Oversight considers how terminology and reference sets are actually used. It identifies inappropriate application, inconsistency or semantic drift that the existence of an approved terminology does not prevent. Its purpose is to preserve coherent use without making each consuming capability the owner of terminology meaning.

Definition

Oversee actual terminology/reference-set use and identify inappropriate, inconsistent or drifting usage.

Boundary

Use oversight does not redefine terminology or automatically authorise a local semantic variation.

2.10.2.3. CT3: Terminology Localisation

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-terminology-localisation`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.

Name	Value	Comment / Description
Ancestry	CT1: Standards, Semantics & Reference Governance (p. 83) → CT2: Clinical Semantics Governance (p. 90)	Established BC responsibility path.
Structural Canonical ID	`CT1-STANDARDS-SEMANTICS-REFERENCE-GOVERNANCE.CT2-CLINICAL-SEMANTICS-GOVERNANCE.CT3-TERMINOLOGY-LOCALISATION`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:723 (p. 544); business:717 (p. 544); business:728 (p. 545)	Linked RAD detail; exact locators in manifest.

Capability Overview

Terminology Localisation manages controlled local responses where adopted terminology does not satisfy legitimate organisational semantic needs. It makes the local variation explicit rather than allowing ad hoc usage to conceal a gap or change in meaning. Within clinical-semantics governance, this preserves a distinction between governed local adaptation and uncontrolled drift.

Definition

Manage controlled local responses where adopted terminology does not meet legitimate organisational semantic requirements.

Boundary

The established localisation responsibility concerns terminology; no corresponding Classification Localisation capability is inferred.

2.10.2.4. CT3: Classification Governance

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-classification-governance`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Standards, Semantics & Reference Governance (p. 83) → CT2: Clinical Semantics Governance (p. 90)	Established BC responsibility path.
Structural Canonical ID	`CT1-STANDARDS-SEMANTICS-REFERENCE-GOVERNANCE.CT2-CLINICAL-SEMANTICS-GOVERNANCE.CT3-CLASSIFICATION-GOVERNANCE`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:724 (p. 544); business:717 (p. 544); business:728 (p. 545)	Linked RAD detail; exact locators in manifest.

Capability Overview

Classification Governance establishes the definitions and organisational standing of applicable classifications. In this governed context, classification concerns sensitivity and the consequences of exposure or mishandling, rather than the clinical meaning supplied by terminology. Its outcome supplies a basis for consistent application and protection considerations.

Definition

Govern applicable classification definitions and their organisational standing.

Boundary

Classification is orthogonal to contextual compartment membership and does not by itself determine an access decision.

2.10.2.5. CT3: Classification Utilisation Oversight

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-classification-utilisation-oversight`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Standards, Semantics & Reference Governance (p. 83) → CT2: Clinical Semantics Governance (p. 90)	Established BC responsibility path.
Structural Canonical ID	`CT1-STANDARDS-SEMANTICS-REFERENCE-GOVERNANCE.CT2-CLINICAL-SEMANTICS-GOVERNANCE.CT3-CLASSIFICATION-UTILISATION-OVERSIGHT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:725 (p. 544); business:717 (p. 544); business:728 (p. 545)	Linked RAD detail; exact locators in manifest.

Capability Overview

Classification Utilisation Oversight evaluates appropriate organisational application of classifications. It addresses whether information is treated consistently with the governed sensitivity meaning, rather than assuming that an established classification scheme guarantees correct use. This retains a distinct responsibility for practice alongside definition governance.

Definition

Oversee appropriate organisational application of classifications.

Boundary

Oversight neither creates classification definitions nor establishes a new localisation peer.

2.10.2.6. CT3: Semantic Mapping Governance

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-semantic-mapping-governance`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Standards, Semantics & Reference Governance (p. 83) → CT2: Clinical Semantics Governance (p. 90)	Established BC responsibility path.

Name	Value	Comment / Description
Structural Canonical ID	`CT1-STANDARDS-SEMANTICS-REFERENCE-GOVERNANCE.CT2-CLINICAL-SEMANTICS-GOVERNANCE.CT3-SEMANTIC-MAPPING-GOVERNANCE`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:726 (p. 544); business:717 (p. 544); business:728 (p. 545)	Linked RAD detail; exact locators in manifest.

Capability Overview

Semantic Mapping Governance maintains mappings between applicable semantic artefacts while preserving the meaning and authority of each side. Its purpose is intelligible correspondence rather than assumed equivalence. A governed mapping can support exchange or interpretation without suggesting that the mapped artefacts have become the same concept or share one authority.

Definition

Govern mappings between applicable semantic artefacts without collapsing their source meanings or authorities.

Boundary

A mapping does not erase source distinctions or give a consuming system authority to redefine either source.

2.11. CT1: Connected Health Services

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-connected-health-services`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT1	Explicit source assertion.
Ancestry	CT1 root	Explicitly established.
Structural Canonical ID	`CT1-CONNECTED-HEALTH-SERVICES`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Harmonia-Core	Independent of status; never inherited.
Source	business:730 (p. 545); business:734 (p. 545); business:120 (p. 531)	Linked RAD detail; exact locators in manifest.

Capability Overview

Connected Health Services represents the organisation’s ability to exchange health information and communicate digitally between health-service participants. Its two children distinguish exchange of health-information artefacts from participant communication, notification and collaboration. Both support connected care, but a user viewing information through a screen is not thereby participating in a communication channel.

Definition

Exchange health information appropriately and provide digitally mediated communication, notifications and collaborative interaction between health-service participants.

Intent

Enable appropriate information movement and interaction between otherwise separate participants and services.

Boundary

This organisational capability is not defined by an interface engine or messaging product. General presentation remains a separate enabling concern.

Harmonia Architectural Relevance

Information exchange and digitally mediated interaction are direct platform responsibilities within their assigned contracts, supporting the Core classification without transferring payload or clinical authority.

Scope

Contained Capabilities

Capability	Reference Id	Tier
CT2: Health Information Exchange (p. 96)	`HCM-BC-health-information-exchange`	CT2
CT2: Digital Communication Channel (p. 97)	`HCM-BC-digital-communication-channel`	CT2

2.11.1. CT2: Health Information Exchange

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-information-exchange`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	CT1: Connected Health Services (p. 95)	Established BC responsibility path.
Structural Canonical ID	`CT1-CONNECTED-HEALTH-SERVICES.CT2-HEALTH-INFORMATION-EXCHANGE`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:738 (p. 545); business:740 (p. 545); business:742 (p. 545); business:746 (p. 545); business:1225 (p. 557)	Linked RAD detail; exact locators in manifest.

Capability Overview

As a Business Capability, Health Information Exchange is the organisational ability to move health information appropriately between participating parties, systems and services. It explains the enterprise purpose served by exchange without defining that purpose as a protocol, server or software product. Its placement under Connected Health Services distinguishes information exchange from the broader interaction and notification purposes of Digital Communication Channel.

Definition

Exchange health information appropriately between participating parties, systems and services.

Boundary

No CT3 decomposition is established. The identically named BEC is a distinct enabling responsibility; the naming conflict remains unresolved, and this entry does not merge the two or assign the sharing-programme concern a parent.

Additional Notes / Unresolved Matters

Name Uniqueness: The same canonical name identifies distinct BC and BEC responsibilities. Global naming and the BC/BEC naming boundary conflict remains unresolved; capture does not merge identities or waive the rule.

2.11.2. CT2: Digital Communication Channel

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-digital-communication-channel`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	CT1: Connected Health Services (p. 95)	Established BC responsibility path.
Structural Canonical ID	`CT1-CONNECTED-HEALTH-SERVICES.CT2-DIGITAL-COMMUNICATION-CHANNEL`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:748 (p. 545); business:750 (p. 545); business:752 (p. 545); business:756 (p. 545)	Linked RAD detail; exact locators in manifest.

Capability Overview

Digital Communication Channel supports communication between patients, practitioners and other participants through digitally mediated messages, notifications and collaborative interaction. Its organisational purpose is the exchange between participants: for example, a notification can connect someone to a care event without itself delivering care or changing the event's originating facts.

Definition

Provide digitally mediated communication, notification and collaborative interaction between patients, practitioners and other health-service participants.

Boundary

A presentation interface used to exercise another capability is not automatically this communication ability. No CT3 set or general presentation BEC placement is established here.

2.12. CT1: Security, Privacy & Digital Trust

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-security-privacy-digital-trust`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT1	Explicit source assertion.
Ancestry	CT1 root	Explicitly established.
Structural Canonical ID	`CT1-SECURITY-PRIVACY-DIGITAL-TRUST`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Harmonia-Core	Independent of status; never inherited.
Source	business:758 (p. 545); business:762 (p. 545); business:120 (p. 531)	Linked RAD detail; exact locators in manifest.

Capability Overview

This capability establishes confidence in identity, coherent control of access, protection of information and accountability for its use. Its three branches distinguish identity/access governance, protection and privacy, and attribution/accountability. The scope includes physical and procedural arrangements as well as digital controls, preventing a software authorisation check from standing in for the organisation's whole trust responsibility.

Definition

Establish adequate identity confidence, coherent access controls, privacy/protection safeguards, governed activity attribution and accountability for security-relevant activity and information use.

Intent

Support information use and governed activity within legitimate identity, authority, privacy and protection constraints.

Boundary

Identifying an actor, permitting an action, recording participation and judging appropriateness are different outcomes. Evidence production is not itself independent assurance.

Harmonia Architectural Relevance

Security and governance are intrinsic to Harmonia-managed operations. The Core classification concerns assigned platform controls and evidence; it does not acquire every organisational security or privacy responsibility.

Scope

Contained Capabilities

Capability	Reference Id	Tier
CT2: Digital Identity & Access Governance (p. 99)	`HCM-BC-digital-identity-access-governance`	CT2

Capability	Reference Id	Tier
CT2: Health Information Protection (p. 105)	HCM-BC-health-information-protection	CT2
CT2: Digital Accountability & Trust (p. 110)	HCM-BC-digital-accountability-trust	CT2

2.12.1. CT2: Digital Identity & Access Governance

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	HCM-BC-digital-identity-access-governance	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	CT1: Security, Privacy & Digital Trust (p. 98)	Established BC responsibility path.
Structural Canonical ID	CT1-SECURITY-PRIVACY-DIGITAL-TRUST.CT2-DIGITAL-IDENTITY-ACCESS-GOVERNANCE	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:766 (p. 545); business:768 (p. 546); business:770 (p. 546); business:785 (p. 546)	Linked RAD detail; exact locators in manifest.

Capability Overview

This capability governs who may act and how controlled concerns are identified and protected across the organisation. Its children separate confidence in a claimed identity, identity models, contextual identity quality, authentication, access-control context, assurance, collaboration and enforcement. That separation prevents successful authentication from being treated as sufficient authority for every use.

Definition

Govern sufficient identity confidence and coherent access controls across the organisation.

Intent

Make identity confidence and control requirements appropriate to the circumstances in which access or activity occurs.

Boundary

Federation or Single Sign-On does not guarantee identity quality. Enforcement spans physical, procedural and digital controls; it is broader than a runtime policy engine.

Scope

Contained Capabilities

Capability	Reference Id	Tier
CT3: Identity Assurance (p. 100)	HCM-BC-identity-assurance	CT3
CT3: Identity Modelling & Interoperability (p. 101)	HCM-BC-identity-modelling-interoperability	CT3
CT3: Identity Qualification Control (p. 101)	HCM-BC-identity-qualification-control	CT3

Capability	Reference Id	Tier
CT3: Identity Identification & Authentication (p. 102)	`HCM-BC-identity-identification-authentication`	CT3
CT3: Access Control Context Management (p. 103)	`HCM-BC-access-control-context-management`	CT3
CT3: Access Control Assurance & Reporting (p. 103)	`HCM-BC-access-control-assurance-reporting`	CT3
CT3: Access Control Collaboration (p. 104)	`HCM-BC-access-control-collaboration`	CT3
CT3: Access Control Enforcement (p. 105)	`HCM-BC-access-control-enforcement`	CT3

2.12.1.1. CT3: Identity Assurance

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-identity-assurance`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Security, Privacy & Digital Trust (p. 98) → CT2: Digital Identity & Access Governance (p. 99)	Established BC responsibility path.
Structural Canonical ID	`CT1-SECURITY-PRIVACY-DIGITAL-TRUST.CT2-DIGITAL-IDENTITY-ACCESS-GOVERNANCE.CT3-IDENTITY-ASSURANCE`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:776 (p. 546); business:772 (p. 546); business:785 (p. 546)	Linked RAD detail; exact locators in manifest.

Capability Overview

Identity Assurance establishes confidence that a person is who they claim to be using appropriate evidence and processes. It concerns the basis and strength of the identity claim, rather than only successful use of an authentication mechanism. Within identity governance it makes confidence a question that can be considered separately from contextual sufficiency and access authority.

Definition

Establish sufficient confidence that a person is who they claim to be through appropriate evidence and processes.

Boundary

An assured identity is not automatically sufficient for every context or permitted to perform every action.

2.12.1.2. CT3: Identity Modelling & Interoperability

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-identity-modelling-interoperability`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Security, Privacy & Digital Trust (p. 98) → CT2: Digital Identity & Access Governance (p. 99)	Established BC responsibility path.
Structural Canonical ID	`CT1-SECURITY-PRIVACY-DIGITAL-TRUST.CT2-DIGITAL-IDENTITY-ACCESS-GOVERNANCE.CT3-IDENTITY-MODELLING-INTEROPERABILITY`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:777 (p. 546); business:772 (p. 546); business:785 (p. 546)	Linked RAD detail; exact locators in manifest.

Capability Overview

This capability establishes identity models and arrangements through which identities can be understood across organisational use. It addresses the coherence needed when parties exchange or rely on identity information without assuming that their identity assertions have identical quality. Its purpose is meaningful interoperability, rather than a particular federation or Single Sign-On technology.

Definition

Establish identity models and interoperability arrangements appropriate to organisational identity use.

Boundary

Interoperability does not guarantee assurance quality or transfer each party's control ownership.

2.12.1.3. CT3: Identity Qualification Control

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-identity-qualification-control`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Security, Privacy & Digital Trust (p. 98) → CT2: Digital Identity & Access Governance (p. 99)	Established BC responsibility path.
Structural Canonical ID	`CT1-SECURITY-PRIVACY-DIGITAL-TRUST.CT2-DIGITAL-IDENTITY-ACCESS-GOVERNANCE.CT3-IDENTITY-QUALIFICATION-CONTROL`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:778 (p. 546); business:772 (p. 546); business:785 (p. 546)	Linked RAD detail; exact locators in manifest.

Capability Overview

Identity Qualification Control evaluates whether presented identity characteristics, provenance and assurance are sufficient for the intended context. It adds a contextual judgement beyond whether the actor can be authenticated. This permits an identity adequate for one purpose to remain insufficient for another without claiming that the identity is necessarily false.

Definition

Determine whether presented identity characteristics, provenance and assurance quality are sufficient for its intended context.

Boundary

Identity qualification does not establish practitioner clinical qualification or a general permission to act.

2.12.1.4. CT3: Identity Identification & Authentication

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-identity-identification-authentication`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Security, Privacy & Digital Trust (p. 98) → CT2: Digital Identity & Access Governance (p. 99)	Established BC responsibility path.
Structural Canonical ID	`CT1-SECURITY-PRIVACY-DIGITAL-TRUST.CT2-DIGITAL-IDENTITY-ACCESS-GOVERNANCE.CT3-IDENTITY-IDENTIFICATION-AUTHENTICATION`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:779 (p. 546); business:772 (p. 546); business:785 (p. 546)	Linked RAD detail; exact locators in manifest.

Capability Overview

This capability identifies the acting participant and authenticates the asserted identity. It establishes who is acting under the applicable identity arrangements so that later control and accountability can be grounded in an identified actor. The outcome remains distinct from confidence established through identity evidence and from authority to access a controlled concern.

Definition

Identify an actor and authenticate the asserted identity.

Boundary

Authentication neither establishes every required identity-quality condition nor authorises the requested activity.

2.12.1.5. CT3: Access Control Context Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-access-control-context-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Security, Privacy & Digital Trust (p. 98) → CT2: Digital Identity & Access Governance (p. 99)	Established BC responsibility path.
Structural Canonical ID	`CT1-SECURITY-PRIVACY-DIGITAL-TRUST.CT2-DIGITAL-IDENTITY-ACCESS-GOVERNANCE.CT3-ACCESS-CONTROL-CONTEXT-MANAGEMENT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:780 (p. 546); business:772 (p. 546); business:785 (p. 546)	Linked RAD detail; exact locators in manifest.

Capability Overview

Access Control Context Management establishes what requires control, the circumstances in which control applies and how those requirements are modelled. It gives access governance an explicit subject and context rather than reducing it to assembling runtime attributes. The responsibility can encompass information, functions and activities under physical, procedural or digital arrangements.

Definition

Determine what information, functions, activities or other concerns require control, under what circumstances and how requirements are modelled.

Boundary

Defining control context does not itself enforce the controls or establish that they operate effectively.

2.12.1.6. CT3: Access Control Assurance & Reporting

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-access-control-assurance-reporting`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Security, Privacy & Digital Trust (p. 98) → CT2: Digital Identity & Access Governance (p. 99)	Established BC responsibility path.
Structural Canonical ID	`CT1-SECURITY-PRIVACY-DIGITAL-TRUST.CT2-DIGITAL-IDENTITY-ACCESS-GOVERNANCE.CT3-ACCESS-CONTROL-ASSURANCE-REPORTING`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:781 (p. 546); business:772 (p. 546); business:785 (p. 546)	Linked RAD detail; exact locators in manifest.

Capability Overview

This capability establishes organisational evidence and assurance about whether access-control arrangements are appropriately established and operating. It makes control effectiveness a distinct concern from defining requirements or performing an individual enforcement action. Reporting communicates that established position without making an isolated successful check proof of end-to-end control.

Definition

Establish organisational assurance/evidence of whether access-control arrangements are appropriately established and operating.

Boundary

Subject-specific access-control assurance does not create another macro-level Health Service Assurance hierarchy.

2.12.1.7. CT3: Access Control Collaboration

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-access-control-collaboration`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Security, Privacy & Digital Trust (p. 98) → CT2: Digital Identity & Access Governance (p. 99)	Established BC responsibility path.
Structural Canonical ID	`CT1-SECURITY-PRIVACY-DIGITAL-TRUST.CT2-DIGITAL-IDENTITY-ACCESS-GOVERNANCE.CT3-ACCESS-CONTROL-COLLABORATION`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:782 (p. 546); business:772 (p. 546); business:785 (p. 546)	Linked RAD detail; exact locators in manifest.

Capability Overview

Access Control Collaboration coordinates arrangements across parties or domains that retain independent control ownership. Its purpose is coherent cross-boundary control where a strong local arrangement alone cannot establish the whole result. Collaboration therefore concerns agreed control participation rather than silently centralising every participant's authority.

Definition

Coordinate arrangements across independently controlled domains/parties while retaining their control ownership.

Boundary

Coordination does not transfer the parties' control ownership or guarantee identity quality across domains.

2.12.1.8. CT3: Access Control Enforcement

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-access-control-enforcement`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Security, Privacy & Digital Trust (p. 98) → CT2: Digital Identity & Access Governance (p. 99)	Established BC responsibility path.
Structural Canonical ID	`CT1-SECURITY-PRIVACY-DIGITAL-TRUST.CT2-DIGITAL-IDENTITY-ACCESS-GOVERNANCE.CT3-ACCESS-CONTROL-ENFORCEMENT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:783 (p. 546); business:772 (p. 546); business:785 (p. 546)	Linked RAD detail; exact locators in manifest.

Capability Overview

Access Control Enforcement ensures that intended controls are coherently imposed through the applicable physical, procedural and digital mechanisms. It represents the organisational ability to make controls effective, rather than equating enforcement with one software decision. Its outcome depends on the controlled circumstances and the arrangements intended to apply there.

Definition

Ensure physical, procedural and digital control mechanisms coherently impose intended controls.

Boundary

A local software authorisation result alone does not establish coherent end-to-end organisational enforcement.

2.12.2. CT2: Health Information Protection

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-information-protection`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	CT1: Security, Privacy & Digital Trust (p. 98)	Established BC responsibility path.
Structural Canonical ID	`CT1-SECURITY-PRIVACY-DIGITAL-TRUST.CT2-HEALTH-INFORMATION-PROTECTION`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:787 (p. 546); business:789 (p. 546); business:791 (p. 546); business:803 (p. 547)	Linked RAD detail; exact locators in manifest.

Capability Overview

Health Information Protection governs the consequences of information exposure and the safeguards protecting information and its operating environment. Privacy safeguards, estate protection, access-practice monitoring, compartment assurance and classification assurance address different questions. Together they distinguish appropriate arrangements from evidence of how information is actually handled, including physical and printed handling.

Definition

Govern privacy consequences and adequate protection of the information estate and its environment.

Intent

Protect information and the people affected by its use, with an intelligible basis for evaluating protective arrangements.

Boundary

Compartment assurance does not decide access. Subject-specific protection assurance does not create additional macro-level Health Service Assurance trees.

Scope

Contained Capabilities

Capability	Reference Id	Tier
CT3: Health Privacy Safeguard Management (p. 106)	`HCM-BC-health-privacy-safeguard-management`	CT3
CT3: Health Information Safeguarding (p. 107)	`HCM-BC-health-information-safeguarding`	CT3
CT3: Health Information Access Monitoring (p. 108)	`HCM-BC-health-information-access-monitoring`	CT3
CT3: Health Information Compartment Assurance (p. 108)	`HCM-BC-health-information-compartment-assurance`	CT3
CT3: Health Information Classification Assurance (p. 109)	`HCM-BC-health-information-classification-assurance`	CT3

2.12.2.1. CT3: Health Privacy Safeguard Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-privacy-safeguard-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Security, Privacy & Digital Trust (p. 98) → CT2: Health Information Protection (p. 105)	Established BC responsibility path.
Structural Canonical ID	`CT1-SECURITY-PRIVACY-DIGITAL-TRUST.CT2-HEALTH-INFORMATION-PROTECTION.CT3-HEALTH-PRIVACY-SAFEGUARD-MANAGEMENT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.

Name	Value	Comment / Description
Source	business:797 (p. 546); business:793 (p. 546); business:803 (p. 547)	Linked RAD detail; exact locators in manifest.

Capability Overview

This capability establishes safeguards and assurance arrangements concerning the privacy consequences of supplying, exposing or using information. It focuses on the effect on people and legitimate information use, distinguishing that concern from protection of the information estate itself. Technical access restrictions may contribute without exhausting the organisational privacy responsibility.

Definition

Establish organisational safeguards and assurance processes concerning privacy consequences of information supply, exposure or use.

Boundary

Privacy safeguards do not replace broader estate resilience, access decisions or originating consent authority.

2.12.2.2. CT3: Health Information Safeguarding

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-information-safeguarding`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Security, Privacy & Digital Trust (p. 98) → CT2: Health Information Protection (p. 105)	Established BC responsibility path.
Structural Canonical ID	`CT1-SECURITY-PRIVACY-DIGITAL-TRUST.CT2-HEALTH-INFORMATION-PROTECTION.CT3-HEALTH-INFORMATION-SAFEGUARDING`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:798 (p. 546); business:793 (p. 546); business:803 (p. 547)	Linked RAD detail; exact locators in manifest.

Capability Overview

Health Information Safeguarding protects information and its operational environment against loss, compromise, inappropriate access and operational failure. It frames resilience and protection as organisational outcomes, allowing physical controls, recovery arrangements and testing to contribute without defining the capability as a product. The concern is broader than any individual access transaction.

Definition

Ensure information and its operational environment are protected and resilient against loss, compromise, inappropriate access or operational failure.

Boundary

Estate protection remains distinct from evaluating privacy consequences and from assurance conclusions about service delivery.

2.12.2.3. CT3: Health Information Access Monitoring

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-information-access-monitoring`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Security, Privacy & Digital Trust (p. 98) → CT2: Health Information Protection (p. 105)	Established BC responsibility path.
Structural Canonical ID	`CT1-SECURITY-PRIVACY-DIGITAL-TRUST.CT2-HEALTH-INFORMATION-PROTECTION.CT3-HEALTH-INFORMATION-ACCESS-MONITORING`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:799 (p. 546); business:793 (p. 546); business:803 (p. 547)	Linked RAD detail; exact locators in manifest.

Capability Overview

Health Information Access Monitoring evaluates actual access and handling practice across organisational circumstances. Digital audit information can contribute, but so can physical access, printed handling, compartments and classification practice. Its purpose is to understand what is happening in use rather than merely observe software events or assume configured controls describe reality.

Definition

Evaluate actual organisational access/handling practice, including controls, accessibility, compartments, classification and physical/printed handling.

Boundary

Evidence supply does not transfer business accountability, and observation is not automatically independent assurance.

2.12.2.4. CT3: Health Information Compartment Assurance

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-information-compartment-assurance`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Security, Privacy & Digital Trust (p. 98) → CT2: Health Information Protection (p. 105)	Established BC responsibility path.
Structural Canonical ID	`CT1-SECURITY-PRIVACY-DIGITAL-TRUST.CT2-HEALTH-INFORMATION-PROTECTION.CT3-HEALTH-INFORMATION-COMPARTMENT-ASSURANCE`	Allocated v0.1; structural responsibility identity.

Name	Value	Comment / Description
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:800 (p. 546); business:793 (p. 546); business:803 (p. 547)	Linked RAD detail; exact locators in manifest.

Capability Overview

This capability evaluates whether contextual information compartments are appropriately established and applied. It treats the correctness of the grouping as a concern distinct from permission to disclose a particular item. Such assurance can make compartment use more trustworthy without changing the business relationships that define membership.

Definition

Assure appropriate establishment and application of compartmentalisation without establishing access decisions.

Boundary

Assuring compartmentalisation does not establish access decisions or sensitivity classification.

2.12.2.5. CT3: Health Information Classification Assurance

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-information-classification-assurance`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Security, Privacy & Digital Trust (p. 98) → CT2: Health Information Protection (p. 105)	Established BC responsibility path.
Structural Canonical ID	`CT1-SECURITY-PRIVACY-DIGITAL-TRUST.CT2-HEALTH-INFORMATION-PROTECTION.CT3-HEALTH-INFORMATION-CLASSIFICATION-ASSURANCE`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:801 (p. 546); business:793 (p. 546); business:803 (p. 547)	Linked RAD detail; exact locators in manifest.

Capability Overview

This capability evaluates whether classification is appropriate to information sensitivity and the consequences of exposure or mishandling. It examines the application of classification rather than merely the presence of a label. The responsibility supports protection decisions while retaining the distinction between the classification basis and the control applied to a particular request.

Definition

Assure appropriate classification according to sensitivity and potential consequences of exposure/mishandling.

Boundary

Classification assurance does not define compartment membership or grant permission to access classified information.

2.12.3. CT2: Digital Accountability & Trust

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-digital-accountability-trust`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	CT1: Security, Privacy & Digital Trust (p. 98)	Established BC responsibility path.
Structural Canonical ID	`CT1-SECURITY-PRIVACY-DIGITAL-TRUST.CT2-DIGITAL-ACCOUNTABILITY-TRUST`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:805 (p. 547); business:807 (p. 547); business:809 (p. 547); business:818 (p. 547)	Linked RAD detail; exact locators in manifest.

Capability Overview

Digital Accountability & Trust makes activity attributable and evaluates whether security-relevant activity and information use were appropriate. Attribution records participation together with acting capacity and relevant permission or constraint context; accountability uses that context to consider conduct against policy and obligations. A record of who acted is therefore necessary context, rather than proof that the action was permitted.

Definition

Establish governed attribution and accountability for activity and information use.

Intent

Support meaningful responsibility for information use and security-relevant conduct.

Boundary

Authentication, attribution and authorisation must remain distinguishable. Expected downstream evidentiary use does not establish new BC-to-BEC relationships.

Scope

Contained Capabilities

Capability	Reference Id	Tier
CT3: Governed Activity Attribution (p. 111)	`HCM-BC-governed-activity-attribution`	CT3
CT3: Security & Information-Use Accountability (p. 111)	`HCM-BC-security-information-use-accountability`	CT3

2.12.3.1. CT3: Governed Activity Attribution

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-governed-activity-attribution`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Security, Privacy & Digital Trust (p. 98) → CT2: Digital Accountability & Trust (p. 110)	Established BC responsibility path.
Structural Canonical ID	`CT1-SECURITY-PRIVACY-DIGITAL-TRUST.CT2-DIGITAL-ACCOUNTABILITY-TRUST.CT3-GOVERNED-ACTIVITY-ATTRIBUTION`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:815 (p. 547); business:811 (p. 547); business:818 (p. 547)	Linked RAD detail; exact locators in manifest.

Capability Overview

Governed Activity Attribution preserves who participated and in what acting capacity, together with the relevant permission, authority and constraint context. It makes conduct explainable beyond a bare actor identifier and timestamp. Within Digital Accountability & Trust, this supplies the contextual record needed to distinguish participation from entitlement.

Definition

Preserve who participated, what was done, the attributable person/role/unit/organisation capacity and applicable permission/authority/constraint context.

Boundary

Attributing an action to a person or role does not establish that the action was permitted or appropriate.

2.12.3.2. CT3: Security & Information-Use Accountability

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-security-information-use-accountability`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Security, Privacy & Digital Trust (p. 98) → CT2: Digital Accountability & Trust (p. 110)	Established BC responsibility path.
Structural Canonical ID	`CT1-SECURITY-PRIVACY-DIGITAL-TRUST.CT2-DIGITAL-ACCOUNTABILITY-TRUST.CT3-SECURITY-INFORMATION-USE-ACCOUNTABILITY`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:816 (p. 547); business:811 (p. 547); business:818 (p. 547)	Linked RAD detail; exact locators in manifest.

Capability Overview

This capability evaluates attributed activity and information use against applicable policy, obligations and authority constraints. It establishes accountability rather than simply recording participation. Its purpose depends on preserving the circumstances of action so that appropriateness is considered within the relevant mandate instead of inferred from identity or technical success.

Definition

Evaluate whether attributed security-relevant activity/information use was appropriate within policy, obligation and authority constraints, and establish accountability.

Boundary

An authenticated actor and a successful operation are not proof of legitimate information use.

2.13. CT1: Health Research & Innovation

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-research-innovation`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT1	Explicit source assertion.
Ancestry	CT1 root	Explicitly established.
Structural Canonical ID	`CT1-HEALTH-RESEARCH-INNOVATION`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Harmonia-Relevant	Independent of status; never inherited.
Source	business:824 (p. 547); business:828 (p. 547); business:120 (p. 531)	Linked RAD detail; exact locators in manifest.

Capability Overview

This capability frames the deliberately bounded research and innovation responsibilities material to Harmonia's R1/R2 architecture. Research Information Support concerns provision of existing information for authorised research; Health Innovation concerns evaluation of proposed or emerging innovation. These establish useful enterprise context without implying a complete research organisation or innovation lifecycle.

Definition

Provide existing health information for authorised research under applicable constraints and evaluate health innovation; deliberately under-decomposed for R1/R2.

Intent

Support legitimate research information needs and informed consideration of changes to health-service or clinical practice.

Boundary

Information provision does not own trials, ethics approval or specialist research analysis. Evaluating innovation does not authorise adoption or implementation.

Harmonia Architectural Relevance

Governed provision of existing information materially enables authorised research. Innovation evaluation has no separate platform assignment; the CT1 classification must not be inherited by each descendant.

Scope

Contained Capabilities

Capability	Reference Id	Tier
CT2: Research Information Support (p. 113)	`HCM-BC-research-information-support`	CT2
CT2: Health Innovation (p. 115)	`HCM-BC-health-innovation`	CT2

2.13.1. CT2: Research Information Support

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-research-information-support`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	CT1: Health Research & Innovation (p. 112)	Established BC responsibility path.
Structural Canonical ID	`CT1-HEALTH-RESEARCH-INNOVATION.CT2-RESEARCH-INFORMATION-SUPPORT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:832 (p. 547); business:834 (p. 547); business:836 (p. 547); business:844 (p. 548)	Linked RAD detail; exact locators in manifest.

Capability Overview

Research Information Support represents the ability to make existing health information available for a legitimate research purpose. Its single established child concentrates on governed provision, preserving the constraints under which information can be used rather than treating technical availability as research permission. The narrow decomposition reflects R1/R2 sufficiency rather than a claim to encompass every form of research support.

Definition

Provide the existing health information needed for authorised research within the deliberately bounded R1/R2 responsibility.

Intent

Support authorised research with information supplied under the applicable ethical, privacy and governance conditions.

Boundary

Research governance, trial oversight and specialist analysis remain unmodelled and are not absorbed into provision.

Scope

Contained Capabilities

Capability	Reference Id	Tier
CT3: Governed Research Information Provision (p. 114)	`HCM-BC-governed-research-information-provision`	CT3

2.13.1.1. CT3: Governed Research Information Provision

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-governed-research-information-provision`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Health Research & Innovation (p. 112) → CT2: Research Information Support (p. 113)	Established BC responsibility path.
Structural Canonical ID	`CT1-HEALTH-RESEARCH-INNOVATION.CT2-RESEARCH-INFORMATION-SUPPORT.CT3-GOVERNED-RESEARCH-INFORMATION-PROVISION`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:842 (p. 547); business:838 (p. 547); business:844 (p. 548)	Linked RAD detail; exact locators in manifest.

Capability Overview

Governed Research Information Provision makes existing health information available for an authorised research purpose under its applicable constraints. It represents a substantive information contribution while keeping ethics, governance, consent, privacy, security and permitted use material to provision. The responsibility concerns supplying existing information legitimately, rather than running the research that uses it.

Definition

Provide existing health information for an authorised research purpose under applicable ethics, governance, privacy, consent, security and information-use constraints.

Boundary

Availability is not research authorisation. Trials, genomic analysis and research-governance responsibilities are not subsumed by this deliberately narrow capability.

2.13.2. CT2: Health Innovation

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-innovation`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	CT1: Health Research & Innovation (p. 112)	Established BC responsibility path.
Structural Canonical ID	`CT1-HEALTH-RESEARCH-INNOVATION.CT2-HEALTH-INNOVATION`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:846 (p. 548); business:848 (p. 548); business:850 (p. 548); business:858 (p. 548)	Linked RAD detail; exact locators in manifest.

Capability Overview

Health Innovation provides the business frame for considering changes in health-service and clinical practice. Its established evaluation child examines suitability, value and implications so that an innovation can be understood in the context in which it might be used. The model stops at that evaluative responsibility rather than presenting a complete pipeline from idea to operational adoption.

Definition

Consider health-service and clinical innovation within the R1/R2 business context.

Intent

Make consideration of innovation informed by its prospective benefits, consequences and suitability.

Boundary

Evaluation does not establish an adoption decision, professional authority or an implementation programme.

Scope

Contained Capabilities

Capability	Reference Id	Tier
CT3: Health Innovation Evaluation (p. 115)	`HCM-BC-health-innovation-evaluation`	CT3

2.13.2.1. CT3: Health Innovation Evaluation

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-innovation-evaluation`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.

Name	Value	Comment / Description
Ancestry	CT1: Health Research & Innovation (p. 112) → CT2: Health Innovation (p. 115)	Established BC responsibility path.
Structural Canonical ID	`CT1-HEALTH-RESEARCH-INNOVATION.CT2-HEALTH-INNOVATION.CT3-HEALTH-INNOVATION-EVALUATION`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:856 (p. 548); business:852 (p. 548); business:858 (p. 548)	Linked RAD detail; exact locators in manifest.

Capability Overview

Health Innovation Evaluation is the organisation's ability to form a considered view of a proposed or emerging innovation before treating it as suitable for use. Within Health Innovation it brings suitability, value and implications into the same evaluative context, so that potential benefit can be considered alongside consequences for the health-service or clinical setting. The outcome supports informed consideration of adoption rather than making adoption inevitable.

Definition

Evaluate proposed/emerging innovation for suitability, value, implications and potential adoption.

Intent

Give decision makers an intelligible basis for considering whether an innovation merits further adoption consideration.

Boundary

Evaluation does not establish adoption, authorise a clinical change or create an implementation responsibility. The single established child does not imply a complete innovation lifecycle.

2.14. CT1: Health Service Direction & Stewardship

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-service-direction-stewardship`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT1	Explicit source assertion.
Ancestry	CT1 root	Explicitly established.
Structural Canonical ID	`CT1-HEALTH-SERVICE-DIRECTION-STEWARDSHIP`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Harmonia-Relevant	Independent of status; never inherited.
Source	business:864 (p. 548); business:868 (p. 548); business:120 (p. 531)	Linked RAD detail; exact locators in manifest.

Capability Overview

This capability considers what health services should exist, what makes them viable and how they fit within the wider service ecosystem. Modelling frames the need and required abilities; alignment addresses dependencies, resources, financial arrangements and surrounding

change. The distinction keeps the rationale for a service separate from its identifiable offering and its current operational delivery state.

Definition

Model needed health services and their viability requirements, support establishment/improvement and align services with the wider ecosystem of dependencies, resources, financial arrangements and organisational change.

Intent

Support viable, coherent service development and improvement within organisational and ecosystem constraints.

Boundary

Identifying dependencies and resources does not acquire their operating authority. Service modelling and alignment do not establish the authoritative service catalogue.

Harmonia Architectural Relevance

Service need, viability and alignment facts materially shape service information and derivation requirements. That relevance does not assign executive authority or establish a future BEC mapping.

Scope

Contained Capabilities

Capability	Reference Id	Tier
CT2: Health Service Modelling (p. 117)	`HCM-BC-health-service-modelling`	CT2
CT2: Health Service Alignment (p. 121)	`HCM-BC-health-service-alignment`	CT2

2.14.1. CT2: Health Service Modelling

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-service-modelling`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	CT1: Health Service Direction & Stewardship (p. 116)	Established BC responsibility path.
Structural Canonical ID	`CT1-HEALTH-SERVICE-DIRECTION-STEWARDSHIP.CT2-HEALTH-SERVICE-MODELLING`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:872 (p. 548); business:874 (p. 548); business:876 (p. 548); business:887 (p. 549)	Linked RAD detail; exact locators in manifest.

Capability Overview

Health Service Modelling establishes the service needed and the ordinary business abilities required to make it viable. Its children distinguish recognising unmet need, identifying required abilities, supporting establishment and evaluating an existing service for improvement. The model can therefore revisit service assumptions without treating the original definition as permanently sufficient.

Definition

Establish what service should exist and what it needs to become viable.

Intent

Connect a service's purpose with the abilities and preparation required for it to exist usefully.

Boundary

Capability Identification uses ordinary business meaning; it does not allocate Harmonia capability tiers. Implementation support does not take over recruitment, facilities, procurement or ICT responsibilities.

Scope

Contained Capabilities

Capability	Reference Id	Tier
CT3: Health Service Need Identification (p. 118)	`HCM-BC-health-service-need-identification`	CT3
CT3: Health Service Capability Identification (p. 119)	`HCM-BC-health-service-capability-identification`	CT3
CT3: Health Service Implementation Support (p. 120)	`HCM-BC-health-service-implementation-support`	CT3
CT3: Health Service Continuous Improvement (p. 120)	`HCM-BC-health-service-continuous-improvement`	CT3

2.14.1.1. CT3: Health Service Need Identification

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-service-need-identification`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Health Service Direction & Stewardship (p. 116) → CT2: Health Service Modelling (p. 117)	Established BC responsibility path.
Structural Canonical ID	`CT1-HEALTH-SERVICE-DIRECTION-STEWARDSHIP.CT2-HEALTH-SERVICE-MODELLING.CT3-HEALTH-SERVICE-NEED-IDENTIFICATION`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:882 (p. 548); business:878 (p. 548); business:887 (p. 549)	Linked RAD detail; exact locators in manifest.

Capability Overview

Health Service Need Identification establishes the unmet service need for which an organisational response is being considered. It gives service modelling a reason grounded in what is missing rather than beginning with a predetermined solution. That identified need can then frame the abilities and resources required without assuming that a new offering has already been established.

Definition

Establish an unmet health-service need requiring response.

Boundary

Identifying need does not define an offering, approve funding or determine the surrounding implementation arrangements.

2.14.1.2. CT3: Health Service Capability Identification

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-service-capability-identification`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Health Service Direction & Stewardship (p. 116) → CT2: Health Service Modelling (p. 117)	Established BC responsibility path.
Structural Canonical ID	`CT1-HEALTH-SERVICE-DIRECTION-STEWARDSHIP.CT2-HEALTH-SERVICE-MODELLING.CT3-HEALTH-SERVICE-CAPABILITY-IDENTIFICATION`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:883 (p. 548); business:878 (p. 548); business:887 (p. 549)	Linked RAD detail; exact locators in manifest.

Capability Overview

This capability identifies the ordinary business abilities a proposed service needs across clinical, workforce, facility, information, technology and support concerns. It broadens service modelling beyond a clinical description to the conditions needed for viability. Its contribution is an understanding of required abilities, leaving their fulfilment and governing authority with the appropriate responsibilities.

Definition

Identify the ordinary business abilities needed to satisfy the service need across clinical, workforce, facility, technology, information and support concerns.

Boundary

The word capability here does not allocate Harmonia CT tiers or establish new catalogue elements.

2.14.1.3. CT3: Health Service Implementation Support

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-service-implementation-support`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Health Service Direction & Stewardship (p. 116) → CT2: Health Service Modelling (p. 117)	Established BC responsibility path.
Structural Canonical ID	`CT1-HEALTH-SERVICE-DIRECTION-STEWARDSHIP.CT2-HEALTH-SERVICE-MODELLING.CT3-HEALTH-SERVICE-IMPLEMENTATION-SUPPORT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:884 (p. 548); business:878 (p. 548); business:887 (p. 549)	Linked RAD detail; exact locators in manifest.

Capability Overview

Health Service Implementation Support assists establishment of the framed service through organisational preparation and programmes of work. It connects the service model to practical readiness for introduction while recognising that recruitment, training, equipment and technology changes retain their own owners. Support therefore contributes to establishment without becoming an all-purpose implementation authority.

Definition

Support establishment of the framed service through applicable organisational preparation and programmes of work.

Boundary

It does not absorb HR, facilities, procurement or ICT responsibilities or establish operational-readiness ancestry.

2.14.1.4. CT3: Health Service Continuous Improvement

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-service-continuous-improvement`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Health Service Direction & Stewardship (p. 116) → CT2: Health Service Modelling (p. 117)	Established BC responsibility path.
Structural Canonical ID	`CT1-HEALTH-SERVICE-DIRECTION-STEWARDSHIP.CT2-HEALTH-SERVICE-MODELLING.CT3-HEALTH-SERVICE-CONTINUOUS-IMPROVEMENT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:885 (p. 548); business:878 (p. 548); business:887 (p. 549)	Linked RAD detail; exact locators in manifest.

Capability Overview

This capability evaluates an established service and identifies correction or improvement needs across its service-wide purpose. It allows experience of actual operation to inform reconsideration of the service model and implementation support. The scope is broader than reporting a clinical-quality finding, while remaining anchored to the service being improved.

Definition

Evaluate the established service and identify correction/improvement needs that may feed back into modelling and implementation support.

Boundary

It does not establish generic Continuous Improvement placement or transfer the authority of specialist activities affected by improvement.

2.14.2. CT2: Health Service Alignment

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-service-alignment`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	CT1: Health Service Direction & Stewardship (p. 116)	Established BC responsibility path.
Structural Canonical ID	`CT1-HEALTH-SERVICE-DIRECTION-STEWARDSHIP.CT2-HEALTH-SERVICE-ALIGNMENT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:889 (p. 549); business:891 (p. 549); business:893 (p. 549); business:904 (p. 549)	Linked RAD detail; exact locators in manifest.

Capability Overview

Health Service Alignment positions a service among the other services, resources and arrangements on which it depends. It considers reciprocal dependencies, required resources, financial fit and the organisational changes needed for introduction or material change. Where modelling asks what service is needed, alignment asks how it can fit coherently into its environment.

Definition

Position the service coherently in its wider health-service ecosystem.

Intent

Expose the dependencies and surrounding commitments that affect service viability.

Boundary

Alignment does not establish the service offering, own the required resources or settle the unresolved ancestry of collective operational readiness.

Scope

Contained Capabilities

Capability	Reference Id	Tier
CT3: Health Service Co-Dependency Management (p. 122)	`HCM-BC-health-service-co-dependency-management`	CT3
CT3: Health Service Resource Identification (p. 123)	`HCM-BC-health-service-resource-identification`	CT3
CT3: Health Service Financial Management (p. 123)	`HCM-BC-health-service-financial-management`	CT3
CT3: Health Service Implementation Planning (p. 124)	`HCM-BC-health-service-implementation-planning`	CT3

2.14.2.1. CT3: Health Service Co-Dependency Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-service-co-dependency-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Health Service Direction & Stewardship (p. 116) → CT2: Health Service Alignment (p. 121)	Established BC responsibility path.
Structural Canonical ID	`CT1-HEALTH-SERVICE-DIRECTION-STEWARDSHIP.CT2-HEALTH-SERVICE-ALIGNMENT.CT3-HEALTH-SERVICE-CO-DEPENDENCY-MANAGEMENT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:899 (p. 549); business:895 (p. 549); business:904 (p. 549)	Linked RAD detail; exact locators in manifest.

Capability Overview

Health Service Co-Dependency Management identifies and manages reciprocal needs between a service and its wider ecosystem. It makes explicit both what the service requires from others and what others rely on it to provide. This supports alignment by exposing dependencies that a service description considered in isolation may conceal.

Definition

Identify and manage what the service needs from others and what others need from it.

Boundary

Recognising a dependency does not acquire the provider’s responsibility or establish a new technical Service dependency.

2.14.2.2. CT3: Health Service Resource Identification

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-service-resource-identification`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Health Service Direction & Stewardship (p. 116) → CT2: Health Service Alignment (p. 121)	Established BC responsibility path.
Structural Canonical ID	`CT1-HEALTH-SERVICE-DIRECTION-STEWARDSHIP.CT2-HEALTH-SERVICE-ALIGNMENT.CT3-HEALTH-SERVICE-RESOURCE-IDENTIFICATION`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:900 (p. 549); business:895 (p. 549); business:904 (p. 549)	Linked RAD detail; exact locators in manifest.

Capability Overview

This capability establishes the resources a service requires across workforce, facilities, equipment, clinical technology, ICT and support. It makes resource needs visible in the service's alignment context without treating identification as procurement, ownership or allocation. Required resources can therefore be discussed distinctly from resources presently available or deployed.

Definition

Identify required workforce, facilities, equipment, clinical technology, ICT, administration and support resources without owning them.

Boundary

Identifying resources does not operate them, assign them to care or establish collective service readiness.

2.14.2.3. CT3: Health Service Financial Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-service-financial-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Health Service Direction & Stewardship (p. 116) → CT2: Health Service Alignment (p. 121)	Established BC responsibility path.
Structural Canonical ID	`CT1-HEALTH-SERVICE-DIRECTION-STEWARDSHIP.CT2-HEALTH-SERVICE-ALIGNMENT.CT3-HEALTH-SERVICE-FINANCIAL-MANAGEMENT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:901 (p. 549); business:895 (p. 549); business:904 (p. 549)	Linked RAD detail; exact locators in manifest.

Capability Overview

Health Service Financial Management establishes how the service fits applicable funding, financial and charging arrangements. It connects service viability with its surrounding financial context without attempting to become the enterprise finance function. The responsibility is service-specific alignment rather than the production of all financial transactions associated with care.

Definition

Establish the service's fit within funding, financial and charging arrangements without absorbing enterprise finance/accounting.

Boundary

Financial fit does not confer pricing, accounting, billing or enterprise revenue-management authority.

2.14.2.4. CT3: Health Service Implementation Planning

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-service-implementation-planning`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Health Service Direction & Stewardship (p. 116) → CT2: Health Service Alignment (p. 121)	Established BC responsibility path.
Structural Canonical ID	`CT1-HEALTH-SERVICE-DIRECTION-STEWARDSHIP.CT2-HEALTH-SERVICE-ALIGNMENT.CT3-HEALTH-SERVICE-IMPLEMENTATION-PLANNING`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:902 (p. 549); business:895 (p. 549); business:904 (p. 549)	Linked RAD detail; exact locators in manifest.

Capability Overview

Health Service Implementation Planning establishes and coordinates the organisational and surrounding service changes needed for introduction or material change. It considers how the service fits its environment, complementing modelling's establishment support with an alignment view of interdependent change. Its purpose is coherence of the planned transition rather than ownership of every change activity.

Definition

Establish and coordinate surrounding organisational/service changes needed to introduce or materially change the service.

Boundary

Planning does not perform all supporting changes or define the authoritative service offering.

2.15. CT1: Workforce & Organisational Capability

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-workforce-organisational-capability`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT1	Explicit source assertion.
Ancestry	CT1 root	Explicitly established.
Structural Canonical ID	`CT1-WORKFORCE-ORGANISATIONAL-CAPABILITY`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Harmonia-Relevant	Independent of status; never inherited.
Source	business:906 (p. 549); business:910 (p. 549); business:120 (p. 531)	Linked RAD detail; exact locators in manifest.

Capability Overview

This capability governs the workforce and organisational conditions under which health services can operate. Its workforce branch addresses professional standing, participation, privilege and enterprise coverage; its structural branch addresses organisations, recognised locations and identifiable service offerings. Directories can represent these facts, but their representation is separate from business authority to establish or change them.

Definition

Govern practitioner qualification, engagement/privilege and enterprise service coverage, together with organisation identity/structure, recognised locations and identifiable healthcare service offerings.

Intent

Make the organisational and workforce context of care explicit enough to constrain legitimate participation and service availability.

Boundary

Qualification, engagement, privilege, service coverage and deployment are distinct. Organisation or location association does not substitute for material Healthcare Service context.

Harmonia Architectural Relevance

These facts materially constrain care enablement, routing and operational context. Directory support preserves the originating workforce and organisational governance rather than making it platform-owned.

Scope

Contained Capabilities

Capability	Reference Id	Tier
CT2: Health Workforce Management (p. 126)	`HCM-BC-health-workforce-management`	CT2

Capability	Reference Id	Tier
CT2: Healthcare Organisation & Service Structure Management (p. 129)	`HCM-BC-healthcare-organisation-service-structure-management`	CT2

2.15.1. CT2: Health Workforce Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-workforce-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	CT1: Workforce & Organisational Capability (p. 125)	Established BC responsibility path.
Structural Canonical ID	`CT1-WORKFORCE-ORGANISATIONAL-CAPABILITY.CT2-HEALTH-WORKFORCE-MANAGEMENT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:916 (p. 549); business:918 (p. 549); business:920 (p. 549); business:930 (p. 550)	Linked RAD detail; exact locators in manifest.

Capability Overview

Health Workforce Management governs practitioners' professional standing, authorised participation and the ability to cover required enterprise work. Its three children separate qualification from engagement and privilege, and both from whole-enterprise coverage. This prevents an available or rostered person from being treated as automatically qualified and privileged for a particular activity.

Definition

Govern practitioners' professional standing, engagement/privilege and whole-enterprise service coverage.

Intent

Support legitimate practitioner participation and an informed understanding of service-coverage gaps and risks.

Boundary

Coverage is broader than rostering. Applying an available resource to a care requirement and progressing a particular support work item remain separate operational responsibilities.

Scope

Contained Capabilities

Capability	Reference Id	Tier
CT3: Practitioner Qualification Management (p. 127)	`HCM-BC-practitioner-qualification-management`	CT3
CT3: Practitioner Engagement & Privilege Management (p. 127)	`HCM-BC-practitioner-engagement-privilege-management`	CT3

Capability	Reference Id	Tier
CT3: Workforce Service Coverage Management (p. 128)	`HCM-BC-workforce-service-coverage-management`	CT3

2.15.1.1. CT3: Practitioner Qualification Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-practitioner-qualification-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Workforce & Organisational Capability (p. 125) → CT2: Health Workforce Management (p. 126)	Established BC responsibility path.
Structural Canonical ID	`CT1-WORKFORCE-ORGANISATIONAL-CAPABILITY.CT2-HEALTH-WORKFORCE-MANAGEMENT.CT3-PRACTITIONER-QUALIFICATION-MANAGEMENT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:926 (p. 549); business:922 (p. 549); business:930 (p. 550)	Linked RAD detail; exact locators in manifest.

Capability Overview

Practitioner Qualification Management governs professional and clinical standing, including review and changes such as suspension or revocation. It makes qualification a continuing governed concern rather than a static directory attribute. That standing can constrain engagement and privilege while remaining distinguishable from either an employment relationship or permission for a particular activity.

Definition

Govern professional/clinical standing and qualifications, including applicable review, escalation, complaints, penalties/issues, suspension and revocation.

Boundary

Qualification alone does not establish engagement, clinical privilege, device-specific enablement or operational assignment.

2.15.1.2. CT3: Practitioner Engagement & Privilege Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-practitioner-engagement-privilege-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Workforce & Organisational Capability (p. 125) → CT2: Health Workforce Management (p. 126)	Established BC responsibility path.

Name	Value	Comment / Description
Structural Canonical ID	`CT1-WORKFORCE-ORGANISATIONAL-CAPABILITY.CT2-HEALTH-WORKFORCE-MANAGEMENT.CT3-PRACTITIONER-ENGAGEMENT-PRIVILEGE-MANAGEMENT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:927 (p. 549); business:922 (p. 549); business:930 (p. 550)	Linked RAD detail; exact locators in manifest.

Capability Overview

This capability manages how practitioners participate in services and the privileges applicable to that participation. It connects professional standing with roles and permitted capacities in the service context, beyond ordinary employment administration. Separating engagement and privilege from qualification makes it possible to recognise a qualified practitioner who is not authorised for a particular service activity.

Definition

Manage practitioners as service participants, their roles/capacities, applicable privileges and qualification context above ordinary HR employment administration.

Boundary

Representation or consumption of privilege information does not transfer the underlying workforce decision authority to Harmonia.

2.15.1.3. CT3: Workforce Service Coverage Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-workforce-service-coverage-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Workforce & Organisational Capability (p. 125) → CT2: Health Workforce Management (p. 126)	Established BC responsibility path.
Structural Canonical ID	`CT1-WORKFORCE-ORGANISATIONAL-CAPABILITY.CT2-HEALTH-WORKFORCE-MANAGEMENT.CT3-WORKFORCE-SERVICE-COVERAGE-MANAGEMENT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:928 (p. 549); business:922 (p. 549); business:930 (p. 550)	Linked RAD detail; exact locators in manifest.

Capability Overview

Workforce Service Coverage Management considers whether required enterprise work and service obligations can be fulfilled and how coverage should change when gaps or risks arise. Its concern is the adequacy of coverage, broader than the existence of a roster. This supplies a service-wide workforce view distinct from deploying a person to a particular care requirement.

Definition

Observe whether required enterprise work/service obligations can be fulfilled, identify coverage gaps/risks and adjust coverage in response.

Boundary

Coverage management is not individual support-work progression or automatic authority to assign clinical work.

2.15.2. CT2: Healthcare Organisation & Service Structure Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-healthcare-organisation-service-structure-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	CT1: Workforce & Organisational Capability (p. 125)	Established BC responsibility path.
Structural Canonical ID	`CT1-WORKFORCE-ORGANISATIONAL-CAPABILITY.CT2-HEALTHCARE-ORGANISATION-SERVICE-STRUCTURE-MANAGEMENT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:932 (p. 550); business:934 (p. 550); business:936 (p. 550); business:946 (p. 550)	Linked RAD detail; exact locators in manifest.

Capability Overview

This capability establishes the organisation, location and service-offering context through which health services are recognised. Its children distinguish the identity and structure of participants, the definition of places, and what identifiable services are offered under temporal and conditional availability constraints. The distinction allows a service to exist without assuming it is offered everywhere or currently available.

Definition

Manage organisational identity/structure, recognised locations and identifiable organisational service offerings.

Intent

Provide an intelligible account of who offers what service, where and under which conditions.

Boundary

Location definition is not facilities operation. Offering availability does not establish that all resources and dependencies are collectively ready to deliver the service.

Scope

Contained Capabilities

Capability	Reference Id	Tier
CT3: Healthcare Organisation Identity & Structure Management (p. 130)	`HCM-BC-healthcare-organisation-identity-structure-management`	CT3
CT3: Healthcare Location Definition Management (p. 131)	`HCM-BC-healthcare-location-definition-management`	CT3
CT3: Healthcare Service Offering Management (p. 131)	`HCM-BC-healthcare-service-offering-management`	CT3

2.15.2.1. CT3: Healthcare Organisation Identity & Structure Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-healthcare-organisation-identity-structure-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Workforce & Organisational Capability (p. 125) → CT2: Healthcare Organisation & Service Structure Management (p. 129)	Established BC responsibility path.
Structural Canonical ID	`CT1-WORKFORCE-ORGANISATIONAL-CAPABILITY.CT2-HEALTHCARE-ORGANISATION-SERVICE-STRUCTURE-MANAGEMENT.CT3-HEALTHCARE-ORGANISATION-IDENTITY-STRUCTURE-MANAGEMENT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:942 (p. 550); business:938 (p. 550); business:946 (p. 550)	Linked RAD detail; exact locators in manifest.

Capability Overview

This capability establishes the identity and structural relationships of participating organisations and organisational units. It provides the business account of who the participants are and how they relate, which directory representations can then convey. The distinction matters because a represented hierarchy should not create organisational authority merely through document or interface nesting.

Definition

Establish and manage identity and structural relationships of participating organisations and organisational units.

Boundary

Organisation identity does not substitute for location, practitioner or material Healthcare Service context.

2.15.2.2. CT3: Healthcare Location Definition Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-healthcare-location-definition-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Workforce & Organisational Capability (p. 125) → CT2: Healthcare Organisation & Service Structure Management (p. 129)	Established BC responsibility path.
Structural Canonical ID	`CT1-WORKFORCE-ORGANISATIONAL-CAPABILITY.CT2-HEALTHCARE-ORGANISATION-SERVICE-STRUCTURE-MANAGEMENT.CT3-HEALTHCARE-LOCATION-DEFINITION-MANAGEMENT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:943 (p. 550); business:938 (p. 550); business:946 (p. 550)	Linked RAD detail; exact locators in manifest.

Capability Overview

Healthcare Location Definition Management establishes recognised places and their business characteristics and relationships. It gives health-service activity an intelligible location context while remaining separate from maintaining the physical facilities. A defined location can be referenced in offerings and operational activity without implying that it is currently usable or ready for care.

Definition

Establish and manage recognised locations and their applicable business characteristics and relationships.

Boundary

Location definition is not facilities operation, care-place readiness or service availability.

2.15.2.3. CT3: Healthcare Service Offering Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-healthcare-service-offering-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Workforce & Organisational Capability (p. 125) → CT2: Healthcare Organisation & Service Structure Management (p. 129)	Established BC responsibility path.
Structural Canonical ID	`CT1-WORKFORCE-ORGANISATIONAL-CAPABILITY.CT2-HEALTHCARE-ORGANISATION-SERVICE-STRUCTURE-MANAGEMENT.CT3-HEALTHCARE-SERVICE-OFFERING-MANAGEMENT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.

Name	Value	Comment / Description
Source	business:944 (p. 550); business:938 (p. 550); business:946 (p. 550)	Linked RAD detail; exact locators in manifest.

Capability Overview

This capability establishes the identifiable service offered, including provider, place, timing and availability conditions. It makes a service discoverable as an offering rather than merely an abstract service idea. Temporal and conditional availability are material: an offering may exist but be unavailable in the present context, and another pathway may cover different hours.

Definition

Establish/manage what identifiable service is offered, by whom, where, when and under which availability conditions/constraints.

Boundary

An available offering does not establish collective readiness to deliver it; organisation and location alone do not supply the service context.

2.16. CT1: Corporate Resources & Enterprise Services

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-corporate-resources-enterprise-services`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT1	Explicit source assertion.
Ancestry	CT1 root	Explicitly established.
Structural Canonical ID	`CT1-CORPORATE-RESOURCES-ENTERPRISE-SERVICES`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Harmonia-Relevant	Independent of status; never inherited.
Source	business:948 (p. 550); business:952 (p. 550); business:120 (p. 531)	Linked RAD detail; exact locators in manifest.

Capability Overview

This capability retains the financial and corporate-support context material to R1/R2. The financial branch concentrates on information about clinical activity relevant to charging or cost; corporate support covers administrative, physical, technical, people and communication conditions affecting service activity. The limited decomposition makes these dependencies visible without assigning a complete corporate enterprise architecture to Harmonia.

Definition

Provide the bounded clinical-activity charging information and enterprise support context required for R1/R2, without assigning enterprise finance, HR, facilities or ICT operating authority to Harmonia.

Intent

Support health-service activity with the required corporate context and legitimate activity information.

Boundary

Evidence about cost-relevant activity does not perform billing or accounting. Consuming corporate resource state does not acquire the authority to operate those resources.

Harmonia Architectural Relevance

Clinical activity evidence provides material financial support. Corporate constraints may also affect enablement, but contextual consumption does not classify every corporate descendant as Relevant or Core.

Scope

Contained Capabilities

Capability	Reference Id	Tier
CT2: Financial & Commercial Services (p. 133)	`HCM-BC-financial-commercial-services`	CT2
CT2: Corporate Support Services (p. 135)	`HCM-BC-corporate-support-services`	CT2

2.16.1. CT2: Financial & Commercial Services

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-financial-commercial-services`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	CT1: Corporate Resources & Enterprise Services (p. 132)	Established BC responsibility path.
Structural Canonical ID	`CT1-CORPORATE-RESOURCES-ENTERPRISE-SERVICES.CT2-FINANCIAL-COMMERCIAL-SERVICES`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:956 (p. 550); business:958 (p. 550); business:960 (p. 550); business:968 (p. 551)	Linked RAD detail; exact locators in manifest.

Capability Overview

Financial & Commercial Services provides the financial business frame needed for the bounded R1/R2 responsibility. Its established child identifies and supplies information about chargeable or cost-relevant clinical activity. The purpose is to make applicable activity evidence available to the responsible financial context without expanding the model into pricing, claims or revenue management.

Definition

Provide the financial/commercial business context materially required for R1/R2.

Intent

Support legitimate funding, financial and charging purposes with relevant clinical-activity information.

Boundary

Not every exchanged request or activity is chargeable. Provision of evidence does not establish a charging decision or an enterprise financial system.

Scope

Contained Capabilities

Capability	Reference Id	Tier
CT3: Clinical Activity Charging Information Provision (p. 134)	`HCM-BC-clinical-activity-charging-information-provision`	CT3

2.16.1.1. CT3: Clinical Activity Charging Information Provision

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-activity-charging-information-provision`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Corporate Resources & Enterprise Services (p. 132) → CT2: Financial & Commercial Services (p. 133)	Established BC responsibility path.
Structural Canonical ID	`CT1-CORPORATE-RESOURCES-ENTERPRISE-SERVICES.CT2-FINANCIAL-COMMERCIAL-SERVICES.CT3-CLINICAL-ACTIVITY-CHARGING-INFORMATION-PROVISION`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:966 (p. 551); business:962 (p. 550); business:968 (p. 551)	Linked RAD detail; exact locators in manifest.

Capability Overview

This capability identifies and supplies information about clinical or health-service activity relevant to legitimate charging, funding or cost purposes. Requests, service types, quantities and utilisation context may contribute evidence, but chargeability cannot be inferred from the fact that Harmonia mediated an interaction. Its outcome is usable activity information for the responsible financial context.

Definition

Identify and provide information about chargeable or cost-relevant clinical/health-service activity where required for applicable funding, financial or charging purposes.

Boundary

Evidence provision does not perform pricing, billing, claims, accounting or revenue management.

2.16.2. CT2: Corporate Support Services

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-corporate-support-services`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	CT1: Corporate Resources & Enterprise Services (p. 132)	Established BC responsibility path.
Structural Canonical ID	`CT1-CORPORATE-RESOURCES-ENTERPRISE-SERVICES.CT2-CORPORATE-SUPPORT-SERVICES`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:970 (p. 551); business:972 (p. 551); business:974 (p. 551); business:987 (p. 551)	Linked RAD detail; exact locators in manifest.

Capability Overview

Corporate Support Services frames enterprise support whose resources, availability or constraints affect health-service activity. Its children retain recognisable administrative, facilities, infrastructure, communications, people and operational-oversight responsibilities. This explains why their context matters to service delivery while keeping the people who operate those concerns accountable for them.

Definition

Provide enterprise support whose resources, availability, state or constraints may materially affect service activity.

Intent

Make material support conditions visible to health-service activity without confusing consumption with ownership.

Boundary

No further R1/R2 decomposition is established. Context from these abilities does not settle the collective service-readiness placement question.

Scope

Contained Capabilities

Capability	Reference Id	Tier
CT3: Back-Office Services (p. 136)	`HCM-BC-back-office-services`	CT3
CT3: Facilities Management (p. 136)	`HCM-BC-facilities-management`	CT3
CT3: IT Infrastructure Management (p. 137)	`HCM-BC-it-infrastructure-management`	CT3
CT3: Communications & Publicity (p. 138)	`HCM-BC-communications-publicity`	CT3
CT3: People Management (p. 138)	`HCM-BC-people-management`	CT3
CT3: Operational Oversight (p. 139)	`HCM-BC-operational-oversight`	CT3

2.16.2.1. CT3: Back-Office Services

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-back-office-services`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Corporate Resources & Enterprise Services (p. 132) → CT2: Corporate Support Services (p. 135)	Established BC responsibility path.
Structural Canonical ID	`CT1-CORPORATE-RESOURCES-ENTERPRISE-SERVICES.CT2-CORPORATE-SUPPORT-SERVICES.CT3-BACK-OFFICE-SERVICES`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:980 (p. 551); business:976 (p. 551); business:987 (p. 551)	Linked RAD detail; exact locators in manifest.

Capability Overview

Back-Office Services provides the administrative and support activity on which health-service operations depend. Within Corporate Support Services it retains an enterprise context for dependencies that may affect whether service activity can proceed. The model stops at this broad responsibility rather than allocating each administrative function to Harmonia.

Definition

Provide administrative/support services on which health-service activity depends.

Boundary

Consumption of support status does not transfer administrative operating authority or establish a more detailed decomposition.

2.16.2.2. CT3: Facilities Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-facilities-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Corporate Resources & Enterprise Services (p. 132) → CT2: Corporate Support Services (p. 135)	Established BC responsibility path.
Structural Canonical ID	`CT1-CORPORATE-RESOURCES-ENTERPRISE-SERVICES.CT2-CORPORATE-SUPPORT-SERVICES.CT3-FACILITIES-MANAGEMENT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:981 (p. 551); business:976 (p. 551); business:987 (p. 551)	Linked RAD detail; exact locators in manifest.

Capability Overview

Facilities Management manages physical facilities and services, including the state and availability context relevant to health activity. It contributes the real-world conditions that location definitions alone cannot establish. This distinguishes a place recognised in a directory from a facility being operated and maintained for use.

Definition

Manage physical facilities/services and applicable resource, location, availability and facility-state context.

Boundary

Harmonia's consumption of facility constraints does not make it the facilities operator or establish collective service readiness.

2.16.2.3. CT3: IT Infrastructure Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-it-infrastructure-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Corporate Resources & Enterprise Services (p. 132) → CT2: Corporate Support Services (p. 135)	Established BC responsibility path.
Structural Canonical ID	`CT1-CORPORATE-RESOURCES-ENTERPRISE-SERVICES.CT2-CORPORATE-SUPPORT-SERVICES.CT3-IT-INFRASTRUCTURE-MANAGEMENT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:982 (p. 551); business:976 (p. 551); business:987 (p. 551)	Linked RAD detail; exact locators in manifest.

Capability Overview

IT Infrastructure Management operates the underlying technology infrastructure and its availability. It supplies an enterprise support context for health-service activity that relies on functioning technology. Its business outcome is operational infrastructure, distinct from governance of health-information meaning or the application responsibilities realised on that infrastructure.

Definition

Operate/manage underlying technology infrastructure and its availability.

Boundary

Infrastructure machinery does not define Harmonia's information semantics or acquire clinical authority.

2.16.2.4. CT3: Communications & Publicity

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-communications-publicity`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Corporate Resources & Enterprise Services (p. 132) → CT2: Corporate Support Services (p. 135)	Established BC responsibility path.
Structural Canonical ID	`CT1-CORPORATE-RESOURCES-ENTERPRISE-SERVICES.CT2-CORPORATE-SUPPORT-SERVICES.CT3-COMMUNICATIONS-PUBLICITY`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:983 (p. 551); business:976 (p. 551); business:987 (p. 551)	Linked RAD detail; exact locators in manifest.

Capability Overview

Communications & Publicity manages organisational publication and communication arrangements relevant to information availability and distribution. It provides enterprise communication context without being identical to clinical record exchange or digitally mediated participant interaction. The governed decomposition retains that broad support role without adding specific publicity or communications functions.

Definition

Manage organisational communication/publication arrangements relevant to information availability/distribution.

Boundary

Organisational communication does not originate the clinical facts or governed conclusions it may convey.

2.16.2.5. CT3: People Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-people-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Corporate Resources & Enterprise Services (p. 132) → CT2: Corporate Support Services (p. 135)	Established BC responsibility path.
Structural Canonical ID	`CT1-CORPORATE-RESOURCES-ENTERPRISE-SERVICES.CT2-CORPORATE-SUPPORT-SERVICES.CT3-PEOPLE-MANAGEMENT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:984 (p. 551); business:976 (p. 551); business:987 (p. 551)	Linked RAD detail; exact locators in manifest.

Capability Overview

People Management addresses people across the organisation as an enterprise support responsibility. It retains a scope broader than the practitioner-specific standing, privilege and coverage concerns of Health Workforce Management. This distinction allows health-service activity to consume relevant people context without redefining all workforce administration as clinical governance.

Definition

Manage people across the organisation, distinct from practitioner-specific Health Workforce Management.

Boundary

General people administration does not itself establish practitioner clinical qualification or privilege.

2.16.2.6. CT3: Operational Oversight

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-operational-oversight`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Corporate Resources & Enterprise Services (p. 132) → CT2: Corporate Support Services (p. 135)	Established BC responsibility path.
Structural Canonical ID	`CT1-CORPORATE-RESOURCES-ENTERPRISE-SERVICES.CT2-CORPORATE-SUPPORT-SERVICES.CT3-OPERATIONAL-OVERSIGHT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:985 (p. 551); business:976 (p. 551); business:987 (p. 551)	Linked RAD detail; exact locators in manifest.

Capability Overview

Operational Oversight observes broader operating conditions affecting the organisation's ability to work and deliver services. It provides context beyond an individual work item or care place, helping make material constraints visible. Within corporate support it remains a broad enterprise responsibility rather than an inferred owner for every operational-readiness question.

Definition

Observe broader operational conditions affecting the organisation's ability to work or deliver services.

Boundary

Oversight does not resolve collective service-readiness placement or constitute independent assurance merely through observation.

2.17. CT1: Health Service Governance

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-service-governance`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT1	Explicit source assertion.
Ancestry	CT1 root	Explicitly established.
Structural Canonical ID	`CT1-HEALTH-SERVICE-GOVERNANCE`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:993 (p. 551); business:997 (p. 551); business:120 (p. 531)	Linked RAD detail; exact locators in manifest.

Capability Overview

Health Service Governance establishes the normative envelope within which health-service activity operates: what obligations apply, who may decide, what constraints bind decisions and how adherence will be assessed and reported. Its framework and compliance branches distinguish the authoritative requirements from the arrangements used to judge satisfaction. Governance therefore shapes legitimate activity without becoming the manager of every governed operation.

Definition

Establish and maintain the obligations, authorities, constraints, policies and governance framework under which health-service activity must operate, and define how adherence will be assessed, checked and reported.

Intent

Make obligations, decision authority and compliance expectations explicit and traceable.

Boundary

Operational management chooses and progresses means within the governing envelope; assurance evaluates evidence. Neither responsibility is absorbed merely because governance defines its constraints.

Harmonia Architectural Relevance

Formal relevance remains Unresolved. The requirements plainly constrain managed activity, but the RAD has not assigned enough platform participation or enabling derivation to adjudicate its relevance classification.

Scope

Contained Capabilities

Capability	Reference Id	Tier
CT2: Governance Framework Management (p. 141)	`HCM-BC-governance-framework-management`	CT2
CT2: Compliance Governance (p. 143)	`HCM-BC-compliance-governance`	CT2

Additional Notes / Unresolved Matters

Enablement: No Business Enabling derivation is established for Health Service Governance.

2.17.1. CT2: Governance Framework Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-governance-framework-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	CT1: Health Service Governance (p. 140)	Established BC responsibility path.
Structural Canonical ID	`CT1-HEALTH-SERVICE-GOVERNANCE.CT2-GOVERNANCE-FRAMEWORK-MANAGEMENT`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:1003 (p. 551); business:1005 (p. 552); business:1007 (p. 552); business:1016 (p. 552)	Linked RAD detail; exact locators in manifest.

Capability Overview

Governance Framework Management establishes what must be satisfied and the legitimate authority and bounds for organisational decisions. Its two children separate obligations attributable to source authority from the allocation and limitation of decision authority. Together they supply the authoritative reference against which operational arrangements and compliance assessments can be considered.

Definition

Establish the authoritative WHAT and legitimate working parameters.

Intent

Keep required outcomes and legitimate decision parameters distinguishable and traceable to their authority.

Boundary

Interpreting an external obligation or engaging with a regulator does not transfer that body's authority. Operational means remain with management.

Scope

Contained Capabilities

Capability	Reference Id	Tier
CT3: Policy / Obligation Governance (p. 142)	HCM-BC-policy-obligation-governance	CT3
CT3: Authority & Constraint Governance (p. 142)	HCM-BC-authority-constraint-governance	CT3

2.17.1.1. CT3: Policy / Obligation Governance

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	HCM-BC-policy-obligation-governance	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Health Service Governance (p. 140) → CT2: Governance Framework Management (p. 141)	Established BC responsibility path.
Structural Canonical ID	CT1-HEALTH-SERVICE-GOVERNANCE.CT2-GOVERNANCE-FRAMEWORK-MANAGEMENT.CT3-POLICY-OBLIGATION-GOVERNANCE	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:1013 (p. 552); business:1009 (p. 552); business:1016 (p. 552)	Linked RAD detail; exact locators in manifest.

Capability Overview

Policy / Obligation Governance establishes and maintains the authoritative requirements under which the organisation operates. It interprets applicable legislative, regulatory, contractual, policy and other sources while preserving traceability to their authority. The purpose is a legitimate account of what must be satisfied, rather than an operational choice about how to satisfy it.

Definition

Identify, interpret, establish and maintain authoritative legislative, regulatory, policy, standards, guideline, contractual and other obligations, attributable and traceable to source authority.

Boundary

Interpreting obligations does not acquire the regulator's authority or determine operational means.

2.17.1.2. CT3: Authority & Constraint Governance

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	HCM-BC-authority-constraint-governance	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Health Service Governance (p. 140) → CT2: Governance Framework Management (p. 141)	Established BC responsibility path.

Name	Value	Comment / Description
Structural Canonical ID	`CT1-HEALTH-SERVICE-GOVERNANCE.CT2-GOVERNANCE-FRAMEWORK-MANAGEMENT.CT3-AUTHORITY-CONSTRAINT-GOVERNANCE`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:1014 (p. 552); business:1009 (p. 552); business:1016 (p. 552)	Linked RAD detail; exact locators in manifest.

Capability Overview

This capability establishes who may make or approve organisational decisions and the bounds within which that authority applies. It complements obligation governance by distinguishing required outcomes from legitimate decision powers. A participant can therefore be responsible for acting within constraints without being authorised to redefine the governing requirement.

Definition

Establish who may make, approve or govern organisational decisions/artefacts and the bounds within which authority may be exercised.

Boundary

Authority allocation does not itself perform an operational decision or a runtime access-control evaluation.

2.17.2. CT2: Compliance Governance

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-compliance-governance`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	CT1: Health Service Governance (p. 140)	Established BC responsibility path.
Structural Canonical ID	`CT1-HEALTH-SERVICE-GOVERNANCE.CT2-COMPLIANCE-GOVERNANCE`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:1018 (p. 552); business:1020 (p. 552); business:1022 (p. 552); business:1031 (p. 552)	Linked RAD detail; exact locators in manifest.

Capability Overview

Compliance Governance defines how adherence to established obligations, authorities and constraints will be determined. Assessment Definition specifies the evidence and basis for evaluation; Reporting & Checkpoint Definition specifies where adherence is checked and what must be communicated. This makes the compliance test explicit without treating the design of a test as its successful execution.

Definition

Define how adherence to the authoritative WHAT will be determined, checked and reported.

Intent

Provide a defensible basis for checking and reporting whether the governing envelope has been respected.

Boundary

Defining assessment arrangements does not perform assurance or manage the governed activity. The obligations being assessed retain their authoritative source.

Scope

Contained Capabilities

Capability	Reference Id	Tier
CT3: Compliance Assessment Definition (p. 144)	`HCM-BC-compliance-assessment-definition`	CT3
CT3: Governance Reporting & Checkpoint Definition (p. 145)	`HCM-BC-governance-reporting-checkpoint-definition`	CT3

2.17.2.1. CT3: Compliance Assessment Definition

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-compliance-assessment-definition`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Health Service Governance (p. 140) → CT2: Compliance Governance (p. 143)	Established BC responsibility path.
Structural Canonical ID	`CT1-HEALTH-SERVICE-GOVERNANCE.CT2-COMPLIANCE-GOVERNANCE.CT3-COMPLIANCE-ASSESSMENT-DEFINITION`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:1028 (p. 552); business:1024 (p. 552); business:1031 (p. 552)	Linked RAD detail; exact locators in manifest.

Capability Overview

Compliance Assessment Definition specifies the evidence, measures, criteria and arrangements sufficient to evaluate adherence. It makes the assessment basis explicit after obligations, authorities and constraints have been established. This preserves the distinction between defining what compliance means and determining whether the available evidence demonstrates it in a particular case.

Definition

Define the evidence, measures, criteria, controls and assessment arrangements sufficient to evaluate compliance with established obligations, authorities and constraints.

Boundary

Assessment definition does not replace the authoritative obligation, perform assurance or establish a compliance finding.

2.17.2.2. CT3: Governance Reporting & Checkpoint Definition

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-governance-reporting-checkpoint-definition`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Health Service Governance (p. 140) → CT2: Compliance Governance (p. 143)	Established BC responsibility path.
Structural Canonical ID	`CT1-HEALTH-SERVICE-GOVERNANCE.CT2-COMPLIANCE-GOVERNANCE.CT3-GOVERNANCE-REPORTING-CHECKPOINT-DEFINITION`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:1029 (p. 552); business:1024 (p. 552); business:1031 (p. 552)	Linked RAD detail; exact locators in manifest.

Capability Overview

This capability specifies where adherence must be checked and what governance or compliance information must be reported. It gives the governance framework observable expectations without treating reporting requirements as completed reporting. Within Compliance Governance it complements the assessment basis by making checking and communication obligations explicit.

Definition

Establish where/how adherence is checked and what governance/compliance information must be reported.

Boundary

Defining a checkpoint does not perform the check or take over progression of the governed operation.

2.18. CT1: Health Service Assurance

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-service-assurance`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT1	Explicit source assertion.
Ancestry	CT1 root	Explicitly established.
Structural Canonical ID	`CT1-HEALTH-SERVICE-ASSURANCE`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Harmonia-Relevant	Independent of status; never inherited.
Source	business:1033 (p. 552); business:1037 (p. 552); business:120 (p. 531)	Linked RAD detail; exact locators in manifest.
Enabled By	BEC: Assurance Design; BEC: Assurance Criteria Management; BEC: Governed Assurance	Bounded support; not decomposition or runtime dependency.

Capability Overview

Health Service Assurance establishes justified confidence in the ability to deliver health services appropriately. Specification defines how confidence will be established; evaluation determines what the available evidence supports. Its subject is macro-level service delivery, with workforce, clinical, information, security and other concerns contributing evidence or criteria without becoming separate assurance trees.

Where independent governed assurance is required, the subject cannot control its own evaluation or conclusion. Independence concerns control of the assurance judgement rather than requiring a separate organisation or technology installation.

Definition

Establish and maintain justified confidence that health services can be, and are being, delivered in accordance with applicable expectations, obligations, constraints and required levels of performance.

Intent

Distinguish justified confidence, qualified conclusions and insufficient evidence so that assurance communicates what is actually supported.

Boundary

Assurance manages its own evaluation and does not assign, remediate or progress the subject activity. Harmonia does not thereby assume responsibility for clinical adequacy or professional practice.

Harmonia Architectural Relevance

Approved independent governed assurance materially supports service confidence. The three established assurance BECs provide bounded support; they are not remapped to the newer BC descendants and are not assigned implementation components.

Scope

Contained Capabilities

Capability	Reference Id	Tier
CT2: Health Service Assurance Specification (p. 146)	`HCM-BC-health-service-assurance-specification`	CT2
CT2: Assurance Evaluation (p. 149)	`HCM-BC-assurance-evaluation`	CT2

2.18.1. CT2: Health Service Assurance Specification

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-service-assurance-specification`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.

Name	Value	Comment / Description
Ancestry	CT1: Health Service Assurance (p. 145)	Established BC responsibility path.
Structural Canonical ID	`CT1-HEALTH-SERVICE-ASSURANCE.CT2-HEALTH-SERVICE-ASSURANCE-SPECIFICATION`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:1043 (p. 552); business:1045 (p. 553); business:1047 (p. 553); business:1056 (p. 553)	Linked RAD detail; exact locators in manifest.

Capability Overview

This capability defines the approach and evaluation criteria through which service-delivery assurance can be justified. Approach Definition addresses scope, method, responsibility, evidence and applicable independence; Criteria Governance establishes the basis for judging sufficiency. Their separation distinguishes the design of an assurance activity from the normative or evaluative material it applies.

Definition

Specify how macro-level service-delivery assurance will be undertaken and what sufficiently supports confidence.

Intent

Make the intended basis of confidence explicit before a conclusion is treated as assurance.

Boundary

An approach cannot waive a higher-level independence requirement. Contributing specialist concerns do not create separate Business Assurance hierarchies.

Scope

Contained Capabilities

Capability	Reference Id	Tier
CT3: Assurance Approach Definition (p. 147)	`HCM-BC-assurance-approach-definition`	CT3
CT3: Assurance Evaluation Criteria Governance (p. 148)	`HCM-BC-assurance-evaluation-criteria-governance`	CT3

2.18.1.1. CT3: Assurance Approach Definition

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-assurance-approach-definition`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Health Service Assurance (p. 145) → CT2: Health Service Assurance Specification (p. 146)	Established BC responsibility path.

Name	Value	Comment / Description
Structural Canonical ID	`CT1-HEALTH-SERVICE-ASSURANCE.CT2-HEALTH-SERVICE-ASSURANCE-SPECIFICATION.CT3-ASSURANCE-APPROACH-DEFINITION`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:1053 (p. 553); business:1049 (p. 553); business:1056 (p. 553)	Linked RAD detail; exact locators in manifest.

Capability Overview

Assurance Approach Definition establishes how service-delivery assurance will be undertaken, including scope, evidence, methods, responsibilities, timing and required independence. It gives assurance activity a defensible approach rather than assuming any available observation can support confidence. The approach must respect applicable higher-level requirements when deciding what constitutes sufficient evaluation.

Definition

Define scope, evidence expectations, responsibilities, methods, applicable degree of independence and timing/frequency for service-delivery assurance.

Boundary

Self-attestation cannot replace required independent governed assurance, and an approach does not itself establish a conclusion.

2.18.1.2. CT3: Assurance Evaluation Criteria Governance

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-assurance-evaluation-criteria-governance`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Health Service Assurance (p. 145) → CT2: Health Service Assurance Specification (p. 146)	Established BC responsibility path.
Structural Canonical ID	`CT1-HEALTH-SERVICE-ASSURANCE.CT2-HEALTH-SERVICE-ASSURANCE-SPECIFICATION.CT3-ASSURANCE-EVALUATION-CRITERIA-GOVERNANCE`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:1054 (p. 553); business:1049 (p. 553); business:1056 (p. 553)	Linked RAD detail; exact locators in manifest.

Capability Overview

This capability establishes and maintains the criteria against which assurance sufficiency is judged. It makes the evaluative basis of confidence explicit across the different concerns that can affect service delivery. Criteria can draw on clinical, workforce, technology, information or other contexts without turning those subjects into independent assurance capability trees.

Definition

Establish and maintain criteria against which sufficient assurance can be determined.

Boundary

Criteria governance is distinct from applying criteria to evidence and does not remap the existing assurance BEC responsibilities.

2.18.2. CT2: Assurance Evaluation

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-assurance-evaluation`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT2	Explicit source assertion.
Ancestry	CT1: Health Service Assurance (p. 145)	Established BC responsibility path.
Structural Canonical ID	`CT1-HEALTH-SERVICE-ASSURANCE.CT2-ASSURANCE-EVALUATION`	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:1058 (p. 553); business:1060 (p. 553); business:1062 (p. 553); business:1071 (p. 553)	Linked RAD detail; exact locators in manifest.

Capability Overview

Assurance Evaluation determines what confidence the evidence legitimately supports under the established approach and criteria. Evidence Assessment considers the material against that basis; Conclusion Determination establishes the resulting position, including qualifications and unresolved concerns. The distinction prevents accumulated evidence or execution completion from being mistaken for a justified assurance conclusion.

Definition

Determine what justified confidence the applicable evidence supports.

Intent

State the supported level and limits of confidence without concealing insufficient evidence.

Boundary

Insufficient evidence establishes neither assurance nor non-assurance. Operational response remains with the activity's responsible capability, and the existing assurance BEC ownership is unchanged.

Scope

Contained Capabilities

Capability	Reference Id	Tier
CT3: Assurance Evidence Assessment (p. 150)	HCM-BC-assurance-evidence-assessment	CT3
CT3: Assurance Conclusion Determination (p. 150)	HCM-BC-assurance-conclusion-determination	CT3

2.18.2.1. CT3: Assurance Evidence Assessment

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	HCM-BC-assurance-evidence-assessment	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Health Service Assurance (p. 145) → CT2: Assurance Evaluation (p. 149)	Established BC responsibility path.
Structural Canonical ID	CT1-HEALTH-SERVICE-ASSURANCE.CT2-ASSURANCE-EVALUATION.CT3-ASSURANCE-EVIDENCE-ASSESSMENT	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:1068 (p. 553); business:1064 (p. 553); business:1071 (p. 553)	Linked RAD detail; exact locators in manifest.

Capability Overview

Assurance Evidence Assessment evaluates available evidence against the established approach and criteria. It produces the assessed basis from which a legitimate conclusion may be determined, retaining qualifications and evidentiary limits. The responsibility is evaluative rather than the operational production of the source information or remediation of what it reveals.

Definition

Evaluate available evidence against the established assurance approach and criteria.

Boundary

Assessment is distinct from conclusion determination; unavailable or insufficient evidence must remain explicit.

2.18.2.2. CT3: Assurance Conclusion Determination

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	HCM-BC-assurance-conclusion-determination	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	CT3	Explicit source assertion.
Ancestry	CT1: Health Service Assurance (p. 145) → CT2: Assurance Evaluation (p. 149)	Established BC responsibility path.
Structural Canonical ID	CT1-HEALTH-SERVICE-ASSURANCE.CT2-ASSURANCE-EVALUATION.CT3-ASSURANCE-CONCLUSION-DETERMINATION	Allocated v0.1; structural responsibility identity.
Harmonia Relevance	Unresolved	Independent of status; never inherited.

Name	Value	Comment / Description
Source	business:1069 (p. 553); business:1064 (p. 553); business:1071 (p. 553)	Linked RAD detail; exact locators in manifest.

Capability Overview

Assurance Conclusion Determination establishes what confidence is legitimately supported by the assessment. It preserves qualifications and unresolved concerns rather than forcing incomplete evidence into a positive or negative result. Within Assurance Evaluation it is the responsibility that gives the conclusion its standing, distinct from gathering or assessing the supporting material.

Definition

Establish the legitimately supported conclusion, including insufficiency, qualification and unresolved concerns where applicable.

Boundary

Insufficiency proves neither assurance nor non-assurance, and the conclusion does not manage the subject's operational response.

2.19. Candidate concerns with unresolved placement

These three named concerns remain candidates; no tier or parent is allocated. Other provisional proposals remain in the linked RAD prose and are not promoted to independently established elements.

2.19.1. Candidate BC: Health Service Operational Readiness

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-service-operational-readiness`	Stable publication identity.
Architectural Status	Candidate	Not approved hierarchy; retain candidate standing.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Candidate placement/identity not approved.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:1201 (p. 557)	Linked RAD detail; exact locators in manifest.

Capability Overview

This candidate concern asks whether an identifiable health-service offering can actually be delivered from its operational context. Its meaning is collective: workforce, places, equipment, materials, utilities and other dependencies must be considered together without acquiring their ownership. It explains why a defined offering, available practitioner or ready bed alone cannot establish service readiness.

Boundary

Tier, parent and CT1 ancestry remain unresolved. Site-level and session-level exercises do not establish separate capabilities, and no convenient parent is allocated here.

Additional Notes / Unresolved Matters

Identified Readiness Placement: OI-BC-002: collective service-readiness meaning is identified, but tier/CT2 parent/CT1 ancestry and placement remain unresolved.

2.19.2. Candidate BC: Clinical Device Maintenance Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-clinical-device-maintenance-management`	Stable publication identity.
Architectural Status	Candidate	Not approved hierarchy; retain candidate standing.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Candidate placement/identity not approved.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:1216 (p. 557)	Linked RAD detail; exact locators in manifest.

Capability Overview

This retained concern addresses maintaining or restoring acceptable device condition through testing, calibration, inspection, repair and related maintenance activity. Its purpose is distinguishable from observing device condition and deciding overall lifecycle disposition. The definition is useful architectural context, but the explicit seven-child device set omits this concern.

Boundary

Membership, tier and ancestry require adjudication; this description does not establish an eighth Clinical Technology Management child.

Additional Notes / Unresolved Matters

Maintenance Membership: DN-BC-015 defines maintenance separately but omits it from its explicit seven-child CT3 set; inclusion/tier/ancestry requires adjudication.

2.19.3. Candidate BC: Health Information Sharing Programme Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BC-health-information-sharing-programme-management`	Stable publication identity.
Architectural Status	Candidate	Not approved hierarchy; retain candidate standing.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Candidate placement/identity not approved.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	business:1222 (p. 557)	Linked RAD detail; exact locators in manifest.

Capability Overview

The RAD retains this named working concern while establishing only its corrected spelling. The name suggests an organised information-sharing context, but it does not provide enough governed meaning to decide what that programme manages or how it differs from the established exchange responsibilities. A fuller architectural overview would require inventing scope that the source has not supplied.

Additional Notes / Unresolved Matters

Definition, tier and parent remain unresolved. This is an explicitly constrained description, not an approved programme-management responsibility or an allocated structural path.

Additional Notes / Unresolved Matters

Sharing Programme Placement: DN-BC-027 establishes spelling of this named working concern, not a precise tier/parent or definition.

3. Business Enabling Capabilities and owned behaviour

All 76 established BEC contexts have unresolved CT tiers and complete ancestry. This chapter groups them by documented contextual view and responsibility; its heading depth is navigation, not a CT assertion. Features, Functions, Services and Processes below retain their actual owning context. Function/Service/Process and Feature associations form a graph, recorded in the relationship register. A Feature listing is not a claim of complete CT1 → CT2 → CT3 → FT reconciliation.

3.1. Entity Management

Capability	Reference Id
BEC: Client Administration (p. 154)	`HCM-BEC-client-administration`
BEC: Provider Administration (p. 155)	`HCM-BEC-provider-administration`
BEC: Organisation Administration (p. 166)	`HCM-BEC-organisation-administration`
BEC: Location Administration (p. 173)	`HCM-BEC-location-administration`
BEC: Health Service Administration (p. 178)	`HCM-BEC-health-service-administration`
BEC: Health Product Administration (p. 185)	`HCM-BEC-health-product-administration`
BEC: Clinical Device Administration (p. 185)	`HCM-BEC-clinical-device-administration`
BEC: Person Identity (p. 192)	`HCM-BEC-person-identity`
BEC: Healthcare Subject (p. 200)	`HCM-BEC-healthcare-subject`
BEC: Client Relationship (p. 203)	`HCM-BEC-client-relationship`
BEC: Client Privacy (p. 207)	`HCM-BEC-client-privacy`

3.1.1. BEC: Client Administration

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-client-administration`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	enabling:76 (p. 564); enabling:112 (p. 565)	Linked RAD detail; exact locators in manifest.

Capability Overview

Client Administration provides the representation and demographic context needed to recognise individuals receiving care. It brings identity, healthcare-subject context, relationships and

privacy into the same administrative frame while preserving their different responsibilities. The established partial chain to Person Identity is retained; the catalogue's other contextual descriptions do not supply a complete capability hierarchy.

Intent

Keep the individual and the circumstances of their representation intelligible across care information and operational activity.

Boundary

R1/R2 identifier correlation and administrative correction do not make Harmonia the authoritative clinical EMPI. Probabilistic matching and master identity merge/unmerge remain an uncommitted roadmap candidate.

Scope

Established Partial Responsibility Relationship

Capability	Reference Id	Tier
BEC: Person Identity (p. 192)	`HCM-BEC-person-identity`	Unresolved

3.1.2. BEC: Provider Administration

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-provider-administration`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	enabling:81 (p. 564); enabling:137 (p. 565); em:86 (p. 636)	Linked RAD detail; exact locators in manifest.

Capability Overview

Provider Administration maintains the practitioner, role, affiliation and engagement context needed to interpret healthcare participation. National identifiers, professional profiles, role-specific endpoints and privilege records support different questions: who the practitioner is, in what capacity they act and how they can be reached. Directory representation makes those distinctions usable without replacing the business governance that establishes professional standing.

Intent

Provide a trustworthy provider context for routing, participation and the application of relevant qualification and privilege constraints.

Boundary

Practitioner Role Resolution establishes neither Clinical Privilege nor Operational Privilege. The descriptive practitioner and role groupings do not establish reconciled CT ancestry.

Scope

Contained Features and Established Owned Behaviour

Element	Type	Reference Id
Feature: National Practitioner Verification (p. 156)	Feature	FEAT-EM-11
Feature: Practitioner Profile Governance (p. 157)	Feature	FEAT-EM-12
Feature: Role & Specialty Modeling (p. 158)	Feature	FEAT-EM-13
Feature: Electronic Endpoint Binding (p. 158)	Feature	FEAT-EM-14
Feature: Practitioner-Organisation Affiliation (p. 159)	Feature	FEAT-EM-15
Feature: Clinical Privileging Record (p. 160)	Feature	FEAT-EM-16
Function: Verify Practitioner Registration (p. 160)	Function	HCM-FN-verify-practitioner-registration
Service: Practitioner Verification Service (p. 161)	Service	HCM-SV-practitioner-verification-service
Function: Govern Practitioner Profile (p. 161)	Function	HCM-FN-govern-practitioner-profile
Service: Practitioner Directory Query (p. 161)	Service	HCM-SV-practitioner-directory-query
Function: Maintain Practitioner Roles (p. 162)	Function	HCM-FN-maintain-practitioner-roles
Service: Practitioner Role Resolution (p. 162)	Service	HCM-SV-practitioner-role-resolution
Function: Bind Practitioner Electronic Endpoints (p. 163)	Function	HCM-FN-bind-practitioner-electronic-endpoints
Service: Practitioner Endpoint Lookup (p. 163)	Service	HCM-SV-practitioner-endpoint-lookup
Function: Maintain Practitioner Affiliations (p. 163)	Function	HCM-FN-maintain-practitioner-affiliations
Service: Practitioner Affiliation Resolution (p. 164)	Service	HCM-SV-practitioner-affiliation-resolution
Function: Maintain Clinical Privileges (p. 164)	Function	HCM-FN-maintain-clinical-privileges
Service: Clinical Privilege Verification (p. 165)	Service	HCM-SV-clinical-privilege-verification
Process: Practitioner Verification Process (p. 165)	Process	HCM-PR-practitioner-verification-process

3.1.2.1. Feature: National Practitioner Verification

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-EM-11	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Provider Administration (p. 155)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:142 (p. 565)	Linked RAD detail; exact locators in manifest.

Feature Overview

National verification connects practitioner identifiers and registration information to their recognised source context. It helps distinguish an identifiable professional from an unverified directory assertion without implying that verification alone grants local clinical privileges.

Definition

Ingest, validate, and verify national practitioner identifiers (e.g., HPI-I) and professional registration status.

Established Behavioural Context

Element	Relationship to this Feature
Function: Verify Practitioner Registration (p. 160)	realises feature
Service: Practitioner Verification Service (p. 161)	associated feature

3.1.2.2. Feature: Practitioner Profile Governance

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-EM-12	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Provider Administration (p. 155)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:143 (p. 565)	Linked RAD detail; exact locators in manifest.

Feature Overview

Practitioner profile governance maintains the professional context used across care information and communication. Qualifications and practice status remain attributable facts, distinct from the organisation-specific role or privilege under which the practitioner may act.

Definition

Maintain core professional demographic details, qualifications, and active practice status.

Established Behavioural Context

Element	Relationship to this Feature
Function: Govern Practitioner Profile (p. 161)	realises feature
Service: Practitioner Directory Query (p. 161)	associated feature

3.1.2.3. Feature: Role & Specialty Modeling

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-EM-13`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Provider Administration (p. 155)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:147 (p. 565)	Linked RAD detail; exact locators in manifest.

Feature Overview

Role and specialty modelling describes the capacity in which a practitioner participates at an organisation or location. This keeps the person separate from each acting role and its service context; role resolution itself establishes neither Clinical Privilege nor Operational Privilege.

Definition

Define and associate clinical specialties, practice roles, and service capacities for a practitioner.

Established Behavioural Context

Element	Relationship to this Feature
Function: Maintain Practitioner Roles (p. 162)	realises feature
Service: Practitioner Role Resolution (p. 162)	associated feature

3.1.2.4. Feature: Electronic Endpoint Binding

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-EM-14`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Provider Administration (p. 155)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:148 (p. 565)	Linked RAD detail; exact locators in manifest.

Feature Overview

Endpoint binding makes a verified communication destination specific to the practitioner's acting role. It supports routing to the intended professional capacity rather than assuming that every address attached to the person is interchangeable across organisations and services.

Definition

Associate validated communication endpoints (secure messaging addresses, direct routing queues) with specific practitioner roles.

Established Behavioural Context

Element	Relationship to this Feature
Function: Bind Practitioner Electronic Endpoints (p. 163)	realises feature
Service: Practitioner Endpoint Lookup (p. 163)	associated feature

3.1.2.5. Feature: Practitioner-Organisation Affiliation

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-EM-15`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Provider Administration (p. 155)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:152 (p. 565)	Linked RAD detail; exact locators in manifest.

Feature Overview

Affiliation records keep practitioner participation in an organisation temporally intelligible. They support interpretation of an actor's organisational context while preserving that affiliation, employment, qualification and permission to undertake a particular activity answer different questions.

Definition

Maintain temporal affiliations between practitioners and healthcare organizations.

Established Behavioural Context

Element	Relationship to this Feature
Function: Maintain Practitioner Affiliations (p. 163)	realises feature
Service: Practitioner Affiliation Resolution (p. 164)	associated feature

3.1.2.6. Feature: Clinical Privileging Record

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-EM-16`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Provider Administration (p. 155)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:156 (p. 565)	Linked RAD detail; exact locators in manifest.

Feature Overview

The privileging record makes applicable admitting and practice permissions available as contextual evidence. It represents the health service's established privilege standing rather than granting new authority merely because a record exists in the directory.

Definition

Track active clinical admitting and practice privileges within healthcare facilities.

Established Behavioural Context

Element	Relationship to this Feature
Function: Maintain Clinical Privileges (p. 164)	realises feature
Service: Clinical Privilege Verification (p. 165)	associated feature

3.1.2.7. Function: Verify Practitioner Registration

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-verify-practitioner-registration`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Provider Administration (p. 155)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	em:92 (p. 636)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: National Practitioner Verification (p. 156)	Established Feature realisation; not structural containment.
Governed Process	Process: Practitioner Verification Process (p. 165)	Established progression relationship.

Governed Description and Qualifications

Validates national registration, specialty endorsements, and disciplinary sanctions against regulatory registries (e.g., AHPRA).

3.1.2.8. Service: Practitioner Verification Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-practitioner-verification-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Provider Administration (p. 155)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	em:93 (p. 636)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: National Practitioner Verification (p. 156)	Established association only.

Governed Description and Qualifications

Discloses verified registration and status to credentialing and clinical systems.

3.1.2.9. Function: Govern Practitioner Profile

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-govern-practitioner-profile`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Provider Administration (p. 155)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	em:96 (p. 636)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Practitioner Profile Governance (p. 157)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Maintains official names, professional identifiers (e.g., HPI-I, Prescriber Number), and clinical qualifications.

3.1.2.10. Service: Practitioner Directory Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-practitioner-directory-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.

Name	Value	Comment / Description
Owning Context	BEC: Provider Administration (p. 155)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	em:97 (p. 636)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Practitioner Profile Governance (p. 157)	Established association only.

Governed Description and Qualifications

Provides provider directory lookups across the healthcare network.

3.1.2.11. Function: Maintain Practitioner Roles

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-maintain-practitioner-roles`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Provider Administration (p. 155)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	em:99 (p. 636)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Role & Specialty Modeling (p. 158)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Assigns and maintains clinical roles (e.g., Specialist Physician, Registrar, General Practitioner) fulfilled by a practitioner.

3.1.2.12. Service: Practitioner Role Resolution

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-practitioner-role-resolution`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Provider Administration (p. 155)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	em:100 (p. 636)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Role & Specialty Modeling (p. 158)	Established association only.

Governed Description and Qualifications

Resolves the specific clinical capacities of a practitioner.

3.1.2.13. Function: Bind Practitioner Electronic Endpoints

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-bind-practitioner-electronic-endpoints`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Provider Administration (p. 155)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	em:102 (p. 636)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Electronic Endpoint Binding (p. 158)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Associates secure messaging addresses, direct communication endpoints, and notification preferences to practitioner roles.

3.1.2.14. Service: Practitioner Endpoint Lookup

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-practitioner-endpoint-lookup`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Provider Administration (p. 155)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	em:103 (p. 636)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Electronic Endpoint Binding (p. 158)	Established association only.

Governed Description and Qualifications

Provides secure message routing destinations for clinical communication.

3.1.2.15. Function: Maintain Practitioner Affiliations

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-maintain-practitioner-affiliations`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Provider Administration (p. 155)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	em:105 (p. 636)	Linked RAD detail; exact locators in manifest.

Name	Value	Comment / Description
Derived From	Feature: Practitioner-Organisation Affiliation (p. 159)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Tracks employment, admitting rights, and consulting appointments at specific healthcare facilities.

3.1.2.16. Service: Practitioner Affiliation Resolution

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-practitioner-affiliation-resolution`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Provider Administration (p. 155)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	em:106 (p. 636)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Practitioner-Organisation Affiliation (p. 159)	Established association only.

Governed Description and Qualifications

Discloses organizational ties and practice locations.

3.1.2.17. Function: Maintain Clinical Privileges

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-maintain-clinical-privileges`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Provider Administration (p. 155)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	em:108 (p. 636)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Clinical Privileging Record (p. 160)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Records approved clinical scopes of practice, procedural authorizations, and prescribing privileges.

3.1.2.18. Service: Clinical Privilege Verification

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-clinical-privilege-verification`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Provider Administration (p. 155)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	em:109 (p. 636)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Clinical Privileging Record (p. 160)	Established association only.

Governed Description and Qualifications

Verifies procedural and prescribing authority for clinical ordering systems.

3.1.2.19. Process: Practitioner Verification Process

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-PR-practitioner-verification-process`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Provider Administration (p. 155)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	processes:85 (p. 615); processes:86 (p. 615)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Process Overview

Verification progresses from checking the recognised professional source to the locally applicable endorsement and continuing verification status. The distinction matters because a nationally identifiable practitioner is not automatically privileged for every service activity. The illustrated progression retains the health service's endorsement responsibility rather than letting directory ingestion grant clinical authority.

Governed Description and Qualifications

- **Owning Capability**: `ProviderAdministration`
- **Process Purpose**: Governs the lifecycle of validating, certifying, and periodically re-verifying a healthcare practitioner's professional credentials, registration status, and clinical scope of practice.
- **State Progression Lifecycle**: Verification Initiated → Regulatory Registry Queried → Credentials Validated → Privileges Endorsed → Active Verified → Expired / Suspended
- **Key State Dispositions**:
  - `VerificationInitiated`: New practitioner registration or periodic re-credentialing triggered.

- `RegulatoryRegistryQueried`: Real-time query executed against national registry (e.g., AHPRA).
- `CredentialsValidated`: Primary qualifications, specialist endorsements, and sanctions checked.
- `PrivilegesEndorsed`: Health service medical administration endorses admitting/procedural scope.
- `ActiveVerified`: Practitioner verified for clinical ordering and electronic communication.
- `Expired/Suspended`: Verification lapses or regulatory sanction triggers privilege suspension.

3.1.3. BEC: Organisation Administration

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-organisation-administration`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	enabling:86 (p. 564); enabling:158 (p. 566); em:118 (p. 637)	Linked RAD detail; exact locators in manifest.

Capability Overview

Organisation Administration maintains the identity, profiles, structural relationships and contact context of participating healthcare organisations. It enables a legal or operational entity to be distinguished from a department, network relationship or communication channel. This supports consistent interpretation of providers and destinations across information flows rather than allowing each interface to create an incompatible organisational account.

Boundary

Representing organisational structure does not acquire corporate authority or ownership of every service offered by the organisation.

Scope

Contained Features and Established Owned Behaviour

Element	Type	Reference Id
Feature: National Organisation Verification (p. 167)	Feature	`FEAT-EM-17`
Feature: Organisation Profile Management (p. 167)	Feature	`FEAT-EM-18`
Feature: Hierarchical Structure Navigation (p. 168)	Feature	`FEAT-EM-19`
Feature: Directory Contact Management (p. 169)	Feature	`FEAT-EM-20`
Function: Verify Healthcare Organisation (p. 169)	Function	`HCM-FN-verify-healthcare-organisation`
Service: Organisation Verification Service (p. 170)	Service	`HCM-SV-organisation-verification-service`

Element	Type	Reference Id
Function: Govern Organisation Profile (p. 170)	Function	`HCM-FN-govern-organisation-profile`
Service: Organisation Directory Query (p. 171)	Service	`HCM-SV-organisation-directory-query`
Function: Maintain Organisational Structure (p. 171)	Function	`HCM-FN-maintain-organisational-structure`
Service: Organisational Hierarchy Resolution (p. 171)	Service	`HCM-SV-organisational-hierarchy-resolution`
Function: Maintain Organisation Contacts (p. 172)	Function	`HCM-FN-maintain-organisation-contacts`
Service: Organisation Contact Lookup (p. 172)	Service	`HCM-SV-organisation-contact-lookup`

3.1.3.1. Feature: National Organisation Verification

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-EM-17`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Organisation Administration (p. 166)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:163 (p. 566)	Linked RAD detail; exact locators in manifest.

Feature Overview

National organisation verification grounds an organisation identifier and entity classification in their recognised context. This supports reliable interpretation of the healthcare participant without assuming that its identity establishes every service, location or authority associated with it.

Definition

Validate national organization identifiers (e.g., HPI-O) and legal entity classifications.

Established Behavioural Context

Element	Relationship to this Feature
Function: Verify Healthcare Organisation (p. 169)	realises feature
Service: Organisation Verification Service (p. 170)	associated feature

3.1.3.2. Feature: Organisation Profile Management

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-EM-18`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.

Name	Value	Comment / Description
Owning Context	BEC: Organisation Administration (p. 166)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:164 (p. 566)	Linked RAD detail; exact locators in manifest.

Feature Overview

Organisation profiles provide a governed account of the participant's status and published service descriptions. They support consistent directory use across systems while leaving the organisation's real-world governance and the definition of individual offerings distinguishable.

Definition

Govern directory profiles, operational status, and published service descriptions.

Established Behavioural Context

Element	Relationship to this Feature
Function: Govern Organisation Profile (p. 170)	realises feature
Service: Organisation Directory Query (p. 171)	associated feature

3.1.3.3. Feature: Hierarchical Structure Navigation

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-EM-19`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Organisation Administration (p. 166)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:168 (p. 566)	Linked RAD detail; exact locators in manifest.

Feature Overview

Hierarchy navigation makes departmental, divisional and network relationships usable without flattening them into unrelated organisations. Traversal reflects established structure; it does not create corporate authority or infer responsibility from the way a directory happens to display a tree.

Definition

Model, traverse, and resolve nested departmental, divisional, and regional healthcare networks.

Established Behavioural Context

Element	Relationship to this Feature
Function: Maintain Organisational Structure (p. 171)	realises feature
Service: Organisational Hierarchy Resolution (p. 171)	associated feature

3.1.3.4. Feature: Directory Contact Management

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-EM-20`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Organisation Administration (p. 166)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:172 (p. 566)	Linked RAD detail; exact locators in manifest.

Feature Overview

Contact management provides verified administrative and clinical communication channels for organisations. It supports reaching the appropriate participant while distinguishing a directory contact from the role-specific destination and authority relevant to a particular clinical exchange.

Definition

Manage verified phone, email, and administrative contact channels for healthcare facilities.

Established Behavioural Context

Element	Relationship to this Feature
Function: Maintain Organisation Contacts (p. 172)	realises feature
Service: Organisation Contact Lookup (p. 172)	associated feature

3.1.3.5. Function: Verify Healthcare Organisation

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-verify-healthcare-organisation`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Organisation Administration (p. 166)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.

Name	Value	Comment / Description
Source	em:121 (p. 637)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: National Organisation Verification (p. 167)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Validates enterprise legal identity, national facility identifiers (e.g., HPI-O), and accreditation status.

3.1.3.6. Service: Organisation Verification Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-organisation-verification-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Organisation Administration (p. 166)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	em:122 (p. 637)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: National Organisation Verification (p. 167)	Established association only.

Governed Description and Qualifications

Provides verified organisation credentials to external and internal partners.

3.1.3.7. Function: Govern Organisation Profile

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-govern-organisation-profile`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Organisation Administration (p. 166)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	em:124 (p. 637)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Organisation Profile Management (p. 167)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Maintains official trading names, enterprise registration numbers, and governing bodies.

3.1.3.8. Service: Organisation Directory Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-organisation-directory-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Organisation Administration (p. 166)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	em:125 (p. 637)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Organisation Profile Management (p. 167)	Established association only.

Governed Description and Qualifications

Discloses healthcare organisation profiles across the region.

3.1.3.9. Function: Maintain Organisational Structure

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-maintain-organisational-structure`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Organisation Administration (p. 166)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	em:127 (p. 637)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Hierarchical Structure Navigation (p. 168)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Models parent-subsidiary relationships, clinical directorates, departments, and operational divisions.

3.1.3.10. Service: Organisational Hierarchy Resolution

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-organisational-hierarchy-resolution`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Organisation Administration (p. 166)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	em:128 (p. 637)	Linked RAD detail; exact locators in manifest.

Name	Value	Comment / Description
Associated Feature	Feature: Hierarchical Structure Navigation (p. 168)	Established association only.

Governed Description and Qualifications

Resolves department nesting and reporting hierarchies.

3.1.3.11. Function: Maintain Organisation Contacts

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-maintain-organisation-contacts`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Organisation Administration (p. 166)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	em:130 (p. 637)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Directory Contact Management (p. 169)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Manages administrative, clinical, and emergency contact details for facilities.

3.1.3.12. Service: Organisation Contact Lookup

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-organisation-contact-lookup`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Organisation Administration (p. 166)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	em:131 (p. 637)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Directory Contact Management (p. 169)	Established association only.

Governed Description and Qualifications

Provides official operational contact details.

3.1.4. BEC: Location Administration

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-location-administration`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	enabling:90 (p. 564); enabling:174 (p. 566); em:140 (p. 638)	Linked RAD detail; exact locators in manifest.

Capability Overview

Location Administration represents the places and physical relationships relevant to healthcare activity, from facilities to particular care places. Address and geospatial context answer where a place is; hierarchy explains its surroundings; care-place definition records the local characteristics relevant to care. These are stable descriptive responsibilities, distinct from the changing operational state of a bed or ward.

Boundary

Defining a place does not establish occupancy, readiness, facilities maintenance or service capacity.

Scope

Contained Features and Established Owned Behaviour

Element	Type	Reference Id
Feature: Location Geospatial & Address Resolution (p. 174)	Feature	`FEAT-EM-21`
Feature: Physical Hierarchy Modeling (p. 174)	Feature	`FEAT-EM-22`
Feature: Atomic Care-Place Modeling (p. 175)	Feature	`FEAT-EM-23`
Function: Maintain Healthcare Location (p. 176)	Function	`HCM-FN-maintain-healthcare-location`
Service: Location Directory Query (p. 176)	Service	`HCM-SV-location-directory-query`
Function: Maintain Physical Location Hierarchy (p. 176)	Function	`HCM-FN-maintain-physical-location-hierarchy`
Service: Location Hierarchy Resolution (p. 177)	Service	`HCM-SV-location-hierarchy-resolution`
Function: Maintain Care-Place Definition (p. 177)	Function	`HCM-FN-maintain-care-place-definition`
Service: Care-Place Specification Lookup (p. 178)	Service	`HCM-SV-care-place-specification-lookup`

3.1.4.1. Feature: Location Geospatial & Address Resolution

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-EM-21`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Location Administration (p. 173)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:179 (p. 566)	Linked RAD detail; exact locators in manifest.

Feature Overview

Address and geospatial resolution gives a recognised healthcare location an intelligible physical reference. It supports place interpretation without treating a valid address or recorded operating status as proof that a particular service or care place is currently ready.

Definition

Validate physical addresses, geospatial coordinates, and location operational status.

Established Behavioural Context

Element	Relationship to this Feature
Function: Maintain Healthcare Location (p. 176)	realises feature
Service: Location Directory Query (p. 176)	associated feature

3.1.4.2. Feature: Physical Hierarchy Modeling

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-EM-22`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Location Administration (p. 173)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:183 (p. 566)	Linked RAD detail; exact locators in manifest.

Feature Overview

Physical hierarchy modelling relates a care environment to its campus, building, floor or ward context. The relationships describe where places sit physically; they do not automatically reproduce organisational hierarchy, service responsibility or operational capacity.

Definition

Represent campus, building, wing, floor, and ward architectural relationships.

Established Behavioural Context

Element	Relationship to this Feature
Function: Maintain Physical Location Hierarchy (p. 176)	realises feature
Service: Location Hierarchy Resolution (p. 177)	associated feature

3.1.4.3. Feature: Atomic Care-Place Modeling

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-EM-23`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Location Administration (p. 173)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:187 (p. 566)	Linked RAD detail; exact locators in manifest.

Feature Overview

Care-place modelling identifies the particular physical place and characteristics relevant to care, such as isolation or telemetry capability. It supplies the descriptive frame against which changing occupancy and readiness can be considered without making those states part of the place's identity.

Definition

Define specific beds, care-bays, and examination rooms with their physical capabilities (e.g., telemetry-capable, negative-pressure).

Established Behavioural Context

Element	Relationship to this Feature
Function: Maintain Care-Place Definition (p. 177)	realises feature
Service: Care-Place Specification Lookup (p. 178)	associated feature

3.1.4.4. Function: Maintain Healthcare Location

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-maintain-healthcare-location`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Location Administration (p. 173)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	em:145 (p. 638)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Location Geospatial & Address Resolution (p. 174)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Records physical sites, campuses, outpatient clinics, and community health centres.

3.1.4.5. Service: Location Directory Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-location-directory-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Location Administration (p. 173)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	em:146 (p. 638)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Location Geospatial & Address Resolution (p. 174)	Established association only.

Governed Description and Qualifications

Exposes location profiles and geospatial addresses.

3.1.4.6. Function: Maintain Physical Location Hierarchy

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-maintain-physical-location-hierarchy`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Location Administration (p. 173)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	em:148 (p. 638)	Linked RAD detail; exact locators in manifest.

Name	Value	Comment / Description
Derived From	Feature: Physical Hierarchy Modeling (p. 174)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Models the nested structural containment tree: `Campus` → `Building` → `Floor` → `Ward/Suite` → `Room/Bay` → `BedCare-Place`.

3.1.4.7. Service: Location Hierarchy Resolution

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-location-hierarchy-resolution`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Location Administration (p. 173)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	em:149 (p. 638)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Physical Hierarchy Modeling (p. 174)	Established association only.

Governed Description and Qualifications

Discloses physical containment and navigation paths for clinical facilities.

3.1.4.8. Function: Maintain Care-Place Definition

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-maintain-care-place-definition`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Location Administration (p. 173)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	em:151 (p. 638)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Atomic Care-Place Modeling (p. 175)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Defines static bed bays, treatment chairs, surgical tables, and emergency resuscitation cubicles, including equipment capabilities and oxygen/telemetry infrastructure.

3.1.4.9. Service: Care-Place Specification Lookup

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-care-place-specification-lookup`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Location Administration (p. 173)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	em:152 (p. 638)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Atomic Care-Place Modeling (p. 175)	Established association only.

Governed Description and Qualifications

Discloses care-place physical capabilities to Bed & Care-Place Management.

3.1.5. BEC: Health Service Administration

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-health-service-administration`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	enabling:94 (p. 564); enabling:189 (p. 567); em:161 (p. 638)	Linked RAD detail; exact locators in manifest.

Capability Overview

Health Service Administration represents identifiable services and the conditions under which organisations, locations and practitioner roles provide them. Catalogue publishing, service-location-provider association, operating schedules and eligibility definitions allow a service to be understood as more than a location or specialty label. The service context remains material wherever activity meaning or accountability depends on it.

Boundary

This capability owns the service mapping associations, not the mapped organisations, locations or practitioners. The supplied Domain03-to-Domain04 trace preserves that distinction; it does not transfer entity ownership.

Scope

Contained Features and Established Owned Behaviour

Element	Type	Reference Id
Feature: Service Catalogue Publishing (p. 179)	Feature	FEAT-EM-24
Feature: Service-Location-Provider Binding (p. 180)	Feature	FEAT-EM-25
Feature: Operating Schedule Specification (p. 180)	Feature	FEAT-EM-26
Feature: Eligibility & Catchment Specification (p. 181)	Feature	FEAT-EM-27
Function: Maintain Healthcare Service Definition (p. 182)	Function	HCM-FN-maintain-healthcare-service-definition
Service: Healthcare Service Directory Query (p. 182)	Service	HCM-SV-healthcare-service-directory-query
Function: Maintain Service / Location / Provider Map (p. 182)	Function	HCM-FN-maintain-service-location-provider-map
Service: Service Provision Resolution (p. 183)	Service	HCM-SV-service-provision-resolution
Function: Maintain Service Availability (p. 183)	Function	HCM-FN-maintain-service-availability
Service: Service Availability Query (p. 184)	Service	HCM-SV-service-availability-query
Function: Maintain Service Eligibility Rules (p. 184)	Function	HCM-FN-maintain-service-eligibility-rules
Service: Service Eligibility Evaluation (p. 184)	Service	HCM-SV-service-eligibility-evaluation

3.1.5.1. Feature: Service Catalogue Publishing

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-EM-24	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Health Service Administration (p. 178)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:194 (p. 567)	Linked RAD detail; exact locators in manifest.

Feature Overview

Service catalogue publishing makes identifiable service definitions and intake context available to regional participants. A published service can be discovered as an offering rather than inferred from an organisation's name; publication alone does not establish present availability or collective readiness.

Definition

Publish structured healthcare service definitions, clinical specialties, and intake criteria to regional directories.

Established Behavioural Context

Element	Relationship to this Feature
Function: Maintain Healthcare Service Definition (p. 182)	realises feature
Service: Healthcare Service Directory Query (p. 182)	associated feature

3.1.5.2. Feature: Service-Location-Provider Binding

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-EM-25	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Health Service Administration (p. 178)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:198 (p. 567)	Linked RAD detail; exact locators in manifest.

Feature Overview

Service-location-provider binding explains which participant provides an identifiable service in which place and acting capacities. The mapping associations belong to Health Service Administration, while the organisations, locations and practitioner entities retain their own information responsibility.

Definition

Correlate which organization provides a named health service at which physical location with which practitioner roles.

Established Behavioural Context

Element	Relationship to this Feature
Function: Maintain Service / Location / Provider Map (p. 182)	realises feature
Service: Service Provision Resolution (p. 183)	associated feature

3.1.5.3. Feature: Operating Schedule Specification

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-EM-26	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Health Service Administration (p. 178)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.

Name	Value	Comment / Description
Source	enabling:202 (p. 567)	Linked RAD detail; exact locators in manifest.

Feature Overview

Operating schedules make temporal availability and exceptions explicit for a service. They distinguish an existing offering from one available at the relevant time, while leaving actual resource readiness and operational capacity as separate concerns.

Definition

Define temporal availability, clinic operating hours, and on-call service windows.

Established Behavioural Context

Element	Relationship to this Feature
Function: Maintain Service Availability (p. 183)	realises feature
Service: Service Availability Query (p. 184)	associated feature

3.1.5.4. Feature: Eligibility & Catchment Specification

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-EM-27`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Health Service Administration (p. 178)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:206 (p. 567)	Linked RAD detail; exact locators in manifest.

Feature Overview

Eligibility specification captures the clinical thresholds and geographical bounds applicable to service access. It provides the criteria context needed to consider a request without making publication of those criteria an individual eligibility decision or a completed allocation.

Definition

Define clinical referral thresholds and geographic catchment boundaries.

Established Behavioural Context

Element	Relationship to this Feature
Function: Maintain Service Eligibility Rules (p. 184)	realises feature

Element	Relationship to this Feature
Service: Service Eligibility Evaluation (p. 184)	associated feature

3.1.5.5. Function: Maintain Healthcare Service Definition

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-maintain-healthcare-service-definition`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Health Service Administration (p. 178)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	em:174 (p. 638)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Service Catalogue Publishing (p. 179)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Defines clinical service offerings (e.g., Cardiology Outpatient Clinic, MRI Imaging, Acute Stroke Care), specialty classifications, and clinical scope.

3.1.5.6. Service: Healthcare Service Directory Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-healthcare-service-directory-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Health Service Administration (p. 178)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	em:175 (p. 638)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Service Catalogue Publishing (p. 179)	Established association only.

Governed Description and Qualifications

Exposes published clinical service offerings.

3.1.5.7. Function: Maintain Service / Location / Provider Map

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-maintain-service-location-provider-map`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.

Name	Value	Comment / Description
Owning Context	BEC: Health Service Administration (p. 178)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	em:177 (p. 638)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Service-Location-Provider Binding (p. 180)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Binds health service definitions to delivering organisations, physical clinic locations, and participating practitioner roles.

3.1.5.8. Service: Service Provision Resolution

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-service-provision-resolution`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Health Service Administration (p. 178)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	em:178 (p. 638)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Service-Location-Provider Binding (p. 180)	Established association only.

Governed Description and Qualifications

Exposes multi-dimensional resolution queries answering who, where, and when healthcare services are delivered.

3.1.5.9. Function: Maintain Service Availability

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-maintain-service-availability`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Health Service Administration (p. 178)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	em:180 (p. 638)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Operating Schedule Specification (p. 180)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Governs regular operating hours, emergency intake availability, and seasonal clinic schedules.

3.1.5.10. Service: Service Availability Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-service-availability-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Health Service Administration (p. 178)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	em:181 (p. 638)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Operating Schedule Specification (p. 180)	Established association only.

Governed Description and Qualifications

Discloses operational intake hours and open status.

3.1.5.11. Function: Maintain Service Eligibility Rules

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-maintain-service-eligibility-rules`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Health Service Administration (p. 178)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	em:183 (p. 638)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Eligibility & Catchment Specification (p. 181)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Configures catchment criteria, age thresholds, referral requirements, and clinical indications for service access.

3.1.5.12. Service: Service Eligibility Evaluation

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-service-eligibility-evaluation`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.

Name	Value	Comment / Description
Owning Context	BEC: Health Service Administration (p. 178)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	em:184 (p. 638)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Eligibility & Catchment Specification (p. 181)	Established association only.

Governed Description and Qualifications

Evaluates referral eligibility for intake coordinators.

3.1.6. BEC: Health Product Administration

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-health-product-administration`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	enabling:99 (p. 564); enabling:208 (p. 567)	Linked RAD detail; exact locators in manifest.

Capability Overview

Health Product Administration retains the reference context for medications, implants, blood products and consumables. Product definitions, formularies and batch or serial information help explain what is being referenced without assigning a supply-chain implementation to Harmonia. No Feature decomposition is established for this responsibility in the present baseline.

Boundary

Product representation does not perform procurement, stock control, dispensing or clinical administration. The contextual child descriptions do not establish additional canonical capabilities.

3.1.7. BEC: Clinical Device Administration

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-clinical-device-administration`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.

Name	Value	Comment / Description
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	enabling:103 (p. 564); enabling:215 (p. 567); em:193 (p. 639)	Linked RAD detail; exact locators in manifest.

Capability Overview

Clinical Device Administration represents device types, identifiable instances, patient or location associations and communication endpoints. This lets device-originated information be interpreted in relation to the actual device and care context, rather than merely a network address. Type, instance, assignment and connectivity each answer a different question about the source of device data.

Boundary

Device registration is not maintenance, release management or clinical interpretation. An endpoint credential does not by itself authorise clinical use of the device.

Scope

Contained Features and Established Owned Behaviour

Element	Type	Reference Id
Feature: Device Type Specification (p. 186)	Feature	`FEAT-EM-28`
Feature: Device Serial Tracking (p. 187)	Feature	`FEAT-EM-29`
Feature: Device-Patient/Location Assignment (p. 188)	Feature	`FEAT-EM-30`
Feature: Device Endpoint Registration (p. 188)	Feature	`FEAT-EM-31`
Function: Maintain Device Definition (p. 189)	Function	`HCM-FN-maintain-device-definition`
Service: Device Definition Query (p. 189)	Service	`HCM-SV-device-definition-query`
Function: Maintain Device Instance Identity (p. 190)	Function	`HCM-FN-maintain-device-instance-identity`
Service: Device Instance Resolution (p. 190)	Service	`HCM-SV-device-instance-resolution`
Function: Maintain Device Association (p. 190)	Function	`HCM-FN-maintain-device-association`
Service: Device Association Query (p. 191)	Service	`HCM-SV-device-association-query`
Function: Maintain Device Communication Endpoints (p. 191)	Function	`HCM-FN-maintain-device-communication-endpoints`
Service: Device Endpoint Resolution (p. 192)	Service	`HCM-SV-device-endpoint-resolution`

3.1.7.1. Feature: Device Type Specification

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-EM-28`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Clinical Device Administration (p. 185)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.

Name	Value	Comment / Description
Source	enabling:220 (p. 567)	Linked RAD detail; exact locators in manifest.

Feature Overview

Device type specification provides model-level capabilities and regulatory context against which individual devices can be understood. It describes the kind of equipment without substituting for the release, condition or deployment of a particular instance.

Definition

Catalog medical device models, regulatory approvals, and capabilities.

Established Behavioural Context

Element	Relationship to this Feature
Function: Maintain Device Definition (p. 189)	realises feature
Service: Device Definition Query (p. 189)	associated feature

3.1.7.2. Feature: Device Serial Tracking

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-EM-29	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Clinical Device Administration (p. 185)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:224 (p. 567)	Linked RAD detail; exact locators in manifest.

Feature Overview

Serial tracking makes individual hardware distinguishable within the device estate. A unique device reference supports subsequent assignment and source interpretation, while remaining separate from evidence that the device is maintained, connected or fit for a particular clinical use.

Definition

Ingest and maintain unique device identifier (UDI) records and serial numbers.

Established Behavioural Context

Element	Relationship to this Feature
Function: Maintain Device Instance Identity (p. 190)	realises feature
Service: Device Instance Resolution (p. 190)	associated feature

3.1.7.3. Feature: Device-Patient/Location Assignment

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-EM-30	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Clinical Device Administration (p. 185)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:228 (p. 567)	Linked RAD detail; exact locators in manifest.

Feature Overview

Dynamic device association preserves which patient or physical place a device is linked to as assignments change. This is essential context for interpreting telemetry: a stable device identity does not imply that every observation concerns the same patient over time.

Definition

Track dynamic association between a telemetry monitor, patient, and physical bed.

Established Behavioural Context

Element	Relationship to this Feature
Function: Maintain Device Association (p. 190)	realises feature
Service: Device Association Query (p. 191)	associated feature

3.1.7.4. Feature: Device Endpoint Registration

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-EM-31	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Clinical Device Administration (p. 185)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:232 (p. 567)	Linked RAD detail; exact locators in manifest.

Feature Overview

Endpoint registration supplies the interfaces and credential context through which a diagnostic or telemetry feed can participate. It describes provisioned connectivity rather than proving that data has arrived, been clinically interpreted or acquired authoritative standing.

Definition

Register communication interfaces and security credentials for diagnostic telemetry feeds.

Established Behavioural Context

Element	Relationship to this Feature
Function: Maintain Device Communication Endpoints (p. 191)	realises feature
Service: Device Endpoint Resolution (p. 192)	associated feature

3.1.7.5. Function: Maintain Device Definition

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-maintain-device-definition`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Clinical Device Administration (p. 185)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	em:196 (p. 639)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Device Type Specification (p. 186)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Models medical device models, manufacturers, regulatory classifications (e.g., TGA/FDA classes), and calibration specifications.

3.1.7.6. Service: Device Definition Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-device-definition-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Clinical Device Administration (p. 185)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	em:197 (p. 639)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Device Type Specification (p. 186)	Established association only.

Governed Description and Qualifications

Discloses approved medical device models and technical parameters.

3.1.7.7. Function: Maintain Device Instance Identity

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-maintain-device-instance-identity`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Clinical Device Administration (p. 185)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	em:199 (p. 639)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Device Serial Tracking (p. 187)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Tracks physical serial numbers, Unique Device Identifiers (UDI), asset tags, and firmware versions.

3.1.7.8. Service: Device Instance Resolution

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-device-instance-resolution`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Clinical Device Administration (p. 185)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	em:200 (p. 639)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Device Serial Tracking (p. 187)	Established association only.

Governed Description and Qualifications

Resolves physical device instances from scanned barcodes or UDIs.

3.1.7.9. Function: Maintain Device Association

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-maintain-device-association`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Clinical Device Administration (p. 185)	Established responsibility only; complete CT ancestry Unresolved.

Name	Value	Comment / Description
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	em:202 (p. 639)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Device-Patient/Location Assignment (p. 188)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Records active associations between clinical devices, patient encounters, and specific care-places (e.g., Bed 4 Vital Signs Monitor).

3.1.7.10. Service: Device Association Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-device-association-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Clinical Device Administration (p. 185)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	em:203 (p. 639)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Device-Patient/Location Assignment (p. 188)	Established association only.

Governed Description and Qualifications

Discloses active device-to-patient and device-to-location bindings.

3.1.7.11. Function: Maintain Device Communication Endpoints

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-maintain-device-communication-endpoints`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Clinical Device Administration (p. 185)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	em:205 (p. 639)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Device Endpoint Registration (p. 188)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Manages network telemetry addresses, gateway ports, and protocol bindings for medical devices.

3.1.7.12. Service: Device Endpoint Resolution

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-device-endpoint-resolution`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Clinical Device Administration (p. 185)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	em:206 (p. 639)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Device Endpoint Registration (p. 188)	Established association only.

Governed Description and Qualifications

Exposes network addresses for telemetry routing.

3.1.8. BEC: Person Identity

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-person-identity`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	BEC: Client Administration (p. 154)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Harmonia-Core	Independent of status; never inherited.
Source	enabling:115 (p. 565); em:16 (p. 634)	Linked RAD detail; exact locators in manifest.

Capability Overview

Person Identity resolves and correlates the identities and identifiers assigned to one individual by different authorities. It enables an incoming local identifier to be understood in its issuing namespace and related to other known identifiers without destructively replacing their history. Resolution, cross-authority correlation, alias association and governed correction preserve different aspects of that responsibility.

Definition

Resolving, maintaining, and correlating identities and identifiers assigned to an individual across different authorities.

Intent

Support reliable association of care information with the intended individual across participating systems.

Boundary

Harmonia relies on authoritative upstream PAS/MPI information for R1/R2. Correlation and correction notifications do not establish an autonomous master-patient reconciliation engine.

Harmonia Architectural Relevance

Identity association is a direct platform concern because clinical records, orders and operational activity need an enduring subject context. Its Core standing does not imply originating master identity authority.

Scope

Contained Features and Established Owned Behaviour

Element	Type	Reference Id
Feature: Identifier Resolution (p. 193)	Feature	FEAT-EM-01
Feature: Cross-Authority Identifier Correlation (p. 194)	Feature	FEAT-EM-02
Feature: Identity Alias Association (p. 195)	Feature	FEAT-EM-03
Feature: Governed Identity Correction (p. 195)	Feature	FEAT-EM-04
Function: Resolve Person Identifier (p. 196)	Function	HCM-FN-resolve-person-identifier
Service: Person Identifier Resolution (p. 196)	Service	HCM-SV-person-identifier-resolution
Function: Correlate Person Identifiers (p. 197)	Function	HCM-FN-correlate-person-identifiers
Service: Person Identifier Correlation (p. 197)	Service	HCM-SV-person-identifier-correlation
Function: Maintain Person Identity Aliases (p. 197)	Function	HCM-FN-maintain-person-identity-aliases
Function: Apply Governed Person Identity Correction (p. 198)	Function	HCM-FN-apply-governed-person-identity-correction
Service: Person Identity Correction (p. 198)	Service	HCM-SV-person-identity-correction
Process: Governed Person Identity Correction Process (p. 199)	Process	HCM-PR-governed-person-identity-correction-process

3.1.8.1. Feature: Identifier Resolution

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-EM-01	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Client Administration (p. 154) → BEC: Person Identity (p. 192)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:117 (p. 565)	Linked RAD detail; exact locators in manifest.

Feature Overview

Identifier Resolution interprets an identifier in its issuing-authority and namespace context, so that the same local value is not assumed to identify the same person everywhere. It provides an identifiable subject reference without deciding an authoritative master-person match.

Definition

Ingest and resolve external identifiers against registered issuing authorities and namespace systems.

Established Behavioural Context

Element	Relationship to this Feature
Function: Resolve Person Identifier (p. 196)	realises feature
Service: Person Identifier Resolution (p. 196)	associated feature

3.1.8.2. Feature: Cross-Authority Identifier Correlation

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-EM-02`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Client Administration (p. 154) → BEC: Person Identity (p. 192)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:118 (p. 565)	Linked RAD detail; exact locators in manifest.

Feature Overview

Cross-authority correlation preserves the association between identifiers used for one person by different participants. It enables information to be related across regional systems without erasing the separate identifier namespaces or claiming that Harmonia originated the identity decision.

Definition

Correlate disparate local identifiers (e.g., hospital MRN, clinic client ID, national IHI) belonging to the same individual.

Established Behavioural Context

Element	Relationship to this Feature
Function: Correlate Person Identifiers (p. 197)	realises feature
Service: Person Identifier Correlation (p. 197)	associated feature

3.1.8.3. Feature: Identity Alias Association

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-EM-03`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Client Administration (p. 154) → BEC: Person Identity (p. 192)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:119 (p. 565)	Linked RAD detail; exact locators in manifest.

Feature Overview

Alias association keeps historical and demographic identity references navigable alongside active identifiers. The non-destructive relationship preserves how earlier information identified the person rather than rewriting that history whenever the current identity representation changes.

Definition

Maintain explicit associations between active identifiers and historical or demographic aliases without destructive overwriting.

Established Behavioural Context

Element	Relationship to this Feature
Function: Maintain Person Identity Aliases (p. 197)	realises feature

3.1.8.4. Feature: Governed Identity Correction

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-EM-04`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Client Administration (p. 154) → BEC: Person Identity (p. 192)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:120 (p. 565)	Linked RAD detail; exact locators in manifest.

Feature Overview

Governed correction applies externally authoritative identity updates and their consequences to Harmonia-managed associations. Administrative review concerns the legitimate application of the source outcome; it does not grant Harmonia an EMPI merge engine or authority to select a golden person record.

Definition

Apply administrative identity updates, linked-record corrections, and split/merge notifications propagated from authoritative sources.

Established Behavioural Context

Element	Relationship to this Feature
Function: Apply Governed Person Identity Correction (p. 198)	realises feature
Service: Person Identity Correction (p. 198)	associated feature

3.1.8.5. Function: Resolve Person Identifier

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-resolve-person-identifier`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Client Administration (p. 154) → BEC: Person Identity (p. 192)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	em:24 (p. 634)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Identifier Resolution (p. 193)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Validates and retrieves the authoritative demographic record bound to a specific identifier.

3.1.8.6. Service: Person Identifier Resolution

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-person-identifier-resolution`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Client Administration (p. 154) → BEC: Person Identity (p. 192)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	em:25 (p. 634)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Identifier Resolution (p. 193)	Established association only.

Governed Description and Qualifications

Exposed to Service Administration and Service Delivery to disambiguate client identities.

3.1.8.7. Function: Correlate Person Identifiers

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-correlate-person-identifiers`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Client Administration (p. 154) → BEC: Person Identity (p. 192)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	em:27 (p. 634)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Cross-Authority Identifier Correlation (p. 194)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Correlates local identifiers (e.g., Hospital MRN, National IHI, Pathology ID) using relationships established by authoritative information, retaining their source and namespace.

3.1.8.8. Service: Person Identifier Correlation

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-person-identifier-correlation`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Client Administration (p. 154) → BEC: Person Identity (p. 192)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	em:28 (p. 634)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Cross-Authority Identifier Correlation (p. 194)	Established association only.

Governed Description and Qualifications

Exposed across the HIE to establish patient cross-reference mappings.

3.1.8.9. Function: Maintain Person Identity Aliases

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-maintain-person-identity-aliases`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Client Administration (p. 154) → BEC: Person Identity (p. 192)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	em:30 (p. 634)	Linked RAD detail; exact locators in manifest.

Name	Value	Comment / Description
Derived From	Feature: Identity Alias Association (p. 195)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Records and qualifies alias names, titles, and emergency aliases for a governed person identity.

3.1.8.10. Function: Apply Governed Person Identity Correction

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-apply-governed-person-identity-correction`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Client Administration (p. 154) → BEC: Person Identity (p. 192)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	em:33 (p. 634)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Governed Identity Correction (p. 195)	Established Feature realisation; not structural containment.
Governed Process	Process: Governed Person Identity Correction Process (p. 199)	Established progression relationship.

Governed Description and Qualifications

Processes externally authoritative corrections and merge/unlink outcomes, updates governed identity associations and propagates resulting managed-state consequences with audit history. Harmonia does not adjudicate the originating person-merge decision.

3.1.8.11. Service: Person Identity Correction

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-person-identity-correction`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Client Administration (p. 154) → BEC: Person Identity (p. 192)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	em:34 (p. 634)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Governed Identity Correction (p. 195)	Established association only.

Governed Description and Qualifications

Exposed to authorized HIM/Medical Records administrators.

3.1.8.12. Process: Governed Person Identity Correction Process

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-PR-governed-person-identity-correction-process`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Client Administration (p. 154) → BEC: Person Identity (p. 192)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	processes:69 (p. 614); processes:70 (p. 614)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Process Overview

This Process governs the application of an externally authoritative correction rather than originating a person-merge judgement. Triage, evidence verification and approval establish that the received outcome should be applied to Harmonia's associations. The progression and change communication preserve why the managed identity account changed without making internal approval a master-person decision.

Governed Description and Qualifications

- **Owning Capability**: `PersonIdentity` (under `ClientAdministration`)
- **Process Purpose**: Processes externally authoritative identity corrections and merge/unlink outcomes, applies their consequences to Harmonia-managed identity information and identifier associations, and communicates resulting changes with an audit history.
- **Authority boundary**: Person correction/merge decisions originate externally. Internal triage, evidence verification and approval concern applicability and governed processing of that authoritative outcome, not person matching, golden-record determination, master-person selection or originating merge adjudication.
- **State Progression Lifecycle**: Correction Requested → HIM Triage → Evidence Verified → Correction Approved → Merge/Unlink Executed → Change Broadcasted → Closed
- **Key State Dispositions**:
  - `CorrectionRequested`: Externally authoritative correction or merge/unlink outcome received for governed application.
  - `HIMTriage`: Health Information Manager reviews the received decision/outcome and its applicability.
  - `EvidenceVerified`: Authoritative source decision and supporting provenance/documentation verified.
  - `CorrectionApproved`: Governed application of the externally authoritative outcome approved; this does not originate a person-merge decision.
  - `Merge/UnlinkExecuted`: External merge/unlink consequences applied to Harmonia's managed identifier associations and audit history.
  - `ChangeBroadcasted`: Downstream clinical systems notified of merged or rectified identity.

3.1.9. BEC: Healthcare Subject

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-healthcare-subject`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Harmonia-Core	Independent of status; never inherited.
Source	enabling:122 (p. 565); em:41 (p. 635)	Linked RAD detail; exact locators in manifest.

Capability Overview

Healthcare Subject maintains the subject-of-care context attached to clinical records and operational activity. It connects a resolved person identity with the encounters, orders and observations concerning that person, while preserving the provenance and temporal standing of demographic attributes. Its responsibility differs from resolving identifiers: it establishes how the person is represented as the continuing subject of care.

Definition

Managing the core healthcare subject context bound to clinical records and operational activities.

Boundary

Subject association does not originate clinical findings or confer authority to change the source person’s identity.

Harmonia Architectural Relevance

Verifiable subject binding prevents otherwise valid information from being interpreted in the wrong person’s care context, making it a substantive information-management responsibility.

Scope

Contained Features and Established Owned Behaviour

Element	Type	Reference Id
Feature: Subject Context Binding (p. 201)	Feature	`FEAT-EM-05`
Feature: Demographic Context Governance (p. 201)	Feature	`FEAT-EM-06`
Function: Establish Healthcare Subject Context (p. 202)	Function	`HCM-FN-establish-healthcare-subject-context`
Service: Healthcare Subject Context Resolution (p. 202)	Service	`HCM-SV-healthcare-subject-context-resolution`
Function: Govern Subject Demographic Context (p. 203)	Function	`HCM-FN-govern-subject-demographic-context`

Element	Type	Reference Id
Service: Demographic Change Distribution (p. 203)	Service	`HCM-SV-demographic-change-distribution`

3.1.9.1. Feature: Subject Context Binding

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-EM-05`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Healthcare Subject (p. 200)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:124 (p. 565)	Linked RAD detail; exact locators in manifest.

Feature Overview

Subject binding connects resolved identity with the records and activities concerning that person. It makes the association verifiable across encounters, orders and observations rather than assuming that proximity in a message or screen establishes the enduring subject of care.

Definition

Bind authenticated person identity to clinical encounters, orders, and observations as the enduring subject of care with verifiable association.

Established Behavioural Context

Element	Relationship to this Feature
Function: Establish Healthcare Subject Context (p. 202)	realises feature
Service: Healthcare Subject Context Resolution (p. 202)	associated feature

3.1.9.2. Feature: Demographic Context Governance

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-EM-06`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Healthcare Subject (p. 200)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:125 (p. 565)	Linked RAD detail; exact locators in manifest.

Feature Overview

Demographic governance retains the provenance and temporal context of attributes used to interpret the subject of care. A changed date, status or demographic assertion can therefore be understood as sourced information rather than an unexplained overwrite of previously known context.

Definition

Manage essential demographic attributes (date of birth, sex, vital status) with full provenance and timestamping.

Established Behavioural Context

Element	Relationship to this Feature
Function: Govern Subject Demographic Context (p. 203)	realises feature
Service: Demographic Change Distribution (p. 203)	associated feature

3.1.9.3. Function: Establish Healthcare Subject Context

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-establish-healthcare-subject-context`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Healthcare Subject (p. 200)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	em:45 (p. 635)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Subject Context Binding (p. 201)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Binds demographic, cultural, and communication requirements to a patient record.

3.1.9.4. Service: Healthcare Subject Context Resolution

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-healthcare-subject-context-resolution`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Healthcare Subject (p. 200)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	em:46 (p. 635)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Subject Context Binding (p. 201)	Established association only.

Governed Description and Qualifications

Provides comprehensive demographic, language, and indigenous status context to clinical applications.

3.1.9.5. Function: Govern Subject Demographic Context

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-govern-subject-demographic-context`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Healthcare Subject (p. 200)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	em:48 (p. 635)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Demographic Context Governance (p. 201)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Applies validation rules to address changes, date of birth verification, and death notifications.

3.1.9.6. Service: Demographic Change Distribution

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-demographic-change-distribution`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Healthcare Subject (p. 200)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	em:49 (p. 635)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Demographic Context Governance (p. 201)	Established association only.

Governed Description and Qualifications

Emits governed subject demographic updates to subscribed clinical systems.

3.1.10. BEC: Client Relationship

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-client-relationship`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.

Name	Value	Comment / Description
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Harmonia-Relevant	Independent of status; never inherited.
Source	enabling:127 (p. 565); em:55 (p. 635)	Linked RAD detail; exact locators in manifest.

Capability Overview

Client Relationship represents nominated carers, guardians, contacts and care agents together with the effective period and authority scope of those relationships. It supports correct interpretation of who is related to a client and in what capacity, rather than treating a contact name as unrestricted authority. The relationship context remains distinct from the client's own identity and privacy directives.

Definition

Representing relationships between the client and carers, guardians, or contacts.

Boundary

Representation of a relationship does not by itself authorise every disclosure or establish a new legal mandate.

Harmonia Architectural Relevance

Temporal relationship information materially supports appropriate care communication and context without assigning guardianship decisions to Harmonia.

Scope

Contained Features and Established Owned Behaviour

Element	Type	Reference Id
Feature: Next-of-Kin & Guardian Association (p. 205)	Feature	`FEAT-EM-07`
Feature: Relationship Validity Tracking (p. 205)	Feature	`FEAT-EM-08`
Function: Maintain Client Relationships (p. 206)	Function	`HCM-FN-maintain-client-relationships`
Service: Client Relationship Query (p. 206)	Service	`HCM-SV-client-relationship-query`
Function: Govern Client Relationship Validity (p. 207)	Function	`HCM-FN-govern-client-relationship-validity`
Service: Representative Authority Verification (p. 207)	Service	`HCM-SV-representative-authority-verification`

3.1.10.1. Feature: Next-of-Kin & Guardian Association

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-EM-07`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Client Relationship (p. 203)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:129 (p. 565)	Linked RAD detail; exact locators in manifest.

Feature Overview

This Feature records the nominated people and capacities through which a client can be contacted or represented. Association preserves who the contact or guardian is in relation to the client; it does not make every nominated person an unrestricted recipient of clinical information.

Definition

Associate nominated emergency contacts, legal guardians, and care agents with the client record.

Established Behavioural Context

Element	Relationship to this Feature
Function: Maintain Client Relationships (p. 206)	realises feature
Service: Client Relationship Query (p. 206)	associated feature

3.1.10.2. Feature: Relationship Validity Tracking

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-EM-08`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Client Relationship (p. 203)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:130 (p. 565)	Linked RAD detail; exact locators in manifest.

Feature Overview

Relationship validity makes effective periods and authority scope part of the relationship's meaning. A contact association that was legitimate in one period or capacity need not be treated as currently valid for every purpose.

Definition

Maintain effective date intervals and authority scopes for nominated client relationships.

Established Behavioural Context

Element	Relationship to this Feature
Function: Govern Client Relationship Validity (p. 207)	realises feature
Service: Representative Authority Verification (p. 207)	associated feature

3.1.10.3. Function: Maintain Client Relationships

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-maintain-client-relationships`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Client Relationship (p. 203)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	em:59 (p. 635)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Next-of-Kin & Guardian Association (p. 205)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Records links between patients and carers, next of kin, nominated representatives, and legal guardians.

3.1.10.4. Service: Client Relationship Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-client-relationship-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Client Relationship (p. 203)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	em:60 (p. 635)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Next-of-Kin & Guardian Association (p. 205)	Established association only.

Governed Description and Qualifications

Discloses verified support network members and contact details to clinical teams.

3.1.10.5. Function: Govern Client Relationship Validity

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-govern-client-relationship-validity`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Client Relationship (p. 203)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	em:62 (p. 635)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Relationship Validity Tracking (p. 205)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Verifies power-of-attorney documents, guardianship orders, and proxy validity periods.

3.1.10.6. Service: Representative Authority Verification

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-representative-authority-verification`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Client Relationship (p. 203)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	em:63 (p. 635)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Relationship Validity Tracking (p. 205)	Established association only.

Governed Description and Qualifications

Confirms legal decision-making authority for substitute consent.

3.1.11. BEC: Client Privacy

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-client-privacy`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Harmonia-Core	Independent of status; never inherited.
Source	enabling:132 (p. 565); em:69 (p. 636)	Linked RAD detail; exact locators in manifest.

Capability Overview

Client Privacy applies client-level consent directives, confidentiality markers and privacy preferences to governed information use. It makes the person's restrictions operationally relevant at disclosure, query and presentation boundaries instead of leaving them as passive record annotations. Its responsibility complements the broader policy boundary while retaining the distinction between the directive and an action evaluated against it.

Definition

Enforcing client-level privacy preferences, consent directives, and confidentiality markers.

Boundary

A privacy flag does not become a caller-controlled grant of authority. Consent evaluation and information access must respect established policy ownership.

Harmonia Architectural Relevance

Consistent application of client restrictions across information pathways is a direct platform responsibility, supporting Core classification.

Scope

Contained Features and Established Owned Behaviour

Element	Type	Reference Id
Feature: Consent Directive Evaluation (p. 208)	Feature	`FEAT-EM-09`
Feature: Confidentiality Flag Enforcement (p. 209)	Feature	`FEAT-EM-10`
Function: Evaluate Client Consent (p. 210)	Function	`HCM-FN-evaluate-client-consent`
Service: Consent Decision Service (p. 210)	Service	`HCM-SV-consent-decision-service`
Function: Evaluate Client Confidentiality Restrictions (p. 210)	Function	`HCM-FN-evaluate-client-confidentiality-restrictions`
Service: Confidentiality Restriction Evaluation (p. 211)	Service	`HCM-SV-confidentiality-restriction-evaluation`

3.1.11.1. Feature: Consent Directive Evaluation

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-EM-09`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Client Privacy (p. 207)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:134 (p. 565)	Linked RAD detail; exact locators in manifest.

Feature Overview

Consent evaluation brings the client's applicable directives into consideration of a requested disclosure or exchange. It distinguishes the recorded directive from the particular action being evaluated, preserving the need for established policy and authority rather than treating consent data as a universal permission token.

Definition

Evaluate client consent directives against requested information disclosure and exchange actions.

Established Behavioural Context

Element	Relationship to this Feature
Function: Evaluate Client Consent (p. 210)	realises feature
Service: Consent Decision Service (p. 210)	associated feature

3.1.11.2. Feature: Confidentiality Flag Enforcement

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-EM-10`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Client Privacy (p. 207)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:135 (p. 565)	Linked RAD detail; exact locators in manifest.

Feature Overview

Confidentiality enforcement makes a restricted-record condition effective across discovery and presentation paths. Its purpose is consistent governed treatment of sensitive information, rather than a flag displayed in one interface while other paths disclose the same content without restriction.

Definition

Enforce restricted access flags (e.g., VIP, sensitive diagnosis, restricted record) across query and presentation interfaces.

Established Behavioural Context

Element	Relationship to this Feature
Function: Evaluate Client Confidentiality Restrictions (p. 210)	realises feature
Service: Confidentiality Restriction Evaluation (p. 211)	associated feature

3.1.11.3. Function: Evaluate Client Consent

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-evaluate-client-consent`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Client Privacy (p. 207)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	em:73 (p. 636)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Consent Directive Evaluation (p. 208)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Evaluates patient opt-in, opt-out, and general information sharing preferences against proposed exchanges.

3.1.11.4. Service: Consent Decision Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-consent-decision-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Client Privacy (p. 207)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	em:74 (p. 636)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Consent Directive Evaluation (p. 208)	Established association only.

Governed Description and Qualifications

Provides real-time consent evaluation decisions to Health Information Control.

3.1.11.5. Function: Evaluate Client Confidentiality Restrictions

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-evaluate-client-confidentiality-restrictions`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Client Privacy (p. 207)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	em:76 (p. 636)	Linked RAD detail; exact locators in manifest.

Name	Value	Comment / Description
Derived From	Feature: Confidentiality Flag Enforcement (p. 209)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Evaluates sensitive record flags (e.g., VIP masking, restricted department exclusions).

3.1.11.6. Service: Confidentiality Restriction Evaluation

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-confidentiality-restriction-evaluation`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Client Privacy (p. 207)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	em:77 (p. 636)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Confidentiality Flag Enforcement (p. 209)	Established association only.

Governed Description and Qualifications

Enforces record-level masking directives across the HIE.

3.2. Service Administration

Capability	Reference Id
BEC: Client-Centred Administration (p. 212)	`HCM-BEC-client-centred-administration`
BEC: Referral Administration (p. 215)	`HCM-BEC-referral-administration`
BEC: Scheduling Administration (p. 222)	`HCM-BEC-scheduling-administration`
BEC: Episode & Encounter Administration (p. 225)	`HCM-BEC-episode-encounter-administration`
BEC: Order Administration (p. 232)	`HCM-BEC-order-administration`
BEC: Diagnostic Administration (p. 241)	`HCM-BEC-diagnostic-administration`
BEC: Medication Administration (p. 246)	`HCM-BEC-medication-administration`
BEC: Procedure Administration (p. 251)	`HCM-BEC-procedure-administration`
BEC: Clinical Record Administration (p. 255)	`HCM-BEC-clinical-record-administration`
BEC: Care Plan Administration (p. 261)	`HCM-BEC-care-plan-administration`
BEC: Clinical Communication Administration (p. 265)	`HCM-BEC-clinical-communication-administration`

Capability	Reference Id
BEC: Funding & Coverage Administration (p. 271)	`HCM-BEC-funding-coverage-administration`
BEC: Clinical Charging & Claims Administration (p. 271)	`HCM-BEC-clinical-charging-claims-administration`

3.2.1. BEC: Client-Centred Administration

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-client-centred-administration`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Harmonia-Relevant	Independent of status; never inherited.
Source	enabling:241 (p. 568); enabling:279 (p. 569); sa:12 (p. 640)	Linked RAD detail; exact locators in manifest.

Capability Overview

Client-Centred Administration receives demographic and registration changes and makes verified updates available to participating clinical systems. It connects administrative events from external PAS environments with the client context used across care. The enabling responsibility concerns integration of changes, rather than independently deciding the authoritative demographic truth whenever a source sends an update.

Boundary

Ingestion and distribution do not establish master clinical identity reconciliation or authority over the originating registration process.

Harmonia Architectural Relevance

Demographic changes materially affect downstream association and continuity, so their governed propagation is active enablement rather than passive directory context.

Scope

Contained Features and Established Owned Behaviour

Element	Type	Reference Id
Feature: Demographic Update Ingestion (p. 213)	Feature	`FEAT-SA-01`
Feature: Demographic Change Distribution (p. 213)	Feature	`FEAT-SA-02`
Function: Receive Client Demographic Change (p. 214)	Function	`HCM-FN-receive-client-demographic-change`
Service: Demographic Intake Service (p. 214)	Service	`HCM-SV-demographic-intake-service`
Function: Distribute Governed Demographic Change (p. 215)	Function	`HCM-FN-distribute-governed-demographic-change`

Element	Type	Reference Id
Service: Demographic Update Notification (p. 215)	Service	`HCM-SV-demographic-update-notification`

3.2.1.1. Feature: Demographic Update Ingestion

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SA-01`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Client-Centred Administration (p. 212)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:281 (p. 569)	Linked RAD detail; exact locators in manifest.

Feature Overview

Demographic ingestion receives administrative changes from upstream admission and registration systems. It makes externally established updates available for governed processing without treating each incoming assertion as an independent master-person adjudication by Harmonia.

Definition

Ingest and validate patient admission and registration updates from external PAS systems.

Established Behavioural Context

Element	Relationship to this Feature
Function: Receive Client Demographic Change (p. 214)	realises feature
Service: Demographic Intake Service (p. 214)	associated feature

3.2.1.2. Feature: Demographic Change Distribution

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SA-02`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Client-Centred Administration (p. 212)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:282 (p. 569)	Linked RAD detail; exact locators in manifest.

Feature Overview

Change distribution propagates verified demographic updates to participating clinical systems. It supports consistent subject context across the network while keeping delivery to each destination distinguishable from that destination's eventual incorporation or business response.

Definition

Fan out verified client demographic updates to downstream clinical subsystems.

Established Behavioural Context

Element	Relationship to this Feature
Function: Distribute Governed Demographic Change (p. 215)	realises feature
Service: Demographic Update Notification (p. 215)	associated feature

3.2.1.3. Function: Receive Client Demographic Change

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-receive-client-demographic-change`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Client-Centred Administration (p. 212)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sa:18 (p. 640)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Demographic Update Ingestion (p. 213)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Ingests administrative demographic updates from PAS/registration feeds.

3.2.1.4. Service: Demographic Intake Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-demographic-intake-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Client-Centred Administration (p. 212)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sa:19 (p. 640)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Demographic Update Ingestion (p. 213)	Established association only.

Governed Description and Qualifications

Exposed to departmental registration systems.

3.2.1.5. Function: Distribute Governed Demographic Change

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-distribute-governed-demographic-change`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Client-Centred Administration (p. 212)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sa:21 (p. 640)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Demographic Change Distribution (p. 213)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Propagates verified demographic modifications to subscribed clinical subsystems.

3.2.1.6. Service: Demographic Update Notification

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-demographic-update-notification`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Client-Centred Administration (p. 212)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sa:22 (p. 640)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Demographic Change Distribution (p. 213)	Established association only.

Governed Description and Qualifications

Emits change notifications across the HIE.

3.2.2. BEC: Referral Administration

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-referral-administration`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.

Name	Value	Comment / Description
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Harmonia-Relevant	Independent of status; never inherited.
Source	enabling:242 (p. 568); enabling:284 (p. 569); sa:28 (p. 640)	Linked RAD detail; exact locators in manifest.

Capability Overview

Referral Administration receives referral material, associates the supporting clinical context and tracks the recipient's explicit disposition and outcome. Its purpose is to keep the request and its progression intelligible across care boundaries. It distinguishes a request for another participant's involvement from an order for bounded fulfilment and from actual acceptance of care responsibility.

Boundary

Transport acceptance does not establish clinical acceptance. A referral need not universally produce an appointment, encounter, order or transfer of care.

Harmonia Architectural Relevance

Harmonia materially connects the referral, supporting information and reported disposition while the receiving service retains clinical and responsibility decisions.

Scope

Contained Features and Established Owned Behaviour

Element	Type	Reference Id
Feature: Referral Document Ingestion (p. 216)	Feature	`FEAT-SA-03`
Feature: Referral Supporting Information (p. 217)	Feature	`FEAT-SA-04`
Feature: Referral Status & Outcome Tracking (p. 218)	Feature	`FEAT-SA-05`
Function: Receive Referral (p. 218)	Function	`HCM-FN-receive-referral`
Service: Referral Submission Service (p. 219)	Service	`HCM-SV-referral-submission-service`
Function: Assemble Referral Context (p. 219)	Function	`HCM-FN-assemble-referral-context`
Service: Referral Context Query (p. 220)	Service	`HCM-SV-referral-context-query`
Function: Manage Referral Progression (p. 220)	Function	`HCM-FN-manage-referral-progression`
Service: Referral Status & Outcome Service (p. 220)	Service	`HCM-SV-referral-status-outcome-service`
Process: Referral Progression Process (p. 221)	Process	`HCM-PR-referral-progression-process`

3.2.2.1. Feature: Referral Document Ingestion

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SA-03`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.

Name	Value	Comment / Description
Owning Context	BEC: Referral Administration (p. 215)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:286 (p. 569)	Linked RAD detail; exact locators in manifest.

Feature Overview

Referral ingestion establishes the received referral material within the integration boundary. It preserves the request for another participant's involvement without treating successful receipt as clinical acceptance, responsibility assumption or a promise that care will be delivered.

Definition

Ingest structured clinical referral messages across integration boundaries.

Established Behavioural Context

Element	Relationship to this Feature
Function: Receive Referral (p. 218)	realises feature
Service: Referral Submission Service (p. 219)	associated feature

3.2.2.2. Feature: Referral Supporting Information

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SA-04`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Referral Administration (p. 215)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:287 (p. 569)	Linked RAD detail; exact locators in manifest.

Feature Overview

Supporting information gives the referral its relevant history and clinical context. Association makes the material available for the recipient's consideration while preserving source provenance and the distinction between evidence supplied and the recipient's own clinical decision.

Definition

Collate and associate supporting clinical documentation, diagnostic history, and patient context with referral requests.

Established Behavioural Context

Element	Relationship to this Feature
Function: Assemble Referral Context (p. 219)	realises feature
Service: Referral Context Query (p. 220)	associated feature

3.2.2.3. Feature: Referral Status & Outcome Tracking

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SA-05`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Referral Administration (p. 215)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:288 (p. 569)	Linked RAD detail; exact locators in manifest.

Feature Overview

Status and outcome tracking makes explicit referral dispositions visible to the referring participant. Acceptance, decline and redirection remain meaningful alternatives; a transport acknowledgement does not fill in a missing business decision or establish a universal referral progression.

Definition

Track referral operational progression and communicate referral outcomes (acceptance, decline, or redirection) to referring providers.

Established Behavioural Context

Element	Relationship to this Feature
Function: Manage Referral Progression (p. 220)	realises feature
Service: Referral Status & Outcome Service (p. 220)	associated feature

3.2.2.4. Function: Receive Referral

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-receive-referral`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Referral Administration (p. 215)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.

Name	Value	Comment / Description
Source	sa:33 (p. 640)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Referral Document Ingestion (p. 216)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Validates and ingests structured electronic referrals, clinical indications, and priority categories.

3.2.2.5. Service: Referral Submission Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-referral-submission-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Referral Administration (p. 215)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sa:34 (p. 640)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Referral Document Ingestion (p. 216)	Established association only.

Governed Description and Qualifications

Exposed to external GP practices and referring health facilities.

3.2.2.6. Function: Assemble Referral Context

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-assemble-referral-context`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Referral Administration (p. 215)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sa:36 (p. 640)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Referral Supporting Information (p. 217)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Gathers relevant clinical history, medications, and diagnostic investigations into the referral package.

3.2.2.7. Service: Referral Context Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-referral-context-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Referral Administration (p. 215)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sa:37 (p. 640)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Referral Supporting Information (p. 217)	Established association only.

Governed Description and Qualifications

Provides comprehensive referral dossiers to intake triage clinicians.

3.2.2.8. Function: Manage Referral Progression

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-manage-referral-progression`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Referral Administration (p. 215)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sa:39 (p. 640)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Referral Status & Outcome Tracking (p. 218)	Established Feature realisation; not structural containment.
Governed Process	Process: Referral Progression Process (p. 221)	Established progression relationship.

Governed Description and Qualifications

Tracks referral disposition in an **illustrative specialist pathway; dispositions vary**: *Submitted → Triaged → Accepted/Waitlisted → Scheduled → Discharged*.

3.2.2.9. Service: Referral Status & Outcome Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-referral-status-outcome-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Referral Administration (p. 215)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.

Name	Value	Comment / Description
Source	sa:40 (p. 640)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Referral Status & Outcome Tracking (p. 218)	Established association only.

Governed Description and Qualifications

Exposes progression milestones to referrers and patients.

3.2.2.10. Process: Referral Progression Process

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-PR-referral-progression-process`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Referral Administration (p. 215)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	processes:102 (p. 615); processes:103 (p. 615)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Process Overview

This Process illustrates one specialist pathway through validation, triage and booking. It makes that pathway's progression visible without imposing it on every referral: rejection, decline or redirection can be legitimate dispositions, and a request need not universally lead to attendance. Neither a transmitted referral nor the local compound acceptance/waitlist checkpoint establishes actual responsibility transfer.

Governed Description and Qualifications

- **Owning Capability**: `ReferralAdministration`
- **Process Purpose**: Governs the end-to-end operational progression of an incoming clinical referral from receipt through specialist clinical triage, booking, and final service acceptance.
- **Scope qualification**: This illustrates a specialist booking/attendance pathway; Referral does not universally require acceptance, scheduling, attendance or discharge, or establish delivery, responsibility actually assumed or Transfer of Care. Decline, rejection and redirection remain valid dispositions without invented transition paths. `Accepted/Waitlisted` is a local compound checkpoint, not universal equivalence between acceptance and waitlisting.
- **State Progression Lifecycle**: Submitted → Intake Validated → Clinically Triaged → Accepted / Waitlisted → Scheduled → Consultation Attended → Discharged / Rejected (Illustrative specialist pathway; dispositions vary)
- **Key State Dispositions**:
  - `Submitted`: Electronic referral received from GP or external facility.
  - `IntakeValidated`: Administrative validation of patient details, mandatory fields, and tests.
  - `ClinicallyTriaged`: Senior clinician assigns urgency category (e.g., Cat 1 urgent within 30 days).
  - `Accepted/Waitlisted`: Referral accepted and placed onto the specialty waitlist.
  - `Scheduled`: Outpatient appointment allocated and patient notified.

- `ConsultationAttended`: Patient seen; initial specialist assessment completed.
- `Discharged/Rejected`: Referral completed and discharged back to primary care, or formally rejected with clinical rationale.

3.2.3. BEC: Scheduling Administration

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-scheduling-administration`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Harmonia-Relevant	Independent of status; never inherited.
Source	enabling:243 (p. 568); enabling:290 (p. 569); sa:48 (p. 641)	Linked RAD detail; exact locators in manifest.

Capability Overview

Scheduling Administration integrates appointment information established by departmental scheduling systems. It receives scheduled events and propagates cancellation, rescheduling and attendance changes so that care-coordination contexts do not diverge. Its concern is the coherence of scheduling information across participants rather than originating every booking decision.

Boundary

A synchronised booking does not prove attendance, service capacity or delivered clinical care.

Harmonia Architectural Relevance

Timely appointment changes materially support coordination across systems without giving Harmonia the scheduler's clinical or departmental authority.

Scope

Contained Features and Established Owned Behaviour

Element	Type	Reference Id
Feature: Appointment Notification Ingestion (p. 223)	Feature	`FEAT-SA-06`
Feature: Appointment Status Synchronization (p. 223)	Feature	`FEAT-SA-07`
Function: Receive Appointment Notification (p. 224)	Function	`HCM-FN-receive-appointment-notification`
Service: Appointment Notification Ingress (p. 224)	Service	`HCM-SV-appointment-notification-ingress`
Function: Synchronise Appointment Status (p. 225)	Function	`HCM-FN-synchronise-appointment-status`
Service: Appointment Schedule Query (p. 225)	Service	`HCM-SV-appointment-schedule-query`

3.2.3.1. Feature: Appointment Notification Ingestion

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SA-06`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Scheduling Administration (p. 222)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:292 (p. 569)	Linked RAD detail; exact locators in manifest.

Feature Overview

Appointment ingestion receives scheduling facts established by departmental systems. It supports shared awareness of planned care opportunities without claiming authority over the underlying booking decision or proving that attendance and clinical fulfilment will occur.

Definition

Ingest scheduled appointment events from departmental scheduling systems.

Established Behavioural Context

Element	Relationship to this Feature
Function: Receive Appointment Notification (p. 224)	realises feature
Service: Appointment Notification Ingress (p. 224)	associated feature

3.2.3.2. Feature: Appointment Status Synchronization

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SA-07`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Scheduling Administration (p. 222)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:293 (p. 569)	Linked RAD detail; exact locators in manifest.

Feature Overview

Status synchronisation keeps cancellation, rescheduling and attendance information coherent across coordination contexts. The purpose is propagation of the externally established booking change rather than independent scheduling or inference of a clinical outcome from appointment state.

Definition

Propagate booking cancellations, reschedules, and attendances across care coordination systems.

Established Behavioural Context

Element	Relationship to this Feature
Function: Synchronise Appointment Status (p. 225)	realises feature
Service: Appointment Schedule Query (p. 225)	associated feature

3.2.3.3. Function: Receive Appointment Notification

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-receive-appointment-notification`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Scheduling Administration (p. 222)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sa:53 (p. 641)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Appointment Notification Ingestion (p. 223)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Ingests appointment creation, rescheduling, cancellation, and check-in events from host PAS engines.

3.2.3.4. Service: Appointment Notification Ingress

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-appointment-notification-ingress`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Scheduling Administration (p. 222)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sa:54 (p. 641)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Appointment Notification Ingestion (p. 223)	Established association only.

Governed Description and Qualifications

Ingests booking events from departmental schedulers.

3.2.3.5. Function: Synchronise Appointment Status

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-synchronise-appointment-status`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Scheduling Administration (p. 222)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sa:56 (p. 641)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Appointment Status Synchronization (p. 223)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Projects synchronised appointment timelines to clinical dashboards and care coordinators.

3.2.3.6. Service: Appointment Schedule Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-appointment-schedule-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Scheduling Administration (p. 222)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sa:57 (p. 641)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Appointment Status Synchronization (p. 223)	Established association only.

Governed Description and Qualifications

Exposes consolidated patient appointment schedules across facilities.

3.2.4. BEC: Episode & Encounter Administration

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-episode-encounter-administration`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Harmonia-Core	Independent of status; never inherited.

Name	Value	Comment / Description
Source	enabling:244 (p. 568); enabling:295 (p. 569); sa:63 (p. 641)	Linked RAD detail; exact locators in manifest.

Capability Overview

This capability maintains identifiable encounter contexts and their governed progression, including association with care teams and locations. It enables information from an admission, visit or community contact to be understood within the relevant episode of activity. Encounter progression is an administrative and operational account, separate from the clinical acts that occur during it.

Boundary

The source lists ON_LEAVE in the Feature description but does not fully connect it in the Process illustration. That transition gap remains unresolved rather than completed by convention.

Harmonia Architectural Relevance

Encounter context and tracked state are substantive platform responsibilities supporting correlation across records, orders and operational events.

Scope

Contained Features and Established Owned Behaviour

Element	Type	Reference Id
Feature: Encounter Context Creation (p. 226)	Feature	`FEAT-SA-08`
Feature: Encounter State Progression (p. 227)	Feature	`FEAT-SA-09`
Feature: Encounter Bed/Location Association (p. 228)	Feature	`FEAT-SA-10`
Function: Establish Encounter Context (p. 228)	Function	`HCM-FN-establish-encounter-context`
Service: Encounter Context Resolution (p. 229)	Service	`HCM-SV-encounter-context-resolution`
Function: Progress Encounter State (p. 229)	Function	`HCM-FN-progress-encounter-state`
Service: Encounter Lifecycle Event Notification (p. 230)	Service	`HCM-SV-encounter-lifecycle-event-notification`
Function: Maintain Encounter Care-Place Association (p. 230)	Function	`HCM-FN-maintain-encounter-care-place-association`
Service: Encounter Location History Query (p. 230)	Service	`HCM-SV-encounter-location-history-query`
Process: Encounter Lifecycle Process (p. 231)	Process	`HCM-PR-encounter-lifecycle-process`

3.2.4.1. Feature: Encounter Context Creation

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SA-08`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Episode & Encounter Administration (p. 225)	Established responsibility only; complete CT ancestry Unresolved.

Name	Value	Comment / Description
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:297 (p. 569)	Linked RAD detail; exact locators in manifest.

Feature Overview

Encounter context creation supplies an identifiable frame for information concerning a particular admission, visit or contact. Its governed tracking authority concerns the encounter representation within Harmonia; it does not make Harmonia the originator of every clinical fact associated with that encounter.

Definition

Create and maintain authoritative encounter tracking contexts for hospital admissions, outpatient visits, and community contacts.

Established Behavioural Context

Element	Relationship to this Feature
Function: Establish Encounter Context (p. 228)	realises feature
Service: Encounter Context Resolution (p. 229)	associated feature

3.2.4.2. Feature: Encounter State Progression

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SA-09`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Episode & Encounter Administration (p. 225)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:298 (p. 569)	Linked RAD detail; exact locators in manifest.

Feature Overview

Encounter progression preserves the difference between planned, arrived, active, leave and discharged circumstances. These states make administrative activity interpretable without implying a clinical outcome. The source's ON_LEAVE transition detail remains incomplete in the corresponding Process and is not repaired here.

Definition

Track encounter status transitions (`PLANNED` → `ARRIVED` → `IN_PROGRESS` → `ON_LEAVE` → `DISCHARGED`).

Established Behavioural Context

Element	Relationship to this Feature
Function: Progress Encounter State (p. 229)	realises feature
Service: Encounter Lifecycle Event Notification (p. 230)	associated feature

3.2.4.3. Feature: Encounter Bed/Location Association

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SA-10`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Episode & Encounter Administration (p. 225)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:299 (p. 569)	Linked RAD detail; exact locators in manifest.

Feature Overview

Encounter association keeps the relevant place and care-team context aligned with the episode as the patient moves. Location changes are meaningful operational facts, but they do not themselves establish a transfer of clinical responsibility.

Definition

Dynamically maintain the association between an encounter, care team, and physical bed or ward location.

Established Behavioural Context

Element	Relationship to this Feature
Function: Maintain Encounter Care-Place Association (p. 230)	realises feature
Service: Encounter Location History Query (p. 230)	associated feature

3.2.4.4. Function: Establish Encounter Context

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-establish-encounter-context`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Episode & Encounter Administration (p. 225)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.

Name	Value	Comment / Description
Source	sa:68 (p. 641)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Encounter Context Creation (p. 226)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Binds patient, attending practitioner, clinical class (e.g., Inpatient, Emergency, Ambulatory), and admitting diagnosis to a unique encounter identifier.

3.2.4.5. Service: Encounter Context Resolution

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-encounter-context-resolution`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Episode & Encounter Administration (p. 225)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sa:69 (p. 641)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Encounter Context Creation (p. 226)	Established association only.

Governed Description and Qualifications

Discloses active encounter details to clinical applications.

3.2.4.6. Function: Progress Encounter State

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-progress-encounter-state`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Episode & Encounter Administration (p. 225)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sa:71 (p. 641)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Encounter State Progression (p. 227)	Established Feature realisation; not structural containment.
Governed Process	Process: Encounter Lifecycle Process (p. 231)	Established progression relationship.

Governed Description and Qualifications

Coordinates encounter progression (**scoped illustration; ON_LEAVE relationship unresolved**): *Planned → Arrived → In-Progress → Discharged → Completed*.

3.2.4.7. Service: Encounter Lifecycle Event Notification

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-encounter-lifecycle-event-notification`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Episode & Encounter Administration (p. 225)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sa:72 (p. 641)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Encounter State Progression (p. 227)	Established association only.

Governed Description and Qualifications

Emits encounter transition events across the HIE.

3.2.4.8. Function: Maintain Encounter Care-Place Association

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-maintain-encounter-care-place-association`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Episode & Encounter Administration (p. 225)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sa:76 (p. 641)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Encounter Bed/Location Association (p. 228)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Binds the encounter to physical wards, rooms, and beds over time.

3.2.4.9. Service: Encounter Location History Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-encounter-location-history-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Episode & Encounter Administration (p. 225)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sa:77 (p. 641)	Linked RAD detail; exact locators in manifest.

Name	Value	Comment / Description
Associated Feature	Feature: Encounter Bed/Location Association (p. 228)	Established association only.

Governed Description and Qualifications

Discloses patient location tracking history.

3.2.4.10. Process: Encounter Lifecycle Process

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-PR-encounter-lifecycle-process`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Episode & Encounter Administration (p. 225)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	processes:119 (p. 616); processes:120 (p. 616)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Process Overview

The encounter lifecycle distinguishes the planned opportunity, presence, active encounter and locally applicable completion context. It supplies an administrative progression frame for care information rather than evidence that every clinical objective has been met. The relation to Strategy's ON_LEAVE state remains unresolved, and local signing or legal-closure descriptions do not become universal completion rules.

Governed Description and Qualifications

- **Owning Capability**: `Episode&EncounterAdministration`
- **Process Purpose**: Governs the clinical and administrative lifecycle of an acute, emergency, inpatient, or outpatient encounter between a patient and healthcare services.
- **State Progression Lifecycle**: Planned / Booked → Arrived → Triaged / Ingested → Active In-Progress → Discharged → Completed / Encoded (Scoped illustration; ON_LEAVE relationship unresolved)
- **Key State Dispositions**:
  - `Planned/Booked`: Elective admission or clinic appointment scheduled.
  - `Arrived`: Patient presents at facility or emergency desk.
  - `Triaged/Ingested`: Clinical triage category assigned; encounter record activated.
  - `ActiveIn-Progress`: Inpatient care, bedside monitoring, and clinical orders actively underway.
  - `Discharged`: Patient clinically discharged; care-place vacated.
  - `Completed/Encoded`: Clinical documentation finalized, ICD/DRG coding completed, and encounter legally closed.
- **Scope qualification**: These completion criteria retain their locally described applicability; they do not establish universal signing, legal closure or originating authority over clinical information. Finalisation, signing and legal qualification remain distinct. The relationship with Strategy `ON_LEAVE`, including applicability and transitions, remains unresolved.

3.2.5. BEC: Order Administration

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-order-administration`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Harmonia-Core	Independent of status; never inherited.
Source	enabling:245 (p. 568); enabling:301 (p. 570); sa:83 (p. 642)	Linked RAD detail; exact locators in manifest.

Capability Overview

Order Administration manages the closed loop around a request for defined fulfilment. It receives the request, resolves and routes to a destination, distinguishes delivery from business disposition, tracks progress and associates the eventual outcome. This makes the difference between an order sent and an order fulfilled visible throughout the requester's interaction.

Intent

Preserve an intelligible connection between the original request, its destination, progression and returned outcome.

Boundary

Technical acknowledgements have only their contractual meaning. Receipt does not universally establish durable acceptance, business acceptance or completion; outcome association has no invented Feature equivalence.

Harmonia Architectural Relevance

Reliable routing and closed-loop state are direct platform responsibilities. The performing service retains authority over clinical execution and its result.

Scope

Contained Features and Established Owned Behaviour

Element	Type	Reference Id
Feature: Order Request Ingestion (p. 233)	Feature	`FEAT-SA-11`
Feature: Order Destination Resolution & Routing (p. 234)	Feature	`FEAT-SA-12`
Feature: Order Closed-Loop Progression Tracking (p. 234)	Feature	`FEAT-SA-13`
Feature: Order Cancellation & Modification Coordination (p. 235)	Feature	`FEAT-SA-14`
Function: Receive Order Request (p. 236)	Function	`HCM-FN-receive-order-request`
Service: Order Requisition Ingress (p. 236)	Service	`HCM-SV-order-requisition-ingress`
Function: Resolve Order Destination (p. 236)	Function	`HCM-FN-resolve-order-destination`

Element	Type	Reference Id
Service: Order Dispatch Service (p. 237)	Service	HCM-SV-order-dispatch-service
Function: Manage Order Progression (p. 237)	Function	HCM-FN-manage-order-progression
Service: Order Status & Tracking Query (p. 238)	Service	HCM-SV-order-status-tracking-query
Function: Coordinate Order Modification / Cancellation (p. 238)	Function	HCM-FN-coordinate-order-modification-cancellation
Service: Order Cancellation Service (p. 238)	Service	HCM-SV-order-cancellation-service
Function: Associate Order Outcome (p. 239)	Function	HCM-FN-associate-order-outcome
Service: Order Outcome Notification (p. 239)	Service	HCM-SV-order-outcome-notification
Process: Closed-Loop Order Progression Process (p. 240)	Process	HCM-PR-closed-loop-order-progression-process

3.2.5.1. Feature: Order Request Ingestion

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-SA-11	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Order Administration (p. 232)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:303 (p. 570)	Linked RAD detail; exact locators in manifest.

Feature Overview

Order ingestion establishes the received bounded fulfilment request with its ordering-clinician context. It begins the information account needed for a closed loop while leaving the performing service's business acceptance and execution distinguishable from technical ingress acceptance.

Definition

Ingest clinical diagnostic, medication, and procedure order requests with validated ordering clinician context.

Established Behavioural Context

Element	Relationship to this Feature
Function: Receive Order Request (p. 236)	realises feature
Service: Order Requisition Ingress (p. 236)	associated feature

3.2.5.2. Feature: Order Destination Resolution & Routing

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-SA-12	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Order Administration (p. 232)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:304 (p. 570)	Linked RAD detail; exact locators in manifest.

Feature Overview

Destination resolution connects an order with the service endpoint intended to fulfil it. Routing advances the request towards that destination without proving execution or changing the original clinical intent simply because a transport route was chosen.

Definition

Resolve fulfillment endpoints and reliably route orders to performing service systems.

Established Behavioural Context

Element	Relationship to this Feature
Function: Resolve Order Destination (p. 236)	realises feature
Service: Order Dispatch Service (p. 237)	associated feature

3.2.5.3. Feature: Order Closed-Loop Progression Tracking

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-SA-13	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Order Administration (p. 232)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:305 (p. 570)	Linked RAD detail; exact locators in manifest.

Feature Overview

Closed-loop tracking keeps delivery, technical acknowledgement, business disposition, execution and returned outcome distinct. Its purpose is to expose what has actually been established at each stage, preventing a successful handoff from being reported as eventual fulfilment.

Definition

Track order progression across destination determination, routing, delivery, receipt acknowledgement (distinguishing technical delivery acknowledgement from business acknowledgement), execution progress, completion, and outcome association.

Established Behavioural Context

Element	Relationship to this Feature
Function: Manage Order Progression (p. 237)	realises feature
Service: Order Status & Tracking Query (p. 238)	associated feature

3.2.5.4. Feature: Order Cancellation & Modification Coordination

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SA-14`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Order Administration (p. 232)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:306 (p. 570)	Linked RAD detail; exact locators in manifest.

Feature Overview

Cancellation and amendment coordination carries a requested change into the performing-system context. It preserves the difference between asking to change an order and evidence that the change took effect, rather than silently rewriting the original request as though no prior intent existed.

Definition

Coordinate order cancellation and amendment requests, ensuring synchronization with performing systems.

Established Behavioural Context

Element	Relationship to this Feature
Function: Coordinate Order Modification / Cancellation (p. 238)	realises feature
Service: Order Cancellation Service (p. 238)	associated feature

3.2.5.5. Function: Receive Order Request

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-receive-order-request`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Order Administration (p. 232)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sa:89 (p. 642)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Order Request Ingestion (p. 233)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Ingests structured orders containing clinical indication, requesting clinician, and test specifications.

3.2.5.6. Service: Order Requisition Ingress

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-order-requisition-ingress`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Order Administration (p. 232)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sa:90 (p. 642)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Order Request Ingestion (p. 233)	Established association only.

Governed Description and Qualifications

Ingests electronic orders from clinical ordering systems.

3.2.5.7. Function: Resolve Order Destination

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-resolve-order-destination`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Order Administration (p. 232)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sa:92 (p. 642)	Linked RAD detail; exact locators in manifest.

Name	Value	Comment / Description
Derived From	Feature: Order Destination Resolution & Routing (p. 234)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Evaluates order routing rules to determine target laboratory, imaging centre, or procedural unit.

3.2.5.8. Service: Order Dispatch Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-order-dispatch-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Order Administration (p. 232)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sa:93 (p. 642)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Order Destination Resolution & Routing (p. 234)	Established association only.

Governed Description and Qualifications

Transmits orders to performing diagnostic systems.

3.2.5.9. Function: Manage Order Progression

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-manage-order-progression`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Order Administration (p. 232)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sa:95 (p. 642)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Order Closed-Loop Progression Tracking (p. 234)	Established Feature realisation; not structural containment.
Governed Process	Process: Closed-Loop Order Progression Process (p. 240)	Established progression relationship.

Governed Description and Qualifications

Tracks order state (**scoped diagnostic/procedural illustration**): *Placed → Received → Specimen Collected → In-Progress → Preliminary Result → Final Result → Closed*.

3.2.5.10. Service: Order Status & Tracking Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-order-status-tracking-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Order Administration (p. 232)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sa:96 (p. 642)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Order Closed-Loop Progression Tracking (p. 234)	Established association only.

Governed Description and Qualifications

Provides real-time order tracking to ordering clinicians.

3.2.5.11. Function: Coordinate Order Modification / Cancellation

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-coordinate-order-modification-cancellation`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Order Administration (p. 232)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sa:100 (p. 642)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Order Cancellation & Modification Coordination (p. 235)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Handles requests to amend clinical details or cancel unexecuted orders.

3.2.5.12. Service: Order Cancellation Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-order-cancellation-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Order Administration (p. 232)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sa:101 (p. 642)	Linked RAD detail; exact locators in manifest.

Name	Value	Comment / Description
Associated Feature	Feature: Order Cancellation & Modification Coordination (p. 235)	Established association only.

Governed Description and Qualifications

Dispatches cancel directives to performing systems.

3.2.5.13. Function: Associate Order Outcome

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-associate-order-outcome`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Order Administration (p. 232)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sa:103 (p. 642)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Governed Description and Qualifications

Correlates returning diagnostic reports and observations back to the originating order requisition.

3.2.5.14. Service: Order Outcome Notification

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-order-outcome-notification`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Order Administration (p. 232)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sa:104 (p. 642)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Governed Description and Qualifications

Emits completion notices to ordering practitioners.

3.2.5.15. Process: Closed-Loop Order Progression Process

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-PR-closed-loop-order-progression-process`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Order Administration (p. 232)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	processes:135 (p. 616); processes:136 (p. 616)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Process Overview

Closed-loop progression preserves the connection between the requisition, performing context and returned result so that a request does not disappear after dispatch. The diagnostic/procedural illustration includes specimen and report checkpoints without excluding medication from the broader owning capability. Dispatch, recipient acceptance, result binding and requester closure remain distinct facts even where a source checkpoint combines them.

Governed Description and Qualifications

- **Owning Capability**: `OrderAdministration`
- **Process Purpose**: Governs the rigorous, closed-loop tracking of diagnostic pathology, radiology, and procedural orders from requisition to result correlation, preventing dropped or unfulfilled investigations.
- **Scope qualification**: These examples describe a diagnostic/procedural progression and do not exclude medication from the broader Order Administration responsibility. Specimen and result checkpoints are scoped. The signed requisition/report requirements remain within this illustrated scope; signing is not universally equivalent to finalisation or authority. Result consumption/binding does not define originating validity, authority, authorship or legal status.
- **State Progression Lifecycle**: Requisition Placed → Order Dispatched → Specimen Collected / Scheduled → In-Execution → Preliminary Result Bound → Final Result Bound → Closed / Verified (Scoped diagnostic/procedural illustration)
- **Key State Dispositions**:
  - `RequisitionPlaced`: Electronic order created and signed by requesting clinician.
  - `OrderDispatched`: Order routed and accepted by performing diagnostic service. This local compound checkpoint retains routing and recipient acceptance as distinct facts; dispatch alone establishes neither receipt nor acceptance. Behaviour `Received` is not equated with this checkpoint without evidence.
  - `SpecimenCollected/Scheduled`: Bio-specimen collected or imaging appointment booked.
  - `In-Execution`: Laboratory analysis or diagnostic scanning underway.
  - `PreliminaryResultBound`: Critical or interim observations published and linked to order.
  - `FinalResultBound`: Authoritative diagnostic report signed and bound to order.
  - `Closed/Verified`: Requesting clinician acknowledges result; order closed. This Order-closure checkpoint does not establish diagnostic-content verification, approval or clinical incorporation.

3.2.6. BEC: Diagnostic Administration

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-diagnostic-administration`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Harmonia-Core	Independent of status; never inherited.
Source	enabling:246 (p. 568); enabling:308 (p. 570); sa:110 (p. 643)	Linked RAD detail; exact locators in manifest.

Capability Overview

Diagnostic Administration correlates requests with specimens and encounters and integrates diagnostic reports into their request context. It connects the laboratory or imaging contribution with the participants expecting an outcome, preserving report identity and standing as updates occur. Its enabling role differs from performing the diagnostic procedure or professionally interpreting evidence.

Boundary

The finalised-report distribution Feature retains its narrower scope. Broader Domain03 behaviour covering preliminary, corrected or addendum information does not silently expand that Feature.

Harmonia Architectural Relevance

Request, specimen and report correlation and governed distribution are substantive information-integration responsibilities.

Scope

Contained Features and Established Owned Behaviour

Element	Type	Reference Id
Feature: Diagnostic Request Correlation (p. 242)	Feature	`FEAT-SA-15`
Feature: Diagnostic Report Ingestion & Binding (p. 242)	Feature	`FEAT-SA-16`
Feature: Diagnostic Report Distribution (p. 243)	Feature	`FEAT-SA-17`
Function: Correlate Diagnostic Request (p. 244)	Function	`HCM-FN-correlate-diagnostic-request`
Service: Diagnostic Correlation Resolution (p. 244)	Service	`HCM-SV-diagnostic-correlation-resolution`
Function: Bind Diagnostic Report (p. 244)	Function	`HCM-FN-bind-diagnostic-report`
Service: Diagnostic Report Ingress (p. 245)	Service	`HCM-SV-diagnostic-report-ingress`
Function: Distribute Diagnostic Report (p. 245)	Function	`HCM-FN-distribute-diagnostic-report`
Service: Diagnostic Result Delivery Service (p. 246)	Service	`HCM-SV-diagnostic-result-delivery-service`

3.2.6.1. Feature: Diagnostic Request Correlation

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-SA-15	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Diagnostic Administration (p. 241)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:310 (p. 570)	Linked RAD detail; exact locators in manifest.

Feature Overview

Diagnostic request correlation connects the order with its specimen and encounter context. It helps distinguish evidence belonging to the intended fulfilment from superficially similar results, without treating association as proof of specimen suitability or diagnostic interpretation.

Definition

Correlate diagnostic test orders with collected clinical specimens and patient encounters.

Established Behavioural Context

Element	Relationship to this Feature
Function: Correlate Diagnostic Request (p. 244)	realises feature
Service: Diagnostic Correlation Resolution (p. 244)	associated feature

3.2.6.2. Feature: Diagnostic Report Ingestion & Binding

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-SA-16	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Diagnostic Administration (p. 241)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:311 (p. 570)	Linked RAD detail; exact locators in manifest.

Feature Overview

Report ingestion binds a finalised diagnostic contribution to the order it answers and verifies content integrity. The report's professional standing comes from its diagnostic source; successful ingestion does not independently authorise its findings or establish a patient-level clinical conclusion.

Definition

Ingest finalized laboratory and radiology reports, verifying content integrity and binding them to the parent order.

Established Behavioural Context

Element	Relationship to this Feature
Function: Bind Diagnostic Report (p. 244)	realises feature
Service: Diagnostic Report Ingress (p. 245)	associated feature

3.2.6.3. Feature: Diagnostic Report Distribution

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SA-17`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Diagnostic Administration (p. 241)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:312 (p. 570)	Linked RAD detail; exact locators in manifest.

Feature Overview

This Feature distributes finalised diagnostic reports to the ordering and designated care participants. Its narrower finalised-report scope is preserved even though broader Diagnostic Administration behaviour also handles preliminary, corrected and addendum information; those behaviours do not silently redefine the Feature.

Definition

Distribute finalized laboratory, pathology, and imaging reports to ordering clinicians and designated care teams.

Established Behavioural Context

Element	Relationship to this Feature
Function: Distribute Diagnostic Report (p. 245)	realises feature
Service: Diagnostic Result Delivery Service (p. 246)	associated feature

3.2.6.4. Function: Correlate Diagnostic Request

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-correlate-diagnostic-request`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Diagnostic Administration (p. 241)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sa:116 (p. 643)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Diagnostic Request Correlation (p. 242)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Matches incoming lab/radiology results to active patient encounters and order requisitions.

3.2.6.5. Service: Diagnostic Correlation Resolution

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-diagnostic-correlation-resolution`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Diagnostic Administration (p. 241)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sa:117 (p. 643)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Diagnostic Request Correlation (p. 242)	Established association only.

Governed Description and Qualifications

Discloses correlated order/result linkages.

3.2.6.6. Function: Bind Diagnostic Report

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-bind-diagnostic-report`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Diagnostic Administration (p. 241)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sa:119 (p. 643)	Linked RAD detail; exact locators in manifest.

Name	Value	Comment / Description
Derived From	Feature: Diagnostic Report Ingestion & Binding (p. 242)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Associates structured observations, reference ranges, and abnormal flags with patient health records.

3.2.6.7. Service: Diagnostic Report Ingress

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-diagnostic-report-ingress`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Diagnostic Administration (p. 241)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sa:120 (p. 643)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Diagnostic Report Ingestion & Binding (p. 242)	Established association only.

Governed Description and Qualifications

Ingests results from external LIS/RIS systems.

3.2.6.8. Function: Distribute Diagnostic Report

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-distribute-diagnostic-report`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Diagnostic Administration (p. 241)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sa:122 (p. 643)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Diagnostic Report Distribution (p. 243)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Delivers preliminary, final, and corrected reports to ordering and copied practitioners.

3.2.6.9. Service: Diagnostic Result Delivery Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-diagnostic-result-delivery-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Diagnostic Administration (p. 241)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sa:124 (p. 643)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Diagnostic Report Distribution (p. 243)	Established association only.

Governed Description and Qualifications

Emits secure result notifications.

3.2.7. BEC: Medication Administration

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-medication-administration`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Harmonia-Relevant	Independent of status; never inherited.
Source	enabling:247 (p. 568); enabling:316 (p. 570); sa:130 (p. 643)	Linked RAD detail; exact locators in manifest.

Capability Overview

In this enabling catalogue, Medication Administration integrates medication orders, dispensing evidence and point-of-care administration records. It preserves the distinct events through which intended therapy becomes supplied and, where reported, administered. This administrative integration supports a coherent medication account across systems without itself dispensing or administering a medicine.

Boundary

Prescribed, dispensed and administered are different states. The capability name does not assign bedside medication delivery to Harmonia.

Harmonia Architectural Relevance

Connecting medication events materially supports continuity and interpretation while clinical and pharmacy actors retain originating responsibility.

Scope

Contained Features and Established Owned Behaviour

Element	Type	Reference Id
Feature: Medication Order Ingestion (p. 247)	Feature	FEAT-SA-18
Feature: Dispense Event Tracking (p. 248)	Feature	FEAT-SA-19
Feature: Medication Administration Record (MAR) Ingestion (p. 248)	Feature	FEAT-SA-20
Function: Receive Medication Order (p. 249)	Function	HCM-FN-receive-medication-order
Service: Medication Order Ingress (p. 249)	Service	HCM-SV-medication-order-ingress
Function: Track Dispense Event (p. 250)	Function	HCM-FN-track-dispense-event
Service: Dispense Event Ingress (p. 250)	Service	HCM-SV-dispense-event-ingress
Function: Receive Medication Administration Event (p. 250)	Function	HCM-FN-receive-medication-administration-event
Service: Medication Administration Ingress (p. 251)	Service	HCM-SV-medication-administration-ingress

3.2.7.1. Feature: Medication Order Ingestion

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-SA-18	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Medication Administration (p. 246)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:318 (p. 570)	Linked RAD detail; exact locators in manifest.

Feature Overview

Medication order ingestion preserves the intended therapy expressed by inpatient or discharge prescriptions. The received order is evidence of clinical intent, distinct from pharmacy dispensing and the patient's subsequent administration record.

Definition

Ingest inpatient and discharge prescription orders.

Established Behavioural Context

Element	Relationship to this Feature
Function: Receive Medication Order (p. 249)	realises feature
Service: Medication Order Ingress (p. 249)	associated feature

3.2.7.2. Feature: Dispense Event Tracking

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SA-19`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Medication Administration (p. 246)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:319 (p. 570)	Linked RAD detail; exact locators in manifest.

Feature Overview

Dispense tracking receives the pharmacy's evidence that medication was supplied under the applicable context. It connects supply with the medication account without inferring that the patient received a dose or that Harmonia performed the professional dispensing activity.

Definition

Ingest medication dispense verification records from pharmacy management systems.

Established Behavioural Context

Element	Relationship to this Feature
Function: Track Dispense Event (p. 250)	realises feature
Service: Dispense Event Ingress (p. 250)	associated feature

3.2.7.3. Feature: Medication Administration Record (MAR) Ingestion

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SA-20`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Medication Administration (p. 246)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:320 (p. 570)	Linked RAD detail; exact locators in manifest.

Feature Overview

MAR ingestion captures source-reported point-of-care administration events. It preserves the crucial distinction between intended or supplied medication and an administration recorded as performed, without originating that bedside event or its clinical authority.

Definition

Ingest point-of-care medication administration events.

Established Behavioural Context

Element	Relationship to this Feature
Function: Receive Medication Administration Event (p. 250)	realises feature
Service: Medication Administration Ingress (p. 251)	associated feature

3.2.7.4. Function: Receive Medication Order

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-receive-medication-order`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Medication Administration (p. 246)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sa:136 (p. 643)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Medication Order Ingestion (p. 247)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Ingests inpatient and outpatient drug prescriptions.

3.2.7.5. Service: Medication Order Ingress

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-medication-order-ingress`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Medication Administration (p. 246)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sa:137 (p. 643)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Medication Order Ingestion (p. 247)	Established association only.

Governed Description and Qualifications

Ingests prescription events.

3.2.7.6. Function: Track Dispense Event

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-track-dispense-event`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Medication Administration (p. 246)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sa:139 (p. 643)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Dispense Event Tracking (p. 248)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Ingests community and hospital pharmacy dispensing records.

3.2.7.7. Service: Dispense Event Ingress

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-dispense-event-ingress`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Medication Administration (p. 246)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sa:140 (p. 643)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Dispense Event Tracking (p. 248)	Established association only.

Governed Description and Qualifications

Ingests pharmacy supply notifications.

3.2.7.8. Function: Receive Medication Administration Event

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-receive-medication-administration-event`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Medication Administration (p. 246)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sa:142 (p. 643)	Linked RAD detail; exact locators in manifest.

Name	Value	Comment / Description
Derived From	Feature: Medication Administration Record (MAR) Ingestion (p. 248)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Records medication administration-event information, including nurse-administered doses, e-MAR documentation and infusion telemetry as non-exhaustive examples; scope is not limited to nursing, bedside or inpatient settings.

3.2.7.9. Service: Medication Administration Ingress

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-medication-administration-ingress`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Medication Administration (p. 246)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sa:143 (p. 643)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Medication Administration Record (MAR) Ingestion (p. 248)	Established association only.

Governed Description and Qualifications

Ingests administration records.

3.2.8. BEC: Procedure Administration

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-procedure-administration`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Harmonia-Relevant	Independent of status; never inherited.
Source	enabling:248 (p. 568); enabling:322 (p. 570); sa:149 (p. 644)	Linked RAD detail; exact locators in manifest.

Capability Overview

Procedure Administration integrates procedural bookings and the documentation of procedures performed. It keeps the scheduled case distinct from operative, anaesthetic and post-procedure information received later. That separation supports continuity across scheduling and clinical documentation without inferring that a booking establishes performance.

Boundary

Administrative integration neither performs the procedure nor originates its professional findings.

Harmonia Architectural Relevance

Correlated bookings and procedural documents support cross-system care context while the surgical or procedural service retains clinical authority.

Scope

Contained Features and Established Owned Behaviour

Element	Type	Reference Id
Feature: Procedure Booking Ingestion (p. 252)	Feature	`FEAT-SA-21`
Feature: Procedural Documentation Ingestion (p. 253)	Feature	`FEAT-SA-22`
Function: Receive Procedure Booking / Request (p. 253)	Function	`HCM-FN-receive-procedure-booking-request`
Service: Procedure Booking Ingress (p. 254)	Service	`HCM-SV-procedure-booking-ingress`
Function: Receive Procedural Documentation (p. 254)	Function	`HCM-FN-receive-procedural-documentation`
Service: Procedural Documentation Ingress (p. 255)	Service	`HCM-SV-procedural-documentation-ingress`

3.2.8.1. Feature: Procedure Booking Ingestion

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SA-21`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Procedure Administration (p. 251)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:324 (p. 570)	Linked RAD detail; exact locators in manifest.

Feature Overview

Procedure booking ingestion makes scheduled cases and procedural requests available in the wider care context. It conveys intended or arranged activity while retaining the distinction between a booked procedure and one actually performed.

Definition

Ingest scheduled surgical case bookings and procedural requests.

Established Behavioural Context

Element	Relationship to this Feature
Function: Receive Procedure Booking / Request (p. 253)	realises feature
Service: Procedure Booking Ingress (p. 254)	associated feature

3.2.8.2. Feature: Procedural Documentation Ingestion

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SA-22`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Procedure Administration (p. 251)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:325 (p. 570)	Linked RAD detail; exact locators in manifest.

Feature Overview

Procedural documentation ingestion preserves the operative, anaesthetic and post-procedure account supplied by the clinical service. It connects performed-care documentation with the wider record without giving the integration process authority to originate or judge the clinical findings.

Definition

Ingest operative reports, anesthesia records, and post-procedure summaries.

Established Behavioural Context

Element	Relationship to this Feature
Function: Receive Procedural Documentation (p. 254)	realises feature
Service: Procedural Documentation Ingress (p. 255)	associated feature

3.2.8.3. Function: Receive Procedure Booking / Request

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-receive-procedure-booking-request`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Procedure Administration (p. 251)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sa:153 (p. 644)	Linked RAD detail; exact locators in manifest.

Name	Value	Comment / Description
Derived From	Feature: Procedure Booking Ingestion (p. 252)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Ingests procedural and surgical requisitions.

3.2.8.4. Service: Procedure Booking Ingress

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-procedure-booking-ingress`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Procedure Administration (p. 251)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sa:154 (p. 644)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Procedure Booking Ingestion (p. 252)	Established association only.

Governed Description and Qualifications

Receives surgical/procedural requests.

3.2.8.5. Function: Receive Procedural Documentation

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-receive-procedural-documentation`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Procedure Administration (p. 251)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sa:156 (p. 644)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Procedural Documentation Ingestion (p. 253)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Ingests operation reports, anaesthetic records, and post-procedure notes.

3.2.8.6. Service: Procedural Documentation Ingress

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-procedural-documentation-ingress`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Procedure Administration (p. 251)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sa:157 (p. 644)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Procedural Documentation Ingestion (p. 253)	Established association only.

Governed Description and Qualifications

Ingests procedural documentation, including surgical notes, into patient record streams.

3.2.9. BEC: Clinical Record Administration

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-clinical-record-administration`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Harmonia-Core	Independent of status; never inherited.
Source	enabling:249 (p. 568); enabling:327 (p. 570); sa:163 (p. 644)	Linked RAD detail; exact locators in manifest.

Capability Overview

Clinical Record Administration manages clinical documents as governed information with authorship, classification, versions and supersession history. It distinguishes a new document from an amendment or withdrawal and maintains metadata for discovery. Its purpose is to preserve an intelligible documentary record across change, rather than merely accumulate files.

Boundary

This responsibility owns document governance; exchange of documents does not transfer that ownership to the transporting capability. Preservation does not originate the clinical facts.

Harmonia Architectural Relevance

Non-destructive document lifecycle and governed discovery are direct platform information-management responsibilities.

Scope

Contained Features and Established Owned Behaviour

Element	Type	Reference Id
Feature: Clinical Document Ingestion (p. 256)	Feature	FEAT-SA-23
Feature: Document Versioning & Supersession (p. 257)	Feature	FEAT-SA-24
Feature: Document Metadata Indexing (p. 257)	Feature	FEAT-SA-25
Function: Receive Clinical Document (p. 258)	Function	HCM-FN-receive-clinical-document
Service: Clinical Document Ingress (p. 258)	Service	HCM-SV-clinical-document-ingress
Function: Govern Clinical Document Lifecycle (p. 259)	Function	HCM-FN-govern-clinical-document-lifecycle
Service: Clinical Document Lifecycle Service (p. 259)	Service	HCM-SV-clinical-document-lifecycle-service
Function: Index Clinical Document Metadata (p. 260)	Function	HCM-FN-index-clinical-document-metadata
Service: Document Metadata Registry Query (p. 260)	Service	HCM-SV-document-metadata-registry-query
Process: Clinical Document Lifecycle Process (p. 260)	Process	HCM-PR-clinical-document-lifecycle-process

3.2.9.1. Feature: Clinical Document Ingestion

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-SA-23	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Clinical Record Administration (p. 255)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:329 (p. 571)	Linked RAD detail; exact locators in manifest.

Feature Overview

Clinical document ingestion receives authored documents into governed information management. It supports both structured and unstructured contributions while preserving that receipt establishes a managed contribution, not independent clinical verification of every assertion in the document.

Definition

Ingest structured and unstructured clinical documents (discharge summaries, consult notes, letters).

Established Behavioural Context

Element	Relationship to this Feature
Function: Receive Clinical Document (p. 258)	realises feature

Element	Relationship to this Feature
Service: Clinical Document Ingress (p. 258)	associated feature

3.2.9.2. Feature: Document Versioning & Supersession

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-SA-24	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Clinical Record Administration (p. 255)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:330 (p. 571)	Linked RAD detail; exact locators in manifest.

Feature Overview

Versioning and supersession make document change understandable without destructive loss of the previous account. Author and amendment attribution distinguish who established each contribution; withdrawal or correction changes current standing without retrospectively erasing what Harmonia previously received or managed.

Definition

Manage document classification, author attribution, amendment attribution, versioning, and supersession or withdrawal without destructive data loss.

Established Behavioural Context

Element	Relationship to this Feature
Function: Govern Clinical Document Lifecycle (p. 259)	realises feature
Service: Clinical Document Lifecycle Service (p. 259)	associated feature

3.2.9.3. Feature: Document Metadata Indexing

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-SA-25	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Clinical Record Administration (p. 255)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:331 (p. 571)	Linked RAD detail; exact locators in manifest.

Feature Overview

Metadata indexing makes documents discoverable by clinically and operationally relevant attributes. It describes content for search without making the index the document itself or granting access merely because a matching record can be located.

Definition

Index clinical document metadata (author, specialty, encounter, date, confidentiality) for rapid discovery.

Established Behavioural Context

Element	Relationship to this Feature
Function: Index Clinical Document Metadata (p. 260)	realises feature
Service: Document Metadata Registry Query (p. 260)	associated feature

3.2.9.4. Function: Receive Clinical Document

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-receive-clinical-document`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Clinical Record Administration (p. 255)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sa:168 (p. 644)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Clinical Document Ingestion (p. 256)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Receives clinical documents from information suppliers.

3.2.9.5. Service: Clinical Document Ingress

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-clinical-document-ingress`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Clinical Record Administration (p. 255)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sa:169 (p. 644)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Clinical Document Ingestion (p. 256)	Established association only.

Governed Description and Qualifications

Exposes document submission endpoints across the HIE.

3.2.9.6. Function: Govern Clinical Document Lifecycle

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-govern-clinical-document-lifecycle`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Clinical Record Administration (p. 255)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sa:171 (p. 644)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Document Versioning & Supersession (p. 257)	Established Feature realisation; not structural containment.
Governed Process	Process: Clinical Document Lifecycle Process (p. 260)	Established progression relationship.

Governed Description and Qualifications

Manages document states (**scoped illustration, not a universal transition model**): *Draft* → *Preliminary* → *Final* → *Amended* → *Superseded* → *Entered-in-Error*.

3.2.9.7. Service: Clinical Document Lifecycle Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-clinical-document-lifecycle-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Clinical Record Administration (p. 255)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sa:172 (p. 644)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Document Versioning & Supersession (p. 257)	Established association only.

Governed Description and Qualifications

Allows authoring clinicians to publish addenda or corrections.

3.2.9.8. Function: Index Clinical Document Metadata

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-index-clinical-document-metadata`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Clinical Record Administration (p. 255)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sa:176 (p. 644)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Document Metadata Indexing (p. 257)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Indexes author, specialty, encounter, date, and document type (LOINC/SNOMED).

3.2.9.9. Service: Document Metadata Registry Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-document-metadata-registry-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Clinical Record Administration (p. 255)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sa:177 (p. 644)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Document Metadata Indexing (p. 257)	Established association only.

Governed Description and Qualifications

Discloses document registry metadata to clinical viewers.

3.2.9.10. Process: Clinical Document Lifecycle Process

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-PR-clinical-document-lifecycle-process`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Clinical Record Administration (p. 255)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	processes:152 (p. 616); processes:153 (p. 617)	Linked RAD detail; exact locators in manifest.

Name	Value	Comment / Description
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Process Overview

This Process makes authorship and changing document standing intelligible through the locally described review, amendment and withdrawal circumstances. It preserves prior versions rather than treating correction as erasure. The illustration does not require every document to pass through every stage or make finalisation, signing, legal qualification and originating authority equivalent.

Governed Description and Qualifications

- **Owning Capability**: `ClinicalRecordAdministration`
- **Process Purpose**: Governs the versioning, clinical sign-off, addenda, superseding, and legal status of clinical documents (discharge summaries, specialist letters, advance care directives).
- **State Progression Lifecycle**: Draft → Preliminary → Final Signed → Amended / Addended → Superseded → Entered-in-Error (Scoped illustration; no universal document lifecycle)
- **Key State Dispositions**:
  - `Draft`: Incomplete clinical document saved during consultation.
  - `Preliminary`: Document authored pending senior registrar or consultant countersignature.
  - `FinalSigned`: Authoritative clinical document legally signed and published.
  - `Amended/Addended`: Governed supplementary clinical addendum appended to signed document.
  - `Superseded`: Entire document replaced by a newer revision; prior version retained for audit.
  - `Entered-in-Error`: Document formally retracted due to wrong patient or invalid content; content struck through with retraction notice.
- **Scope qualification**: `Final≠automaticallyFinalSigned`. The described signing/countersigning and legal qualifications remain local requirements, not a universal document lifecycle. Authorship, attestation, approval, authentication, verification, signature, finalisation, authority and legal qualification remain distinct. Amendment, supersession and entered-in-error are not mandatory stages for every document, and consumer processing does not establish originating authority or legal status.

3.2.10. BEC: Care Plan Administration

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-care-plan-administration`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Harmonia-Relevant	Independent of status; never inherited.
Source	enabling:250 (p. 568); enabling:333 (p. 571); sa:183 (p. 645)	Linked RAD detail; exact locators in manifest.

Capability Overview

Care Plan Administration receives care plans and coordinates the distribution of updates to authorised team members. It supports shared understanding of established clinical intent across participating organisations. The responsibility concerns the plan as managed information, distinct from deciding the intended course of care or performing the actions expressed by it.

Boundary

Synchronising a plan does not author its clinical intent, approve treatment or prove planned care occurred.

Harmonia Architectural Relevance

Timely propagation of multidisciplinary plan information materially supports continuity without transferring clinical planning authority.

Scope

Contained Features and Established Owned Behaviour

Element	Type	Reference Id
Feature: Care Plan Ingestion & Distribution (p. 262)	Feature	FEAT-SA-26
Function: Receive Care Plan (p. 263)	Function	HCM-FN-receive-care-plan
Service: Care Plan Ingress (p. 263)	Service	HCM-SV-care-plan-ingress
Function: Associate Care Plan Context (p. 264)	Function	HCM-FN-associate-care-plan-context
Service: Care Plan Context Query (p. 264)	Service	HCM-SV-care-plan-context-query
Function: Distribute Care Plan Change (p. 264)	Function	HCM-FN-distribute-care-plan-change
Service: Care Plan Change Notification (p. 265)	Service	HCM-SV-care-plan-change-notification

3.2.10.1. Feature: Care Plan Ingestion & Distribution

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-SA-26	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Care Plan Administration (p. 261)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:335 (p. 571)	Linked RAD detail; exact locators in manifest.

Feature Overview

Care-plan exchange keeps authorised team members aware of established multidisciplinary intent and its updates. It provides information continuity while preserving clinical planning authority and the distinction between a distributed plan and care actually performed.

Definition

Ingest multidisciplinary care plans and syndicate updates to authorized care team members.

Established Behavioural Context

Element	Relationship to this Feature
Function: Receive Care Plan (p. 263)	realises feature
Service: Care Plan Ingress (p. 263)	associated feature

3.2.10.2. Function: Receive Care Plan

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-receive-care-plan`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Care Plan Administration (p. 261)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sa:186 (p. 645)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Care Plan Ingestion & Distribution (p. 262)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Ingests multidisciplinary care plans, clinical goals, and patient action steps.

3.2.10.3. Service: Care Plan Ingress

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-care-plan-ingress`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Care Plan Administration (p. 261)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sa:187 (p. 645)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Care Plan Ingestion & Distribution (p. 262)	Established association only.

Governed Description and Qualifications

Ingests shared care plans.

3.2.10.4. Function: Associate Care Plan Context

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-associate-care-plan-context`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Care Plan Administration (p. 261)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sa:189 (p. 645)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Governed Description and Qualifications

Links care plans to active diagnoses, health problems, and care team members.

3.2.10.5. Service: Care Plan Context Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-care-plan-context-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Care Plan Administration (p. 261)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sa:190 (p. 645)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Governed Description and Qualifications

Discloses active care plans to treating clinicians.

3.2.10.6. Function: Distribute Care Plan Change

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-distribute-care-plan-change`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Care Plan Administration (p. 261)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.

Name	Value	Comment / Description
Source	sa:192 (p. 645)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Governed Description and Qualifications

Broadcasts care plan updates to all enrolled care team participants.

3.2.10.7. Service: Care Plan Change Notification

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-care-plan-change-notification`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Care Plan Administration (p. 261)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sa:193 (p. 645)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Governed Description and Qualifications

Emits care plan revision notices.

3.2.11. BEC: Clinical Communication Administration

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-clinical-communication-administration`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Harmonia-Core	Independent of status; never inherited.
Source	enabling:251 (p. 568); enabling:337 (p. 571); sa:199 (p. 645)	Linked RAD detail; exact locators in manifest.

Capability Overview

This capability manages secure clinical communication transactions, delivery and receipt evidence, and the required enduring audit trail. Verified provider and team endpoints support

appropriate addressing while transaction correlation makes the communication history intelligible. The responsibility preserves separate meanings for transport delivery, recipient acknowledgement and clinical or business disposition.

Boundary

Delivery or read receipts do not prove clinical review, incorporation, approval or acceptance. Audit evidence must remain governed without unmasked PHI in diagnostic streams.

Harmonia Architectural Relevance

Secure addressed communication and attributable transaction evidence are substantive platform responsibilities.

Scope

Contained Features and Established Owned Behaviour

Element	Type	Reference Id
Feature: Clinical Communication Distribution (p. 266)	Feature	`FEAT-SA-27`
Feature: Delivery Acknowledgement Tracking (p. 267)	Feature	`FEAT-SA-28`
Feature: Communication Audit Logging (p. 268)	Feature	`FEAT-SA-29`
Function: Distribute Clinical Communication (p. 268)	Function	`HCM-FN-distribute-clinical-communication`
Service: Secure Clinical Message Dispatch (p. 269)	Service	`HCM-SV-secure-clinical-message-dispatch`
Function: Track Business Delivery Acknowledgement (p. 269)	Function	`HCM-FN-track-business-delivery-acknowledgement`
Service: Communication Delivery Status Query (p. 269)	Service	`HCM-SV-communication-delivery-status-query`
Function: Record Communication Transaction (p. 270)	Function	`HCM-FN-record-communication-transaction`
Service: Communication Audit Query (p. 270)	Service	`HCM-SV-communication-audit-query`

3.2.11.1. Feature: Clinical Communication Distribution

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SA-27`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Clinical Communication Administration (p. 265)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:339 (p. 571)	Linked RAD detail; exact locators in manifest.

Feature Overview

Clinical communication distribution connects verified providers and designated team endpoints through governed addressed delivery. The responsibility concerns reaching the intended par-

ticipant, rather than assuming that delivered information has been read, incorporated or acted upon clinically.

Definition

Distribute secure clinical communications between verified healthcare providers and designated care team endpoints.

Established Behavioural Context

Element	Relationship to this Feature
Function: Distribute Clinical Communication (p. 268)	realises feature
Service: Secure Clinical Message Dispatch (p. 269)	associated feature

3.2.11.2. Feature: Delivery Acknowledgement Tracking

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-SA-28	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Clinical Communication Administration (p. 265)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:340 (p. 571)	Linked RAD detail; exact locators in manifest.

Feature Overview

Acknowledgement tracking associates delivery and read evidence with the communication transaction. Each receipt retains its contractual meaning; neither delivery nor reading alone proves business acceptance, clinical review, approval, finality or originating authority.

Definition

Correlate recipient delivery and read receipts with outbound clinical communications, distinguishing transport delivery from recipient acknowledgement. Receipt/read evidence does not by itself establish business acceptance, clinical review, incorporation, approval, finality or originating authority.

Established Behavioural Context

Element	Relationship to this Feature
Function: Track Business Delivery Acknowledgement (p. 269)	realises feature
Service: Communication Delivery Status Query (p. 269)	associated feature

3.2.11.3. Feature: Communication Audit Logging

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SA-29`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Clinical Communication Administration (p. 265)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:341 (p. 571)	Linked RAD detail; exact locators in manifest.

Feature Overview

Communication audit evidence preserves the attributable transaction history required for scrutiny of clinical communication. It records meaningful communication facts under the zero-PHI constraints rather than turning the audit trail into a copy of message content or a permanent diagnostic trace.

Definition

Maintain an enduring, non-PHI, tamper-evident log of all clinical communication transactions.

Established Behavioural Context

Element	Relationship to this Feature
Function: Record Communication Transaction (p. 270)	realises feature
Service: Communication Audit Query (p. 270)	associated feature

3.2.11.4. Function: Distribute Clinical Communication

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-distribute-clinical-communication`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Clinical Communication Administration (p. 265)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sa:206 (p. 645)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Clinical Communication Distribution (p. 266)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Routes secure messages, referral letters, and critical alerts to practitioner endpoints.

3.2.11.5. Service: Secure Clinical Message Dispatch

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-secure-clinical-message-dispatch`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Clinical Communication Administration (p. 265)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sa:207 (p. 645)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Clinical Communication Distribution (p. 266)	Established association only.

Governed Description and Qualifications

Exposes message dispatch to clinical authors.

3.2.11.6. Function: Track Business Delivery Acknowledgement

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-track-business-delivery-acknowledgement`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Clinical Communication Administration (p. 265)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sa:209 (p. 645)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Delivery Acknowledgement Tracking (p. 267)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Tracks end-to-end delivery state and records business-level delivery/rejection acknowledgements.

3.2.11.7. Service: Communication Delivery Status Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-communication-delivery-status-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Clinical Communication Administration (p. 265)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.

Name	Value	Comment / Description
Source	sa:211 (p. 645)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Delivery Acknowledgement Tracking (p. 267)	Established association only.

Governed Description and Qualifications

Discloses delivery confirmation to message senders.

3.2.11.8. Function: Record Communication Transaction

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-record-communication-transaction`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Clinical Communication Administration (p. 265)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sa:213 (p. 645)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Communication Audit Logging (p. 268)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Maintains an immutable audit trail of clinical message dispatch and receipt.

3.2.11.9. Service: Communication Audit Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-communication-audit-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Clinical Communication Administration (p. 265)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sa:214 (p. 645)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Communication Audit Logging (p. 268)	Established association only.

Governed Description and Qualifications

Discloses transmission history for medico-legal verification.

3.2.12. BEC: Funding & Coverage Administration

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-funding-coverage-administration`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Adjacent	Independent of status; never inherited.
Source	enabling:252 (p. 568); enabling:343 (p. 571)	Linked RAD detail; exact locators in manifest.

Capability Overview

Funding & Coverage Administration represents externally supplied insurance coverage and eligibility flags needed as care context. It allows those administrative conditions to be available without implying that Harmonia determines entitlements. The established Adjacent standing and absence of Features bound the current responsibility to this information contribution.

Boundary

Coverage information is not claims adjudication, funding governance or a universal determination of care eligibility.

Harmonia Architectural Relevance

Harmonia consumes the administrative context; it does not acquire the payer's authority or manage the underlying coverage arrangement.

3.2.13. BEC: Clinical Charging & Claims Administration

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-clinical-charging-claims-administration`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Adjacent	Independent of status; never inherited.
Source	enabling:253 (p. 568); enabling:346 (p. 571)	Linked RAD detail; exact locators in manifest.

Capability Overview

This capability supplies clinical activity summaries and charging records to billing contexts. It preserves a route from activity information to the systems responsible for financial processing

without treating the export as a completed bill or claim. No Feature decomposition or comprehensive revenue-cycle responsibility is established.

Boundary

Export of activity evidence does not confer pricing, billing, claim or accounting authority.

Harmonia Architectural Relevance

Its Adjacent standing reflects information supplied to financial systems while the financial activity remains externally governed.

3.3. Service Delivery

Capability	Reference Id
BEC: Clinical Assessment (p. 272)	`HCM-BEC-clinical-assessment`
BEC: Primary Care (p. 273)	`HCM-BEC-primary-care`
BEC: Acute Care (p. 277)	`HCM-BEC-acute-care`
BEC: Emergency Care (p. 281)	`HCM-BEC-emergency-care`
BEC: Inpatient Care (p. 285)	`HCM-BEC-inpatient-care`
BEC: Ambulatory Care (p. 289)	`HCM-BEC-ambulatory-care`
BEC: Surgical & Procedural Care (p. 290)	`HCM-BEC-surgical-procedural-care`
BEC: Diagnostic Services (p. 290)	`HCM-BEC-diagnostic-services`
BEC: Therapeutic Services (p. 294)	`HCM-BEC-therapeutic-services`
BEC: Medication Therapy (p. 295)	`HCM-BEC-medication-therapy`
BEC: Allied Health (p. 299)	`HCM-BEC-allied-health`
BEC: Preventive Care (p. 299)	`HCM-BEC-preventive-care`
BEC: Rehabilitation (p. 303)	`HCM-BEC-rehabilitation`
BEC: Community Care (p. 304)	`HCM-BEC-community-care`
BEC: Outreach Care (p. 308)	`HCM-BEC-outreach-care`
BEC: Remote & Virtual Care (p. 311)	`HCM-BEC-remote-virtual-care`

3.3.1. BEC: Clinical Assessment

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-clinical-assessment`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Adjacent	Independent of status; never inherited.
Source	enabling:361 (p. 571)	Linked RAD detail; exact locators in manifest.

Capability Overview

Clinical Assessment is retained as an Adjacent Service Delivery context. It identifies the setting of assessment-related information needs without establishing a specialised Harmonia Feature set or replacing the Business Capability Individual Health Assessment. The distinction allows the catalogue to recognise clinical activity while keeping actual examination, interpretation and professional judgement outside the platform's assigned practice.

Boundary

No atomic Features are established. The contextual name does not make Harmonia a clinical assessor or establish BC ancestry.

Harmonia Architectural Relevance

The Adjacent classification denotes contextual information participation rather than material execution of clinical assessment.

3.3.2. BEC: Primary Care

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-primary-care`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Harmonia-Relevant	Independent of status; never inherited.
Source	enabling:362 (p. 571); enabling:389 (p. 572); sd:51 (p. 648)	Linked RAD detail; exact locators in manifest.

Capability Overview

Primary Care enables information continuity between general practice and other care settings through shared-record ingestion and notifications of acute events. A GP's notes, summaries and immunisation records can contribute to the longitudinal view, while admission and discharge information can return to the registered practitioner. The enabling responsibility is the connection between contexts, rather than practice management or primary clinical care.

Boundary

Receiving a summary or dispatching a notification does not perform consultation, clinical review or follow-up.

Harmonia Architectural Relevance

These flows materially support continuity across primary and acute care without assigning general-practice authority to Harmonia.

Scope

Contained Features and Established Owned Behaviour

Element	Type	Reference Id
Feature: General Practice Shared Record Ingestion (p. 274)	Feature	FEAT-SD-01
Feature: Primary Care Notification Dispatch (p. 275)	Feature	FEAT-SD-02
Function: Correlate Primary Care Consultation Context (p. 275)	Function	HCM-FN-correlate-primary-care-consultation-context
Service: Primary Care Summary Ingress (p. 276)	Service	HCM-SV-primary-care-summary-ingress
Function: Coordinate Multi-Agency Chronic Care Plan (p. 276)	Function	HCM-FN-coordinate-multi-agency-chronic-care-plan
Service: Chronic Care Timeline Query (p. 276)	Service	HCM-SV-chronic-care-timeline-query

3.3.2.1. Feature: General Practice Shared Record Ingestion

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-SD-01	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Primary Care (p. 273)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:391 (p. 572)	Linked RAD detail; exact locators in manifest.

Feature Overview

Shared-record ingestion brings general-practice contributions into cross-provider continuity. The summary or encounter note retains its clinical source while becoming available alongside regional information; consolidation does not transfer the GP's professional authority to the platform.

Definition

Ingest shared health summaries, encounter notes, and immunization records from primary care practice management systems.

Established Behavioural Context

Element	Relationship to this Feature
Function: Correlate Primary Care Consultation Context (p. 275)	realises feature
Service: Primary Care Summary Ingress (p. 276)	associated feature

3.3.2.2. Feature: Primary Care Notification Dispatch

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SD-02`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Primary Care (p. 273)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:392 (p. 572)	Linked RAD detail; exact locators in manifest.

Feature Overview

Primary-care notifications inform the registered GP of significant acute-care events. This outward notification responsibility is distinct from querying a chronic-care timeline: the latter shares context but has no established equivalence to dispatching the required arrival, admission or discharge notice.

Definition

Dispatch emergency department arrival, acute admission, and discharge notifications to the patient's registered general practitioner.

3.3.2.3. Function: Correlate Primary Care Consultation Context

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-correlate-primary-care-consultation-context`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Primary Care (p. 273)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sd:55 (p. 648)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: General Practice Shared Record Ingestion (p. 274)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Binds GP consultation notes, vital signs, and encounter episodes to the regional Longitudinal Health Record (LHR).

3.3.2.4. Service: Primary Care Summary Ingress

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-primary-care-summary-ingress`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Primary Care (p. 273)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sd:56 (p. 648)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: General Practice Shared Record Ingestion (p. 274)	Established association only.

Governed Description and Qualifications

Ingests GP event summaries into the HIE.

3.3.2.5. Function: Coordinate Multi-Agency Chronic Care Plan

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-coordinate-multi-agency-chronic-care-plan`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Primary Care (p. 273)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sd:59 (p. 648)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Governed Description and Qualifications

Enables GPs to track multi-provider disease management milestones (e.g., Diabetes or Cardiovascular Care Plans).

Feature association not established: Strategy `FEAT-SD-02—PrimaryCareNotificationDispatch` remains valid, but semantic evidence does not establish that the following Function or Service belongs to, realises, specialises or is equivalent to it. No replacement Feature is assigned.

3.3.2.6. Service: Chronic Care Timeline Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-chronic-care-timeline-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.

Name	Value	Comment / Description
Owning Context	BEC: Primary Care (p. 273)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sd:60 (p. 648)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Governed Description and Qualifications

Exposes aggregated multidisciplinary care timelines.

Feature association not established: Strategy `FEAT-SD-02_PrimaryCareNotificationDispatch` remains valid, but semantic evidence does not establish that the following Function or Service belongs to, realises, specialises or is equivalent to it. No replacement Feature is assigned.

3.3.3. BEC: Acute Care

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-acute-care`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Harmonia-Relevant	Independent of status; never inherited.
Source	enabling:363 (p. 571); enabling:394 (p. 572); sd:71 (p. 649)	Linked RAD detail; exact locators in manifest.

Capability Overview

Acute Care provides the information context needed around acute clinical events, including externally reported deterioration and timely access to history and active medications. It helps current events be interpreted alongside prior information rather than as isolated messages. Its Service Delivery context does not become a child of Individual Care Delivery's setting-independent Business Capability hierarchy.

Boundary

Capturing a deterioration event does not determine clinical significance or prescribe an acute response.

Harmonia Architectural Relevance

Harmonia materially supplies and correlates acute-care information while clinical decisions and intervention remain with the care service.

Scope

Contained Features and Established Owned Behaviour

Element	Type	Reference Id
Feature: Acute Clinical State Event Capture (p. 278)	Feature	FEAT-SD-03
Feature: Acute Care Clinical Context Provision (p. 278)	Feature	FEAT-SD-04
Function: Assemble Acute Clinical Baseline (p. 279)	Function	HCM-FN-assemble-acute-clinical-baseline
Service: Acute Admission Dossier Query (p. 280)	Service	HCM-SV-acute-admission-dossier-query
Function: Route Acute Critical Result Alert (p. 280)	Function	HCM-FN-route-acute-critical-result-alert
Service: Acute Clinical Alert Dispatch (p. 280)	Service	HCM-SV-acute-clinical-alert-dispatch

3.3.3.1. Feature: Acute Clinical State Event Capture

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-SD-03	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Acute Care (p. 277)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:396 (p. 572)	Linked RAD detail; exact locators in manifest.

Feature Overview

Acute-state capture records deterioration notifications and current clinical status as externally established information. Approved reusable clinical-information composition is sufficient for this responsibility; routing a critical-result alert does not become a specialised capture Function merely because both concern urgent care.

Definition

Ingest clinical deterioration notifications and acute clinical status updates.

3.3.3.2. Feature: Acute Care Clinical Context Provision

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-SD-04	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Acute Care (p. 277)	Established responsibility only; complete CT ancestry Unresolved.

Name	Value	Comment / Description
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:397 (p. 572)	Linked RAD detail; exact locators in manifest.

Feature Overview

Acute context provision brings relevant history and active medications together for clinicians facing a current presentation. It supports rapid understanding without substituting the consolidated view for clinical assessment, triage or a decision about treatment.

Definition

Provide consolidated longitudinal history and active medication lists to acute care clinicians at point of triage.

Established Behavioural Context

Element	Relationship to this Feature
Function: Assemble Acute Clinical Baseline (p. 279)	realises feature
Service: Acute Admission Dossier Query (p. 280)	associated feature

3.3.3.3. Function: Assemble Acute Clinical Baseline

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-assemble-acute-clinical-baseline`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Acute Care (p. 277)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sd:75 (p. 649)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Acute Care Clinical Context Provision (p. 278)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Rapidly aggregates previous discharge summaries, active medications, allergies, and diagnostic alerts upon acute hospital presentation.

3.3.3.4. Service: Acute Admission Dossier Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-acute-admission-dossier-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Acute Care (p. 277)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sd:76 (p. 649)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Acute Care Clinical Context Provision (p. 278)	Established association only.

Governed Description and Qualifications

Delivers a high-priority clinical briefing to admitting teams.

3.3.3.5. Function: Route Acute Critical Result Alert

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-route-acute-critical-result-alert`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Acute Care (p. 277)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sd:79 (p. 649)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Governed Description and Qualifications

Immediately directs urgent lab/radiology alerts (e.g., critical troponin, intracranial bleed) to active acute attending clinicians.

Feature association not established: `FEAT-SD-03–AcuteClinicalStateEventCapture` is adjudicated sufficient through general clinical-information responsibilities above. Routing a critical-result alert remains distinct from capture; no specialised capture Function or replacement association is assigned.

3.3.3.6. Service: Acute Clinical Alert Dispatch

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-acute-clinical-alert-dispatch`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.

Name	Value	Comment / Description
Owning Context	BEC: Acute Care (p. 277)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sd:80 (p. 649)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Governed Description and Qualifications

Dispatches urgent interruptive alerts to on-duty ward teams.

Feature association not established: `FEAT-SD-03–AcuteClinicalStateEventCapture` is adjudicated sufficient through general clinical-information responsibilities above. Routing a critical-result alert remains distinct from capture; no specialised capture Function or replacement association is assigned.

3.3.4. BEC: Emergency Care

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-emergency-care`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Harmonia-Relevant	Independent of status; never inherited.
Source	enabling:364 (p. 571); enabling:399 (p. 572); sd:91 (p. 649)	Linked RAD detail; exact locators in manifest.

Capability Overview

Emergency Care integrates presentation and triage information and correlates it with recent admissions, summaries and known allergies. It supports a rapidly intelligible account of the patient arriving for emergency care without replacing the clinical triage decision. The context is valuable because the regional history may be distributed across participants outside the receiving department.

Boundary

Ingested triage categories remain externally established clinical information; correlation does not confer emergency decision authority.

Harmonia Architectural Relevance

Cross-facility context and current presentation information materially support emergency care continuity.

Scope

Contained Features and Established Owned Behaviour

Element	Type	Reference Id
Feature: Emergency Department Arrival & Triage Ingestion (p. 282)	Feature	FEAT-SD-05
Feature: Emergency Care Cross-Facility Correlation (p. 283)	Feature	FEAT-SD-06
Function: Bind ED Triage Assessment (p. 283)	Function	HCM-FN-bind-ed-triage-assessment
Service: ED Triage Stream Ingress (p. 284)	Service	HCM-SV-ed-triage-stream-ingress
Function: Provide Emergency LHR Summary (p. 284)	Function	HCM-FN-provide-emergency-lhr-summary
Service: Emergency Fast-Path LHR Query (p. 285)	Service	HCM-SV-emergency-fast-path-lhr-query

3.3.4.1. Feature: Emergency Department Arrival & Triage Ingestion

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-SD-05	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Emergency Care (p. 281)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:401 (p. 573)	Linked RAD detail; exact locators in manifest.

Feature Overview

Emergency presentation ingestion retains the arrival, triage category and presenting complaint reported by the emergency context. It makes that event available for regional correlation while preserving that the clinical triage judgement was established by the responsible source.

Definition

Ingest emergency presentation, triage category, and chief complaint records.

Established Behavioural Context

Element	Relationship to this Feature
Function: Bind ED Triage Assessment (p. 283)	realises feature
Service: ED Triage Stream Ingress (p. 284)	associated feature

3.3.4.2. Feature: Emergency Care Cross-Facility Correlation

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SD-06`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Emergency Care (p. 281)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:402 (p. 573)	Linked RAD detail; exact locators in manifest.

Feature Overview

Cross-facility correlation connects an emergency presentation with relevant recent history and allergy information. It supports situational understanding across institutional boundaries without deciding the clinical significance of the combined evidence or assigning emergency treatment.

Definition

Correlate incoming emergency presentations with recent regional admissions, prior discharge summaries, and known allergy profiles.

Established Behavioural Context

Element	Relationship to this Feature
Function: Provide Emergency LHR Summary (p. 284)	realises feature
Service: Emergency Fast-Path LHR Query (p. 285)	associated feature

3.3.4.3. Function: Bind ED Triage Assessment

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-bind-ed-triage-assessment`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Emergency Care (p. 281)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sd:95 (p. 649)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Emergency Department Arrival & Triage Ingestion (p. 282)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Captures ATS (Australasian Triage Scale) category, presenting complaint, and ambulance paramedic handover data.

3.3.4.4. Service: ED Triage Stream Ingress

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-ed-triage-stream-ingress`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Emergency Care (p. 281)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sd:96 (p. 649)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Emergency Department Arrival & Triage Ingestion (p. 282)	Established association only.

Governed Description and Qualifications

Ingests emergency triage presentations.

3.3.4.5. Function: Provide Emergency LHR Summary

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-provide-emergency-lhr-summary`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Emergency Care (p. 281)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sd:98 (p. 649)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Emergency Care Cross-Facility Correlation (p. 283)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Delivers rapid, override-capable access to Advance Care Directives, allergy records, and recent hospital admissions.

3.3.4.6. Service: Emergency Fast-Path LHR Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-emergency-fast-path-lhr-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Emergency Care (p. 281)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sd:99 (p. 649)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Emergency Care Cross-Facility Correlation (p. 283)	Established association only.

Governed Description and Qualifications

Provides low-latency emergency record summaries.

3.3.5. BEC: Inpatient Care

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-inpatient-care`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Harmonia-Relevant	Independent of status; never inherited.
Source	enabling:365 (p. 571); enabling:404 (p. 573); sd:110 (p. 650)	Linked RAD detail; exact locators in manifest.

Capability Overview

Inpatient Care brings together information for multidisciplinary review and captures externally established clinical milestones during an admission. Results, medication records and nursing notes support a shared contextual view, while progression events make reported care status visible. The responsibility concerns information availability and correlation rather than control of the inpatient clinical worklist.

Boundary

Clinical milestone capture does not establish the milestone's clinical authority or manage rounds and handover.

Harmonia Architectural Relevance

The platform materially supplies consolidated inpatient context; milestone tracking may be realised through approved reusable composition without an invented specialised Function.

Scope

Contained Features and Established Owned Behaviour

Element	Type	Reference Id
Feature: Inpatient Rounding Context Provision (p. 286)	Feature	FEAT-SD-07
Feature: Inpatient Clinical Progression Tracking (p. 287)	Feature	FEAT-SD-08
Function: Consolidate Inpatient Clinical Round Feed (p. 287)	Function	HCM-FN-consolidate-inpatient-clinical-round-feed
Service: Ward Round Clinical Briefing Query (p. 288)	Service	HCM-SV-ward-round-clinical-briefing-query
Function: Assemble Inpatient Discharge Dossier (p. 288)	Function	HCM-FN-assemble-inpatient-discharge-dossier
Service: Discharge Dossier Packaging Service (p. 288)	Service	HCM-SV-discharge-dossier-packaging-service

3.3.5.1. Feature: Inpatient Rounding Context Provision

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-SD-07	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Inpatient Care (p. 285)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:406 (p. 573)	Linked RAD detail; exact locators in manifest.

Feature Overview

Rounding context brings results, medication records and nursing notes into an accessible multidisciplinary view. Assembly serves the clinical discussion but does not perform rounds, determine pending clinical work or establish the team's professional conclusions.

Definition

Aggregate diagnostic results, current medication administration records, and nursing notes for multidisciplinary inpatient rounds.

Established Behavioural Context

Element	Relationship to this Feature
Function: Consolidate Inpatient Clinical Round Feed (p. 287)	realises feature
Service: Ward Round Clinical Briefing Query (p. 288)	associated feature

3.3.5.2. Feature: Inpatient Clinical Progression Tracking

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SD-08`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Inpatient Care (p. 285)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:407 (p. 573)	Linked RAD detail; exact locators in manifest.

Feature Overview

Inpatient progression tracking captures clinical milestones reported by the responsible participants. General clinical-information capabilities and configured workflow composition suffice for the established Feature; capture does not require inventing a specialised Function or make Harmonia the authority declaring clinical stability.

Definition

Capture inpatient clinical milestone completions (e.g., medical stability achieved, allied health clearance).

3.3.5.3. Function: Consolidate Inpatient Clinical Round Feed

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-consolidate-inpatient-clinical-round-feed`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Inpatient Care (p. 285)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sd:114 (p. 650)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Inpatient Rounding Context Provision (p. 286)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Aggregates daily vital signs, overnight pathology, allied health notes, and medication administration records.

3.3.5.4. Service: Ward Round Clinical Briefing Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-ward-round-clinical-briefing-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Inpatient Care (p. 285)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sd:115 (p. 650)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Inpatient Rounding Context Provision (p. 286)	Established association only.

Governed Description and Qualifications

Discloses consolidated 24-hour clinical briefing views.

3.3.5.5. Function: Assemble Inpatient Discharge Dossier

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-assemble-inpatient-discharge-dossier`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Inpatient Care (p. 285)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sd:118 (p. 650)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Governed Description and Qualifications

Synthesises inpatient diagnoses, procedural summaries, reconciled discharge medications, and follow-up instructions into a canonical discharge summary package.

Feature association not established: `FEAT-SD-08–InpatientClinicalProgressionTracking` is adjudicated sufficient through general clinical-information responsibilities above. Discharge-dossier assembly/publication remains distinct; no separate clinical-tracking Function or replacement association is assigned.

3.3.5.6. Service: Discharge Dossier Packaging Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-discharge-dossier-packaging-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.

Name	Value	Comment / Description
Owning Context	BEC: Inpatient Care (p. 285)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sd:119 (p. 650)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Governed Description and Qualifications

Emits canonical discharge summaries to GPs and community providers.

Feature association not established: `FEAT-SD-08–InpatientClinicalProgressionTracking` is adjudicated sufficient through general clinical-information responsibilities above. Discharge-dossier assembly/publication remains distinct; no separate clinical-tracking Function or replacement association is assigned.

3.3.6. BEC: Ambulatory Care

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-ambulatory-care`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Adjacent	Independent of status; never inherited.
Source	enabling:366 (p. 571)	Linked RAD detail; exact locators in manifest.

Capability Overview

Ambulatory Care is an established Adjacent delivery context retained for understanding information used around care without inpatient admission. The RAD establishes no atomic enabling Feature decomposition for it. The entry therefore locates the context without claiming a separate scheduling, treatment or clinical progression mandate for Harmonia.

Boundary

The contextual entry does not allocate BC tiers or require a distinct platform function for every ambulatory activity.

Harmonia Architectural Relevance

Adjacent standing preserves contextual participation while clinical service delivery remains external.

3.3.7. BEC: Surgical & Procedural Care

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-surgical-procedural-care`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Adjacent	Independent of status; never inherited.
Source	enabling:367 (p. 571)	Linked RAD detail; exact locators in manifest.

Capability Overview

Surgical & Procedural Care retains the clinical delivery context around procedures as Adjacent. It is distinct from the established integration of bookings and documents in Procedure Administration and from theatre operational coordination. Recognising those neighbouring responsibilities prevents information about a procedure from being read as an assignment to perform or clinically manage it.

Boundary

No atomic Features are allocated to this context, and Harmonia does not acquire surgical decision or procedural authority.

Harmonia Architectural Relevance

The context is represented without assigning direct clinical delivery responsibility.

3.3.8. BEC: Diagnostic Services

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-diagnostic-services`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Harmonia-Relevant	Independent of status; never inherited.
Source	enabling:368 (p. 571); enabling:409 (p. 573); sd:129 (p. 650)	Linked RAD detail; exact locators in manifest.

Capability Overview

Diagnostic Services supports cross-provider availability of reports and diagnostic history. Result distribution brings established findings to the ordering and designated participants; history con-

solidation makes relevant evidence visible across institutional boundaries. This delivery context complements Diagnostic Administration's request and report associations without making information integration the professional diagnostic service itself.

Boundary

Consolidating trends does not interpret diagnostic evidence or establish a patient-level clinical conclusion.

Harmonia Architectural Relevance

Governed report distribution and integrated history materially enable diagnostic information use across care boundaries.

Scope

Contained Features and Established Owned Behaviour

Element	Type	Reference Id
Feature: Laboratory & Imaging Result Distribution (p. 291)	Feature	`FEAT-SD-09`
Feature: Diagnostic History Consolidation (p. 292)	Feature	`FEAT-SD-10`
Function: Receive Diagnostic Requisition Feed (p. 292)	Function	`HCM-FN-receive-diagnostic-requisition-feed`
Service: Modality Worklist Distribution (p. 293)	Service	`HCM-SV-modality-worklist-distribution`
Function: Publish Governed Diagnostic Report (p. 293)	Function	`HCM-FN-publish-governed-diagnostic-report`
Service: Diagnostic Publication Service (p. 294)	Service	`HCM-SV-diagnostic-publication-service`

3.3.8.1. Feature: Laboratory & Imaging Result Distribution

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SD-09`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Diagnostic Services (p. 290)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:411 (p. 573)	Linked RAD detail; exact locators in manifest.

Feature Overview

Result distribution connects established diagnostic reports with ordering and designated secondary recipients. It supports continuity of diagnostic information across care settings while retaining the distinction between delivery, recipient review and the clinical decision based on the result.

Definition

Reliably distribute structured diagnostic reports (pathology, radiology) to ordering clinicians and designated secondary recipients.

3.3.8.2. Feature: Diagnostic History Consolidation

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SD-10`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Diagnostic Services (p. 290)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:412 (p. 573)	Linked RAD detail; exact locators in manifest.

Feature Overview

Diagnostic history consolidation makes prior evidence visible across regional providers. A combined historical view can support comparison over time without itself establishing a diagnostic interpretation, reconciling conflicting clinical assertions or replacing each report's source standing.

Definition

Consolidate historical diagnostic trends (e.g., cumulative laboratory results) across multiple regional providers into an integrated view.

3.3.8.3. Function: Receive Diagnostic Requisition Feed

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-receive-diagnostic-requisition-feed`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Diagnostic Services (p. 290)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sd:134 (p. 650)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Governed Description and Qualifications

Normalises order requisitions from across the health network into modality worklists.

Feature association not established: Strategy `FEAT-SD-09_Laboratory&ImagingResultDistribution` remains valid, but semantic evidence does not establish that the following Function or Service belongs to, realises, specialises or is equivalent to it. No replacement Feature is assigned.

3.3.8.4. Service: Modality Worklist Distribution

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-modality-worklist-distribution`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Diagnostic Services (p. 290)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sd:135 (p. 650)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Governed Description and Qualifications

Supplies order worklists to diagnostic laboratory/imaging systems.

Feature association not established: Strategy `FEAT-SD-09_Laboratory&ImagingResultDistribution` remains valid, but semantic evidence does not establish that the following Function or Service belongs to, realises, specialises or is equivalent to it. No replacement Feature is assigned.

3.3.8.5. Function: Publish Governed Diagnostic Report

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-publish-governed-diagnostic-report`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Diagnostic Services (p. 290)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sd:138 (p. 650)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Governed Description and Qualifications

Normalises and distributes structured diagnostic observations, reference ranges, and diagnostic image study links into the LHR.

Feature association not established: Strategy `FEAT-SD-10_DiagnosticHistoryConsolidation` remains valid, but semantic evidence does not establish that the following Function or Service belongs to, realises, specialises or is equivalent to it. No replacement Feature is assigned.

3.3.8.6. Service: Diagnostic Publication Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-diagnostic-publication-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Diagnostic Services (p. 290)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sd:139 (p. 650)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Governed Description and Qualifications

Publishes verified diagnostic reports across the enterprise.

Feature association not established: Strategy `FEAT-SD-10—DiagnosticHistoryConsolidation` remains valid, but semantic evidence does not establish that the following Function or Service belongs to, realises, specialises or is equivalent to it. No replacement Feature is assigned.

3.3.9. BEC: Therapeutic Services

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-therapeutic-services`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Adjacent	Independent of status; never inherited.
Source	enabling:369 (p. 571)	Linked RAD detail; exact locators in manifest.

Capability Overview

Therapeutic Services is retained as an Adjacent clinical delivery context, recognising care activity whose information may concern Harmonia without assigning treatment performance to it. The current model establishes no atomic Features for this context. The broad label is not used to invent therapy-specific Functions or reclassify clinical interventions as platform execution.

Boundary

Information participation does not confer authority to select, perform or assess the clinical adequacy of therapy.

Harmonia Architectural Relevance

Adjacent standing preserves the contextual role while treatment remains the responsible service's activity.

3.3.10. BEC: Medication Therapy

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-medication-therapy`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Harmonia-Relevant	Independent of status; never inherited.
Source	enabling:370 (p. 571); enabling:414 (p. 573); sd:147 (p. 651)	Linked RAD detail; exact locators in manifest.

Capability Overview

Medication Therapy connects reconciled histories and medication events across care transitions. The clinically established medication account is received and made available with order, dispensing and administration information still distinguishable. This supports continuity of therapy understanding without turning the platform into the clinician performing medication reconciliation or choosing treatment.

Boundary

Ingesting a reconciled history is not clinical reconciliation, and an order or dispense event does not prove administration.

Harmonia Architectural Relevance

Cross-boundary medication information materially supports care continuity and interpretation.

Scope

Contained Features and Established Owned Behaviour

Element	Type	Reference Id
Feature: Medication History Reconciliation Ingestion (p. 296)	Feature	`FEAT-SD-11`
Feature: Medication Administration Tracking (p. 296)	Feature	`FEAT-SD-12`
Function: Project Longitudinal Medication History (p. 297)	Function	`HCM-FN-project-longitudinal-medication-history`
Service: Longitudinal Medication History Query (p. 297)	Service	`HCM-SV-longitudinal-medication-history-query`
Function: Correlate Adverse Reaction & Allergy Profile (p. 298)	Function	`HCM-FN-correlate-adverse-reaction-allergy-profile`

Element	Type	Reference Id
Service: Allergy & Adverse Reaction Query (p. 298)	Service	`HCM-SV-allergy-adverse-reaction-query`

3.3.10.1. Feature: Medication History Reconciliation Ingestion

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SD-11`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Medication Therapy (p. 295)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:416 (p. 573)	Linked RAD detail; exact locators in manifest.

Feature Overview

This Feature receives medication histories already reconciled by clinical participants during care transitions. The platform preserves and connects that account; ingestion is distinct from performing the clinical reconciliation or independently deciding which regimen is correct.

Definition

Ingest reconciled medication histories compiled during clinical transitions of care.

Established Behavioural Context

Element	Relationship to this Feature
Function: Project Longitudinal Medication History (p. 297)	realises feature
Service: Longitudinal Medication History Query (p. 297)	associated feature

3.3.10.2. Feature: Medication Administration Tracking

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SD-12`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Medication Therapy (p. 295)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:417 (p. 573)	Linked RAD detail; exact locators in manifest.

Feature Overview

Medication tracking conveys orders, dispensing changes and administration information across care boundaries while keeping their meanings separate. This supports a coherent therapy account without treating a supplied medicine as an administered dose or a distributed order as treatment performed.

Definition

Distribute medication therapy orders, dispensing updates, and administration records across clinical care boundaries.

3.3.10.3. Function: Project Longitudinal Medication History

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-project-longitudinal-medication-history`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Medication Therapy (p. 295)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sd:151 (p. 651)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Medication History Reconciliation Ingestion (p. 296)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Aggregates community dispensing, hospital e-MAR administrations, and specialist prescriptions into a unified chronological medication profile.

3.3.10.4. Service: Longitudinal Medication History Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-longitudinal-medication-history-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Medication Therapy (p. 295)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sd:152 (p. 651)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Medication History Reconciliation Ingestion (p. 296)	Established association only.

Governed Description and Qualifications

Discloses full medication timelines for medication reconciliation.

3.3.10.5. Function: Correlate Adverse Reaction & Allergy Profile

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-correlate-adverse-reaction-allergy-profile`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Medication Therapy (p. 295)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sd:155 (p. 651)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Governed Description and Qualifications

Binds confirmed substance allergies, adverse drug reactions, and severity grades to the patient identity.

Feature association not established: Strategy `FEAT-SD-12–MedicationAdministrationTracking` remains valid, but semantic evidence does not establish that the following Function or Service belongs to, realises, specialises or is equivalent to it. No replacement Feature is assigned.

3.3.10.6. Service: Allergy & Adverse Reaction Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-allergy-adverse-reaction-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Medication Therapy (p. 295)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sd:156 (p. 651)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Governed Description and Qualifications

Exposes active allergy profiles to clinical decision support systems.

Feature association not established: Strategy `FEAT-SD-12–MedicationAdministrationTracking` remains valid, but semantic evidence does not establish that the following Function or Service belongs to, realises, specialises or is equivalent to it. No replacement Feature is assigned.

3.3.11. BEC: Allied Health

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-allied-health`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Adjacent	Independent of status; never inherited.
Source	enabling:371 (p. 571)	Linked RAD detail; exact locators in manifest.

Capability Overview

Allied Health is preserved as an Adjacent Service Delivery context without an established atomic Feature set. It acknowledges the professional care context in which information can be needed or produced, while leaving the specific therapeutic and assessment responsibilities with their clinical participants. The model does not create a generic allied-health workflow from the contextual name.

Boundary

Care coordination and record access do not transfer professional judgement or therapy delivery to Harmonia.

Harmonia Architectural Relevance

The formal Adjacent position is retained rather than inherited from neighbouring Relevant delivery contexts.

3.3.12. BEC: Preventive Care

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-preventive-care`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Harmonia-Relevant	Independent of status; never inherited.
Source	enabling:372 (p. 571); enabling:419 (p. 573); sd:164 (p. 651)	Linked RAD detail; exact locators in manifest.

Capability Overview

Preventive Care enables information flows around immunisation and screening recall. Administration records can be conveyed to national registers, while established reminders can reach

designated providers. This Service Delivery context supplies bounded information enablement; it does not replace the broader Business Capability Preventative Health or its programme and cohort responsibilities.

Boundary

Submission does not perform immunisation, and a recall notice does not determine individual clinical eligibility or complete screening.

Harmonia Architectural Relevance

Register integration and recall distribution materially connect preventative activity across participants.

Scope

Contained Features and Established Owned Behaviour

Element	Type	Reference Id
Feature: Immunisation Event Submission (p. 300)	Feature	`FEAT-SD-13`
Feature: Screening Recall Notification Distribution (p. 301)	Feature	`FEAT-SD-14`
Function: Syndicate Immunisation Record (p. 301)	Function	`HCM-FN-syndicate-immunisation-record`
Service: Immunisation History Query (p. 302)	Service	`HCM-SV-immunisation-history-query`
Function: Project Screening Eligibility Timeline (p. 302)	Function	`HCM-FN-project-screening-eligibility-timeline`
Service: Preventive Screening Status Query (p. 303)	Service	`HCM-SV-preventive-screening-status-query`

3.3.12.1. Feature: Immunisation Event Submission

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SD-13`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Preventive Care (p. 299)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:421 (p. 573)	Linked RAD detail; exact locators in manifest.

Feature Overview

Immunisation submission connects a source-recorded administration event with the national register context. It supports continuity and reporting without performing the immunisation or making successful transmission equivalent to every downstream registry outcome.

Definition

Ingest point-of-care immunisation administration records and syndicate to national immunization registers.

Established Behavioural Context

Element	Relationship to this Feature
Function: Syndicate Immunisation Record (p. 301)	realises feature
Service: Immunisation History Query (p. 302)	associated feature

3.3.12.2. Feature: Screening Recall Notification Distribution

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SD-14`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Preventive Care (p. 299)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:422 (p. 573)	Linked RAD detail; exact locators in manifest.

Feature Overview

Screening recall distribution conveys established reminders to the designated care providers. Its notification responsibility is expressible through approved event and workflow composition; the catalogue does not invent a specialised Function or transfer the clinical decision to recall the patient.

Definition

Distribute clinical screening reminders and recall notices to designated healthcare providers.

3.3.12.3. Function: Syndicate Immunisation Record

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-syndicate-immunisation-record`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Preventive Care (p. 299)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sd:168 (p. 651)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Immunisation Event Submission (p. 300)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Records vaccine encounters and syndicates records to national immunisation registers (e.g., AIR).

3.3.12.4. Service: Immunisation History Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-immunisation-history-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Preventive Care (p. 299)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sd:169 (p. 651)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Immunisation Event Submission (p. 300)	Established association only.

Governed Description and Qualifications

Discloses complete vaccination history.

3.3.12.5. Function: Project Screening Eligibility Timeline

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-project-screening-eligibility-timeline`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Preventive Care (p. 299)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sd:172 (p. 651)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Governed Description and Qualifications

Tracks age-based and risk-based screening schedules across primary care.

Feature association not established: `FEAT-SD-14–ScreeningRecallNotificationDistribution` is adjudicated sufficient through reusable composition/workflow above. Projecting/querying eligibility dates remains distinct from distribution; no dedicated recall Function or replacement association is assigned.

3.3.12.6. Service: Preventive Screening Status Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-preventive-screening-status-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Preventive Care (p. 299)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sd:173 (p. 651)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Governed Description and Qualifications

Discloses screening due dates to primary care providers.

Feature association not established: `FEAT-SD-14–ScreeningRecallNotificationDistribution` is adjudicated sufficient through reusable composition/workflow above. Projecting/querying eligibility dates remains distinct from distribution; no dedicated recall Function or replacement association is assigned.

3.3.13. BEC: Rehabilitation

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-rehabilitation`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Adjacent	Independent of status; never inherited.
Source	enabling:373 (p. 571)	Linked RAD detail; exact locators in manifest.

Capability Overview

Rehabilitation is an Adjacent clinical delivery context whose information can be represented without assigning the rehabilitation programme to Harmonia. No atomic Features are established in the current catalogue. Its presence records a healthcare context rather than requiring a new specialised coordination engine or a clinical outcome authority.

Boundary

Information integration does not perform therapy, determine rehabilitation goals or judge clinical effectiveness.

Harmonia Architectural Relevance

Contextual participation is retained within the formal Adjacent classification.

3.3.14. BEC: Community Care

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-community-care`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Harmonia-Relevant	Independent of status; never inherited.
Source	enabling:374 (p. 571); enabling:424 (p. 573); sd:183 (p. 652)	Linked RAD detail; exact locators in manifest.

Capability Overview

Community Care integrates encounters, observations and plans across home, community and hospital outreach participants. It supports continuity where clinical activity occurs outside the hospital's immediate information environment. Receiving visit records and synchronising plan changes helps make community activity visible without turning the platform into the provider of that activity.

Boundary

Plan synchronisation does not originate clinical intent, assign community work or establish that a visit occurred beyond its source evidence.

Harmonia Architectural Relevance

Cross-organisational encounter and care-plan information materially supports community care continuity.

Scope

Contained Features and Established Owned Behaviour

Element	Type	Reference Id
Feature: Community Encounter Ingestion (p. 305)	Feature	`FEAT-SD-15`
Feature: Community Care Plan Synchronization (p. 305)	Feature	`FEAT-SD-16`
Function: Synchronize Community Visit Record (p. 306)	Function	`HCM-FN-synchronize-community-visit-record`
Service: Community Care Encounter Ingress (p. 306)	Service	`HCM-SV-community-care-encounter-ingress`
Function: Bind Community Support Network Context (p. 307)	Function	`HCM-FN-bind-community-support-network-context`
Service: Community Care Team Query (p. 307)	Service	`HCM-SV-community-care-team-query`

3.3.14.1. Feature: Community Encounter Ingestion

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-SD-15	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Community Care (p. 304)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:426 (p. 573)	Linked RAD detail; exact locators in manifest.

Feature Overview

Community encounter ingestion brings home-visit and community assessments into the longitudinal information context. It supports continuity across organisational boundaries while preserving source attribution and the difference between the documentation received and care performed by the platform.

Definition

Ingest home visit notes, community nursing observations, and social support assessments.

Established Behavioural Context

Element	Relationship to this Feature
Function: Synchronize Community Visit Record (p. 306)	realises feature
Service: Community Care Encounter Ingress (p. 306)	associated feature

3.3.14.2. Feature: Community Care Plan Synchronization

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-SD-16	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Community Care (p. 304)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:427 (p. 573)	Linked RAD detail; exact locators in manifest.

Feature Overview

Community plan synchronisation keeps hospital outreach and community participants informed of established plan changes. It supports shared clinical intent without allowing synchronisation to become authority to choose that intent or allocate clinical work.

Definition

Synchronize community care plan updates between hospital outreach teams and community health organizations.

3.3.14.3. Function: Synchronize Community Visit Record

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-synchronize-community-visit-record`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Community Care (p. 304)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sd:187 (p. 652)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Community Encounter Ingestion (p. 305)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Ingests clinical notes, wound assessments, and vital signs recorded in community and home settings.

3.3.14.4. Service: Community Care Encounter Ingress

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-community-care-encounter-ingress`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Community Care (p. 304)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sd:188 (p. 652)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Community Encounter Ingestion (p. 305)	Established association only.

Governed Description and Qualifications

Ingests community care documentation.

3.3.14.5. Function: Bind Community Support Network Context

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-bind-community-support-network-context`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Community Care (p. 304)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sd:191 (p. 652)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Governed Description and Qualifications

Links community nursing teams with local non-clinical social care and home care packages.

Feature association not established: Strategy `FEAT-SD-16–CommunityCarePlanSynchronization` remains valid, but semantic evidence does not establish that the following Function or Service belongs to, realises, specialises or is equivalent to it. No replacement Feature is assigned.

3.3.14.6. Service: Community Care Team Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-community-care-team-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Community Care (p. 304)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sd:192 (p. 652)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Governed Description and Qualifications

Exposes community provider contact rosters.

Feature association not established: Strategy `FEAT-SD-16–CommunityCarePlanSynchronization` remains valid, but semantic evidence does not establish that the following Function or Service belongs to, realises, specialises or is equivalent to it. No replacement Feature is assigned.

3.3.15. BEC: Outreach Care

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-outreach-care`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Harmonia-Relevant	Independent of status; never inherited.
Source	enabling:375 (p. 571); enabling:429 (p. 573); sd:202 (p. 652)	Linked RAD detail; exact locators in manifest.

Capability Overview

Outreach Care supplies portable clinical context and receives documentation from rural or remote outreach activity. It addresses continuity where the relevant care information may not be available through the usual on-site environment. The responsibility connects the prior record with the outcomes reported after the visit while keeping source and temporal context intelligible.

Boundary

An offline-capable summary does not acquire clinical authority or guarantee that the remote copy represents current authoritative state.

Harmonia Architectural Relevance

Context provision and outcome ingestion materially connect outreach activity to the longitudinal record.

Scope

Contained Features and Established Owned Behaviour

Element	Type	Reference Id
Feature: Outreach Visit Context Provision (p. 309)	Feature	`FEAT-SD-17`
Feature: Outreach Encounter Outcome Ingestion (p. 309)	Feature	`FEAT-SD-18`
Function: Provide Outreach Visit Context (p. 310)	Function	`HCM-FN-provide-outreach-visit-context`
Function: Reconcile Outreach Clinical Batch (p. 310)	Function	`HCM-FN-reconcile-outreach-clinical-batch`
Service: Outreach Batch Reconciliation Service (p. 311)	Service	`HCM-SV-outreach-batch-reconciliation-service`

3.3.15.1. Feature: Outreach Visit Context Provision

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-SD-17	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Outreach Care (p. 308)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:431 (p. 574)	Linked RAD detail; exact locators in manifest.

Feature Overview

Outreach context provision prepares portable summaries for care beyond the usual connected environment. Their usefulness depends on retaining clinical and temporal context; an offline-capable copy is not automatically current authoritative state or a new origin of clinical facts.

Definition

Package and deliver portable clinical summaries and offline-capable care summaries for remote outreach clinics.

Established Behavioural Context

Element	Relationship to this Feature
Function: Provide Outreach Visit Context (p. 310)	realises feature

3.3.15.2. Feature: Outreach Encounter Outcome Ingestion

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-SD-18	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Outreach Care (p. 308)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:432 (p. 574)	Linked RAD detail; exact locators in manifest.

Feature Overview

Outreach outcome ingestion brings documentation collected during remote visits back into the governed record. It reconnects previously separated activity with the regional context without erasing when, where and by whom the source information was established.

Definition

Ingest and reconcile clinical documentation gathered during rural and remote outreach visits.

Established Behavioural Context

Element	Relationship to this Feature
Function: Reconcile Outreach Clinical Batch (p. 310)	realises feature
Service: Outreach Batch Reconciliation Service (p. 311)	associated feature

3.3.15.3. Function: Provide Outreach Visit Context

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-provide-outreach-visit-context`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Outreach Care (p. 308)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sd:206 (p. 652)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Outreach Visit Context Provision (p. 309)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Packages and delivers portable clinical summaries and offline-capable care summaries for remote outreach clinics.

Derivation: `FEAT-SD-17` in Strategy's Outreach Care Continuity establishes this preparation/provision responsibility. Outcome reconciliation below does not supply pre-visit context. This Function remains owned by Outreach Care, uses governed source information without acquiring its originating authority and establishes no copying, storage or offline implementation mechanism.

3.3.15.4. Function: Reconcile Outreach Clinical Batch

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-reconcile-outreach-clinical-batch`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Outreach Care (p. 308)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sd:210 (p. 652)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Outreach Encounter Outcome Ingestion (p. 309)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Reconciles and merges clinical documentation recorded during disconnected remote clinic operations.

3.3.15.5. Service: Outreach Batch Reconciliation Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-outreach-batch-reconciliation-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Outreach Care (p. 308)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sd:211 (p. 652)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Outreach Encounter Outcome Ingestion (p. 309)	Established association only.

Governed Description and Qualifications

Reconciles batch clinical encounters upon network reconnection.

3.3.16. BEC: Remote & Virtual Care

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-remote-virtual-care`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Harmonia-Relevant	Independent of status; never inherited.
Source	enabling:376 (p. 571); enabling:434 (p. 574); sd:221 (p. 653)	Linked RAD detail; exact locators in manifest.

Capability Overview

Remote & Virtual Care integrates remotely obtained observations and associates virtual consultations with their patient and episode context. It distinguishes a telemetry contribution from the encounter in which a practitioner may interpret it. The enabling responsibility concerns information continuity across delivery modes, preserving the setting-independent meaning of the Business care capabilities.

Boundary

Telemetry acquisition is not clinical monitoring judgement or treatment; session association does not establish clinical adequacy.

Harmonia Architectural Relevance

Remote data and virtual-session context materially extend information continuity across care locations and modes.

Scope

Contained Features and Established Owned Behaviour

Element	Type	Reference Id
Feature: Remote Telemetry Ingestion (p. 312)	Feature	`FEAT-SD-19`
Feature: Virtual Care Session Context Binding (p. 313)	Feature	`FEAT-SD-20`
Function: Correlate Virtual Care Consultation Context (p. 313)	Function	`HCM-FN-correlate-virtual-care-consultation-context`
Service: Virtual Encounter Context Resolution (p. 314)	Service	`HCM-SV-virtual-encounter-context-resolution`
Function: Ingest Remote Patient Telemetry (p. 314)	Function	`HCM-FN-ingest-remote-patient-telemetry`
Service: Remote Telemetry Ingress (p. 314)	Service	`HCM-SV-remote-telemetry-ingress`

3.3.16.1. Feature: Remote Telemetry Ingestion

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SD-19`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Remote & Virtual Care (p. 311)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:436 (p. 574)	Linked RAD detail; exact locators in manifest.

Feature Overview

Remote telemetry ingestion accepts measurements and patient-reported contributions from outside the usual care site. The information can inform subsequent monitoring and interpretation, but acquisition alone does not establish clinical significance or authorise an intervention.

Definition

Ingest home patient monitoring data, biometric telemetry, and patient-reported outcome measures (PROMs).

Established Behavioural Context

Element	Relationship to this Feature
Function: Ingest Remote Patient Telemetry (p. 314)	realises feature
Service: Remote Telemetry Ingress (p. 314)	associated feature

3.3.16.2. Feature: Virtual Care Session Context Binding

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-SD-20`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Remote & Virtual Care (p. 311)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:437 (p. 574)	Linked RAD detail; exact locators in manifest.

Feature Overview

Virtual-session binding associates a consultation with the enduring record and the relevant episode. It preserves the clinical context across delivery modes without turning the communication session itself into proof of clinical conclusions or adequacy of care.

Definition

Associate synchronous virtual/telehealth consultations with the longitudinal health record and active care episode.

Established Behavioural Context

Element	Relationship to this Feature
Function: Correlate Virtual Care Consultation Context (p. 313)	realises feature
Service: Virtual Encounter Context Resolution (p. 314)	associated feature

3.3.16.3. Function: Correlate Virtual Care Consultation Context

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-correlate-virtual-care-consultation-context`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Remote & Virtual Care (p. 311)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sd:225 (p. 653)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Virtual Care Session Context Binding (p. 313)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Binds video consultation metadata, remote practitioner identity, and shared digital whiteboard artifacts to the encounter.

3.3.16.4. Service: Virtual Encounter Context Resolution

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-virtual-encounter-context-resolution`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Remote & Virtual Care (p. 311)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sd:226 (p. 653)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Virtual Care Session Context Binding (p. 313)	Established association only.

Governed Description and Qualifications

Binds virtual session identifiers to patient encounters.

3.3.16.5. Function: Ingest Remote Patient Telemetry

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-ingest-remote-patient-telemetry`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Remote & Virtual Care (p. 311)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sd:228 (p. 653)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Remote Telemetry Ingestion (p. 312)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Ingests continuous or periodic physiological measurements (e.g., blood pressure, pulse oximetry, continuous glucose) from home monitoring devices.

3.3.16.6. Service: Remote Telemetry Ingress

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-remote-telemetry-ingress`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Remote & Virtual Care (p. 311)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	sd:229 (p. 653)	Linked RAD detail; exact locators in manifest.

Name	Value	Comment / Description
Associated Feature	Feature: Remote Telemetry Ingestion (p. 312)	Established association only.

Governed Description and Qualifications

Ingests remote patient data streams into the LHR.

3.4. Health Service Operations

Capability	Reference Id
BEC: Clinic & Practice Operations (p. 316)	HCM-BEC-clinic-practice-operations
BEC: Ward Operations (p. 320)	HCM-BEC-ward-operations
BEC: Theatre Operations (p. 324)	HCM-BEC-theatre-operations
BEC: Emergency Department Operations (p. 328)	HCM-BEC-emergency-department-operations
BEC: Outpatient Operations (p. 331)	HCM-BEC-outpatient-operations
BEC: Bed & Care-Place Management (p. 335)	HCM-BEC-bed-care-place-management
BEC: Clinical Resource Management (p. 341)	HCM-BEC-clinical-resource-management
BEC: Service Capacity Management (p. 343)	HCM-BEC-service-capacity-management
BEC: Clinical Workforce Management (p. 345)	HCM-BEC-clinical-workforce-management
BEC: Clinical Rostering (p. 346)	HCM-BEC-clinical-rostering
BEC: Mobile Staff Management (p. 347)	HCM-BEC-mobile-staff-management
BEC: On-Call Management (p. 351)	HCM-BEC-on-call-management
BEC: Work Allocation & Dispatch (p. 354)	HCM-BEC-work-allocation-dispatch
BEC: Clinical Training & Education (p. 360)	HCM-BEC-clinical-training-education
BEC: Clinical Competency Management (p. 360)	HCM-BEC-clinical-competency-management
BEC: Clinical Qualification Management (p. 361)	HCM-BEC-clinical-qualification-management
BEC: Clinical Supervision (p. 363)	HCM-BEC-clinical-supervision
BEC: Patient Transport (p. 364)	HCM-BEC-patient-transport
BEC: Clinical Logistics Coordination (p. 368)	HCM-BEC-clinical-logistics-coordination
BEC: Discharge Management (p. 371)	HCM-BEC-discharge-management
BEC: Credential Management (p. 376)	HCM-BEC-credential-management

3.4.1. BEC: Clinic & Practice Operations

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-clinic-practice-operations`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Harmonia-Relevant	Independent of status; never inherited.
Source	enabling:448 (p. 574); enabling:481 (p. 575); hso:72 (p. 656)	Linked RAD detail; exact locators in manifest.

Capability Overview

Clinic & Practice Operations tracks operating sessions and the operational progression of people through the clinic environment. It makes delays, waiting and completion context visible alongside service-specific capacity responsibilities. This is real-world clinic operation, distinct from the technical queueing machinery that might realise the information flow.

Boundary

Operational queue progression does not determine clinical priority or own the clinician's worklist.

Harmonia Architectural Relevance

Session and flow information materially supports clinic coordination while the clinic retains its contextual capacity responsibility.

Scope

Contained Features and Established Owned Behaviour

Element	Type	Reference Id
Feature: Clinic Session Tracking (p. 317)	Feature	`FEAT-HSO-01`
Feature: Operational Queue Progression (p. 317)	Feature	`FEAT-HSO-02`
Function: Manage Clinic & Practice Capacity (p. 318)	Function	`HCM-FN-manage-clinic-practice-capacity`
Function: Track Clinic Operating Session (p. 318)	Function	`HCM-FN-track-clinic-operating-session`
Function: Manage Clinic Queue Progression (p. 319)	Function	`HCM-FN-manage-clinic-queue-progression`
Service: Clinic Queue Telemetry Service (p. 319)	Service	`HCM-SV-clinic-queue-telemetry-service`
Process: Clinic / Operational Queue Progression Process (p. 320)	Process	`HCM-PR-clinic-operational-queue-progression-process`

3.4.1.1. Feature: Clinic Session Tracking

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-HSO-01`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Clinic & Practice Operations (p. 316)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:483 (p. 575)	Linked RAD detail; exact locators in manifest.

Feature Overview

Clinic-session tracking records the operating session's start, delay and completion as service-level context. It is distinct from an individual patient's queue state: knowing where one patient is waiting does not establish whether the clinic session is running as planned.

Definition

Ingest and track clinic operating session start, delay, and completion states.

Established Behavioural Context

Element	Relationship to this Feature
Function: Track Clinic Operating Session (p. 318)	realises feature

3.4.1.2. Feature: Operational Queue Progression

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-HSO-02`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Clinic & Practice Operations (p. 316)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:484 (p. 575)	Linked RAD detail; exact locators in manifest.

Feature Overview

Operational queue progression makes the patient's passage through clinic stages visible. The queue concerns real-world arrival, waiting and consultation context, rather than a broker queue, and does not give Harmonia authority to determine clinical priority or own the clinician's work-list.

Definition

Track client queue progression through reception, waiting area, consultation, and billing.

Established Behavioural Context

Element	Relationship to this Feature
Function: Manage Clinic Queue Progression (p. 319)	realises feature
Service: Clinic Queue Telemetry Service (p. 319)	associated feature

3.4.1.3. Function: Manage Clinic & Practice Capacity

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-manage-clinic-practice-capacity`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Clinic & Practice Operations (p. 316)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	hso:75 (p. 656)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Governed Description and Qualifications

Understands and manages available, committed and utilised clinic/practice capacity in relation to practitioner availability, sessions, rooms, appointments and patient demand.

Responsibility boundary: Operating-session tracking and individual queue progression contribute to this responsibility; neither alone expresses overall service capacity. Capacity coordination does not own source bookings, clinical decisions or practitioner rostering.

3.4.1.4. Function: Track Clinic Operating Session

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-track-clinic-operating-session`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Clinic & Practice Operations (p. 316)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	hso:78 (p. 656)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Clinic Session Tracking (p. 317)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Ingests and tracks clinic operating-session start, delay and completion, distinct from an individual patient's queue progression.

Derivation: `FEAT-HSO-01` directly establishes this session-level behaviour; Manage Clinic Queue Progression and its Process concern individual patients and do not supply it.

3.4.1.5. Function: Manage Clinic Queue Progression

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-manage-clinic-queue-progression`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Clinic & Practice Operations (p. 316)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	hso:82 (p. 656)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Operational Queue Progression (p. 317)	Established Feature realisation; not structural containment.
Governed Process	Process: Clinic / Operational Queue Progression Process (p. 320)	Established progression relationship.

Governed Description and Qualifications

Tracks patient arrival, rooming, consultation in-progress, and check-out states.

3.4.1.6. Service: Clinic Queue Telemetry Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-clinic-queue-telemetry-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Clinic & Practice Operations (p. 316)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	hso:83 (p. 656)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Operational Queue Progression (p. 317)	Established association only.

Governed Description and Qualifications

Discloses real-time clinic waiting times and queue states.

3.4.1.7. Process: Clinic / Operational Queue Progression Process

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-PR-clinic-operational-queue-progression-process`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Clinic & Practice Operations (p. 316)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	processes:187 (p. 617); processes:188 (p. 617)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Process Overview

This Process follows an individual's operational passage through the clinic. Waiting, rooming, consultation and departure have different implications for flow and communication, while the clinic's overall session has its own state. The progression is therefore patient-specific operational context rather than a complete account of clinic capacity or clinical priority.

Governed Description and Qualifications

- **Owning Capability**: `Clinic&PracticeOperations`
- **Process Purpose**: Governs the movement and state tracking of outpatients through an ambulatory specialty clinic session.
- **State Progression Lifecycle**: Scheduled → Patient Arrived → Checked-In → Roomed / Pre-Consult → Consultation In-Progress → Consultation Concluded → Departed
- **Key State Dispositions**:
  - `Scheduled`: Appointment booked on session list.
  - `PatientArrived`: Patient enters waiting area.
  - `Checked-In`: Reception verifies demographics and marks patient ready.
  - `Roomed/Pre-Consult`: Nursing staff record baseline vitals and room patient.
  - `ConsultationIn-Progress`: Clinician actively conducting consultation.
  - `ConsultationConcluded`: Clinical discussion finished; follow-up orders placed.
  - `Departed`: Patient checks out and departs clinic facility.

3.4.2. BEC: Ward Operations

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-ward-operations`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Harmonia-Relevant	Independent of status; never inherited.

Name	Value	Comment / Description
Source	enabling:449 (p. 574); enabling:486 (p. 575); hso:90 (p. 656)	Linked RAD detail; exact locators in manifest.

Capability Overview

Ward Operations coordinates ward occupancy and care-place context and assembles the information needed around shift changes. Its responsibility includes the ward-specific meaning of capacity and patient flow rather than a generic resource counter. It can make pending concerns and tasks visible without becoming the authority managing clinical handover.

Boundary

Handover information assembly is distinct from performing or managing handover. Context-specific capacity remains with Ward Operations.

Harmonia Architectural Relevance

Correlated ward state and contextual information materially support operational continuity.

Scope

Contained Features and Established Owned Behaviour

Element	Type	Reference Id
Feature: Ward Patient Flow Coordination (p. 321)	Feature	`FEAT-HSO-03`
Feature: Clinical Handover Context Assembly (p. 322)	Feature	`FEAT-HSO-04`
Function: Manage Ward Capacity (p. 322)	Function	`HCM-FN-manage-ward-capacity`
Service: Ward Operational Summary Query (p. 323)	Service	`HCM-SV-ward-operational-summary-query`
Function: Assemble Ward Handover Context (p. 323)	Function	`HCM-FN-assemble-ward-handover-context`

3.4.2.1. Feature: Ward Patient Flow Coordination

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-HSO-03`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Ward Operations (p. 320)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:488 (p. 575)	Linked RAD detail; exact locators in manifest.

Feature Overview

Ward flow coordination connects occupancy, place allocation and bed status within the ward's operational context. Those facts contribute to ward capacity but do not exhaust its acuity, staffing and cohorting responsibilities or prove that another patient can be accepted.

Definition

Coordinate inpatient ward occupancy, care-place allocation, and patient bed status across care facilities.

Established Behavioural Context

Element	Relationship to this Feature
Function: Manage Ward Capacity (p. 322)	contributes feature

3.4.2.2. Feature: Clinical Handover Context Assembly

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-HS0-04`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Ward Operations (p. 320)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:489 (p. 575)	Linked RAD detail; exact locators in manifest.

Feature Overview

Handover context assembly brings patient lists, current concerns and pending tasks together for the participants conducting a shift handover. Assembly is the established responsibility: it does not perform clinical review, decide pending clinical work or manage the handover itself.

Definition

Collate current ward patient lists, active medical concerns, and pending tasks for nursing shift handovers.

Established Behavioural Context

Element	Relationship to this Feature
Function: Assemble Ward Handover Context (p. 323)	realises feature

3.4.2.3. Function: Manage Ward Capacity

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-manage-ward-capacity`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Ward Operations (p. 320)	Established responsibility only; complete CT ancestry Unresolved.

Name	Value	Comment / Description
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	hso:93 (p. 656)	Linked RAD detail; exact locators in manifest.
Name Aliases	Coordinate Ward Operational State	Same element only.

Governed Description and Qualifications

Coordinates available, committed and utilised ward capacity and its patient-flow implications, retaining acuity distribution, nurse-to-patient ratios and isolation cohorting alongside bed/care-place state, patient demand and operational constraints.

Capacity boundary: Bed & Care-Place Management contributes resource state. Ward capacity is assessed in its service context; an available bed alone does not establish capacity to accept a patient. Source staffing and clinical assertions retain their authority.

3.4.2.4. Service: Ward Operational Summary Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-ward-operational-summary-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Ward Operations (p. 320)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	hso:95 (p. 656)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Governed Description and Qualifications

Discloses ward-level operational telemetry.

Capacity boundary: Bed & Care-Place Management contributes resource state. Ward capacity is assessed in its service context; an available bed alone does not establish capacity to accept a patient. Source staffing and clinical assertions retain their authority.

3.4.2.5. Function: Assemble Ward Handover Context

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-assemble-ward-handover-context`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Ward Operations (p. 320)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	hso:99 (p. 656)	Linked RAD detail; exact locators in manifest.

Name	Value	Comment / Description
Derived From	Feature: Clinical Handover Context Assembly (p. 322)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Collates current ward patient lists, active medical concerns and pending tasks for nursing shift handover.

Derivation: `FEAT-HSO-04` directly requires this assembly. Ward acuity/cohorting telemetry alone does not provide the stated handover content. Source clinical information retains its originating authority; assembly does not perform clinical review, manage clinical handover, determine pending clinical work or acquire clinical judgement.

3.4.3. BEC: Theatre Operations

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-theatre-operations`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Harmonia-Relevant	Independent of status; never inherited.
Source	enabling:450 (p. 574); enabling:491 (p. 575); hso:106 (p. 657)	Linked RAD detail; exact locators in manifest.

Capability Overview

Theatre Operations integrates surgical case progression and the operational consequences relevant to portering, sterilisation and recovery participants. It gives reported milestones an operational coordination context without originating the surgical acts they describe. Theatre-specific capacity remains a business responsibility even when shared information machinery supports several service contexts.

Boundary

Tracking incision or recovery information does not authorise surgery or transfer control of the clinical procedure.

Harmonia Architectural Relevance

Milestone-driven operational notifications materially connect supporting activity around the theatre service.

Scope

Contained Features and Established Owned Behaviour

Element	Type	Reference Id
Feature: Operating Theatre Case Progression (p. 325)	Feature	FEAT-HSO-05
Feature: Perioperative Resource Notification (p. 326)	Feature	FEAT-HSO-06
Function: Manage Theatre Capacity (p. 326)	Function	HCM-FN-manage-theatre-capacity
Function: Manage Theatre Case Progression (p. 327)	Function	HCM-FN-manage-theatre-case-progression
Service: Theatre Case Progression Telemetry (p. 327)	Service	HCM-SV-theatre-case-progression-telemetry
Process: Theatre Case Progression Process (p. 328)	Process	HCM-PR-theatre-case-progression-process

3.4.3.1. Feature: Operating Theatre Case Progression

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-HSO-05	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Theatre Operations (p. 324)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:493 (p. 575)	Linked RAD detail; exact locators in manifest.

Feature Overview

Case progression records externally established surgical milestones in an operationally meaningful sequence. It makes the theatre situation visible without authorising or performing the clinical acts denoted by anaesthesia, incision, closure or recovery.

Definition

Track surgical case state transitions (CALLED → IN_THEATRE → ANAESTHETISED → INCISION → CLOSED → RECOVERY).

Established Behavioural Context

Element	Relationship to this Feature
Function: Manage Theatre Case Progression (p. 327)	realises feature
Service: Theatre Case Progression Telemetry (p. 327)	associated feature

3.4.3.2. Feature: Perioperative Resource Notification

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-HSO-06`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Theatre Operations (p. 324)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:494 (p. 575)	Linked RAD detail; exact locators in manifest.

Feature Overview

Perioperative notification connects relevant case milestones with portering, sterilisation and recovery participants. Observation alone does not realise the Feature's dispatch responsibility; the established case-progression behaviour includes the required alerts without adding another Function or Service.

Definition

Dispatch alerts to portering, sterilization, and recovery teams based on surgical case progression milestones.

Established Behavioural Context

Element	Relationship to this Feature
Function: Manage Theatre Case Progression (p. 327)	realises feature

3.4.3.3. Function: Manage Theatre Capacity

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-manage-theatre-capacity`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Theatre Operations (p. 324)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	hso:109 (p. 657)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Governed Description and Qualifications

Understands and manages available, committed and utilised theatre capacity in relation to theatre, team and equipment availability, scheduled cases and case progression.

Responsibility boundary: Case milestones and targeted resource notifications contribute; they do not by themselves establish available or committed theatre service capacity. Clinical case determination and originating team/equipment information remain separately governed.

3.4.3.4. Function: Manage Theatre Case Progression

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-manage-theatre-case-progression`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Theatre Operations (p. 324)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	hso:112 (p. 657)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Operating Theatre Case Progression (p. 325); Feature: Perioperative Resource Notification (p. 326)	Established Feature realisation; not structural containment.
Governed Process	Process: Theatre Case Progression Process (p. 328)	Established progression relationship.

Governed Description and Qualifications

Coordinates surgical case milestones: *Case Called → Patient in Anaesthetic Bay → Anaesthesia Commenced → Knife to Skin → Procedure Finished → In PACU/Recovery → Ward Handover Complete*.

Additional Feature responsibility: `FEAT-HSO-06–PerioperativeResourceNotification` is realised by dispatching progression-triggered alerts to portering, sterilisation and recovery teams within this same case-progression behaviour/exposure. Milestone observation alone does not establish notification; no new Function or Service is created.

3.4.3.5. Service: Theatre Case Progression Telemetry

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-theatre-case-progression-telemetry`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Theatre Operations (p. 324)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	hso:113 (p. 657)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Operating Theatre Case Progression (p. 325)	Established association only.

Governed Description and Qualifications

Emits real-time surgical milestone updates.

Additional Feature responsibility: `FEAT-HSO-06–PerioperativeResourceNotification` is realised by dispatching progression-triggered alerts to portering, sterilisation and recovery teams within

this same case-progression behaviour/exposure. Milestone observation alone does not establish notification; no new Function or Service is created.

3.4.3.6. Process: Theatre Case Progression Process

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-PR-theatre-case-progression-process`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Theatre Operations (p. 324)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	processes:170 (p. 617); processes:171 (p. 617)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Process Overview

Theatre case progression follows reported milestones of the surgical encounter so that supporting participants can coordinate around the actual situation. The state account concerns what the clinical service reports has occurred. It does not make the integration mechanism the actor administering anaesthesia, performing surgery or managing clinical handover.

Governed Description and Qualifications

- **Owning Capability**: `TheatreOperations`
- **Process Purpose**: Governs the perioperative operational milestones of a surgical procedure within the operating theatre suite.
- **State Progression Lifecycle**: Case Called → In Anaesthetic Bay → Anaesthesia Commenced → Patient in Theatre → Knife-to-Skin → Procedure Finished → In PACU Recovery → Ward Handover Complete
- **Key State Dispositions**:
  - `CaseCalled`: Theatre team requests ward to send patient for surgery.
  - `InAnaestheticBay`: Patient arrives in theatre holding and identity/consent verified.
  - `AnaesthesiaCommenced`: Anaesthetic induction underway.
  - `PatientinTheatre`: Patient transferred onto operating table.
  - `Knife-to-Skin`: Surgical incision initiated (operative start).
  - `ProcedureFinished`: Wound closure completed and dressings applied.
  - `InPACURecovery`: Patient monitored in post-anaesthesia care unit.
  - `WardHandoverComplete`: Post-operative clinical handover to inpatient ward completed.

3.4.4. BEC: Emergency Department Operations

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-emergency-department-operations`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.

Name	Value	Comment / Description
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Harmonia-Relevant	Independent of status; never inherited.
Source	enabling:451 (p. 574); enabling:496 (p. 575); hso:121 (p. 657)	Linked RAD detail; exact locators in manifest.

Capability Overview

This capability maintains the operational context of emergency places, waits and length-of-stay milestones. It supports understanding of occupancy and impediments in the department while keeping capacity's emergency-service meaning intact. Its concern differs from Emergency Care's clinical presentation context, even though both can refer to the same patient encounter.

Boundary

Operational timing and escalation do not determine triage or acquire clinical decision authority.

Harmonia Architectural Relevance

Care-place state and operational waiting information materially support coordination of the emergency service.

Scope

Contained Features and Established Owned Behaviour

Element	Type	Reference Id
Feature: ED Bed & Bay Tracking (p. 329)	Feature	`FEAT-HSO-07`
Feature: ED Length of Stay Tracking (p. 330)	Feature	`FEAT-HSO-08`
Function: Coordinate ED Departmental Capacity (p. 330)	Function	`HCM-FN-coordinate-ed-departmental-capacity`
Service: ED Operational Status Query (p. 331)	Service	`HCM-SV-ed-operational-status-query`

3.4.4.1. Feature: ED Bed & Bay Tracking

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-HSO-07`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Emergency Department Operations (p. 328)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:498 (p. 575)	Linked RAD detail; exact locators in manifest.

Feature Overview

Emergency-place tracking distinguishes occupancy and status across cubicles, resuscitation bays and short-stay beds. It provides place context for departmental operations while leaving the service's usable capacity and clinical triage decisions separate from an inventory of spaces.

Definition

Track occupancy and status of emergency cubicles, resuscitation bays, and short-stay beds.

Established Behavioural Context

Element	Relationship to this Feature
Function: Coordinate ED Departmental Capacity (p. 330)	realises feature

3.4.4.2. Feature: ED Length of Stay Tracking

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-HSO-08`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Emergency Department Operations (p. 328)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:499 (p. 575)	Linked RAD detail; exact locators in manifest.

Feature Overview

Length-of-stay tracking makes waits and operational timing thresholds visible for escalation. The timing information supports management of impediments without determining clinical urgency or making an elapsed-time alert an independent assurance finding.

Definition

Monitor operational wait times and length-of-stay milestones, generating operational escalation notifications.

3.4.4.3. Function: Coordinate ED Departmental Capacity

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-coordinate-ed-departmental-capacity`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Emergency Department Operations (p. 328)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.

Name	Value	Comment / Description
Source	hso:124 (p. 657)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: ED Bed & Bay Tracking (p. 329)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Understands and manages available, committed and utilised ED capacity in relation to care-place availability, demand, patient presence/progression, acuity and operational constraints; retains waiting room occupancy, resuscitation bay availability and time-to-triage metrics.

Capacity boundary: Existing departmental capacity responsibility is clarified, not duplicated. Care-place state contributes to service capacity; capacity coordination neither performs clinical triage nor establishes clinical acuity assertions.

3.4.4.4. Service: ED Operational Status Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-ed-operational-status-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Emergency Department Operations (p. 328)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	hso:125 (p. 657)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Governed Description and Qualifications

Discloses facility ED occupancy and surge level.

Capacity boundary: Existing departmental capacity responsibility is clarified, not duplicated. Care-place state contributes to service capacity; capacity coordination neither performs clinical triage nor establishes clinical acuity assertions.

3.4.5. BEC: Outpatient Operations

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-outpatient-operations`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Harmonia-Relevant	Independent of status; never inherited.

Name	Value	Comment / Description
Source	enabling:452 (p. 574); enabling:501 (p. 575); hso:134 (p. 658)	Linked RAD detail; exact locators in manifest.

Capability Overview

Outpatient Operations integrates arrival events and follow-up requirements generated as clinics progress. It connects a person's operational presence with the relevant service context and makes post-clinic booking or diagnostic needs available for subsequent activity. The responsibility retains outpatient-specific capacity semantics without requiring a separate arrival Function where approved reusable composition suffices.

Boundary

An arrival does not establish consultation completion, and a captured follow-up requirement does not perform the clinical activity.

Harmonia Architectural Relevance

Arrival and clinic-outcome information materially connect outpatient progression to care coordination.

Scope

Contained Features and Established Owned Behaviour

Element	Type	Reference Id
Feature: Outpatient Arrival Registration (p. 332)	Feature	`FEAT-HSO-09`
Feature: Post-Clinic Order Coordination (p. 333)	Feature	`FEAT-HSO-10`
Function: Manage Outpatient Capacity (p. 334)	Function	`HCM-FN-manage-outpatient-capacity`
Service: Ambulatory Session Status Query (p. 334)	Service	`HCM-SV-ambulatory-session-status-query`
Function: Capture Post-Clinic Follow-up Requirements (p. 335)	Function	`HCM-FN-capture-post-clinic-follow-up-requirements`

3.4.5.1. Feature: Outpatient Arrival Registration

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-HSO-09`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Outpatient Operations (p. 331)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:503 (p. 576)	Linked RAD detail; exact locators in manifest.

Feature Overview

Outpatient arrival registration connects a check-in event with the treating context and required notification. Approved reusable composition supplies this integration responsibility; it does not require a specialised arrival Function or prove that a consultation has begun or finished.

Definition

Ingest self-service check-in and clinic arrival events, notifying treating clinicians.

Established Behavioural Context

Element	Relationship to this Feature
Function: Manage Outpatient Capacity (p. 334)	realises feature

3.4.5.2. Feature: Post-Clinic Order Coordination

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-HSO-10`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Outpatient Operations (p. 331)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:504 (p. 576)	Linked RAD detail; exact locators in manifest.

Feature Overview

Post-clinic coordination captures the follow-up bookings and diagnostic requirements established at clinic completion. It connects those requirements with subsequent operational activity while preserving that capture is not the clinical decision, a completed booking or diagnostic fulfilment.

Definition

Capture follow-up booking and diagnostic order requirements generated at clinic completion.

Established Behavioural Context

Element	Relationship to this Feature
Function: Capture Post-Clinic Follow-up Requirements (p. 335)	realises feature

3.4.5.3. Function: Manage Outpatient Capacity

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-manage-outpatient-capacity`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Outpatient Operations (p. 331)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	hso:137 (p. 658)	Linked RAD detail; exact locators in manifest.
Name Aliases	Coordinate Outpatient Session Utilisation	Same element only.
Derived From	Feature: Outpatient Arrival Registration (p. 332)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Understands and manages available, committed and utilised outpatient capacity through clinic/session, practitioner and room availability, appointment demand, patient arrival/waiting and operational progression. Retains specialty room allocation and clinician arrival tracking.

Exposure qualification: Query behaviour and Outpatient Operations ownership remain established; a meaningful outside consumer/mandate is not identified by the current description. That exposure relationship remains unestablished rather than populated with an inferred Actor, Role or Capability.

Capacity boundary: Operational coordination does not acquire authoritative booking/rostering or clinical-work management. Room-occupancy query remains narrower than the capacity responsibility; no enlarged exposure or new named Service is inferred.

3.4.5.4. Service: Ambulatory Session Status Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-ambulatory-session-status-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Outpatient Operations (p. 331)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	hso:140 (p. 658)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.
Exposure / Consumer	Unresolved	Named Service is established but outside consumer/exposure mandate is unestablished.

Governed Description and Qualifications

Exposes clinic room occupancy.

Exposure qualification: Query behaviour and Outpatient Operations ownership remain established; a meaningful outside consumer/mandate is not identified by the current description.

That exposure relationship remains unestablished rather than populated with an inferred Actor, Role or Capability.

Capacity boundary: Operational coordination does not acquire authoritative booking/rostering or clinical-work management. Room-occupancy query remains narrower than the capacity responsibility; no enlarged exposure or new named Service is inferred.

3.4.5.5. Function: Capture Post-Clinic Follow-up Requirements

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-capture-post-clinic-follow-up-requirements`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Outpatient Operations (p. 331)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	hso:144 (p. 658)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Post-Clinic Order Coordination (p. 333)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Captures follow-up booking and diagnostic-order requirements generated at clinic completion.

Derivation: `FEAT-HSO-10` directly establishes this capture responsibility. The Clinic queue Process's consultation-concluded checkpoint describes orders placed, not Outpatient Operations' capture of follow-up requirements. Capture does not authorise clinical orders, own authoritative bookings or establish a new Service dependency.

3.4.6. BEC: Bed & Care-Place Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-bed-care-place-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Harmonia-Core	Independent of status; never inherited.
Source	enabling:453 (p. 574); enabling:506 (p. 576); hso:151 (p. 658)	Linked RAD detail; exact locators in manifest.

Capability Overview

Bed & Care-Place Management maintains operational place state and the turnover progression that makes a place usable again. Vacancy, reservation, cleaning and readiness must remain distinguishable so that an empty bed is not automatically treated as available for allocation. Notifications expose meaningful readiness changes to the responsible operational participants.

Intent

Support credible care-place decisions using governed state rather than physical presence alone.

Boundary

A ready bed does not establish service-level capacity or collective readiness, and place state does not determine clinical suitability or care responsibility.

Harmonia Architectural Relevance

Governed place-state progression and operational notification are substantive assigned platform responsibilities.

Scope

Contained Features and Established Owned Behaviour

Element	Type	Reference Id
Feature: Bed Availability Tracking (p. 336)	Feature	`FEAT-HSO-11`
Feature: Bed State Progression (p. 337)	Feature	`FEAT-HSO-12`
Feature: Bed Readiness Notification (p. 338)	Feature	`FEAT-HSO-13`
Function: Maintain Operational Bed State (p. 338)	Function	`HCM-FN-maintain-operational-bed-state`
Service: Bed Availability Query (p. 339)	Service	`HCM-SV-bed-availability-query`
Function: Manage Bed Turnover Progression (p. 339)	Function	`HCM-FN-manage-bed-turnover-progression`
Service: Bed Turnover Lifecycle Service (p. 339)	Service	`HCM-SV-bed-turnover-lifecycle-service`
Process: Bed Turnover Process (p. 340)	Process	`HCM-PR-bed-turnover-process`

3.4.6.1. Feature: Bed Availability Tracking

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-HSO-11`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Bed & Care-Place Management (p. 335)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:508 (p. 576)	Linked RAD detail; exact locators in manifest.

Feature Overview

Availability tracking preserves the operational distinctions among vacancy, occupancy, reservation, blockage and cleaning conditions. These states support meaningful care-place coordination without equating an unoccupied bed with a ready place or usable service capacity.

Definition

Maintain operational status of all registered beds and care-places (`VACANT`, `OCCUPIED`, `BLOCKED`, `RESERVED`, `DIRTY`, `CLEANING`, `CLEAN`).

Established Behavioural Context

Element	Relationship to this Feature
Function: Maintain Operational Bed State (p. 338)	realises feature
Service: Bed Availability Query (p. 339)	associated feature

3.4.6.2. Feature: Bed State Progression

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-HSO-12`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Bed & Care-Place Management (p. 335)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:509 (p. 576)	Linked RAD detail; exact locators in manifest.

Feature Overview

Bed-state progression applies the consequences of relevant clinical and environmental events to the managed place state. The resulting change is an operational fact, such as a need for cleaning after departure, rather than a transfer of clinical responsibility.

Definition

Progress bed states in response to clinical and environmental events (e.g., patient discharge automatically transitions bed to `DIRTY`).

Established Behavioural Context

Element	Relationship to this Feature
Function: Manage Bed Turnover Progression (p. 339)	realises feature
Service: Bed Turnover Lifecycle Service (p. 339)	associated feature

3.4.6.3. Feature: Bed Readiness Notification

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-HSO-13`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Bed & Care-Place Management (p. 335)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:510 (p. 576)	Linked RAD detail; exact locators in manifest.

Feature Overview

Readiness notification exposes the established readiness of a care place to the responsible allocation participants. It makes a relevant transition actionable without deciding the clinical suitability of the next allocation or establishing whole-service readiness.

Definition

Notify bed allocation managers and clinical units when a cleaned bed achieves `READY` status.

Established Behavioural Context

Element	Relationship to this Feature
Function: Manage Bed Turnover Progression (p. 339)	realises feature

3.4.6.4. Function: Maintain Operational Bed State

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-maintain-operational-bed-state`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Bed & Care-Place Management (p. 335)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	hso:157 (p. 658)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Bed Availability Tracking (p. 336)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Tracks real-time bed status and environmental locks.

3.4.6.5. Service: Bed Availability Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-bed-availability-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Bed & Care-Place Management (p. 335)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	hso:158 (p. 658)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Bed Availability Tracking (p. 336)	Established association only.

Governed Description and Qualifications

Discloses real-time bed availability across hospital directorates.

3.4.6.6. Function: Manage Bed Turnover Progression

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-manage-bed-turnover-progression`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Bed & Care-Place Management (p. 335)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	hso:160 (p. 658)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Bed State Progression (p. 337); Feature: Bed Readiness Notification (p. 338)	Established Feature realisation; not structural containment.
Governed Process	Process: Bed Turnover Process (p. 340)	Established progression relationship.

Governed Description and Qualifications

Coordinates cleaning, disinfection, and preparation: *Vacated → Cleaning Requested → Cleaning In-Progress → Inspected/Ready → Occupied*.

Additional Feature responsibility: `FEAT-HSO-13–BedReadinessNotification` requires the existing turnover behaviour/exposure to notify bed allocation managers and clinical units when a cleaned bed is ready. Readiness notification remains distinct from availability query and from physical placement; no new lifecycle state or Service is introduced.

3.4.6.7. Service: Bed Turnover Lifecycle Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-bed-turnover-lifecycle-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.

Name	Value	Comment / Description
Owning Context	BEC: Bed & Care-Place Management (p. 335)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	hso:161 (p. 658)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Bed State Progression (p. 337)	Established association only.

Governed Description and Qualifications

Coordinates turnover requests with environmental services.

Additional Feature responsibility: `FEAT-HSO-13–BedReadinessNotification` requires the existing turnover behaviour/exposure to notify bed allocation managers and clinical units when a cleaned bed is ready. Readiness notification remains distinct from availability query and from physical placement; no new lifecycle state or Service is introduced.

3.4.6.8. Process: Bed Turnover Process

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-PR-bed-turnover-process`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Bed & Care-Place Management (p. 335)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	processes:203 (p. 618); processes:204 (p. 618)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Process Overview

Bed turnover connects the release of a care place with the operational work needed to return it to use. Its progression preserves the difference between vacancy, cleaning and readiness. A place can be empty while work remains outstanding, and a completed turnover does not by itself prove that the service has capacity for the next patient.

Governed Description and Qualifications

- **Owning Capability**: `Bed&Care-PlaceManagement`
- **Process Purpose**: Governs the rapid physical turnover, cleaning, sanitisation, and reallocation of inpatient and emergency care-places.
- **State Progression Lifecycle**: Bed Vacated → Cleaning Dispatched → Sanitisation In-Progress → Terminal Cleaning Verified → Bed Ready / Available → Bed Allocated / Occupied
- **Key State Dispositions**:
  - `BedVacated`: Prior patient discharged or transferred.
  - `CleaningDispatched`: Cleaning job allocated to environmental services team.
  - `SanitisationIn-Progress`: Ward cleaning or isolation decontamination underway.
  - `TerminalCleaningVerified`: Ward coordinator or supervisor verifies care-place readiness.
  - `BedReady/Available`: Bed marked available for incoming patient allocation.

- `BedAllocated/Occupied`: Incoming patient allocated and physically placed in bed.

3.4.7. BEC: Clinical Resource Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-clinical-resource-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Harmonia-Relevant	Independent of status; never inherited.
Source	enabling:454 (p. 574); enabling:512 (p. 576); hso:169 (p. 659)	Linked RAD detail; exact locators in manifest.

Capability Overview

Clinical Resource Management integrates location and allocation information for shared clinical equipment. It makes the current operational resource context available rather than treating device inventory as proof that equipment can be used for a particular need. The responsibility supports coordination while retaining the distinction between resource state and a service's capacity.

Boundary

Equipment tracking does not acquire enterprise asset ownership, maintenance responsibility or clinical authority to use it.

Harmonia Architectural Relevance

Dynamic equipment context materially supports operational coordination across care environments.

Scope

Contained Features and Established Owned Behaviour

Element	Type	Reference Id
Feature: Mobile Clinical Equipment Tracking (p. 342)	Feature	`FEAT-HSO-14`
Function: Track Mobile Clinical Asset Operational State (p. 342)	Function	`HCM-FN-track-mobile-clinical-asset-operational-state`
Service: Clinical Resource Availability Query (p. 343)	Service	`HCM-SV-clinical-resource-availability-query`

3.4.7.1. Feature: Mobile Clinical Equipment Tracking

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-HSO-14`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Clinical Resource Management (p. 341)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:514 (p. 576)	Linked RAD detail; exact locators in manifest.

Feature Overview

Equipment tracking provides the dynamic location and allocation context of shared devices. It distinguishes an identifiable asset from one available for the present operational need, while leaving maintenance, ownership and clinical authorisation with their respective responsibilities.

Definition

Track physical location and allocation status of shared clinical equipment (infusion pumps, ventilators, mobile telemetry).

Established Behavioural Context

Element	Relationship to this Feature
Function: Track Mobile Clinical Asset Operational State (p. 342)	realises feature
Service: Clinical Resource Availability Query (p. 343)	associated feature

3.4.7.2. Function: Track Mobile Clinical Asset Operational State

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-track-mobile-clinical-asset-operational-state`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Clinical Resource Management (p. 341)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	hso:172 (p. 659)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Mobile Clinical Equipment Tracking (p. 342)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Manages the real-time location and operational readiness of mobile ventilators, infusion pumps, and telemetry transmitters.

3.4.7.3. Service: Clinical Resource Availability Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-clinical-resource-availability-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Clinical Resource Management (p. 341)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	hso:173 (p. 659)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Mobile Clinical Equipment Tracking (p. 342)	Established association only.

Governed Description and Qualifications

Exposes mobile equipment availability.

3.4.8. BEC: Service Capacity Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-service-capacity-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Harmonia-Relevant	Independent of status; never inherited.
Source	enabling:455 (p. 574); enabling:516 (p. 576); hso:179 (p. 659)	Linked RAD detail; exact locators in manifest.

Capability Overview

Service Capacity Management provides a higher-order view by aggregating capacity information across services and facilities. It can consider occupancy, waits and diversion while preserving the service-specific meaning of the underlying capacity. Its purpose is not to erase differences between clinic, ward, theatre, emergency, outpatient and mobile-service operation.

Boundary

Consuming contextual capacity does not acquire the originating capacity-management responsibility or infer capacity solely from resource counts.

Harmonia Architectural Relevance

Cross-service operational visibility materially supports coordination while source service contexts retain their authority.

Scope

Contained Features and Established Owned Behaviour

Element	Type	Reference Id
Feature: Operational Capacity Metric Aggregation (p. 344)	Feature	FEAT-HSO-15
Function: Aggregate Service Capacity Telemetry (p. 344)	Function	HCM-FN-aggregate-service-capacity-telemetry
Service: Regional Health System Capacity Query (p. 345)	Service	HCM-SV-regional-health-system-capacity-query

3.4.8.1. Feature: Operational Capacity Metric Aggregation

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-HSO-15	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Service Capacity Management (p. 343)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:518 (p. 576)	Linked RAD detail; exact locators in manifest.

Feature Overview

Capacity aggregation combines occupancy, waiting and diversion context across facilities. The higher-order view must retain the originating service-specific capacity meaning; a shared metric does not transfer clinic, ward or theatre capacity responsibility to the aggregator.

Definition

Aggregate operational capacity telemetry (occupancy rates, wait times, diversion status) across facilities.

Established Behavioural Context

Element	Relationship to this Feature
Function: Aggregate Service Capacity Telemetry (p. 344)	realises feature
Service: Regional Health System Capacity Query (p. 345)	associated feature

3.4.8.2. Function: Aggregate Service Capacity Telemetry

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	HCM-FN-aggregate-service-capacity-telemetry	Stable publication identity.
Architectural Status	Canonical	Established by RAD.

Name	Value	Comment / Description
Owning Context	BEC: Service Capacity Management (p. 343)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	hso:182 (p. 659)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Operational Capacity Metric Aggregation (p. 344)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Consumes and coordinates context-specific service-capacity information for broader situational awareness, alongside bed occupancy, ICU surge capacity and ventilator availability across regional hospitals.

3.4.8.3. Service: Regional Health System Capacity Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-regional-health-system-capacity-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Service Capacity Management (p. 343)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	hso:183 (p. 659)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Operational Capacity Metric Aggregation (p. 344)	Established association only.

Governed Description and Qualifications

Provides executive operational situational awareness.

3.4.9. BEC: Clinical Workforce Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-clinical-workforce-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Adjacent	Independent of status; never inherited.
Source	enabling:456 (p. 574)	Linked RAD detail; exact locators in manifest.

Capability Overview

Clinical Workforce Management is retained as an Adjacent operational context for the workforce on which services depend. The current enabling catalogue does not assign an atomic Feature set. Its presence allows workforce constraints to be recognised without treating integration of workforce facts as authority to govern employment, coverage or clinical allocation.

Boundary

No new workforce system responsibility or complete CT ancestry is inferred from this contextual entry.

Harmonia Architectural Relevance

Adjacent standing reflects contextual participation rather than management of the workforce itself.

3.4.10. BEC: Clinical Rostering

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-clinical-rostering`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Adjacent	Independent of status; never inherited.
Source	enabling:457 (p. 574)	Linked RAD detail; exact locators in manifest.

Capability Overview

Clinical Rostering provides the Adjacent context of planned workforce availability. It is distinct from consuming roster projections for operational views or resolving who is currently on call. The present architecture retains the context without establishing a Harmonia Feature decomposition for authoring or managing the authoritative roster.

Boundary

A projected roster does not confer qualification, privilege, deployment or ownership of the originating schedule.

Harmonia Architectural Relevance

Roster information can inform operations while the formal Adjacent classification preserves its external management.

3.4.11. BEC: Mobile Staff Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-mobile-staff-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Harmonia-Relevant	Independent of status; never inherited.
Source	enabling:458 (p. 574); enabling:520 (p. 576); hso:190 (p. 659)	Linked RAD detail; exact locators in manifest.

Capability Overview

Mobile Staff Management integrates with external business mobile-work mechanisms and receives visit progression and safety milestones. Its established dispatch Feature exchanges assignment, dispatch and status information with the relevant worker context. This is an information integration boundary around mobile activity, rather than a platform claim to decide the clinician's work.

Boundary

Harmonia does not determine clinical allocation, handover or EMR worklists. External work/task language does not redefine the internal Synthetic Task.

Harmonia Architectural Relevance

Mobile assignment and progression exchanges materially connect roving activity to the wider care environment within that bounded integration role.

Scope

Contained Features and Established Owned Behaviour

Element	Type	Reference Id
Feature: Mobile Worker Task Dispatch (p. 348)	Feature	`FEAT-HSO-16`
Feature: Mobile Visit Status Tracking (p. 348)	Feature	`FEAT-HSO-17`
Function: Manage Mobile Service Capacity (p. 349)	Function	`HCM-FN-manage-mobile-service-capacity`
Function: Track Operational Staff Presence (p. 349)	Function	`HCM-FN-track-operational-staff-presence`
Service: Staff Presence Telemetry Query (p. 350)	Service	`HCM-SV-staff-presence-telemetry-query`
Function: Receive Mobile Visit Progression (p. 350)	Function	`HCM-FN-receive-mobile-visit-progression`
Function: Integrate Mobile Work Management (p. 351)	Function	`HCM-FN-integrate-mobile-work-management`

3.4.11.1. Feature: Mobile Worker Task Dispatch

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-HSO-16	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Mobile Staff Management (p. 347)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:522 (p. 576)	Linked RAD detail; exact locators in manifest.

Feature Overview

Mobile dispatch exchanges externally established assignment, dispatch and progression information with business mobile-work mechanisms. The retained name does not grant Harmonia authority to allocate clinical work, manage handover or own EMR worklists, and external task terminology does not redefine Synthetic Task.

Definition

Dispatch community care tasks and urgent visits to roving clinicians' mobile devices.

Boundary

The proposed replacement name remains unconfirmed. This entry preserves the existing identity and the human-adjudicated integration boundary.

Established Behavioural Context

Element	Relationship to this Feature
Function: Integrate Mobile Work Management (p. 351)	realises feature

3.4.11.2. Feature: Mobile Visit Status Tracking

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-HSO-17	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Mobile Staff Management (p. 347)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:524 (p. 576)	Linked RAD detail; exact locators in manifest.

Feature Overview

Mobile visit tracking receives arrival, progression and safety checkpoints from roving workers. It makes reported activity visible in the wider operational context without originating the clinical visit, deciding its clinical content or assuming that a checkpoint proves the intended clinical outcome.

Definition

Ingest arrival, visit progression, and safety check-in milestones from roving staff.

Established Behavioural Context

Element	Relationship to this Feature
Function: Receive Mobile Visit Progression (p. 350)	realises feature

3.4.11.3. Function: Manage Mobile Service Capacity

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-manage-mobile-service-capacity`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Mobile Staff Management (p. 347)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	hso:193 (p. 659)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Governed Description and Qualifications

Understands and manages available, committed and utilised mobile-service capacity in relation to workforce availability, geography, time, workload and applicable service demand.

Responsibility boundary: Staff presence and externally established assignments/visit progression contribute to capacity; they do not alone express it. Capacity coordination does not determine clinical work, own the source workforce roster or allocate underlying clinical duties.

3.4.11.4. Function: Track Operational Staff Presence

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-track-operational-staff-presence`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Mobile Staff Management (p. 347)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.

Name	Value	Comment / Description
Source	hso:197 (p. 659)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Governed Description and Qualifications

Tracks active on-duty presence and physical zone assignment for porters, phlebotomists, and roving nurses.

Feature association not established: `FEAT-HSO-16—MobileWorkerTaskDispatch` retains its Strategy identity with the integration boundary below. Presence/zone tracking remains distinct from mobile work-management integration; no replacement association to presence is assigned.

3.4.11.5. Service: Staff Presence Telemetry Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-staff-presence-telemetry-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Mobile Staff Management (p. 347)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	hso:198 (p. 659)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Governed Description and Qualifications

Discloses on-duty staff availability by hospital zone.

Feature association not established: `FEAT-HSO-16—MobileWorkerTaskDispatch` retains its Strategy identity with the integration boundary below. Presence/zone tracking remains distinct from mobile work-management integration; no replacement association to presence is assigned.

3.4.11.6. Function: Receive Mobile Visit Progression

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-receive-mobile-visit-progression`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Mobile Staff Management (p. 347)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	hso:200 (p. 659)	Linked RAD detail; exact locators in manifest.

Name	Value	Comment / Description
Derived From	Feature: Mobile Visit Status Tracking (p. 348)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Ingests arrival, visit-progression and safety check-in milestones from roving staff.

Derivation: `FEAT-HSO-17` directly requires visit-specific milestone ingestion. It remains distinct from presence, capacity assessment and work-management integration. Received progression can inform mobile capacity without determining clinical work or clinical completion.

3.4.11.7. Function: Integrate Mobile Work Management

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-integrate-mobile-work-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Mobile Staff Management (p. 347)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	hso:203 (p. 659)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Mobile Worker Task Dispatch (p. 348)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Exchanges externally established work/assignment, dispatch and operational progression/status information, with applicable worker/context information, between mobile work-management mechanisms and Harmonia operational coordination.

Derivation and boundary: The human adjudication of `FEAT-HSO-16` establishes integration with mobile work/task management. This responsibility is absent from presence observation and visit-specific milestone ingestion. It does not determine, allocate or manage the underlying clinical work, clinical handover or EMR worklists. Work Allocation & Dispatch retains non-clinical operational Work Order responsibility.

3.4.12. BEC: On-Call Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-on-call-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Harmonia-Relevant	Independent of status; never inherited.
Source	enabling:459 (p. 574); enabling:526 (p. 576); hso:211 (p. 660)	Linked RAD detail; exact locators in manifest.

Capability Overview

On-Call Management resolves the active provider for a specialty, service or hospital at the relevant time. It turns changing coverage information into usable routing context so that operational communication can reach the intended capacity. This differs from authoring the roster or deciding who is professionally qualified for the requested activity.

Boundary

Being on call does not by itself establish clinical privilege or assignment of a particular care responsibility.

Harmonia Architectural Relevance

Current provider resolution materially enables appropriate contact and routing across operational boundaries.

Scope

Contained Features and Established Owned Behaviour

Element	Type	Reference Id
Feature: Active On-Call Provider Resolution (p. 352)	Feature	FEAT-HSO-18
Function: Resolve Active On-Call Coverage (p. 353)	Function	HCM-FN-resolve-active-on-call-coverage
Service: On-Call Specialist Lookup (p. 353)	Service	HCM-SV-on-call-specialist-lookup

3.4.12.1. Feature: Active On-Call Provider Resolution

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-HSO-18	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: On-Call Management (p. 351)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:528 (p. 576)	Linked RAD detail; exact locators in manifest.

Feature Overview

Active on-call resolution identifies the current provider in the required specialty and hospital context. It supports correct routing at the relevant time without authoring the roster or treating current coverage as proof of qualification, privilege or responsibility acceptance.

Definition

Resolve currently active on-call specialists, registrars, and operational managers for a given specialty and hospital at any point in time.

Established Behavioural Context

Element	Relationship to this Feature
Function: Resolve Active On-Call Coverage (p. 353)	realises feature
Service: On-Call Specialist Lookup (p. 353)	associated feature

3.4.12.2. Function: Resolve Active On-Call Coverage

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-resolve-active-on-call-coverage`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: On-Call Management (p. 351)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	hso:214 (p. 660)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Active On-Call Provider Resolution (p. 352)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Resolves primary, secondary, and tertiary on-call clinical specialists for emergency escalation.

3.4.12.3. Service: On-Call Specialist Lookup

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-on-call-specialist-lookup`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: On-Call Management (p. 351)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	hso:215 (p. 660)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Active On-Call Provider Resolution (p. 352)	Established association only.

Governed Description and Qualifications

Discloses active on-call contacts for switchboard and emergency teams.

3.4.13. BEC: Work Allocation & Dispatch

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-work-allocation-dispatch`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Harmonia-Core	Independent of status; never inherited.
Source	enabling:460 (p. 574); enabling:530 (p. 577); hso:221 (p. 661)	Linked RAD detail; exact locators in manifest.

Capability Overview

Work Allocation & Dispatch progresses authorised operational work from a recognised requirement through matching, dispatch, acknowledgement and oversight. It distinguishes an instantiated work item from its assignment and from evidence of performance, allowing stalled activity to be identified and escalated. Examples such as cleaning or transport concern real-world support work, not technical message queues.

Intent

Make operational work progression accountable from requirement to reported outcome within applicable policy and role constraints.

Boundary

This assignment responsibility does not confer clinical work allocation, qualification or privilege. A technical receipt has only its interaction contract’s meaning.

Harmonia Architectural Relevance

Governed operational work progression is a substantive platform responsibility supporting the Core classification.

Scope

Contained Features and Established Owned Behaviour

Element	Type	Reference Id
Feature: Work Item Instantiation (p. 355)	Feature	`FEAT-HSO-19`
Feature: Worker Matching & Allocation (p. 355)	Feature	`FEAT-HSO-20`
Feature: Work Dispatch Delivery (p. 356)	Feature	`FEAT-HSO-21`
Feature: Work Progress Oversight & Escalation (p. 357)	Feature	`FEAT-HSO-22`
Function: Dispatch Operational Work Order (p. 357)	Function	`HCM-FN-dispatch-operational-work-order`
Service: Work Dispatch Service (p. 358)	Service	`HCM-SV-work-dispatch-service`
Function: Manage Operational Work Progression (p. 358)	Function	`HCM-FN-manage-operational-work-progression`

Element	Type	Reference Id
Service: Operational Work Status Query (p. 359)	Service	`HCM-SV-operational-work-status-query`
Process: Operational Work Progression Process (p. 359)	Process	`HCM-PR-operational-work-progression-process`

3.4.13.1. Feature: Work Item Instantiation

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-HSO-19`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Work Allocation & Dispatch (p. 354)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:532 (p. 577)	Linked RAD detail; exact locators in manifest.

Feature Overview

Work-item instantiation gives an authorised operational requirement an identifiable unit of work. It distinguishes recognising that support activity is needed from assigning it, dispatching it and recording its completion; a clinical trigger does not make the resulting support item clinical work authority.

Definition

Instantiate operational units of work from clinical triggers (e.g., bed cleaning required, specimen transport required).

Established Behavioural Context

Element	Relationship to this Feature
Function: Dispatch Operational Work Order (p. 357)	realises feature
Service: Work Dispatch Service (p. 358)	associated feature

3.4.13.2. Feature: Worker Matching & Allocation

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-HSO-20`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Work Allocation & Dispatch (p. 354)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.

Name	Value	Comment / Description
Source	enabling:533 (p. 577)	Linked RAD detail; exact locators in manifest.

Feature Overview

Worker matching relates an operational requirement to eligible roles, credentials, locations and availability under policy. It consumes those constraints rather than changing their authoritative records, and does not turn operational allocation into permission to decide clinical work.

Definition

Match work items against available staff roles, credentials, and locations based on allocation policies.

Established Behavioural Context

Element	Relationship to this Feature
Function: Dispatch Operational Work Order (p. 357)	realises feature

3.4.13.3. Feature: Work Dispatch Delivery

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-HS0-21	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Work Allocation & Dispatch (p. 354)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:534 (p. 577)	Linked RAD detail; exact locators in manifest.

Feature Overview

Dispatch delivery brings assigned operational work to the intended worker or team and retains acknowledgement evidence. Technical receipt and operational acknowledgement must remain distinguishable; neither alone proves that the work was performed or its intended outcome achieved.

Definition

Deliver work orders to target workers, devices, or team queues with technical and operational acknowledgement capture.

Established Behavioural Context

Element	Relationship to this Feature
Function: Dispatch Operational Work Order (p. 357)	realises feature

3.4.13.4. Feature: Work Progress Oversight & Escalation

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-HSO-22`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Work Allocation & Dispatch (p. 354)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:535 (p. 577)	Linked RAD detail; exact locators in manifest.

Feature Overview

Progress oversight identifies stalled or unacknowledged operational work against applicable timing expectations. Escalation serves completion management and does not itself constitute independent assurance or transfer authority over clinical decisions related to the support activity.

Definition

Monitor work progression against time-to-completion thresholds, escalating stalled or unacknowledged tasks.

Established Behavioural Context

Element	Relationship to this Feature
Function: Manage Operational Work Progression (p. 358)	realises feature
Service: Operational Work Status Query (p. 359)	associated feature

3.4.13.5. Function: Dispatch Operational Work Order

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-dispatch-operational-work-order`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Work Allocation & Dispatch (p. 354)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	hso:227 (p. 661)	Linked RAD detail; exact locators in manifest.

Name	Value	Comment / Description
Derived From	Feature: Work Item Instantiation (p. 355); Feature: Worker Matching & Allocation (p. 355); Feature: Work Dispatch Delivery (p. 356)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Allocates work requests to available wardspersons or support teams using applicable allocation policies, staff roles, credentials, location and skill, and delivers allocated work to the target worker, device or team queue.

3.4.13.6. Service: Work Dispatch Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-work-dispatch-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Work Allocation & Dispatch (p. 354)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	hso:228 (p. 661)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Work Item Instantiation (p. 355)	Established association only.

Governed Description and Qualifications

Delivers allocated work and captures distinct technical and operational acknowledgements; acknowledgement alone does not establish completion.

3.4.13.7. Function: Manage Operational Work Progression

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-manage-operational-work-progression`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Work Allocation & Dispatch (p. 354)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	hso:231 (p. 661)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Work Progress Oversight & Escalation (p. 357)	Established Feature realisation; not structural containment.
Governed Process	Process: Operational Work Progression Process (p. 359)	Established progression relationship.

Governed Description and Qualifications

Tracks execution states: *Queued → Assigned → Accepted → In-Progress → Completed / Aborted*.

3.4.13.8. Service: Operational Work Status Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-operational-work-status-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Work Allocation & Dispatch (p. 354)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	hso:232 (p. 661)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Work Progress Oversight & Escalation (p. 357)	Established association only.

Governed Description and Qualifications

Discloses task progression to requesting wards.

3.4.13.9. Process: Operational Work Progression Process

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-PR-operational-work-progression-process`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Work Allocation & Dispatch (p. 354)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	processes:218 (p. 618); processes:219 (p. 618)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Process Overview

Operational work progression makes the path from a recognised support requirement to assigned and reported work explicit. Acknowledgement and activity progress help distinguish a dispatch from actual performance and expose impediments requiring operational attention. This responsibility remains bounded to authorised support work rather than ownership of the clinician's worklist.

Governed Description and Qualifications

- **Owning Capability**: `WorkAllocation&Dispatch`
- **Process Purpose**: Governs the dispatch, assignment, execution, and completion of facility operational jobs (e.g., equipment moves, waste removal, linen supply).
- **State Progression Lifecycle**: Work Requested → Queued in Dispatch → Assigned to Worker → Job Accepted → Work In-Progress → Work Completed / Aborted
- **Key State Dispositions**:
  - `WorkRequested`: Facility support task submitted by clinical unit.
  - `QueuedinDispatch`: Job prioritized in operational queue.

- `AssignedtoWorker`: Job allocated to specific staff member or team.
- `JobAccepted`: Worker acknowledges dispatch on mobile device.
- `WorkIn-Progress`: Task actively being executed.
- `WorkCompleted/Aborted`: Job marked finished or formally cancelled with reason.

3.4.14. BEC: Clinical Training & Education

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-clinical-training-education`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Reference	Independent of status; never inherited.
Source	enabling:461 (p. 574)	Linked RAD detail; exact locators in manifest.

Capability Overview

Clinical Training & Education remains a Reference context for developing practitioners' knowledge and skills. The catalogue recognises its enterprise significance without assigning training delivery or a Feature decomposition to Harmonia. Its separation from qualification checks prevents a represented certificate or training record from being mistaken for the education activity itself.

Boundary

No platform training programme, assessment process or clinical teaching authority is established.

Harmonia Architectural Relevance

Reference standing retains enterprise context without a material assigned processing responsibility in this entry.

3.4.15. BEC: Clinical Competency Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-clinical-competency-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Reference	Independent of status; never inherited.

Name	Value	Comment / Description
Source	enabling:462 (p. 574)	Linked RAD detail; exact locators in manifest.

Capability Overview

Clinical Competency Management is retained as a Reference context concerning practitioner competence. It is distinguishable from checking supplied qualification or competency evidence before specialised work. The model does not establish that representing evidence makes Harmonia the authority that assesses or grants competence.

Boundary

No atomic Features or competency-assessment authority are allocated to this context.

Harmonia Architectural Relevance

The formal Reference classification remains separate from the relevance of operational qualification checking elsewhere.

3.4.16. BEC: Clinical Qualification Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-clinical-qualification-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Harmonia-Relevant	Independent of status; never inherited.
Source	enabling:463 (p. 574); enabling:539 (p. 577); hso:292 (p. 663); metamodel:397 (p. 595)	Linked RAD detail; exact locators in manifest.
Name Aliases	Clinical Credential Management	Same element only.

Capability Overview

Clinical Qualification Management checks professional and clinical qualification or competency context before specialised operational work is dispatched. It makes relevant standing available at the point where an operational action must respect it. The approved rename preserves the professional responsibility while distinguishing it from operational badges and physical access credentials.

Boundary

The historical name Clinical Credential Management is an alias only for this element. It is not equivalent to Credential Management, and role resolution alone establishes neither clinical nor operational privilege.

Harmonia Architectural Relevance

Qualification context materially constrains legitimate dispatch without making Harmonia the originating professional regulator or training authority.

Scope

Contained Features and Established Owned Behaviour

Element	Type	Reference Id
Feature: Operational Credential Check (p. 362)	Feature	FEAT-HS0-23
Function: Verify Clinical Qualification for Work Dispatch (p. 363)	Function	HCM-FN-verify-clinical-qualification-for-work-dispatch

3.4.16.1. Feature: Operational Credential Check

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-HS0-23	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Clinical Qualification Management (p. 361)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:541 (p. 577)	Linked RAD detail; exact locators in manifest.

Feature Overview

The qualification checkpoint considers active professional credentials and competency evidence before specialised dispatch. Its historical credential terminology does not make it the operational badge or physical-access responsibility of Credential Management, nor does a qualification check alone establish all required privilege.

Definition

Validate that a practitioner possesses active credentials and required competency certs prior to dispatching specialized work.

Established Behavioural Context

Element	Relationship to this Feature
Function: Verify Clinical Qualification for Work Dispatch (p. 363)	realises feature

3.4.16.2. Function: Verify Clinical Qualification for Work Dispatch

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-verify-clinical-qualification-for-work-dispatch`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Clinical Qualification Management (p. 361)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	hso:295 (p. 663)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Operational Credential Check (p. 362)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Validates that a practitioner possesses active clinical credentials and required competency certificates before dispatch of specialised work.

Derivation: `FEAT-HSO-23` belongs to Strategy Clinical Qualification Management. Provider Administration establishes professional qualification/registration and privilege information, but the dispatch-specific qualification/competency checkpoint was not expressed by that information management alone.

3.4.17. BEC: Clinical Supervision

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-clinical-supervision`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Reference	Independent of status; never inherited.
Source	enabling:464 (p. 574)	Linked RAD detail; exact locators in manifest.

Capability Overview

Clinical Supervision is a Reference context for the professional supervision of care activity. Its inclusion locates that enterprise responsibility without assigning a Harmonia supervision Feature or clinical-work mandate. Operational visibility into activity and qualifications can be useful context without becoming the professional act of supervising practice.

Boundary

Workflow oversight or platform escalation does not confer clinical supervision or responsibility for clinical adequacy.

Harmonia Architectural Relevance

Reference standing is preserved; no direct platform supervision responsibility is established.

3.4.18. BEC: Patient Transport

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-patient-transport`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Harmonia-Core	Independent of status; never inherited.
Source	enabling:465 (p. 574); enabling:543 (p. 577); hso:250 (p. 661)	Linked RAD detail; exact locators in manifest.

Capability Overview

Patient Transport coordinates the operational progression of movement requests, including the patient's applicable mobility constraints. Departure, transit and arrival checkpoints make the physical journey visible to participants. The responsibility supports internal and inter-facility movement without treating arrival as acceptance of clinical responsibility at the destination.

Boundary

Transport completion does not perform clinical handover or establish a care responsibility transition.

Harmonia Architectural Relevance

Transport request and dispatch progression are substantive assigned operational coordination responsibilities.

Scope

Contained Features and Established Owned Behaviour

Element	Type	Reference Id
Feature: Transport Request Ingestion (p. 365)	Feature	`FEAT-HSO-24`
Feature: Transport Dispatch & Progress Tracking (p. 365)	Feature	`FEAT-HSO-25`
Function: Manage Patient Transport Progression (p. 366)	Function	`HCM-FN-manage-patient-transport-progression`
Service: Patient Transport Tracking Service (p. 366)	Service	`HCM-SV-patient-transport-tracking-service`
Process: Patient Transport Process (p. 367)	Process	`HCM-PR-patient-transport-process`

3.4.18.1. Feature: Transport Request Ingestion

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-HSO-24	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Patient Transport (p. 364)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:545 (p. 577)	Linked RAD detail; exact locators in manifest.

Feature Overview

Transport ingestion establishes the requested movement and applicable clinical mobility constraints. It gives operational coordination the context needed to arrange transit without making the request itself a departure, arrival or accepted care transition.

Definition

Ingest internal (portering) and external (ambulance, inter-hospital) patient transfer requests with clinical mobility requirements.

Established Behavioural Context

Element	Relationship to this Feature
Function: Manage Patient Transport Progression (p. 366)	realises feature

3.4.18.2. Feature: Transport Dispatch & Progress Tracking

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-HSO-25	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Patient Transport (p. 364)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:546 (p. 577)	Linked RAD detail; exact locators in manifest.

Feature Overview

Transport progression connects dispatch, departure, transit and arrival evidence for a physical patient journey. The checkpoints make movement intelligible while preserving that reaching a destination does not establish clinical handover or assumption of responsibility there.

Definition

Dispatch transport orders, tracking departure, transit, and destination arrival checkpoints.

Established Behavioural Context

Element	Relationship to this Feature
Function: Manage Patient Transport Progression (p. 366)	realises feature
Service: Patient Transport Tracking Service (p. 366)	associated feature

3.4.18.3. Function: Manage Patient Transport Progression

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-manage-patient-transport-progression`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Patient Transport (p. 364)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	hso:253 (p. 661)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Transport Dispatch & Progress Tracking (p. 365); Feature: Transport Request Ingestion (p. 365)	Established Feature realisation; not structural containment.
Governed Process	Process: Patient Transport Process (p. 367)	Established progression relationship.

Governed Description and Qualifications

Coordinates intra-facility and inter-facility patient movements: *Transport Requested → Porter Dispatched → Patient Collected → In-Transit → Delivered at Destination → Handover Completed*.

Additional Feature responsibility: `FEAT-HSO-24–TransportRequestIngestion` is directly supported by the request stage of this same Function/Process. It ingests internal portering and external ambulance/inter-hospital requests with clinical mobility requirements; request ingestion is distinct from dispatch, transit and arrival.

3.4.18.4. Service: Patient Transport Tracking Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-patient-transport-tracking-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Patient Transport (p. 364)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	hso:254 (p. 661)	Linked RAD detail; exact locators in manifest.

Name	Value	Comment / Description
Associated Feature	Feature: Transport Dispatch & Progress Tracking (p. 365)	Established association only.

Governed Description and Qualifications

Provides real-time transit telemetry.

Additional Feature responsibility: `FEAT-HSO-24–TransportRequestIngestion` is directly supported by the request stage of this same Function/Process. It ingests internal portering and external ambulance/inter-hospital requests with clinical mobility requirements; request ingestion is distinct from dispatch, transit and arrival.

3.4.18.5. Process: Patient Transport Process

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-PR-patient-transport-process`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Patient Transport (p. 364)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	processes:233 (p. 618); processes:234 (p. 618)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Process Overview

Patient transport progression records the physical journey from a movement request through departure, transit and destination arrival. Its checkpoints allow operational participants to follow the journey with relevant mobility context intact. Arrival closes a movement concern only under its applicable scope; it does not establish clinical handover or destination acceptance of care responsibility.

Governed Description and Qualifications

- **Owning Capability**: `PatientTransport`
- **Process Purpose**: Governs the physical portering and transport of patients between hospital departments, wards, diagnostic suites, and external facilities.
- **State Progression Lifecycle**: Transport Requested → Porter Dispatched → Patient Collected → In-Transit → Delivered at Destination → Clinical Handover Complete
- **Key State Dispositions**:
  - `TransportRequested`: Clinical unit books wheelchair, bed, or ambulance transport.
  - `PorterDispatched`: Wardsperson assigned and en route to pickup location.
  - `PatientCollected`: Patient identity verified and transfer initiated.
  - `In-Transit`: Patient actively moving through facility corridors or ambulance transit.
  - `DeliveredatDestination`: Patient arrives at target diagnostic suite or ward.
  - `ClinicalHandoverComplete`: Safe custody and clinical briefing transferred to receiving staff.

3.4.19. BEC: Clinical Logistics Coordination

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-clinical-logistics-coordination`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Harmonia-Relevant	Independent of status; never inherited.
Source	enabling:466 (p. 574); enabling:548 (p. 577); hso:264 (p. 662)	Linked RAD detail; exact locators in manifest.

Capability Overview

Clinical Logistics Coordination integrates the movement and custody context of clinical material, notably specimen pickup, transport conditions and laboratory delivery. It keeps a material's journey connected to the fulfilment context requiring it. This enables clinical logistics to be followed without reducing specimen integrity to a delivery timestamp alone.

Boundary

Tracking material movement does not perform diagnostic interpretation or establish general enterprise supply-chain ownership.

Harmonia Architectural Relevance

Custody and transport information materially support diagnostic continuity and appropriate interpretation of specimen handling.

Scope

Contained Features and Established Owned Behaviour

Element	Type	Reference Id
Feature: Pathology Specimen Transport Tracking (p. 368)	Feature	`FEAT-HSO-26`
Function: Manage Specimen Transport Progression (p. 369)	Function	`HCM-FN-manage-specimen-transport-progression`
Service: Specimen Transit Tracking Service (p. 370)	Service	`HCM-SV-specimen-transit-tracking-service`
Process: Specimen Transport Process (p. 370)	Process	`HCM-PR-specimen-transport-process`

3.4.19.1. Feature: Pathology Specimen Transport Tracking

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-HSO-26`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.

Name	Value	Comment / Description
Owning Context	BEC: Clinical Logistics Coordination (p. 368)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:550 (p. 577)	Linked RAD detail; exact locators in manifest.

Feature Overview

Specimen transport tracking retains pickup, custody and condition context through laboratory delivery. It supports interpretation of the material's journey without treating a delivered package as proof of specimen suitability, completed analysis or an authorised diagnostic report.

Definition

Track pickup, custody chain, transport conditions, and laboratory delivery of clinical specimens.

Established Behavioural Context

Element	Relationship to this Feature
Function: Manage Specimen Transport Progression (p. 369)	realises feature
Service: Specimen Transit Tracking Service (p. 370)	associated feature

3.4.19.2. Function: Manage Specimen Transport Progression

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-manage-specimen-transport-progression`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Clinical Logistics Coordination (p. 368)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	hso:267 (p. 662)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Pathology Specimen Transport Tracking (p. 368)	Established Feature realisation; not structural containment.
Governed Process	Process: Specimen Transport Process (p. 370)	Established progression relationship.

Governed Description and Qualifications

Tracks the physical transit of pathology bio-specimens, blood products, and surgical trays: *Collected → Courier Picked Up → In-Transit → Laboratory Ingress Received*.

3.4.19.3. Service: Specimen Transit Tracking Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-specimen-transit-tracking-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Clinical Logistics Coordination (p. 368)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	hso:268 (p. 662)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Pathology Specimen Transport Tracking (p. 368)	Established association only.

Governed Description and Qualifications

Exposes chain-of-custody tracking.

3.4.19.4. Process: Specimen Transport Process

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-PR-specimen-transport-process`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Clinical Logistics Coordination (p. 368)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	processes:248 (p. 618); processes:249 (p. 618)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Process Overview

Specimen transport preserves the movement and custody account connecting collection context with laboratory receipt. It makes the journey of the material explainable rather than reducing it to a sent/received pair. Delivery remains distinct from specimen acceptance, diagnostic performance and the eventual professional interpretation of its evidence.

Governed Description and Qualifications

- **Owning Capability**: `ClinicalLogisticsCoordination`
- **Process Purpose**: Governs the physical courier chain-of-custody, transport, and delivery of pathology bio-specimens, blood products, and surgical biopsies.
- **State Progression Lifecycle**: Specimen Packaged → Courier Dispatched → Chain-of-Custody Signed → In-Transit (Monitored) → Lab Ingress Received → Specimen Accepted
- **Key State Dispositions**:
  - `SpecimenPackaged`: Clinical unit packages bio-sample with barcode manifest.
  - `CourierDispatched`: Courier route assigned for pickup.
  - `Chain-of-CustodySigned`: Courier scans barcode and accepts legal custody.

- `In-Transit(Monitored)`: Specimen in transit under temperature/time constraints.
- `LabIngressReceived`: Central pathology specimen reception logs physical receipt.
- `SpecimenAccepted`: Specimen integrity verified and passed to analytical track.

3.4.20. BEC: Discharge Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-discharge-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Harmonia-Core	Independent of status; never inherited.
Source	enabling:467 (p. 574); enabling:552 (p. 577); hso:275 (p. 662)	Linked RAD detail; exact locators in manifest.

Capability Overview

Discharge Management coordinates the reported milestones needed around discharge, including medication supply, transport and home-care confirmations. It makes readiness and impediments visible across disciplines and communicates departure information when physical exit occurs. The responsibility preserves a distinction between gathering confirmations and making the clinical decision that discharge is appropriate.

Boundary

Milestone completion does not transfer clinical responsibility or prove the receiving provider accepted it. Readiness, exit and clinical decision authority remain distinct.

Harmonia Architectural Relevance

Cross-team milestone coordination and departure notification are substantive assigned platform responsibilities.

Scope

Contained Features and Established Owned Behaviour

Element	Type	Reference Id
Feature: Discharge Milestone Tracking (p. 372)	Feature	`FEAT-HSO-27`
Feature: Discharge Readiness & Coordination Oversight (p. 372)	Feature	`FEAT-HSO-28`
Feature: Discharge Notification Dispatch (p. 373)	Feature	`FEAT-HSO-29`
Function: Manage Discharge Coordination Progression (p. 374)	Function	`HCM-FN-manage-discharge-coordination-progression`
Service: Discharge Readiness Telemetry (p. 374)	Service	`HCM-SV-discharge-readiness-telemetry`
Process: Discharge Progression Process (p. 374)	Process	`HCM-PR-discharge-progression-process`

3.4.20.1. Feature: Discharge Milestone Tracking

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-HSO-27	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Discharge Management (p. 371)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:554 (p. 577)	Linked RAD detail; exact locators in manifest.

Feature Overview

Discharge milestone tracking records confirmations contributed by the relevant clinical and operational participants. It makes outstanding requirements visible without allowing the tracking mechanism to declare clinical readiness or assume that every local milestone defines a universal discharge pathway.

Definition

Track clinical criteria-led discharge milestones (medications dispensed, transportation arranged, home care confirmed).

Established Behavioural Context

Element	Relationship to this Feature
Function: Manage Discharge Coordination Progression (p. 374)	realises feature

3.4.20.2. Feature: Discharge Readiness & Coordination Oversight

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-HSO-28	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Discharge Management (p. 371)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:555 (p. 577)	Linked RAD detail; exact locators in manifest.

Feature Overview

Readiness oversight brings multidisciplinary, home-care and transport confirmations into one coordination view. Aggregation supports the responsible participants' decisions but does not itself confer authority to discharge or establish acceptance of continuing care by another provider.

Definition

Track and aggregate multidisciplinary discharge milestones, home care confirmations, and transit readiness across care teams.

Established Behavioural Context

Element	Relationship to this Feature
Function: Manage Discharge Coordination Progression (p. 374)	realises feature
Service: Discharge Readiness Telemetry (p. 374)	associated feature

3.4.20.3. Feature: Discharge Notification Dispatch

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-HS0-29`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Discharge Management (p. 371)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:556 (p. 577)	Linked RAD detail; exact locators in manifest.

Feature Overview

Departure notification communicates the physical exit and associated record and place-state context to the applicable participants. It distinguishes departure information from earlier readiness and from the clinical decision, while retaining the separate question of responsibility actually assumed elsewhere.

Definition

Dispatch departure notifications, updated bed states, and finalized discharge summaries to external care providers upon physical patient exit.

Established Behavioural Context

Element	Relationship to this Feature
Function: Manage Discharge Coordination Progression (p. 374)	realises feature

3.4.20.4. Function: Manage Discharge Coordination Progression

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-manage-discharge-coordination-progression`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Discharge Management (p. 371)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	hso:279 (p. 662)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Discharge Readiness & Coordination Oversight (p. 372); Feature: Discharge Milestone Tracking (p. 372); Feature: Discharge Notification Dispatch (p. 373)	Established Feature realisation; not structural containment.
Governed Process	Process: Discharge Progression Process (p. 374)	Established progression relationship.

Governed Description and Qualifications

Synchronises multi-agency discharge readiness (**illustrative preparation; publication, authorisation and confirmed exit are distinct**): *Discharge Planning Initiated → Medications Reconciled → Transport Booked → Discharge Summary Finalised → Physically Departed*.

3.4.20.5. Service: Discharge Readiness Telemetry

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-discharge-readiness-telemetry`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Discharge Management (p. 371)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	hso:283 (p. 662)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Discharge Readiness & Coordination Oversight (p. 372)	Established association only.

Governed Description and Qualifications

Discloses discharge barrier checklists and planned departure times.

3.4.20.6. Process: Discharge Progression Process

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-PR-discharge-progression-process`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.

Name	Value	Comment / Description
Owning Context	BEC: Discharge Management (p. 371)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	processes:263 (p. 619); processes:264 (p. 619)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Process Overview

Discharge progression coordinates the confirmations and practical requirements around leaving care. Clinical readiness, medications, destination arrangements and transport are related but distinct contributions, and the source progression retains its stated local applicability. Operational coordination does not originate the clinical discharge decision or prove that another provider assumed responsibility.

Governed Description and Qualifications

- **Owning Capability**: `DischargeManagement`
- **Process Purpose**: Governs the multidisciplinary coordination of patient discharge planning, pharmacy reconciliation, transport, and community handover.
- **State Progression Lifecycle**: Discharge Planning Initiated → Clinical Readiness Confirmed → Medications Reconciled → Transport & Services Booked → Discharge Summary Signed → Physically Departed (Illustrative preparation; publication, authorisation and exit distinct)
- **Key State Dispositions**:
  - `DischargePlanningInitiated`: Estimated Date of Discharge (EDD) set on admission.
  - `ClinicalReadinessConfirmed`: Attending medical team declares patient fit for discharge.
  - `MedicationsReconciled`: Hospital pharmacy reconciles and dispenses discharge medications.
  - `Transport&ServicesBooked`: Community nursing, home equipment, and patient transport booked.
  - `DischargeSummarySigned`: Applicable signed summary is available/published and may be communicated before departure where the Business behaviour requires it; signing, finalisation, publication and discharge authorisation remain distinct.
  - `PhysicallyDeparted`: Confirmed physical exit triggers departure notifications, updated bed-state communication and finalised-summary dispatch to external care providers under `FEAT-HSO-29`; care-place turnover follows its own responsibility.
- **Timing reconciliation**: Preparation, information availability/publication, applicable discharge authorisation and physical exit are distinct. Earlier preparation/communication is permitted; it does not realise a Feature triggered by confirmed exit. This illustrated local pathway establishes neither universal publication timing nor a universal discharge state machine, and adds no inferred publication event or authorisation state.
- **Preserved uncertainty**: Summary signing and finalisation are not universal equivalents. Detailed applicability, discharge-authority workflows and timing beyond the established preparation/confirmed-exit distinction remain unestablished.

3.4.21. BEC: Credential Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-credential-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	hso:239 (p. 661)	Linked RAD detail; exact locators in manifest.

Capability Overview

Credential Management concerns operational badges and physical-access credentials used in authorised operational participation. It retains a distinct Domain03 responsibility even though Strategy has no matching catalogue entry. This prevents the word credential from merging physical-access context with the professional qualifications and competency evidence checked by Clinical Qualification Management.

Boundary

No Strategy correspondence or Feature association is inferred from name similarity. Operational credentials do not themselves establish professional clinical qualification or privilege.

Harmonia Architectural Relevance

Relevance remains Unresolved. Established operational behaviour provides context for possible enablement but does not authorise a formal classification.

Additional Notes / Unresolved Matters

Strategy Catalogue Correspondence: Domain03 establishes this Capability; Strategy contains no matching entry. No equivalence is inferred.

Scope

Contained Features and Established Owned Behaviour

Element	Type	Reference Id
Function: Verify Operational Access Credential (p. 377)	Function	`HCM-FN-verify-operational-access-credential`
Service: Operational Credential Checkpoint Service (p. 377)	Service	`HCM-SV-operational-credential-checkpoint-service`

3.4.21.1. Function: Verify Operational Access Credential

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-verify-operational-access-credential`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Credential Management (p. 376)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	hso:243 (p. 661)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Governed Description and Qualifications

Verifies physical access badges, specialty ward entry authorizations, and restricted area permissions.

3.4.21.2. Service: Operational Credential Checkpoint Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-operational-credential-checkpoint-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Credential Management (p. 376)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	hso:244 (p. 661)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Governed Description and Qualifications

Evaluates physical/operational access rights.

3.5. Intrinsic / Shared Enablement

Capability	Reference Id
BEC: Patient Clinical Record (p. 378)	`HCM-BEC-patient-clinical-record`
BEC: Health Information Exchange (p. 383)	`HCM-BEC-health-information-exchange`
BEC: Health Information Access (p. 391)	`HCM-BEC-health-information-access`

Capability	Reference Id
BEC: Health Information Communication (p. 395)	`HCM-BEC-health-information-communication`
BEC: Health Information Control (p. 398)	`HCM-BEC-health-information-control`
BEC: Clinical Knowledge Services (p. 404)	`HCM-BEC-clinical-knowledge-services`
BEC: Clinical Collaboration (p. 408)	`HCM-BEC-clinical-collaboration`
BEC: General Collaboration (p. 414)	`HCM-BEC-general-collaboration`
BEC: Workflow & Activity Coordination (p. 416)	`HCM-BEC-workflow-activity-coordination`
BEC: Calendar Management (p. 426)	`HCM-BEC-calendar-management`
BEC: Presentation Services (p. 430)	`HCM-BEC-presentation-services`
BEC: Information Design Governance (p. 432)	`HCM-BEC-information-design-governance`

3.5.1. BEC: Patient Clinical Record

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-patient-clinical-record`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Harmonia-Core	Independent of status; never inherited.
Source	enabling:565 (p. 578); enabling:608 (p. 579); ise:35 (p. 665)	Linked RAD detail; exact locators in manifest.

Capability Overview

Patient Clinical Record provides a governed longitudinal representation across encounters, providers and source systems. Assembly supplies the timeline, active access supports current care context, and durable preservation retains the history of what Harmonia received, knew or managed. These contributions are complementary: a responsive working view must not become the authority for durable history.

Intent

Make cross-institutional information usable over time without erasing source provenance or prior knowledge through correction.

Boundary

Preservation does not originate clinical facts. Active distributed access and authoritative durable preservation remain separate under AX-05.

Harmonia Architectural Relevance

Longitudinal assembly, active access and governed durable history are direct information-management responsibilities within the Core boundary.

Scope

Contained Features and Established Owned Behaviour

Element	Type	Reference Id
Feature: Longitudinal Health Record (LHR) Assembly (p. 379)	Feature	`FEAT-ISE-01`
Feature: Active Clinical Record Access (p. 380)	Feature	`FEAT-ISE-02`
Feature: Durable Clinical Record Preservation (p. 380)	Feature	`FEAT-ISE-03`
Function: Assemble Longitudinal Clinical Record (p. 381)	Function	`HCM-FN-assemble-longitudinal-clinical-record`
Service: Longitudinal Clinical Record Query (p. 381)	Service	`HCM-SV-longitudinal-clinical-record-query`
Function: Maintain Active Clinical Record (p. 382)	Function	`HCM-FN-maintain-active-clinical-record`
Service: Active Problem & Allergy Summary Query (p. 382)	Service	`HCM-SV-active-problem-allergy-summary-query`
Function: Preserve Durable Clinical Record (p. 383)	Function	`HCM-FN-preserve-durable-clinical-record`
Service: Historical Clinical Record Access (p. 383)	Service	`HCM-SV-historical-clinical-record-access`

3.5.1.1. Feature: Longitudinal Health Record (LHR) Assembly

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-ISE-01`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Patient Clinical Record (p. 378)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:610 (p. 579)	Linked RAD detail; exact locators in manifest.

Feature Overview

Longitudinal assembly relates contributions from disparate systems into a usable chronology. The unified view preserves source and subject context rather than turning consolidation into originating clinical authority or erasing differences in the standing of individual assertions.

Definition

Assemble disparate encounters, diagnostic reports, medications, and clinical documents into a unified, vendor-neutral longitudinal timeline.

Established Behavioural Context

Element	Relationship to this Feature
Function: Assemble Longitudinal Clinical Record (p. 381)	realises feature
Service: Longitudinal Clinical Record Query (p. 381)	associated feature

3.5.1.2. Feature: Active Clinical Record Access

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-ISE-02`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Patient Clinical Record (p. 378)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:611 (p. 579)	Linked RAD detail; exact locators in manifest.

Feature Overview

Active access supports responsive use of consolidated clinical summaries in ongoing care. The working representation remains reconstructable active state; speed and availability do not promote it to authoritative durable truth when persistence or convergence is unavailable.

Definition

Provide low-latency, active distributed access to consolidated patient clinical summaries.

Established Behavioural Context

Element	Relationship to this Feature
Function: Maintain Active Clinical Record (p. 382)	realises feature
Service: Active Problem & Allergy Summary Query (p. 382)	associated feature

3.5.1.3. Feature: Durable Clinical Record Preservation

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-ISE-03`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Patient Clinical Record (p. 378)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.

Name	Value	Comment / Description
Source	enabling:612 (p. 579)	Linked RAD detail; exact locators in manifest.

Feature Overview

Durable preservation retains the governed longitudinal representation and the history of what Harmonia received or managed. Corrections establish changed knowledge without retrospectively rewriting that history, and preservation neither originates clinical facts nor automatically establishes legal qualification of the record.

Definition

Durably maintain governed, vendor-neutral longitudinal clinical representation and preservation with verifiable integrity controls.

Established Behavioural Context

Element	Relationship to this Feature
Function: Preserve Durable Clinical Record (p. 383)	realises feature
Service: Historical Clinical Record Access (p. 383)	associated feature

3.5.1.4. Function: Assemble Longitudinal Clinical Record

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-assemble-longitudinal-clinical-record`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Patient Clinical Record (p. 378)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	ise:39 (p. 665)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Longitudinal Health Record (LHR) Assembly (p. 379)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Synthesises clinical documents, encounters, diagnostic reports, and medication histories across disparate source systems into a unified chronological clinical view.

3.5.1.5. Service: Longitudinal Clinical Record Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-longitudinal-clinical-record-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.

Name	Value	Comment / Description
Owning Context	BEC: Patient Clinical Record (p. 378)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	ise:40 (p. 665)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Longitudinal Health Record (LHR) Assembly (p. 379)	Established association only.

Governed Description and Qualifications

Discloses the comprehensive clinical timeline to authorised clinicians.

3.5.1.6. Function: Maintain Active Clinical Record

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-maintain-active-clinical-record`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Patient Clinical Record (p. 378)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	ise:42 (p. 665)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Active Clinical Record Access (p. 380)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Maintains active problem lists, current medication regimens, allergies, and open clinical alerts for active care coordination.

3.5.1.7. Service: Active Problem & Allergy Summary Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-active-problem-allergy-summary-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Patient Clinical Record (p. 378)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	ise:43 (p. 665)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Active Clinical Record Access (p. 380)	Established association only.

Governed Description and Qualifications

Exposes immediate active clinical summaries.

3.5.1.8. Function: Preserve Durable Clinical Record

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-preserve-durable-clinical-record`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Patient Clinical Record (p. 378)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	ise:45 (p. 665)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Durable Clinical Record Preservation (p. 380)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Preserves the governed longitudinal representation and durable, immutable, append-only history of what Harmonia received, knew, asserted or managed, with provenance and versioning. Corrections/supersession establish changed knowledge without retrospectively altering that history or acquiring originating clinical authority.

3.5.1.9. Service: Historical Clinical Record Access

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-historical-clinical-record-access`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Patient Clinical Record (p. 378)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	ise:46 (p. 665)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Durable Clinical Record Preservation (p. 380)	Established association only.

Governed Description and Qualifications

Provides governed historical record retrieval for authorised clinical and legal review; preservation does not itself establish legal qualification or originating authority.

3.5.2. BEC: Health Information Exchange

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-health-information-exchange`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.

Name	Value	Comment / Description
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Harmonia-Core	Independent of status; never inherited.
Source	enabling:566 (p. 578); enabling:614 (p. 579); ise:52 (p. 665); business:1225 (p. 557)	Linked RAD detail; exact locators in manifest.

Capability Overview

As a BEC, Health Information Exchange provides reusable information-movement enablement through submission, retrieval mediation, addressed distribution and syndication. The models distinguish sending to a participant, requesting information, deliberately distributing to named recipients and publishing changes to matching subscribers. Their common responsibility is the exchange context, including routing and delivery state, rather than the business meaning of every payload.

Boundary

The capability owns exchange transaction state without acquiring clinical-document ownership. Its name collision with the BC remains unresolved; exchange egress ends management of the emitted representation.

Harmonia Architectural Relevance

Multi-party exchange and per-destination delivery tracking are substantive Core responsibilities, bounded by the external contract and originating information authority.

Additional Notes / Unresolved Matters

Name Uniqueness: The same canonical name identifies distinct BC and BEC responsibilities. Global naming and the BC/BEC naming boundary conflict remains unresolved; capture does not merge identities or waive the rule.

Scope

Contained Features and Established Owned Behaviour

Element	Type	Reference Id
Feature: Direct Submission Ingestion & Delivery (p. 385)	Feature	`FEAT-ISE-04`
Feature: Clinical Record Retrieval Mediation (p. 385)	Feature	`FEAT-ISE-05`
Feature: Addressed Clinical Distribution (p. 386)	Feature	`FEAT-ISE-06`
Feature: Event-Driven Syndication (p. 387)	Feature	`FEAT-ISE-07`
Function: Receive Information Submission (p. 387)	Function	`HCM-FN-receive-information-submission`
Service: HIE Submission Gateway Service (p. 388)	Service	`HCM-SV-hie-submission-gateway-service`
Function: Mediate Information Retrieval (p. 388)	Function	`HCM-FN-mediate-information-retrieval`
Service: Federated Health Information Query (p. 389)	Service	`HCM-SV-federated-health-information-query`

Element	Type	Reference Id
Function: Distribute Addressed Information (p. 389)	Function	`HCM-FN-distribute-addressed-information`
Service: Addressed Document Delivery Service (p. 389)	Service	`HCM-SV-addressed-document-delivery-service`
Function: Syndicate Information Change (p. 390)	Function	`HCM-FN-syndicate-information-change`
Service: Clinical Event Syndication Stream (p. 390)	Service	`HCM-SV-clinical-event-syndication-stream`

3.5.2.1. Feature: Direct Submission Ingestion & Delivery

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-ISE-04`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Health Information Exchange (p. 383)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:616 (p. 579)	Linked RAD detail; exact locators in manifest.

Feature Overview

Direct submission supports a sender's deliberate push of information across an exchange boundary. It preserves the distinction between ingress or delivery acceptance and the recipient's business handling, under the actual contract rather than an assumed meaning of transport success.

Definition

Support push submissions of clinical documents and messages across boundaries.

Established Behavioural Context

Element	Relationship to this Feature
Function: Receive Information Submission (p. 387)	realises feature
Service: HIE Submission Gateway Service (p. 388)	associated feature

3.5.2.2. Feature: Clinical Record Retrieval Mediation

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-ISE-05`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Health Information Exchange (p. 383)	Established responsibility only; complete CT ancestry Unresolved.

Name	Value	Comment / Description
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:617 (p. 579)	Linked RAD detail; exact locators in manifest.

Feature Overview

Retrieval mediation connects a request with information held in participating repositories. It makes distributed retrieval possible without transferring repository authority, acquiring the clinical payload or treating a resolvable request as automatic permission to disclose.

Definition

Mediate query-based retrieval requests across external and internal repositories.

Established Behavioural Context

Element	Relationship to this Feature
Function: Mediate Information Retrieval (p. 388)	realises feature
Service: Federated Health Information Query (p. 389)	associated feature

3.5.2.3. Feature: Addressed Clinical Distribution

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-ISE-06`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Health Information Exchange (p. 383)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:618 (p. 579)	Linked RAD detail; exact locators in manifest.

Feature Overview

Addressed distribution sends information to explicitly designated recipients and tracks each destination's delivery separately. This preserves partial outcomes in a fan-out rather than collapsing several deliveries into a single success that hides a missing recipient.

Definition

Fan out clinical events and diagnostic reports to designated external and internal recipients with per-destination delivery tracking.

Established Behavioural Context

Element	Relationship to this Feature
Function: Distribute Addressed Information (p. 389)	realises feature
Service: Addressed Document Delivery Service (p. 389)	associated feature

3.5.2.4. Feature: Event-Driven Syndication

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-ISE-07`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Health Information Exchange (p. 383)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:619 (p. 579)	Linked RAD detail; exact locators in manifest.

Feature Overview

Syndication connects published changes with subscribers whose registered interests match them. It differs from named-recipient distribution in how consumers are selected, while retaining governed delivery and the distinction between notification and the subscriber's eventual business action.

Definition

Publish clinical and directory change events to subscribed systems matching interest filters.

Established Behavioural Context

Element	Relationship to this Feature
Function: Syndicate Information Change (p. 390)	realises feature
Service: Clinical Event Syndication Stream (p. 390)	associated feature

3.5.2.5. Function: Receive Information Submission

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-receive-information-submission`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Health Information Exchange (p. 383)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	ise:57 (p. 665)	Linked RAD detail; exact locators in manifest.

Name	Value	Comment / Description
Derived From	Feature: Direct Submission Ingestion & Delivery (p. 385)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Validates and ingests standards-compliant clinical messages from external health providers.

3.5.2.6. Service: HIE Submission Gateway Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-hie-submission-gateway-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Health Information Exchange (p. 383)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	ise:58 (p. 665)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Direct Submission Ingestion & Delivery (p. 385)	Established association only.

Governed Description and Qualifications

Ingress endpoint for regional data contributions.

3.5.2.7. Function: Mediate Information Retrieval

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-mediate-information-retrieval`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Health Information Exchange (p. 383)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	ise:60 (p. 665)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Clinical Record Retrieval Mediation (p. 385)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Federates queries across participating hospital and diagnostic node repositories.

3.5.2.8. Service: Federated Health Information Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-federated-health-information-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Health Information Exchange (p. 383)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	ise:61 (p. 665)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Clinical Record Retrieval Mediation (p. 385)	Established association only.

Governed Description and Qualifications

Resolves distributed record queries.

3.5.2.9. Function: Distribute Addressed Information

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-distribute-addressed-information`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Health Information Exchange (p. 383)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	ise:63 (p. 665)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Addressed Clinical Distribution (p. 386)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Directs point-to-point clinical documents to specific recipient practitioner endpoints.

3.5.2.10. Service: Addressed Document Delivery Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-addressed-document-delivery-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Health Information Exchange (p. 383)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	ise:64 (p. 665)	Linked RAD detail; exact locators in manifest.

Name	Value	Comment / Description
Associated Feature	Feature: Addressed Clinical Distribution (p. 386)	Established association only.

Governed Description and Qualifications

Securely delivers documents to designated recipient mailboxes.

3.5.2.11. Function: Syndicate Information Change

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-syndicate-information-change`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Health Information Exchange (p. 383)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	ise:66 (p. 665)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Event-Driven Syndication (p. 387)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Broadcasts event notifications and record updates to subscribed regional care network systems.

3.5.2.12. Service: Clinical Event Syndication Stream

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-clinical-event-syndication-stream`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Health Information Exchange (p. 383)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	ise:67 (p. 665)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Event-Driven Syndication (p. 387)	Established association only.

Governed Description and Qualifications

Emits publish-subscribe clinical event feeds.

3.5.3. BEC: Health Information Access

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-health-information-access`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Harmonia-Core	Independent of status; never inherited.
Source	enabling:567 (p. 578); enabling:621 (p. 579); ise:73 (p. 666)	Linked RAD detail; exact locators in manifest.
Supports	CT1: Individual Care Delivery; CT2: Individual Health Assessment; CT2: Clinical Determination; CT2: Individual Care Planning; CT2: Clinical Intervention; CT2: Individual Health Monitoring; CT2: Individual Care Review	Source-scoped; no inherited descendant edges.

Capability Overview

Health Information Access enables authorised discovery and retrieval across clinical information. It combines semantic search and discrete retrieval with access-qualified filtering so that a discoverable item is not automatically disclosable. It consumes established policy and consent determinations rather than making the search interface an alternative security authority.

Boundary

Retrieval alone does not realise the filtering Feature. Unestablished Feature associations remain unestablished even where the behaviour shares the same owner.

Harmonia Architectural Relevance

Responsive governed access is a direct platform responsibility. The approved support to Individual Care Delivery remains bounded and does not establish runtime dependencies for every clinical ability.

Scope

Contained Features and Established Owned Behaviour

Element	Type	Reference Id
Feature: Federated Clinical Query (p. 392)	Feature	`FEAT-ISE-08`
Feature: Access-Controlled Information Filtering (p. 392)	Feature	`FEAT-ISE-09`
Function: Search Health Information (p. 393)	Function	`HCM-FN-search-health-information`
Service: Clinical Information Search Service (p. 393)	Service	`HCM-SV-clinical-information-search-service`
Function: Retrieve Health Information (p. 394)	Function	`HCM-FN-retrieve-health-information`
Service: Clinical Resource Retrieval Service (p. 394)	Service	`HCM-SV-clinical-resource-retrieval-service`

Element	Type	Reference Id
Function: Apply Access-Qualified Result Filtering (p. 395)	Function	`HCM-FN-apply-access-qualified-result-filtering`

3.5.3.1. Feature: Federated Clinical Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-ISE-08`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Health Information Access (p. 391)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:623 (p. 579)	Linked RAD detail; exact locators in manifest.

Feature Overview

Federated query makes relevant clinical and directory information discoverable across distributed contexts using bounded criteria. Discovery is a governed information-access responsibility rather than authority to disclose every match or reinterpret the clinical meaning of the results.

Definition

Execute bounded, indexed queries across distributed clinical resources and directory graphs.

Established Behavioural Context

Element	Relationship to this Feature
Function: Search Health Information (p. 393)	realises feature
Service: Clinical Information Search Service (p. 393)	associated feature

3.5.3.2. Feature: Access-Controlled Information Filtering

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-ISE-09`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Health Information Access (p. 391)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:624 (p. 579)	Linked RAD detail; exact locators in manifest.

Feature Overview

Access-qualified filtering restricts the disclosed result to the verified caller's authority and applicable consent. The responsibility is more specific than retrieving a resource; the latter's shared owner and subject matter do not establish a Feature realisation relationship.

Definition

Filter search results dynamically according to requesting user authorizations and patient consent directives.

3.5.3.3. Function: Search Health Information

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-search-health-information`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Health Information Access (p. 391)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	ise:78 (p. 666)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Federated Clinical Query (p. 392)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Executes structured semantic searches across diagnostic reports, clinical notes, and medication histories.

3.5.3.4. Service: Clinical Information Search Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-clinical-information-search-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Health Information Access (p. 391)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	ise:79 (p. 666)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Federated Clinical Query (p. 392)	Established association only.

Governed Description and Qualifications

Discloses search capabilities to clinical user interfaces.

3.5.3.5. Function: Retrieve Health Information

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-retrieve-health-information`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Health Information Access (p. 391)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	ise:82 (p. 666)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Function Overview

Retrieval obtains discrete clinical information or attachments under the owning access responsibility. It is distinct from applying the access-qualified filtering required by FEAT-ISE-09. A shared owner and related data do not justify making that unresolved realisation association authoritative.

Governed Description and Qualifications

Fetches discrete clinical observations and original attachments.

Feature association not established: `FEAT-ISE-09—Access-ControlledInformationFiltering` requires access-controlled disclosure/filtering; retrieval of discrete information alone does not materially realise that responsibility. No replacement Feature is assigned.

3.5.3.6. Service: Clinical Resource Retrieval Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-clinical-resource-retrieval-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Health Information Access (p. 391)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	ise:83 (p. 666)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Governed Description and Qualifications

Delivers structured clinical information to requesting applications.

Feature association not established: `FEAT-ISE-09—Access-ControlledInformationFiltering` requires access-controlled disclosure/filtering; retrieval of discrete information alone does not materially realise that responsibility. No replacement Feature is assigned.

3.5.3.7. Function: Apply Access-Qualified Result Filtering

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-apply-access-qualified-result-filtering`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Health Information Access (p. 391)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	ise:85 (p. 666)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Function Overview

Result filtering changes what can be disclosed according to the verified context and applicable consent determinations. It applies established control outcomes without owning their policy. Its capability-scoped standing is retained instead of promoting the behaviour to a Feature simply to obtain traceability symmetry.

Governed Description and Qualifications

Redacts or masks search results and clinical content based on the caller's verified security context and patient consent directives.

3.5.4. BEC: Health Information Communication

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-health-information-communication`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Harmonia-Core	Independent of status; never inherited.
Source	enabling:568 (p. 578); enabling:626 (p. 579); ise:92 (p. 666)	Linked RAD detail; exact locators in manifest.

Capability Overview

Health Information Communication supplies standards-based transmission mediation between participating parties. It addresses protocol-facing transaction coordination and technical error handling while the business interaction retains its own meaning and authority. This differentiates the communication mechanism from the exchange model and from the clinical or operational purpose of the message.

Boundary

Transport mediation does not own every communicated interaction or payload. External contracts must preserve standards semantics and exclude private operational metadata.

Harmonia Architectural Relevance

Standards-conformant communication boundaries are a direct interoperability responsibility.

Scope

Contained Features and Established Owned Behaviour

Element	Type	Reference Id
Feature: Standards-Based Boundary Exchange (p. 396)	Feature	`FEAT-ISE-10`
Function: Mediate Standards-Based Health Information Communication (p. 397)	Function	`HCM-FN-mediate-standards-based-health-information-communication`
Service: Standards Communication Gateway Service (p. 397)	Service	`HCM-SV-standards-communication-gateway-service`

3.5.4.1. Feature: Standards-Based Boundary Exchange

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-ISE-10`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Health Information Communication (p. 395)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:628 (p. 579)	Linked RAD detail; exact locators in manifest.

Feature Overview

Boundary exchange preserves the standards contract understood by the external participant. Harmonia's internal management metadata must not leak into that contract, and publication must construct an allowed external representation without destructively changing the internally managed information.

Definition

Provide standards-conformant boundary exchange for healthcare messaging and resource representations.

Established Behavioural Context

Element	Relationship to this Feature
Function: Mediate Standards-Based Health Information Communication (p. 397)	realises feature
Service: Standards Communication Gateway Service (p. 397)	associated feature

3.5.4.2. Function: Mediate Standards-Based Health Information Communication

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-mediate-standards-based-health-information-communication`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Health Information Communication (p. 395)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	ise:97 (p. 666)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Standards-Based Boundary Exchange (p. 396)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Handles standards-based communication mediation, transaction coordination, and technical error handling.

3.5.4.3. Service: Standards Communication Gateway Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-standards-communication-gateway-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Health Information Communication (p. 395)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	ise:98 (p. 666)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Standards-Based Boundary Exchange (p. 396)	Established association only.

Governed Description and Qualifications

Protocol interface for external client integrations.

3.5.5. BEC: Health Information Control

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-health-information-control`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Harmonia-Core	Independent of status; never inherited.
Source	enabling:569 (p. 578); enabling:630 (p. 579); ise:104 (p. 667)	Linked RAD detail; exact locators in manifest.

Capability Overview

Health Information Control establishes the policy and security basis for governed information operations. Default-deny authority evaluation, consent constraints, security-context propagation and security-significant evidence have distinct purposes: deciding permission, retaining restrictions, carrying attributable operational context and preserving required evidence. The capability makes security intrinsic to the operation rather than optional presentation behaviour.

Boundary

Security context is operational context and does not automatically become resource content. Security-context validation has no invented one-to-one mapping to propagation, and audit recording is not an assurance conclusion.

Harmonia Architectural Relevance

Assigned policy enforcement and attributable context are substantive Core responsibilities across managed operations.

Scope

Contained Features and Established Owned Behaviour

Element	Type	Reference Id
Feature: Health Information Access & Consent Control (p. 399)	Feature	`FEAT-ISE-11`
Feature: Security Context Propagation (p. 399)	Feature	`FEAT-ISE-12`
Feature: Non-PHI Compliance Auditing (p. 400)	Feature	`FEAT-ISE-13`
Function: Evaluate Information Access Authority (p. 401)	Function	`HCM-FN-evaluate-information-access-authority`
Service: Policy Evaluation & Authorisation Service (p. 401)	Service	`HCM-SV-policy-evaluation-authorisation-service`
Function: Evaluate Consent Constraint (p. 401)	Function	`HCM-FN-evaluate-consent-constraint`
Service: Consent Enforcement Service (p. 402)	Service	`HCM-SV-consent-enforcement-service`
Function: Propagate Security Context (p. 402)	Function	`HCM-FN-propagate-security-context`
Service: Security Context Validation Service (p. 403)	Service	`HCM-SV-security-context-validation-service`
Function: Record Security-Significant Activity (p. 404)	Function	`HCM-FN-record-security-significant-activity`

Element	Type	Reference Id
Service: Security Audit Ingress (p. 404)	Service	`HCM-SV-security-audit-ingress`

3.5.5.1. Feature: Health Information Access & Consent Control

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-ISE-11`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Health Information Control (p. 398)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:632 (p. 580)	Linked RAD detail; exact locators in manifest.

Feature Overview

Access and consent control evaluates whether an information operation is legitimate in its practitioner, patient and purpose context. It supplies policy determinations consumed by access and presentation paths rather than allowing those consumers to establish authority from caller-supplied content.

Definition

Evaluate patient consent preferences, practitioner authority boundaries, and access control policies for managed clinical records.

Established Behavioural Context

Element	Relationship to this Feature
Function: Evaluate Information Access Authority (p. 401)	realises feature
Service: Policy Evaluation & Authorisation Service (p. 401)	associated feature

3.5.5.2. Feature: Security Context Propagation

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-ISE-12`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Health Information Control (p. 398)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:633 (p. 580)	Linked RAD detail; exact locators in manifest.

Feature Overview

Security-context propagation carries attributable operational authority across asynchronous boundaries. Its responsibility is preservation of established context, distinct from evaluating a consent constraint; shared security subject matter is insufficient to assign that Function to this Feature.

Definition

Propagate tamper-evident, attributable security context across asynchronous processing boundaries.

3.5.5.3. Feature: Non-PHI Compliance Auditing

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-ISE-13`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Health Information Control (p. 398)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:634 (p. 580)	Linked RAD detail; exact locators in manifest.

Feature Overview

Security-significant audit evidence makes governed actions attributable and verifiable while respecting PHI restrictions. The record concerns meaningful security facts under policy rather than routine implementation events, and recording it does not independently establish an assurance conclusion.

Definition

Generate tamper-evident, verifiable audit records of security-significant events without emitting unmasked PHI.

Established Behavioural Context

Element	Relationship to this Feature
Function: Record Security-Significant Activity (p. 404)	realises feature
Service: Security Audit Ingress (p. 404)	associated feature

3.5.5.4. Function: Evaluate Information Access Authority

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-evaluate-information-access-authority`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Health Information Control (p. 398)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	ise:108 (p. 667)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Health Information Access & Consent Control (p. 399)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Evaluates user role, organisation, patient relationship, purpose of use, and break-glass overrides against default-deny policies.

3.5.5.5. Service: Policy Evaluation & Authorisation Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-policy-evaluation-authorisation-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Health Information Control (p. 398)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	ise:109 (p. 667)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Health Information Access & Consent Control (p. 399)	Established association only.

Governed Description and Qualifications

Evaluates access requests across the platform.

3.5.5.6. Function: Evaluate Consent Constraint

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-evaluate-consent-constraint`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Health Information Control (p. 398)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.

Name	Value	Comment / Description
Source	ise:112 (p. 667)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Governed Description and Qualifications

Cross-references access requests against client consent directives and sensitive health category restrictions.

Feature association not established: `FEAT-ISE-12–SecurityContextPropagation` propagates attributable security context; evaluating consent constraints does not materially realise that responsibility. No replacement Feature is assigned.

3.5.5.7. Service: Consent Enforcement Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-consent-enforcement-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Health Information Control (p. 398)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	ise:113 (p. 667)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Governed Description and Qualifications

Returns masking and redaction instructions.

Feature association not established: `FEAT-ISE-12–SecurityContextPropagation` propagates attributable security context; evaluating consent constraints does not materially realise that responsibility. No replacement Feature is assigned.

3.5.5.8. Function: Propagate Security Context

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-propagate-security-context`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Health Information Control (p. 398)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	ise:115 (p. 667)	Linked RAD detail; exact locators in manifest.

Name	Value	Comment / Description
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Function Overview

Propagation preserves attributable operational security context across managed transactions. It provides continuity of context rather than independently deciding permission for every downstream action. The nearby validation Service has its own declared exposure; juxtaposition does not establish an exact one-to-one implementation relationship.

Governed Description and Qualifications

Generates and binds canonical, immutable security contexts to all intra-platform transactions.

Exposure qualification: Validation is the declared Service behaviour. Its juxtaposition with Propagate Security Context does not establish a one-to-one Function/Service relationship; the precise internal mapping remains unestablished.

3.5.5.9. Service: Security Context Validation Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-security-context-validation-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Health Information Control (p. 398)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	ise:116 (p. 667)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.
Internal Function Mapping	Unresolved	Precise internal Service-to-Function mapping is unestablished.

Service Overview

This Service lets a client obtain validation of transaction security credentials. The exposure is established even though its precise internal Function mapping is not. Validation must not be rewritten as context propagation merely to manufacture a symmetrical Function-to-Service pair.

Governed Description and Qualifications

Verifies transaction security credentials.

Exposure qualification: Validation is the declared Service behaviour. Its juxtaposition with Propagate Security Context does not establish a one-to-one Function/Service relationship; the precise internal mapping remains unestablished.

3.5.5.10. Function: Record Security-Significant Activity

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-record-security-significant-activity`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Health Information Control (p. 398)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	ise:119 (p. 667)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Non-PHI Compliance Auditing (p. 400)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Records tamper-evident audit evidence (who, what, when, why, patient MRN, policy outcome) without logging unmasked PHI.

3.5.5.11. Service: Security Audit Ingress

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-security-audit-ingress`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Health Information Control (p. 398)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	ise:120 (p. 667)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Non-PHI Compliance Auditing (p. 400)	Established association only.

Governed Description and Qualifications

Ingests audit events across all capabilities.

3.5.6. BEC: Clinical Knowledge Services

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-clinical-knowledge-services`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Harmonia-Relevant	Independent of status; never inherited.

Name	Value	Comment / Description
Source	enabling:570 (p. 578); enabling:636 (p. 580); ise:126 (p. 667)	Linked RAD detail; exact locators in manifest.

Capability Overview

Clinical Knowledge Services resolves governed clinical concepts and applies governed terminology mappings. It makes codes usable across local and standard semantic systems while retaining their source meaning. The responsibility is semantic reference and mapping enablement, distinct from formulating reusable clinical knowledge or applying that knowledge to an individual.

Boundary

Code translation does not establish equivalence beyond the governed map or confer clinical decision authority.

Harmonia Architectural Relevance

Concept resolution and mapping materially support meaningful information exchange and interpretation across regional systems.

Scope

Contained Features and Established Owned Behaviour

Element	Type	Reference Id
Feature: Clinical Concept Resolution (p. 405)	Feature	`FEAT-ISE-14`
Feature: Bidirectional Terminology Mapping (p. 406)	Feature	`FEAT-ISE-15`
Function: Resolve Clinical Concept (p. 407)	Function	`HCM-FN-resolve-clinical-concept`
Service: Terminology Concept Resolution Service (p. 407)	Service	`HCM-SV-terminology-concept-resolution-service`
Function: Map Clinical Concept (p. 407)	Function	`HCM-FN-map-clinical-concept`
Service: Terminology Mapping Service (p. 408)	Service	`HCM-SV-terminology-mapping-service`

3.5.6.1. Feature: Clinical Concept Resolution

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-ISE-14`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Clinical Knowledge Services (p. 404)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:638 (p. 580)	Linked RAD detail; exact locators in manifest.

Feature Overview

Concept resolution makes a coded clinical reference interpretable against its governed terminology. It validates the reference context without making a recognised code proof of clinical truth or giving the consuming system authority to redefine the concept.

Definition

Resolve local clinical codes against standard terminologies (SNOMED CT, LOINC, ICD-10).

Established Behavioural Context

Element	Relationship to this Feature
Function: Resolve Clinical Concept (p. 407)	realises feature
Service: Terminology Concept Resolution Service (p. 407)	associated feature

3.5.6.2. Feature: Bidirectional Terminology Mapping

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-ISE-15	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Clinical Knowledge Services (p. 404)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:639 (p. 580)	Linked RAD detail; exact locators in manifest.

Feature Overview

Terminology mapping applies governed correspondence between local and canonical semantic systems. Translation preserves the limitations and authority of the map; it does not justify silently treating related source concepts as identical or clinically adjudicating the underlying information.

Definition

Translate between legacy local code systems and canonical national healthcare profiles.

Established Behavioural Context

Element	Relationship to this Feature
Function: Map Clinical Concept (p. 407)	realises feature
Service: Terminology Mapping Service (p. 408)	associated feature

3.5.6.3. Function: Resolve Clinical Concept

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-resolve-clinical-concept`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Clinical Knowledge Services (p. 404)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	ise:130 (p. 667)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Clinical Concept Resolution (p. 405)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Validates concept identifiers and retrieves preferred clinical rubrics and hierarchy paths.

3.5.6.4. Service: Terminology Concept Resolution Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-terminology-concept-resolution-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Clinical Knowledge Services (p. 404)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	ise:131 (p. 667)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Clinical Concept Resolution (p. 405)	Established association only.

Governed Description and Qualifications

Exposes concept validation lookups.

3.5.6.5. Function: Map Clinical Concept

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-map-clinical-concept`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Clinical Knowledge Services (p. 404)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	ise:133 (p. 667)	Linked RAD detail; exact locators in manifest.

Name	Value	Comment / Description
Derived From	Feature: Bidirectional Terminology Mapping (p. 406)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Translates local lab codes and legacy terms to canonical SNOMED/LOINC codes using governed translation maps.

3.5.6.6. Service: Terminology Mapping Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-terminology-mapping-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Clinical Knowledge Services (p. 404)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	ise:134 (p. 667)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Bidirectional Terminology Mapping (p. 406)	Established association only.

Governed Description and Qualifications

Translates codes across terminology systems.

3.5.7. BEC: Clinical Collaboration

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-clinical-collaboration`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Harmonia-Core	Independent of status; never inherited.
Source	enabling:571 (p. 578); enabling:641 (p. 580); ise:140 (p. 668)	Linked RAD detail; exact locators in manifest.

Capability Overview

Clinical Collaboration provides secure multidisciplinary spaces bound to clinical context, including governed access to longitudinal information during discussion. Its purpose is contextual conversation between care participants rather than a separate source of authoritative clinical facts. A statement in a conversation remains discourse until explicitly authored through the governed record boundary.

Boundary

Channel metadata must remain free of PHI. The source Function's mention of managing clinical handovers conflicts with the metamodel's clinical-work boundary; both are preserved for review without assigning that authority here.

Harmonia Architectural Relevance

LHR-aware collaboration and controlled contextual queries are substantive Core responsibilities, distinct from clinical practice or record authorship.

Scope

Contained Features and Established Owned Behaviour

Element	Type	Reference Id
Feature: Care-Team Clinical Collaboration Coordination (p. 409)	Feature	`FEAT-ISE-16`
Feature: In-Conversation LHR Query Resolution (p. 410)	Feature	`FEAT-ISE-17`
Feature: Zero-PHI Collaboration Metadata Governance (p. 411)	Feature	`FEAT-ISE-18`
Function: Coordinate Care-Team Collaboration (p. 411)	Function	`HCM-FN-coordinate-care-team-collaboration`
Service: Clinical Collaboration Space Service (p. 412)	Service	`HCM-SV-clinical-collaboration-space-service`
Function: Resolve LHR Query within Collaboration (p. 412)	Function	`HCM-FN-resolve-lhr-query-within-collaboration`
Service: Collaboration Clinical Summary Resolution (p. 413)	Service	`HCM-SV-collaboration-clinical-summary-resolution`
Function: Govern Collaboration Metadata (p. 413)	Function	`HCM-FN-govern-collaboration-metadata`
Service: Collaboration Space Governance Service (p. 414)	Service	`HCM-SV-collaboration-space-governance-service`

3.5.7.1. Feature: Care-Team Clinical Collaboration Coordination

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-ISE-16`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Clinical Collaboration (p. 408)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:643 (p. 580)	Linked RAD detail; exact locators in manifest.

Feature Overview

Clinical collaboration coordination binds secure multidisciplinary exchange to the relevant encounter or clinical topic. It supports conversation among authorised participants without making discourse an authoritative observation or acquiring the clinical handover authority excluded by the metamodel.

Definition

Coordinate secure multidisciplinary clinical communication and consultation exchange bound to patient encounters or clinical topics.

Established Behavioural Context

Element	Relationship to this Feature
Function: Coordinate Care-Team Collaboration (p. 411)	realises feature
Service: Clinical Collaboration Space Service (p. 412)	associated feature

3.5.7.2. Feature: In-Conversation LHR Query Resolution

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-ISE-17`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Clinical Collaboration (p. 408)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:644 (p. 580)	Linked RAD detail; exact locators in manifest.

Feature Overview

In-conversation query supplies governed clinical summaries within an authorised collaboration context. The convenience of retrieving information during discussion does not weaken consent or access controls, and displaying a summary does not promote the surrounding conversation into the clinical record.

Definition

Resolve clinical data queries within an authorized collaboration context, presenting verified clinical summaries directly to care team members.

Established Behavioural Context

Element	Relationship to this Feature
Function: Resolve LHR Query within Collaboration (p. 412)	realises feature
Service: Collaboration Clinical Summary Resolution (p. 413)	associated feature

3.5.7.3. Feature: Zero-PHI Collaboration Metadata Governance

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-ISE-18`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Clinical Collaboration (p. 408)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:645 (p. 580)	Linked RAD detail; exact locators in manifest.

Feature Overview

Metadata governance keeps patient names, identifiers and sensitive detail out of collaboration titles, topics and aliases. Opaque references preserve useful contextual association without placing PHI in broadly visible channel metadata or diagnostic surfaces.

Definition

Ensure collaboration channel titles, topics, and aliases use opaque identifiers free of patient names, MRNs, or sensitive details.

Established Behavioural Context

Element	Relationship to this Feature
Function: Govern Collaboration Metadata (p. 413)	realises feature
Service: Collaboration Space Governance Service (p. 414)	associated feature

3.5.7.4. Function: Coordinate Care-Team Collaboration

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-coordinate-care-team-collaboration`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Clinical Collaboration (p. 408)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	ise:146 (p. 668); metamodel:175 (p. 589)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Care-Team Clinical Collaboration Coordination (p. 409)	Established Feature realisation; not structural containment.

Function Overview

This Function maintains clinical collaboration spaces and participant context, enabling team discourse around care. The governed source also says it manages clinical handovers, which

conflicts with the metamodel's explicit exclusion of that authority. The conflict is preserved for adjudication; this explanatory catalogue does not resolve it by quietly deleting either statement.

Governed Description and Qualifications

Manages care-team communication spaces, participant memberships, and clinical handovers.

Additional Notes / Unresolved Matters

Semantic Boundary Conflict: An existing description appears to conflict with a governing responsibility boundary; preserved for architectural review under AX-17 without reinterpretation.

3.5.7.5. Service: Clinical Collaboration Space Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-clinical-collaboration-space-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Clinical Collaboration (p. 408)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	ise:147 (p. 668)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Care-Team Clinical Collaboration Coordination (p. 409)	Established association only.

Governed Description and Qualifications

Manages team collaboration spaces and discussions.

3.5.7.6. Function: Resolve LHR Query within Collaboration

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-resolve-lhr-query-within-collaboration`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Clinical Collaboration (p. 408)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	ise:149 (p. 668)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: In-Conversation LHR Query Resolution (p. 410)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Contextually projects active patient summaries directly into collaborative clinical discussion channels.

3.5.7.7. Service: Collaboration Clinical Summary Resolution

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-collaboration-clinical-summary-resolution`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Clinical Collaboration (p. 408)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	ise:150 (p. 668)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: In-Conversation LHR Query Resolution (p. 410)	Established association only.

Governed Description and Qualifications

Embeds clinical summaries in collaboration feeds.

3.5.7.8. Function: Govern Collaboration Metadata

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-govern-collaboration-metadata`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Clinical Collaboration (p. 408)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	ise:154 (p. 668)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Zero-PHI Collaboration Metadata Governance (p. 411)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Enforces privacy policies, participant access gates, and zero-PHI space metadata rules.

3.5.7.9. Service: Collaboration Space Governance Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-collaboration-space-governance-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Clinical Collaboration (p. 408)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	ise:155 (p. 668)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Zero-PHI Collaboration Metadata Governance (p. 411)	Established association only.

Governed Description and Qualifications

Governs space lifecycles and membership.

3.5.8. BEC: General Collaboration

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-general-collaboration`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Harmonia-Relevant	Independent of status; never inherited.
Source	enabling:572 (p. 578); enabling:647 (p. 580); ise:161 (p. 668)	Linked RAD detail; exact locators in manifest.

Capability Overview

General Collaboration provides secure operational messaging for departmental, administrative and facility-support participants. It gives non-clinical coordination an interaction context without assuming every discussion requires a patient-linked clinical workspace. Its role is communication enablement rather than ownership of the support activity being discussed.

Boundary

Conversation does not assign operational authority, complete work or become an authoritative record merely through retention.

Harmonia Architectural Relevance

Operational messaging materially enables coordination while the participants' business responsibilities remain external to the collaboration mechanism.

Scope

Contained Features and Established Owned Behaviour

Element	Type	Reference Id
Feature: Operational Team Chat Enablement (p. 415)	Feature	FEAT-ISE-19
Function: Coordinate Operational Collaboration (p. 415)	Function	HCM-FN-coordinate-operational-collaboration
Service: Operational Collaboration Service (p. 416)	Service	HCM-SV-operational-collaboration-service

3.5.8.1. Feature: Operational Team Chat Enablement

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-ISE-19	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: General Collaboration (p. 414)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:649 (p. 580)	Linked RAD detail; exact locators in manifest.

Feature Overview

Operational chat provides secure communication for administrative and support teams. It enables participants to discuss work while leaving assignment, completion and the authority of operational decisions with the capabilities responsible for that activity.

Definition

Provide secure instant messaging channels for departmental, administrative, and facility support staff.

Established Behavioural Context

Element	Relationship to this Feature
Function: Coordinate Operational Collaboration (p. 415)	realises feature
Service: Operational Collaboration Service (p. 416)	associated feature

3.5.8.2. Function: Coordinate Operational Collaboration

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	HCM-FN-coordinate-operational-collaboration	Stable publication identity.
Architectural Status	Canonical	Established by RAD.

Name	Value	Comment / Description
Owning Context	BEC: General Collaboration (p. 414)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	ise:164 (p. 668)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Operational Team Chat Enablement (p. 415)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Provides communication spaces for facility logistics, disaster management teams, and administrative committees.

3.5.8.3. Service: Operational Collaboration Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-operational-collaboration-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: General Collaboration (p. 414)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	ise:165 (p. 668)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Operational Team Chat Enablement (p. 415)	Established association only.

Governed Description and Qualifications

Provides non-clinical operational messaging channels.

3.5.9. BEC: Workflow & Activity Coordination

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-workflow-activity-coordination`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Harmonia-Core	Independent of status; never inherited.
Source	enabling:573 (p. 578); enabling:651 (p. 580); ise:171 (p. 668)	Linked RAD detail; exact locators in manifest.

Capability Overview

Workflow & Activity Coordination provides governed progression for three distinct work meanings: human operational Work Orders, human review or decision To Dos, and non-human Synthetic Tasks. It supports dispatch, state and deadline oversight without collapsing those meanings into a generic technical task. The common coordination responsibility makes progression visible while preserving who has authority to perform or decide the underlying activity.

Intent

Keep assigned activity, acknowledgement, progress and outcome distinguishable across human and automated work.

Boundary

Coordination does not confer clinical work ownership. Synthetic Tasks are not external clinical worklists, and the incomplete stalled/failed Process paths remain unresolved.

Harmonia Architectural Relevance

Governed activity progression is a substantive Core responsibility; information traceability preserves archetype, work and undertaking distinctions without transferring outcome authority.

Scope

Contained Features and Established Owned Behaviour

Element	Type	Reference Id
Feature: Work Order Progression (p. 418)	Feature	FEAT-ISE-20
Feature: To Do Decision & Approval Coordination (p. 418)	Feature	FEAT-ISE-21
Feature: Synthetic Task Orchestration (p. 419)	Feature	FEAT-ISE-22
Feature: Workflow Timeout & Escalation Oversight (p. 419)	Feature	FEAT-ISE-23
Function: Coordinate Work Order (p. 420)	Function	HCM-FN-coordinate-work-order
Service: Work Order Coordination Service (p. 421)	Service	HCM-SV-work-order-coordination-service
Function: Coordinate To Do (p. 421)	Function	HCM-FN-coordinate-to-do
Service: To Do Management Service (p. 421)	Service	HCM-SV-to-do-management-service
Function: Coordinate Synthetic Task (p. 422)	Function	HCM-FN-coordinate-synthetic-task
Service: Synthetic Task Execution Service (p. 422)	Service	HCM-SV-synthetic-task-execution-service
Function: Supervise Activity Timeout / Escalation (p. 423)	Function	HCM-FN-supervise-activity-timeout-escalation
Service: Activity Escalation Telemetry (p. 423)	Service	HCM-SV-activity-escalation-telemetry
Process: Work Order Progression Process (p. 423)	Process	HCM-PR-work-order-progression-process
Process: To Do Progression Process (p. 424)	Process	HCM-PR-to-do-progression-process
Process: Synthetic Task Progression Process (p. 425)	Process	HCM-PR-synthetic-task-progression-process

3.5.9.1. Feature: Work Order Progression

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-ISE-20`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Workflow & Activity Coordination (p. 416)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:653 (p. 580)	Linked RAD detail; exact locators in manifest.

Feature Overview

Work Order progression coordinates human operational doing as a distinct work meaning. Dispatch, acceptance and completion describe the undertaking without making the workflow engine the authority over clinical work or treating a receipt as performance.

Definition

Manage execution states and dispatch of human operational work orders.

Established Behavioural Context

Element	Relationship to this Feature
Function: Coordinate Work Order (p. 420)	realises feature
Service: Work Order Coordination Service (p. 421)	associated feature

3.5.9.2. Feature: To Do Decision & Approval Coordination

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-ISE-21`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Workflow & Activity Coordination (p. 416)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:654 (p. 580)	Linked RAD detail; exact locators in manifest.

Feature Overview

To Do coordination supports human review, sign-off and decision activity. Its progression can expose whether the review item has been handled while leaving the meaning and authority of the professional or governance decision with the responsible participant.

Definition

Coordinate human clinical review items, sign-offs, and approval workflows.

Established Behavioural Context

Element	Relationship to this Feature
Function: Coordinate To Do (p. 421)	realises feature
Service: To Do Management Service (p. 421)	associated feature

3.5.9.3. Feature: Synthetic Task Orchestration

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-ISE-22	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Workflow & Activity Coordination (p. 416)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:655 (p. 580)	Linked RAD detail; exact locators in manifest.

Feature Overview

Synthetic Task orchestration governs non-human executable work and its sequencing. This technical work meaning remains distinct from human Work Orders, clinical worklists and external systems' generic task terminology; incomplete stalled/failed Process paths are not supplied by convention.

Definition

Execute non-human, automated platform activity units and sequence orchestration.

Established Behavioural Context

Element	Relationship to this Feature
Function: Coordinate Synthetic Task (p. 422)	realises feature
Service: Synthetic Task Execution Service (p. 422)	associated feature

3.5.9.4. Feature: Workflow Timeout & Escalation Oversight

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-ISE-23	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.

Name	Value	Comment / Description
Owning Context	BEC: Workflow & Activity Coordination (p. 416)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:656 (p. 580)	Linked RAD detail; exact locators in manifest.

Feature Overview

Timeout oversight observes deadlines and initiates the established coordination response to stalled activity. It is management of progression rather than an assurance conclusion, and escalation does not grant the coordinator authority over the clinical meaning of the underlying work.

Definition

Supervise operational deadlines, executing automated timeout notifications and supervisory escalations.

Established Behavioural Context

Element	Relationship to this Feature
Function: Supervise Activity Timeout / Escalation (p. 423)	realises feature
Service: Activity Escalation Telemetry (p. 423)	associated feature

3.5.9.5. Function: Coordinate Work Order

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-coordinate-work-order`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Workflow & Activity Coordination (p. 416)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	ise:181 (p. 668)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Work Order Progression (p. 418)	Established Feature realisation; not structural containment.
Governed Process	Process: Work Order Progression Process (p. 423)	Established progression relationship.

Governed Description and Qualifications

Coordinates operational task lifecycles and dispatches.

3.5.9.6. Service: Work Order Coordination Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-work-order-coordination-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Workflow & Activity Coordination (p. 416)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	ise:182 (p. 668)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Work Order Progression (p. 418)	Established association only.

Governed Description and Qualifications

Manages operational work progression.

3.5.9.7. Function: Coordinate To Do

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-coordinate-to-do`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Workflow & Activity Coordination (p. 416)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	ise:185 (p. 668)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: To Do Decision & Approval Coordination (p. 418)	Established Feature realisation; not structural containment.
Governed Process	Process: To Do Progression Process (p. 424)	Established progression relationship.

Governed Description and Qualifications

Coordinates human review/update/approval items, reminders and sign-off queues within Harmonia's operational responsibility; clinical requirements and decisions remain with accountable clinical authority.

3.5.9.8. Service: To Do Management Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-to-do-management-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Workflow & Activity Coordination (p. 416)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.

Name	Value	Comment / Description
Source	ise:186 (p. 668)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: To Do Decision & Approval Coordination (p. 418)	Established association only.

Governed Description and Qualifications

Provides To Do inbox and sign-off coordination without acquiring underlying clinical-work management.

3.5.9.9. Function: Coordinate Synthetic Task

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-coordinate-synthetic-task`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Workflow & Activity Coordination (p. 416)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	ise:189 (p. 668)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Synthetic Task Orchestration (p. 419)	Established Feature realisation; not structural containment.
Governed Process	Process: Synthetic Task Progression Process (p. 425)	Established progression relationship.

Governed Description and Qualifications

Orchestrates automated system workflows and operational progression.

3.5.9.10. Service: Synthetic Task Execution Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-synthetic-task-execution-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Workflow & Activity Coordination (p. 416)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	ise:190 (p. 668)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Synthetic Task Orchestration (p. 419)	Established association only.

Governed Description and Qualifications

Coordinates automated tasks.

3.5.9.11. Function: Supervise Activity Timeout / Escalation

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-supervise-activity-timeout-escalation`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Workflow & Activity Coordination (p. 416)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	ise:193 (p. 668)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Workflow Timeout & Escalation Oversight (p. 419)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Monitors SLAs and triggers escalations when clinical or operational deadlines elapse.

3.5.9.12. Service: Activity Escalation Telemetry

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-activity-escalation-telemetry`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Workflow & Activity Coordination (p. 416)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	ise:194 (p. 668)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Workflow Timeout & Escalation Oversight (p. 419)	Established association only.

Governed Description and Qualifications

Discloses SLA breaches.

3.5.9.13. Process: Work Order Progression Process

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-PR-work-order-progression-process`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Workflow & Activity Coordination (p. 416)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	processes:282 (p. 619); processes:283 (p. 619)	Linked RAD detail; exact locators in manifest.

Name	Value	Comment / Description
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Process Overview

Work Order progression describes a human undertaking physical or operational activity. Assignment, acknowledgement and outcome make the undertaking accountable without collapsing dispatch into completion. Its generic work meaning can support different operational contexts while leaving their domain authority and the clinical-work boundary intact.

Governed Description and Qualifications

- **Owning Capability**: `Workflow&ActivityCoordination`
- **Activity Archetype**: **Human Doing** — physical or operational task executed by healthcare staff.
- **State Progression Lifecycle**: Created → Ready → Dispatched → Accepted → In-Progress → Completed / Failed / Cancelled
- **Key State Dispositions**:
  - `Created`: Work order instantiated with scope, target, and required skills.
  - `Ready`: Preconditions satisfied; available for assignment.
  - `Dispatched`: Sent to target individual or worker group queue.
  - `Accepted`: Worker claims task.
  - `In-Progress`: Execution underway.
  - `Completed`: Task finished with recorded completion telemetry.

3.5.9.14. Process: To Do Progression Process

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-PR-to-do-progression-process`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Workflow & Activity Coordination (p. 416)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	processes:297 (p. 620); processes:298 (p. 620)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Process Overview

To Do progression coordinates a human review or decision item. Completion records progression of the item, while the significance and authority of the resulting judgement depend on the governed activity. A review being marked complete therefore does not independently establish clinical adequacy, signing authority or an assurance conclusion.

Governed Description and Qualifications

- **Owning Capability**: `Workflow&ActivityCoordination`

- **Activity Archetype**: **Human Reviewing / Updating / Deciding** — clinical review, document countersignature, or administrative authorization task.
- **State Progression Lifecycle**:

RAD diagram: textual source notation.

```
graph LR
    I["Issued"] --> B["In-Inbox"]
    B --> R["Opened / Under-Review"]
    R --> A["Actioned / Decided"]
    D["Dismissed / Delegated<br/>Disposition/position unresolved"]
    NOTE["Illustrative progression, not a normative transition model.<br/>Relationship of dismissal/delegation
    ↪  to action/decision is unestablished."]
    classDef note fill:#fff8dc,stroke:#8a6d3b,stroke-dasharray:5 5;
    class NOTE,D note;
```

- **Key State Dispositions**:
  - `Issued`: To Do generated (e.g., review abnormal potassium result).
  - `In-Inbox`: Displayed in practitioner's actionable task list.
  - `Opened/Under-Review`: Clinician inspecting relevant clinical context.
  - `Actioned/Decided`: Decision executed (e.g., signed off, order placed, acknowledged).
  - `Dismissed/Delegated`: Task reassigned to registrar or dismissed with comment.
- **Preserved uncertainty**: Whether dismissal/delegation is an alternative disposition, a subsequent action or a scoped variant remains unresolved; its disconnected diagram node establishes no transition or position in the illustrative progression.

3.5.9.15. Process: Synthetic Task Progression Process

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-PR-synthetic-task-progression-process`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Workflow & Activity Coordination (p. 416)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	processes:322 (p. 620); processes:323 (p. 620)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Process Overview

Synthetic Task progression concerns automated non-human execution, including waiting for a dependency and producing an output. It must remain distinct from human work orders and external clinical task lists. The disconnected stalled/failed outcome in the illustration leaves its relation to the success path unresolved; no reopening or post-completion failure transition is inferred.

Governed Description and Qualifications

- **Owning Capability**: `Workflow&ActivityCoordination`
- **Activity Archetype**: **Non-Human Executable Work** — automated system workflows, batch syndications, or policy evaluation tasks.
- **State Progression Lifecycle**:

RAD diagram: textual source notation.

```
graph LR
    D["Draft / Scheduled"] --> E["Executing"]
    E --> W["Awaiting-Dependency"]
    W --> C["Completed"]
    F["Progression Stalled / Failed<br/>Outcome/position unresolved"]
    NOTE["Illustrative progression, not a normative transition model.<br/>No Completed-to-Stalled/Failed
    ↪  transition is established."]
    classDef note fill:#fff8dc,stroke:#8a6d3b,stroke-dasharray:5 5;
    class NOTE,F note;
```

- **Key State Dispositions**:
  - `Draft/Scheduled`: Automated task instantiated and scheduled for processing.
  - `Executing`: System actively processing task.
  - `Awaiting-Dependency`: Paused awaiting asynchronous external reply or event.
  - `Completed`: Automated execution succeeded and output artifact produced.
  - `ProgressionStalled/Failed`: Transient fault triggers scheduled recovery attempt; permanent failure records failure evidence.
- **Preserved uncertainty**: Whether stalled/failed is an alternative outcome, reopening/post-completion behaviour or a presentation defect remains unresolved. Its disconnected diagram node does not make success and failure equivalent or establish post-completion transitions.

3.5.10. BEC: Calendar Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-calendar-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Harmonia-Core	Independent of status; never inherited.
Source	enabling:574 (p. 578); enabling:658 (p. 580); ise:200 (p. 669)	Linked RAD detail; exact locators in manifest.

Capability Overview

Calendar Management projects schedules and correlates events with temporal commitments. It brings rosters, on-call windows, facility calendars and appointment milestones into operational views without replacing their authoritative sources. This distinguishes the temporal context used for coordination from the business decisions that establish the schedule.

Boundary

Projection does not author the roster or confer qualification, privilege or clinical assignment authority.

Harmonia Architectural Relevance

Consistent temporal projections and event correlation are direct platform coordination responsibilities.

Scope

Contained Features and Established Owned Behaviour

Element	Type	Reference Id
Feature: Schedule Projection Integration (p. 427)	Feature	FEAT-ISE-24
Feature: Temporal Event Correlation (p. 428)	Feature	FEAT-ISE-25
Function: Project Operational Schedule (p. 428)	Function	HCM-FN-project-operational-schedule
Service: Unified Calendar Projection Query (p. 429)	Service	HCM-SV-unified-calendar-projection-query
Function: Correlate Activity with Temporal Context (p. 429)	Function	HCM-FN-correlate-activity-with-temporal-context
Service: Temporal Context Resolution (p. 429)	Service	HCM-SV-temporal-context-resolution

3.5.10.1. Feature: Schedule Projection Integration

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-ISE-24	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Calendar Management (p. 426)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:660 (p. 581)	Linked RAD detail; exact locators in manifest.

Feature Overview

Schedule projection makes external roster, on-call and facility calendars available in operational views. It preserves their temporal usefulness without making the projection the authoritative schedule or granting authority to alter staffing and coverage decisions.

Definition

Project clinician rosters, on-call schedules, and facility operating calendars into operational views.

Established Behavioural Context

Element	Relationship to this Feature
Function: Project Operational Schedule (p. 428)	realises feature
Service: Unified Calendar Projection Query (p. 429)	associated feature

3.5.10.2. Feature: Temporal Event Correlation

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-ISE-25`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Calendar Management (p. 426)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:661 (p. 581)	Linked RAD detail; exact locators in manifest.

Feature Overview

Temporal correlation relates incoming events to the relevant appointment or calendar milestone. Association provides context for interpretation and coordination without assuming that temporal proximity proves attendance, fulfilment or the clinical outcome expected at that time.

Definition

Correlate incoming clinical events with scheduled appointments and operational calendar milestones.

Established Behavioural Context

Element	Relationship to this Feature
Function: Correlate Activity with Temporal Context (p. 429)	realises feature
Service: Temporal Context Resolution (p. 429)	associated feature

3.5.10.3. Function: Project Operational Schedule

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-project-operational-schedule`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Calendar Management (p. 426)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	ise:205 (p. 669)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Schedule Projection Integration (p. 427)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Aggregates and overlays multidisciplinary schedules onto a unified temporal view.

3.5.10.4. Service: Unified Calendar Projection Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-unified-calendar-projection-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Calendar Management (p. 426)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	ise:206 (p. 669)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Schedule Projection Integration (p. 427)	Established association only.

Governed Description and Qualifications

Discloses consolidated temporal schedules.

3.5.10.5. Function: Correlate Activity with Temporal Context

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-correlate-activity-with-temporal-context`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Calendar Management (p. 426)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	ise:208 (p. 669)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Temporal Event Correlation (p. 428)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Associates clinical events with shift boundaries and appointment slots.

3.5.10.6. Service: Temporal Context Resolution

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-temporal-context-resolution`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Calendar Management (p. 426)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	ise:209 (p. 669)	Linked RAD detail; exact locators in manifest.

Name	Value	Comment / Description
Associated Feature	Feature: Temporal Event Correlation (p. 428)	Established association only.

Governed Description and Qualifications

Resolves operational shifts.

3.5.11. BEC: Presentation Services

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-presentation-services`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Harmonia-Core	Independent of status; never inherited.
Source	enabling:575 (p. 578); enabling:663 (p. 581); ise:215 (p. 670)	Linked RAD detail; exact locators in manifest.

Capability Overview

Presentation Services supplies contextual human interaction through clinical timelines, directory views and operational dashboards. It makes governed information usable by people while consuming the identity, security and clinical context established elsewhere. The responsibility is the interaction and presentation experience rather than a new authority for the information shown.

Boundary

Presentation does not own authentication, credential storage, consent decisions, EMPI reconciliation or authoritative clinical state. A displayed value is not made authoritative by its appearance in a dashboard.

Harmonia Architectural Relevance

Decoupled contextual interaction is a substantive Core responsibility that preserves the established information-access and control boundaries.

Scope

Contained Features and Established Owned Behaviour

Element	Type	Reference Id
Feature: Longitudinal Clinical & Operational Presentation (p. 431)	Feature	`FEAT-ISE-26`
Function: Present Contextual Health and Operational Information (p. 431)	Function	`HCM-FN-present-contextual-health-and-operational-information`
Service: Contextual Presentation Service (p. 432)	Service	`HCM-SV-contextual-presentation-service`

3.5.11.1. Feature: Longitudinal Clinical & Operational Presentation

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`FEAT-ISE-26`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Presentation Services (p. 430)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:665 (p. 581)	Linked RAD detail; exact locators in manifest.

Feature Overview

Contextual presentation makes clinical timelines, directories and operational information usable for people. It consumes established identity and security context while preserving the originating authority of what is shown; a user interface does not become an alternative persistence or policy boundary.

Definition

Deliver contextual clinical timelines, directory administration views, and operational management dashboards.

Established Behavioural Context

Element	Relationship to this Feature
Function: Present Contextual Health and Operational Information (p. 431)	realises feature
Service: Contextual Presentation Service (p. 432)	associated feature

3.5.11.2. Function: Present Contextual Health and Operational Information

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-present-contextual-health-and-operational-information`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Presentation Services (p. 430)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	ise:220 (p. 670)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Longitudinal Clinical & Operational Presentation (p. 431)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Composes clinical summaries, work queues, and patient records into responsive role-based user interfaces.

3.5.11.3. Service: Contextual Presentation Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-contextual-presentation-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Presentation Services (p. 430)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	ise:221 (p. 670)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Longitudinal Clinical & Operational Presentation (p. 431)	Established association only.

Governed Description and Qualifications

Delivers contextual health and operational information for presentation.

3.5.12. BEC: Information Design Governance

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-information-design-governance`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Harmonia-Core	Independent of status; never inherited.
Source	enabling:576 (p. 578); enabling:667 (p. 581); ise:227 (p. 670)	Linked RAD detail; exact locators in manifest.

Capability Overview

Information Design Governance maintains canonical definitions, semantic bindings, exchange profiles and the conformance rules applied to representations. It makes meaning and constraints available across regional information flows so that an accepted technical format is not mistaken for semantic correctness. Governance and verification remain distinguishable from the clinical facts represented.

Boundary

Consumers of definitions do not acquire their semantic authority, and conformance checking does not establish clinical truth or permission to disclose.

Harmonia Architectural Relevance

Governed semantics and representation conformance are direct platform responsibilities that constrain both internal use and boundary exchange.

Scope

Contained Features and Established Owned Behaviour

Element	Type	Reference Id
Feature: Information Standards & Semantic Governance (p. 433)	Feature	FEAT-ISE-27
Feature: Semantic Conformance Verification (p. 434)	Feature	FEAT-ISE-28
Function: Govern Information Semantics (p. 435)	Function	HCM-FN-govern-information-semantics
Service: Semantic Model Definition Service (p. 435)	Service	HCM-SV-semantic-model-definition-service
Function: Govern Information Definition (p. 435)	Function	HCM-FN-govern-information-definition
Service: Information Dictionary Query (p. 436)	Service	HCM-SV-information-dictionary-query
Function: Govern Terminology Binding (p. 436)	Function	HCM-FN-govern-terminology-binding
Service: Terminology Binding Specification Lookup (p. 437)	Service	HCM-SV-terminology-binding-specification-lookup
Function: Govern Exchange Profile (p. 437)	Function	HCM-FN-govern-exchange-profile
Service: Exchange Profile Specification Query (p. 437)	Service	HCM-SV-exchange-profile-specification-query
Function: Verify Semantic Conformance (p. 438)	Function	HCM-FN-verify-semantic-conformance
Service: Semantic Conformance Verification Service (p. 438)	Service	HCM-SV-semantic-conformance-verification-service

3.5.12.1. Feature: Information Standards & Semantic Governance

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-ISE-27	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Information Design Governance (p. 432)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:669 (p. 581)	Linked RAD detail; exact locators in manifest.

Feature Overview

Semantic governance provides canonical definitions, bindings and profiles through which regional representations can be understood consistently. Its purpose concerns meaning and constraints rather than a particular schema technology, and consuming a definition does not transfer its governing authority.

Definition

Govern canonical clinical data definitions, terminology bindings, and exchange profiles across regional systems.

Established Behavioural Context

Element	Relationship to this Feature
Function: Govern Information Semantics (p. 435)	realises feature
Service: Semantic Model Definition Service (p. 435)	associated feature

3.5.12.2. Feature: Semantic Conformance Verification

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	FEAT-ISE-28	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Type	FT / Feature	Not a Capability Tier.
Owning Context	BEC: Information Design Governance (p. 432)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	enabling:670 (p. 581)	Linked RAD detail; exact locators in manifest.

Feature Overview

Conformance verification evaluates a representation against published semantic rules and constraints. It distinguishes meaningful conformity from simple parseability while retaining that a conformant message is not necessarily clinically true, authoritative or permitted for disclosure.

Definition

Verify inbound and outbound representations against published semantic rules and conformance constraints.

Established Behavioural Context

Element	Relationship to this Feature
Function: Verify Semantic Conformance (p. 438)	realises feature
Service: Semantic Conformance Verification Service (p. 438)	associated feature

3.5.12.3. Function: Govern Information Semantics

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-govern-information-semantics`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Information Design Governance (p. 432)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	ise:231 (p. 670)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Information Standards & Semantic Governance (p. 433)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Authors and validates canonical data concepts and entity relationships.

3.5.12.4. Service: Semantic Model Definition Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-semantic-model-definition-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Information Design Governance (p. 432)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	ise:232 (p. 670)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Information Standards & Semantic Governance (p. 433)	Established association only.

Governed Description and Qualifications

Discloses canonical model definitions.

3.5.12.5. Function: Govern Information Definition

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-govern-information-definition`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Information Design Governance (p. 432)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	ise:234 (p. 670)	Linked RAD detail; exact locators in manifest.

Name	Value	Comment / Description
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Governed Description and Qualifications

Maintains governed data element dictionaries, data types, and structural rules.

3.5.12.6. Service: Information Dictionary Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-information-dictionary-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Information Design Governance (p. 432)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	ise:235 (p. 670)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Governed Description and Qualifications

Provides data dictionary specifications.

3.5.12.7. Function: Govern Terminology Binding

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-govern-terminology-binding`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Information Design Governance (p. 432)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	ise:237 (p. 670)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Governed Description and Qualifications

Binds canonical data elements to authoritative terminology value sets.

3.5.12.8. Service: Terminology Binding Specification Lookup

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-terminology-binding-specification-lookup`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Information Design Governance (p. 432)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	ise:238 (p. 670)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Governed Description and Qualifications

Discloses value set bindings.

3.5.12.9. Function: Govern Exchange Profile

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-govern-exchange-profile`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Information Design Governance (p. 432)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	ise:240 (p. 670)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Governed Description and Qualifications

Defines standards-based interchange profiles and message constraints.

3.5.12.10. Service: Exchange Profile Specification Query

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-exchange-profile-specification-query`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Information Design Governance (p. 432)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.

Name	Value	Comment / Description
Source	ise:241 (p. 670)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Governed Description and Qualifications

Discloses interoperability schemas.

3.5.12.11. Function: Verify Semantic Conformance

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-verify-semantic-conformance`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Information Design Governance (p. 432)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	ise:243 (p. 670)	Linked RAD detail; exact locators in manifest.
Derived From	Feature: Semantic Conformance Verification (p. 434)	Established Feature realisation; not structural containment.

Governed Description and Qualifications

Validates candidate messages against canonical governance schemas.

3.5.12.12. Service: Semantic Conformance Verification Service

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-semantic-conformance-verification-service`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Information Design Governance (p. 432)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	ise:244 (p. 670)	Linked RAD detail; exact locators in manifest.
Associated Feature	Feature: Semantic Conformance Verification (p. 434)	Established association only.

Governed Description and Qualifications

Validates representations for compliance.

3.6. Contextual view unestablished

Capability	Reference Id
BEC: Assurance Design (p. 439)	`HCM-BEC-assurance-design`
BEC: Assurance Criteria Management (p. 442)	`HCM-BEC-assurance-criteria-management`
BEC: Governed Assurance (p. 443)	`HCM-BEC-governed-assurance`

3.6.1. BEC: Assurance Design

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-assurance-design`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	enabling:680 (p. 581)	Linked RAD detail; exact locators in manifest.
Supports	CT1: Health Service Assurance	Source-scoped; no inherited descendant edges.

Capability Overview

Assurance Design determines how satisfaction of a governed requirement or expected behaviour will be assured. It makes the disposition, criteria, evidence and evaluation expectations explicit alongside behaviour and failure/recovery design. Its output is a basis for assurance rather than proof that the implemented behaviour has satisfied the requirement.

Definition

The capability to determine and define how satisfaction of a governed requirement, constraint or expected behaviour will be assured, including its assurance disposition, applicable criteria, required evidence and evaluation expectations.

Boundary

Designing assurance does not confer approval authority, define the underlying obligation or allocate a runtime component. No Feature decomposition or CT ancestry is established.

Harmonia Architectural Relevance

Formal classification remains Unresolved. Its approved support to Health Service Assurance establishes purpose and Enterprise Capability contributions without adjudicating this separate relevance dimension.

Additional Notes / Unresolved Matters

Contextual View: No contextual-view placement is established.

Feature Decomposition: No Feature decomposition is established.

Scope

Contained Features and Established Owned Behaviour

Element	Type	Reference Id
Function: Design Service Assurance (p. 440)	Function	`HCM-FN-design-service-assurance`
Process: Assurance Process Design (p. 441)	Process	`HCM-PR-assurance-process-design`

3.6.1.1. Function: Design Service Assurance

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-design-service-assurance`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Assurance Design (p. 439)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	assurance:71 (p. 625)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Function Overview

Designing service assurance turns a requirement's assurance question into an explicit disposition and evaluation basis. The modeller determines how satisfaction is to be assessed, not what the authoritative requirement should mean or who may approve it. Design completion is not evidence that the implemented behaviour satisfies that requirement.

Governed Description and Qualifications

- **Canonical Name**: `DesignServiceAssurance`.
- **Definition**: Defines how satisfaction of a governed requirement, constraint or expected behaviour is to be independently established, including the applicable assurance disposition, assurance criteria, required evidence and evaluation expectations.
- **Boundary**: Concerns assurance design. It does not establish the underlying governance requirement, grant authority to it, perform the resulting assurance, manage the subject activity or prescribe application or implementation mechanisms. Modelling the resulting Assurance Definition does not grant approval authority. The Strategy peer relationship to behaviour design and failure/recovery design creates no additional Capability or Function.

Additional Notes / Unresolved Matters

Contextual View: No contextual-view placement is established.

3.6.1.2. Process: Assurance Process Design

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-PR-assurance-process-design`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Assurance Design (p. 439)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	processes:355 (p. 621); processes:358 (p. 621)	Linked RAD detail; exact locators in manifest.
Participating Functions	Function: Design Service Assurance (p. 440); Function: Manage Assurance Criteria (p. 443)	Participation retains each Function owner.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Process Overview

Assurance design establishes a reusable basis for how satisfaction will be evaluated, rather than running the evaluation itself. Criteria-management participation retains its separate owner and does not create an exchange Service or a shared criteria lifecycle. An Assurance Definition can be reused, so each execution need not imply a freshly performed Design Process.

Governed Description and Qualifications

- **Canonical Name**: `AssuranceProcessDesign`.
- **Owning Capability**: Assurance Design.
- **Process Purpose**: Establishes and maintains how satisfaction of a governed requirement, constraint or expected behaviour is to be assured.
- **Principal Performing Role**: Service Assurance Modeller.
- **Participating Functions**: Design Service Assurance and Manage Assurance Criteria. Manage Assurance Criteria retains ownership within Assurance Criteria Management; its participation does not subsume that Capability's criteria lifecycle or transfer its information responsibility to Assurance Design.
- **Conceptual Output**: Assurance Definition, retaining its conceptual Business Architecture output/boundary treatment.
- **Boundary**: Governance Authority establishes what is authoritative. Design does not grant Service Assurance Modeller approval authority. Detailed Assurance Definition lifecycle, approval/publication workflows, criteria lifecycle and information modelling remain unresolved. Participation of the two Functions does not establish a cross-capability lifecycle or a new exchange/Service between their owners.

Additional Notes / Unresolved Matters

Contextual View: No contextual-view placement is established.

3.6.2. BEC: Assurance Criteria Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-assurance-criteria-management`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	enabling:686 (p. 581)	Linked RAD detail; exact locators in manifest.
Supports	CT1: Health Service Assurance	Source-scoped; no inherited descendant edges.

Capability Overview

Assurance Criteria Management maintains reusable evaluative criteria with governed standing and temporal applicability. Version and effective context matter because a conclusion must be understood against the criteria applicable to the subject and time being assessed. Criteria may operationalise governing requirements, but their management is not authority to change those requirements.

Definition

The capability to establish and manage reusable, governed and temporally identifiable assurance criteria through which subjects may be evaluated.

Boundary

Modelling criteria does not inherently approve them. Detailed criterion states and approval workflows remain unresolved, and criteria need not all originate in formal Governance.

Harmonia Architectural Relevance

Formal relevance remains Unresolved. Established assurance support explains the enabling purpose without classifying it by inference or allocating software responsibility.

Additional Notes / Unresolved Matters

Contextual View: No contextual-view placement is established.

Feature Decomposition: No Feature decomposition is established.

Scope

Contained Features and Established Owned Behaviour

Element	Type	Reference Id
Function: Manage Assurance Criteria (p. 443)	Function	`HCM-FN-manage-assurance-criteria`

3.6.2.1. Function: Manage Assurance Criteria

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-manage-assurance-criteria`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Assurance Criteria Management (p. 442)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	assurance:84 (p. 626)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Function Overview

Criteria management maintains the reusable evaluative basis and its temporal standing. It permits an assessment to be understood against the criterion applicable at the relevant time, rather than today's unqualified version. Management of that basis does not inherently grant approval authority or permit alteration of the governing obligation.

Governed Description and Qualifications

- **Canonical Name**: `ManageAssuranceCriteria`.
- **Definition**: Establishes and manages reusable, governed and temporally identifiable assurance criteria through which governed subjects may be evaluated.
- **Scope**: Criteria may express governance rule boundaries, quality constraints, performance expectations, compliance criteria, temporal criteria, evidence requirements or completeness expectations. These examples are not an exhaustive taxonomy. Criteria may have lifecycle, version and temporal applicability.
- **Boundary**: Managing a criterion does not transfer authority for the governing requirement from which it may derive. Criteria may operationalise a requirement without silently altering, weakening, strengthening or replacing it; not every criterion must derive from formal Governance. Modelling/managing criteria does not inherently confer approval authority. Detailed criterion lifecycle states and approval workflows remain unresolved.

Additional Notes / Unresolved Matters

Contextual View: No contextual-view placement is established.

3.6.3. BEC: Governed Assurance

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-BEC-governed-assurance`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Capability Tier	Unresolved	No tier inferred from presentation.
Ancestry	Unresolved	No root/parent inferred.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.

Name	Value	Comment / Description
Harmonia Relevance	Unresolved	Independent of status; never inherited.
Source	enabling:694 (p. 582)	Linked RAD detail; exact locators in manifest.
Supports	CT1: Health Service Assurance	Source-scoped; no inherited descendant edges.

Capability Overview

Governed Assurance independently evaluates a subject through applicable criteria and trustworthy evidence, establishing explicit findings and conclusions. Context establishment associates source-owned information with a particular assurance purpose; assessment evaluates it; adjudication establishes the supported outcome. Insufficient evidence remains a meaningful limit rather than a forced positive or negative result.

The capability can manage its own assurance activity without managing the subject. Request, progression status and established outcome are different client concerns; the requester does not thereby control the evaluation or gain unrestricted status access.

Definition

The capability to independently evaluate a governed subject against applicable assurance criteria through the assembly and evaluation of sufficient trustworthy evidence, establishing explicit findings, exceptions and assurance conclusions.

Boundary

No remediation, assignment or operational escalation of the subject is acquired. Financial governance is deliberately excluded; assurance does not recursively assure its own execution, and no component allocation is established.

Harmonia Architectural Relevance

Formal classification remains Unresolved. The approved independent-assurance responsibility and contribution model establish material purpose without resolving that metadata dimension.

Additional Notes / Unresolved Matters

Contextual View: No contextual-view placement is established.

Feature Decomposition: No Feature decomposition is established.

Scope

Contained Features and Established Owned Behaviour

Element	Type	Reference Id
Function: Establish Assurance Context (p. 445)	Function	`HCM-FN-establish-assurance-context`
Function: Assess Assurance Evidence (p. 446)	Function	`HCM-FN-assess-assurance-evidence`
Function: Adjudicate Assurance Assessment (p. 446)	Function	`HCM-FN-adjudicate-assurance-assessment`
Service: Request Service Assurance (p. 447)	Service	`HCM-SV-request-service-assurance`

Element	Type	Reference Id
Service: Request Service Assurance Status (p. 448)	Service	`HCM-SV-request-service-assurance-status`
Service: Communicate Service Assurance Outcome (p. 449)	Service	`HCM-SV-communicate-service-assurance-outcome`
Process: Assurance Process Execution (p. 450)	Process	`HCM-PR-assurance-process-execution`
Process: Assurance Process Reporting / Communication (p. 451)	Process	`HCM-PR-assurance-process-reporting-communication`

3.6.3.1. Function: Establish Assurance Context

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-establish-assurance-context`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Governed Assurance (p. 443)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	assurance:98 (p. 626)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Function Overview

Context establishment makes particular source-owned information evidence for a particular assurance purpose, with applicable subject, criteria and time identified. It includes evidence assembly without assuming sufficiency or prescribing copying and storage. This contextual evidentiary association is the assurance responsibility; ownership of the source material remains unchanged.

Governed Description and Qualifications

- **Canonical Name**: `EstablishAssuranceContext`.
- **Definition**: Establishes the governed subject, applicable assurance purpose and criteria, relevant temporal context, and contextual association of sufficient trustworthy source information as evidence required for an assurance activity.
- **Scope**: Includes the evidence-assembly responsibility previously described as Assurance Evidence Assembly; no separate evidence-assembly Function is established. Sufficiency is required for conclusion, not assumed merely because context is established; unavailable or insufficient evidence remains explicit.
- **Information Boundary**: Source information remains owned by its responsible source Capability. Its use as evidence does not transfer ownership or originating authority. Governed Assurance owns the contextual association that particular information constitutes evidence for a particular assurance purpose. Evidence association and applicable criteria may each be temporal and version-specific; association distinguishes the information state relevant to assurance from its current state.
- **Realisation Boundary**: Uses generic EC-02 context-management capability together with EC-14 assurance-specific semantics as documented in §4.1. Establishment of Assurance Context prescribes no copying, persistence, aggregation, caching or other technical mechanism.

Additional Notes / Unresolved Matters

Contextual View: No contextual-view placement is established.

3.6.3.2. Function: Assess Assurance Evidence

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-assess-assurance-evidence`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Governed Assurance (p. 443)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	assurance:108 (p. 626)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Function Overview

Evidence assessment applies the established evaluative basis to the available trustworthy material. It produces assessed detail rather than treating source availability or successful processing as a conclusion. Explicit gaps and qualifications preserve what the evidence cannot establish for subsequent adjudication.

Governed Description and Qualifications

- **Canonical Name**: `AssessAssuranceEvidence`.
- **Definition**: Evaluates the available trustworthy evidence against the applicable assurance criteria and establishes what that evidence demonstrates for each relevant criterion or assessment concern.
- **Boundary**: Assessment remains distinct from adjudication. It may identify satisfaction, exception, insufficiency or other criterion-level results where applicable; no universal result taxonomy is established. Quantitative, qualitative, compound, temporal and confidence-bearing assessment remain possible. **Insufficient evidence is not equivalent to either satisfaction or non-satisfaction.**

Additional Notes / Unresolved Matters

Contextual View: No contextual-view placement is established.

3.6.3.3. Function: Adjudicate Assurance Assessment

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-FN-adjudicate-assurance-assessment`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Governed Assurance (p. 443)	Established responsibility only; complete CT ancestry Unresolved.

Name	Value	Comment / Description
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	assurance:116 (p. 627)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Function Overview

Adjudication establishes the supported finding or conclusion from assessed detail. It is distinct from gathering evidence and from reporting an outcome already reached. Insufficiency must remain explicit rather than be forced into satisfaction or non-satisfaction, and the resulting finding does not acquire authority to remediate the subject.

Governed Description and Qualifications

- **Canonical Name**: `AdjudicateAssuranceAssessment`.
- **Definition**: Determines the assurance finding or conclusion that follows from the assessed evidence and applicable assurance criteria.
- **Boundary**: Establishment of the finding/conclusion is part of adjudication. It remains distinct from evidence assessment, operational response, remediation, escalation, workflow progression and clinical judgement. No separate conclusion-establishment or reporting Function is introduced merely because a conclusion is established or may be communicated.

Additional Notes / Unresolved Matters

Contextual View: No contextual-view placement is established.

3.6.3.4. Service: Request Service Assurance

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-request-service-assurance`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Governed Assurance (p. 443)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	assurance:164 (p. 628)	Linked RAD detail; exact locators in manifest.
Uses Process	Process: Assurance Process Execution (p. 450)	Established Service-to-Process relationship.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.
Internal Function Mapping	Unresolved	Precise internal Service-to-Function mapping is unestablished.

Service Overview

This Service gives approved requesting roles a governed way to seek assurance within their authority. A request expresses the legitimate assurance need; it does not confer control over criteria, evidence assessment or adjudication. The requester can be involved in the subject while the required independence of the conclusion remains intact.

Governed Description and Qualifications

- **Canonical Name**: `RequestServiceAssurance`.
- **Definition**: Enables an authorised consumer to request that Service Assurance be performed for an applicable governed subject in accordance with an established Assurance Definition.
- **Approved Consumers**: `CareCoordinator`, `ServiceCoordinator`, `Regulator`, `PolicyAuthority`, `SystemSteward`.
- **Associated Interaction**: Service Assurance Request, with Service Guardian and one authorised requesting Role.
- **Process Relationship**: Requests Assurance Process Execution under an applicable established Assurance Definition. It does not require a new Assurance Process Design instance for each request.
- **Independence Boundary**: The requesting Role identifies the assurance need within its responsibility and authority. Requesting does not grant authority to define or alter the Assurance Definition, alter Assurance Criteria, direct evidence assessment, determine adjudication or control the resulting Assurance Finding / Conclusion. REQ-FND-005 control independence remains applicable, including where a requester performs or manages the subject. Detailed payloads, contracts, initiation mechanics and states remain unresolved.

Additional Notes / Unresolved Matters

Contextual View: No contextual-view placement is established.

3.6.3.5. Service: Request Service Assurance Status

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-request-service-assurance-status`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Governed Assurance (p. 443)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	assurance:175 (p. 629)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.
Internal Function Mapping	Unresolved	Precise internal Service-to-Function mapping is unestablished.

Service Overview

The status Service exposes progression of an identifiable assurance activity exclusively to System Steward. Its client question is where the activity has reached, not what the evidence will

establish. Requesting assurance does not automatically grant status access, and progression observation is stewardship rather than recursive assurance.

Governed Description and Qualifications

- **Canonical Name**: `RequestServiceAssuranceStatus`.
- **Definition**: Enables the System Steward to obtain the current business progression state of an identifiable Service Assurance activity for stewardship purposes.
- **Only Approved Consumer**: `SystemSteward`.
- **Associated Interaction**: Service Assurance Status, between System Steward and Service Guardian.
- **Status Boundary**: Concerns **activity progression state**. It does not communicate provisional findings, incomplete assessment results as conclusions, predicted outcomes or unadjudicated conclusions. Other requesting Roles do not gain status access merely because they requested assurance. No detailed activity-status taxonomy is established.

Service Assurance Status answers "where is the assurance activity in its progression?" It does not answer "what has the assurance established?" The latter requires an adjudicated Assurance Finding / Conclusion and subsequent outcome communication; neither status nor execution completion establishes that conclusion.

Management Monitoring evaluates activity state in order to progress or manage activity. Assurance evaluates activity against governing criteria in order to establish an assurance conclusion. System Steward's use of this Service is stewardship / management observation of assurance progression, not itself assurance or recursive assurance. The non-recursion boundary in §7.2 applies.

Additional Notes / Unresolved Matters

Contextual View: No contextual-view placement is established.

3.6.3.6. Service: Communicate Service Assurance Outcome

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-SV-communicate-service-assurance-outcome`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Governed Assurance (p. 443)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	assurance:189 (p. 629)	Linked RAD detail; exact locators in manifest.
Exposes Process	Process: Assurance Process Reporting / Communication (p. 451)	Exposure is distinct from Process ownership.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.
Internal Function Mapping	Unresolved	Precise internal Service-to-Function mapping is unestablished.

Service Overview

Outcome communication exposes already established findings to applicable authorised recipients from the approved roles. It preserves the adjudicated meaning and does not require a

broadcast to every possible recipient. Receipt does not transfer the recipient's subsequent clinical, governance or operational action to Service Guardian.

Governed Description and Qualifications

- **Canonical Name**: `CommunicateServiceAssuranceOutcome`.
- **Definition**: Communicates applicable established Assurance Findings and Conclusions to authorised recipients according to their responsibilities, authority and information requirements.
- **Approved Potential Recipients**: `CareCoordinator`, `ServiceCoordinator`, `Regulator`, `PolicyAuthority`, `SystemSteward`.
- **Associated Interaction**: Service Assurance Outcome Communication, with Service Guardian and one or more authorised recipient Roles.
- **Process Relationship**: Exposes the communication behaviour of Assurance Process Reporting / Communication, beginning from an already established finding/conclusion. It does not perform assessment or adjudication or alter/reinterpret the established outcome. A documentary report is not required.
- **Recipient Boundary**: No mandatory broadcast is established. A particular outcome may be communicated to one, some or all legitimate recipients according to applicability, authority, responsibility and information requirements. Requester, status consumer and outcome recipient need not be the same Role. Communication does not transfer responsibility for recipients' interpretation within their authority, operational/clinical management, remediation, escalation, policy/compliance response, process improvement or other subsequent action to Service Guardian.

Additional Notes / Unresolved Matters

Contextual View: No contextual-view placement is established.

3.6.3.7. Process: Assurance Process Execution

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-PR-assurance-process-execution`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Governed Assurance (p. 443)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	processes:367 (p. 621); processes:370 (p. 621)	Linked RAD detail; exact locators in manifest.
Participating Functions	Function: Establish Assurance Context (p. 445); Function: Assess Assurance Evidence (p. 446); Function: Adjudicate Assurance Assessment (p. 446)	Participation retains each Function owner.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Process Overview

Assurance execution evaluates an identifiable subject under an applicable definition and establishes a finding through adjudication where sufficient evidence permits. Correct execution

with insufficient evidence remains different from execution failure and from a positive or negative conclusion about the subject. Participating Functions describe responsibilities without supplying unapproved lifecycle states.

Governed Description and Qualifications

- **Canonical Name**: `AssuranceProcessExecution`.
- **Owning Capability**: Governed Assurance.
- **Process Purpose**: Performs defined assurance for an identifiable governed subject in accordance with an applicable Assurance Definition and establishes an Assurance Finding / Conclusion from applicable trustworthy evidence.
- **Principal Performing Role**: Service Guardian.
- **Participating Functions**: Establish Assurance Context, Assess Assurance Evidence and Adjudicate Assurance Assessment, all retaining Governed Assurance ownership.
- **Conceptual Basis and Output**: Applicable Assurance Definition plus identifiable governed subject; Assurance Finding / Conclusion established through adjudication where sufficient trustworthy evidence permits. Correct execution with insufficient evidence does not imply satisfaction or non-satisfaction or force a conclusion.
- **Boundary**: Function completion does not define Process lifecycle states. No Context Established, Evidence Assessed, Adjudicated or Concluded states are inferred. Detailed lifecycle states, transitions and initiation mechanics remain unresolved. Requesting this Process does not grant control of evidence assessment, adjudication or the resulting finding/conclusion.

RAD diagram: textual source notation.

```
graph TD
    DEF["Applicable Assurance Definition<br/>Conceptual basis"] -->|Assurance basis| EXEC["PR: Assurance Process
    ↪  Execution<br/>Owner: Governed Assurance"]
    SUBJECT["Identifiable Governed Subject"] -->|Subject of assurance| EXEC
    EXEC -->|Participating Function| CONTEXT["FN: Establish Assurance Context"]
    EXEC -->|Participating Function| ASSESS["FN: Assess Assurance Evidence"]
    EXEC -->|Participating Function| ADJ["FN: Adjudicate Assurance Assessment"]
    EXEC -->|Establishes through adjudication| FINDING["Assurance Finding / Conclusion"]
```

Additional Notes / Unresolved Matters

Contextual View: No contextual-view placement is established.

3.6.3.8. Process: Assurance Process Reporting / Communication

Element Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`HCM-PR-assurance-process-reporting-communication`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Owning Context	BEC: Governed Assurance (p. 443)	Established responsibility only; complete CT ancestry Unresolved.
Structural Canonical ID	Unresolved	Required BEC tier ancestry is not established.
Source	processes:389 (p. 621); processes:392 (p. 622)	Linked RAD detail; exact locators in manifest.
Feature Association	Unresolved	No Feature association asserted; preserve owning Capability and do not infer an edge.

Process Overview

This Process starts from an established finding and communicates it to legitimate recipients without altering the outcome. Communication need not create a report document, and it does not repeat assessment or adjudication. Recipients retain authority and responsibility for any clinical, operational, policy or improvement action they take in response.

Governed Description and Qualifications

- **Canonical Name**: `AssuranceProcessReporting/Communication`.
- **Owning Capability**: Governed Assurance.
- **Process Purpose**: Communicates applicable established Assurance Findings and Conclusions to legitimate recipients according to their responsibilities, authority and information requirements, without altering the established assurance outcome or assuming responsibility for the recipient's subsequent action.
- **Starting Basis**: An already established Assurance Finding / Conclusion. This Process does not perform assessment or adjudication, alter or reinterpret the established outcome.
- **Communication Participation**: Service Guardian participates through Service Assurance Outcome Communication, associated with Communicate Service Assurance Outcome. No additional Function is introduced to populate this Process.
- **Boundary**: Reporting / Communication is not limited to documentary report production; an appropriate Business Interaction may communicate the outcome without a report document. It assumes no operational response, remediation, escalation, policy/compliance response, process improvement, clinical management, clinical judgement or other recipient action. Detailed communication progression and completion semantics remain unresolved.

Additional Notes / Unresolved Matters

Contextual View: No contextual-view placement is established.

4. Enterprise Capabilities

These are the fourteen governed reusable architectural capabilities. Functional responsibility bullets are descriptive responsibilities, not additional named Function or Service identities. No CT decomposition or additional Enterprise peers are inferred.

4.1. Enterprise Capability: Managed Entity & Relationship

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`EC-01`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Structural Canonical ID	`EC-01`	Existing governed EC identifier; no CT path applies.
Source	enterprise:103 (p. 674)	Linked RAD detail; exact locators in manifest.

Capability Overview

This reusable capability keeps managed healthcare entities identifiable and their relationships structurally intelligible. It supplies common identity, alias and relationship behaviour recurring in persons, practitioners, organisations, locations and devices. Reuse concerns those architectural responsibilities, allowing the healthcare-specific meaning of each entity to remain with its governing context.

Governed Description and Qualifications

- **Architectural Scope**: Identify, classify, relate, and maintain the structural integrity of managed healthcare and administrative entities throughout the enterprise landscape.
- **Functional Responsibilities**:
  - Uniquely identifying core domain entities (Persons, Practitioners, Roles, Organisations, Locations, Services, Devices).
  - Resolving and maintaining multi-authority identifier systems and aliases without destructive overwriting.
  - Representing and traversing multi-dimensional relationships (affiliations, organizational hierarchies, physical location trees, care team memberships).
  - Maintaining referential integrity across interconnected entity graphs.
- **Anti-Responsibilities**: Does not own physical database relational tables or foreign key constraints (Domain 04/07).

Boundary

Relationship integrity is not a physical database design. Correlating entities does not establish originating clinical identity authority or a new standalone Correlation capability.

4.2. Enterprise Capability: Context Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	EC-02	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Structural Canonical ID	EC-02	Existing governed EC identifier; no CT path applies.
Source	enterprise:112 (p. 674)	Linked RAD detail; exact locators in manifest.

Capability Overview

Context Management preserves the circumstances in which information and activity have meaning as they cross processing boundaries. Subject, encounter, provider, service and security context must remain attributable and consistent rather than be reconstructed from convenient local assumptions. Its reuse is especially significant in multi-hop activity where a syntactically valid message can otherwise lose the context required for legitimate interpretation.

Governed Description and Qualifications

- **Architectural Scope**: Establish, validate, propagate, and preserve the execution, clinical, security, and administrative context within which information and activities have meaning.
- **Functional Responsibilities**:
  - Binding the subject of care, encounter, ordering provider, and healthcare service context to in-flight messages and transactions.
  - Propagating tamper-evident, attributable security context across asynchronous, multi-hop operational boundaries.
  - Ensuring context preservation during cross-protocol transformations across disparate healthcare exchange formats and protocols.
  - Detecting and rejecting requests where mandatory operational or clinical context is missing or inconsistent.
- **Collaborative Nature**: Bounded reusable context responsibility may be realised through participation across gateways, activity execution and information-access boundaries. Participation does not transfer semantic ownership.

Boundary

Context participation does not transfer semantic ownership. Generic context management and assurance-specific context semantics contribute together without allocating a new central Context Service.

4.3. Enterprise Capability: Managed State & Lifecycle

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	EC-03	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Structural Canonical ID	EC-03	Existing governed EC identifier; no CT path applies.
Source	enterprise:121 (p. 675)	Linked RAD detail; exact locators in manifest.

Capability Overview

Managed State & Lifecycle provides reusable responsibility for valid progression, temporal applicability and non-destructive change. A directory relationship, clinical document and operational work item have different business states, yet all need an intelligible distinction between current standing, historical state and invalid transitions. The capability captures that recurrence without replacing each subject's lifecycle meaning.

Governed Description and Qualifications

- **Architectural Scope**: Govern the current and historical state, effective temporal periods, state transition validations, and lifecycle progression of managed integration artifacts and entities.
- **Functional Responsibilities**:
  - Defining and enforcing valid state transition state machines (e.g., `REQUESTED` → `VALIDATED` → `IN_PROGRESS` → `COMPLETED`).
  - Maintaining temporal validity intervals (effective start and end dates) for relationships, credentials, and states.
  - Supporting non-destructive state progression, preserving historical state versions for longitudinal review.
  - Rejecting illegal or out-of-order state transitions while accommodating idempotent replay.
- **Anti-Responsibilities**: Does not define thread-level mutexes or database transaction isolation levels.

Boundary

Business lifecycle integrity does not prescribe locks, database isolation or worker implementation, and illustrative state names do not define every subject's transitions.

4.4. Enterprise Capability: Information Management

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`EC-04`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Structural Canonical ID	`EC-04`	Existing governed EC identifier; no CT path applies.
Source	enterprise:130 (p. 675)	Linked RAD detail; exact locators in manifest.

Capability Overview

Information Management provides the recurring ability to represent and maintain governed information through validation, preservation, retrieval and versioned change. It accommodates both responsive active use and authoritative durable preservation while keeping those responsibilities separate. This distinction allows availability and working state to support care without turning a cache into durable truth.

Governed Description and Qualifications

- **Architectural Scope**: Validate, represent, persist, retrieve, version, and manage governed clinical, administrative, and operational information assets.
- **Functional Responsibilities**:

  - Validating inbound information structures against canonical information constraints.
  - Providing authoritative durable persistence for historical health records and administrative registries.
  - Managing active distributed working memory representations for low-latency retrieval.
  - Governing document versioning, supersession, amendments, and retractions without data loss.
- **Architectural Principle**: Enforces the foundational separation between *active distributed state access* and *authoritative durable state preservation* (Axiom AX-05).

Boundary

Engine capabilities provide machinery under Harmonia semantics. Persistence success does not establish information authority or make the platform the originator of the represented facts.

4.5. Enterprise Capability: Search & Discovery

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`EC-05`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Structural Canonical ID	`EC-05`	Existing governed EC identifier; no CT path applies.
Source	enterprise:139 (p. 675)	Linked RAD detail; exact locators in manifest.

Capability Overview

Search & Discovery makes information, entities and service endpoints findable through governed criteria and bounded queries. It addresses the recurring need to locate relevant material or a destination before it can be used. Security-qualified results preserve the distinction between knowing that something exists and being entitled to obtain or disclose it.

Governed Description and Qualifications

- **Architectural Scope**: Index, search, resolve, filter, and discover information assets, entities, directory endpoints, and healthcare services across the enterprise.
- **Functional Responsibilities**:
  - Providing multi-criteria indexed search across complex entity graphs (e.g., find active cardiologists at Location X accepting new referrals).
  - Resolving service and practitioner communication endpoints for addressed delivery.
  - Supporting bounded, paginated clinical queries across longitudinal patient records.
  - Applying dynamic security and consent filters to search results prior to egress.
- **Anti-Responsibilities**: Does not dictate specific search engine indexes, inverted index implementations, or SQL query syntaxes.

Boundary

Discovery does not decide policy or prescribe a search engine, index format or query language.

4.6. Enterprise Capability: Policy & Control

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`EC-06`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Structural Canonical ID	`EC-06`	Existing governed EC identifier; no CT path applies.
Source	enterprise:148 (p. 676)	Linked RAD detail; exact locators in manifest.

Capability Overview

Policy & Control supplies the reusable evaluation and enforcement of authority, consent, privacy and handling constraints. Its purpose is consistent governed action across ingress, information access, execution and presentation boundaries. Default-deny evaluation preserves the need for established authority rather than treating a caller's intent or a technically valid request as permission.

Governed Description and Qualifications

- **Architectural Scope**: Evaluate and enforce security, consent, privacy, and regulatory policies including authority, access control, and information handling rules.
- **Functional Responsibilities**:
  - Executing impartial, default-deny policy evaluations for all integration transactions.
  - Enforcing Role-Based and Attribute-Based Access Control (RBAC/ABAC) policies.
  - Evaluating patient consent directives and privacy restrictions at boundary and query checkpoints.
  - Enforcing statutory information disclosure boundaries (e.g., mental health, VIP records).
- **Collaborative Nature**: Evaluated across boundary gateways, internal bus dispatch, storage interfaces, and presentation layers.

Boundary

Participation at multiple enforcement points does not distribute policy ownership or turn operational security context into persistent clinical content.

4.7. Enterprise Capability: Provenance & Traceability

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`EC-07`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Structural Canonical ID	`EC-07`	Existing governed EC identifier; no CT path applies.
Source	enterprise:157 (p. 676)	Linked RAD detail; exact locators in manifest.

Capability Overview

Provenance & Traceability preserves the meaningful attribution and history needed to explain information and significant activity. It connects source, transformation and required check-

points without making every internal mechanism permanent evidence. Its recurring contribution supports accountability, reconstruction and assurance use while retaining the context that makes a recorded fact evidentially relevant.

Governed Description and Qualifications

- **Architectural Scope**: Preserve required origin attribution, meaningful transformation history, significant execution checkpoints, tamper-evident audit evidence and end-to-end traceability. Harmonia SHALL preserve provenance, audit and operational evidence where required by governing obligations, architectural significance, accountability, reconstruction, security or assurance needs. Routine technical activity SHALL NOT automatically require durable evidentiary recording.
- **Functional Responsibilities**:
  - Recording attributable, verifiable origin metadata for information sources, authors, and timestamps.
  - Tracking information-significant transformation history and required mapping checkpoints as information crosses system seams.
  - Capturing non-PHI operational and security audit trails for regulatory compliance (HIPAA, GDPR).
  - Supporting end-to-end transaction tracing across asynchronous processing pipelines.
- **Collaborative Nature**: Participating elements preserve required provenance and audit context according to policy. Telemetry, operational facts, provenance, audit and assurance evidence remain distinct. Being evidence is contextual, not an intrinsic property of every recorded fact; preservation does not establish an assurance conclusion.

Boundary

Telemetry, provenance, audit and assurance evidence remain distinct. Recording evidence neither establishes an assurance conclusion nor automatically requires retention of routine technical activity.

4.8. Enterprise Capability: Interoperability & Exchange

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`EC-08`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Structural Canonical ID	`EC-08`	Existing governed EC identifier; no CT path applies.
Source	enterprise:166 (p. 676)	Linked RAD detail; exact locators in manifest.

Capability Overview

Interoperability & Exchange moves information across participating boundaries through destination resolution, transformation, delivery and acknowledgements. It preserves the difference between technical transmission and the recipient's business response so that an exchange can be understood under its actual contract. Standards govern the external representation while internal management semantics remain private.

Governed Description and Qualifications

- **Architectural Scope**: Resolve destinations, transform schemas, route, transport, deliver, acknowledge, and coordinate standards-based information exchange across enterprise boundaries.

- **Functional Responsibilities**:
  - Providing multi-protocol boundary adaptation for healthcare standards (HL7 v2, FHIR REST, DICOM, secure messaging).
  - Executing bidirectional structural and semantic transformations between legacy formats and canonical representations.
  - Ensuring reliable addressed delivery and fan-out distribution to external clinical endpoints.
  - Processing and distinguishing technical delivery acknowledgements from business acknowledgements.
- **Boundary Rule**: Egress terminates Harmonia management of the emitted representation (Axiom AX-13).

Boundary

Egress ends Harmonia management of the emitted representation. Retained delivery evidence does not extend control over an external recipient or transfer payload authority.

4.9. Enterprise Capability: Event & Subscription

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	`EC-09`	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Structural Canonical ID	`EC-09`	Existing governed EC identifier; no CT path applies.
Source	enterprise:175 (p. 677)	Linked RAD detail; exact locators in manifest.

Capability Overview

Event & Subscription connects significant changes with participants whose registered interests match them. It makes event ingestion, correlation, interest criteria and notification reusable across clinical and operational contexts. The architectural meaning is the relationship between a change and an interested consumer, rather than the broker constructs used to carry it.

Governed Description and Qualifications

- **Architectural Scope**: Publish, ingest, correlate, filter, and syndicate events and subscription-driven changes across the enterprise landscape.
- **Functional Responsibilities**:
  - Ingesting operational and clinical event triggers from external systems and internal state changes.
  - Managing topic- and criteria-based event subscriptions for internal and external consumers.
  - Evaluating dynamic subscription filters to match event payloads with interested subscriber criteria.
  - Delivering asynchronous event notifications reliably to registered subscribers.
- **Decoupling Note**: Defined strictly in terms of event publishing, criteria matching, and notification delivery; entirely independent of broker topics, partitions, or queues.

Boundary

Receiving a notification does not determine the consumer's business response; topics and queues do not define the capability.

4.10. Enterprise Capability: Activity & Execution

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	EC-10	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Structural Canonical ID	EC-10	Existing governed EC identifier; no CT path applies.
Source	enterprise:184 (p. 677)	Linked RAD detail; exact locators in manifest.

Capability Overview

Activity & Execution provides reusable instantiation, matching, dispatch and supervision for governed activity units. Human Work Orders, human decision To Dos and automated Synthetic Tasks retain different performer and authority semantics while sharing a need for accountable progression. This reuse supports operational coordination without making a runtime engine the definition of the work.

Governed Description and Qualifications

- **Architectural Scope**: Define, instantiate, match, assign, dispatch, supervise, and progress coordinated operational and clinical activity units.
- **Functional Responsibilities**:
  - Instantiating operational work items (Work Orders, To Dos, synthetic platform Tasks) in response to triggers.
  - Matching and assigning work units to eligible human or automated performers and roles based on policy rules.
  - Dispatching work units to their assigned performers with acknowledgement according to the applicable interaction contract.
  - Supervising execution progress, tracking timeouts, managing escalations, and handling activity interruptions.
- **Decoupling Note**: Free of worker threads, thread pools, execution daemons, or specific workflow runtime engines.

Boundary

Coordination does not confer clinical work allocation or judgement. The capability specifies neither worker threads nor a particular execution engine.

4.11. Enterprise Capability: Interaction & Experience

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	EC-11	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Structural Canonical ID	EC-11	Existing governed EC identifier; no CT path applies.
Source	enterprise:193 (p. 677)	Linked RAD detail; exact locators in manifest.

Capability Overview

Interaction & Experience makes contextual information and governed actions available to human participants through presentation and collaboration integration. Clinical timelines, directory administration and operational dashboards have different purposes, but share a need to retain established identity, security and information context. The recurring responsibility is usable interaction within those boundaries.

Governed Description and Qualifications

- **Architectural Scope**: Provide contextual interaction between human actors, Harmonia, and enterprise presentation services, including visualization, alerting, and collaboration integration.
- **Functional Responsibilities**:
  - Mediating presentation interactions via dedicated backend-for-frontend gateways decoupled from internal databases.
  - Rendering clinical timelines, provider directory management interfaces, and operational console dashboards.
  - Providing contextual notifications and alerts to human users across web and mobile channels.
  - Projecting longitudinal record context into secure multidisciplinary collaboration spaces.
- **Boundary Note**: Does not absorb the healthcare semantics of Clinical Collaboration or medical documentation. Consumes established identity and security context; does not own authentication, consent, or clinical truth.

Boundary

Presentation does not own consent, authentication or clinical truth, and collaboration projection does not absorb clinical documentation semantics.

4.12. Enterprise Capability: Operational Assurance

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	EC-12	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Structural Canonical ID	EC-12	Existing governed EC identifier; no CT path applies.
Source	enterprise:202 (p. 677)	Linked RAD detail; exact locators in manifest.

Capability Overview

Operational Assurance preserves processing integrity under concurrency, duplicate delivery and failure. It brings together resilience, durable ingress acceptance, recovery and operational telemetry so that technical degradation does not silently weaken the managed operation's semantics. Its contribution is collaborative across operational boundaries while remaining a bounded architectural responsibility.

Governed Description and Qualifications

- **Architectural Scope**: Ensure concurrency integrity, system resilience, duplicate detection, exception handling, automated recovery, and operational telemetry across platform operations.

- **Functional Responsibilities**:
  - Enforcing sliding-window duplicate message detection and idempotent suppression.
  - Guaranteeing ingress dual-write safety (durable acceptance before emitting positive application ACK).
  - Managing structured failure recovery, exception escalation, and dead-letter isolation.
  - Emitting platform telemetry, operational health metrics, and queue processing metrics.
- **Collaborative Nature**: Realized collaboratively across boundary gateways, messaging infrastructure, execution workers, and storage layers.

EC-12 / EC-14 Boundary

EC-12 retains the established scope and responsibilities above. It may contribute directly to generic operational/processing assurance, limited to that contribution in the approved assurance matrix. It does not own the general semantics of assurance disposition, assurance-specific criteria applicability, evidentiary association, assessment against assurance criteria, adjudication, or independent assurance findings and conclusions. Those distinguishing responsibilities are introduced through EC-14 Service Guardian. Operational health and business-level processing assurance remain distinct; EC-12 is not expanded to absorb EC-14.

Boundary

Operational health is distinct from independent assessment of a governed subject. Assurance-specific criteria applicability, adjudication and conclusions belong to the established EC-14 contribution.

4.13. Enterprise Capability: Semantic Governance & Conformance

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	EC-13	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Structural Canonical ID	EC-13	Existing governed EC identifier; no CT path applies.
Source	enterprise:215 (p. 678)	Linked RAD detail; exact locators in manifest.

Capability Overview

This capability makes canonical information models, terminology bindings, mappings and conformance rules governable across enterprise use. It addresses both what representations mean and whether they respect the published rules, keeping version and compatibility concerns explicit. A conformant representation can therefore be distinguished from one that merely uses a supported transport or parser.

Governed Description and Qualifications

- **Architectural Scope**: Govern canonical models, schemas, terminologies, value sets, mapping tables, constraint rules, and semantic conformance validation across the enterprise.
- **Functional Responsibilities**:
  - Defining and publishing canonical healthcare data models and profile structures.
  - Governing clinical terminology mappings, value sets, and concept lookups (SNOMED CT, LOINC).

  - Validating inbound and outbound message representations against published semantic rules.
  - Governing semantic versioning, deprecation, and backward compatibility across schema revisions.
- **Component Relationship**: Informs the Calliope strategic logical component, but Calliope is the responsibility centre, not the capability definition.

Boundary

Conformance does not establish clinical truth, information authority or disclosure permission. Calliope is an established responsibility centre, not the definition of the capability.

4.14. Enterprise Capability: Service Guardian

Capability Metadata		
Name	**Value**	**Comment / Description**
Reference Id	EC-14	Stable publication identity.
Architectural Status	Canonical	Established by RAD.
Structural Canonical ID	EC-14	Existing governed EC identifier; no CT path applies.
Source	enterprise:224 (p. 678)	Linked RAD detail; exact locators in manifest.

Capability Overview

Service Guardian supplies the reusable assurance semantics that bind a governed subject to applicable criteria and trustworthy evidence, support assessment and adjudication, and establish reportable findings. Its contribution distinguishes explicit evidence insufficiency from a conclusion about the subject. Existing Enterprise Capabilities contribute collaboratively; EC-14 does not duplicate all their information, policy or execution responsibilities.

Independence concerns control of evaluation and conclusion. A subject may supply evidence but cannot solely determine, suppress or retrospectively change its own assurance outcome.

Governed Description and Qualifications

- **Architectural Scope**: Service Guardian provides the reusable capability to independently establish whether governed service behaviour, state or outcomes remain within defined assurance expectations by associating applicable criteria and trustworthy evidence with a governed subject, assessing that evidence, adjudicating the assessment, and establishing reportable assurance findings and conclusions.
- **Functional Responsibilities**:
  - Supplying assurance-specific semantics for assurance disposition and assurance criteria applicability.
  - Associating a governed subject/context with applicable criteria and sufficient trustworthy evidence for a particular assurance purpose.
  - Assessing evidence against applicable criteria and adjudicating assessed detail to establish reportable findings, exceptions and conclusions.
  - Preserving assurance independence, temporal/version-specific evidence and criteria applicability, and explicit inability to establish a conclusion where evidence is insufficient.
- **Collaborative Nature**: Supplies the assurance-specific semantics required to bind existing reusable capabilities into governed assurance. It contributes directly to Assurance Design, Assurance Criteria Management and Governed Assurance; direct contribution is not sole realisation. The approved matrix preserves the contributions of EC-01 through EC-13.

- **Anti-Responsibilities**: Does not own the underlying governed subject, authoritative governance requirements, or source information used as evidence. Does not perform, manage, remediate, assign, delegate or operationally escalate the subject activity. Does not replace System Operations or EC-12 Operational Assurance, and does not provide Clinical Services Delivery Assurance.
- **Allocation Boundary**: No strategic logical component, application component, runtime service or implementation construct is allocated EC-14. The catalogue defines a capability, not a software component or deployment boundary.

Conceptual Behavioural Sequence

RAD source listing (presentation only; no inferred ancestry).

Define Good Behaviour

↓

Gather Evidence

↓

Assess Evidence against Good Behaviour

↓

Adjudicate Assessed Detail

↓

Report

This sequence explains the capability; it is not an implementation workflow specification or a derived Business Process. “Define Good Behaviour” identifies assurance expectations and criteria; it does not transfer ownership of governing requirements to Service Guardian. Source information ownership remains with its responsible capability; assurance owns its contextual evidentiary association.

The Business Enabling assurance principles govern disposition, generic processing assurance, evidence/criteria temporality, assessment/adjudication, independence, failure/recovery and explicit evidence insufficiency. These semantics are not universally reducible to PASS / FAIL / UNKNOWN and do not imply an assurance information model or persistence mechanism.

Boundary

The capability does not manage or remediate the subject, replace operational assurance, or assure clinical adequacy. No application component, runtime service or deployment boundary is allocated, and the explanatory sequence is not a derived Business Process.

5. Established and unresolved relationship register

Only source-evidenced assertions are listed. Unresolved associations are explicitly unestablished, not approved edges. The BC evidence/context table and the approved EC contribution matrix retain their exact semantics in the source detail; they are not coerced into these relationship types.

5.1. Capability Decomposition

From	To	Status / evidence
CT1: Individual Care Delivery (p. 4)	CT2: Individual Health Assessment (p. 5)	Canonical; business:162 (p. 532)
CT1: Individual Care Delivery (p. 4)	CT2: Clinical Determination (p. 8)	Canonical; business:188 (p. 532)
CT1: Individual Care Delivery (p. 4)	CT2: Individual Care Planning (p. 9)	Canonical; business:213 (p. 533)
CT1: Individual Care Delivery (p. 4)	CT2: Clinical Intervention (p. 10)	Canonical; business:239 (p. 533)
CT1: Individual Care Delivery (p. 4)	CT2: Individual Health Monitoring (p. 11)	Canonical; business:265 (p. 533)
CT1: Individual Care Delivery (p. 4)	CT2: Individual Care Review (p. 13)	Canonical; business:291 (p. 534)
CT2: Individual Health Assessment (p. 5)	CT3: Health History Assessment (p. 6)	Canonical; business:326 (p. 534); business:322 (p. 534)
CT2: Individual Health Assessment (p. 5)	CT3: Clinical Examination (p. 7)	Canonical; business:327 (p. 534); business:322 (p. 534)
CT2: Individual Health Assessment (p. 5)	CT3: Health Status Measurement (p. 7)	Canonical; business:328 (p. 534); business:322 (p. 534)
CT2: Individual Health Monitoring (p. 11)	CT3: Health State Monitoring (p. 12)	Canonical; business:342 (p. 535); business:338 (p. 535)
CT2: Individual Health Monitoring (p. 11)	CT3: Care Response Monitoring (p. 12)	Canonical; business:343 (p. 535); business:338 (p. 535)
CT2: Individual Care Review (p. 13)	CT3: Care Appropriateness Review (p. 14)	Canonical; business:357 (p. 535); business:353 (p. 535)
CT2: Individual Care Review (p. 13)	CT3: Care Effectiveness Review (p. 15)	Canonical; business:358 (p. 535); business:353 (p. 535)
CT1: Care Access & Coordination (p. 15)	CT2: Care Access Management (p. 16)	Canonical; business:372 (p. 535)
CT2: Care Access Management (p. 16)	CT3: Care Intake Management (p. 17)	Canonical; business:380 (p. 535); business:376 (p. 535)
CT2: Care Access Management (p. 16)	CT3: Care Referral Management (p. 18)	Canonical; business:381 (p. 535); business:376 (p. 535)
CT2: Care Access Management (p. 16)	CT3: Care Eligibility Determination (p. 19)	Canonical; business:382 (p. 535); business:376 (p. 535)
CT1: Care Access & Coordination (p. 15)	CT2: Care Scheduling & Capacity Management (p. 19)	Canonical; business:388 (p. 536)
CT2: Care Scheduling & Capacity Management (p. 19)	CT3: Care Appointment Management (p. 20)	Canonical; business:396 (p. 536); business:392 (p. 536)
CT2: Care Scheduling & Capacity Management (p. 19)	CT3: Care-Place Allocation (p. 21)	Canonical; business:397 (p. 536); business:392 (p. 536)
CT2: Care Scheduling & Capacity Management (p. 19)	CT3: Care-Place Readiness Management (p. 22)	Canonical; business:398 (p. 536); business:392 (p. 536)

From	To	Status / evidence
CT2: Care Scheduling & Capacity Management (p. 19)	CT3: Care Resource Deployment (p. 22)	Canonical; business:399 (p. 536); business:392 (p. 536)
CT2: Care Scheduling & Capacity Management (p. 19)	CT3: Operational Care Flow Coordination (p. 23)	Canonical; business:400 (p. 536); business:392 (p. 536)
CT2: Care Scheduling & Capacity Management (p. 19)	CT3: Operational Care Capacity Management (p. 24)	Canonical; business:401 (p. 536); business:392 (p. 536)
CT1: Care Access & Coordination (p. 15)	CT2: Care Coordination & Transition (p. 24)	Canonical; business:407 (p. 536)
CT2: Care Coordination & Transition (p. 24)	CT3: Care-Team Coordination (p. 25)	Canonical; business:415 (p. 536); business:411 (p. 536)
CT2: Care Coordination & Transition (p. 24)	CT3: Care Responsibility Transition (p. 26)	Canonical; business:416 (p. 536); business:411 (p. 536)
CT2: Care Coordination & Transition (p. 24)	CT3: Care Discharge Coordination (p. 27)	Canonical; business:417 (p. 536); business:411 (p. 536)
CT1: Care Access & Coordination (p. 15)	CT2: Care Support Logistics Management (p. 27)	Canonical; business:423 (p. 537)
CT2: Care Support Logistics Management (p. 27)	CT3: Patient Movement Coordination (p. 28)	Canonical; business:431 (p. 537); business:427 (p. 537)
CT2: Care Support Logistics Management (p. 27)	CT3: Clinical Material Logistics Coordination (p. 29)	Canonical; business:432 (p. 537); business:427 (p. 537)
CT2: Care Support Logistics Management (p. 27)	CT3: Care Support Work Coordination (p. 30)	Canonical; business:433 (p. 537); business:427 (p. 537)
CT1: Diagnostic, Therapeutic & Clinical Support Services (p. 31)	CT2: Diagnostic Support Provision (p. 32)	Canonical; business:459 (p. 537)
CT2: Diagnostic Support Provision (p. 32)	CT3: Diagnostic Service Appointment Management (p. 33)	Canonical; business:467 (p. 538); business:463 (p. 537)
CT2: Diagnostic Support Provision (p. 32)	CT3: Diagnostic Service Delivery (p. 34)	Canonical; business:468 (p. 538); business:463 (p. 537)
CT2: Diagnostic Support Provision (p. 32)	CT3: Diagnostic Specimen Management (p. 35)	Canonical; business:469 (p. 538); business:463 (p. 537)
CT2: Diagnostic Support Provision (p. 32)	CT3: Diagnostic Data Management (p. 35)	Canonical; business:470 (p. 538); business:463 (p. 537)
CT2: Diagnostic Support Provision (p. 32)	CT3: Diagnostic Interpretation (p. 36)	Canonical; business:471 (p. 538); business:463 (p. 537)
CT2: Diagnostic Support Provision (p. 32)	CT3: Diagnostic Report Management (p. 37)	Canonical; business:472 (p. 538); business:463 (p. 537)
CT2: Diagnostic Support Provision (p. 32)	CT3: Diagnostic Report Distribution (p. 37)	Canonical; business:473 (p. 538); business:463 (p. 537)
CT1: Diagnostic, Therapeutic & Clinical Support Services (p. 31)	CT2: Medication & Pharmacy Services (p. 38)	Canonical; business:479 (p. 538)
CT2: Medication & Pharmacy Services (p. 38)	CT3: Prescription Medication Dispensing (p. 39)	Canonical; business:487 (p. 538); business:483 (p. 538)
CT2: Medication & Pharmacy Services (p. 38)	CT3: Non-Prescription Medication Provision (p. 40)	Canonical; business:488 (p. 538); business:483 (p. 538)
CT2: Medication & Pharmacy Services (p. 38)	CT3: Medication Advice (p. 40)	Canonical; business:489 (p. 538); business:483 (p. 538)
CT2: Medication & Pharmacy Services (p. 38)	CT3: Medication Dose Administration Aid Provision (p. 41)	Canonical; business:490 (p. 538); business:483 (p. 538)
CT1: Health Products & Clinical Technology (p. 42)	CT2: Clinical Technology Management (p. 43)	Canonical; business:504 (p. 539)
CT2: Clinical Technology Management (p. 43)	CT3: Clinical Device Inventory Management (p. 44)	Canonical; business:512 (p. 539); business:508 (p. 539)
CT2: Clinical Technology Management (p. 43)	CT3: Clinical Device Feature & Release Management (p. 45)	Canonical; business:513 (p. 539); business:508 (p. 539)

From	To	Status / evidence
CT2: Clinical Technology Management (p. 43)	CT3: Clinical Device Loan & Deployment Management (p. 45)	Canonical; business:514 (p. 539); business:508 (p. 539)
CT2: Clinical Technology Management (p. 43)	CT3: Clinical Device Utilisation Support (p. 46)	Canonical; business:515 (p. 539); business:508 (p. 539)
CT2: Clinical Technology Management (p. 43)	CT3: Clinical Device Integration Provision (p. 47)	Canonical; business:516 (p. 539); business:508 (p. 539)
CT2: Clinical Technology Management (p. 43)	CT3: Clinical Device Data Integration (p. 47)	Canonical; business:517 (p. 539); business:508 (p. 539)
CT2: Clinical Technology Management (p. 43)	CT3: Clinical Device Lifecycle Management (p. 48)	Canonical; business:518 (p. 539); business:508 (p. 539)
CT1: Clinical Quality, Safety & Improvement (p. 49)	CT2: Clinical Safety Management (p. 50)	Canonical; business:532 (p. 539)
CT2: Clinical Safety Management (p. 50)	CT3: Clinical Risk Identification (p. 51)	Canonical; business:540 (p. 540); business:536 (p. 540)
CT2: Clinical Safety Management (p. 50)	CT3: Clinical Risk Mitigation Management (p. 51)	Canonical; business:541 (p. 540); business:536 (p. 540)
CT2: Clinical Safety Management (p. 50)	CT3: Clinical Risk Response Management (p. 52)	Canonical; business:542 (p. 540); business:536 (p. 540)
CT1: Clinical Quality, Safety & Improvement (p. 49)	CT2: Clinical Quality Management (p. 53)	Canonical; business:548 (p. 540)
CT2: Clinical Quality Management (p. 53)	CT3: Clinical Quality Criteria Management (p. 54)	Canonical; business:556 (p. 540); business:552 (p. 540)
CT2: Clinical Quality Management (p. 53)	CT3: Clinical Quality Evidence Management (p. 54)	Canonical; business:557 (p. 540); business:552 (p. 540)
CT2: Clinical Quality Management (p. 53)	CT3: Clinical Quality Review & Assessment Management (p. 55)	Canonical; business:558 (p. 540); business:552 (p. 540)
CT2: Clinical Quality Management (p. 53)	CT3: Clinical Quality Reporting (p. 56)	Canonical; business:559 (p. 540); business:552 (p. 540)
CT1: Community Health & Wellbeing (p. 56)	CT2: Preventative Health (p. 57)	Canonical; business:573 (p. 540)
CT2: Preventative Health (p. 57)	CT3: Preventative Health Problem Identification (p. 58)	Canonical; business:581 (p. 541); business:577 (p. 541)
CT2: Preventative Health (p. 57)	CT3: Preventative Health Programme Management (p. 59)	Canonical; business:582 (p. 541); business:577 (p. 541)
CT2: Preventative Health (p. 57)	CT3: Preventative Health Education Services (p. 60)	Canonical; business:583 (p. 541); business:577 (p. 541)
CT2: Preventative Health (p. 57)	CT3: Preventative Health Programme Monitoring (p. 60)	Canonical; business:584 (p. 541); business:577 (p. 541)
CT2: Preventative Health (p. 57)	CT3: Preventative Health Reporting (p. 61)	Canonical; business:585 (p. 541); business:577 (p. 541)
CT2: Preventative Health (p. 57)	CT3: Preventative Health Cohort Management (p. 62)	Canonical; business:586 (p. 541); business:577 (p. 541)
CT1: Population Health & Health-System Planning (p. 62)	CT2: Population Health Surveillance (p. 63)	Canonical; business:600 (p. 541)
CT2: Population Health Surveillance (p. 63)	CT3: Population Health Longitudinal Assessment (p. 65)	Canonical; business:608 (p. 541); business:604 (p. 541)
CT2: Population Health Surveillance (p. 63)	CT3: Population Health Surveillance Management (p. 65)	Canonical; business:609 (p. 541); business:604 (p. 541)
CT2: Population Health Surveillance (p. 63)	CT3: Population Health Event Identification (p. 66)	Canonical; business:610 (p. 541); business:604 (p. 541)
CT2: Population Health Surveillance (p. 63)	CT3: Notifiable Health Event Management (p. 67)	Canonical; business:611 (p. 541); business:604 (p. 541)
CT2: Population Health Surveillance (p. 63)	CT3: Population Health Reporting (p. 67)	Canonical; business:612 (p. 541); business:604 (p. 541)

From	To	Status / evidence
CT1: Population Health & Health-System Planning (p. 62)	CT2: Health-System Planning (p. 68)	Canonical; business:618 (p. 542)
CT2: Health-System Planning (p. 68)	CT3: Health Service Demand Assessment (p. 69)	Canonical; business:626 (p. 542); business:622 (p. 542)
CT2: Health-System Planning (p. 68)	CT3: Health Service Capacity Planning (p. 70)	Canonical; business:627 (p. 542); business:622 (p. 542)
CT2: Health-System Planning (p. 68)	CT3: Health Service Distribution Planning (p. 70)	Canonical; business:628 (p. 542); business:622 (p. 542)
CT1: Health Information & Knowledge Management (p. 71)	CT2: Health Information Management (p. 72)	Canonical; business:646 (p. 542)
CT2: Health Information Management (p. 72)	CT3: Health Information Data Management (p. 73)	Canonical; business:654 (p. 542); business:650 (p. 542)
CT2: Health Information Management (p. 72)	CT3: Health Information Structure Management (p. 74)	Canonical; business:655 (p. 542); business:650 (p. 542)
CT2: Health Information Management (p. 72)	CT3: Health Information Lifecycle Management (p. 75)	Canonical; business:656 (p. 542); business:650 (p. 542)
CT2: Health Information Management (p. 72)	CT3: Health Information Catalogue and Index Management (p. 75)	Canonical; business:657 (p. 542); business:650 (p. 542)
CT2: Health Information Management (p. 72)	CT3: Health Information Compartmentalisation (p. 76)	Canonical; business:658 (p. 542); business:650 (p. 542)
CT2: Health Information Management (p. 72)	CT3: Health Information Data Correlation (p. 77)	Canonical; business:659 (p. 542); business:650 (p. 542)
CT2: Health Information Management (p. 72)	CT3: Health Information Data Retrieval (p. 77)	Canonical; business:660 (p. 542); business:650 (p. 542)
CT1: Health Information & Knowledge Management (p. 71)	CT2: Clinical Knowledge Management (p. 78)	Canonical; business:666 (p. 543)
CT2: Clinical Knowledge Management (p. 78)	CT3: Clinical Knowledge Context Identification (p. 79)	Canonical; business:674 (p. 543); business:670 (p. 543)
CT2: Clinical Knowledge Management (p. 78)	CT3: Clinical Knowledge Context Extraction (p. 80)	Canonical; business:675 (p. 543); business:670 (p. 543)
CT2: Clinical Knowledge Management (p. 78)	CT3: Clinical Knowledge Information Formulation (p. 80)	Canonical; business:676 (p. 543); business:670 (p. 543)
CT2: Clinical Knowledge Management (p. 78)	CT3: Clinical Knowledge Concept Association (p. 81)	Canonical; business:677 (p. 543); business:670 (p. 543)
CT2: Clinical Knowledge Management (p. 78)	CT3: Clinical Knowledge Review & Approval Management (p. 82)	Canonical; business:678 (p. 543); business:670 (p. 543)
CT2: Clinical Knowledge Management (p. 78)	CT3: Clinical Knowledge Publication & Sharing (p. 83)	Canonical; business:679 (p. 543); business:670 (p. 543)
CT1: Standards, Semantics & Reference Governance (p. 83)	CT2: Health Information Standards Governance (p. 84)	Canonical; business:693 (p. 544)
CT2: Health Information Standards Governance (p. 84)	CT3: Information Model Discovery (p. 85)	Canonical; business:701 (p. 544); business:697 (p. 544)
CT2: Health Information Standards Governance (p. 84)	CT3: Information Model Adoption (p. 86)	Canonical; business:702 (p. 544); business:697 (p. 544)
CT2: Health Information Standards Governance (p. 84)	CT3: Information Model Education (p. 87)	Canonical; business:703 (p. 544); business:697 (p. 544)
CT2: Health Information Standards Governance (p. 84)	CT3: Information Model Compliance (p. 87)	Canonical; business:704 (p. 544); business:697 (p. 544)
CT2: Health Information Standards Governance (p. 84)	CT3: Information Model Governance Oversight (p. 88)	Canonical; business:705 (p. 544); business:697 (p. 544)
CT2: Health Information Standards Governance (p. 84)	CT3: Standards Adoption (p. 89)	Canonical; business:706 (p. 544); business:697 (p. 544)
CT2: Health Information Standards Governance (p. 84)	CT3: Standards Impact Identification (p. 89)	Canonical; business:707 (p. 544); business:697 (p. 544)
CT1: Standards, Semantics & Reference Governance (p. 83)	CT2: Clinical Semantics Governance (p. 90)	Canonical; business:713 (p. 544)

From	To	Status / evidence
CT2: Clinical Semantics Governance (p. 90)	CT3: Terminology Governance (p. 91)	Canonical; business:721 (p. 544); business:717 (p. 544)
CT2: Clinical Semantics Governance (p. 90)	CT3: Terminology Utilisation Oversight (p. 92)	Canonical; business:722 (p. 544); business:717 (p. 544)
CT2: Clinical Semantics Governance (p. 90)	CT3: Terminology Localisation (p. 92)	Canonical; business:723 (p. 544); business:717 (p. 544)
CT2: Clinical Semantics Governance (p. 90)	CT3: Classification Governance (p. 93)	Canonical; business:724 (p. 544); business:717 (p. 544)
CT2: Clinical Semantics Governance (p. 90)	CT3: Classification Utilisation Oversight (p. 94)	Canonical; business:725 (p. 544); business:717 (p. 544)
CT2: Clinical Semantics Governance (p. 90)	CT3: Semantic Mapping Governance (p. 94)	Canonical; business:726 (p. 544); business:717 (p. 544)
CT1: Connected Health Services (p. 95)	CT2: Health Information Exchange (p. 96)	Canonical; business:740 (p. 545)
CT1: Connected Health Services (p. 95)	CT2: Digital Communication Channel (p. 97)	Canonical; business:750 (p. 545)
CT1: Security, Privacy & Digital Trust (p. 98)	CT2: Digital Identity & Access Governance (p. 99)	Canonical; business:768 (p. 546)
CT2: Digital Identity & Access Governance (p. 99)	CT3: Identity Assurance (p. 100)	Canonical; business:776 (p. 546); business:772 (p. 546)
CT2: Digital Identity & Access Governance (p. 99)	CT3: Identity Modelling & Interoperability (p. 101)	Canonical; business:777 (p. 546); business:772 (p. 546)
CT2: Digital Identity & Access Governance (p. 99)	CT3: Identity Qualification Control (p. 101)	Canonical; business:778 (p. 546); business:772 (p. 546)
CT2: Digital Identity & Access Governance (p. 99)	CT3: Identity Identification & Authentication (p. 102)	Canonical; business:779 (p. 546); business:772 (p. 546)
CT2: Digital Identity & Access Governance (p. 99)	CT3: Access Control Context Management (p. 103)	Canonical; business:780 (p. 546); business:772 (p. 546)
CT2: Digital Identity & Access Governance (p. 99)	CT3: Access Control Assurance & Reporting (p. 103)	Canonical; business:781 (p. 546); business:772 (p. 546)
CT2: Digital Identity & Access Governance (p. 99)	CT3: Access Control Collaboration (p. 104)	Canonical; business:782 (p. 546); business:772 (p. 546)
CT2: Digital Identity & Access Governance (p. 99)	CT3: Access Control Enforcement (p. 105)	Canonical; business:783 (p. 546); business:772 (p. 546)
CT1: Security, Privacy & Digital Trust (p. 98)	CT2: Health Information Protection (p. 105)	Canonical; business:789 (p. 546)
CT2: Health Information Protection (p. 105)	CT3: Health Privacy Safeguard Management (p. 106)	Canonical; business:797 (p. 546); business:793 (p. 546)
CT2: Health Information Protection (p. 105)	CT3: Health Information Safeguarding (p. 107)	Canonical; business:798 (p. 546); business:793 (p. 546)
CT2: Health Information Protection (p. 105)	CT3: Health Information Access Monitoring (p. 108)	Canonical; business:799 (p. 546); business:793 (p. 546)
CT2: Health Information Protection (p. 105)	CT3: Health Information Compartment Assurance (p. 108)	Canonical; business:800 (p. 546); business:793 (p. 546)
CT2: Health Information Protection (p. 105)	CT3: Health Information Classification Assurance (p. 109)	Canonical; business:801 (p. 546); business:793 (p. 546)
CT1: Security, Privacy & Digital Trust (p. 98)	CT2: Digital Accountability & Trust (p. 110)	Canonical; business:807 (p. 547)
CT2: Digital Accountability & Trust (p. 110)	CT3: Governed Activity Attribution (p. 111)	Canonical; business:815 (p. 547); business:811 (p. 547)
CT2: Digital Accountability & Trust (p. 110)	CT3: Security & Information-Use Accountability (p. 111)	Canonical; business:816 (p. 547); business:811 (p. 547)
CT1: Health Research & Innovation (p. 112)	CT2: Research Information Support (p. 113)	Canonical; business:834 (p. 547)

From	To	Status / evidence
CT2: Research Information Support (p. 113)	CT3: Governed Research Information Provision (p. 114)	Canonical; business:842 (p. 547); business:838 (p. 547)
CT1: Health Research & Innovation (p. 112)	CT2: Health Innovation (p. 115)	Canonical; business:848 (p. 548)
CT2: Health Innovation (p. 115)	CT3: Health Innovation Evaluation (p. 115)	Canonical; business:856 (p. 548); business:852 (p. 548)
CT1: Health Service Direction & Stewardship (p. 116)	CT2: Health Service Modelling (p. 117)	Canonical; business:874 (p. 548)
CT2: Health Service Modelling (p. 117)	CT3: Health Service Need Identification (p. 118)	Canonical; business:882 (p. 548); business:878 (p. 548)
CT2: Health Service Modelling (p. 117)	CT3: Health Service Capability Identification (p. 119)	Canonical; business:883 (p. 548); business:878 (p. 548)
CT2: Health Service Modelling (p. 117)	CT3: Health Service Implementation Support (p. 120)	Canonical; business:884 (p. 548); business:878 (p. 548)
CT2: Health Service Modelling (p. 117)	CT3: Health Service Continuous Improvement (p. 120)	Canonical; business:885 (p. 548); business:878 (p. 548)
CT1: Health Service Direction & Stewardship (p. 116)	CT2: Health Service Alignment (p. 121)	Canonical; business:891 (p. 549)
CT2: Health Service Alignment (p. 121)	CT3: Health Service Co-Dependency Management (p. 122)	Canonical; business:899 (p. 549); business:895 (p. 549)
CT2: Health Service Alignment (p. 121)	CT3: Health Service Resource Identification (p. 123)	Canonical; business:900 (p. 549); business:895 (p. 549)
CT2: Health Service Alignment (p. 121)	CT3: Health Service Financial Management (p. 123)	Canonical; business:901 (p. 549); business:895 (p. 549)
CT2: Health Service Alignment (p. 121)	CT3: Health Service Implementation Planning (p. 124)	Canonical; business:902 (p. 549); business:895 (p. 549)
CT1: Workforce & Organisational Capability (p. 125)	CT2: Health Workforce Management (p. 126)	Canonical; business:918 (p. 549)
CT2: Health Workforce Management (p. 126)	CT3: Practitioner Qualification Management (p. 127)	Canonical; business:926 (p. 549); business:922 (p. 549)
CT2: Health Workforce Management (p. 126)	CT3: Practitioner Engagement & Privilege Management (p. 127)	Canonical; business:927 (p. 549); business:922 (p. 549)
CT2: Health Workforce Management (p. 126)	CT3: Workforce Service Coverage Management (p. 128)	Canonical; business:928 (p. 549); business:922 (p. 549)
CT1: Workforce & Organisational Capability (p. 125)	CT2: Healthcare Organisation & Service Structure Management (p. 129)	Canonical; business:934 (p. 550)
CT2: Healthcare Organisation & Service Structure Management (p. 129)	CT3: Healthcare Organisation Identity & Structure Management (p. 130)	Canonical; business:942 (p. 550); business:938 (p. 550)
CT2: Healthcare Organisation & Service Structure Management (p. 129)	CT3: Healthcare Location Definition Management (p. 131)	Canonical; business:943 (p. 550); business:938 (p. 550)
CT2: Healthcare Organisation & Service Structure Management (p. 129)	CT3: Healthcare Service Offering Management (p. 131)	Canonical; business:944 (p. 550); business:938 (p. 550)
CT1: Corporate Resources & Enterprise Services (p. 132)	CT2: Financial & Commercial Services (p. 133)	Canonical; business:958 (p. 550)
CT2: Financial & Commercial Services (p. 133)	CT3: Clinical Activity Charging Information Provision (p. 134)	Canonical; business:966 (p. 551); business:962 (p. 550)
CT1: Corporate Resources & Enterprise Services (p. 132)	CT2: Corporate Support Services (p. 135)	Canonical; business:972 (p. 551)
CT2: Corporate Support Services (p. 135)	CT3: Back-Office Services (p. 136)	Canonical; business:980 (p. 551); business:976 (p. 551)

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CT2: Corporate Support Services (p. 135)	CT3: Facilities Management (p. 136)	Canonical; business:981 (p. 551); business:976 (p. 551)
CT2: Corporate Support Services (p. 135)	CT3: IT Infrastructure Management (p. 137)	Canonical; business:982 (p. 551); business:976 (p. 551)
CT2: Corporate Support Services (p. 135)	CT3: Communications & Publicity (p. 138)	Canonical; business:983 (p. 551); business:976 (p. 551)
CT2: Corporate Support Services (p. 135)	CT3: People Management (p. 138)	Canonical; business:984 (p. 551); business:976 (p. 551)
CT2: Corporate Support Services (p. 135)	CT3: Operational Oversight (p. 139)	Canonical; business:985 (p. 551); business:976 (p. 551)
CT1: Health Service Governance (p. 140)	CT2: Governance Framework Management (p. 141)	Canonical; business:1005 (p. 552)
CT2: Governance Framework Management (p. 141)	CT3: Policy / Obligation Governance (p. 142)	Canonical; business:1013 (p. 552); business:1009 (p. 552)
CT2: Governance Framework Management (p. 141)	CT3: Authority & Constraint Governance (p. 142)	Canonical; business:1014 (p. 552); business:1009 (p. 552)
CT1: Health Service Governance (p. 140)	CT2: Compliance Governance (p. 143)	Canonical; business:1020 (p. 552)
CT2: Compliance Governance (p. 143)	CT3: Compliance Assessment Definition (p. 144)	Canonical; business:1028 (p. 552); business:1024 (p. 552)
CT2: Compliance Governance (p. 143)	CT3: Governance Reporting & Checkpoint Definition (p. 145)	Canonical; business:1029 (p. 552); business:1024 (p. 552)
CT1: Health Service Assurance (p. 145)	CT2: Health Service Assurance Specification (p. 146)	Canonical; business:1045 (p. 553)
CT2: Health Service Assurance Specification (p. 146)	CT3: Assurance Approach Definition (p. 147)	Canonical; business:1053 (p. 553); business:1049 (p. 553)
CT2: Health Service Assurance Specification (p. 146)	CT3: Assurance Evaluation Criteria Governance (p. 148)	Canonical; business:1054 (p. 553); business:1049 (p. 553)
CT1: Health Service Assurance (p. 145)	CT2: Assurance Evaluation (p. 149)	Canonical; business:1060 (p. 553)
CT2: Assurance Evaluation (p. 149)	CT3: Assurance Evidence Assessment (p. 150)	Canonical; business:1068 (p. 553); business:1064 (p. 553)
CT2: Assurance Evaluation (p. 149)	CT3: Assurance Conclusion Determination (p. 150)	Canonical; business:1069 (p. 553); business:1064 (p. 553)

5.2. Realises Feature

From	To	Status / evidence
Function: Resolve Person Identifier (p. 196)	Feature: Identifier Resolution (p. 193)	Canonical; em:24 (p. 634)
Function: Correlate Person Identifiers (p. 197)	Feature: Cross-Authority Identifier Correlation (p. 194)	Canonical; em:27 (p. 634)
Function: Maintain Person Identity Aliases (p. 197)	Feature: Identity Alias Association (p. 195)	Canonical; em:30 (p. 634)
Function: Apply Governed Person Identity Correction (p. 198)	Feature: Governed Identity Correction (p. 195)	Canonical; em:33 (p. 634)
Function: Establish Healthcare Subject Context (p. 202)	Feature: Subject Context Binding (p. 201)	Canonical; em:45 (p. 635)
Function: Govern Subject Demographic Context (p. 203)	Feature: Demographic Context Governance (p. 201)	Canonical; em:48 (p. 635)
Function: Maintain Client Relationships (p. 206)	Feature: Next-of-Kin & Guardian Association (p. 205)	Canonical; em:59 (p. 635)
Function: Govern Client Relationship Validity (p. 207)	Feature: Relationship Validity Tracking (p. 205)	Canonical; em:62 (p. 635)
Function: Evaluate Client Consent (p. 210)	Feature: Consent Directive Evaluation (p. 208)	Canonical; em:73 (p. 636)

From	To	Status / evidence
Function: Evaluate Client Confidentiality Restrictions (p. 210)	Feature: Confidentiality Flag Enforcement (p. 209)	Canonical; em:76 (p. 636)
Function: Verify Practitioner Registration (p. 160)	Feature: National Practitioner Verification (p. 156)	Canonical; em:92 (p. 636)
Function: Govern Practitioner Profile (p. 161)	Feature: Practitioner Profile Governance (p. 157)	Canonical; em:96 (p. 636)
Function: Maintain Practitioner Roles (p. 162)	Feature: Role & Specialty Modeling (p. 158)	Canonical; em:99 (p. 636)
Function: Bind Practitioner Electronic Endpoints (p. 163)	Feature: Electronic Endpoint Binding (p. 158)	Canonical; em:102 (p. 636)
Function: Maintain Practitioner Affiliations (p. 163)	Feature: Practitioner-Organisation Affiliation (p. 159)	Canonical; em:105 (p. 636)
Function: Maintain Clinical Privileges (p. 164)	Feature: Clinical Privileging Record (p. 160)	Canonical; em:108 (p. 636)
Function: Verify Healthcare Organisation (p. 169)	Feature: National Organisation Verification (p. 167)	Canonical; em:121 (p. 637)
Function: Govern Organisation Profile (p. 170)	Feature: Organisation Profile Management (p. 167)	Canonical; em:124 (p. 637)
Function: Maintain Organisational Structure (p. 171)	Feature: Hierarchical Structure Navigation (p. 168)	Canonical; em:127 (p. 637)
Function: Maintain Organisation Contacts (p. 172)	Feature: Directory Contact Management (p. 169)	Canonical; em:130 (p. 637)
Function: Maintain Healthcare Location (p. 176)	Feature: Location Geospatial & Address Resolution (p. 174)	Canonical; em:145 (p. 638)
Function: Maintain Physical Location Hierarchy (p. 176)	Feature: Physical Hierarchy Modeling (p. 174)	Canonical; em:148 (p. 638)
Function: Maintain Care-Place Definition (p. 177)	Feature: Atomic Care-Place Modeling (p. 175)	Canonical; em:151 (p. 638)
Function: Maintain Healthcare Service Definition (p. 182)	Feature: Service Catalogue Publishing (p. 179)	Canonical; em:174 (p. 638)
Function: Maintain Service / Location / Provider Map (p. 182)	Feature: Service-Location-Provider Binding (p. 180)	Canonical; em:177 (p. 638)
Function: Maintain Service Availability (p. 183)	Feature: Operating Schedule Specification (p. 180)	Canonical; em:180 (p. 638)
Function: Maintain Service Eligibility Rules (p. 184)	Feature: Eligibility & Catchment Specification (p. 181)	Canonical; em:183 (p. 638)
Function: Maintain Device Definition (p. 189)	Feature: Device Type Specification (p. 186)	Canonical; em:196 (p. 639)
Function: Maintain Device Instance Identity (p. 190)	Feature: Device Serial Tracking (p. 187)	Canonical; em:199 (p. 639)
Function: Maintain Device Association (p. 190)	Feature: Device-Patient/Location Assignment (p. 188)	Canonical; em:202 (p. 639)
Function: Maintain Device Communication Endpoints (p. 191)	Feature: Device Endpoint Registration (p. 188)	Canonical; em:205 (p. 639)
Function: Receive Client Demographic Change (p. 214)	Feature: Demographic Update Ingestion (p. 213)	Canonical; sa:18 (p. 640)
Function: Distribute Governed Demographic Change (p. 215)	Feature: Demographic Change Distribution (p. 213)	Canonical; sa:21 (p. 640)
Function: Receive Referral (p. 218)	Feature: Referral Document Ingestion (p. 216)	Canonical; sa:33 (p. 640)
Function: Assemble Referral Context (p. 219)	Feature: Referral Supporting Information (p. 217)	Canonical; sa:36 (p. 640)

From	To	Status / evidence
Function: Manage Referral Progression (p. 220)	Feature: Referral Status & Outcome Tracking (p. 218)	Canonical; sa:39 (p. 640)
Function: Receive Appointment Notification (p. 224)	Feature: Appointment Notification Ingestion (p. 223)	Canonical; sa:53 (p. 641)
Function: Synchronise Appointment Status (p. 225)	Feature: Appointment Status Synchronization (p. 223)	Canonical; sa:56 (p. 641)
Function: Establish Encounter Context (p. 228)	Feature: Encounter Context Creation (p. 226)	Canonical; sa:68 (p. 641)
Function: Progress Encounter State (p. 229)	Feature: Encounter State Progression (p. 227)	Canonical; sa:71 (p. 641)
Function: Maintain Encounter Care-Place Association (p. 230)	Feature: Encounter Bed/Location Association (p. 228)	Canonical; sa:76 (p. 641)
Function: Receive Order Request (p. 236)	Feature: Order Request Ingestion (p. 233)	Canonical; sa:89 (p. 642)
Function: Resolve Order Destination (p. 236)	Feature: Order Destination Resolution & Routing (p. 234)	Canonical; sa:92 (p. 642)
Function: Manage Order Progression (p. 237)	Feature: Order Closed-Loop Progression Tracking (p. 234)	Canonical; sa:95 (p. 642)
Function: Coordinate Order Modification / Cancellation (p. 238)	Feature: Order Cancellation & Modification Coordination (p. 235)	Canonical; sa:100 (p. 642)
Function: Correlate Diagnostic Request (p. 244)	Feature: Diagnostic Request Correlation (p. 242)	Canonical; sa:116 (p. 643)
Function: Bind Diagnostic Report (p. 244)	Feature: Diagnostic Report Ingestion & Binding (p. 242)	Canonical; sa:119 (p. 643)
Function: Distribute Diagnostic Report (p. 245)	Feature: Diagnostic Report Distribution (p. 243)	Canonical; sa:122 (p. 643)
Function: Receive Medication Order (p. 249)	Feature: Medication Order Ingestion (p. 247)	Canonical; sa:136 (p. 643)
Function: Track Dispense Event (p. 250)	Feature: Dispense Event Tracking (p. 248)	Canonical; sa:139 (p. 643)
Function: Receive Medication Administration Event (p. 250)	Feature: Medication Administration Record (MAR) Ingestion (p. 248)	Canonical; sa:142 (p. 643)
Function: Receive Procedure Booking / Request (p. 253)	Feature: Procedure Booking Ingestion (p. 252)	Canonical; sa:153 (p. 644)
Function: Receive Procedural Documentation (p. 254)	Feature: Procedural Documentation Ingestion (p. 253)	Canonical; sa:156 (p. 644)
Function: Receive Clinical Document (p. 258)	Feature: Clinical Document Ingestion (p. 256)	Canonical; sa:168 (p. 644)
Function: Govern Clinical Document Lifecycle (p. 259)	Feature: Document Versioning & Supersession (p. 257)	Canonical; sa:171 (p. 644)
Function: Index Clinical Document Metadata (p. 260)	Feature: Document Metadata Indexing (p. 257)	Canonical; sa:176 (p. 644)
Function: Receive Care Plan (p. 263)	Feature: Care Plan Ingestion & Distribution (p. 262)	Canonical; sa:186 (p. 645)
Function: Distribute Clinical Communication (p. 268)	Feature: Clinical Communication Distribution (p. 266)	Canonical; sa:206 (p. 645)
Function: Track Business Delivery Acknowledgement (p. 269)	Feature: Delivery Acknowledgement Tracking (p. 267)	Canonical; sa:209 (p. 645)
Function: Record Communication Transaction (p. 270)	Feature: Communication Audit Logging (p. 268)	Canonical; sa:213 (p. 645)

From	To	Status / evidence
Function: Correlate Primary Care Consultation Context (p. 275)	Feature: General Practice Shared Record Ingestion (p. 274)	Canonical; sd:55 (p. 648)
Function: Assemble Acute Clinical Baseline (p. 279)	Feature: Acute Care Clinical Context Provision (p. 278)	Canonical; sd:75 (p. 649)
Function: Bind ED Triage Assessment (p. 283)	Feature: Emergency Department Arrival & Triage Ingestion (p. 282)	Canonical; sd:95 (p. 649)
Function: Provide Emergency LHR Summary (p. 284)	Feature: Emergency Care Cross-Facility Correlation (p. 283)	Canonical; sd:98 (p. 649)
Function: Consolidate Inpatient Clinical Round Feed (p. 287)	Feature: Inpatient Rounding Context Provision (p. 286)	Canonical; sd:114 (p. 650)
Function: Project Longitudinal Medication History (p. 297)	Feature: Medication History Reconciliation Ingestion (p. 296)	Canonical; sd:151 (p. 651)
Function: Syndicate Immunisation Record (p. 301)	Feature: Immunisation Event Submission (p. 300)	Canonical; sd:168 (p. 651)
Function: Synchronize Community Visit Record (p. 306)	Feature: Community Encounter Ingestion (p. 305)	Canonical; sd:187 (p. 652)
Function: Provide Outreach Visit Context (p. 310)	Feature: Outreach Visit Context Provision (p. 309)	Canonical; sd:206 (p. 652)
Function: Reconcile Outreach Clinical Batch (p. 310)	Feature: Outreach Encounter Outcome Ingestion (p. 309)	Canonical; sd:210 (p. 652)
Function: Correlate Virtual Care Consultation Context (p. 313)	Feature: Virtual Care Session Context Binding (p. 313)	Canonical; sd:225 (p. 653)
Function: Ingest Remote Patient Telemetry (p. 314)	Feature: Remote Telemetry Ingestion (p. 312)	Canonical; sd:228 (p. 653)
Function: Track Clinic Operating Session (p. 318)	Feature: Clinic Session Tracking (p. 317)	Canonical; hso:78 (p. 656)
Function: Manage Clinic Queue Progression (p. 319)	Feature: Operational Queue Progression (p. 317)	Canonical; hso:82 (p. 656)
Function: Assemble Ward Handover Context (p. 323)	Feature: Clinical Handover Context Assembly (p. 322)	Canonical; hso:99 (p. 656)
Function: Manage Theatre Case Progression (p. 327)	Feature: Operating Theatre Case Progression (p. 325)	Canonical; hso:112 (p. 657)
Function: Capture Post-Clinic Follow-up Requirements (p. 335)	Feature: Post-Clinic Order Coordination (p. 333)	Canonical; hso:144 (p. 658)
Function: Maintain Operational Bed State (p. 338)	Feature: Bed Availability Tracking (p. 336)	Canonical; hso:157 (p. 658)
Function: Manage Bed Turnover Progression (p. 339)	Feature: Bed State Progression (p. 337)	Canonical; hso:160 (p. 658)
Function: Track Mobile Clinical Asset Operational State (p. 342)	Feature: Mobile Clinical Equipment Tracking (p. 342)	Canonical; hso:172 (p. 659)
Function: Aggregate Service Capacity Telemetry (p. 344)	Feature: Operational Capacity Metric Aggregation (p. 344)	Canonical; hso:182 (p. 659)
Function: Receive Mobile Visit Progression (p. 350)	Feature: Mobile Visit Status Tracking (p. 348)	Canonical; hso:200 (p. 659)
Function: Integrate Mobile Work Management (p. 351)	Feature: Mobile Worker Task Dispatch (p. 348)	Canonical; hso:203 (p. 659)
Function: Resolve Active On-Call Coverage (p. 353)	Feature: Active On-Call Provider Resolution (p. 352)	Canonical; hso:214 (p. 660)
Function: Dispatch Operational Work Order (p. 357)	Feature: Work Item Instantiation (p. 355)	Canonical; hso:227 (p. 661)
Function: Manage Operational Work Progression (p. 358)	Feature: Work Progress Oversight & Escalation (p. 357)	Canonical; hso:231 (p. 661)

From	To	Status / evidence
Function: Manage Patient Transport Progression (p. 366)	Feature: Transport Dispatch & Progress Tracking (p. 365)	Canonical; hso:253 (p. 661)
Function: Manage Specimen Transport Progression (p. 369)	Feature: Pathology Specimen Transport Tracking (p. 368)	Canonical; hso:267 (p. 662)
Function: Manage Discharge Coordination Progression (p. 374)	Feature: Discharge Readiness & Coordination Oversight (p. 372)	Canonical; hso:279 (p. 662)
Function: Verify Clinical Qualification for Work Dispatch (p. 363)	Feature: Operational Credential Check (p. 362)	Canonical; hso:295 (p. 663)
Function: Assemble Longitudinal Clinical Record (p. 381)	Feature: Longitudinal Health Record (LHR) Assembly (p. 379)	Canonical; ise:39 (p. 665)
Function: Maintain Active Clinical Record (p. 382)	Feature: Active Clinical Record Access (p. 380)	Canonical; ise:42 (p. 665)
Function: Preserve Durable Clinical Record (p. 383)	Feature: Durable Clinical Record Preservation (p. 380)	Canonical; ise:45 (p. 665)
Function: Receive Information Submission (p. 387)	Feature: Direct Submission Ingestion & Delivery (p. 385)	Canonical; ise:57 (p. 665)
Function: Mediate Information Retrieval (p. 388)	Feature: Clinical Record Retrieval Mediation (p. 385)	Canonical; ise:60 (p. 665)
Function: Distribute Addressed Information (p. 389)	Feature: Addressed Clinical Distribution (p. 386)	Canonical; ise:63 (p. 665)
Function: Syndicate Information Change (p. 390)	Feature: Event-Driven Syndication (p. 387)	Canonical; ise:66 (p. 665)
Function: Search Health Information (p. 393)	Feature: Federated Clinical Query (p. 392)	Canonical; ise:78 (p. 666)
Function: Mediate Standards-Based Health Information Communication (p. 397)	Feature: Standards-Based Boundary Exchange (p. 396)	Canonical; ise:97 (p. 666)
Function: Evaluate Information Access Authority (p. 401)	Feature: Health Information Access & Consent Control (p. 399)	Canonical; ise:108 (p. 667)
Function: Record Security-Significant Activity (p. 404)	Feature: Non-PHI Compliance Auditing (p. 400)	Canonical; ise:119 (p. 667)
Function: Resolve Clinical Concept (p. 407)	Feature: Clinical Concept Resolution (p. 405)	Canonical; ise:130 (p. 667)
Function: Map Clinical Concept (p. 407)	Feature: Bidirectional Terminology Mapping (p. 406)	Canonical; ise:133 (p. 667)
Function: Coordinate Care-Team Collaboration (p. 411)	Feature: Care-Team Clinical Collaboration Coordination (p. 409)	Canonical; ise:146 (p. 668)
Function: Resolve LHR Query within Collaboration (p. 412)	Feature: In-Conversation LHR Query Resolution (p. 410)	Canonical; ise:149 (p. 668)
Function: Govern Collaboration Metadata (p. 413)	Feature: Zero-PHI Collaboration Metadata Governance (p. 411)	Canonical; ise:154 (p. 668)
Function: Coordinate Operational Collaboration (p. 415)	Feature: Operational Team Chat Enablement (p. 415)	Canonical; ise:164 (p. 668)
Function: Coordinate Work Order (p. 420)	Feature: Work Order Progression (p. 418)	Canonical; ise:181 (p. 668)
Function: Coordinate To Do (p. 421)	Feature: To Do Decision & Approval Coordination (p. 418)	Canonical; ise:185 (p. 668)
Function: Coordinate Synthetic Task (p. 422)	Feature: Synthetic Task Orchestration (p. 419)	Canonical; ise:189 (p. 668)
Function: Supervise Activity Timeout / Escalation (p. 423)	Feature: Workflow Timeout & Escalation Oversight (p. 419)	Canonical; ise:193 (p. 668)

From	To	Status / evidence
Function: Project Operational Schedule (p. 428)	Feature: Schedule Projection Integration (p. 427)	Canonical; ise:205 (p. 669)
Function: Correlate Activity with Temporal Context (p. 429)	Feature: Temporal Event Correlation (p. 428)	Canonical; ise:208 (p. 669)
Function: Present Contextual Health and Operational Information (p. 431)	Feature: Longitudinal Clinical & Operational Presentation (p. 431)	Canonical; ise:220 (p. 670)
Function: Govern Information Semantics (p. 435)	Feature: Information Standards & Semantic Governance (p. 433)	Canonical; ise:231 (p. 670)
Function: Verify Semantic Conformance (p. 438)	Feature: Semantic Conformance Verification (p. 434)	Canonical; ise:243 (p. 670)
Function: Manage Theatre Case Progression (p. 327)	Feature: Perioperative Resource Notification (p. 326)	Canonical; hso:114 (p. 657)
Function: Coordinate ED Departmental Capacity (p. 330)	Feature: ED Bed & Bay Tracking (p. 329)	Canonical; hso:127 (p. 657)
Function: Manage Outpatient Capacity (p. 334)	Feature: Outpatient Arrival Registration (p. 332)	Canonical; hso:139 (p. 658)
Function: Manage Bed Turnover Progression (p. 339)	Feature: Bed Readiness Notification (p. 338)	Canonical; hso:162 (p. 658)
Function: Dispatch Operational Work Order (p. 357)	Feature: Worker Matching & Allocation (p. 355)	Canonical; hso:229 (p. 661)
Function: Dispatch Operational Work Order (p. 357)	Feature: Work Dispatch Delivery (p. 356)	Canonical; hso:229 (p. 661)
Function: Manage Patient Transport Progression (p. 366)	Feature: Transport Request Ingestion (p. 365)	Canonical; hso:256 (p. 661)
Function: Manage Discharge Coordination Progression (p. 374)	Feature: Discharge Milestone Tracking (p. 372)	Canonical; hso:280 (p. 662)
Function: Manage Discharge Coordination Progression (p. 374)	Feature: Discharge Notification Dispatch (p. 373)	Canonical; hso:281 (p. 662)
Function: Coordinate Multi-Agency Chronic Care Plan (p. 276)	Feature: Primary Care Notification Dispatch (p. 275)	**Unresolved; association not established**; sd:58 (p. 648)
Function: Route Acute Critical Result Alert (p. 280)	Feature: Acute Clinical State Event Capture (p. 278)	**Unresolved; association not established**; sd:78 (p. 649)
Function: Assemble Inpatient Discharge Dossier (p. 288)	Feature: Inpatient Clinical Progression Tracking (p. 287)	**Unresolved; association not established**; sd:117 (p. 650)
Function: Receive Diagnostic Requisition Feed (p. 292)	Feature: Laboratory & Imaging Result Distribution (p. 291)	**Unresolved; association not established**; sd:133 (p. 650)
Function: Publish Governed Diagnostic Report (p. 293)	Feature: Diagnostic History Consolidation (p. 292)	**Unresolved; association not established**; sd:137 (p. 650)
Function: Correlate Adverse Reaction & Allergy Profile (p. 298)	Feature: Medication Administration Tracking (p. 296)	**Unresolved; association not established**; sd:154 (p. 651)
Function: Project Screening Eligibility Timeline (p. 302)	Feature: Screening Recall Notification Distribution (p. 301)	**Unresolved; association not established**; sd:171 (p. 651)
Function: Bind Community Support Network Context (p. 307)	Feature: Community Care Plan Synchronization (p. 305)	**Unresolved; association not established**; sd:190 (p. 652)
Function: Track Operational Staff Presence (p. 349)	Feature: Mobile Worker Task Dispatch (p. 348)	**Unresolved; association not established**; hso:196 (p. 659)
Function: Retrieve Health Information (p. 394)	Feature: Access-Controlled Information Filtering (p. 392)	**Unresolved; association not established**; ise:81 (p. 666)
Function: Evaluate Consent Constraint (p. 401)	Feature: Security Context Propagation (p. 399)	**Unresolved; association not established**; ise:111 (p. 667)

5.3. Associated Feature

From	To	Status / evidence
Service: Person Identifier Resolution (p. 196)	Feature: Identifier Resolution (p. 193)	Canonical; em:25 (p. 634)
Service: Person Identifier Correlation (p. 197)	Feature: Cross-Authority Identifier Correlation (p. 194)	Canonical; em:28 (p. 634)
Service: Person Identity Correction (p. 198)	Feature: Governed Identity Correction (p. 195)	Canonical; em:34 (p. 634)
Service: Healthcare Subject Context Resolution (p. 202)	Feature: Subject Context Binding (p. 201)	Canonical; em:46 (p. 635)
Service: Demographic Change Distribution (p. 203)	Feature: Demographic Context Governance (p. 201)	Canonical; em:49 (p. 635)
Service: Client Relationship Query (p. 206)	Feature: Next-of-Kin & Guardian Association (p. 205)	Canonical; em:60 (p. 635)
Service: Representative Authority Verification (p. 207)	Feature: Relationship Validity Tracking (p. 205)	Canonical; em:63 (p. 635)
Service: Consent Decision Service (p. 210)	Feature: Consent Directive Evaluation (p. 208)	Canonical; em:74 (p. 636)
Service: Confidentiality Restriction Evaluation (p. 211)	Feature: Confidentiality Flag Enforcement (p. 209)	Canonical; em:77 (p. 636)
Service: Practitioner Verification Service (p. 161)	Feature: National Practitioner Verification (p. 156)	Canonical; em:93 (p. 636)
Service: Practitioner Directory Query (p. 161)	Feature: Practitioner Profile Governance (p. 157)	Canonical; em:97 (p. 636)
Service: Practitioner Role Resolution (p. 162)	Feature: Role & Specialty Modeling (p. 158)	Canonical; em:100 (p. 636)
Service: Practitioner Endpoint Lookup (p. 163)	Feature: Electronic Endpoint Binding (p. 158)	Canonical; em:103 (p. 636)
Service: Practitioner Affiliation Resolution (p. 164)	Feature: Practitioner-Organisation Affiliation (p. 159)	Canonical; em:106 (p. 636)
Service: Clinical Privilege Verification (p. 165)	Feature: Clinical Privileging Record (p. 160)	Canonical; em:109 (p. 636)
Service: Organisation Verification Service (p. 170)	Feature: National Organisation Verification (p. 167)	Canonical; em:122 (p. 637)
Service: Organisation Directory Query (p. 171)	Feature: Organisation Profile Management (p. 167)	Canonical; em:125 (p. 637)
Service: Organisational Hierarchy Resolution (p. 171)	Feature: Hierarchical Structure Navigation (p. 168)	Canonical; em:128 (p. 637)
Service: Organisation Contact Lookup (p. 172)	Feature: Directory Contact Management (p. 169)	Canonical; em:131 (p. 637)
Service: Location Directory Query (p. 176)	Feature: Location Geospatial & Address Resolution (p. 174)	Canonical; em:146 (p. 638)
Service: Location Hierarchy Resolution (p. 177)	Feature: Physical Hierarchy Modeling (p. 174)	Canonical; em:149 (p. 638)
Service: Care-Place Specification Lookup (p. 178)	Feature: Atomic Care-Place Modeling (p. 175)	Canonical; em:152 (p. 638)
Service: Healthcare Service Directory Query (p. 182)	Feature: Service Catalogue Publishing (p. 179)	Canonical; em:175 (p. 638)
Service: Service Provision Resolution (p. 183)	Feature: Service-Location-Provider Binding (p. 180)	Canonical; em:178 (p. 638)
Service: Service Availability Query (p. 184)	Feature: Operating Schedule Specification (p. 180)	Canonical; em:181 (p. 638)
Service: Service Eligibility Evaluation (p. 184)	Feature: Eligibility & Catchment Specification (p. 181)	Canonical; em:184 (p. 638)

From	To	Status / evidence
Service: Device Definition Query (p. 189)	Feature: Device Type Specification (p. 186)	Canonical; em:197 (p. 639)
Service: Device Instance Resolution (p. 190)	Feature: Device Serial Tracking (p. 187)	Canonical; em:200 (p. 639)
Service: Device Association Query (p. 191)	Feature: Device-Patient/Location Assignment (p. 188)	Canonical; em:203 (p. 639)
Service: Device Endpoint Resolution (p. 192)	Feature: Device Endpoint Registration (p. 188)	Canonical; em:206 (p. 639)
Service: Demographic Intake Service (p. 214)	Feature: Demographic Update Ingestion (p. 213)	Canonical; sa:19 (p. 640)
Service: Demographic Update Notification (p. 215)	Feature: Demographic Change Distribution (p. 213)	Canonical; sa:22 (p. 640)
Service: Referral Submission Service (p. 219)	Feature: Referral Document Ingestion (p. 216)	Canonical; sa:34 (p. 640)
Service: Referral Context Query (p. 220)	Feature: Referral Supporting Information (p. 217)	Canonical; sa:37 (p. 640)
Service: Referral Status & Outcome Service (p. 220)	Feature: Referral Status & Outcome Tracking (p. 218)	Canonical; sa:40 (p. 640)
Service: Appointment Notification Ingress (p. 224)	Feature: Appointment Notification Ingestion (p. 223)	Canonical; sa:54 (p. 641)
Service: Appointment Schedule Query (p. 225)	Feature: Appointment Status Synchronization (p. 223)	Canonical; sa:57 (p. 641)
Service: Encounter Context Resolution (p. 229)	Feature: Encounter Context Creation (p. 226)	Canonical; sa:69 (p. 641)
Service: Encounter Lifecycle Event Notification (p. 230)	Feature: Encounter State Progression (p. 227)	Canonical; sa:72 (p. 641)
Service: Encounter Location History Query (p. 230)	Feature: Encounter Bed/Location Association (p. 228)	Canonical; sa:77 (p. 641)
Service: Order Requisition Ingress (p. 236)	Feature: Order Request Ingestion (p. 233)	Canonical; sa:90 (p. 642)
Service: Order Dispatch Service (p. 237)	Feature: Order Destination Resolution & Routing (p. 234)	Canonical; sa:93 (p. 642)
Service: Order Status & Tracking Query (p. 238)	Feature: Order Closed-Loop Progression Tracking (p. 234)	Canonical; sa:96 (p. 642)
Service: Order Cancellation Service (p. 238)	Feature: Order Cancellation & Modification Coordination (p. 235)	Canonical; sa:101 (p. 642)
Service: Diagnostic Correlation Resolution (p. 244)	Feature: Diagnostic Request Correlation (p. 242)	Canonical; sa:117 (p. 643)
Service: Diagnostic Report Ingress (p. 245)	Feature: Diagnostic Report Ingestion & Binding (p. 242)	Canonical; sa:120 (p. 643)
Service: Diagnostic Result Delivery Service (p. 246)	Feature: Diagnostic Report Distribution (p. 243)	Canonical; sa:124 (p. 643)
Service: Medication Order Ingress (p. 249)	Feature: Medication Order Ingestion (p. 247)	Canonical; sa:137 (p. 643)
Service: Dispense Event Ingress (p. 250)	Feature: Dispense Event Tracking (p. 248)	Canonical; sa:140 (p. 643)
Service: Medication Administration Ingress (p. 251)	Feature: Medication Administration Record (MAR) Ingestion (p. 248)	Canonical; sa:143 (p. 643)
Service: Procedure Booking Ingress (p. 254)	Feature: Procedure Booking Ingestion (p. 252)	Canonical; sa:154 (p. 644)
Service: Procedural Documentation Ingress (p. 255)	Feature: Procedural Documentation Ingestion (p. 253)	Canonical; sa:157 (p. 644)
Service: Clinical Document Ingress (p. 258)	Feature: Clinical Document Ingestion (p. 256)	Canonical; sa:169 (p. 644)

From	To	Status / evidence
Service: Clinical Document Lifecycle Service (p. 259)	Feature: Document Versioning & Supersession (p. 257)	Canonical; sa:172 (p. 644)
Service: Document Metadata Registry Query (p. 260)	Feature: Document Metadata Indexing (p. 257)	Canonical; sa:177 (p. 644)
Service: Care Plan Ingress (p. 263)	Feature: Care Plan Ingestion & Distribution (p. 262)	Canonical; sa:187 (p. 645)
Service: Secure Clinical Message Dispatch (p. 269)	Feature: Clinical Communication Distribution (p. 266)	Canonical; sa:207 (p. 645)
Service: Communication Delivery Status Query (p. 269)	Feature: Delivery Acknowledgement Tracking (p. 267)	Canonical; sa:211 (p. 645)
Service: Communication Audit Query (p. 270)	Feature: Communication Audit Logging (p. 268)	Canonical; sa:214 (p. 645)
Service: Primary Care Summary Ingress (p. 276)	Feature: General Practice Shared Record Ingestion (p. 274)	Canonical; sd:56 (p. 648)
Service: Acute Admission Dossier Query (p. 280)	Feature: Acute Care Clinical Context Provision (p. 278)	Canonical; sd:76 (p. 649)
Service: ED Triage Stream Ingress (p. 284)	Feature: Emergency Department Arrival & Triage Ingestion (p. 282)	Canonical; sd:96 (p. 649)
Service: Emergency Fast-Path LHR Query (p. 285)	Feature: Emergency Care Cross-Facility Correlation (p. 283)	Canonical; sd:99 (p. 649)
Service: Ward Round Clinical Briefing Query (p. 288)	Feature: Inpatient Rounding Context Provision (p. 286)	Canonical; sd:115 (p. 650)
Service: Longitudinal Medication History Query (p. 297)	Feature: Medication History Reconciliation Ingestion (p. 296)	Canonical; sd:152 (p. 651)
Service: Immunisation History Query (p. 302)	Feature: Immunisation Event Submission (p. 300)	Canonical; sd:169 (p. 651)
Service: Community Care Encounter Ingress (p. 306)	Feature: Community Encounter Ingestion (p. 305)	Canonical; sd:188 (p. 652)
Service: Outreach Batch Reconciliation Service (p. 311)	Feature: Outreach Encounter Outcome Ingestion (p. 309)	Canonical; sd:211 (p. 652)
Service: Virtual Encounter Context Resolution (p. 314)	Feature: Virtual Care Session Context Binding (p. 313)	Canonical; sd:226 (p. 653)
Service: Remote Telemetry Ingress (p. 314)	Feature: Remote Telemetry Ingestion (p. 312)	Canonical; sd:229 (p. 653)
Service: Clinic Queue Telemetry Service (p. 319)	Feature: Operational Queue Progression (p. 317)	Canonical; hso:83 (p. 656)
Service: Theatre Case Progression Telemetry (p. 327)	Feature: Operating Theatre Case Progression (p. 325)	Canonical; hso:113 (p. 657)
Service: Bed Availability Query (p. 339)	Feature: Bed Availability Tracking (p. 336)	Canonical; hso:158 (p. 658)
Service: Bed Turnover Lifecycle Service (p. 339)	Feature: Bed State Progression (p. 337)	Canonical; hso:161 (p. 658)
Service: Clinical Resource Availability Query (p. 343)	Feature: Mobile Clinical Equipment Tracking (p. 342)	Canonical; hso:173 (p. 659)
Service: Regional Health System Capacity Query (p. 345)	Feature: Operational Capacity Metric Aggregation (p. 344)	Canonical; hso:183 (p. 659)
Service: On-Call Specialist Lookup (p. 353)	Feature: Active On-Call Provider Resolution (p. 352)	Canonical; hso:215 (p. 660)
Service: Work Dispatch Service (p. 358)	Feature: Work Item Instantiation (p. 355)	Canonical; hso:228 (p. 661)
Service: Operational Work Status Query (p. 359)	Feature: Work Progress Oversight & Escalation (p. 357)	Canonical; hso:232 (p. 661)

From	To	Status / evidence
Service: Patient Transport Tracking Service (p. 366)	Feature: Transport Dispatch & Progress Tracking (p. 365)	Canonical; hso:254 (p. 661)
Service: Specimen Transit Tracking Service (p. 370)	Feature: Pathology Specimen Transport Tracking (p. 368)	Canonical; hso:268 (p. 662)
Service: Discharge Readiness Telemetry (p. 374)	Feature: Discharge Readiness & Coordination Oversight (p. 372)	Canonical; hso:283 (p. 662)
Service: Longitudinal Clinical Record Query (p. 381)	Feature: Longitudinal Health Record (LHR) Assembly (p. 379)	Canonical; ise:40 (p. 665)
Service: Active Problem & Allergy Summary Query (p. 382)	Feature: Active Clinical Record Access (p. 380)	Canonical; ise:43 (p. 665)
Service: Historical Clinical Record Access (p. 383)	Feature: Durable Clinical Record Preservation (p. 380)	Canonical; ise:46 (p. 665)
Service: HIE Submission Gateway Service (p. 388)	Feature: Direct Submission Ingestion & Delivery (p. 385)	Canonical; ise:58 (p. 665)
Service: Federated Health Information Query (p. 389)	Feature: Clinical Record Retrieval Mediation (p. 385)	Canonical; ise:61 (p. 665)
Service: Addressed Document Delivery Service (p. 389)	Feature: Addressed Clinical Distribution (p. 386)	Canonical; ise:64 (p. 665)
Service: Clinical Event Syndication Stream (p. 390)	Feature: Event-Driven Syndication (p. 387)	Canonical; ise:67 (p. 665)
Service: Clinical Information Search Service (p. 393)	Feature: Federated Clinical Query (p. 392)	Canonical; ise:79 (p. 666)
Service: Standards Communication Gateway Service (p. 397)	Feature: Standards-Based Boundary Exchange (p. 396)	Canonical; ise:98 (p. 666)
Service: Policy Evaluation & Authorisation Service (p. 401)	Feature: Health Information Access & Consent Control (p. 399)	Canonical; ise:109 (p. 667)
Service: Security Audit Ingress (p. 404)	Feature: Non-PHI Compliance Auditing (p. 400)	Canonical; ise:120 (p. 667)
Service: Terminology Concept Resolution Service (p. 407)	Feature: Clinical Concept Resolution (p. 405)	Canonical; ise:131 (p. 667)
Service: Terminology Mapping Service (p. 408)	Feature: Bidirectional Terminology Mapping (p. 406)	Canonical; ise:134 (p. 667)
Service: Clinical Collaboration Space Service (p. 412)	Feature: Care-Team Clinical Collaboration Coordination (p. 409)	Canonical; ise:147 (p. 668)
Service: Collaboration Clinical Summary Resolution (p. 413)	Feature: In-Conversation LHR Query Resolution (p. 410)	Canonical; ise:150 (p. 668)
Service: Collaboration Space Governance Service (p. 414)	Feature: Zero-PHI Collaboration Metadata Governance (p. 411)	Canonical; ise:155 (p. 668)
Service: Operational Collaboration Service (p. 416)	Feature: Operational Team Chat Enablement (p. 415)	Canonical; ise:165 (p. 668)
Service: Work Order Coordination Service (p. 421)	Feature: Work Order Progression (p. 418)	Canonical; ise:182 (p. 668)
Service: To Do Management Service (p. 421)	Feature: To Do Decision & Approval Coordination (p. 418)	Canonical; ise:186 (p. 668)
Service: Synthetic Task Execution Service (p. 422)	Feature: Synthetic Task Orchestration (p. 419)	Canonical; ise:190 (p. 668)
Service: Activity Escalation Telemetry (p. 423)	Feature: Workflow Timeout & Escalation Oversight (p. 419)	Canonical; ise:194 (p. 668)
Service: Unified Calendar Projection Query (p. 429)	Feature: Schedule Projection Integration (p. 427)	Canonical; ise:206 (p. 669)
Service: Temporal Context Resolution (p. 429)	Feature: Temporal Event Correlation (p. 428)	Canonical; ise:209 (p. 669)

From	To	Status / evidence
Service: Contextual Presentation Service (p. 432)	Feature: Longitudinal Clinical & Operational Presentation (p. 431)	Canonical; ise:221 (p. 670)
Service: Semantic Model Definition Service (p. 435)	Feature: Information Standards & Semantic Governance (p. 433)	Canonical; ise:232 (p. 670)
Service: Semantic Conformance Verification Service (p. 438)	Feature: Semantic Conformance Verification (p. 434)	Canonical; ise:244 (p. 670)
Service: Chronic Care Timeline Query (p. 276)	Feature: Primary Care Notification Dispatch (p. 275)	**Unresolved; association not established**; sd:58 (p. 648)
Service: Acute Clinical Alert Dispatch (p. 280)	Feature: Acute Clinical State Event Capture (p. 278)	**Unresolved; association not established**; sd:78 (p. 649)
Service: Discharge Dossier Packaging Service (p. 288)	Feature: Inpatient Clinical Progression Tracking (p. 287)	**Unresolved; association not established**; sd:117 (p. 650)
Service: Modality Worklist Distribution (p. 293)	Feature: Laboratory & Imaging Result Distribution (p. 291)	**Unresolved; association not established**; sd:133 (p. 650)
Service: Diagnostic Publication Service (p. 294)	Feature: Diagnostic History Consolidation (p. 292)	**Unresolved; association not established**; sd:137 (p. 650)
Service: Allergy & Adverse Reaction Query (p. 298)	Feature: Medication Administration Tracking (p. 296)	**Unresolved; association not established**; sd:154 (p. 651)
Service: Preventive Screening Status Query (p. 303)	Feature: Screening Recall Notification Distribution (p. 301)	**Unresolved; association not established**; sd:171 (p. 651)
Service: Community Care Team Query (p. 307)	Feature: Community Care Plan Synchronization (p. 305)	**Unresolved; association not established**; sd:190 (p. 652)
Service: Staff Presence Telemetry Query (p. 350)	Feature: Mobile Worker Task Dispatch (p. 348)	**Unresolved; association not established**; hso:196 (p. 659)
Service: Clinical Resource Retrieval Service (p. 394)	Feature: Access-Controlled Information Filtering (p. 392)	**Unresolved; association not established**; ise:81 (p. 666)
Service: Consent Enforcement Service (p. 402)	Feature: Security Context Propagation (p. 399)	**Unresolved; association not established**; ise:111 (p. 667)

5.4. Participating Function

From	To	Status / evidence
Process: Assurance Process Design (p. 441)	Function: Design Service Assurance (p. 440)	Canonical; processes:361 (p. 621)
Process: Assurance Process Design (p. 441)	Function: Manage Assurance Criteria (p. 443)	Canonical; processes:361 (p. 621)
Process: Assurance Process Execution (p. 450)	Function: Establish Assurance Context (p. 445)	Canonical; processes:373 (p. 621)
Process: Assurance Process Execution (p. 450)	Function: Assess Assurance Evidence (p. 446)	Canonical; processes:373 (p. 621)
Process: Assurance Process Execution (p. 450)	Function: Adjudicate Assurance Assessment (p. 446)	Canonical; processes:373 (p. 621)

5.5. Responsibility Parent

From	To	Status / evidence
BEC: Client Administration (p. 154)	BEC: Person Identity (p. 192)	Canonical; em:16 (p. 634)

5.6. Contributes Feature

From	To	Status / evidence
Function: Manage Ward Capacity (p. 322)	Feature: Ward Patient Flow Coordination (p. 321)	Canonical; hso:96 (p. 656)

5.7. Enabled By

From	To	Status / evidence
CT1: Individual Care Delivery (p. 4)	BEC: Health Information Access (p. 391)	Canonical; business:314 (p. 534)
CT2: Individual Health Assessment (p. 5)	BEC: Health Information Access (p. 391)	Canonical; business:314 (p. 534)
CT2: Clinical Determination (p. 8)	BEC: Health Information Access (p. 391)	Canonical; business:314 (p. 534)
CT2: Individual Care Planning (p. 9)	BEC: Health Information Access (p. 391)	Canonical; business:314 (p. 534)
CT2: Clinical Intervention (p. 10)	BEC: Health Information Access (p. 391)	Canonical; business:314 (p. 534)
CT2: Individual Health Monitoring (p. 11)	BEC: Health Information Access (p. 391)	Canonical; business:314 (p. 534)
CT2: Individual Care Review (p. 13)	BEC: Health Information Access (p. 391)	Canonical; business:314 (p. 534)
CT1: Health Service Assurance (p. 145)	BEC: Assurance Design (p. 439)	Canonical; enabling:676 (p. 581)
CT1: Health Service Assurance (p. 145)	BEC: Assurance Criteria Management (p. 442)	Canonical; enabling:676 (p. 581)
CT1: Health Service Assurance (p. 145)	BEC: Governed Assurance (p. 443)	Canonical; enabling:676 (p. 581)

5.8. Governed Process

From	To	Status / evidence
Function: Apply Governed Person Identity Correction (p. 198)	Process: Governed Person Identity Correction Process (p. 199)	Canonical; em:35 (p. 634)
Function: Verify Practitioner Registration (p. 160)	Process: Practitioner Verification Process (p. 165)	Canonical; em:94 (p. 636)
Function: Manage Referral Progression (p. 220)	Process: Referral Progression Process (p. 221)	Canonical; sa:41 (p. 640)
Function: Progress Encounter State (p. 229)	Process: Encounter Lifecycle Process (p. 231)	Canonical; sa:73 (p. 641)
Function: Manage Order Progression (p. 237)	Process: Closed-Loop Order Progression Process (p. 240)	Canonical; sa:97 (p. 642)
Function: Govern Clinical Document Lifecycle (p. 259)	Process: Clinical Document Lifecycle Process (p. 260)	Canonical; sa:173 (p. 644)
Function: Manage Clinic Queue Progression (p. 319)	Process: Clinic / Operational Queue Progression Process (p. 320)	Canonical; hso:84 (p. 656)
Function: Manage Theatre Case Progression (p. 327)	Process: Theatre Case Progression Process (p. 328)	Canonical; hso:115 (p. 657)
Function: Manage Bed Turnover Progression (p. 339)	Process: Bed Turnover Process (p. 340)	Canonical; hso:163 (p. 658)
Function: Manage Operational Work Progression (p. 358)	Process: Operational Work Progression Process (p. 359)	Canonical; hso:233 (p. 661)

From	To	Status / evidence
Function: Manage Patient Transport Progression (p. 366)	Process: Patient Transport Process (p. 367)	Canonical; hso:255 (p. 661)
Function: Manage Specimen Transport Progression (p. 369)	Process: Specimen Transport Process (p. 370)	Canonical; hso:269 (p. 662)
Function: Manage Discharge Coordination Progression (p. 374)	Process: Discharge Progression Process (p. 374)	Canonical; hso:284 (p. 662)
Function: Coordinate Work Order (p. 420)	Process: Work Order Progression Process (p. 423)	Canonical; ise:183 (p. 668)
Function: Coordinate To Do (p. 421)	Process: To Do Progression Process (p. 424)	Canonical; ise:187 (p. 668)
Function: Coordinate Synthetic Task (p. 422)	Process: Synthetic Task Progression Process (p. 425)	Canonical; ise:191 (p. 668)

5.9. Uses Process

From	To	Status / evidence
Service: Request Service Assurance (p. 447)	Process: Assurance Process Execution (p. 450)	Canonical; assurance:170 (p. 629)

5.10. Exposes Process

From	To	Status / evidence
Service: Communicate Service Assurance Outcome (p. 449)	Process: Assurance Process Reporting / Communication (p. 451)	Canonical; assurance:195 (p. 629)

6. Catalogue descriptions with unresolved element identity

Of 73 named enabling decomposition descriptions, four independently identify established BEC contexts. The other 69 do not establish additional Capabilities or tiers. Their names and source wording remain in the full BEC transcription. The context keys below are navigation locators, not allocated architectural identities, ancestors or aliases.

Source description	Identity standing	Source / established context
Person Identity	Canonical Capability identity	enabling:115 (p. 565); BEC: Person Identity (p. 192)
Healthcare Subject	Canonical Capability identity	enabling:122 (p. 565); BEC: Healthcare Subject (p. 200)
Client Relationship	Canonical Capability identity	enabling:127 (p. 565); BEC: Client Relationship (p. 203)
Client Privacy	Canonical Capability identity	enabling:132 (p. 565); BEC: Client Privacy (p. 207)
Practitioner	Unresolved standalone element identity	enabling:140 (p. 565)
Practitioner Role	Unresolved standalone element identity	enabling:145 (p. 565)
Provider Relationship	Unresolved standalone element identity	enabling:150 (p. 565)
Provider Engagement	Unresolved standalone element identity	enabling:154 (p. 565)
Healthcare Organisation	Unresolved standalone element identity	enabling:161 (p. 566)
Organisational Structure	Unresolved standalone element identity	enabling:166 (p. 566)
Organisation Contact	Unresolved standalone element identity	enabling:170 (p. 566)
Healthcare Location	Unresolved standalone element identity	enabling:177 (p. 566)
Location Structure	Unresolved standalone element identity	enabling:181 (p. 566)
Care-Place Definition	Unresolved standalone element identity	enabling:185 (p. 566)
Healthcare Service Definition	Unresolved standalone element identity	enabling:192 (p. 567)
Service Provision Relationship	Unresolved standalone element identity	enabling:196 (p. 567)
Service Availability Definition	Unresolved standalone element identity	enabling:200 (p. 567)
Service Eligibility Definition	Unresolved standalone element identity	enabling:204 (p. 567)
Health Product Definition	Unresolved standalone element identity	enabling:210 (p. 567)
Product Formulary & Catalogue	Unresolved standalone element identity	enabling:211 (p. 567)
Product Batch & Serial Tracking	Unresolved standalone element identity	enabling:212 (p. 567)
Device Definition	Unresolved standalone element identity	enabling:218 (p. 567)
Device Instance	Unresolved standalone element identity	enabling:222 (p. 567)

Source description	Identity standing	Source / established context
Device Association	Unresolved standalone element identity	enabling:226 (p. 567)
Device Connectivity	Unresolved standalone element identity	enabling:230 (p. 567)
Client Intake & Demographic Update	Unresolved standalone element identity	enabling:280 (p. 569)
Referral Intake & Progression	Unresolved standalone element identity	enabling:285 (p. 569)
Appointment & Booking Administration	Unresolved standalone element identity	enabling:291 (p. 569)
Encounter Lifecycle Management	Unresolved standalone element identity	enabling:296 (p. 569)
Closed-Loop Order Progression	Unresolved standalone element identity	enabling:302 (p. 570)
Diagnostic Workflow Management	Unresolved standalone element identity	enabling:309 (p. 570)
Medication Order & Dispense Management	Unresolved standalone element identity	enabling:317 (p. 570)
Procedure & Surgical Administration	Unresolved standalone element identity	enabling:323 (p. 570)
Clinical Documentation Lifecycle	Unresolved standalone element identity	enabling:328 (p. 571)
Care Plan Coordination	Unresolved standalone element identity	enabling:334 (p. 571)
Secure Clinical Communication	Unresolved standalone element identity	enabling:338 (p. 571)
Primary Care Coordination	Unresolved standalone element identity	enabling:390 (p. 572)
Acute Care Monitoring & Event Coordination	Unresolved standalone element identity	enabling:395 (p. 572)
Emergency Care Progression	Unresolved standalone element identity	enabling:400 (p. 573)
Multidisciplinary Inpatient Care Coordination	Unresolved standalone element identity	enabling:405 (p. 573)
Diagnostic Service Integration	Unresolved standalone element identity	enabling:410 (p. 573)
Medication Therapy Provision and Monitoring	Unresolved standalone element identity	enabling:415 (p. 573)
Screening and Immunisation Enablement	Unresolved standalone element identity	enabling:420 (p. 573)
Community Care Coordination	Unresolved standalone element identity	enabling:425 (p. 573)
Outreach Care Continuity	Unresolved standalone element identity	enabling:430 (p. 574)
Remote Patient Monitoring	Unresolved standalone element identity	enabling:435 (p. 574)
Session & Queue Coordination	Unresolved standalone element identity	enabling:482 (p. 575)
Ward Logistics Coordination	Unresolved standalone element identity	enabling:487 (p. 575)
Perioperative Logistics	Unresolved standalone element identity	enabling:492 (p. 575)
ED Operational Coordination	Unresolved standalone element identity	enabling:497 (p. 575)
Outpatient Flow Enablement	Unresolved standalone element identity	enabling:502 (p. 576)
Bed State & Turnover Management	Unresolved standalone element identity	enabling:507 (p. 576)

Source description	Identity standing	Source / established context
Specialized Equipment Coordination	Unresolved standalone element identity	enabling:513 (p. 576)
Facility Demand & Capacity Telemetry	Unresolved standalone element identity	enabling:517 (p. 576)
Mobile Worker Dispatch & Safety	Unresolved standalone element identity	enabling:521 (p. 576)
On-Call Directory Resolution	Unresolved standalone element identity	enabling:527 (p. 576)
Operational Work Assignment & Supervision	Unresolved standalone element identity	enabling:531 (p. 577)
Credential Verification Checkpoint	Unresolved standalone element identity	enabling:540 (p. 577)
Patient Transfer Coordination	Unresolved standalone element identity	enabling:544 (p. 577)
Specimen & Supply Courier Tracking	Unresolved standalone element identity	enabling:549 (p. 577)
Discharge Progression & Coordination	Unresolved standalone element identity	enabling:553 (p. 577)
Longitudinal Clinical Record Governance	Unresolved standalone element identity	enabling:609 (p. 579)
Multi-Model Information Exchange	Unresolved standalone element identity	enabling:615 (p. 579)
Governed Query & Search	Unresolved standalone element identity	enabling:622 (p. 579)
Boundary Protocol Mediation	Unresolved standalone element identity	enabling:627 (p. 579)
Policy & Security Governance	Unresolved standalone element identity	enabling:631 (p. 580)
Terminology & Reference Mapping	Unresolved standalone element identity	enabling:637 (p. 580)
LHR-Aware Multidisciplinary Collaboration	Unresolved standalone element identity	enabling:642 (p. 580)
Operational Messaging	Unresolved standalone element identity	enabling:648 (p. 580)
Multi-Work-Unit Orchestration	Unresolved standalone element identity	enabling:652 (p. 580)
Operational & Clinical Calendar Sync	Unresolved standalone element identity	enabling:659 (p. 581)
Decoupled Human Interaction & Dashboards	Unresolved standalone element identity	enabling:664 (p. 581)
Canonical Semantics & Schema Governance	Unresolved standalone element identity	enabling:668 (p. 581)

7. Identifier register

Reference IDs and labels are frozen in manifest.json. The register is sorted by Reference Id and includes candidates with their actual status.

Reference Id / Structural Canonical ID	Element / page	Type / status
EC-01 EC-01	Enterprise Capability: Managed Entity & Relationship (p. 453)	enterprise capability / Canonical
EC-02 EC-02	Enterprise Capability: Context Management (p. 454)	enterprise capability / Canonical
EC-03 EC-03	Enterprise Capability: Managed State & Lifecycle (p. 454)	enterprise capability / Canonical
EC-04 EC-04	Enterprise Capability: Information Management (p. 455)	enterprise capability / Canonical
EC-05 EC-05	Enterprise Capability: Search & Discovery (p. 456)	enterprise capability / Canonical
EC-06 EC-06	Enterprise Capability: Policy & Control (p. 457)	enterprise capability / Canonical
EC-07 EC-07	Enterprise Capability: Provenance & Traceability (p. 457)	enterprise capability / Canonical
EC-08 EC-08	Enterprise Capability: Interoperability & Exchange (p. 458)	enterprise capability / Canonical
EC-09 EC-09	Enterprise Capability: Event & Subscription (p. 459)	enterprise capability / Canonical
EC-10 EC-10	Enterprise Capability: Activity & Execution (p. 460)	enterprise capability / Canonical
EC-11 EC-11	Enterprise Capability: Interaction & Experience (p. 460)	enterprise capability / Canonical
EC-12 EC-12	Enterprise Capability: Operational Assurance (p. 461)	enterprise capability / Canonical
EC-13 EC-13	Enterprise Capability: Semantic Governance & Conformance (p. 462)	enterprise capability / Canonical
EC-14 EC-14	Enterprise Capability: Service Guardian (p. 463)	enterprise capability / Canonical
FEAT-EM-01 Structural ID: Unresolved	Feature: Identifier Resolution (p. 193)	feature / Canonical
FEAT-EM-02 Structural ID: Unresolved	Feature: Cross-Authority Identifier Correlation (p. 194)	feature / Canonical
FEAT-EM-03 Structural ID: Unresolved	Feature: Identity Alias Association (p. 195)	feature / Canonical
FEAT-EM-04 Structural ID: Unresolved	Feature: Governed Identity Correction (p. 195)	feature / Canonical
FEAT-EM-05 Structural ID: Unresolved	Feature: Subject Context Binding (p. 201)	feature / Canonical
FEAT-EM-06 Structural ID: Unresolved	Feature: Demographic Context Governance (p. 201)	feature / Canonical
FEAT-EM-07 Structural ID: Unresolved	Feature: Next-of-Kin & Guardian Association (p. 205)	feature / Canonical
FEAT-EM-08 Structural ID: Unresolved	Feature: Relationship Validity Tracking (p. 205)	feature / Canonical
FEAT-EM-09 Structural ID: Unresolved	Feature: Consent Directive Evaluation (p. 208)	feature / Canonical
FEAT-EM-10 Structural ID: Unresolved	Feature: Confidentiality Flag Enforcement (p. 209)	feature / Canonical

Reference Id / Structural Canonical ID	Element / page	Type / status
FEAT-EM-11 Structural ID: Unresolved	Feature: National Practitioner Verification (p. 156)	feature / Canonical
FEAT-EM-12 Structural ID: Unresolved	Feature: Practitioner Profile Governance (p. 157)	feature / Canonical
FEAT-EM-13 Structural ID: Unresolved	Feature: Role & Specialty Modeling (p. 158)	feature / Canonical
FEAT-EM-14 Structural ID: Unresolved	Feature: Electronic Endpoint Binding (p. 158)	feature / Canonical
FEAT-EM-15 Structural ID: Unresolved	Feature: Practitioner-Organisation Affiliation (p. 159)	feature / Canonical
FEAT-EM-16 Structural ID: Unresolved	Feature: Clinical Privileging Record (p. 160)	feature / Canonical
FEAT-EM-17 Structural ID: Unresolved	Feature: National Organisation Verification (p. 167)	feature / Canonical
FEAT-EM-18 Structural ID: Unresolved	Feature: Organisation Profile Management (p. 167)	feature / Canonical
FEAT-EM-19 Structural ID: Unresolved	Feature: Hierarchical Structure Navigation (p. 168)	feature / Canonical
FEAT-EM-20 Structural ID: Unresolved	Feature: Directory Contact Management (p. 169)	feature / Canonical
FEAT-EM-21 Structural ID: Unresolved	Feature: Location Geospatial & Address Resolution (p. 174)	feature / Canonical
FEAT-EM-22 Structural ID: Unresolved	Feature: Physical Hierarchy Modeling (p. 174)	feature / Canonical
FEAT-EM-23 Structural ID: Unresolved	Feature: Atomic Care-Place Modeling (p. 175)	feature / Canonical
FEAT-EM-24 Structural ID: Unresolved	Feature: Service Catalogue Publishing (p. 179)	feature / Canonical
FEAT-EM-25 Structural ID: Unresolved	Feature: Service-Location-Provider Binding (p. 180)	feature / Canonical
FEAT-EM-26 Structural ID: Unresolved	Feature: Operating Schedule Specification (p. 180)	feature / Canonical
FEAT-EM-27 Structural ID: Unresolved	Feature: Eligibility & Catchment Specification (p. 181)	feature / Canonical
FEAT-EM-28 Structural ID: Unresolved	Feature: Device Type Specification (p. 186)	feature / Canonical
FEAT-EM-29 Structural ID: Unresolved	Feature: Device Serial Tracking (p. 187)	feature / Canonical
FEAT-EM-30 Structural ID: Unresolved	Feature: Device-Patient/Location Assignment (p. 188)	feature / Canonical
FEAT-EM-31 Structural ID: Unresolved	Feature: Device Endpoint Registration (p. 188)	feature / Canonical
FEAT-HSO-01 Structural ID: Unresolved	Feature: Clinic Session Tracking (p. 317)	feature / Canonical
FEAT-HSO-02 Structural ID: Unresolved	Feature: Operational Queue Progression (p. 317)	feature / Canonical
FEAT-HSO-03 Structural ID: Unresolved	Feature: Ward Patient Flow Coordination (p. 321)	feature / Canonical
FEAT-HSO-04 Structural ID: Unresolved	Feature: Clinical Handover Context Assembly (p. 322)	feature / Canonical
FEAT-HSO-05 Structural ID: Unresolved	Feature: Operating Theatre Case Progression (p. 325)	feature / Canonical
FEAT-HSO-06 Structural ID: Unresolved	Feature: Perioperative Resource Notification (p. 326)	feature / Canonical
FEAT-HSO-07 Structural ID: Unresolved	Feature: ED Bed & Bay Tracking (p. 329)	feature / Canonical

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FEAT-HSO-08 Structural ID: Unresolved	Feature: ED Length of Stay Tracking (p. 330)	feature / Canonical
FEAT-HSO-09 Structural ID: Unresolved	Feature: Outpatient Arrival Registration (p. 332)	feature / Canonical
FEAT-HSO-10 Structural ID: Unresolved	Feature: Post-Clinic Order Coordination (p. 333)	feature / Canonical
FEAT-HSO-11 Structural ID: Unresolved	Feature: Bed Availability Tracking (p. 336)	feature / Canonical
FEAT-HSO-12 Structural ID: Unresolved	Feature: Bed State Progression (p. 337)	feature / Canonical
FEAT-HSO-13 Structural ID: Unresolved	Feature: Bed Readiness Notification (p. 338)	feature / Canonical
FEAT-HSO-14 Structural ID: Unresolved	Feature: Mobile Clinical Equipment Tracking (p. 342)	feature / Canonical
FEAT-HSO-15 Structural ID: Unresolved	Feature: Operational Capacity Metric Aggregation (p. 344)	feature / Canonical
FEAT-HSO-16 Structural ID: Unresolved	Feature: Mobile Worker Task Dispatch (p. 348)	feature / Canonical
FEAT-HSO-17 Structural ID: Unresolved	Feature: Mobile Visit Status Tracking (p. 348)	feature / Canonical
FEAT-HSO-18 Structural ID: Unresolved	Feature: Active On-Call Provider Resolution (p. 352)	feature / Canonical
FEAT-HSO-19 Structural ID: Unresolved	Feature: Work Item Instantiation (p. 355)	feature / Canonical
FEAT-HSO-20 Structural ID: Unresolved	Feature: Worker Matching & Allocation (p. 355)	feature / Canonical
FEAT-HSO-21 Structural ID: Unresolved	Feature: Work Dispatch Delivery (p. 356)	feature / Canonical
FEAT-HSO-22 Structural ID: Unresolved	Feature: Work Progress Oversight & Escalation (p. 357)	feature / Canonical
FEAT-HSO-23 Structural ID: Unresolved	Feature: Operational Credential Check (p. 362)	feature / Canonical
FEAT-HSO-24 Structural ID: Unresolved	Feature: Transport Request Ingestion (p. 365)	feature / Canonical
FEAT-HSO-25 Structural ID: Unresolved	Feature: Transport Dispatch & Progress Tracking (p. 365)	feature / Canonical
FEAT-HSO-26 Structural ID: Unresolved	Feature: Pathology Specimen Transport Tracking (p. 368)	feature / Canonical
FEAT-HSO-27 Structural ID: Unresolved	Feature: Discharge Milestone Tracking (p. 372)	feature / Canonical
FEAT-HSO-28 Structural ID: Unresolved	Feature: Discharge Readiness & Coordination Oversight (p. 372)	feature / Canonical
FEAT-HSO-29 Structural ID: Unresolved	Feature: Discharge Notification Dispatch (p. 373)	feature / Canonical
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FEAT-ISE-03 Structural ID: Unresolved	Feature: Durable Clinical Record Preservation (p. 380)	feature / Canonical
FEAT-ISE-04 Structural ID: Unresolved	Feature: Direct Submission Ingestion & Delivery (p. 385)	feature / Canonical
FEAT-ISE-05 Structural ID: Unresolved	Feature: Clinical Record Retrieval Mediation (p. 385)	feature / Canonical
FEAT-ISE-06 Structural ID: Unresolved	Feature: Addressed Clinical Distribution (p. 386)	feature / Canonical

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FEAT-ISE-07 Structural ID: Unresolved	Feature: Event-Driven Syndication (p. 387)	feature / Canonical
FEAT-ISE-08 Structural ID: Unresolved	Feature: Federated Clinical Query (p. 392)	feature / Canonical
FEAT-ISE-09 Structural ID: Unresolved	Feature: Access-Controlled Information Filtering (p. 392)	feature / Canonical
FEAT-ISE-10 Structural ID: Unresolved	Feature: Standards-Based Boundary Exchange (p. 396)	feature / Canonical
FEAT-ISE-11 Structural ID: Unresolved	Feature: Health Information Access & Consent Control (p. 399)	feature / Canonical
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FEAT-ISE-13 Structural ID: Unresolved	Feature: Non-PHI Compliance Auditing (p. 400)	feature / Canonical
FEAT-ISE-14 Structural ID: Unresolved	Feature: Clinical Concept Resolution (p. 405)	feature / Canonical
FEAT-ISE-15 Structural ID: Unresolved	Feature: Bidirectional Terminology Mapping (p. 406)	feature / Canonical
FEAT-ISE-16 Structural ID: Unresolved	Feature: Care-Team Clinical Collaboration Coordination (p. 409)	feature / Canonical
FEAT-ISE-17 Structural ID: Unresolved	Feature: In-Conversation LHR Query Resolution (p. 410)	feature / Canonical
FEAT-ISE-18 Structural ID: Unresolved	Feature: Zero-PHI Collaboration Metadata Governance (p. 411)	feature / Canonical
FEAT-ISE-19 Structural ID: Unresolved	Feature: Operational Team Chat Enablement (p. 415)	feature / Canonical
FEAT-ISE-20 Structural ID: Unresolved	Feature: Work Order Progression (p. 418)	feature / Canonical
FEAT-ISE-21 Structural ID: Unresolved	Feature: To Do Decision & Approval Coordination (p. 418)	feature / Canonical
FEAT-ISE-22 Structural ID: Unresolved	Feature: Synthetic Task Orchestration (p. 419)	feature / Canonical
FEAT-ISE-23 Structural ID: Unresolved	Feature: Workflow Timeout & Escalation Oversight (p. 419)	feature / Canonical
FEAT-ISE-24 Structural ID: Unresolved	Feature: Schedule Projection Integration (p. 427)	feature / Canonical
FEAT-ISE-25 Structural ID: Unresolved	Feature: Temporal Event Correlation (p. 428)	feature / Canonical
FEAT-ISE-26 Structural ID: Unresolved	Feature: Longitudinal Clinical & Operational Presentation (p. 431)	feature / Canonical
FEAT-ISE-27 Structural ID: Unresolved	Feature: Information Standards & Semantic Governance (p. 433)	feature / Canonical
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FEAT-SA-03 Structural ID: Unresolved	Feature: Referral Document Ingestion (p. 216)	feature / Canonical
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FEAT-SA-05 Structural ID: Unresolved	Feature: Referral Status & Outcome Tracking (p. 218)	feature / Canonical
FEAT-SA-06 Structural ID: Unresolved	Feature: Appointment Notification Ingestion (p. 223)	feature / Canonical

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FEAT-SA-08 Structural ID: Unresolved	Feature: Encounter Context Creation (p. 226)	feature / Canonical
FEAT-SA-09 Structural ID: Unresolved	Feature: Encounter State Progression (p. 227)	feature / Canonical
FEAT-SA-10 Structural ID: Unresolved	Feature: Encounter Bed/Location Association (p. 228)	feature / Canonical
FEAT-SA-11 Structural ID: Unresolved	Feature: Order Request Ingestion (p. 233)	feature / Canonical
FEAT-SA-12 Structural ID: Unresolved	Feature: Order Destination Resolution & Routing (p. 234)	feature / Canonical
FEAT-SA-13 Structural ID: Unresolved	Feature: Order Closed-Loop Progression Tracking (p. 234)	feature / Canonical
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FEAT-SA-15 Structural ID: Unresolved	Feature: Diagnostic Request Correlation (p. 242)	feature / Canonical
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FEAT-SA-21 Structural ID: Unresolved	Feature: Procedure Booking Ingestion (p. 252)	feature / Canonical
FEAT-SA-22 Structural ID: Unresolved	Feature: Procedural Documentation Ingestion (p. 253)	feature / Canonical
FEAT-SA-23 Structural ID: Unresolved	Feature: Clinical Document Ingestion (p. 256)	feature / Canonical
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FEAT-SD-02 Structural ID: Unresolved	Feature: Primary Care Notification Dispatch (p. 275)	feature / Canonical
FEAT-SD-03 Structural ID: Unresolved	Feature: Acute Clinical State Event Capture (p. 278)	feature / Canonical
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FEAT-SD-05 Structural ID: Unresolved	Feature: Emergency Department Arrival & Triage Ingestion (p. 282)	feature / Canonical

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FEAT-SD-07 Structural ID: Unresolved	Feature: Inpatient Rounding Context Provision (p. 286)	feature / Canonical
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FEAT-SD-10 Structural ID: Unresolved	Feature: Diagnostic History Consolidation (p. 292)	feature / Canonical
FEAT-SD-11 Structural ID: Unresolved	Feature: Medication History Reconciliation Ingestion (p. 296)	feature / Canonical
FEAT-SD-12 Structural ID: Unresolved	Feature: Medication Administration Tracking (p. 296)	feature / Canonical
FEAT-SD-13 Structural ID: Unresolved	Feature: Immunisation Event Submission (p. 300)	feature / Canonical
FEAT-SD-14 Structural ID: Unresolved	Feature: Screening Recall Notification Distribution (p. 301)	feature / Canonical
FEAT-SD-15 Structural ID: Unresolved	Feature: Community Encounter Ingestion (p. 305)	feature / Canonical
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FEAT-SD-17 Structural ID: Unresolved	Feature: Outreach Visit Context Provision (p. 309)	feature / Canonical
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FEAT-SD-19 Structural ID: Unresolved	Feature: Remote Telemetry Ingestion (p. 312)	feature / Canonical
FEAT-SD-20 Structural ID: Unresolved	Feature: Virtual Care Session Context Binding (p. 313)	feature / Canonical
HCM-BC-access-control-assurance-reporting CT1-SECURITY-PRIVACY-DIGITAL-TRUST.CT2-DIGITAL-IDENTITY-ACCESS-GOVERNANCE.CT3-ACCESS-CONTROL-ASSURANCE-REPORTING	CT3: Access Control Assurance & Reporting (p. 103)	capability / Canonical
HCM-BC-access-control-collaboration CT1-SECURITY-PRIVACY-DIGITAL-TRUST.CT2-DIGITAL-IDENTITY-ACCESS-GOVERNANCE.CT3-ACCESS-CONTROL-COLLABORATION	CT3: Access Control Collaboration (p. 104)	capability / Canonical
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HCM-BC-access-control-enforcement CT1-SECURITY-PRIVACY-DIGITAL-TRUST.CT2-DIGITAL-IDENTITY-ACCESS-GOVERNANCE.CT3-ACCESS-CONTROL-ENFORCEMENT	CT3: Access Control Enforcement (p. 105)	capability / Canonical
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HCM-BC-assurance-conclusion-determination CT1-HEALTH-SERVICE-ASSURANCE.CT2-ASSURANCE-EVALUATION.CT3-ASSURANCE-CONCLUSION-DETERMINATION	CT3: Assurance Conclusion Determination (p. 150)	capability / Canonical

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HCM-BC-assurance-evaluation CT1-HEALTH-SERVICE-ASSURANCE.CT2-ASSURANCE-EVALUATION	CT2: Assurance Evaluation (p. 149)	capability / Canonical
HCM-BC-assurance-evaluation-criteria-governance CT1-HEALTH-SERVICE-ASSURANCE.CT2-HEALTH-SERVICE-ASSURANCE-SPECIFICATION.CT3-ASSURANCE-EVALUATION-CRITERIA-GOVERNANCE	CT3: Assurance Evaluation Criteria Governance (p. 148)	capability / Canonical
HCM-BC-assurance-evidence-assessment CT1-HEALTH-SERVICE-ASSURANCE.CT2-ASSURANCE-EVALUATION.CT3-ASSURANCE-EVIDENCE-ASSESSMENT	CT3: Assurance Evidence Assessment (p. 150)	capability / Canonical
HCM-BC-authority-constraint-governance CT1-HEALTH-SERVICE-GOVERNANCE.CT2-GOVERNANCE-FRAMEWORK-MANAGEMENT.CT3-AUTHORITY-CONSTRAINT-GOVERNANCE	CT3: Authority & Constraint Governance (p. 142)	capability / Canonical
HCM-BC-back-office-services CT1-CORPORATE-RESOURCES-ENTERPRISE-SERVICES.CT2-CORPORATE-SUPPORT-SERVICES.CT3-BACK-OFFICE-SERVICES	CT3: Back-Office Services (p. 136)	capability / Canonical
HCM-BC-care-access-coordination CT1-CARE-ACCESS-COORDINATION	CT1: Care Access & Coordination (p. 15)	capability / Canonical
HCM-BC-care-access-management CT1-CARE-ACCESS-COORDINATION.CT2-CARE-ACCESS-MANAGEMENT	CT2: Care Access Management (p. 16)	capability / Canonical
HCM-BC-care-appointment-management CT1-CARE-ACCESS-COORDINATION.CT2-CARE-SCHEDULING-CAPACITY-MANAGEMENT.CT3-CARE-APPOINTMENT-MANAGEMENT	CT3: Care Appointment Management (p. 20)	capability / Canonical
HCM-BC-care-appropriateness-review CT1-INDIVIDUAL-CARE-DELIVERY.CT2-INDIVIDUAL-CARE-REVIEW.CT3-CARE-APPROPRIATENESS-REVIEW	CT3: Care Appropriateness Review (p. 14)	capability / Canonical
HCM-BC-care-coordination-transition CT1-CARE-ACCESS-COORDINATION.CT2-CARE-COORDINATION-TRANSITION	CT2: Care Coordination & Transition (p. 24)	capability / Canonical
HCM-BC-care-discharge-coordination CT1-CARE-ACCESS-COORDINATION.CT2-CARE-COORDINATION-TRANSITION.CT3-CARE-DISCHARGE-COORDINATION	CT3: Care Discharge Coordination (p. 27)	capability / Canonical
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HCM-BC-care-intake-management CT1-CARE-ACCESS-COORDINATION.CT2-CARE-ACCESS-MANAGEMENT.CT3-CARE-INTAKE-MANAGEMENT	CT3: Care Intake Management (p. 17)	capability / Canonical
HCM-BC-care-place-allocation CT1-CARE-ACCESS-COORDINATION.CT2-CARE-SCHEDULING-CAPACITY-MANAGEMENT.CT3-CARE-PLACE-ALLOCATION	CT3: Care-Place Allocation (p. 21)	capability / Canonical

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HCM-BC-care-place-readiness-management CT1-CARE-ACCESS-COORDINATION.CT2-CARE-SCHEDULING-CAPACITY-MANAGEMENT.CT3-CARE-PLACE-READINESS-MANAGEMENT	CT3: Care-Place Readiness Management (p. 22)	capability / Canonical
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HCM-BC-care-resource-deployment CT1-CARE-ACCESS-COORDINATION.CT2-CARE-SCHEDULING-CAPACITY-MANAGEMENT.CT3-CARE-RESOURCE-DEPLOYMENT	CT3: Care Resource Deployment (p. 22)	capability / Canonical
HCM-BC-care-response-monitoring CT1-INDIVIDUAL-CARE-DELIVERY.CT2-INDIVIDUAL-HEALTH-MONITORING.CT3-CARE-RESPONSE-MONITORING	CT3: Care Response Monitoring (p. 12)	capability / Canonical
HCM-BC-care-responsibility-transition CT1-CARE-ACCESS-COORDINATION.CT2-CARE-COORDINATION-TRANSITION.CT3-CARE-RESPONSIBILITY-TRANSITION	CT3: Care Responsibility Transition (p. 26)	capability / Canonical
HCM-BC-care-scheduling-capacity-management CT1-CARE-ACCESS-COORDINATION.CT2-CARE-SCHEDULING-CAPACITY-MANAGEMENT	CT2: Care Scheduling & Capacity Management (p. 19)	capability / Canonical
HCM-BC-care-support-logistics-management CT1-CARE-ACCESS-COORDINATION.CT2-CARE-SUPPORT-LOGISTICS-MANAGEMENT	CT2: Care Support Logistics Management (p. 27)	capability / Canonical
HCM-BC-care-support-work-coordination CT1-CARE-ACCESS-COORDINATION.CT2-CARE-SUPPORT-LOGISTICS-MANAGEMENT.CT3-CARE-SUPPORT-WORK-COORDINATION	CT3: Care Support Work Coordination (p. 30)	capability / Canonical
HCM-BC-care-team-coordination CT1-CARE-ACCESS-COORDINATION.CT2-CARE-COORDINATION-TRANSITION.CT3-CARE-TEAM-COORDINATION	CT3: Care-Team Coordination (p. 25)	capability / Canonical
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HCM-BC-clinical-determination CT1-INDIVIDUAL-CARE-DELIVERY.CT2-CLINICAL-DETERMINATION	CT2: Clinical Determination (p. 8)	capability / Canonical

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HCM-BC-clinical-device-data-integration CT1-HEALTH-PRODUCTS-CLINICAL-TECHNOLOGY.CT2-CLINICAL-TECHNOLOGY-MANAGEMENT.CT3-CLINICAL-DEVICE-DATA-INTEGRATION	CT3: Clinical Device Data Integration (p. 47)	capability / Canonical
HCM-BC-clinical-device-feature-release-management CT1-HEALTH-PRODUCTS-CLINICAL-TECHNOLOGY.CT2-CLINICAL-TECHNOLOGY-MANAGEMENT.CT3-CLINICAL-DEVICE-FEATURE-RELEASE-MANAGEMENT	CT3: Clinical Device Feature & Release Management (p. 45)	capability / Canonical
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HCM-BC-clinical-device-loan-deployment-management CT1-HEALTH-PRODUCTS-CLINICAL-TECHNOLOGY.CT2-CLINICAL-TECHNOLOGY-MANAGEMENT.CT3-CLINICAL-DEVICE-LOAN-DEPLOYMENT-MANAGEMENT	CT3: Clinical Device Loan & Deployment Management (p. 45)	capability / Canonical
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HCM-BC-clinical-device-utilisation-support CT1-HEALTH-PRODUCTS-CLINICAL-TECHNOLOGY.CT2-CLINICAL-TECHNOLOGY-MANAGEMENT.CT3-CLINICAL-DEVICE-UTILISATION-SUPPORT	CT3: Clinical Device Utilisation Support (p. 46)	capability / Canonical
HCM-BC-clinical-examination CT1-INDIVIDUAL-CARE-DELIVERY.CT2-INDIVIDUAL-HEALTH-ASSESSMENT.CT3-CLINICAL-EXAMINATION	CT3: Clinical Examination (p. 7)	capability / Canonical
HCM-BC-clinical-intervention CT1-INDIVIDUAL-CARE-DELIVERY.CT2-CLINICAL-INTERVENTION	CT2: Clinical Intervention (p. 10)	capability / Canonical
HCM-BC-clinical-knowledge-concept-association CT1-HEALTH-INFORMATION-KNOWLEDGE-MANAGEMENT.CT2-CLINICAL-KNOWLEDGE-MANAGEMENT.CT3-CLINICAL-KNOWLEDGE-CONCEPT-ASSOCIATION	CT3: Clinical Knowledge Concept Association (p. 81)	capability / Canonical

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HCM-BC-clinical-knowledge-context-identification CT1-HEALTH-INFORMATION-KNOWLEDGE-MANAGEMENT.CT2-CLINICAL-KNOWLEDGE-MANAGEMENT.CT3-CLINICAL-KNOWLEDGE-CONTEXT-IDENTIFICATION	CT3: Clinical Knowledge Context Identification (p. 79)	capability / Canonical
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HCM-BC-clinical-material-logistics-coordination CT1-CARE-ACCESS-COORDINATION.CT2-CARE-SUPPORT-LOGISTICS-MANAGEMENT.CT3-CLINICAL-MATERIAL-LOGISTICS-COORDINATION	CT3: Clinical Material Logistics Coordination (p. 29)	capability / Canonical
HCM-BC-clinical-quality-criteria-management CT1-CLINICAL-QUALITY-SAFETY-IMPROVEMENT.CT2-CLINICAL-QUALITY-MANAGEMENT.CT3-CLINICAL-QUALITY-CRITERIA-MANAGEMENT	CT3: Clinical Quality Criteria Management (p. 54)	capability / Canonical
HCM-BC-clinical-quality-evidence-management CT1-CLINICAL-QUALITY-SAFETY-IMPROVEMENT.CT2-CLINICAL-QUALITY-MANAGEMENT.CT3-CLINICAL-QUALITY-EVIDENCE-MANAGEMENT	CT3: Clinical Quality Evidence Management (p. 54)	capability / Canonical
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HCM-BC-clinical-quality-reporting CT1-CLINICAL-QUALITY-SAFETY-IMPROVEMENT.CT2-CLINICAL-QUALITY-MANAGEMENT.CT3-CLINICAL-QUALITY-REPORTING	CT3: Clinical Quality Reporting (p. 56)	capability / Canonical

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HCM-BC-clinical-quality-review-assessment-management CT1-CLINICAL-QUALITY-SAFETY-IMPROVEMENT.CT2-CLINICAL-QUALITY-MANAGEMENT.CT3-CLINICAL-QUALITY-REVIEW-ASSESSMENT-MANAGEMENT	CT3: Clinical Quality Review & Assessment Management (p. 55)	capability / Canonical
HCM-BC-clinical-quality-safety-improvement CT1-CLINICAL-QUALITY-SAFETY-IMPROVEMENT	CT1: Clinical Quality, Safety & Improvement (p. 49)	capability / Canonical
HCM-BC-clinical-risk-identification CT1-CLINICAL-QUALITY-SAFETY-IMPROVEMENT.CT2-CLINICAL-SAFETY-MANAGEMENT.CT3-CLINICAL-RISK-IDENTIFICATION	CT3: Clinical Risk Identification (p. 51)	capability / Canonical
HCM-BC-clinical-risk-mitigation-management CT1-CLINICAL-QUALITY-SAFETY-IMPROVEMENT.CT2-CLINICAL-SAFETY-MANAGEMENT.CT3-CLINICAL-RISK-MITIGATION-MANAGEMENT	CT3: Clinical Risk Mitigation Management (p. 51)	capability / Canonical
HCM-BC-clinical-risk-response-management CT1-CLINICAL-QUALITY-SAFETY-IMPROVEMENT.CT2-CLINICAL-SAFETY-MANAGEMENT.CT3-CLINICAL-RISK-RESPONSE-MANAGEMENT	CT3: Clinical Risk Response Management (p. 52)	capability / Canonical
HCM-BC-clinical-safety-management CT1-CLINICAL-QUALITY-SAFETY-IMPROVEMENT.CT2-CLINICAL-SAFETY-MANAGEMENT	CT2: Clinical Safety Management (p. 50)	capability / Canonical
HCM-BC-clinical-semantics-governance CT1-STANDARDS-SEMANTICS-REFERENCE-GOVERNANCE.CT2-CLINICAL-SEMANTICS-GOVERNANCE	CT2: Clinical Semantics Governance (p. 90)	capability / Canonical
HCM-BC-clinical-technology-management CT1-HEALTH-PRODUCTS-CLINICAL-TECHNOLOGY.CT2-CLINICAL-TECHNOLOGY-MANAGEMENT	CT2: Clinical Technology Management (p. 43)	capability / Canonical
HCM-BC-communications-publicity CT1-CORPORATE-RESOURCES-ENTERPRISE-SERVICES.CT2-CORPORATE-SUPPORT-SERVICES.CT3-COMMUNICATIONS-PUBLICITY	CT3: Communications & Publicity (p. 138)	capability / Canonical
HCM-BC-community-health-wellbeing CT1-COMMUNITY-HEALTH-WELLBEING	CT1: Community Health & Wellbeing (p. 56)	capability / Canonical
HCM-BC-compliance-assessment-definition CT1-HEALTH-SERVICE-GOVERNANCE.CT2-COMPLIANCE-GOVERNANCE.CT3-COMPLIANCE-ASSESSMENT-DEFINITION	CT3: Compliance Assessment Definition (p. 144)	capability / Canonical
HCM-BC-compliance-governance CT1-HEALTH-SERVICE-GOVERNANCE.CT2-COMPLIANCE-GOVERNANCE	CT2: Compliance Governance (p. 143)	capability / Canonical
HCM-BC-connected-health-services CT1-CONNECTED-HEALTH-SERVICES	CT1: Connected Health Services (p. 95)	capability / Canonical
HCM-BC-corporate-resources-enterprise-services CT1-CORPORATE-RESOURCES-ENTERPRISE-SERVICES	CT1: Corporate Resources & Enterprise Services (p. 132)	capability / Canonical
HCM-BC-corporate-support-services CT1-CORPORATE-RESOURCES-ENTERPRISE-SERVICES.CT2-CORPORATE-SUPPORT-SERVICES	CT2: Corporate Support Services (p. 135)	capability / Canonical

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HCM-BC-diagnostic-data-management CT1-DIAGNOSTIC-THERAPEUTIC-CLINICAL-SUPPORT-SERVICES.CT2-DIAGNOSTIC-SUPPORT-PROVISION.CT3-DIAGNOSTIC-DATA-MANAGEMENT	CT3: Diagnostic Data Management (p. 35)	capability / Canonical
HCM-BC-diagnostic-interpretation CT1-DIAGNOSTIC-THERAPEUTIC-CLINICAL-SUPPORT-SERVICES.CT2-DIAGNOSTIC-SUPPORT-PROVISION.CT3-DIAGNOSTIC-INTERPRETATION	CT3: Diagnostic Interpretation (p. 36)	capability / Canonical
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HCM-BC-diagnostic-report-management CT1-DIAGNOSTIC-THERAPEUTIC-CLINICAL-SUPPORT-SERVICES.CT2-DIAGNOSTIC-SUPPORT-PROVISION.CT3-DIAGNOSTIC-REPORT-MANAGEMENT	CT3: Diagnostic Report Management (p. 37)	capability / Canonical
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HCM-BC-digital-communication-channel CT1-CONNECTED-HEALTH-SERVICES.CT2-DIGITAL-COMMUNICATION-CHANNEL	CT2: Digital Communication Channel (p. 97)	capability / Canonical
HCM-BC-digital-identity-access-governance CT1-SECURITY-PRIVACY-DIGITAL-TRUST.CT2-DIGITAL-IDENTITY-ACCESS-GOVERNANCE	CT2: Digital Identity & Access Governance (p. 99)	capability / Canonical
HCM-BC-facilities-management CT1-CORPORATE-RESOURCES-ENTERPRISE-SERVICES.CT2-CORPORATE-SUPPORT-SERVICES.CT3-FACILITIES-MANAGEMENT	CT3: Facilities Management (p. 136)	capability / Canonical

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HCM-BC-financial-commercial-services CT1-CORPORATE-RESOURCES-ENTERPRISE-SERVICES.CT2-FINANCIAL-COMMERCIAL-SERVICES	CT2: Financial & Commercial Services (p. 133)	capability / Canonical
HCM-BC-governance-framework-management CT1-HEALTH-SERVICE-GOVERNANCE.CT2-GOVERNANCE-FRAMEWORK-MANAGEMENT	CT2: Governance Framework Management (p. 141)	capability / Canonical
HCM-BC-governance-reporting-checkpoint-definition CT1-HEALTH-SERVICE-GOVERNANCE.CT2-COMPLIANCE-GOVERNANCE.CT3-GOVERNANCE-REPORTING-CHECKPOINT-DEFINITION	CT3: Governance Reporting & Checkpoint Definition (p. 145)	capability / Canonical
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HCM-BC-health-information-access-monitoring CT1-SECURITY-PRIVACY-DIGITAL-TRUST.CT2-HEALTH-INFORMATION-PROTECTION.CT3-HEALTH-INFORMATION-ACCESS-MONITORING	CT3: Health Information Access Monitoring (p. 108)	capability / Canonical
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HCM-BC-health-information-data-management CT1-HEALTH-INFORMATION-KNOWLEDGE-MANAGEMENT.CT2-HEALTH-INFORMATION-MANAGEMENT.CT3-HEALTH-INFORMATION-DATA-MANAGEMENT	CT3: Health Information Data Management (p. 73)	capability / Canonical
HCM-BC-health-information-data-retrieval CT1-HEALTH-INFORMATION-KNOWLEDGE-MANAGEMENT.CT2-HEALTH-INFORMATION-MANAGEMENT.CT3-HEALTH-INFORMATION-DATA-RETRIEVAL	CT3: Health Information Data Retrieval (p. 77)	capability / Canonical
HCM-BC-health-information-exchange CT1-CONNECTED-HEALTH-SERVICES.CT2-HEALTH-INFORMATION-EXCHANGE	CT2: Health Information Exchange (p. 96)	capability / Canonical
HCM-BC-health-information-knowledge-management CT1-HEALTH-INFORMATION-KNOWLEDGE-MANAGEMENT	CT1: Health Information & Knowledge Management (p. 71)	capability / Canonical
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HCM-BC-healthcare-location-definition-management CT1-WORKFORCE-ORGANISATIONAL-CAPABILITY.CT2-HEALTHCARE-ORGANISATION-SERVICE-STRUCTURE-MANAGEMENT.CT3-HEALTHCARE-LOCATION-DEFINITION-MANAGEMENT	CT3: Healthcare Location Definition Management (p. 131)	capability / Canonical
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HCM-BC-healthcare-organisation-service-structure-management CT1-WORKFORCE-ORGANISATIONAL-CAPABILITY.CT2-HEALTHCARE-ORGANISATION-SERVICE-STRUCTURE-MANAGEMENT	CT2: Healthcare Organisation & Service Structure Management (p. 129)	capability / Canonical
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HCM-BC-individual-health-assessment CT1-INDIVIDUAL-CARE-DELIVERY.CT2-INDIVIDUAL-HEALTH-ASSESSMENT	CT2: Individual Health Assessment (p. 5)	capability / Canonical
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HCM-BC-patient-movement-coordination CT1-CARE-ACCESS-COORDINATION.CT2-CARE-SUPPORT-LOGISTICS-MANAGEMENT.CT3-PATIENT-MOVEMENT-COORDINATION	CT3: Patient Movement Coordination (p. 28)	capability / Canonical
HCM-BC-people-management CT1-CORPORATE-RESOURCES-ENTERPRISE-SERVICES.CT2-CORPORATE-SUPPORT-SERVICES.CT3-PEOPLE-MANAGEMENT	CT3: People Management (p. 138)	capability / Canonical
HCM-BC-policy-obligation-governance CT1-HEALTH-SERVICE-GOVERNANCE.CT2-GOVERNANCE-FRAMEWORK-MANAGEMENT.CT3-POLICY-OBLIGATION-GOVERNANCE	CT3: Policy / Obligation Governance (p. 142)	capability / Canonical
HCM-BC-population-health-event-identification CT1-POPULATION-HEALTH-HEALTH-SYSTEM-PLANNING.CT2-POPULATION-HEALTH-SURVEILLANCE.CT3-POPULATION-HEALTH-EVENT-IDENTIFICATION	CT3: Population Health Event Identification (p. 66)	capability / Canonical
HCM-BC-population-health-health-system-planning CT1-POPULATION-HEALTH-HEALTH-SYSTEM-PLANNING	CT1: Population Health & Health-System Planning (p. 62)	capability / Canonical
HCM-BC-population-health-longitudinal-assessment CT1-POPULATION-HEALTH-HEALTH-SYSTEM-PLANNING.CT2-POPULATION-HEALTH-SURVEILLANCE.CT3-POPULATION-HEALTH-LONGITUDINAL-ASSESSMENT	CT3: Population Health Longitudinal Assessment (p. 65)	capability / Canonical
HCM-BC-population-health-reporting CT1-POPULATION-HEALTH-HEALTH-SYSTEM-PLANNING.CT2-POPULATION-HEALTH-SURVEILLANCE.CT3-POPULATION-HEALTH-REPORTING	CT3: Population Health Reporting (p. 67)	capability / Canonical
HCM-BC-population-health-surveillance CT1-POPULATION-HEALTH-HEALTH-SYSTEM-PLANNING.CT2-POPULATION-HEALTH-SURVEILLANCE	CT2: Population Health Surveillance (p. 63)	capability / Canonical
HCM-BC-population-health-surveillance-management CT1-POPULATION-HEALTH-HEALTH-SYSTEM-PLANNING.CT2-POPULATION-HEALTH-SURVEILLANCE.CT3-POPULATION-HEALTH-SURVEILLANCE-MANAGEMENT	CT3: Population Health Surveillance Management (p. 65)	capability / Canonical

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HCM-BC-practitioner-engagement-privilege-management CT1-WORKFORCE-ORGANISATIONAL-CAPABILITY.CT2-HEALTH-WORKFORCE-MANAGEMENT.CT3-PRACTITIONER-ENGAGEMENT-PRIVILEGE-MANAGEMENT	CT3: Practitioner Engagement & Privilege Management (p. 127)	capability / Canonical
HCM-BC-practitioner-qualification-management CT1-WORKFORCE-ORGANISATIONAL-CAPABILITY.CT2-HEALTH-WORKFORCE-MANAGEMENT.CT3-PRACTITIONER-QUALIFICATION-MANAGEMENT	CT3: Practitioner Qualification Management (p. 127)	capability / Canonical
HCM-BC-prescription-medication-dispensing CT1-DIAGNOSTIC-THERAPEUTIC-CLINICAL-SUPPORT-SERVICES.CT2-MEDICATION-PHARMACY-SERVICES.CT3-PRESCRIPTION-MEDICATION-DISPENSING	CT3: Prescription Medication Dispensing (p. 39)	capability / Canonical
HCM-BC-preventative-health CT1-COMMUNITY-HEALTH-WELLBEING.CT2-PREVENTATIVE-HEALTH	CT2: Preventative Health (p. 57)	capability / Canonical
HCM-BC-preventative-health-cohort-management CT1-COMMUNITY-HEALTH-WELLBEING.CT2-PREVENTATIVE-HEALTH.CT3-PREVENTATIVE-HEALTH-COHORT-MANAGEMENT	CT3: Preventative Health Cohort Management (p. 62)	capability / Canonical
HCM-BC-preventative-health-education-services CT1-COMMUNITY-HEALTH-WELLBEING.CT2-PREVENTATIVE-HEALTH.CT3-PREVENTATIVE-HEALTH-EDUCATION-SERVICES	CT3: Preventative Health Education Services (p. 60)	capability / Canonical
HCM-BC-preventative-health-problem-identification CT1-COMMUNITY-HEALTH-WELLBEING.CT2-PREVENTATIVE-HEALTH.CT3-PREVENTATIVE-HEALTH-PROBLEM-IDENTIFICATION	CT3: Preventative Health Problem Identification (p. 58)	capability / Canonical
HCM-BC-preventative-health-programme-management CT1-COMMUNITY-HEALTH-WELLBEING.CT2-PREVENTATIVE-HEALTH.CT3-PREVENTATIVE-HEALTH-PROGRAMME-MANAGEMENT	CT3: Preventative Health Programme Management (p. 59)	capability / Canonical
HCM-BC-preventative-health-programme-monitoring CT1-COMMUNITY-HEALTH-WELLBEING.CT2-PREVENTATIVE-HEALTH.CT3-PREVENTATIVE-HEALTH-PROGRAMME-MONITORING	CT3: Preventative Health Programme Monitoring (p. 60)	capability / Canonical
HCM-BC-preventative-health-reporting CT1-COMMUNITY-HEALTH-WELLBEING.CT2-PREVENTATIVE-HEALTH.CT3-PREVENTATIVE-HEALTH-REPORTING	CT3: Preventative Health Reporting (p. 61)	capability / Canonical
HCM-BC-research-information-support CT1-HEALTH-RESEARCH-INNOVATION.CT2-RESEARCH-INFORMATION-SUPPORT	CT2: Research Information Support (p. 113)	capability / Canonical
HCM-BC-security-information-use-accountability CT1-SECURITY-PRIVACY-DIGITAL-TRUST.CT2-DIGITAL-ACCOUNTABILITY-TRUST.CT3-SECURITY-INFORMATION-USE-ACCOUNTABILITY	CT3: Security & Information-Use Accountability (p. 111)	capability / Canonical
HCM-BC-security-privacy-digital-trust CT1-SECURITY-PRIVACY-DIGITAL-TRUST	CT1: Security, Privacy & Digital Trust (p. 98)	capability / Canonical

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HCM-BC-standards-adoption CT1-STANDARDS-SEMANTICS-REFERENCE-GOVERNANCE.CT2-HEALTH-INFORMATION-STANDARDS-GOVERNANCE.CT3-STANDARDS-ADOPTION	CT3: Standards Adoption (p. 89)	capability / Canonical
HCM-BC-standards-impact-identification CT1-STANDARDS-SEMANTICS-REFERENCE-GOVERNANCE.CT2-HEALTH-INFORMATION-STANDARDS-GOVERNANCE.CT3-STANDARDS-IMPACT-IDENTIFICATION	CT3: Standards Impact Identification (p. 89)	capability / Canonical
HCM-BC-standards-semantics-reference-governance CT1-STANDARDS-SEMANTICS-REFERENCE-GOVERNANCE	CT1: Standards, Semantics & Reference Governance (p. 83)	capability / Canonical
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HCM-BC-terminology-localisation CT1-STANDARDS-SEMANTICS-REFERENCE-GOVERNANCE.CT2-CLINICAL-SEMANTICS-GOVERNANCE.CT3-TERMINOLOGY-LOCALISATION	CT3: Terminology Localisation (p. 92)	capability / Canonical
HCM-BC-terminology-utilisation-oversight CT1-STANDARDS-SEMANTICS-REFERENCE-GOVERNANCE.CT2-CLINICAL-SEMANTICS-GOVERNANCE.CT3-TERMINOLOGY-UTILISATION-OVERSIGHT	CT3: Terminology Utilisation Oversight (p. 92)	capability / Canonical
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HCM-BC-workforce-service-coverage-management CT1-WORKFORCE-ORGANISATIONAL-CAPABILITY.CT2-HEALTH-WORKFORCE-MANAGEMENT.CT3-WORKFORCE-SERVICE-COVERAGE-MANAGEMENT	CT3: Workforce Service Coverage Management (p. 128)	capability / Canonical
HCM-BEC-acute-care Structural ID: Unresolved	BEC: Acute Care (p. 277)	capability / Canonical
HCM-BEC-allied-health Structural ID: Unresolved	BEC: Allied Health (p. 299)	capability / Canonical
HCM-BEC-ambulatory-care Structural ID: Unresolved	BEC: Ambulatory Care (p. 289)	capability / Canonical
HCM-BEC-assurance-criteria-management Structural ID: Unresolved	BEC: Assurance Criteria Management (p. 442)	capability / Canonical
HCM-BEC-assurance-design Structural ID: Unresolved	BEC: Assurance Design (p. 439)	capability / Canonical
HCM-BEC-bed-care-place-management Structural ID: Unresolved	BEC: Bed & Care-Place Management (p. 335)	capability / Canonical
HCM-BEC-calendar-management Structural ID: Unresolved	BEC: Calendar Management (p. 426)	capability / Canonical
HCM-BEC-care-plan-administration Structural ID: Unresolved	BEC: Care Plan Administration (p. 261)	capability / Canonical

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HCM-BEC-client-administration Structural ID: Unresolved	BEC: Client Administration (p. 154)	capability / Canonical
HCM-BEC-client-centred-administration Structural ID: Unresolved	BEC: Client-Centred Administration (p. 212)	capability / Canonical
HCM-BEC-client-privacy Structural ID: Unresolved	BEC: Client Privacy (p. 207)	capability / Canonical
HCM-BEC-client-relationship Structural ID: Unresolved	BEC: Client Relationship (p. 203)	capability / Canonical
HCM-BEC-clinic-practice-operations Structural ID: Unresolved	BEC: Clinic & Practice Operations (p. 316)	capability / Canonical
HCM-BEC-clinical-assessment Structural ID: Unresolved	BEC: Clinical Assessment (p. 272)	capability / Canonical
HCM-BEC-clinical-charging-claims-administration Structural ID: Unresolved	BEC: Clinical Charging & Claims Administration (p. 271)	capability / Canonical
HCM-BEC-clinical-collaboration Structural ID: Unresolved	BEC: Clinical Collaboration (p. 408)	capability / Canonical
HCM-BEC-clinical-communication-administration Structural ID: Unresolved	BEC: Clinical Communication Administration (p. 265)	capability / Canonical
HCM-BEC-clinical-competency-management Structural ID: Unresolved	BEC: Clinical Competency Management (p. 360)	capability / Canonical
HCM-BEC-clinical-device-administration Structural ID: Unresolved	BEC: Clinical Device Administration (p. 185)	capability / Canonical
HCM-BEC-clinical-knowledge-services Structural ID: Unresolved	BEC: Clinical Knowledge Services (p. 404)	capability / Canonical
HCM-BEC-clinical-logistics-coordination Structural ID: Unresolved	BEC: Clinical Logistics Coordination (p. 368)	capability / Canonical
HCM-BEC-clinical-qualification-management Structural ID: Unresolved	BEC: Clinical Qualification Management (p. 361)	capability / Canonical
HCM-BEC-clinical-record-administration Structural ID: Unresolved	BEC: Clinical Record Administration (p. 255)	capability / Canonical
HCM-BEC-clinical-resource-management Structural ID: Unresolved	BEC: Clinical Resource Management (p. 341)	capability / Canonical
HCM-BEC-clinical-rostering Structural ID: Unresolved	BEC: Clinical Rostering (p. 346)	capability / Canonical
HCM-BEC-clinical-supervision Structural ID: Unresolved	BEC: Clinical Supervision (p. 363)	capability / Canonical
HCM-BEC-clinical-training-education Structural ID: Unresolved	BEC: Clinical Training & Education (p. 360)	capability / Canonical
HCM-BEC-clinical-workforce-management Structural ID: Unresolved	BEC: Clinical Workforce Management (p. 345)	capability / Canonical
HCM-BEC-community-care Structural ID: Unresolved	BEC: Community Care (p. 304)	capability / Canonical
HCM-BEC-credential-management Structural ID: Unresolved	BEC: Credential Management (p. 376)	capability / Canonical
HCM-BEC-diagnostic-administration Structural ID: Unresolved	BEC: Diagnostic Administration (p. 241)	capability / Canonical
HCM-BEC-diagnostic-services Structural ID: Unresolved	BEC: Diagnostic Services (p. 290)	capability / Canonical
HCM-BEC-discharge-management Structural ID: Unresolved	BEC: Discharge Management (p. 371)	capability / Canonical

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`HCM-BEC-emergency-care` Structural ID: Unresolved	BEC: Emergency Care (p. 281)	capability / Canonical
`HCM-BEC-emergency-department-operations` Structural ID: Unresolved	BEC: Emergency Department Operations (p. 328)	capability / Canonical
`HCM-BEC-episode-encounter-administration` Structural ID: Unresolved	BEC: Episode & Encounter Administration (p. 225)	capability / Canonical
`HCM-BEC-funding-coverage-administration` Structural ID: Unresolved	BEC: Funding & Coverage Administration (p. 271)	capability / Canonical
`HCM-BEC-general-collaboration` Structural ID: Unresolved	BEC: General Collaboration (p. 414)	capability / Canonical
`HCM-BEC-governed-assurance` Structural ID: Unresolved	BEC: Governed Assurance (p. 443)	capability / Canonical
`HCM-BEC-health-information-access` Structural ID: Unresolved	BEC: Health Information Access (p. 391)	capability / Canonical
`HCM-BEC-health-information-communication` Structural ID: Unresolved	BEC: Health Information Communication (p. 395)	capability / Canonical
`HCM-BEC-health-information-control` Structural ID: Unresolved	BEC: Health Information Control (p. 398)	capability / Canonical
`HCM-BEC-health-information-exchange` Structural ID: Unresolved	BEC: Health Information Exchange (p. 383)	capability / Canonical
`HCM-BEC-health-product-administration` Structural ID: Unresolved	BEC: Health Product Administration (p. 185)	capability / Canonical
`HCM-BEC-health-service-administration` Structural ID: Unresolved	BEC: Health Service Administration (p. 178)	capability / Canonical
`HCM-BEC-healthcare-subject` Structural ID: Unresolved	BEC: Healthcare Subject (p. 200)	capability / Canonical
`HCM-BEC-information-design-governance` Structural ID: Unresolved	BEC: Information Design Governance (p. 432)	capability / Canonical
`HCM-BEC-inpatient-care` Structural ID: Unresolved	BEC: Inpatient Care (p. 285)	capability / Canonical
`HCM-BEC-location-administration` Structural ID: Unresolved	BEC: Location Administration (p. 173)	capability / Canonical
`HCM-BEC-medication-administration` Structural ID: Unresolved	BEC: Medication Administration (p. 246)	capability / Canonical
`HCM-BEC-medication-therapy` Structural ID: Unresolved	BEC: Medication Therapy (p. 295)	capability / Canonical
`HCM-BEC-mobile-staff-management` Structural ID: Unresolved	BEC: Mobile Staff Management (p. 347)	capability / Canonical
`HCM-BEC-on-call-management` Structural ID: Unresolved	BEC: On-Call Management (p. 351)	capability / Canonical
`HCM-BEC-order-administration` Structural ID: Unresolved	BEC: Order Administration (p. 232)	capability / Canonical
`HCM-BEC-organisation-administration` Structural ID: Unresolved	BEC: Organisation Administration (p. 166)	capability / Canonical
`HCM-BEC-outpatient-operations` Structural ID: Unresolved	BEC: Outpatient Operations (p. 331)	capability / Canonical
`HCM-BEC-outreach-care` Structural ID: Unresolved	BEC: Outreach Care (p. 308)	capability / Canonical
`HCM-BEC-patient-clinical-record` Structural ID: Unresolved	BEC: Patient Clinical Record (p. 378)	capability / Canonical
`HCM-BEC-patient-transport` Structural ID: Unresolved	BEC: Patient Transport (p. 364)	capability / Canonical
`HCM-BEC-person-identity` Structural ID: Unresolved	BEC: Person Identity (p. 192)	capability / Canonical

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`HCM-BEC-presentation-services` Structural ID: Unresolved	BEC: Presentation Services (p. 430)	capability / Canonical
`HCM-BEC-preventive-care` Structural ID: Unresolved	BEC: Preventive Care (p. 299)	capability / Canonical
`HCM-BEC-primary-care` Structural ID: Unresolved	BEC: Primary Care (p. 273)	capability / Canonical
`HCM-BEC-procedure-administration` Structural ID: Unresolved	BEC: Procedure Administration (p. 251)	capability / Canonical
`HCM-BEC-provider-administration` Structural ID: Unresolved	BEC: Provider Administration (p. 155)	capability / Canonical
`HCM-BEC-referral-administration` Structural ID: Unresolved	BEC: Referral Administration (p. 215)	capability / Canonical
`HCM-BEC-rehabilitation` Structural ID: Unresolved	BEC: Rehabilitation (p. 303)	capability / Canonical
`HCM-BEC-remote-virtual-care` Structural ID: Unresolved	BEC: Remote & Virtual Care (p. 311)	capability / Canonical
`HCM-BEC-scheduling-administration` Structural ID: Unresolved	BEC: Scheduling Administration (p. 222)	capability / Canonical
`HCM-BEC-service-capacity-management` Structural ID: Unresolved	BEC: Service Capacity Management (p. 343)	capability / Canonical
`HCM-BEC-surgical-procedural-care` Structural ID: Unresolved	BEC: Surgical & Procedural Care (p. 290)	capability / Canonical
`HCM-BEC-theatre-operations` Structural ID: Unresolved	BEC: Theatre Operations (p. 324)	capability / Canonical
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`HCM-BEC-ward-operations` Structural ID: Unresolved	BEC: Ward Operations (p. 320)	capability / Canonical
`HCM-BEC-work-allocation-dispatch` Structural ID: Unresolved	BEC: Work Allocation & Dispatch (p. 354)	capability / Canonical
`HCM-BEC-workflow-activity-coordination` Structural ID: Unresolved	BEC: Workflow & Activity Coordination (p. 416)	capability / Canonical
`HCM-FN-adjudicate-assurance-assessment` Structural ID: Unresolved	Function: Adjudicate Assurance Assessment (p. 446)	function / Canonical
`HCM-FN-aggregate-service-capacity-telemetry` Structural ID: Unresolved	Function: Aggregate Service Capacity Telemetry (p. 344)	function / Canonical
`HCM-FN-apply-access-qualified-result-filtering` Structural ID: Unresolved	Function: Apply Access-Qualified Result Filtering (p. 395)	function / Canonical
`HCM-FN-apply-governed-person-identity-correction` Structural ID: Unresolved	Function: Apply Governed Person Identity Correction (p. 198)	function / Canonical
`HCM-FN-assemble-acute-clinical-baseline` Structural ID: Unresolved	Function: Assemble Acute Clinical Baseline (p. 279)	function / Canonical
`HCM-FN-assemble-inpatient-discharge-dossier` Structural ID: Unresolved	Function: Assemble Inpatient Discharge Dossier (p. 288)	function / Canonical
`HCM-FN-assemble-longitudinal-clinical-record` Structural ID: Unresolved	Function: Assemble Longitudinal Clinical Record (p. 381)	function / Canonical
`HCM-FN-assemble-referral-context` Structural ID: Unresolved	Function: Assemble Referral Context (p. 219)	function / Canonical
`HCM-FN-assemble-ward-handover-context` Structural ID: Unresolved	Function: Assemble Ward Handover Context (p. 323)	function / Canonical

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HCM-FN-assess-assurance-evidence Structural ID: Unresolved	Function: Assess Assurance Evidence (p. 446)	function / Canonical
HCM-FN-associate-care-plan-context Structural ID: Unresolved	Function: Associate Care Plan Context (p. 264)	function / Canonical
HCM-FN-associate-order-outcome Structural ID: Unresolved	Function: Associate Order Outcome (p. 239)	function / Canonical
HCM-FN-bind-community-support-network-context Structural ID: Unresolved	Function: Bind Community Support Network Context (p. 307)	function / Canonical
HCM-FN-bind-diagnostic-report Structural ID: Unresolved	Function: Bind Diagnostic Report (p. 244)	function / Canonical
HCM-FN-bind-ed-triage-assessment Structural ID: Unresolved	Function: Bind ED Triage Assessment (p. 283)	function / Canonical
HCM-FN-bind-practitioner-electronic-endpoints Structural ID: Unresolved	Function: Bind Practitioner Electronic Endpoints (p. 163)	function / Canonical
HCM-FN-capture-post-clinic-follow-up-requirements Structural ID: Unresolved	Function: Capture Post-Clinic Follow-up Requirements (p. 335)	function / Canonical
HCM-FN-consolidate-inpatient-clinical-round-feed Structural ID: Unresolved	Function: Consolidate Inpatient Clinical Round Feed (p. 287)	function / Canonical
HCM-FN-coordinate-care-team-collaboration Structural ID: Unresolved	Function: Coordinate Care-Team Collaboration (p. 411)	function / Canonical
HCM-FN-coordinate-ed-departmental-capacity Structural ID: Unresolved	Function: Coordinate ED Departmental Capacity (p. 330)	function / Canonical
HCM-FN-coordinate-multi-agency-chronic-care-plan Structural ID: Unresolved	Function: Coordinate Multi-Agency Chronic Care Plan (p. 276)	function / Canonical
HCM-FN-coordinate-operational-collaboration Structural ID: Unresolved	Function: Coordinate Operational Collaboration (p. 415)	function / Canonical
HCM-FN-coordinate-order-modification-cancellation Structural ID: Unresolved	Function: Coordinate Order Modification / Cancellation (p. 238)	function / Canonical
HCM-FN-coordinate-synthetic-task Structural ID: Unresolved	Function: Coordinate Synthetic Task (p. 422)	function / Canonical
HCM-FN-coordinate-to-do Structural ID: Unresolved	Function: Coordinate To Do (p. 421)	function / Canonical
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HCM-FN-correlate-adverse-reaction-allergy-profile Structural ID: Unresolved	Function: Correlate Adverse Reaction & Allergy Profile (p. 298)	function / Canonical
HCM-FN-correlate-diagnostic-request Structural ID: Unresolved	Function: Correlate Diagnostic Request (p. 244)	function / Canonical
HCM-FN-correlate-person-identifiers Structural ID: Unresolved	Function: Correlate Person Identifiers (p. 197)	function / Canonical
HCM-FN-correlate-primary-care-consultation-context Structural ID: Unresolved	Function: Correlate Primary Care Consultation Context (p. 275)	function / Canonical

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`HCM-FN-correlate-virtual-care-consultation-context` Structural ID: Unresolved	Function: Correlate Virtual Care Consultation Context (p. 313)	function / Canonical
`HCM-FN-design-service-assurance` Structural ID: Unresolved	Function: Design Service Assurance (p. 440)	function / Canonical
`HCM-FN-dispatch-operational-work-order` Structural ID: Unresolved	Function: Dispatch Operational Work Order (p. 357)	function / Canonical
`HCM-FN-distribute-addressed-information` Structural ID: Unresolved	Function: Distribute Addressed Information (p. 389)	function / Canonical
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`HCM-FN-distribute-clinical-communication` Structural ID: Unresolved	Function: Distribute Clinical Communication (p. 268)	function / Canonical
`HCM-FN-distribute-diagnostic-report` Structural ID: Unresolved	Function: Distribute Diagnostic Report (p. 245)	function / Canonical
`HCM-FN-distribute-governed-demographic-change` Structural ID: Unresolved	Function: Distribute Governed Demographic Change (p. 215)	function / Canonical
`HCM-FN-establish-assurance-context` Structural ID: Unresolved	Function: Establish Assurance Context (p. 445)	function / Canonical
`HCM-FN-establish-encounter-context` Structural ID: Unresolved	Function: Establish Encounter Context (p. 228)	function / Canonical
`HCM-FN-establish-healthcare-subject-context` Structural ID: Unresolved	Function: Establish Healthcare Subject Context (p. 202)	function / Canonical
`HCM-FN-evaluate-client-confidentiality-restrictions` Structural ID: Unresolved	Function: Evaluate Client Confidentiality Restrictions (p. 210)	function / Canonical
`HCM-FN-evaluate-client-consent` Structural ID: Unresolved	Function: Evaluate Client Consent (p. 210)	function / Canonical
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`HCM-FN-govern-client-relationship-validity` Structural ID: Unresolved	Function: Govern Client Relationship Validity (p. 207)	function / Canonical
`HCM-FN-govern-clinical-document-lifecycle` Structural ID: Unresolved	Function: Govern Clinical Document Lifecycle (p. 259)	function / Canonical
`HCM-FN-govern-collaboration-metadata` Structural ID: Unresolved	Function: Govern Collaboration Metadata (p. 413)	function / Canonical
`HCM-FN-govern-exchange-profile` Structural ID: Unresolved	Function: Govern Exchange Profile (p. 437)	function / Canonical
`HCM-FN-govern-information-definition` Structural ID: Unresolved	Function: Govern Information Definition (p. 435)	function / Canonical
`HCM-FN-govern-information-semantics` Structural ID: Unresolved	Function: Govern Information Semantics (p. 435)	function / Canonical
`HCM-FN-govern-organisation-profile` Structural ID: Unresolved	Function: Govern Organisation Profile (p. 170)	function / Canonical
`HCM-FN-govern-practitioner-profile` Structural ID: Unresolved	Function: Govern Practitioner Profile (p. 161)	function / Canonical

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`HCM-FN-govern-subject-demographic-context` Structural ID: Unresolved	Function: Govern Subject Demographic Context (p. 203)	function / Canonical
`HCM-FN-govern-terminology-binding` Structural ID: Unresolved	Function: Govern Terminology Binding (p. 436)	function / Canonical
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`HCM-FN-ingest-remote-patient-telemetry` Structural ID: Unresolved	Function: Ingest Remote Patient Telemetry (p. 314)	function / Canonical
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`HCM-FN-maintain-clinical-privileges` Structural ID: Unresolved	Function: Maintain Clinical Privileges (p. 164)	function / Canonical
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`HCM-FN-maintain-device-communication-endpoints` Structural ID: Unresolved	Function: Maintain Device Communication Endpoints (p. 191)	function / Canonical
`HCM-FN-maintain-device-definition` Structural ID: Unresolved	Function: Maintain Device Definition (p. 189)	function / Canonical
`HCM-FN-maintain-device-instance-identity` Structural ID: Unresolved	Function: Maintain Device Instance Identity (p. 190)	function / Canonical
`HCM-FN-maintain-encounter-care-place-association` Structural ID: Unresolved	Function: Maintain Encounter Care-Place Association (p. 230)	function / Canonical
`HCM-FN-maintain-healthcare-location` Structural ID: Unresolved	Function: Maintain Healthcare Location (p. 176)	function / Canonical
`HCM-FN-maintain-healthcare-service-definition` Structural ID: Unresolved	Function: Maintain Healthcare Service Definition (p. 182)	function / Canonical
`HCM-FN-maintain-operational-bed-state` Structural ID: Unresolved	Function: Maintain Operational Bed State (p. 338)	function / Canonical
`HCM-FN-maintain-organisation-contacts` Structural ID: Unresolved	Function: Maintain Organisation Contacts (p. 172)	function / Canonical
`HCM-FN-maintain-organisational-structure` Structural ID: Unresolved	Function: Maintain Organisational Structure (p. 171)	function / Canonical
`HCM-FN-maintain-person-identity-aliases` Structural ID: Unresolved	Function: Maintain Person Identity Aliases (p. 197)	function / Canonical
`HCM-FN-maintain-physical-location-hierarchy` Structural ID: Unresolved	Function: Maintain Physical Location Hierarchy (p. 176)	function / Canonical
`HCM-FN-maintain-practitioner-affiliations` Structural ID: Unresolved	Function: Maintain Practitioner Affiliations (p. 163)	function / Canonical

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HCM-FN-maintain-practitioner-roles Structural ID: Unresolved	Function: Maintain Practitioner Roles (p. 162)	function / Canonical
HCM-FN-maintain-service-availability Structural ID: Unresolved	Function: Maintain Service Availability (p. 183)	function / Canonical
HCM-FN-maintain-service-eligibility-rules Structural ID: Unresolved	Function: Maintain Service Eligibility Rules (p. 184)	function / Canonical
HCM-FN-maintain-service-location-provider-map Structural ID: Unresolved	Function: Maintain Service / Location / Provider Map (p. 182)	function / Canonical
HCM-FN-manage-assurance-criteria Structural ID: Unresolved	Function: Manage Assurance Criteria (p. 443)	function / Canonical
HCM-FN-manage-bed-turnover-progression Structural ID: Unresolved	Function: Manage Bed Turnover Progression (p. 339)	function / Canonical
HCM-FN-manage-clinic-practice-capacity Structural ID: Unresolved	Function: Manage Clinic & Practice Capacity (p. 318)	function / Canonical
HCM-FN-manage-clinic-queue-progression Structural ID: Unresolved	Function: Manage Clinic Queue Progression (p. 319)	function / Canonical
HCM-FN-manage-discharge-coordination-progression Structural ID: Unresolved	Function: Manage Discharge Coordination Progression (p. 374)	function / Canonical
HCM-FN-manage-mobile-service-capacity Structural ID: Unresolved	Function: Manage Mobile Service Capacity (p. 349)	function / Canonical
HCM-FN-manage-operational-work-progression Structural ID: Unresolved	Function: Manage Operational Work Progression (p. 358)	function / Canonical
HCM-FN-manage-order-progression Structural ID: Unresolved	Function: Manage Order Progression (p. 237)	function / Canonical
HCM-FN-manage-outpatient-capacity Structural ID: Unresolved	Function: Manage Outpatient Capacity (p. 334)	function / Canonical
HCM-FN-manage-patient-transport-progression Structural ID: Unresolved	Function: Manage Patient Transport Progression (p. 366)	function / Canonical
HCM-FN-manage-referral-progression Structural ID: Unresolved	Function: Manage Referral Progression (p. 220)	function / Canonical
HCM-FN-manage-specimen-transport-progression Structural ID: Unresolved	Function: Manage Specimen Transport Progression (p. 369)	function / Canonical
HCM-FN-manage-theatre-capacity Structural ID: Unresolved	Function: Manage Theatre Capacity (p. 326)	function / Canonical
HCM-FN-manage-theatre-case-progression Structural ID: Unresolved	Function: Manage Theatre Case Progression (p. 327)	function / Canonical
HCM-FN-manage-ward-capacity Structural ID: Unresolved	Function: Manage Ward Capacity (p. 322)	function / Canonical
HCM-FN-map-clinical-concept Structural ID: Unresolved	Function: Map Clinical Concept (p. 407)	function / Canonical
HCM-FN-mediate-information-retrieval Structural ID: Unresolved	Function: Mediate Information Retrieval (p. 388)	function / Canonical
HCM-FN-mediate-standards-based-health-information-communication Structural ID: Unresolved	Function: Mediate Standards-Based Health Information Communication (p. 397)	function / Canonical
HCM-FN-present-contextual-health-and-operational-information Structural ID: Unresolved	Function: Present Contextual Health and Operational Information (p. 431)	function / Canonical

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`HCM-FN-preserve-durable-clinical-record` Structural ID: Unresolved	Function: Preserve Durable Clinical Record (p. 383)	function / Canonical
`HCM-FN-progress-encounter-state` Structural ID: Unresolved	Function: Progress Encounter State (p. 229)	function / Canonical
`HCM-FN-project-longitudinal-medication-history` Structural ID: Unresolved	Function: Project Longitudinal Medication History (p. 297)	function / Canonical
`HCM-FN-project-operational-schedule` Structural ID: Unresolved	Function: Project Operational Schedule (p. 428)	function / Canonical
`HCM-FN-project-screening-eligibility-timeline` Structural ID: Unresolved	Function: Project Screening Eligibility Timeline (p. 302)	function / Canonical
`HCM-FN-propagate-security-context` Structural ID: Unresolved	Function: Propagate Security Context (p. 402)	function / Canonical
`HCM-FN-provide-emergency-lhr-summary` Structural ID: Unresolved	Function: Provide Emergency LHR Summary (p. 284)	function / Canonical
`HCM-FN-provide-outreach-visit-context` Structural ID: Unresolved	Function: Provide Outreach Visit Context (p. 310)	function / Canonical
`HCM-FN-publish-governed-diagnostic-report` Structural ID: Unresolved	Function: Publish Governed Diagnostic Report (p. 293)	function / Canonical
`HCM-FN-receive-appointment-notification` Structural ID: Unresolved	Function: Receive Appointment Notification (p. 224)	function / Canonical
`HCM-FN-receive-care-plan` Structural ID: Unresolved	Function: Receive Care Plan (p. 263)	function / Canonical
`HCM-FN-receive-client-demographic-change` Structural ID: Unresolved	Function: Receive Client Demographic Change (p. 214)	function / Canonical
`HCM-FN-receive-clinical-document` Structural ID: Unresolved	Function: Receive Clinical Document (p. 258)	function / Canonical
`HCM-FN-receive-diagnostic-requisition-feed` Structural ID: Unresolved	Function: Receive Diagnostic Requisition Feed (p. 292)	function / Canonical
`HCM-FN-receive-information-submission` Structural ID: Unresolved	Function: Receive Information Submission (p. 387)	function / Canonical
`HCM-FN-receive-medication-administration-event` Structural ID: Unresolved	Function: Receive Medication Administration Event (p. 250)	function / Canonical
`HCM-FN-receive-medication-order` Structural ID: Unresolved	Function: Receive Medication Order (p. 249)	function / Canonical
`HCM-FN-receive-mobile-visit-progression` Structural ID: Unresolved	Function: Receive Mobile Visit Progression (p. 350)	function / Canonical
`HCM-FN-receive-order-request` Structural ID: Unresolved	Function: Receive Order Request (p. 236)	function / Canonical
`HCM-FN-receive-procedural-documentation` Structural ID: Unresolved	Function: Receive Procedural Documentation (p. 254)	function / Canonical
`HCM-FN-receive-procedure-booking-request` Structural ID: Unresolved	Function: Receive Procedure Booking / Request (p. 253)	function / Canonical
`HCM-FN-receive-referral` Structural ID: Unresolved	Function: Receive Referral (p. 218)	function / Canonical
`HCM-FN-reconcile-outreach-clinical-batch` Structural ID: Unresolved	Function: Reconcile Outreach Clinical Batch (p. 310)	function / Canonical

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`HCM-FN-record-communication-transaction` Structural ID: Unresolved	Function: Record Communication Transaction (p. 270)	function / Canonical
`HCM-FN-record-security-significant-activity` Structural ID: Unresolved	Function: Record Security-Significant Activity (p. 404)	function / Canonical
`HCM-FN-resolve-active-on-call-coverage` Structural ID: Unresolved	Function: Resolve Active On-Call Coverage (p. 353)	function / Canonical
`HCM-FN-resolve-clinical-concept` Structural ID: Unresolved	Function: Resolve Clinical Concept (p. 407)	function / Canonical
`HCM-FN-resolve-lhr-query-within-collaboration` Structural ID: Unresolved	Function: Resolve LHR Query within Collaboration (p. 412)	function / Canonical
`HCM-FN-resolve-order-destination` Structural ID: Unresolved	Function: Resolve Order Destination (p. 236)	function / Canonical
`HCM-FN-resolve-person-identifier` Structural ID: Unresolved	Function: Resolve Person Identifier (p. 196)	function / Canonical
`HCM-FN-retrieve-health-information` Structural ID: Unresolved	Function: Retrieve Health Information (p. 394)	function / Canonical
`HCM-FN-route-acute-critical-result-alert` Structural ID: Unresolved	Function: Route Acute Critical Result Alert (p. 280)	function / Canonical
`HCM-FN-search-health-information` Structural ID: Unresolved	Function: Search Health Information (p. 393)	function / Canonical
`HCM-FN-supervise-activity-timeout-escalation` Structural ID: Unresolved	Function: Supervise Activity Timeout / Escalation (p. 423)	function / Canonical
`HCM-FN-synchronise-appointment-status` Structural ID: Unresolved	Function: Synchronise Appointment Status (p. 225)	function / Canonical
`HCM-FN-synchronize-community-visit-record` Structural ID: Unresolved	Function: Synchronize Community Visit Record (p. 306)	function / Canonical
`HCM-FN-syndicate-immunisation-record` Structural ID: Unresolved	Function: Syndicate Immunisation Record (p. 301)	function / Canonical
`HCM-FN-syndicate-information-change` Structural ID: Unresolved	Function: Syndicate Information Change (p. 390)	function / Canonical
`HCM-FN-track-business-delivery-acknowledgement` Structural ID: Unresolved	Function: Track Business Delivery Acknowledgement (p. 269)	function / Canonical
`HCM-FN-track-clinic-operating-session` Structural ID: Unresolved	Function: Track Clinic Operating Session (p. 318)	function / Canonical
`HCM-FN-track-dispense-event` Structural ID: Unresolved	Function: Track Dispense Event (p. 250)	function / Canonical
`HCM-FN-track-mobile-clinical-asset-operational-state` Structural ID: Unresolved	Function: Track Mobile Clinical Asset Operational State (p. 342)	function / Canonical
`HCM-FN-track-operational-staff-presence` Structural ID: Unresolved	Function: Track Operational Staff Presence (p. 349)	function / Canonical
`HCM-FN-verify-clinical-qualification-for-work-dispatch` Structural ID: Unresolved	Function: Verify Clinical Qualification for Work Dispatch (p. 363)	function / Canonical
`HCM-FN-verify-healthcare-organisation` Structural ID: Unresolved	Function: Verify Healthcare Organisation (p. 169)	function / Canonical
`HCM-FN-verify-operational-access-credential` Structural ID: Unresolved	Function: Verify Operational Access Credential (p. 377)	function / Canonical

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`HCM-FN-verify-practitioner-registration` Structural ID: Unresolved	Function: Verify Practitioner Registration (p. 160)	function / Canonical
`HCM-FN-verify-semantic-conformance` Structural ID: Unresolved	Function: Verify Semantic Conformance (p. 438)	function / Canonical
`HCM-PR-assurance-process-design` Structural ID: Unresolved	Process: Assurance Process Design (p. 441)	process / Canonical
`HCM-PR-assurance-process-execution` Structural ID: Unresolved	Process: Assurance Process Execution (p. 450)	process / Canonical
`HCM-PR-assurance-process-reporting-communication` Structural ID: Unresolved	Process: Assurance Process Reporting / Communication (p. 451)	process / Canonical
`HCM-PR-bed-turnover-process` Structural ID: Unresolved	Process: Bed Turnover Process (p. 340)	process / Canonical
`HCM-PR-clinic-operational-queue-progression-process` Structural ID: Unresolved	Process: Clinic / Operational Queue Progression Process (p. 320)	process / Canonical
`HCM-PR-clinical-document-lifecycle-process` Structural ID: Unresolved	Process: Clinical Document Lifecycle Process (p. 260)	process / Canonical
`HCM-PR-closed-loop-order-progression-process` Structural ID: Unresolved	Process: Closed-Loop Order Progression Process (p. 240)	process / Canonical
`HCM-PR-discharge-progression-process` Structural ID: Unresolved	Process: Discharge Progression Process (p. 374)	process / Canonical
`HCM-PR-encounter-lifecycle-process` Structural ID: Unresolved	Process: Encounter Lifecycle Process (p. 231)	process / Canonical
`HCM-PR-governed-person-identity-correction-process` Structural ID: Unresolved	Process: Governed Person Identity Correction Process (p. 199)	process / Canonical
`HCM-PR-operational-work-progression-process` Structural ID: Unresolved	Process: Operational Work Progression Process (p. 359)	process / Canonical
`HCM-PR-patient-transport-process` Structural ID: Unresolved	Process: Patient Transport Process (p. 367)	process / Canonical
`HCM-PR-practitioner-verification-process` Structural ID: Unresolved	Process: Practitioner Verification Process (p. 165)	process / Canonical
`HCM-PR-referral-progression-process` Structural ID: Unresolved	Process: Referral Progression Process (p. 221)	process / Canonical
`HCM-PR-specimen-transport-process` Structural ID: Unresolved	Process: Specimen Transport Process (p. 370)	process / Canonical
`HCM-PR-synthetic-task-progression-process` Structural ID: Unresolved	Process: Synthetic Task Progression Process (p. 425)	process / Canonical
`HCM-PR-theatre-case-progression-process` Structural ID: Unresolved	Process: Theatre Case Progression Process (p. 328)	process / Canonical
`HCM-PR-to-do-progression-process` Structural ID: Unresolved	Process: To Do Progression Process (p. 424)	process / Canonical
`HCM-PR-work-order-progression-process` Structural ID: Unresolved	Process: Work Order Progression Process (p. 423)	process / Canonical
`HCM-SV-active-problem-allergy-summary-query` Structural ID: Unresolved	Service: Active Problem & Allergy Summary Query (p. 382)	service / Canonical
`HCM-SV-activity-escalation-telemetry` Structural ID: Unresolved	Service: Activity Escalation Telemetry (p. 423)	service / Canonical

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`HCM-SV-acute-admission-dossier-query` Structural ID: Unresolved	Service: Acute Admission Dossier Query (p. 280)	service / Canonical
`HCM-SV-acute-clinical-alert-dispatch` Structural ID: Unresolved	Service: Acute Clinical Alert Dispatch (p. 280)	service / Canonical
`HCM-SV-addressed-document-delivery-service` Structural ID: Unresolved	Service: Addressed Document Delivery Service (p. 389)	service / Canonical
`HCM-SV-allergy-adverse-reaction-query` Structural ID: Unresolved	Service: Allergy & Adverse Reaction Query (p. 298)	service / Canonical
`HCM-SV-ambulatory-session-status-query` Structural ID: Unresolved	Service: Ambulatory Session Status Query (p. 334)	service / Canonical
`HCM-SV-appointment-notification-ingress` Structural ID: Unresolved	Service: Appointment Notification Ingress (p. 224)	service / Canonical
`HCM-SV-appointment-schedule-query` Structural ID: Unresolved	Service: Appointment Schedule Query (p. 225)	service / Canonical
`HCM-SV-bed-availability-query` Structural ID: Unresolved	Service: Bed Availability Query (p. 339)	service / Canonical
`HCM-SV-bed-turnover-lifecycle-service` Structural ID: Unresolved	Service: Bed Turnover Lifecycle Service (p. 339)	service / Canonical
`HCM-SV-care-place-specification-lookup` Structural ID: Unresolved	Service: Care-Place Specification Lookup (p. 178)	service / Canonical
`HCM-SV-care-plan-change-notification` Structural ID: Unresolved	Service: Care Plan Change Notification (p. 265)	service / Canonical
`HCM-SV-care-plan-context-query` Structural ID: Unresolved	Service: Care Plan Context Query (p. 264)	service / Canonical
`HCM-SV-care-plan-ingress` Structural ID: Unresolved	Service: Care Plan Ingress (p. 263)	service / Canonical
`HCM-SV-chronic-care-timeline-query` Structural ID: Unresolved	Service: Chronic Care Timeline Query (p. 276)	service / Canonical
`HCM-SV-client-relationship-query` Structural ID: Unresolved	Service: Client Relationship Query (p. 206)	service / Canonical
`HCM-SV-clinic-queue-telemetry-service` Structural ID: Unresolved	Service: Clinic Queue Telemetry Service (p. 319)	service / Canonical
`HCM-SV-clinical-collaboration-space-service` Structural ID: Unresolved	Service: Clinical Collaboration Space Service (p. 412)	service / Canonical
`HCM-SV-clinical-document-ingress` Structural ID: Unresolved	Service: Clinical Document Ingress (p. 258)	service / Canonical
`HCM-SV-clinical-document-lifecycle-service` Structural ID: Unresolved	Service: Clinical Document Lifecycle Service (p. 259)	service / Canonical
`HCM-SV-clinical-event-syndication-stream` Structural ID: Unresolved	Service: Clinical Event Syndication Stream (p. 390)	service / Canonical
`HCM-SV-clinical-information-search-service` Structural ID: Unresolved	Service: Clinical Information Search Service (p. 393)	service / Canonical
`HCM-SV-clinical-privilege-verification` Structural ID: Unresolved	Service: Clinical Privilege Verification (p. 165)	service / Canonical
`HCM-SV-clinical-resource-availability-query` Structural ID: Unresolved	Service: Clinical Resource Availability Query (p. 343)	service / Canonical

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`HCM-SV-clinical-resource-retrieval-service` Structural ID: Unresolved	Service: Clinical Resource Retrieval Service (p. 394)	service / Canonical
`HCM-SV-collaboration-clinical-summary-resolution` Structural ID: Unresolved	Service: Collaboration Clinical Summary Resolution (p. 413)	service / Canonical
`HCM-SV-collaboration-space-governance-service` Structural ID: Unresolved	Service: Collaboration Space Governance Service (p. 414)	service / Canonical
`HCM-SV-communicate-service-assurance-outcome` Structural ID: Unresolved	Service: Communicate Service Assurance Outcome (p. 449)	service / Canonical
`HCM-SV-communication-audit-query` Structural ID: Unresolved	Service: Communication Audit Query (p. 270)	service / Canonical
`HCM-SV-communication-delivery-status-query` Structural ID: Unresolved	Service: Communication Delivery Status Query (p. 269)	service / Canonical
`HCM-SV-community-care-encounter-ingress` Structural ID: Unresolved	Service: Community Care Encounter Ingress (p. 306)	service / Canonical
`HCM-SV-community-care-team-query` Structural ID: Unresolved	Service: Community Care Team Query (p. 307)	service / Canonical
`HCM-SV-confidentiality-restriction-evaluation` Structural ID: Unresolved	Service: Confidentiality Restriction Evaluation (p. 211)	service / Canonical
`HCM-SV-consent-decision-service` Structural ID: Unresolved	Service: Consent Decision Service (p. 210)	service / Canonical
`HCM-SV-consent-enforcement-service` Structural ID: Unresolved	Service: Consent Enforcement Service (p. 402)	service / Canonical
`HCM-SV-contextual-presentation-service` Structural ID: Unresolved	Service: Contextual Presentation Service (p. 432)	service / Canonical
`HCM-SV-demographic-change-distribution` Structural ID: Unresolved	Service: Demographic Change Distribution (p. 203)	service / Canonical
`HCM-SV-demographic-intake-service` Structural ID: Unresolved	Service: Demographic Intake Service (p. 214)	service / Canonical
`HCM-SV-demographic-update-notification` Structural ID: Unresolved	Service: Demographic Update Notification (p. 215)	service / Canonical
`HCM-SV-device-association-query` Structural ID: Unresolved	Service: Device Association Query (p. 191)	service / Canonical
`HCM-SV-device-definition-query` Structural ID: Unresolved	Service: Device Definition Query (p. 189)	service / Canonical
`HCM-SV-device-endpoint-resolution` Structural ID: Unresolved	Service: Device Endpoint Resolution (p. 192)	service / Canonical
`HCM-SV-device-instance-resolution` Structural ID: Unresolved	Service: Device Instance Resolution (p. 190)	service / Canonical
`HCM-SV-diagnostic-correlation-resolution` Structural ID: Unresolved	Service: Diagnostic Correlation Resolution (p. 244)	service / Canonical
`HCM-SV-diagnostic-publication-service` Structural ID: Unresolved	Service: Diagnostic Publication Service (p. 294)	service / Canonical
`HCM-SV-diagnostic-report-ingress` Structural ID: Unresolved	Service: Diagnostic Report Ingress (p. 245)	service / Canonical
`HCM-SV-diagnostic-result-delivery-service` Structural ID: Unresolved	Service: Diagnostic Result Delivery Service (p. 246)	service / Canonical

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`HCM-SV-discharge-dossier-packaging-service` Structural ID: Unresolved	Service: Discharge Dossier Packaging Service (p. 288)	service / Canonical
`HCM-SV-discharge-readiness-telemetry` Structural ID: Unresolved	Service: Discharge Readiness Telemetry (p. 374)	service / Canonical
`HCM-SV-dispense-event-ingress` Structural ID: Unresolved	Service: Dispense Event Ingress (p. 250)	service / Canonical
`HCM-SV-document-metadata-registry-query` Structural ID: Unresolved	Service: Document Metadata Registry Query (p. 260)	service / Canonical
`HCM-SV-ed-operational-status-query` Structural ID: Unresolved	Service: ED Operational Status Query (p. 331)	service / Canonical
`HCM-SV-ed-triage-stream-ingress` Structural ID: Unresolved	Service: ED Triage Stream Ingress (p. 284)	service / Canonical
`HCM-SV-emergency-fast-path-lhr-query` Structural ID: Unresolved	Service: Emergency Fast-Path LHR Query (p. 285)	service / Canonical
`HCM-SV-encounter-context-resolution` Structural ID: Unresolved	Service: Encounter Context Resolution (p. 229)	service / Canonical
`HCM-SV-encounter-lifecycle-event-notification` Structural ID: Unresolved	Service: Encounter Lifecycle Event Notification (p. 230)	service / Canonical
`HCM-SV-encounter-location-history-query` Structural ID: Unresolved	Service: Encounter Location History Query (p. 230)	service / Canonical
`HCM-SV-exchange-profile-specification-query` Structural ID: Unresolved	Service: Exchange Profile Specification Query (p. 437)	service / Canonical
`HCM-SV-federated-health-information-query` Structural ID: Unresolved	Service: Federated Health Information Query (p. 389)	service / Canonical
`HCM-SV-healthcare-service-directory-query` Structural ID: Unresolved	Service: Healthcare Service Directory Query (p. 182)	service / Canonical
`HCM-SV-healthcare-subject-context-resolution` Structural ID: Unresolved	Service: Healthcare Subject Context Resolution (p. 202)	service / Canonical
`HCM-SV-hie-submission-gateway-service` Structural ID: Unresolved	Service: HIE Submission Gateway Service (p. 388)	service / Canonical
`HCM-SV-historical-clinical-record-access` Structural ID: Unresolved	Service: Historical Clinical Record Access (p. 383)	service / Canonical
`HCM-SV-immunisation-history-query` Structural ID: Unresolved	Service: Immunisation History Query (p. 302)	service / Canonical
`HCM-SV-information-dictionary-query` Structural ID: Unresolved	Service: Information Dictionary Query (p. 436)	service / Canonical
`HCM-SV-location-directory-query` Structural ID: Unresolved	Service: Location Directory Query (p. 176)	service / Canonical
`HCM-SV-location-hierarchy-resolution` Structural ID: Unresolved	Service: Location Hierarchy Resolution (p. 177)	service / Canonical
`HCM-SV-longitudinal-clinical-record-query` Structural ID: Unresolved	Service: Longitudinal Clinical Record Query (p. 381)	service / Canonical
`HCM-SV-longitudinal-medication-history-query` Structural ID: Unresolved	Service: Longitudinal Medication History Query (p. 297)	service / Canonical
`HCM-SV-medication-administration-ingress` Structural ID: Unresolved	Service: Medication Administration Ingress (p. 251)	service / Canonical

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`HCM-SV-medication-order-ingress` Structural ID: Unresolved	Service: Medication Order Ingress (p. 249)	service / Canonical
`HCM-SV-modality-worklist-distribution` Structural ID: Unresolved	Service: Modality Worklist Distribution (p. 293)	service / Canonical
`HCM-SV-on-call-specialist-lookup` Structural ID: Unresolved	Service: On-Call Specialist Lookup (p. 353)	service / Canonical
`HCM-SV-operational-collaboration-service` Structural ID: Unresolved	Service: Operational Collaboration Service (p. 416)	service / Canonical
`HCM-SV-operational-credential-checkpoint-service` Structural ID: Unresolved	Service: Operational Credential Checkpoint Service (p. 377)	service / Canonical
`HCM-SV-operational-work-status-query` Structural ID: Unresolved	Service: Operational Work Status Query (p. 359)	service / Canonical
`HCM-SV-order-cancellation-service` Structural ID: Unresolved	Service: Order Cancellation Service (p. 238)	service / Canonical
`HCM-SV-order-dispatch-service` Structural ID: Unresolved	Service: Order Dispatch Service (p. 237)	service / Canonical
`HCM-SV-order-outcome-notification` Structural ID: Unresolved	Service: Order Outcome Notification (p. 239)	service / Canonical
`HCM-SV-order-requisition-ingress` Structural ID: Unresolved	Service: Order Requisition Ingress (p. 236)	service / Canonical
`HCM-SV-order-status-tracking-query` Structural ID: Unresolved	Service: Order Status & Tracking Query (p. 238)	service / Canonical
`HCM-SV-organisation-contact-lookup` Structural ID: Unresolved	Service: Organisation Contact Lookup (p. 172)	service / Canonical
`HCM-SV-organisation-directory-query` Structural ID: Unresolved	Service: Organisation Directory Query (p. 171)	service / Canonical
`HCM-SV-organisation-verification-service` Structural ID: Unresolved	Service: Organisation Verification Service (p. 170)	service / Canonical
`HCM-SV-organisational-hierarchy-resolution` Structural ID: Unresolved	Service: Organisational Hierarchy Resolution (p. 171)	service / Canonical
`HCM-SV-outreach-batch-reconciliation-service` Structural ID: Unresolved	Service: Outreach Batch Reconciliation Service (p. 311)	service / Canonical
`HCM-SV-patient-transport-tracking-service` Structural ID: Unresolved	Service: Patient Transport Tracking Service (p. 366)	service / Canonical
`HCM-SV-person-identifier-correlation` Structural ID: Unresolved	Service: Person Identifier Correlation (p. 197)	service / Canonical
`HCM-SV-person-identifier-resolution` Structural ID: Unresolved	Service: Person Identifier Resolution (p. 196)	service / Canonical
`HCM-SV-person-identity-correction` Structural ID: Unresolved	Service: Person Identity Correction (p. 198)	service / Canonical
`HCM-SV-policy-evaluation-authorisation-service` Structural ID: Unresolved	Service: Policy Evaluation & Authorisation Service (p. 401)	service / Canonical
`HCM-SV-practitioner-affiliation-resolution` Structural ID: Unresolved	Service: Practitioner Affiliation Resolution (p. 164)	service / Canonical
`HCM-SV-practitioner-directory-query` Structural ID: Unresolved	Service: Practitioner Directory Query (p. 161)	service / Canonical
`HCM-SV-practitioner-endpoint-lookup` Structural ID: Unresolved	Service: Practitioner Endpoint Lookup (p. 163)	service / Canonical
`HCM-SV-practitioner-role-resolution` Structural ID: Unresolved	Service: Practitioner Role Resolution (p. 162)	service / Canonical

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`HCM-SV-practitioner-verification-service` Structural ID: Unresolved	Service: Practitioner Verification Service (p. 161)	service / Canonical
`HCM-SV-preventive-screening-status-query` Structural ID: Unresolved	Service: Preventive Screening Status Query (p. 303)	service / Canonical
`HCM-SV-primary-care-summary-ingress` Structural ID: Unresolved	Service: Primary Care Summary Ingress (p. 276)	service / Canonical
`HCM-SV-procedural-documentation-ingress` Structural ID: Unresolved	Service: Procedural Documentation Ingress (p. 255)	service / Canonical
`HCM-SV-procedure-booking-ingress` Structural ID: Unresolved	Service: Procedure Booking Ingress (p. 254)	service / Canonical
`HCM-SV-referral-context-query` Structural ID: Unresolved	Service: Referral Context Query (p. 220)	service / Canonical
`HCM-SV-referral-status-outcome-service` Structural ID: Unresolved	Service: Referral Status & Outcome Service (p. 220)	service / Canonical
`HCM-SV-referral-submission-service` Structural ID: Unresolved	Service: Referral Submission Service (p. 219)	service / Canonical
`HCM-SV-regional-health-system-capacity-query` Structural ID: Unresolved	Service: Regional Health System Capacity Query (p. 345)	service / Canonical
`HCM-SV-remote-telemetry-ingress` Structural ID: Unresolved	Service: Remote Telemetry Ingress (p. 314)	service / Canonical
`HCM-SV-representative-authority-verification` Structural ID: Unresolved	Service: Representative Authority Verification (p. 207)	service / Canonical
`HCM-SV-request-service-assurance` Structural ID: Unresolved	Service: Request Service Assurance (p. 447)	service / Canonical
`HCM-SV-request-service-assurance-status` Structural ID: Unresolved	Service: Request Service Assurance Status (p. 448)	service / Canonical
`HCM-SV-secure-clinical-message-dispatch` Structural ID: Unresolved	Service: Secure Clinical Message Dispatch (p. 269)	service / Canonical
`HCM-SV-security-audit-ingress` Structural ID: Unresolved	Service: Security Audit Ingress (p. 404)	service / Canonical
`HCM-SV-security-context-validation-service` Structural ID: Unresolved	Service: Security Context Validation Service (p. 403)	service / Canonical
`HCM-SV-semantic-conformance-verification-service` Structural ID: Unresolved	Service: Semantic Conformance Verification Service (p. 438)	service / Canonical
`HCM-SV-semantic-model-definition-service` Structural ID: Unresolved	Service: Semantic Model Definition Service (p. 435)	service / Canonical
`HCM-SV-service-availability-query` Structural ID: Unresolved	Service: Service Availability Query (p. 184)	service / Canonical
`HCM-SV-service-eligibility-evaluation` Structural ID: Unresolved	Service: Service Eligibility Evaluation (p. 184)	service / Canonical
`HCM-SV-service-provision-resolution` Structural ID: Unresolved	Service: Service Provision Resolution (p. 183)	service / Canonical
`HCM-SV-specimen-transit-tracking-service` Structural ID: Unresolved	Service: Specimen Transit Tracking Service (p. 370)	service / Canonical
`HCM-SV-staff-presence-telemetry-query` Structural ID: Unresolved	Service: Staff Presence Telemetry Query (p. 350)	service / Canonical

Reference Id / Structural Canonical ID	Element / page	Type / status
`HCM-SV-standards-communication-gateway-service` Structural ID: Unresolved	Service: Standards Communication Gateway Service (p. 397)	service / Canonical
`HCM-SV-synthetic-task-execution-service` Structural ID: Unresolved	Service: Synthetic Task Execution Service (p. 422)	service / Canonical
`HCM-SV-temporal-context-resolution` Structural ID: Unresolved	Service: Temporal Context Resolution (p. 429)	service / Canonical
`HCM-SV-terminology-binding-specification-lookup` Structural ID: Unresolved	Service: Terminology Binding Specification Lookup (p. 437)	service / Canonical
`HCM-SV-terminology-concept-resolution-service` Structural ID: Unresolved	Service: Terminology Concept Resolution Service (p. 407)	service / Canonical
`HCM-SV-terminology-mapping-service` Structural ID: Unresolved	Service: Terminology Mapping Service (p. 408)	service / Canonical
`HCM-SV-theatre-case-progression-telemetry` Structural ID: Unresolved	Service: Theatre Case Progression Telemetry (p. 327)	service / Canonical
`HCM-SV-to-do-management-service` Structural ID: Unresolved	Service: To Do Management Service (p. 421)	service / Canonical
`HCM-SV-unified-calendar-projection-query` Structural ID: Unresolved	Service: Unified Calendar Projection Query (p. 429)	service / Canonical
`HCM-SV-virtual-encounter-context-resolution` Structural ID: Unresolved	Service: Virtual Encounter Context Resolution (p. 314)	service / Canonical
`HCM-SV-ward-operational-summary-query` Structural ID: Unresolved	Service: Ward Operational Summary Query (p. 323)	service / Canonical
`HCM-SV-ward-round-clinical-briefing-query` Structural ID: Unresolved	Service: Ward Round Clinical Briefing Query (p. 288)	service / Canonical
`HCM-SV-work-dispatch-service` Structural ID: Unresolved	Service: Work Dispatch Service (p. 358)	service / Canonical
`HCM-SV-work-order-coordination-service` Structural ID: Unresolved	Service: Work Order Coordination Service (p. 421)	service / Canonical

8. Description limits and identifier exceptions

All 283 Capability records, including three Candidates, have a Capability Overview. The named sharing-programme concern has insufficient established scope for a full architectural account; its entry explains that limit. The machine-readable `review-exceptions.json` lists every affected element by its stable Reference Id, name and label.

Structural IDs are allocated to 190 canonical Business Capabilities and the fourteen existing Enterprise Capability IDs are retained. The 512 canonical BEC/Feature/behaviour records require unresolved BEC ancestry; three Candidate concerns also lack approved placement. Their publication identities remain usable. No absent tier is omitted or invented to obtain a structural path.

8.1. Material semantic constraints

Element	Constraint preserved for review
CT2: Clinical Determination (p. 8)	No CT3 set is adjudicated; examples are not promoted to children.
CT2: Individual Care Planning (p. 9)	No CT3 set is adjudicated; examples are not promoted to children.
CT2: Clinical Intervention (p. 10)	DN-BC-010 retains the intervention candidate as provisional pending a coherent peer taxonomy.
CT1: Health Service Governance (p. 140)	No Business Enabling derivation is established for Health Service Governance.
CT2: Medication & Pharmacy Services (p. 38)	OI-BC-001: a compounding/preparation CT3 is neither established nor rejected; do not infer subsumption.
CT2: Health Information Exchange (p. 96)	The same canonical name identifies distinct BC and BEC responsibilities. Global naming and the BC/BEC naming boundary conflict remains unresolved; capture does not merge identities or waive the rule.
Candidate BC: Health Service Operational Readiness (p. 151)	OI-BC-002: collective service-readiness meaning is identified, but tier/CT2 parent/CT1 ancestry and placement remain unresolved.
Candidate BC: Clinical Device Maintenance Management (p. 152)	DN-BC-015 defines maintenance separately but omits it from its explicit seven-child CT3 set; inclusion/tier/ancestry requires adjudication.
Candidate BC: Health Information Sharing Programme Management (p. 153)	DN-BC-027 establishes spelling of this named working concern, not a precise tier/parent or definition.
BEC: Health Information Exchange (p. 383)	The same canonical name identifies distinct BC and BEC responsibilities. Global naming and the BC/BEC naming boundary conflict remains unresolved; capture does not merge identities or waive the rule.
BEC: Credential Management (p. 376)	Domain03 establishes this Capability; Strategy contains no matching entry. No equivalence is inferred.
Function: Associate Order Outcome (p. 239)	No Feature association asserted; preserve owning Capability and do not infer an edge.
Service: Order Outcome Notification (p. 239)	No Feature association asserted; preserve owning Capability and do not infer an edge.
Function: Associate Care Plan Context (p. 264)	No Feature association asserted; preserve owning Capability and do not infer an edge.
Service: Care Plan Context Query (p. 264)	No Feature association asserted; preserve owning Capability and do not infer an edge.

Element	Constraint preserved for review
Function: Distribute Care Plan Change (p. 264)	No Feature association asserted; preserve owning Capability and do not infer an edge.
Service: Care Plan Change Notification (p. 265)	No Feature association asserted; preserve owning Capability and do not infer an edge.
Function: Coordinate Multi-Agency Chronic Care Plan (p. 276)	No Feature association asserted; preserve owning Capability and do not infer an edge.
Service: Chronic Care Timeline Query (p. 276)	No Feature association asserted; preserve owning Capability and do not infer an edge.
Function: Route Acute Critical Result Alert (p. 280)	No Feature association asserted; preserve owning Capability and do not infer an edge.
Service: Acute Clinical Alert Dispatch (p. 280)	No Feature association asserted; preserve owning Capability and do not infer an edge.
Function: Assemble Inpatient Discharge Dossier (p. 288)	No Feature association asserted; preserve owning Capability and do not infer an edge.
Service: Discharge Dossier Packaging Service (p. 288)	No Feature association asserted; preserve owning Capability and do not infer an edge.
Function: Receive Diagnostic Requisition Feed (p. 292)	No Feature association asserted; preserve owning Capability and do not infer an edge.
Service: Modality Worklist Distribution (p. 293)	No Feature association asserted; preserve owning Capability and do not infer an edge.
Function: Publish Governed Diagnostic Report (p. 293)	No Feature association asserted; preserve owning Capability and do not infer an edge.
Service: Diagnostic Publication Service (p. 294)	No Feature association asserted; preserve owning Capability and do not infer an edge.
Function: Correlate Adverse Reaction & Allergy Profile (p. 298)	No Feature association asserted; preserve owning Capability and do not infer an edge.
Service: Allergy & Adverse Reaction Query (p. 298)	No Feature association asserted; preserve owning Capability and do not infer an edge.
Function: Project Screening Eligibility Timeline (p. 302)	No Feature association asserted; preserve owning Capability and do not infer an edge.
Service: Preventive Screening Status Query (p. 303)	No Feature association asserted; preserve owning Capability and do not infer an edge.
Function: Bind Community Support Network Context (p. 307)	No Feature association asserted; preserve owning Capability and do not infer an edge.
Service: Community Care Team Query (p. 307)	No Feature association asserted; preserve owning Capability and do not infer an edge.
Function: Manage Clinic & Practice Capacity (p. 318)	No Feature association asserted; preserve owning Capability and do not infer an edge.
Service: Ward Operational Summary Query (p. 323)	No Feature association asserted; preserve owning Capability and do not infer an edge.
Function: Manage Theatre Capacity (p. 326)	No Feature association asserted; preserve owning Capability and do not infer an edge.
Service: ED Operational Status Query (p. 331)	No Feature association asserted; preserve owning Capability and do not infer an edge.
Service: Ambulatory Session Status Query (p. 334)	No Feature association asserted; preserve owning Capability and do not infer an edge. Named Service is established but outside consumer/exposure mandate is unestablished.
Function: Manage Mobile Service Capacity (p. 349)	No Feature association asserted; preserve owning Capability and do not infer an edge.
Function: Track Operational Staff Presence (p. 349)	No Feature association asserted; preserve owning Capability and do not infer an edge.
Service: Staff Presence Telemetry Query (p. 350)	No Feature association asserted; preserve owning Capability and do not infer an edge.
Function: Verify Operational Access Credential (p. 377)	No Feature association asserted; preserve owning Capability and do not infer an edge.
Service: Operational Credential Checkpoint Service (p. 377)	No Feature association asserted; preserve owning Capability and do not infer an edge.

Element	Constraint preserved for review
Function: Retrieve Health Information (p. 394)	No Feature association asserted; preserve owning Capability and do not infer an edge.
Service: Clinical Resource Retrieval Service (p. 394)	No Feature association asserted; preserve owning Capability and do not infer an edge.
Function: Apply Access-Qualified Result Filtering (p. 395)	No Feature association asserted; preserve owning Capability and do not infer an edge.
Function: Evaluate Consent Constraint (p. 401)	No Feature association asserted; preserve owning Capability and do not infer an edge.
Service: Consent Enforcement Service (p. 402)	No Feature association asserted; preserve owning Capability and do not infer an edge.
Function: Propagate Security Context (p. 402)	No Feature association asserted; preserve owning Capability and do not infer an edge.
Service: Security Context Validation Service (p. 403)	No Feature association asserted; preserve owning Capability and do not infer an edge. Precise internal Service-to-Function mapping is unestablished.
Function: Coordinate Care-Team Collaboration (p. 411)	An existing description appears to conflict with a governing responsibility boundary; preserved for architectural review under AX-17 without reinterpretation.
Function: Govern Information Definition (p. 435)	No Feature association asserted; preserve owning Capability and do not infer an edge.
Service: Information Dictionary Query (p. 436)	No Feature association asserted; preserve owning Capability and do not infer an edge.
Function: Govern Terminology Binding (p. 436)	No Feature association asserted; preserve owning Capability and do not infer an edge.
Service: Terminology Binding Specification Lookup (p. 437)	No Feature association asserted; preserve owning Capability and do not infer an edge.
Function: Govern Exchange Profile (p. 437)	No Feature association asserted; preserve owning Capability and do not infer an edge.
Service: Exchange Profile Specification Query (p. 437)	No Feature association asserted; preserve owning Capability and do not infer an edge.
Function: Design Service Assurance (p. 440)	No Feature association asserted; preserve owning Capability and do not infer an edge.
Function: Manage Assurance Criteria (p. 443)	No Feature association asserted; preserve owning Capability and do not infer an edge.
Function: Establish Assurance Context (p. 445)	No Feature association asserted; preserve owning Capability and do not infer an edge.
Function: Assess Assurance Evidence (p. 446)	No Feature association asserted; preserve owning Capability and do not infer an edge.
Function: Adjudicate Assurance Assessment (p. 446)	No Feature association asserted; preserve owning Capability and do not infer an edge.
Service: Request Service Assurance (p. 447)	No Feature association asserted; preserve owning Capability and do not infer an edge. Precise internal Service-to-Function mapping is unestablished.
Service: Request Service Assurance Status (p. 448)	No Feature association asserted; preserve owning Capability and do not infer an edge. Precise internal Service-to-Function mapping is unestablished.
Service: Communicate Service Assurance Outcome (p. 449)	No Feature association asserted; preserve owning Capability and do not infer an edge. Precise internal Service-to-Function mapping is unestablished.
Process: Governed Person Identity Correction Process (p. 199)	No Feature association asserted; preserve owning Capability and do not infer an edge.
Process: Practitioner Verification Process (p. 165)	No Feature association asserted; preserve owning Capability and do not infer an edge.
Process: Referral Progression Process (p. 221)	No Feature association asserted; preserve owning Capability and do not infer an edge.
Process: Encounter Lifecycle Process (p. 231)	No Feature association asserted; preserve owning Capability and do not infer an edge. Strategy ON_LEAVE applicability and transitions are unresolved.

Element	Constraint preserved for review
Process: Closed-Loop Order Progression Process (p. 240)	No Feature association asserted; preserve owning Capability and do not infer an edge.
Process: Clinical Document Lifecycle Process (p. 260)	No Feature association asserted; preserve owning Capability and do not infer an edge.
Process: Theatre Case Progression Process (p. 328)	No Feature association asserted; preserve owning Capability and do not infer an edge.
Process: Clinic / Operational Queue Progression Process (p. 320)	No Feature association asserted; preserve owning Capability and do not infer an edge.
Process: Bed Turnover Process (p. 340)	No Feature association asserted; preserve owning Capability and do not infer an edge.
Process: Operational Work Progression Process (p. 359)	No Feature association asserted; preserve owning Capability and do not infer an edge.
Process: Patient Transport Process (p. 367)	No Feature association asserted; preserve owning Capability and do not infer an edge.
Process: Specimen Transport Process (p. 370)	No Feature association asserted; preserve owning Capability and do not infer an edge.
Process: Discharge Progression Process (p. 374)	No Feature association asserted; preserve owning Capability and do not infer an edge.
Process: Work Order Progression Process (p. 423)	No Feature association asserted; preserve owning Capability and do not infer an edge.
Process: To Do Progression Process (p. 424)	No Feature association asserted; preserve owning Capability and do not infer an edge.
Process: Synthetic Task Progression Process (p. 425)	No Feature association asserted; preserve owning Capability and do not infer an edge. Stalled/failed outcome position and transitions are unresolved.
Process: Assurance Process Design (p. 441)	No Feature association asserted; preserve owning Capability and do not infer an edge.
Process: Assurance Process Execution (p. 450)	No Feature association asserted; preserve owning Capability and do not infer an edge.
Process: Assurance Process Reporting / Communication (p. 451)	No Feature association asserted; preserve owning Capability and do not infer an edge.

Part I.

Governed RAD semantic detail

9. Canonical Business Capability Model (18 CT1 Domains)

RAD source: `docs/markdown/02-strategy/capabilities/business-capabilities.md`

The following is a presentation transcription of the governed source. Source catalogue layout and retained illustrations do not allocate tiers or establish additional identities. Equations wrap as text; diagram notation is textual.

Canonical Business Capability Model (18 CT1 Domains)

Overview & Purpose

Business Capabilities describe **what the health enterprise/business must be capable of doing**. They are organisational abilities independent of technology, products, departments, professions or automation. This catalogue is the canonical R1/R2 reconciliation of the adjudicated Business Capability decisions recorded in DN-BC-001 through DN-BC-037. The scratchpad retains working reasoning only; the definitions, explicit tier/parent assertions and boundaries here govern the reconciled model. Provisional and unresolved matters are not promoted as approved hierarchy.

Business Enabling Capabilities describe **reusable enabling abilities Harmonia provides or supports**. The two hierarchies are distinct:

RAD source listing (presentation only; no inferred ancestry).

> Business Capability: CT1 → CT2 → CT3
>
> enabled-by / supported-by
>
> Business Enabling Capability: CT1 → CT2 → CT3 → Feature

The relationship is enablement/support, never decomposition or implementation equivalence. **Business Capabilities do not decompose into Harmonia Features.** Features are ultimately framed by Business Enabling CT3s. This catalogue establishes no new BC↔BEC mapping and does not reframe existing Features. Previously approved B2.1 and assurance support relationships retain their bounded authority without being extended to the new BC descendants.

Technology-sounding names such as Health Information Exchange, Digital Communication Channel, Identity Assurance and Access Control Enforcement describe organisational abilities. Physical, procedural and digital mechanisms illustrate exercise/enablement, not software-component identities. An interpretation requiring a particular software product to explain a BC is suspect. Where BC and BEC concern the same subject, their names must distinguish organisational ability from system enablement where reuse would imply equivalence or ownership. The existing Exchange name collision is explicitly unresolved below.

R1/R2 Scope and Decomposition Rules

> Capability decomposition SHALL be sufficient to establish Harmonia's architectural context and derivation requirements. It SHALL NOT pursue exhaustive decomposition of healthcare-enterprise capabilities where additional detail does not materially affect Harmonia responsibility, information semantics, interoperability, security, workflow enablement, traceability or another Harmonia architectural concern.
>
> Scope may determine decomposition depth. Scope must not distort decomposition semantics.

The stopping test asks whether distinguishing another ability would materially change how Harmonia behaves, governs, represents, exchanges, secures, coordinates or traces something. Once a branch is decomposed, its materially distinct peers must be coherent from the business perspective, independently of Harmonia's implementation interest. This does not require identical depth, a Feature for every peer or a complete healthcare-enterprise taxonomy. An undecomposed CT1/CT2 does not authorise a skipped tier or an invented CT3.

The metamodel governs tier meaning. Explicit CT1/CT2/CT3 and parent statements below establish the stated BC hierarchy; display ordinals, indentation and regional membership alone do not. CT4 and deeper tiers are outside scope. No ordinal is an allocated local code or Canonical ID. BEC tiers/ancestry remain separately unresolved under K9. Absent decomposition does not deny real-world abilities or assign them to an existing node.

Capability Modelling Quality Rules

Rule	Requirement
CM-R01 Coverage	Preserve the eighteen broad enterprise domains and five regions; decomposition is bounded by R1/R2 semantic sufficiency rather than exhaustive enterprise detail.
CM-R02 Orthogonality	Preserve distinct responsibilities, outcomes and authority even where capabilities share evidence, context or implementation machinery.
CM-R03 Semantic Clarity	Use clear business meanings; examples do not redefine capabilities as technology.
CM-R04 No Junk Drawers	Keep cohesive purposes; do not use residual categories to hide materially distinct abilities.
CM-R05 Global Name Uniqueness	Capability names are globally unique across the architecture. The unresolved BC/BEC Exchange collision is reported; neither identity is merged or renamed here.

Capability decomposition and educational documentation

Decomposition follows architectural distinction, not explanatory detail. Definition, state change, boundary, illustrative activities, care contexts and exclusions explain capabilities without establishing more children. History-taking, diagnosis, prescribing, medication administration, surgery, therapy and counselling are examples unless an explicit adjudication establishes a Capability identity.

B2-R05 — Capability Is Independent of Care Delivery Context

Setting, delivery mode, temporal pattern and organisational arrangement do not by themselves establish separate Business Capabilities.

B2-R06 — Context Independence Test

If changing setting, mode or time context does not materially change the business ability, it remains the same Capability. Acute, Planned/Scheduled and Longitudinal Care are not Individual Care Delivery's CT2 decomposition. Inpatient, ambulatory, community, home, virtual, emergency and elective remain illustrative contexts.

Harmonia Relevance Taxonomy

Classification	Meaning
Reference	Enterprise context without material effect on Harmonia responsibilities or processing obligations.
Adjacent	Harmonia consumes, represents or supplies contextual information without material participation in execution or coordination.
Harmonia-Relevant	Harmonia materially enables, connects, routes, coordinates or supplies information used in performing the capability.
Harmonia-Core	Harmonia provides substantive direct platform responsibility for the capability within its assigned boundary.

An established classification has exactly one of these values. Unresolved is a status, not a fifth value. CT1 relevance describes material participation across its bounded scope; it does not classify every descendant identically or confer enterprise-domain authority. Descendant relevance remains unallocated unless explicitly established. Contextual resource/state consumption alone warrants neither Core nor automatic Relevant classification. AX-18 preserves originating and domain authority. A Harmonia-Core enabling responsibility may support a Harmonia-Relevant BC without Harmonia owning that business activity.

The 18 CT1 Business Capability Domains

Five natural regions group the eighteen CT1s. Regions are contextual views, not capability ancestors, organisational boundaries or application ownership. They are distinct from the five BEC contextual views. All eighteen named catalogue domains below are explicitly **CT1 roots**; each branch statement explicitly establishes its CT2 parent and each table its CT3 parent.

Catalogue ordinal	CT1 Business Capability	Region	Relevance
01	Individual Care Delivery	Care & Health Delivery	Harmonia-Relevant
02	Care Access & Coordination	Care & Health Delivery	Harmonia-Relevant
03	Health Rights, Advocacy & Participation	Care & Health Delivery	Adjacent
04	Diagnostic, Therapeutic & Clinical Support Services	Care & Health Delivery	Harmonia-Relevant
05	Health Products & Clinical Technology	Care & Health Delivery	Harmonia-Relevant
06	Clinical Quality, Safety & Improvement	Care & Health Delivery	Harmonia-Relevant
07	Community Health & Wellbeing	Care & Health Delivery	Harmonia-Relevant
08	Population Health & Health-System Planning	Care & Health Delivery	Harmonia-Relevant
09	Health Information & Knowledge Management	Health Information & Digital Health	Harmonia-Core
10	Standards, Semantics & Reference Governance	Health Information & Digital Health	Harmonia-Core
11	Connected Health Services	Health Information & Digital Health	Harmonia-Core
12	Security, Privacy & Digital Trust	Health Information & Digital Health	Harmonia-Core
13	Health Research & Innovation	Research & Innovation	Harmonia-Relevant
14	Health Service Direction & Stewardship	Health Service Management	Harmonia-Relevant

Catalogue ordinal	CT1 Business Capability	Region	Relevance
15	Workforce & Organisational Capability	Health Service Management	Harmonia-Relevant
16	Corporate Resources & Enterprise Services	Health Service Management	Harmonia-Relevant
17	Health Service Governance	Governance & Assurance	Unresolved
18	Health Service Assurance	Governance & Assurance	Harmonia-Relevant

Region 1: Care & Health Delivery

01. Individual Care Delivery

Tier and ancestry: CT1 root.

Definition: Assess, clinically determine, plan, intervene, monitor and review individual care independently of setting, mode, time pattern or organisational arrangement.

Classification: Harmonia-Relevant.

The six CT2s are Individual Health Assessment, Clinical Determination, Individual Care Planning, Clinical Intervention, Individual Health Monitoring and Individual Care Review. Their semantic state changes do not prescribe execution order.

01.01 Individual Health Assessment

Tier and ancestry: CT2; parent Individual Care Delivery (CT1).

Definition: The capability to establish an understanding of an individual's current health state through the collection, consideration and evaluation of relevant clinical, health and contextual information.

State Change: `Healthinformation→Assessedhealthstate/findings`

Boundary: Assessment establishes the assessed health state or findings. Existing records can contribute, while direct examination, observation, history-taking or measurement may establish information independently of Health Information Access.

Illustrative Activities: History taking, physical examination, consideration of available clinical records and measurement of current observations.

Example Care Contexts: An emergency presentation, a scheduled consultation or a home visit may exercise the same assessment ability.

Explicit Exclusions: Establishing clinical conclusions, selecting the care response and performing an intervention are not the assessment itself.

01.02 Clinical Determination

Tier and ancestry: CT2; parent Individual Care Delivery (CT1).

Definition: The capability to interpret assessed health information and establish clinically meaningful conclusions about an individual's health state, condition, risk or prognosis.

State Change: `Assessedfindings→Clinicallyinterpretedunderstanding`

Boundary: Clinical Determination interprets findings and establishes their clinical meaning, including the clinical significance of change identified by monitoring.

Illustrative Activities: Diagnosis, clinical risk interpretation and prognostic judgement.

Example Care Contexts: Interpretation during acute care or a longitudinal follow-up consultation remains the same business ability.

Explicit Exclusions: Clinical Determination does not include establishing care needs or selecting the response to the determination.

01.03 Individual Care Planning

Tier and ancestry: CT2; parent Individual Care Delivery (CT1).

Definition: The capability to establish the intended clinical response to an individual's assessed and clinically interpreted health state, including the objectives, interventions and monitoring required to progress their care.

State Change: `Clinicallyinterpretedunderstanding→Intendedcourseofcare`

Boundary: Planning establishes what is intended to happen. It does not imply that the planned care has occurred.

Illustrative Activities: Establishing care objectives, selecting intended therapy, prescribing as an expression of clinical intent and planning monitoring.

Example Care Contexts: An intended course of care may be established during an inpatient encounter, an ambulatory consultation or a virtual consultation.

Explicit Exclusions: Performing planned care is Clinical Intervention. Ordering, requesting, scheduling and allocating care do not themselves establish that an intervention has occurred. Operational service/resource availability information is not Health Information Access merely because it informs planning.

01.04 Clinical Intervention

Tier and ancestry: CT2; parent Individual Care Delivery (CT1).

Definition: The capability to perform clinical actions intended to alter, maintain or otherwise manage an individual's health state in accordance with an established care intent.

State Change: `Intendedcare→Careactionperformed`

Boundary: Clinical Intervention establishes that a care action was performed. An intervention need not produce a successful clinical outcome. Access to an intended intervention does not confer authority to perform it.

Illustrative Activities: Medication administration, surgery, therapy and counselling when performed as clinical actions under an established care intent.

Example Care Contexts: An intervention may be performed in inpatient, community or home care, or through a suitable virtual delivery mode.

Explicit Exclusions: Ordering, requesting, scheduling or allocating an intervention is not itself the Clinical Intervention; selecting the intended response belongs to planning.

01.05 Individual Health Monitoring

Tier and ancestry: CT2; parent Individual Care Delivery (CT1).

Definition: The capability to observe and evaluate an individual's health state over time in order to identify clinically meaningful change, stability or deviation from an expected state or trajectory.

State Change: `Observedhealthstatesovertime→Change/stability/deviationidentified`

Boundary: Monitoring establishes change, stability or deviation. Interpretation of the clinical significance of that change remains Clinical Determination.

Illustrative Activities: Comparing serial observations with a baseline or expected trajectory and identifying a trend or deviation.

Example Care Contexts: Serial inpatient observations and longitudinal home monitoring exercise the same ability across different time patterns and settings.

Explicit Exclusions: Clinical interpretation of an identified change and selection of the care response are not monitoring. Evaluating care effectiveness against its objectives belongs to Individual Care Review.

01.06 Individual Care Review

Tier and ancestry: CT2; parent Individual Care Delivery (CT1).

Definition: The capability to evaluate the progress and effectiveness of an individual's care against its intended objectives and establish the continuing appropriateness of the current care intent.

State Change: `Careprogress/outcomes→Effectivenessandcontinuingappropriatenessestablished`

Boundary: Care Review evaluates the care itself against its intent and objectives; it is not simply another name for reassessment.

Illustrative Activities: Reviewing response to delivered therapy against care objectives and evaluating whether the current care intent remains appropriate.

Example Care Contexts: Review during an admission, a scheduled follow-up or longitudinal care retains the same evaluation purpose.

Explicit Exclusions: Reassessment alone does not establish care effectiveness. Establishing a revised intended course of care belongs to Individual Care Planning.

Clinical Determination and Individual Care Planning have no adjudicated CT3 set in the supplied decisions. Clinical Intervention's Therapy & Counselling Intervention / Medication Intervention / Procedural Intervention candidate remains provisional under DN-BC-010; it is not promoted as a final taxonomy. Examples cannot close these gaps.

The previously approved B2.1 support trace is preserved without extending it to CT3s or establishing runtime dependencies.

CT2 — Individual Health Assessment

Tier and ancestry: CT2; parent Individual Care Delivery (CT1).

Definition: Establish an understanding of an individual's current health state.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Individual Health Assessment; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Health History Assessment	Establish relevant health knowledge from the individual's history and applicable historical/contextual information.
Clinical Examination	Establish health findings through clinical examination.
Health Status Measurement	Establish individual health observations or measurements.

Boundary: The decomposition distinguishes means of establishing knowledge. Functional, nutritional, cognitive, mental-health and similar labels are assessment domains, not automatically peer abilities. Risk interpretation belongs to Clinical Determination. Care setting, profession, specialty and information type do not define the decomposition.

CT2 — Individual Health Monitoring

Tier and ancestry: CT2; parent Individual Care Delivery (CT1).

Definition: Observe health over time to identify change, stability or deviation.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Individual Health Monitoring; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Health State Monitoring	Establish an aggregate, temporally situated view of the individual's health from applicable monitored data.
Care Response Monitoring	Scope that health-state view to a particular problem/condition and the treatment applied to it.

Boundary: Care Response Monitoring consumes health-state services provided through Health State Monitoring without independently recreating the aggregate view. This semantic consumption establishes no named Service, interface or runtime invocation. Measurement supplies observations; monitoring establishes a view; Clinical Determination interprets its clinical significance. Deterioration, improvement and stability are possible monitored states, not established peer CT3s.

CT2 — Individual Care Review

Tier and ancestry: CT2; parent Individual Care Delivery (CT1).

Definition: Evaluate care against intent and objectives.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Individual Care Review; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Care Appropriateness Review	Evaluate whether the current intended care remains suitable in the individual's current context.
Care Effectiveness Review	Evaluate whether delivered care is achieving or has achieved its intended goals and outcomes.

Boundary: An effectiveness finding may inform appropriateness without replacing it. Care may be appropriate yet ineffective, or effective yet no longer appropriate to continue. Monitoring the response is distinct from judging effectiveness; identifying a need to reconsider care does not establish the replacement plan. Replanning belongs to Individual Care Planning.

02. Care Access & Coordination

Tier and ancestry: CT1 root.

Definition: Managing patient intake, referral triage, appointment scheduling, multidisciplinary care team coordination, cross-facility patient transfers, and care continuity across organisational boundaries.

Classification: Harmonia-Relevant.

CT2 — Care Access Management

Tier and ancestry: CT2; parent Care Access & Coordination (CT1).

Definition: Establish and manage access to applicable care.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Care Access Management; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Care Intake Management	Establish and manage the intake context through which an individual seeks or enters care.

CT3 Business Capability	Definition
Care Referral Management	Manage a request for another participant/service to assess, manage or assume applicable care responsibility.
Care Eligibility Determination	Determine whether applicable eligibility conditions for care access are satisfied.

Boundary: Intake, referral and eligibility are distinct abilities. Referral submission or receipt does not establish acceptance, care delivery or responsibility actually assumed; eligibility does not itself allocate or deliver care.

CT2 — Care Scheduling & Capacity Management

Tier and ancestry: CT2; parent Care Access & Coordination (CT1).

Definition: Manage scheduled opportunities, care places, usable operational capacity and care-demand resourcing.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Care Scheduling & Capacity Management; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Care Appointment Management	Establish and manage a scheduled opportunity for care.
Care-Place Allocation	Allocate an applicable care place to a care requirement.
Care-Place Readiness Management	Establish and manage whether a particular care place is ready for allocation or use.
Care Resource Deployment	Apply legitimately available and appropriately authorised resources to a current or planned operational care requirement, particularly using roster/on-call and service-coverage context.
Operational Care Flow Coordination	Coordinate operational care flow and its progression within the care-delivery environment.
Operational Care Capacity Management	Understand and manage the service's usable capacity to accommodate care in its current operational context.

Boundary: Available staff, equipment or an unoccupied bed does not establish usable service capacity. Available for service coverage ≠ deployed to a care requirement ≠ assigned operational work. Care Resource Deployment consumes availability, role, qualification, engagement and privilege context without changing authoritative rosters, qualifications, employment or privilege records. It establishes no Harmonia authority over clinical work, enterprise assets or hospital-wide logistics. Operational capacity is distinct from strategic/tactical capacity planning and from collective service readiness (OI-BC-002).

CT2 — Care Coordination & Transition

Tier and ancestry: CT2; parent Care Access & Coordination (CT1).

Definition: Coordinate care participation, actual responsibility transition and discharge.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Care Coordination & Transition; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Care-Team Coordination	Coordinate participating care-team context and collaboration around care.
Care Responsibility Transition	Establish and coordinate who has actually assumed applicable care responsibility.
Care Discharge Coordination	Coordinate applicable discharge requirements and progression.

Boundary: Patient moved ≠ care responsibility transferred. Arrival, referral submission, message receipt, discharge readiness or physical departure does not itself establish acceptance

of responsibility. Information assembly for clinical handover does not confer clinical-handover management authority on Harmonia.

CT2 — Care Support Logistics Management

Tier and ancestry: CT2; parent Care Access & Coordination (CT1).

Definition: Coordinate bounded operational support needed for care access and fulfilment.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Care Support Logistics Management; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Patient Movement Coordination	Coordinate physical patient transit and applicable mobility constraints.
Clinical Material Logistics Coordination	Coordinate clinically relevant material movement, custody, availability or disposition needed for care or diagnostic fulfilment.
Care Support Work Coordination	Coordinate authorised operational support work: what needs doing, who performs it and how progression, completion or escalation is coordinated.

Boundary: These abilities do not establish a general hospital logistics, supply-chain, ERP, facilities or workforce-management architecture. Support work coordination does not determine clinical work; assignment/dispatch does not establish availability, qualification, privilege or clinical authority.

03. Health Rights, Advocacy & Participation

Tier and ancestry: CT1 root.

Definition: Protecting patient rights, facilitating informed consent, managing advance care directives, supporting patient advocacy, and enabling consumer participation in health decisions.

Classification: Adjacent.

No CT2/CT3 set is established by this reconciliation. Consent and record-access evidence can support rights without assigning advocacy or participation management to Harmonia.

04. Diagnostic, Therapeutic & Clinical Support Services

Tier and ancestry: CT1 root.

Definition: The specialised clinical services that support direct patient care, including pathology and laboratory testing, diagnostic imaging (radiology), pharmacy and medication dispensing, and allied health therapies.

Classification: Harmonia-Relevant.

The following diagnostic and pharmacy branches are established. Other therapeutic/support scope remains enterprise context; no further subordinate identity is manufactured.

CT2 — Diagnostic Support Provision

Tier and ancestry: CT2; parent Diagnostic, Therapeutic & Clinical Support Services (CT1).

Definition: Provide diagnostic services and manage their evidence, interpretation and governed reporting.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Diagnostic Support Provision; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Diagnostic Service Appointment Management	Manage the diagnostic-service-specific scheduled opportunity, including preparation, modality, urgency, duration and other service-specific constraints.
Diagnostic Service Delivery	Perform the diagnostic activity that obtains or produces diagnostic evidence.
Diagnostic Specimen Management	Maintain a specimen's identity, integrity, suitability, custody and applicable lifecycle context.
Diagnostic Data Management	Manage acquired or produced diagnostic evidence as information, including identity, association, integrity, organisation, availability and lifecycle.
Diagnostic Interpretation	Establish what diagnostic evidence demonstrates within the diagnostic service's professional scope.
Diagnostic Report Management	Establish and govern the authoritative expression of diagnostic findings, including applicable creation, review, amendment, verification, finalisation and authorisation/signing.
Diagnostic Report Distribution	Provide an established report to applicable authorised recipients or consuming contexts under applicable access, privacy, security and communication controls.

Boundary: Scheduling ≠ performance ≠ evidence management ≠ interpretation ≠ governed report ≠ distribution. Specimen and data responsibilities remain distinct. Diagnostic interpretation does not establish patient-level Clinical Determination from the total clinical context. Generic appointments are distinct from diagnostic-specific constraints. Diagnostic Order Management is not established here as a subordinate capability. Signing/finalisation are report behaviours, not additional CT3s. Services may exercise a subset, concurrently or iteratively; no mandatory diagnostic workflow is established.

CT2 — Medication & Pharmacy Services

Tier and ancestry: CT2; parent Diagnostic, Therapeutic & Clinical Support Services (CT1).

Definition: Provide professional medication/pharmacy services under applicable controls.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Medication & Pharmacy Services; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Prescription Medication Dispensing	Professionally prepare and provide medication according to an applicable authorised prescription/order and dispensing requirements.
Non-Prescription Medication Provision	Provide medication not requiring an applicable prescription, subject to professional, regulatory and safety controls.
Medication Advice	Provide professional medication-specific advice about appropriate use, precautions, interactions, administration, adherence or safe handling.
Medication Dose Administration Aid Provision	Prepare and provide organised medication doses supporting administration according to an already established regimen.

Boundary: Medication prescribed ≠ dispensed ≠ provided ≠ administered. Advice does not establish diagnosis, prescribing or authority to alter treatment. Dose aid provision does not establish the regimen; commercial packaging examples do not define the capability. These are pharmacy responsibilities, distinct from clinical intervention, and do not prescribe a medication sequence. Medication Advice's detailed scope remains subject to refinement. OI-BC-001 remains unresolved; compounding/preparation is neither established, rejected nor silently subsumed.

05. Health Products & Clinical Technology

Tier and ancestry: CT1 root.

Definition: Manage health products and clinical technology in the care environment, with distinct device inventory, functional/release, deployment, utilisation, integration and lifecycle responsibilities. Product procurement and supply context is retained without inferring further Health Product Management decomposition.

Classification: Harmonia-Relevant.

CT2 — Clinical Technology Management

Tier and ancestry: CT2; parent Health Products & Clinical Technology (CT1).

Definition: Manage clinical technology as a coherent business concern rather than only the device information Harmonia happens to use.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Clinical Technology Management; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Clinical Device Inventory Management	Establish what devices exist and their managed asset identity, specification, ownership/custody and inventory context.
Clinical Device Feature & Release Management	Manage functional/release state, including applicable software, firmware, configuration and enabled device characteristics.
Clinical Device Loan & Deployment Management	Manage device loan, issue, assignment, placement, return and deployment.
Clinical Device Utilisation Support	Enable and support otherwise appropriately authorised users to use the particular device safely and effectively.
Clinical Device Integration Provision	Establish and maintain the device's capability and configuration to participate in applicable information exchanges.
Clinical Device Data Integration	Acquire, exchange or convey device data through established integration capability.
Clinical Device Lifecycle Management	Govern introduction, continued service, review, replacement, withdrawal, decommissioning and retirement within the overall device estate.

Boundary: Inventory ≠ release state ≠ deployment ≠ user enablement ≠ use in care. Professional qualification/privilege does not establish device-specific enablement. Integration provisioned ≠ data integrated; neither confers clinical interpretation or downstream information authority. Device monitoring remains externally provided enablement, with provider unestablished. Monitoring, maintenance and lifecycle disposition have distinct meanings. The Maintenance membership discrepancy is preserved under Unresolved Business Capability matters; no eighth CT3 is inferred.

06. Clinical Quality, Safety & Improvement

Tier and ancestry: CT1 root.

Definition: Operational clinical quality, safety and improvement: monitoring clinical outcomes; identifying quality and safety concerns; adverse event and incident management, reporting and response; infection control; operational quality and safety monitoring; continuous quality improvement; and coordination of operational responses to identified concerns.

Classification: Harmonia-Relevant.

CT2 — Clinical Safety Management

Tier and ancestry: CT2; parent Clinical Quality, Safety & Improvement (CT1).

Definition: Manage the clinical-risk distinctions materially required for R1/R2.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Clinical Safety Management; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Clinical Risk Identification	Recognise and characterise a clinical safety risk from applicable evidence.
Clinical Risk Mitigation Management	Manage prospective/preventative measures reducing identified risk likelihood, exposure or consequence.
Clinical Risk Response Management	Manage active response, containment and remediation where risk has materialised, is emerging or requires active intervention.

Boundary: Monitoring supplies evidence but does not itself identify/characterise clinical risk. Monitoring providers remain unestablished. Mitigation/response may require action by other capabilities without acquiring those capabilities' ownership. Response is not a generic enterprise incident-management container. The three CT3s are deliberately non-exhaustive specialist context; identified, mitigated and actively responded-to states are distinct, not a mandatory sequence.

CT2 — Clinical Quality Management

Tier and ancestry: CT2; parent Clinical Quality, Safety & Improvement (CT1).

Definition: Manage the clinical-quality distinctions materially required for R1/R2.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Clinical Quality Management; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Clinical Quality Criteria Management	Establish and manage measures, indicators, expectations and criteria against which clinical quality is considered.
Clinical Quality Evidence Management	Manage the body of applicable evidence used for clinical-quality consideration without owning its originating activity.
Clinical Quality Review & Assessment Management	Review evidence against applicable criteria and establish a clinical-quality assessment, finding or conclusion.
Clinical Quality Reporting	Communicate established clinical-quality measures, assessments, findings and conclusions to authorised consumers.

Boundary: Criteria ≠ evidence ≠ assessment ≠ reporting ≠ subsequent improvement. Observation alone is not quality judgement and reporting does not originate the reported conclusion. Clinical Quality Performance Monitoring is not established as a separate CT3; the provider of monitoring evidence remains unestablished. Continuous Improvement is not a subordinate CT3 here; generic placement remains unestablished. Health Service Continuous Improvement has the separately established service-wide purpose under 14. The four CT3s do not exhaust specialist Clinical Quality.

07. Community Health & Wellbeing

Tier and ancestry: CT1 root.

Definition: Support community health and wellbeing, including the bounded preventative-health abilities required for R1/R2. Wider environmental health, outreach and social determinants remain enterprise context without inferred additional decomposition.

Classification: Harmonia-Relevant.

CT2 — Preventative Health

Tier and ancestry: CT2; parent Community Health & Wellbeing (CT1).

Definition: Address preventative-health needs through organised activity with sufficient R1/R2 distinctions.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Preventative Health; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Preventative Health Problem Identification	Identify and characterise a preventative-health problem warranting organised attention.
Preventative Health Programme Management	Establish and manage an organised response to a preventative-health need/objective without absorbing its delivery capabilities.
Preventative Health Education Services	Provide education intended to prevent adverse health outcomes and influence preventative knowledge or behaviour.
Preventative Health Programme Monitoring	Observe programme operation, reach, participation, coverage, progress and applicable performance.
Preventative Health Reporting	Report established preventative-health problems, programmes, cohorts, activity, performance and outcomes.
Preventative Health Cohort Management	Establish and manage the target population, inclusion/exclusion criteria, membership, stratification and applicable entry/exit context.

Boundary: Screening and immunisation are programme types, not automatically separate CT3s. Population at risk ≠ defined cohort ≠ individual clinical problem. Programme management does not own clinical intervention, appointments or other delivery abilities. Education is purpose-specific rather than generic communication. Programme monitoring is distinct from population-health assessment, individual monitoring and assurance/review conclusions. This is deliberate specialist under-decomposition; unmodelled responsibilities are not assigned to existing CT3s.

08. Population Health & Health-System Planning

Tier and ancestry: CT1 root.

Definition: Understand population health through assessment/surveillance and applicable event/notification management; assess health-service demand and plan required capacity and its distribution.

Classification: Harmonia-Relevant.

CT2 — Population Health Surveillance

Tier and ancestry: CT2; parent Population Health & Health-System Planning (CT1).

Definition: Understand population-health state and manage organised surveillance, significant events and applicable notification.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Population Health Surveillance; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Population Health Longitudinal Assessment	Establish population-health state, patterns and change over time from applicable evidence.
Population Health Surveillance Management	Establish and manage surveillance purposes, populations, concerns and regimes.
Population Health Event Identification	Identify and characterise significant population-health events/patterns from applicable evidence.
Notifiable Health Event Management	Manage formal notification applicability, required information, recipient, status, submission, correction/supplementation and applicable follow-up.

CT3 Business Capability	Definition
Population Health Reporting	Communicate established population-health assessments, surveillance, events, trends and findings.

Boundary: Population state ≠ individual state; observed change ≠ identified event ≠ notifiability ≠ obligation discharged. Notification management neither defines the obligation nor owns operational response, and is distinct from general reporting. Surveillance may provide evidence for preventative problems/cohorts, demand planning, clinical quality or safety without establishing those outcomes. Population assessment differs from preventative-programme performance. Individual observations may contribute population evidence without becoming population conclusions. Epidemiology, outbreak response and public-health scope are deliberately not exhaustively modelled.

CT2 — Health-System Planning

Tier and ancestry: CT2; parent Population Health & Health-System Planning (CT1).

Definition: Assess health-service demand and plan the capacity and footprint required to meet it.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Health-System Planning; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Health Service Demand Assessment	Assess current/expected health-service demand across a population, geography, service domain or health-system context.
Health Service Capacity Planning	Determine required strategic/tactical capacity to address assessed demand.
Health Service Distribution Planning	Determine the distribution of planned capacity across a geographical, organisational, site or service-network footprint.

Boundary: Population-health evidence does not predetermine a demand conclusion. Individual care demand differs from health-system demand; required capacity differs from established or operationally usable capacity. Planning owns neither underlying resources nor operational allocation/deployment. Planned footprint differs from managed locations and identifiable offerings. Detailed commissioning, funding, capital, infrastructure and workforce planning remain intentionally unmodelled.

Region 2: Health Information & Digital Health

09. Health Information & Knowledge Management

Tier and ancestry: CT1 root.

Definition: Manage governed health-information content, structures, lifecycle, catalogues, compartments, correlation and retrieval, alongside reusable clinical knowledge and its governed standing.

Classification: Harmonia-Core.

CT2 — Health Information Management

Tier and ancestry: CT2; parent Health Information & Knowledge Management (CT1).

Definition: Manage health information as governed, semantically meaningful content.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Health Information Management; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Health Information Data Management	Manage actual health-information content within applicable governed structures.
Health Information Structure Management	Manage structures, element/reference definitions, constraints and semantic bindings giving content unambiguous meaning.
Health Information Lifecycle Management	Govern state, amendment, correction, supersession, currency, retention and disposition while preserving provenance and historical meaning.
Health Information Catalogue and Index Management	Manage descriptive catalogues/indexes through which information existence and location can be established.
Health Information Compartmentalisation	Manage contextual association, partitioning and bounded information groupings.
Health Information Data Correlation	Establish meaningful relationships between independently managed information items/sets.
Health Information Data Retrieval	Obtain identified information under an applicable authorised request/context.

Boundary: Semantic authority ≠ semantic binding ≠ content; a persisted value is not self-describing health information. Structure/binding management does not govern the referenced terminology/value set. Catalogue existence/location ≠ retrieval ≠ communication ≠ presentation; discoverability does not authorise access. Compartment membership may inform but does not determine authorisation. Correlation establishes neither clinical interpretation nor entity identity. Current state and immutable historical operational truth remain distinct; managing information does not originate its facts. Structures are not equated with FHIR resources, physical schemas or storage mechanisms.

CT2 — Clinical Knowledge Management

Tier and ancestry: CT2; parent Health Information & Knowledge Management (CT1).

Definition: Identify, extract, formulate, associate, govern and share reusable clinical knowledge.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Clinical Knowledge Management; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Clinical Knowledge Context Identification	Identify sources/contexts potentially containing reusable clinical knowledge.
Clinical Knowledge Context Extraction	Extract relevant material while preserving its source/context relationship.
Clinical Knowledge Information Formulation	Formulate explicit reusable knowledge, including applicability, qualifications, rationale and evidence relationships.
Clinical Knowledge Concept Association	Associate knowledge with applicable governed clinical concepts without acquiring concept authority.
Clinical Knowledge Review & Approval Management	Manage review, endorsement, approval/rejection, conditions and applicable history for intended governed use.
Clinical Knowledge Publication & Sharing	Make approved knowledge available to applicable authorised consumers and use contexts.

Boundary: Source identified ≠ extracted material ≠ formulated knowledge ≠ reviewed/approved knowledge ≠ publication ≠ individual application. Reusable knowledge differs from source health information and individual clinical facts. Formulation is distinct from Health Information Structure Management and does not establish approval. Concept association does not redefine terminology or semantic authority. Publication establishes neither individual applicability nor a clinical conclusion/decision. Clinical knowledge and source provenance/authority remain distinguishable; no mandatory knowledge workflow is prescribed.

10. Standards, Semantics & Reference Governance

Tier and ancestry: CT1 root.

Definition: Govern organisational adoption, oversight and appropriate use of health-information models, standards, terminology, classifications and semantic mappings.

Classification: Harmonia-Core.

CT2 — Health Information Standards Governance

Tier and ancestry: CT2; parent Standards, Semantics & Reference Governance (CT1).

Definition: Govern organisational adoption and use of health-information models and applicable standards.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Health Information Standards Governance; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Information Model Discovery	Identify information models relevant to the organisation's health-information needs.
Information Model Adoption	Adopt applicable information models, including intrinsic assessment of adoption impacts.
Information Model Education	Support organisational understanding and appropriate use of adopted information models.
Information Model Compliance	Address organisational conformity to applicable information-model requirements.
Information Model Governance Oversight	Oversee and evaluate effectiveness, consistency and appropriate operation of information-model governance.
Standards Adoption	Adopt applicable standards within the organisation.
Standards Impact Identification	Identify standards impacts across clinical/work practice, permitted medications, procedures, policy, safety, obligations and reporting as applicable.

Boundary: Oversight does not perform its discovery, adoption, education or compliance peers. Model-adoption impact is intrinsic to adoption; standards impacts can extend beyond information-system implementation and warrant their distinct established identity. These are the peers explicitly named in DN-BC-025; no additional standards peer is inferred to fill out a conventional taxonomy. Conceptual progression does not establish a workflow.

CT2 — Clinical Semantics Governance

Tier and ancestry: CT2; parent Standards, Semantics & Reference Governance (CT1).

Definition: Govern clinical meaning and appropriate organisational use of semantic artefacts.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Clinical Semantics Governance; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Terminology Governance	Manage governed terminology/reference-set lifecycle and standing, including acceptance, release, update, supersession/retirement and authority/provider coordination.
Terminology Utilisation Oversight	Oversee actual terminology/reference-set use and identify inappropriate, inconsistent or drifting usage.
Terminology Localisation	Manage controlled local responses where adopted terminology does not meet legitimate organisational semantic requirements.
Classification Governance	Govern applicable classification definitions and their organisational standing.

CT3 Business Capability	Definition
Classification Utilisation Oversight	Oversee appropriate organisational application of classifications.
Semantic Mapping Governance	Govern mappings between applicable semantic artefacts without collapsing their source meanings or authorities.

Boundary: Compartmentalisation concerns business/context relationships; classification concerns sensitivity and consequences of exposure/mishandling; terminology concerns consistent meaning. They are orthogonal even where metadata/mechanisms are shared. Classification Localisation is not established and SHALL NOT be invented for symmetry. Governance differs from structure binding and knowledge-to-concept association.

11. Connected Health Services

Tier and ancestry: CT1 root.

Definition: Exchange health information appropriately and provide digitally mediated communication, notifications and collaborative interaction between health-service participants.

Classification: Harmonia-Core.

CT2 — Health Information Exchange

Tier and ancestry: CT2; parent Connected Health Services (CT1).

Definition: Exchange health information appropriately between participating parties, systems and services.

No CT3 decomposition is established here.

Boundary: This organisational ability is not a FHIR server, interface engine, protocol or interoperability product. The established rename from Health Information Connectivity does not allocate additional CT3s. Health Information Sharing Programme Management is a named working capability in DN-BC-027 whose precise tier/parent is not supplied there; it is retained separately as unresolved. The collision with the existing BEC name remains for explicit adjudication; no BEC identity is merged or renamed.

CT2 — Digital Communication Channel

Tier and ancestry: CT2; parent Connected Health Services (CT1).

Definition: Provide digitally mediated communication, notification and collaborative interaction between patients, practitioners and other health-service participants.

No CT3 decomposition is established here.

Boundary: Appointment notifications, pharmacy queries, results and collaboration illustrate participant communication. This is not the general UI/presentation capability: exercising another capability through a UI does not itself exercise this communication ability. General presentation enablement remains parked for BEC reconciliation. No CT3 set is established by DN-BC-027.

12. Security, Privacy & Digital Trust

Tier and ancestry: CT1 root.

Definition: Establish adequate identity confidence, coherent access controls, privacy/protection safeguards, governed activity attribution and accountability for security-relevant activity and information use.

Classification: Harmonia-Core.

CT2 — Digital Identity & Access Governance

Tier and ancestry: CT2; parent Security, Privacy & Digital Trust (CT1).

Definition: Govern sufficient identity confidence and coherent access controls across the organisation.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Digital Identity & Access Governance; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Identity Assurance	Establish sufficient confidence that a person is who they claim to be through appropriate evidence and processes.
Identity Modelling & Interoperability	Establish identity models and interoperability arrangements appropriate to organisational identity use.
Identity Qualification Control	Determine whether presented identity characteristics, provenance and assurance quality are sufficient for its intended context.
Identity Identification & Authentication	Identify an actor and authenticate the asserted identity.
Access Control Context Management	Determine what information, functions, activities or other concerns require control, under what circumstances and how requirements are modelled.
Access Control Assurance & Reporting	Establish organisational assurance/evidence of whether access-control arrangements are appropriately established and operating.
Access Control Collaboration	Coordinate arrangements across independently controlled domains/parties while retaining their control ownership.
Access Control Enforcement	Ensure physical, procedural and digital control mechanisms coherently impose intended controls.

Boundary: Who are you? ≠ confidence in the claimed identity ≠ identity quality sufficient here. Federation and Single Sign-On are examples, not equivalent concepts or universal identity-quality guarantees. Context management is not merely runtime attribute assembly; enforcement is not a software authorisation engine. Strong local control does not imply end-to-end control. Identity Assurance is an organisational ability; the possible future BEC name Identity Assurance Representation remains unestablished.

CT2 — Health Information Protection

Tier and ancestry: CT2; parent Security, Privacy & Digital Trust (CT1).

Definition: Govern privacy consequences and adequate protection of the information estate and its environment.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Health Information Protection; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Health Privacy Safeguard Management	Establish organisational safeguards and assurance processes concerning privacy consequences of information supply, exposure or use.
Health Information Safeguarding	Ensure information and its operational environment are protected and resilient against loss, compromise, inappropriate access or operational failure.
Health Information Access Monitoring	Evaluate actual organisational access/handling practice, including controls, accessibility, compartments, classification and physical/printed handling.
Health Information Compartment Assurance	Assure appropriate establishment and application of compartmentalisation without establishing access decisions.

CT3 Business Capability	Definition
Health Information Classification Assurance	Assure appropriate classification according to sensitivity and potential consequences of exposure/mishandling.

Boundary: Privacy consequences differ from estate/environment protection. Storage, backups, disaster recovery, physical controls and testing illustrate organisational safeguards rather than define software components. Access Monitoring evaluates practice using IT, audit, physical-access or other evidence; it is not merely observation of software access events. Evidence supply does not transfer business accountability. These subject-specific protection abilities are not separate macro-level Health Service Assurance trees.

CT2 — Digital Accountability & Trust

Tier and ancestry: CT2; parent Security, Privacy & Digital Trust (CT1).

Definition: Establish governed attribution and accountability for activity and information use.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Digital Accountability & Trust; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Governed Activity Attribution	Preserve who participated, what was done, the attributable person/role/unit/organisation capacity and applicable permission/authority/constraint context.
Security & Information-Use Accountability	Evaluate whether attributed security-relevant activity/information use was appropriate within policy, obligation and authority constraints, and establish accountability.

Boundary: Actor identified ≠ activity attributed ≠ activity permitted. Attribution without permission/constraint context cannot establish entitlement and is richer than simple audit logging or provenance. Accountability evaluates appropriateness rather than merely recording participation. Expected consumption by governance/assurance or other enablement establishes no structural BC↔BEC relationship.

Region 3: Research & Innovation

13. Health Research & Innovation

Tier and ancestry: CT1 root.

Definition: Provide existing health information for authorised research under applicable constraints and evaluate health innovation; deliberately under-decomposed for R1/R2.

Classification: Harmonia-Relevant.

CT2 — Research Information Support

Tier and ancestry: CT2; parent Health Research & Innovation (CT1).

Definition: Provide the existing health information needed for authorised research within the deliberately bounded R1/R2 responsibility.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Research Information Support; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Governed Research Information Provision	Provide existing health information for an authorised research purpose under applicable ethics, governance, privacy, consent, security and information-use constraints.

Boundary: This is deliberate under-decomposition, not a claim that information provision exhausts research support. Research Assessment and Governance, Research/Trials Oversight, clinical-trial management, genomic analysis and other specialist research abilities are intentionally unmodelled unless Harmonia requirements make them material. Their absence does not assign them to information provision.

CT2 — Health Innovation

Tier and ancestry: CT2; parent Health Research & Innovation (CT1).

Definition: Consider health-service and clinical innovation within the R1/R2 business context.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Health Innovation; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Health Innovation Evaluation	Evaluate proposed/emerging innovation for suitability, value, implications and potential adoption.

Boundary: Evaluation does not establish adoption or the clinical/operational authority to implement innovation. The single CT3 is deliberately sufficient, not an exhaustive innovation lifecycle.

Region 4: Health Service Management

14. Health Service Direction & Stewardship

Tier and ancestry: CT1 root.

Definition: Model needed health services and their viability requirements, support establishment/improvement and align services with the wider ecosystem of dependencies, resources, financial arrangements and organisational change.

Classification: Harmonia-Relevant.

CT2 — Health Service Modelling

Tier and ancestry: CT2; parent Health Service Direction & Stewardship (CT1).

Definition: Establish what service should exist and what it needs to become viable.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Health Service Modelling; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Health Service Need Identification	Establish an unmet health-service need requiring response.
Health Service Capability Identification	Identify the ordinary business abilities needed to satisfy the service need across clinical, workforce, facility, technology, information and support concerns.
Health Service Implementation Support	Support establishment of the framed service through applicable organisational preparation and programmes of work.
Health Service Continuous Improvement	Evaluate the established service and identify correction/improvement needs that may feed back into modelling and implementation support.

Boundary: Capability Identification uses ordinary business meaning; it is not Harmonia CT-tier allocation. Implementation support may require recruitment, training, facility, equipment or technology change without absorbing HR, facilities, ICT or procurement responsibility. Identification, fulfilment requirements, support and review describe semantic relationships, not a prescribed workflow.

CT2 — Health Service Alignment

Tier and ancestry: CT2; parent Health Service Direction & Stewardship (CT1).

Definition: Position the service coherently in its wider health-service ecosystem.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Health Service Alignment; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Health Service Co-Dependency Management	Identify and manage what the service needs from others and what others need from it.
Health Service Resource Identification	Identify required workforce, facilities, equipment, clinical technology, ICT, administration and support resources without owning them.
Health Service Financial Management	Establish the service's fit within funding, financial and charging arrangements without absorbing enterprise finance/accounting.
Health Service Implementation Planning	Establish and coordinate surrounding organisational/service changes needed to introduce or materially change the service.

Boundary: Alignment does not establish the authoritative service catalogue; it positions the service within the environment represented by that catalogue. Modelling determines what service is needed; alignment determines how it fits; Offering Management establishes the identifiable offering. Expected future Service Catalogue richness explains architectural relevance, but establishes neither a BEC identity/hierarchy nor a BC↔BEC mapping. Alignment does not resolve operational-readiness ancestry.

15. Workforce & Organisational Capability

Tier and ancestry: CT1 root.

Definition: Govern practitioner qualification, engagement/privilege and enterprise service coverage, together with organisation identity/structure, recognised locations and identifiable healthcare service offerings.

Classification: Harmonia-Relevant.

Workforce & Organisational Capability remains the established CT1. Health Workforce Management and Healthcare Organisation & Service Structure Management are its distinct CT2 frames; directories represent their facts without replacing business governance of those facts.

CT2 — Health Workforce Management

Tier and ancestry: CT2; parent Workforce & Organisational Capability (CT1).

Definition: Govern practitioners' professional standing, engagement/privilege and whole-enterprise service coverage.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Health Workforce Management; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Practitioner Qualification Management	Govern professional/clinical standing and qualifications, including applicable review, escalation, complaints, penalties/issues, suspension and revocation.
Practitioner Engagement & Privilege Management	Manage practitioners as service participants, their roles/ capacities, applicable privileges and qualification context above ordinary HR employment administration.
Workforce Service Coverage Management	Observe whether required enterprise work/service obligations can be fulfilled, identify coverage gaps/risks and adjust coverage in response.

Boundary: Qualified ≠ engaged ≠ privileged for a particular activity. Coverage is broader than rostering; it asks whether required work can be covered and how coverage should change. Care Resource Deployment applies available resources to a particular care requirement; Care Support Work Coordination progresses particular support work. Neither consumption of qualification/coverage information nor directory representation transfers the underlying workforce authority to Harmonia.

CT2 — Healthcare Organisation & Service Structure Management

Tier and ancestry: CT2; parent Workforce & Organisational Capability (CT1).

Definition: Manage organisational identity/structure, recognised locations and identifiable organisational service offerings.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Healthcare Organisation & Service Structure Management; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Healthcare Organisation Identity & Structure Management	Establish and manage identity and structural relationships of participating organisations and organisational units.
Healthcare Location Definition Management	Establish and manage recognised locations and their applicable business characteristics and relationships.
Healthcare Service Offering Management	Establish/manage what identifiable service is offered, by whom, where, when and under which availability conditions/constraints.

Boundary: Location definition is distinct from Facilities Management. Service exists ≠ offered here ≠ available at this time/context. Temporal/conditional availability is material: a walk-in offering may operate 06:00–22:00 while another pathway covers other hours. Offering availability does not establish collective operational readiness; OI-BC-002 remains unplaced. Organisation, location and practitioner associations do not substitute for material Healthcare Service context.

16. Corporate Resources & Enterprise Services

Tier and ancestry: CT1 root.

Definition: Provide the bounded clinical-activity charging information and enterprise support context required for R1/R2, without assigning enterprise finance, HR, facilities or ICT operating authority to Harmonia.

Classification: Harmonia-Relevant.

CT2 — Financial & Commercial Services

Tier and ancestry: CT2; parent Corporate Resources & Enterprise Services (CT1).

Definition: Provide the financial/commercial business context materially required for R1/R2.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Financial & Commercial Services; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Clinical Activity Charging Information Provision	Identify and provide information about chargeable or cost-relevant clinical/health-service activity where required for applicable funding, financial or charging purposes.

Boundary: The single CT3 is deliberately narrow. Referral forwarding, diagnostic requests, service types, quantities and utilisation context can supply evidence; not every mediated activity is chargeable. Evidence provision does not confer pricing, billing, claims, accounting or revenue-management responsibility. Comprehensive Medicare, insurance, activity-based funding and revenue-cycle architecture is not established.

CT2 — Corporate Support Services

Tier and ancestry: CT2; parent Corporate Resources & Enterprise Services (CT1).

Definition: Provide enterprise support whose resources, availability, state or constraints may materially affect service activity.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Corporate Support Services; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Back-Office Services	Provide administrative/support services on which health-service activity depends.
Facilities Management	Manage physical facilities/services and applicable resource, location, availability and facility-state context.
IT Infrastructure Management	Operate/manage underlying technology infrastructure and its availability.
Communications & Publicity	Manage organisational communication/publication arrangements relevant to information availability/distribution.
People Management	Manage people across the organisation, distinct from practitioner-specific Health Workforce Management.
Operational Oversight	Observe broader operational conditions affecting the organisation’s ability to work or deliver services.

Boundary: These are relevant enterprise abilities even where Harmonia only consumes their state/resource/constraint context; contextual consumption alone does not make them Harmonia-Core. They are not decomposed further for R1/R2 absent a material requirement. Consumption transfers neither resource ownership nor corporate operating authority and does not resolve OI-BC-002.

Region 5: Governance & Assurance

17. Health Service Governance

Tier and ancestry: CT1 root.

Definition: Establish and maintain the obligations, authorities, constraints, policies and governance framework under which health-service activity must operate, and define how adherence will be assessed, checked and reported.

Classification: Unresolved.

Health Service Governance establishes what the organisation must do and how adherence will be determined and governed. It does not determine operational means or acquire progression/management of governed work. Enabling derivation remains unestablished; consumption of its requirements alone does not settle its relevance classification.

CT2 — Governance Framework Management

Tier and ancestry: CT2; parent Health Service Governance (CT1).

Definition: Establish the authoritative WHAT and legitimate working parameters.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Governance Framework Management; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Policy / Obligation Governance	Identify, interpret, establish and maintain authoritative legislative, regulatory, policy, standards, guideline, contractual and other obligations, attributable and traceable to source authority.
Authority & Constraint Governance	Establish who may make, approve or govern organisational decisions/artefacts and the bounds within which authority may be exercised.

Boundary: Obligation = what must be satisfied; authority = who may decide/approve; constraint = within what bounds. Engagement with regulators, standards/compliance bodies or other authorities can establish legitimate parameters without transferring their authority.

CT2 — Compliance Governance

Tier and ancestry: CT2; parent Health Service Governance (CT1).

Definition: Define how adherence to the authoritative WHAT will be determined, checked and reported.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Compliance Governance; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Compliance Assessment Definition	Define the evidence, measures, criteria, controls and assessment arrangements sufficient to evaluate compliance with established obligations, authorities and constraints.
Governance Reporting & Checkpoint Definition	Establish where/how adherence is checked and what governance/compliance information must be reported.

Boundary: Obligation definition establishes what must be satisfied; assessment definition establishes the basis for determining satisfaction. This replaces the candidate Compliance Obligation Definition wording. Compliance Governance does not itself perform assurance or assume operational management.

18. Health Service Assurance

Tier and ancestry: CT1 root.

Definition: Establish and maintain justified confidence that health services can be, and are being, delivered in accordance with applicable expectations, obligations, constraints and required levels of performance.

Classification: Harmonia-Relevant.

Health Service Assurance establishes whether sufficient evidence exists to give justified confidence in the organisation's ability to deliver its health services appropriately. Its macro-level subject is that ability. Security, information, technology, process, clinical and other concerns may contribute criteria/evidence without becoming separate Business Assurance Capability trees. The independently governed evaluation already established in the canonical architecture is a required assurance approach where REQ-FND-005 applies; the reconciliation below preserves its requirements.

CT2 — Health Service Assurance Specification

Tier and ancestry: CT2; parent Health Service Assurance (CT1).

Definition: Specify how macro-level service-delivery assurance will be undertaken and what sufficiently supports confidence.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Health Service Assurance Specification; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Assurance Approach Definition	Define scope, evidence expectations, responsibilities, methods, applicable degree of independence and timing/frequency for service-delivery assurance.
Assurance Evaluation Criteria Governance	Establish and maintain criteria against which sufficient assurance can be determined.

Boundary: Criteria may draw on workforce, facilities, clinical practice, information, technology, security, privacy, governance, dependencies and operational state. These contributing concerns do not create separate Business Assurance trees. An assurance approach SHALL preserve every applicable higher-level independence requirement; it cannot designate self-attestation as sufficient where REQ-FND-005 requires independent governed assurance.

CT2 — Assurance Evaluation

Tier and ancestry: CT2; parent Health Service Assurance (CT1).

Definition: Determine what justified confidence the applicable evidence supports.

Decomposition assertion: Each named child in this table is a CT3 Business Capability whose immediate CT2 parent is Assurance Evaluation; that CT2 has the CT1 parent stated above.

CT3 Business Capability	Definition
Assurance Evidence Assessment	Evaluate available evidence against the established assurance approach and criteria.
Assurance Conclusion Determination	Establish the legitimately supported conclusion, including insufficiency, qualification and unresolved concerns where applicable.

Boundary: Evidence assessment ≠ conclusion determination ≠ operational response. Insufficient evidence establishes neither assurance nor non-assurance. Assurance manages its own evaluation, not its subject. These BC responsibilities do not remap existing Assurance Design, Assurance Criteria Management or Governed Assurance BEC ownership.

Assurance Independence Reconciliation

Independence is a required property of applicable assurance approaches/evaluations, rather than the exhaustive definition of the macro-level Business Capability. This does **not** make Harmonia's already-required independent governed assurance optional. REQ-FND-005 requires it for activities, information and outcomes **for which assurance is required**. Its applicability is bounded; it neither defines every enterprise assurance approach nor requires assurance of every activity.

The approved Strategy derivation and Business assurance responsibilities implement that obligation through independently governed progression and conclusion authority. Assurance Approach Definition may determine applicable methods, responsibilities and degree of independence but SHALL NOT weaken those requirements. The subject may contribute evidence; it shall not solely determine, suppress, manufacture or retrospectively alter its assurance outcome. Evidence dependency is distinct from control dependency. No external assessor or physical separation is implied.

The broader organisational capability concerns justified confidence in service delivery. Harmonia's assigned participation remains assurance of explicitly required facts/behaviour; it does not acquire clinical-adequacy assurance or management of the service. Thus the old independent-evaluation definition is preserved as the established governed-assurance obligation within the broader capability, rather than discarded or universalised to all enterprise evaluations. Operational uncertainty under AX-15/REQ-FND-004 remains distinct from evidence insufficiency under REQ-FND-005.

Clinical Services Delivery Assurance Boundary

Health Service Assurance SHALL NOT be represented as Clinical Services Delivery Assurance. The enterprise scope's reference to independent clinical audit/evaluation does not make Harmonia responsible for assuring clinical judgement, professional practice, clinical adequacy of care, or delivery of clinical services by healthcare practitioners or healthcare organisations.

Information concerning clinical service delivery MAY be the subject of governed assurance where an explicit applicable requirement establishes that assurance concern. This does not transfer responsibility for Clinical Services Delivery Assurance to Harmonia. Capability 06 — Clinical Quality, Safety & Improvement retains its established clinical quality/safety responsibilities.

> **Harmonia may assure facts and behaviour concerning clinical activity where explicitly required; it does not thereby assure the clinical adequacy of care.**

Meaning of Independent

Independent Assurance requires an independently governed assurance activity/workflow whose progression and assurance conclusion are not controlled by the activity or mechanism responsible for performing or managing the subject being assured.

Independence does not inherently require an external auditor, a different organisation, department, human actor, application, deployment, database or Kubernetes cluster, or physical infrastructure separation. This is a responsibility and authority boundary consistent with REQ-FND-005, not an Application or Technology Architecture decision about physical execution separation.

> **Evidence dependency does not compromise assurance independence; control dependency does.**

The subject being assured may supply evidence. It shall not thereby be solely responsible for determining, suppressing, manufacturing or retrospectively altering its assurance conclusion.

Assurance / Operational Management Boundary

Health Service Assurance does not thereby assume operational management of its subject, assignment, delegation, reassignment, operational escalation, operational remediation, workflow progression, Incident Management or Incident Response. Findings, exceptions and conclusions may inform or initiate appropriate operational activity; responsibility for that activity remains with its operational capability.

> **Assurance is the QA service, not the Job Foreman.**

An assurance activity may manage its own internal assurance workflow without thereby assuming management of the subject being assured.

Governance, Management, Monitoring and Assurance Boundaries

Concern	Purpose and responsibility	Boundary
Governance	Establishes what must be done or must be true; applicable obligations, constraints and authority; reporting required to demonstrate compliance; and checkpoints required to determine whether obligations and constraints are satisfied.	Establishes the normative operating envelope without determining operational means.
Management	Establishes and coordinates what must be done operationally and how it will be performed within constraints; operational-state monitoring, activity progression, progress reporting, assignment/ reassignment, escalation necessary for completion, response and remediation.	Owns progression of the managed activity. Being governed does not relocate activity management to Region 5.
Management Monitoring	Observation and assessment of the current state of an activity for the purpose of determining progress, completion, fulfilment or impediments, so that the containing management or workflow activity can determine what should happen next.	May cause or inform progression, pause, retry, escalation, reassignment, response or completion. Observation alone is not Independent Assurance.
Independent Assurance	Evaluation of a governed subject against applicable obligations, constraints, controls, performance expectations, quality criteria or compliance checkpoints, using sufficient trustworthy evidence to establish an assurance conclusion.	Independently governs its evaluation and conclusion; does not progress or manage its subject.

Management Monitoring evaluates activity state in order to progress or manage the activity. Independent Assurance evaluates a governed subject against governing criteria in order to establish an assurance conclusion.

Evidence Distinctions

Evidence production does not itself perform assurance. Security, privacy, clinical, operational and information-management capabilities may produce trustworthy evidence used by Health Service Assurance without becoming assurance capabilities.

RAD source listing (presentation only; no inferred ancestry).

Provenance ≠ Assurance

Audit information / AuditEvent ≠ Assurance

Operational monitoring ≠ Assurance

Workflow progression ≠ Assurance

Business Outcome ≠ Assurance Outcome

Workflow completion ≠ Assurance

Incident ≠ Assurance Finding

Incident Response ≠ Assurance

Risk ≠ Assurance Finding

Risk ≠ Assurance Outcome

Subject-Specific Governance

> **Subject-specific governance remains with the capability responsible for that subject where governance is intrinsic to managing the subject itself.**

Health Products & Clinical Technology (05), Standards, Semantics & Reference Governance (10), Security, Privacy & Digital Trust (12), and Workforce & Organisational Capability (15) retain their existing governance semantics and regional placement. Capability 12's compliance auditing and audit-record production do not, merely by that wording, establish independent assurance. Region 5 concerns governance and independent assurance as primary responsibilities.

Classification Reconciliation and Rationale

CT1	Previous → reconciled classification	Semantic rationale
05 Health Products & Clinical Technology	Reference → Harmonia-Relevant	Device integration provision/data integration and service-relevant device context materially affect information exchange and care enablement. Supply-chain and maintenance authority remain external.
07 Community Health & Wellbeing	Reference → Harmonia-Relevant	The established preventative programme/ cohort distinctions materially frame information, notification and care enablement. Wider public-health enterprise activity is not assigned to Harmonia.
08 Population Health & Health-System Planning	Adjacent → Harmonia-Relevant	Formal notification and governed population information materially support required health activity; planning remains externally governed and is not Core through context consumption.
13 Health Research & Innovation	Adjacent → Harmonia-Relevant	Governed provision of existing information for an authorised research purpose is material support, not passive context or ownership of trials/research. Innovation evaluation does not acquire a separate platform assignment.
14 Health Service Direction & Stewardship	Reference → Harmonia-Relevant	Service needs, viability, dependencies and alignment facts materially frame Harmonia's service information and derivation requirements. This does not assign executive/service-management authority or establish a future BEC.
15 Workforce & Organisational Capability	Harmonia-Relevant retained	Professional standing, privilege, coverage and organisational/service-offering facts materially constrain care enablement. Directory support does not make workforce/service governance Core.
16 Corporate Resources & Enterprise Services	Reference → Harmonia-Relevant	Required clinical activity/utilisation evidence is material financial/charging support. Corporate support context also matters, but contextual consumption alone does not classify every support CT3 Relevant or Core.
17 Health Service Governance	Unresolved retained	The normative and compliance-definition responsibilities are established, but material platform participation/ enablement is not yet sufficiently assigned to distinguish Adjacent from Relevant/Core.

CT1	Previous → reconciled classification	Semantic rationale
18 Health Service Assurance	Unresolved → Harmonia-Relevant	Already-approved independent governed assurance materially supports service confidence. Harmonia does not own macro-level clinical/service delivery assurance, so the whole BC is not classified Core.
01–04, 06, 09–12	Established assignments retained	Their material enabling/direct platform responsibilities remain established; organisational definitions do not turn clinical judgement, consent authority or enterprise decisions into software responsibilities. Core is bounded to assigned platform activity.

Unresolved Business Capability Matters

OI-BC-001 — Medication Compounding and Preparation

Unresolved. Whether compounding/specialised preparation warrants a distinct CT3 beneath Medication & Pharmacy Services is neither established nor rejected. Absence does not mean subsumption by Prescription Medication Dispensing, irrelevance, exclusion or representation by another Capability. Adjudication must preserve peer completeness and the R1/R2 materiality test.

OI-BC-002 — Health Service Operational Readiness Placement

The identified semantic concern asks: **Can this Health Service Offering actually be delivered from this operational context?** It establishes collective readiness from workforce, administration, spaces, equipment/devices, materials, utilities, ICT/communications and other dependencies without owning them. Ongoing site/service readiness and readiness for a session are presently exercises of the same concern; session-level exercise does not establish another Capability.

Its provisional name/identified identity and boundary are retained, but **CT2 parent, CT1 ancestry and canonical placement remain unresolved**. No complete CT1→CT2→CT3 path is asserted. Offering defined/temporally available ≠ collectively ready; individual resource ready ≠ service ready; care-place readiness ≠ service readiness; readiness oversight ≠ clinical execution or supporting-resource ownership. It SHALL NOT be placed beneath Health Service Alignment, Healthcare Service Offering Management, Care Scheduling & Capacity Management, Corporate Support Services, Health Product Management or another convenient parent without explicit adjudication.

Other Preserved Uncertainties and Canonical Conflicts

- Clinical Determination and Individual Care Planning have no established CT3 set in the supplied decisions. Clinical Intervention's three-child candidate remains provisional; DN-BC-010's taxonomy question is not closed by the queue.
- DN-BC-015 defines **Clinical Device Maintenance Management** as maintaining or restoring acceptable device condition through maintenance, testing, calibration, inspection, fault remediation and repair, distinct from monitoring and overall lifecycle disposition. Its Working Decision and closing guardrail nevertheless specify seven CT3s omitting Maintenance. Its semantics are preserved, but its inclusion/tier/ancestry requires adjudication; no eighth child is manufactured.
- **Health Information Sharing Programme Management** is named by DN-BC-027's spelling correction, but that note supplies no precise tier/parent assertion. Its name is retained without constructing ancestry or an additional definition.

- **Health Information Exchange** is the approved BC CT2 name and an existing BEC name. Their identities/meanings remain distinct. This conflicts with CM-R05 and the BC/BEC naming guardrail; neither a BEC rename nor an identity merger is authorised here. The naming point remains stopped pending adjudication.
- Generic Continuous Improvement placement, external device/safety/quality monitoring providers, the possible Identity Assurance Representation BEC name and general UI/ presentation enablement remain unestablished/parked. No standards or other peer is added merely for symmetry. Other undecomposed enterprise scope remains explicitly unmodelled.

Semantic Relationships and Downstream Boundary

Business capabilities may provide evidence, context, identified events or established information to other business capabilities while retaining their own outcomes and source authority. Unless explicitly required otherwise, such provision is contextual rather than a universal dependency. **Information consumed ≠ ownership transferred; context supplied ≠ target outcome established.** These relationships are not decomposition, workflow order, runtime invocation, Service identity or application dependency. Any RADS relationship must preserve the exact meaning; where no governed type fits, preserve it as unresolved rather than substitute a stronger convenient type.

Established BC Evidence and Context Relationships

The following relationships preserve the adjudicated business meaning. They do not require every source instance to produce an input for every target instance, or make one capability perform another. A conditional relationship remains conditional. No named Business Service or machine-readable relationship type is allocated by this table.

Source	Recipient	Established meaning
Population Health Surveillance Management	Population Health Longitudinal Assessment; Population Health Event Identification	Supplies the population, concern and purpose of managed surveillance.
Population Health Longitudinal Assessment	Population Health Event Identification	Supplies trends/change evidence that may support recognition of a significant event.
Population Health Event Identification	Notifiable Health Event Management	Supplies an identified event for consideration under applicable notification obligations.
Population Health Longitudinal Assessment; Population Health Surveillance Management; Population Health Event Identification; Notifiable Health Event Management	Population Health Reporting	Supply their established assessments, surveillance state, events and notification information for reporting.
Population Health Longitudinal Assessment; Population Health Event Identification	Preventative Health Problem Identification	Supply population-health evidence/ identified concerns without determining the preventative response.
Population Health Longitudinal Assessment	Preventative Health Cohort Management	Supplies population characteristics relevant to cohort definition, stratification or review.
Population Health Event Identification	Preventative Health Cohort Management	May supply event context for identifying the population requiring preventative attention.

Source	Recipient	Established meaning
Individual Care Delivery	Population Health Surveillance	Individual information may contribute population evidence without becoming a population conclusion.
Population Health Surveillance	Individual Care Delivery	Population findings may inform individual care without establishing an Individual Clinical Determination.
Population Health Surveillance	Clinical Quality Management; Clinical Safety Management	May supply evidence relevant to their distinct assessments or risk identification; their information may also contribute population evidence.
Population Health Longitudinal Assessment; Population Health Event Identification	Health Service Demand Assessment	Supply population state/change or identified-event evidence relevant to expected service demand.
Health Service Demand Assessment	Health Service Capacity Planning	Supplies assessed demand evidence relevant to required capacity.
Health Service Capacity Planning	Health Service Distribution Planning	Supplies planned-capacity requirements relevant to the service footprint.
Operational Care Capacity Management	Health Service Demand Assessment; Health Service Capacity Planning	May supply operational pressure/ utilisation and capacity experience as planning evidence.
Health Service Distribution Planning	Healthcare Service Offering Management	May supply planned-footprint context for identifiable offerings.
Standards, Semantics & Reference Governance	Health Information Structure Management	Supplies governed semantic artefacts for reference/binding while retaining their authority.
Health Information Structure Management	Health Information Data Management	Supplies the structure, references, constraints and bindings through which content is meaningful.
Health Information Data Management	Health Information Lifecycle Management; Health Information Catalogue and Index Management; Health Information Compartmentalisation; Health Information Data Correlation	Supplies the managed information subject/content for these distinct responsibilities.
Health Information Catalogue and Index Management	Health Information Data Retrieval	Supplies existence, discovery and location context.
Health Information Compartmentalisation	Health Information Data Retrieval	Supplies applicable compartment scope/ context without making an access decision.
Health Information Data Correlation	Health Information Data Retrieval	May supply relationships relevant to identifying related requested information.
Health Information Structure Management; Health Information Lifecycle Management	Health Information Data Retrieval	Supply interpretation and applicable current/historical/lifecycle context.
Clinical Knowledge Context Identification	Clinical Knowledge Context Extraction	Supplies the identified source/context for extraction.
Clinical Knowledge Context Extraction	Clinical Knowledge Information Formulation	Supplies relevant extracted material for formulation.
Clinical Knowledge Information Formulation	Clinical Knowledge Concept Association; Clinical Knowledge Review & Approval Management	Supplies formulated knowledge for association and governed review.
Standards, Semantics & Reference Governance	Clinical Knowledge Concept Association	Supplies governed concepts/artefacts without transferring semantic authority.

Source	Recipient	Established meaning
Clinical Knowledge Concept Association	Clinical Knowledge Review & Approval Management	Supplies semantic-association context relevant to review.
Clinical Knowledge Review & Approval Management	Clinical Knowledge Publication & Sharing	Supplies knowledge whose applicable approval standing is established.
Clinical Knowledge Publication & Sharing	Individual Care Delivery	Supplies reusable knowledge; the individual-care capability establishes its clinical application/conclusion.
Health Information Management	Clinical Knowledge Context Identification	May supply source information containing potential reusable knowledge.

Material Traceability Requirements

Later derivation SHALL preserve established CT3 identity and the distinction between the Business purpose it supports and its owning BEC CT3 Feature frame. It SHALL NOT attach a Harmonia Feature directly to a BC CT3. Where the following contexts are material, traceability must preserve them without prescribing an Information Concept, representation, interface or implementation:

Concern	Context/evidence to preserve
Clinical risk	Evidence potentially indicating risk, the characterised risk, mitigation, active response and resulting state remain distinct.
Clinical quality	Underlying criterion authority; evidence source and purpose; criteria/evidence supporting an assessment; assessment authority distinct from its report and subsequent improvement.
Preventative health	Evidence supporting problem identification; target cohort; membership reasoning, inclusions/exclusions and applicable entry/exit or stratification. Programme type does not create another BC.
Population health	Population/cohort/geographical and temporal scope; evidence supporting assessment/event identification; notification obligation, recipient/authority, status and supporting event evidence.
Health-system planning	Demand basis, assumptions and scope; capacity requirements and distribution context; planned capacity distinguishable from operational evidence.
Health information	Applicable structures/reference targets/bindings and semantic authority; lifecycle state/history; catalogue versus content; compartment versus access decision; correlations and source provenance; retrieved information context.
Clinical knowledge	Source/extraction provenance and supporting formulation material; concept authority; review/approval state, conditions and history; publication preserving standing and limits; individual use distinct from general knowledge authority.

Later traceability must preserve source provenance, applicable context, temporal standing and these distinct business outcomes. Criteria do not replace governing requirements; reported representations do not originate the facts/conclusions they present. Programme-specific implementation does not establish another BC. Implementation reuse does not collapse BC peers or reverse-derive their meaning.

The separately approved assurance support derivation and B2.1 trace remain intact, with no extension to the newly reconciled CT3s. This reconciliation establishes no BEC ancestry, no framing of the 137 Features, no new BC↔BEC mapping, no allocation of Canonical IDs/local codes and no Domain05 B2.2 work. The 69 unresolved enabling descriptions remain unresolved. Those are subsequent phases.

- Business Enabling Capabilities
- Capability decomposition and derivation model

- Historical five-region reconciliation
- Business Architecture reference candidate

10. Canonical Business Enabling Capability Model & Features

RAD source: `docs/markdown/02-strategy/capabilities/business-enabling-capabilities.md`

The following is a presentation transcription of the governed source. Source catalogue layout and retained illustrations do not allocate tiers or establish additional identities. Equations wrap as text; diagram notation is textual.

Canonical Business Enabling Capability Model & Features

Overview & Architecture

The Business Enabling Capability model defines **what systems and information infrastructure must enable or provide in support of the healthcare enterprise**.

Where Business Capabilities define the high-level healthcare landscape (what the enterprise does), Business Enabling Capabilities specify the functional abilities systems must deliver to make healthcare delivery, administration, and operations possible.

Architectural Principles Governing this Model

1. **Authentic Healthcare Operating Contexts**: Capabilities are grouped into five authentic healthcare contextual views. They reflect genuine healthcare operating environments (e.g., Ward Operations, Order Administration, Patient Clinical Record), **not** low-level integration engine plumbing or middleware mechanics.
2. **Independence from Commercial Product Boundaries**: Capabilities are defined independently of commercial software product boundaries (such as PAS, EMR, LIS, RIS, HRMS, or FMIS). A commercial product may implement multiple capabilities, drawing ”dotted lines” around parts of the model; the capability model remains orthogonal to vendor packaging.
3. **Atomic Feature Decomposition**: Within established Harmonia-Relevant and Harmonia-Core Capability contexts, system behaviour is decomposed into atomic **Features**—the smallest useful, testable, technology-neutral statements of system-enabled behaviour. Reference and Adjacent capabilities are intentionally not decomposed into features.
4. **Technology Neutrality**: No capability or feature is defined in terms of specific software products (HAPI FHIR, Infinispan, PostgreSQL, ActiveMQ Artemis, Camel, Netty, Vue, Spring) or runtime plumbing (threads, queues, ports, DDL).
5. **Architectural Scope Sufficiency**: The Business Enabling Feature catalogue provides the established R1.x/R2.x Feature baseline for the capabilities and contextual views presently decomposed. Completeness SHALL NOT be interpreted as requiring artificial Feature decomposition of capabilities for which no Feature-level decomposition has been architecturally established. Roadmap items (such as authoritative master-patient EMPI reconciliation) are explicitly demarcated as uncommitted Harmonia 3.x candidates.

Under approved G1 K9, the five views, catalogue numbering, indentation and decomposition presentation do not establish Architectural Element identity, CT1 / CT2 / CT3 assignment, ancestry or root status. The 137 established Features retain their FT type, names and identifiers. Affected complete Capability ancestry and structural Canonical IDs remain unresolved. Derivation may proceed from established ownership and responsibility without inventing intermediate elements.

The fixed Capability decomposition requires **CT1 → CT2 → CT3 → FT** for every fully reconciled Feature. Every Feature SHALL be framed by CT3 with CT2 and CT1 parents; tiers SHALL NOT

be skipped. A Feature cannot compensate for incomplete upstream Capability modelling. Existing Features without established CT3 frames remain valid evidence with Capability ancestry/decomposition requiring separate reconciliation; their current placement does not establish direct CT1/CT2 → FT permission. Neither the 69 unresolved enabling catalogue descriptions nor any other presentation entry becomes a CT3 Capability through this obligation. No Capability, tier assignment, Feature, local ID or complete Canonical ID is created or changed here.

The approved Health Service Assurance derivation additionally establishes **Assurance Design**, **Assurance Criteria Management** and **Governed Assurance** as Business Enabling Capabilities supporting BC-18. Their definitions and assurance principles are recorded below. Their contextual-view placement, relevance classification, Capability Tier, complete ancestry and structural Canonical IDs remain unresolved; they are not assigned through document position or lexical similarity. No additional Feature catalogue is derived.

The Five Contextual Views

The Business Enabling capabilities are structured across five contextual views:

RAD source listing (presentation only; no inferred ancestry).

1. ENTITY MANAGEMENT

   Governance and administration of healthcare entities throughout their lifecycle: Clients, Providers, Organisations, Locations, Services, Products, and Devices.

2. SERVICE ADMINISTRATION

   Administrative, logistical, and financial progression of care: Client Intake, Referrals, Encounters, Orders, Diagnostics, Care Plans, Clinical Messaging, and Billing Extraction.

3. SERVICE DELIVERY

   Systems enablement across clinical delivery settings: Primary Care, Acute Inpatient, Emergency, Surgical, Diagnostic, Medication, Allied Health, Community, and Remote/Virtual Care.

4. HEALTH SERVICE OPERATIONS

   Facility logistics, bed and care-place management, capacity, clinical resource coordination, work allocation and dispatch, patient transport, and discharge progression.

5. INTRINSIC / SHARED ENABLEMENT

   Reusable system capabilities: Longitudinal Clinical Record, Health Information Exchange, Information Access, Policy & Security Control, Collaboration, Workflow Coordination, and Presentation.

Architectural Scope Statement (Harmonia 1.x/2.x vs. 3.x Roadmap)

> The Business Enabling Feature catalogue provides the established R1.x/R2.x Feature baseline for the capabilities and contextual views presently decomposed. Completeness SHALL NOT be interpreted as requiring artificial Feature decomposition of capabilities for which no Feature-level decomposition has been architecturally established.

- **Established Coverage**: The model records established capability responsibilities and the 137-Feature baseline. BC-17 enablement and the assurance capabilities' unestablished contextual-view, tier, ancestry and Feature relationships remain explicit gaps; structural symmetry is not a completion requirement.

- **Sufficient Does Not Mean Frozen Forever**: Additional capabilities and features may be introduced where future Harmonia scope requires them.
- **Roadmap Demarcation — Patient Identity**:
  - **Harmonia 1.x/2.x Scope**: Includes cross-system identifier resolution, identifier correlation, identity alias association, identity provenance, identity federation across regional nodes, and governed administrative correction.
  - **Harmonia 3.x Roadmap Candidate**: Authoritative master-patient identity reconciliation, probabilistic matching algorithms, and Enterprise Master Person Index (EMPI) merge/unmerge engines remain an uncommitted Harmonia 3.x candidate. Harmonia 1.x/2.x relies on authoritative upstream PAS/MPI feeds rather than acting as the master clinical EMPI authority.

View 1: Entity Management

Entity Management encompasses systems enablement for identifying, governing, and managing core healthcare entities throughout their operational lifecycles.

RAD source listing (presentation only; no inferred ancestry).

1. Client Administration

1.1 Person Identity [Harmonia-Core]

1.2 Healthcare Subject [Harmonia-Core]

1.3 Client Relationship [Harmonia-Relevant]

1.4 Client Privacy [Harmonia-Core]

2. Provider Administration

2.1 Practitioner [Harmonia-Core]

2.2 Practitioner Role [Harmonia-Core]

2.3 Provider Relationship [Harmonia-Relevant]

2.4 Provider Engagement [Harmonia-Relevant]

3. Organisation Administration

3.1 Healthcare Organisation [Harmonia-Core]

3.2 Organisational Structure [Harmonia-Core]

3.3 Organisation Contact [Harmonia-Relevant]

4. Location Administration

4.1 Healthcare Location [Harmonia-Core]

4.2 Location Structure [Harmonia-Core]

4.3 Care-Place Definition [Harmonia-Core]

5. Health Service Administration

5.1 Healthcare Service Definition [Harmonia-Core]

5.2 Service Provision Relationship [Harmonia-Core]

5.3 Service Availability Definition [Harmonia-Core]

5.4 Service Eligibility Definition [Harmonia-Relevant]

6. Health Product Administration

6.1 Health Product Definition [Adjacent]

6.2 Product Formulary & Catalogue [Adjacent]

6.3 Product Batch & Serial Tracking [Reference]

7. Clinical Device Administration

7.1 Device Definition [Harmonia-Relevant]

7.2 Device Instance [Harmonia-Relevant]

7.3 Device Association [Harmonia-Relevant]

7.4 Device Connectivity [Harmonia-Relevant]

Detailed Capability & Feature Catalogue: Entity Management

1. Client Administration

Governs the representation, identification, and demographic context of individuals receiving care.

- **1.1 Person Identity** (`Harmonia-Core`)
  - *Definition*: Resolving, maintaining, and correlating identities and identifiers assigned to an individual across different authorities.
  - `FEAT-EM-01`: **Identifier Resolution**: Ingest and resolve external identifiers against registered issuing authorities and namespace systems.
  - `FEAT-EM-02`: **Cross-Authority Identifier Correlation**: Correlate disparate local identifiers (e.g., hospital MRN, clinic client ID, national IHI) belonging to the same individual.
  - `FEAT-EM-03`: **Identity Alias Association**: Maintain explicit associations between active identifiers and historical or demographic aliases without destructive overwriting.
  - `FEAT-EM-04`: **Governed Identity Correction**: Apply administrative identity updates, linked-record corrections, and split/merge notifications propagated from authoritative sources.
- **1.2 Healthcare Subject** (`Harmonia-Core`)
  - *Definition*: Managing the core healthcare subject context bound to clinical records and operational activities.
  - `FEAT-EM-05`: **Subject Context Binding**: Bind authenticated person identity to clinical encounters, orders, and observations as the enduring subject of care with verifiable association.
  - `FEAT-EM-06`: **Demographic Context Governance**: Manage essential demographic attributes (date of birth, sex, vital status) with full provenance and timestamping.
- **1.3 Client Relationship** (`Harmonia-Relevant`)
  - *Definition*: Representing relationships between the client and carers, guardians, or contacts.
  - `FEAT-EM-07`: **Next-of-Kin & Guardian Association**: Associate nominated emergency contacts, legal guardians, and care agents with the client record.
  - `FEAT-EM-08`: **Relationship Validity Tracking**: Maintain effective date intervals and authority scopes for nominated client relationships.
- **1.4 Client Privacy** (`Harmonia-Core`)
  - *Definition*: Enforcing client-level privacy preferences, consent directives, and confidentiality markers.
  - `FEAT-EM-09`: **Consent Directive Evaluation**: Evaluate client consent directives against requested information disclosure and exchange actions.
  - `FEAT-EM-10`: **Confidentiality Flag Enforcement**: Enforce restricted access flags (e.g., VIP, sensitive diagnosis, restricted record) across query and presentation interfaces.

2. Provider Administration

Governs healthcare practitioners, their professional qualifications, operational roles, and organisational engagements.

- **2.1 Practitioner** (`Harmonia-Core`)
  - *Definition*: Identifying and verifying individual healthcare professionals.

  - `FEAT-EM-11`: **National Practitioner Verification**: Ingest, validate, and verify national practitioner identifiers (e.g., HPI-I) and professional registration status.
  - `FEAT-EM-12`: **Practitioner Profile Governance**: Maintain core professional demographic details, qualifications, and active practice status.
- **2.2 Practitioner Role** (`Harmonia-Core`)
  - *Definition*: Modeling the formal role, specialty, and operational capacity of a practitioner at a given organisation or location.
  - `FEAT-EM-13`: **Role & Specialty Modeling**: Define and associate clinical specialties, practice roles, and service capacities for a practitioner.
  - `FEAT-EM-14`: **Electronic Endpoint Binding**: Associate validated communication endpoints (secure messaging addresses, direct routing queues) with specific practitioner roles.
- **2.3 Provider Relationship** (`Harmonia-Relevant`)
  - *Definition*: Managing organizational affiliations and multidisciplinary team relationships.
  - `FEAT-EM-15`: **Practitioner-Organisation Affiliation**: Maintain temporal affiliations between practitioners and healthcare organizations.
- **2.4 Provider Engagement** (`Harmonia-Relevant`)
  - *Definition*: Capturing terms of service and clinical privilege agreements.
  - `FEAT-EM-16`: **Clinical Privileging Record**: Track active clinical admitting and practice privileges within healthcare facilities.

3. Organisation Administration

Governs healthcare delivery organizations, facilities, and network structures.

- **3.1 Healthcare Organisation** (`Harmonia-Core`)
  - *Definition*: Authoritative identification and profiling of legal and operational healthcare entities.
  - `FEAT-EM-17`: **National Organisation Verification**: Validate national organization identifiers (e.g., HPI-O) and legal entity classifications.
  - `FEAT-EM-18`: **Organisation Profile Management**: Govern directory profiles, operational status, and published service descriptions.
- **3.2 Organisational Structure** (`Harmonia-Core`)
  - *Definition*: Managing parent-child hierarchies, departments, and corporate relationships.
  - `FEAT-EM-19`: **Hierarchical Structure Navigation**: Model, traverse, and resolve nested departmental, divisional, and regional healthcare networks.
- **3.3 Organisation Contact** (`Harmonia-Relevant`)
  - *Definition*: Maintaining verified administrative and clinical communication channels.
  - `FEAT-EM-20`: **Directory Contact Management**: Manage verified phone, email, and administrative contact channels for healthcare facilities.

4. Location Administration

Governs the physical spaces, facilities, and care environments where healthcare is delivered.

- **4.1 Healthcare Location** (`Harmonia-Core`)
  - *Definition*: Identifying and profiling geographic locations, physical campuses, and health facilities.
  - `FEAT-EM-21`: **Location Geospatial & Address Resolution**: Validate physical addresses, geospatial coordinates, and location operational status.
- **4.2 Location Structure** (`Harmonia-Core`)
  - *Definition*: Modeling the physical architecture of healthcare campuses.
  - `FEAT-EM-22`: **Physical Hierarchy Modeling**: Represent campus, building, wing, floor, and ward architectural relationships.

- **4.3 Care-Place Definition** (`Harmonia-Core`)
  - *Definition*: Modeling the atomic care delivery places (beds, bays, treatment chairs, surgical tables).
  - `FEAT-EM-23`: **Atomic Care-Place Modeling**: Define specific beds, care-bays, and examination rooms with their physical capabilities (e.g., telemetry-capable, negative-pressure).

5. Health Service Administration

Governs the healthcare services offered by providers and organizations.

- **5.1 Healthcare Service Definition** (`Harmonia-Core`)
  - *Definition*: Specifying clinical and administrative health services offered to the public or clinical referrers.
  - `FEAT-EM-24`: **Service Catalogue Publishing**: Publish structured healthcare service definitions, clinical specialties, and intake criteria to regional directories.
- **5.2 Service Provision Relationship** (`Harmonia-Core`)
  - *Definition*: Binding health services to specific organizations, locations, and practitioner roles.
  - `FEAT-EM-25`: **Service-Location-Provider Binding**: Correlate which organization provides a named health service at which physical location with which practitioner roles.
- **5.3 Service Availability Definition** (`Harmonia-Core`)
  - *Definition*: Modeling regular operating hours, exceptional closures, and emergency availability.
  - `FEAT-EM-26`: **Operating Schedule Specification**: Define temporal availability, clinic operating hours, and on-call service windows.
- **5.4 Service Eligibility Definition** (`Harmonia-Relevant`)
  - *Definition*: Capturing referral criteria, catchment boundaries, and clinical eligibility.
  - `FEAT-EM-27`: **Eligibility & Catchment Specification**: Define clinical referral thresholds and geographic catchment boundaries.

6. Health Product Administration

Governs medications, blood products, implants, and consumable health goods.

- **6.1 Health Product Definition** (`Adjacent`): Classification of pharmaceutical and consumable products.
- **6.2 Product Formulary & Catalogue** (`Adjacent`): Management of approved facility medication formularies.
- **6.3 Product Batch & Serial Tracking** (`Reference`): Supply chain batch and lot management. *(No features decomposed for Reference/Adjacent capabilities).*

7. Clinical Device Administration

Governs medical equipment, patient monitors, diagnostic instruments, and telehealth devices.

- **7.1 Device Definition** (`Harmonia-Relevant`)
  - *Definition*: Specifying medical equipment types, models, and regulatory classifications.
  - `FEAT-EM-28`: **Device Type Specification**: Catalog medical device models, regulatory approvals, and capabilities.
- **7.2 Device Instance** (`Harmonia-Relevant`)
  - *Definition*: Tracking physical device hardware instances and serial numbers.
  - `FEAT-EM-29`: **Device Serial Tracking**: Ingest and maintain unique device identifier (UDI) records and serial numbers.
- **7.3 Device Association** (`Harmonia-Relevant`)

  - *Definition*: Associating clinical devices with patients, beds, or rooms.
  - `FEAT-EM-30`: **Device-Patient/Location Assignment**: Track dynamic association between a telemetry monitor, patient, and physical bed.
- **7.4 Device Connectivity** (`Harmonia-Relevant`)
  - *Definition*: Registering communication protocols and network endpoints for medical devices.
  - `FEAT-EM-31`: **Device Endpoint Registration**: Register communication interfaces and security credentials for diagnostic telemetry feeds.

View 2: Service Administration

Service Administration encompasses systems enablement for the operational, administrative, logistical, and financial progression of care.

RAD source listing (presentation only; no inferred ancestry).

1. Client-Centred Administration [Harmonia-Relevant]
2. Referral Administration [Harmonia-Relevant]
3. Scheduling Administration [Harmonia-Relevant]
4. Episode & Encounter Administration [Harmonia-Core]
5. Order Administration [Harmonia-Core]
6. Diagnostic Administration [Harmonia-Core]
7. Medication Administration [Harmonia-Relevant]
8. Procedure Administration [Harmonia-Relevant]
9. Clinical Record Administration [Harmonia-Core]
10. Care Plan Administration [Harmonia-Relevant]
11. Clinical Communication Administration [Harmonia-Core]
12. Funding & Coverage Administration [Adjacent]
13. Clinical Charging & Claims Administration [Adjacent]

Critical Domain Distinctions: Service Administration

Distinction: Referral vs. Order

- **Referral Administration**: A **Referral** formally requests another care participant or service provider to assess, manage or assume some degree of clinical care responsibility. Referral intent, disposition, responsibility actually assumed and Transfer of Care are distinct. Referral may lead to shared or assumed responsibility or transfer of ongoing care, but does not itself establish acceptance, delivery, assumption or Transfer of Care, or universally require an Appointment, Encounter or Order.
- **Order Administration**: An **Order** requests a defined, bounded **fulfilment activity** (e.g., execute a diagnostic test, dispense a medication, perform a procedure) and strictly supports a **closed-loop progression** returning an outcome to the requester.

Distinction: Technical ACK vs. Business ACK

- **Technical Delivery Acknowledgement**: A technical acknowledgement establishes only the acceptance state defined by the applicable interaction contract. It SHALL NOT be interpreted as durable acceptance, committed business state, successful processing or successful business disposition unless that meaning is explicitly established by the governing

contract. HTTP 200 does not universally establish successful business disposition, and HTTP 202 does not universally establish durable queue acceptance; either may have stronger meaning under an explicit contract.
- **Business Acknowledgement**: A clinical/operational message generated by the receiving application or clinical staff confirming that the order or referral has been reviewed, accepted for execution, scheduled, or rejected. Technical Delivery Acknowledgement ≠ Business Acknowledgement

Acknowledgement of a handoff is not, by itself, evidence of eventual downstream business outcome. REQ-FND-001 continues to require the applicable durable acceptance boundary before Harmonia positively acknowledges ingress acceptance.

Business acknowledgement is contextual and may communicate acceptance, rejection or another explicit disposition; it does not establish a universal reviewed → accepted → scheduled progression or transfer originating authority.

Order Closed-Loop Progression Semantics

All order-related capabilities must support the end-to-end closed loop: Destination → Route → Deliver → Acknowledge → Progress → Complete → Outcome

Detailed Capability & Feature Catalogue: Service Administration

1. Client-Centred Administration (`Harmonia-Relevant`)

- **1.1 Client Intake & Demographic Update**
  - `FEAT-SA-01`: **Demographic Update Ingestion**: Ingest and validate patient admission and registration updates from external PAS systems.
  - `FEAT-SA-02`: **Demographic Change Distribution**: Fan out verified client demographic updates to downstream clinical subsystems.

2. Referral Administration (`Harmonia-Relevant`)

- **2.1 Referral Intake & Progression**
  - `FEAT-SA-03`: **Referral Document Ingestion**: Ingest structured clinical referral messages across integration boundaries.
  - `FEAT-SA-04`: **Referral Supporting Information**: Collate and associate supporting clinical documentation, diagnostic history, and patient context with referral requests.
  - `FEAT-SA-05`: **Referral Status & Outcome Tracking**: Track referral operational progression and communicate referral outcomes (acceptance, decline, or redirection) to referring providers.

3. Scheduling Administration (`Harmonia-Relevant`)

- **3.1 Appointment & Booking Administration**
  - `FEAT-SA-06`: **Appointment Notification Ingestion**: Ingest scheduled appointment events from departmental scheduling systems.
  - `FEAT-SA-07`: **Appointment Status Synchronization**: Propagate booking cancellations, reschedules, and attendances across care coordination systems.

4. Episode & Encounter Administration (`Harmonia-Core`)

- **4.1 Encounter Lifecycle Management**

- FEAT-SA-08: **Encounter Context Creation**: Create and maintain authoritative encounter tracking contexts for hospital admissions, outpatient visits, and community contacts.
- FEAT-SA-09: **Encounter State Progression**: Track encounter status transitions (PLANNED → ARRIVED → IN_PROGRESS → ON_LEAVE → DISCHARGED).
- FEAT-SA-10: **Encounter Bed/Location Association**: Dynamically maintain the association between an encounter, care team, and physical bed or ward location.

5. Order Administration (Harmonia-Core)

- **5.1 Closed-Loop Order Progression**
  - FEAT-SA-11: **Order Request Ingestion**: Ingest clinical diagnostic, medication, and procedure order requests with validated ordering clinician context.
  - FEAT-SA-12: **Order Destination Resolution & Routing**: Resolve fulfillment endpoints and reliably route orders to performing service systems.
  - FEAT-SA-13: **Order Closed-Loop Progression Tracking**: Track order progression across destination determination, routing, delivery, receipt acknowledgement (distinguishing technical delivery acknowledgement from business acknowledgement), execution progress, completion, and outcome association.
  - FEAT-SA-14: **Order Cancellation & Modification Coordination**: Coordinate order cancellation and amendment requests, ensuring synchronization with performing systems.

6. Diagnostic Administration (Harmonia-Core)

- **6.1 Diagnostic Workflow Management**
  - FEAT-SA-15: **Diagnostic Request Correlation**: Correlate diagnostic test orders with collected clinical specimens and patient encounters.
  - FEAT-SA-16: **Diagnostic Report Ingestion & Binding**: Ingest finalized laboratory and radiology reports, verifying content integrity and binding them to the parent order.
  - FEAT-SA-17: **Diagnostic Report Distribution**: Distribute finalized laboratory, pathology, and imaging reports to ordering clinicians and designated care teams.

This Feature retains its finalised-report responsibility. Broader Domain03 Diagnostic Administration behaviour may also handle preliminary, corrected and addendum/correction information without redefining this narrower Feature.

7. Medication Administration (Harmonia-Relevant)

- **7.1 Medication Order & Dispense Management**
  - FEAT-SA-18: **Medication Order Ingestion**: Ingest inpatient and discharge prescription orders.
  - FEAT-SA-19: **Dispense Event Tracking**: Ingest medication dispense verification records from pharmacy management systems.
  - FEAT-SA-20: **Medication Administration Record (MAR) Ingestion**: Ingest point-of-care medication administration events.

8. Procedure Administration (Harmonia-Relevant)

- **8.1 Procedure & Surgical Administration**
  - FEAT-SA-21: **Procedure Booking Ingestion**: Ingest scheduled surgical case bookings and procedural requests.
  - FEAT-SA-22: **Procedural Documentation Ingestion**: Ingest operative reports, anesthesia records, and post-procedure summaries.

9. Clinical Record Administration (`Harmonia-Core`)

- **9.1 Clinical Documentation Lifecycle**
  - `FEAT-SA-23`: **Clinical Document Ingestion**: Ingest structured and unstructured clinical documents (discharge summaries, consult notes, letters).
  - `FEAT-SA-24`: **Document Versioning & Supersession**: Manage document classification, author attribution, amendment attribution, versioning, and supersession or withdrawal without destructive data loss.
  - `FEAT-SA-25`: **Document Metadata Indexing**: Index clinical document metadata (author, specialty, encounter, date, confidentiality) for rapid discovery.

10. Care Plan Administration (`Harmonia-Relevant`)

- **10.1 Care Plan Coordination**
  - `FEAT-SA-26`: **Care Plan Ingestion & Distribution**: Ingest multidisciplinary care plans and syndicate updates to authorized care team members.

11. Clinical Communication Administration (`Harmonia-Core`)

- **11.1 Secure Clinical Communication**
  - `FEAT-SA-27`: **Clinical Communication Distribution**: Distribute secure clinical communications between verified healthcare providers and designated care team endpoints.
  - `FEAT-SA-28`: **Delivery Acknowledgement Tracking**: Correlate recipient delivery and read receipts with outbound clinical communications, distinguishing transport delivery from recipient acknowledgement. Receipt/read evidence does not by itself establish business acceptance, clinical review, incorporation, approval, finality or originating authority.
  - `FEAT-SA-29`: **Communication Audit Logging**: Maintain an enduring, non-PHI, tamper-evident log of all clinical communication transactions.

12. Funding & Coverage Administration (`Adjacent`)

- Ingestion of patient insurance coverage and eligibility flags. *(No features decomposed)*.

13. Clinical Charging & Claims Administration (`Adjacent`)

- Export of clinical activity summaries and charging records to billing systems. *(No features decomposed)*.

View 3: Service Delivery

Service Delivery encompasses systems enablement across genuine healthcare delivery settings and clinical care delivery contexts.

These established Business Enabling contextual entries are not the CT2 decomposition of 01 Individual Care Delivery. The Business Capability's six CT2 abilities remain independent of these contexts under B2-R05 and B2-R06.

RAD source listing (presentation only; no inferred ancestry).

1. Clinical Assessment [Adjacent]
2. Primary Care [Harmonia-Relevant]
3. Acute Care [Harmonia-Relevant]

4. Emergency Care [Harmonia-Relevant]
5. Inpatient Care [Harmonia-Relevant]
6. Ambulatory Care [Adjacent]
7. Surgical & Procedural Care [Adjacent]
8. Diagnostic Services [Harmonia-Relevant]
9. Therapeutic Services [Adjacent]
10. Medication Therapy [Harmonia-Relevant]
11. Allied Health [Adjacent]
12. Preventive Care [Harmonia-Relevant]
13. Rehabilitation [Adjacent]
14. Community Care [Harmonia-Relevant]
15. Outreach Care [Harmonia-Relevant]
16. Remote & Virtual Care [Harmonia-Relevant]

Architectural Principles: Service Delivery

1. **Heterogeneous Axes Preserved**: The 16 Service Delivery capabilities represent heterogeneous clinical settings (acute, emergency, primary), modalities (virtual, outreach), and specialties (diagnostic, therapeutic). They are deliberately preserved as authentic healthcare contexts rather than normalized into artificial abstractions.
2. **Zero Harmonia-Core in Service Delivery**: No Service Delivery capability is classified as `Harmonia-Core`. Harmonia provides integration, transport, state management, and notification; **Harmonia does not practice medicine or deliver clinical therapy**.
3. **Canonical Enabling Pattern for Relevant Features**: Provide Context → Receive Clinical State/Outcome → Correlate → Preserve Provenance → Make Available → Enable Next Activity

Detailed Capability & Feature Catalogue: Service Delivery

2. Primary Care (`Harmonia-Relevant`)

- **2.1 Primary Care Coordination**
  - `FEAT-SD-01`: **General Practice Shared Record Ingestion**: Ingest shared health summaries, encounter notes, and immunization records from primary care practice management systems.
  - `FEAT-SD-02`: **Primary Care Notification Dispatch**: Dispatch emergency department arrival, acute admission, and discharge notifications to the patient's registered general practitioner.

3. Acute Care (`Harmonia-Relevant`)

- **3.1 Acute Care Monitoring & Event Coordination**
  - `FEAT-SD-03`: **Acute Clinical State Event Capture**: Ingest clinical deterioration notifications and acute clinical status updates.
  - `FEAT-SD-04`: **Acute Care Clinical Context Provision**: Provide consolidated longitudinal history and active medication lists to acute care clinicians at point of triage.

4. Emergency Care (`Harmonia-Relevant`)

- **4.1 Emergency Care Progression**
  - `FEAT-SD-05`: **Emergency Department Arrival & Triage Ingestion**: Ingest emergency presentation, triage category, and chief complaint records.
  - `FEAT-SD-06`: **Emergency Care Cross-Facility Correlation**: Correlate incoming emergency presentations with recent regional admissions, prior discharge summaries, and known allergy profiles.

5. Inpatient Care (`Harmonia-Relevant`)

- **5.1 Multidisciplinary Inpatient Care Coordination**
  - `FEAT-SD-07`: **Inpatient Rounding Context Provision**: Aggregate diagnostic results, current medication administration records, and nursing notes for multidisciplinary inpatient rounds.
  - `FEAT-SD-08`: **Inpatient Clinical Progression Tracking**: Capture inpatient clinical milestone completions (e.g., medical stability achieved, allied health clearance).

8. Diagnostic Services (`Harmonia-Relevant`)

- **8.1 Diagnostic Service Integration**
  - `FEAT-SD-09`: **Laboratory & Imaging Result Distribution**: Reliably distribute structured diagnostic reports (pathology, radiology) to ordering clinicians and designated secondary recipients.
  - `FEAT-SD-10`: **Diagnostic History Consolidation**: Consolidate historical diagnostic trends (e.g., cumulative laboratory results) across multiple regional providers into an integrated view.

10. Medication Therapy (`Harmonia-Relevant`)

- **10.1 Medication Therapy Provision and Monitoring**
  - `FEAT-SD-11`: **Medication History Reconciliation Ingestion**: Ingest reconciled medication histories compiled during clinical transitions of care.
  - `FEAT-SD-12`: **Medication Administration Tracking**: Distribute medication therapy orders, dispensing updates, and administration records across clinical care boundaries.

12. Preventive Care (`Harmonia-Relevant`)

- **12.1 Screening and Immunisation Enablement**
  - `FEAT-SD-13`: **Immunisation Event Submission**: Ingest point-of-care immunisation administration records and syndicate to national immunization registers.
  - `FEAT-SD-14`: **Screening Recall Notification Distribution**: Distribute clinical screening reminders and recall notices to designated healthcare providers.

14. Community Care (`Harmonia-Relevant`)

- **14.1 Community Care Coordination**
  - `FEAT-SD-15`: **Community Encounter Ingestion**: Ingest home visit notes, community nursing observations, and social support assessments.
  - `FEAT-SD-16`: **Community Care Plan Synchronization**: Synchronize community care plan updates between hospital outreach teams and community health organizations.

15. Outreach Care (`Harmonia-Relevant`)

- **15.1 Outreach Care Continuity**
  - `FEAT-SD-17`: **Outreach Visit Context Provision**: Package and deliver portable clinical summaries and offline-capable care summaries for remote outreach clinics.
  - `FEAT-SD-18`: **Outreach Encounter Outcome Ingestion**: Ingest and reconcile clinical documentation gathered during rural and remote outreach visits.

16. Remote & Virtual Care (`Harmonia-Relevant`)

- **16.1 Remote Patient Monitoring**
  - `FEAT-SD-19`: **Remote Telemetry Ingestion**: Ingest home patient monitoring data, biometric telemetry, and patient-reported outcome measures (PROMs).
  - `FEAT-SD-20`: **Virtual Care Session Context Binding**: Associate synchronous virtual/telehealth consultations with the longitudinal health record and active care episode.

(Capabilities 1 Clinical Assessment, 6 Ambulatory Care, 7 Surgical & Procedural Care, 9 Therapeutic Services, 11 Allied Health, and 13 Rehabilitation remain Adjacent or Reference and are not decomposed into atomic features).

View 4: Health Service Operations

Health Service Operations encompasses systems enablement for the operational management, logistics, capacity, and resource coordination of healthcare facilities and services.

RAD source listing (presentation only; no inferred ancestry).

1. Clinic & Practice Operations [Harmonia-Relevant]
2. Ward Operations [Harmonia-Relevant]
3. Theatre Operations [Harmonia-Relevant]
4. Emergency Department Operations [Harmonia-Relevant]
5. Outpatient Operations [Harmonia-Relevant]
6. Bed & Care-Place Management [Harmonia-Core]
7. Clinical Resource Management [Harmonia-Relevant]
8. Service Capacity Management [Harmonia-Relevant]
9. Clinical Workforce Management [Adjacent]
10. Clinical Rostering [Adjacent]
11. Mobile Staff Management [Harmonia-Relevant]
12. On-Call Management [Harmonia-Relevant]
13. Work Allocation & Dispatch [Harmonia-Core]
14. Clinical Training & Education [Reference]
15. Clinical Competency Management [Reference]
16. Clinical Qualification Management [Harmonia-Relevant]
17. Clinical Supervision [Reference]
18. Patient Transport [Harmonia-Core]
19. Clinical Logistics Coordination [Harmonia-Relevant]
20. Discharge Management [Harmonia-Core]

Architectural Principles: Health Service Operations

1. **Facility Logistics, Not IT Machinery**: Health Service Operations reflects real-world health facility logistics and coordination (wards, beds, staff allocation, patient transfer); it must **never** be redefined or reinterpreted as protocol mediation, message queuing, deduplication, or integration software machinery.
2. **Healthcare Service Context Rule**:

   > **Where operational activity occurs in the context of a Healthcare Service, that service context must be explicitly associated with the activity and state rather than inferred solely from practitioner, organisation, or location.**

1. **General Operational Progression Pattern**: Event → Entity Context → Required Activity → Assignment → Dispatch → ACK → Progress → Outcome → Entity State Change → Next Activity

Detailed Capability & Feature Catalogue: Health Service Operations

1. Clinic & Practice Operations (`Harmonia-Relevant`)

- **1.1 Session & Queue Coordination**
  - `FEAT-HSO-01`: **Clinic Session Tracking**: Ingest and track clinic operating session start, delay, and completion states.
  - `FEAT-HSO-02`: **Operational Queue Progression**: Track client queue progression through reception, waiting area, consultation, and billing.

2. Ward Operations (`Harmonia-Relevant`)

- **2.1 Ward Logistics Coordination**
  - `FEAT-HSO-03`: **Ward Patient Flow Coordination**: Coordinate inpatient ward occupancy, care-place allocation, and patient bed status across care facilities.
  - `FEAT-HSO-04`: **Clinical Handover Context Assembly**: Collate current ward patient lists, active medical concerns, and pending tasks for nursing shift handovers.

3. Theatre Operations (`Harmonia-Relevant`)

- **3.1 Perioperative Logistics**
  - `FEAT-HSO-05`: **Operating Theatre Case Progression**: Track surgical case state transitions (`CALLED` → `IN_THEATRE` → `ANAESTHETISED` → `INCISION` → `CLOSED` → `RECOVERY`).
  - `FEAT-HSO-06`: **Perioperative Resource Notification**: Dispatch alerts to portering, sterilization, and recovery teams based on surgical case progression milestones.

4. Emergency Department Operations (`Harmonia-Relevant`)

- **4.1 ED Operational Coordination**
  - `FEAT-HSO-07`: **ED Bed & Bay Tracking**: Track occupancy and status of emergency cubicles, resuscitation bays, and short-stay beds.
  - `FEAT-HSO-08`: **ED Length of Stay Tracking**: Monitor operational wait times and length-of-stay milestones, generating operational escalation notifications.

5. Outpatient Operations (`Harmonia-Relevant`)

- **5.1 Outpatient Flow Enablement**
  - `FEAT-HSO-09`: **Outpatient Arrival Registration**: Ingest self-service check-in and clinic arrival events, notifying treating clinicians.
  - `FEAT-HSO-10`: **Post-Clinic Order Coordination**: Capture follow-up booking and diagnostic order requirements generated at clinic completion.

6. Bed & Care-Place Management (`Harmonia-Core`)

- **6.1 Bed State & Turnover Management**
  - `FEAT-HSO-11`: **Bed Availability Tracking**: Maintain operational status of all registered beds and care-places (`VACANT`, `OCCUPIED`, `BLOCKED`, `RESERVED`, `DIRTY`, `CLEANING`, `CLEAN`).
  - `FEAT-HSO-12`: **Bed State Progression**: Progress bed states in response to clinical and environmental events (e.g., patient discharge automatically transitions bed to `DIRTY`).
  - `FEAT-HSO-13`: **Bed Readiness Notification**: Notify bed allocation managers and clinical units when a cleaned bed achieves `READY` status.

7. Clinical Resource Management (`Harmonia-Relevant`)

- **7.1 Specialized Equipment Coordination**
  - `FEAT-HSO-14`: **Mobile Clinical Equipment Tracking**: Track physical location and allocation status of shared clinical equipment (infusion pumps, ventilators, mobile telemetry).

8. Service Capacity Management (`Harmonia-Relevant`)

- **8.1 Facility Demand & Capacity Telemetry**
  - `FEAT-HSO-15`: **Operational Capacity Metric Aggregation**: Aggregate operational capacity telemetry (occupancy rates, wait times, diversion status) across facilities.

11. Mobile Staff Management (`Harmonia-Relevant`)

- **11.1 Mobile Worker Dispatch & Safety**
  - `FEAT-HSO-16`: **Mobile Worker Task Dispatch**: Dispatch community care tasks and urgent visits to roving clinicians' mobile devices.
    - **Architectural scope qualification (2026-10-09 human adjudication)**: Dispatch here concerns integration with external/business mobile work-management mechanisms: exchanging externally established assignment, dispatch and operational progression/status information with applicable worker/context information. Harmonia does not thereby determine or manage clinical work, clinical handover, clinical allocation or EMR worklists. External "work/task" vocabulary does not redefine Harmonia synthetic Task. The Feature name/identifier are retained; the proposed name `MobileWorkTaskManagementIntegration` requires separate human confirmation. See the Business integration boundary.
  - `FEAT-HSO-17`: **Mobile Visit Status Tracking**: Ingest arrival, visit progression, and safety check-in milestones from roving staff.

12. On-Call Management (`Harmonia-Relevant`)

- **12.1 On-Call Directory Resolution**

- FEAT-HSO-18: **Active On-Call Provider Resolution**: Resolve currently active on-call specialists, registrars, and operational managers for a given specialty and hospital at any point in time.

13. Work Allocation & Dispatch (`Harmonia-Core`)

- **13.1 Operational Work Assignment & Supervision**
  - FEAT-HSO-19: **Work Item Instantiation**: Instantiate operational units of work from clinical triggers (e.g., bed cleaning required, specimen transport required).
  - FEAT-HSO-20: **Worker Matching & Allocation**: Match work items against available staff roles, credentials, and locations based on allocation policies.
  - FEAT-HSO-21: **Work Dispatch Delivery**: Deliver work orders to target workers, devices, or team queues with technical and operational acknowledgement capture.
  - FEAT-HSO-22: **Work Progress Oversight & Escalation**: Monitor work progression against time-to-completion thresholds, escalating stalled or unacknowledged tasks.

16. Clinical Qualification Management (`Harmonia-Relevant`)

- **16.1 Credential Verification Checkpoint**
  - FEAT-HSO-23: **Operational Credential Check**: Validate that a practitioner possesses active credentials and required competency certs prior to dispatching specialized work.

18. Patient Transport (`Harmonia-Core`)

- **18.1 Patient Transfer Coordination**
  - FEAT-HSO-24: **Transport Request Ingestion**: Ingest internal (portering) and external (ambulance, inter-hospital) patient transfer requests with clinical mobility requirements.
  - FEAT-HSO-25: **Transport Dispatch & Progress Tracking**: Dispatch transport orders, tracking departure, transit, and destination arrival checkpoints.

19. Clinical Logistics Coordination (`Harmonia-Relevant`)

- **19.1 Specimen & Supply Courier Tracking**
  - FEAT-HSO-26: **Pathology Specimen Transport Tracking**: Track pickup, custody chain, transport conditions, and laboratory delivery of clinical specimens.

20. Discharge Management (`Harmonia-Core`)

- **20.1 Discharge Progression & Coordination**
  - FEAT-HSO-27: **Discharge Milestone Tracking**: Track clinical criteria-led discharge milestones (medications dispensed, transportation arranged, home care confirmed).
  - FEAT-HSO-28: **Discharge Readiness & Coordination Oversight**: Track and aggregate multidisciplinary discharge milestones, home care confirmations, and transit readiness across care teams.
  - FEAT-HSO-29: **Discharge Notification Dispatch**: Dispatch departure notifications, updated bed states, and finalized discharge summaries to external care providers upon physical patient exit.

View 5: Intrinsic / Shared Enablement

Intrinsic / Shared Enablement encompasses the horizontal, platform-wide system capabilities required to support all healthcare operating contexts.

RAD source listing (presentation only; no inferred ancestry).

1. Patient Clinical Record [Harmonia-Core]
2. Health Information Exchange [Harmonia-Core]
3. Health Information Access [Harmonia-Core]
4. Health Information Communication [Harmonia-Core]
5. Health Information Control [Harmonia-Core]
6. Clinical Knowledge Services [Harmonia-Relevant]
7. Clinical Collaboration [Harmonia-Core]
8. General Collaboration [Harmonia-Relevant]
9. Workflow & Activity Coordination [Harmonia-Core]
10. Calendar Management [Harmonia-Core]
11. Presentation Services [Harmonia-Core]
12. Information Design Governance [Harmonia-Core]

Critical Domain Distinctions: Intrinsic / Shared Enablement

Health Information Exchange: Four Interaction Models

1. **Submission**: *"I send this to you."* (Point-to-point push of an authored artifact).
2. **Retrieval**: *"I ask for this."* (Synchronous or asynchronous pull of specific records).
3. **Distribution**: *"I deliberately send this to identified recipients."* (Addressed fan-out delivery to designated systems or clinical actors).
4. **Syndication**: *"I publish information/change and subscribers receive matching interests."* (Publish-subscribe topic/criteria distribution).

Workflow & Activity Coordination: Three Work Units

1. **Work Order**: A unit of work where a **human is doing something** physical or operational (e.g., clean bed 4B, transfer patient to X-ray, draw blood sample).
2. **To Do**: A unit of work where a **human is reviewing, updating, or deciding**, including clinical approvals, sign-offs, and triage reviews.
3. **Task**: A unit of **non-human executable work** (internally a synthetic task) executed by platform automation, daemons, or services. *Execution Semantics*: Workflow Management provides the coordination and state engine through which these three activity types progress.

Clinical Collaboration Context

Clinical Collaboration is **Harmonia-Core** because Harmonia provides **Longitudinal Health Record (LHR)-aware collaboration spaces**. Clinicians can converse within secure multidisciplinary team rooms and query patient context directly (e.g., *"What were Fred's last three HbA1c results?"*). *Important Boundary*: A clinical statement made within a collaboration chat does **not** automatically become an authoritative clinical Observation or medical record entry. It remains collaborative discourse unless formally authored into a clinical document.

Presentation Services Decoupling

Presentation Services consumes established identity, security, and clinical context. Presentation does **not** own:

- Authentication or credential storage.
- Identity lifecycle or EMPI reconciliation.
- Consent decisions or access-policy evaluation.
- Authoritative clinical state preservation.

Detailed Capability & Feature Catalogue: Intrinsic / Shared Enablement

1. Patient Clinical Record (`Harmonia-Core`)

- **1.1 Longitudinal Clinical Record Governance**
  - `FEAT-ISE-01`: **Longitudinal Health Record (LHR) Assembly**: Assemble disparate encounters, diagnostic reports, medications, and clinical documents into a unified, vendor-neutral longitudinal timeline.
  - `FEAT-ISE-02`: **Active Clinical Record Access**: Provide low-latency, active distributed access to consolidated patient clinical summaries.
  - `FEAT-ISE-03`: **Durable Clinical Record Preservation**: Durably maintain governed, vendor-neutral longitudinal clinical representation and preservation with verifiable integrity controls.

2. Health Information Exchange (`Harmonia-Core`)

- **2.1 Multi-Model Information Exchange**
  - `FEAT-ISE-04`: **Direct Submission Ingestion & Delivery**: Support push submissions of clinical documents and messages across boundaries.
  - `FEAT-ISE-05`: **Clinical Record Retrieval Mediation**: Mediate query-based retrieval requests across external and internal repositories.
  - `FEAT-ISE-06`: **Addressed Clinical Distribution**: Fan out clinical events and diagnostic reports to designated external and internal recipients with per-destination delivery tracking.
  - `FEAT-ISE-07`: **Event-Driven Syndication**: Publish clinical and directory change events to subscribed systems matching interest filters.

3. Health Information Access (`Harmonia-Core`)

- **3.1 Governed Query & Search**
  - `FEAT-ISE-08`: **Federated Clinical Query**: Execute bounded, indexed queries across distributed clinical resources and directory graphs.
  - `FEAT-ISE-09`: **Access-Controlled Information Filtering**: Filter search results dynamically according to requesting user authorizations and patient consent directives.

4. Health Information Communication (`Harmonia-Core`)

- **4.1 Boundary Protocol Mediation**
  - `FEAT-ISE-10`: **Standards-Based Boundary Exchange**: Provide standards-conformant boundary exchange for healthcare messaging and resource representations.

5. Health Information Control (`Harmonia-Core`)

- **5.1 Policy & Security Governance**
  - `FEAT-ISE-11`: **Health Information Access & Consent Control**: Evaluate patient consent preferences, practitioner authority boundaries, and access control policies for managed clinical records.
  - `FEAT-ISE-12`: **Security Context Propagation**: Propagate tamper-evident, attributable security context across asynchronous processing boundaries.
  - `FEAT-ISE-13`: **Non-PHI Compliance Auditing**: Generate tamper-evident, verifiable audit records of security-significant events without emitting unmasked PHI.

6. Clinical Knowledge Services (`Harmonia-Relevant`)

- **6.1 Terminology & Reference Mapping**
  - `FEAT-ISE-14`: **Clinical Concept Resolution**: Resolve local clinical codes against standard terminologies (SNOMED CT, LOINC, ICD-10).
  - `FEAT-ISE-15`: **Bidirectional Terminology Mapping**: Translate between legacy local code systems and canonical national healthcare profiles.

7. Clinical Collaboration (`Harmonia-Core`)

- **7.1 LHR-Aware Multidisciplinary Collaboration**
  - `FEAT-ISE-16`: **Care-Team Clinical Collaboration Coordination**: Coordinate secure multidisciplinary clinical communication and consultation exchange bound to patient encounters or clinical topics.
  - `FEAT-ISE-17`: **In-Conversation LHR Query Resolution**: Resolve clinical data queries within an authorized collaboration context, presenting verified clinical summaries directly to care team members.
  - `FEAT-ISE-18`: **Zero-PHI Collaboration Metadata Governance**: Ensure collaboration channel titles, topics, and aliases use opaque identifiers free of patient names, MRNs, or sensitive details.

8. General Collaboration (`Harmonia-Relevant`)

- **8.1 Operational Messaging**
  - `FEAT-ISE-19`: **Operational Team Chat Enablement**: Provide secure instant messaging channels for departmental, administrative, and facility support staff.

9. Workflow & Activity Coordination (`Harmonia-Core`)

- **9.1 Multi-Work-Unit Orchestration**
  - `FEAT-ISE-20`: **Work Order Progression**: Manage execution states and dispatch of human operational work orders.
  - `FEAT-ISE-21`: **To Do Decision & Approval Coordination**: Coordinate human clinical review items, sign-offs, and approval workflows.
  - `FEAT-ISE-22`: **Synthetic Task Orchestration**: Execute non-human, automated platform activity units and sequence orchestration.
  - `FEAT-ISE-23`: **Workflow Timeout & Escalation Oversight**: Supervise operational deadlines, executing automated timeout notifications and supervisory escalations.

10. Calendar Management (`Harmonia-Core`)

- **10.1 Operational & Clinical Calendar Sync**
  - `FEAT-ISE-24`: **Schedule Projection Integration**: Project clinician rosters, on-call schedules, and facility operating calendars into operational views.
  - `FEAT-ISE-25`: **Temporal Event Correlation**: Correlate incoming clinical events with scheduled appointments and operational calendar milestones.

11. Presentation Services (`Harmonia-Core`)

- **11.1 Decoupled Human Interaction & Dashboards**
  - `FEAT-ISE-26`: **Longitudinal Clinical & Operational Presentation**: Deliver contextual clinical timelines, directory administration views, and operational management dashboards.

12. Information Design Governance (`Harmonia-Core`)

- **12.1 Canonical Semantics & Schema Governance**
  - `FEAT-ISE-27`: **Information Standards & Semantic Governance**: Govern canonical clinical data definitions, terminology bindings, and exchange profiles across regional systems.
  - `FEAT-ISE-28`: **Semantic Conformance Verification**: Verify inbound and outbound representations against published semantic rules and conformance constraints.

Health Service Assurance: Approved Business Enabling Capabilities

REQ-FND-005 — Independent Assurance of Governed Activity motivates BC-18 — Health Service Assurance. BC-18 requires the three Business Enabling Capabilities defined here. They are collaboratively realised by the approved contributions of existing Enterprise Capabilities and EC-14 — Service Guardian, as recorded in the contribution matrix.

These are Strategy capabilities and semantic boundaries. Their presentation does not create a sixth contextual view, allocate them to an existing view, establish CT1 / CT2 / CT3 ancestry, or derive Business Functions, Services, Processes, Features or assurance Information Concepts.

Assurance Design

- **Definition**: The capability to determine and define how satisfaction of a governed requirement, constraint or expected behaviour will be assured, including its assurance disposition, applicable criteria, required evidence and evaluation expectations.
- **Fundamental Question**: **How will satisfaction of this requirement, constraint or expected behaviour be assured?**
- **Boundary**: Assurance Design is a peer concern to Behaviour Design and Failure/Recovery Design when implementing governed behaviour. This relationship establishes design concerns, not additional capability entries, application design or implementation specifications.

Assurance Criteria Management

- **Definition**: The capability to establish and manage reusable, governed and temporally identifiable assurance criteria through which subjects may be evaluated.
- **Scope**: Criteria may express governance rule boundaries, quality constraints, performance expectations, compliance criteria, temporal criteria, evidence requirements or completeness expectations. These are examples, not an exhaustive taxonomy.

- **Boundary**: Criteria management does not own or replace the authoritative governance requirement from which a criterion may derive. Not every assurance criterion is required to derive from formal Governance. Criteria may have their own lifecycle, version and temporal applicability.

  Assurance Criteria may operationalise a governing requirement for evaluation, but SHALL NOT silently alter, weaken, strengthen or replace the authoritative requirement from which they derive.

Governed Assurance

- **Definition**: The capability to independently evaluate a governed subject against applicable assurance criteria through the assembly and evaluation of sufficient trustworthy evidence, establishing explicit findings, exceptions and assurance conclusions.
- **Fundamental Question**: **What can we independently establish about this subject against these criteria from the available trustworthy evidence?**
- **Assurance Semantics**: Establishing the subject/context of assurance, applicable assurance criteria, evidentiary association, assessment, adjudication, findings, exceptions and conclusions.
- **Boundary**: Governed Assurance does not manage, remediate, assign, delegate, escalate operationally or complete the subject activity. It may manage its own assurance activity; findings may inform operational response without transferring responsibility for that response.

Assurance Disposition

Every implemented behaviour has an assurance aspect. Every governed obligation, functional requirement, non-functional requirement or other defined implementation requirement SHALL have an explicit assurance disposition identifying how its satisfaction is assured.

The approved disposition concepts are:

- **Generic Assurance sufficient**
- **Existing Specific Assurance applies**
- **Existing Assurance must be extended**
- **New Specific Assurance required**
- **Not Assurable**
- **Not Worth Assuring**

These are architectural semantics, not an information model or software enumeration. **Not Assurable** does not mean the requirement is not required. **Not Worth Assuring** does not mean the requirement is optional or unimportant. Both are explicit governed assurance decisions, not absence of assurance configuration. An explicit disposition does not require specific assurance of every individual activity, information item or outcome; REQ-FND-005 applies where assurance is required.

Generic Processing Assurance and System Operations

Harmonia-managed workflow behaviour SHALL be subject to applicable generic processing assurance. This concerns business-level processing behaviour above and beyond System Operations. Potential concerns include progression, completion, timeliness, expected processing pathway, failure behaviour, recovery behaviour, outcome, and aggregate or pattern behaviour. This is not a final exhaustive criteria catalogue.

System Operations asks whether the technical machinery is operating. Generic processing assurance asks whether governed business behaviour is operating within expected boundaries.

Operational Health ≠ Processing Assurance.

Failure and recovery are part of the expected behavioural space of the activity being executed. The activity/Praxis remains responsible for handling its own expected failure and recovery behaviour. Assurance independently establishes whether normal, failure and recovery behaviour remain within applicable expectations. This responsibility boundary does not allocate assurance execution to Praxis or another solution construct.

EC-12 Operational Assurance retains its established resilience, concurrency, exception/recovery and telemetry responsibilities and may contribute directly to generic operational/processing assurance. EC-14 Service Guardian supplies the distinguishing assurance-specific semantics; neither replaces System Operations or the other Enterprise Capability.

Assurance Evidence Assembly and Information Ownership

Assurance Evidence Assembly is the evidence-assembly concern within Governed Assurance; this statement does not establish a fourth Business Enabling Capability or a downstream Function, Service or Information Concept.

Underlying information used as assurance evidence remains owned and managed by the capability responsible for that information. Assurance owns the contextual association that particular information constitutes evidence for a particular assurance purpose. Being evidence is contextual, not an intrinsic universal property of source information.

Evidence may be associated through reference, linkage, aggregation, representation, mirroring or derivation. These possibilities do not prescribe technical storage or replication. An association may identify a particular version, state or temporal context of the underlying information.

> **Evidence inclusion may be temporally and version-specific. Association of information as assurance evidence SHALL be capable of distinguishing the particular information state relevant to the assurance activity from the information's current state.**

Temporally Appropriate Evidence and Criteria

Applicable assurance criteria may themselves be versioned and temporally applicable. A conclusion may depend both on the information state relevant to the evidence and on the criterion state applicable to the subject at the relevant time. Assurance evaluates temporally appropriate evidence against temporally appropriate criteria. No persistence, active-state, replication or retention mechanism is derived here.

Assessment, Adjudication and Explicit Insufficiency

Concern	Fundamental question	Semantic boundary
Assessment	What does this evidence demonstrate with respect to the applicable criterion?	May supply quantitative, qualitative, compound, temporal or confidence-bearing information.
Adjudication	What assurance finding or conclusion follows from that assessment?	Establishes the finding or conclusion from assessed detail; it is distinct from assessment and operational response.

Assurance SHALL NOT be reduced universally to PASS / FAIL / UNKNOWN. Insufficient evidence is not equivalent to either satisfaction or non-satisfaction. Under REQ-FND-005, where available evidence cannot establish the required assurance conclusion with sufficient confidence, that inability and evidence insufficiency SHALL remain explicit.

An assurance activity may execute correctly while lacking sufficient evidence to establish a conclusion about its subject. This is distinct from an indeterminate execution outcome or execution failure. REQ-FND-004 and AX-15 continue to govern uncertainty about operational state, effect or outcome, including the assurance activity's own execution where applicable.

Assurance Independence

Independent Assurance is performed through an independently governed activity whose progression and assurance conclusion are not controlled by the activity or mechanism responsible for performing or managing the subject being assured.

Independence does not require a different organisation, person, application, Kubernetes cluster or database, an external auditor, or physical separation. The subject activity may supply evidence but SHALL NOT solely determine, suppress, manufacture or retrospectively alter its own assurance conclusion.

> **Evidence dependency does not compromise assurance independence; control dependency does.**

Assurance may manage its own assurance activity without assuming management of the subject activity. No technical or organisational separation is prescribed by this boundary.

Clinical Services Delivery Assurance Boundary

Health Service Assurance SHALL NOT be represented as Clinical Services Delivery Assurance. Harmonia does not thereby assume responsibility for assuring clinical judgement, professional practice, clinical adequacy of care, or delivery of clinical services by healthcare practitioners or healthcare organisations.

Information concerning clinical service delivery MAY be the subject of governed assurance where an explicit applicable requirement establishes that concern. This does not transfer responsibility for Clinical Services Delivery Assurance to Harmonia. Capability 06 — Clinical Quality, Safety & Improvement retains its established clinical quality/safety responsibilities.

> **Harmonia may assure facts and behaviour concerning clinical activity where explicitly required; it does not thereby assure the clinical adequacy of care.**

Strategy Derivation Boundary

The approved derivation view stops at collaborative Enterprise Capability realisation. No strategic logical or application component is allocated these capabilities or EC-14. No Dokimasia, Ponos, Praxis, Pragma, Digital Twin, Mneme, Mnemosyne, Calliope, Iris, Pylai or other implementation relationship is established. Business Architecture, Information Architecture and solution derivation remain subsequent activities.

Downstream Progression

The Business Enabling Capability catalogue and its atomic Features establish what systems must enable for the healthcare enterprise. These features serve as the empirical basis for deriving the reusable, technology-neutral platform capabilities:

- Enterprise Capabilities (EC-01 .. EC-14): The established EC-01 through EC-13 and approved EC-14 Service Guardian; the Health Service Assurance contribution model is established without adding Feature or component allocations.
- Health Service Assurance Derivation: REQ-FND-005 → BC-18 → the three approved Business Enabling Capabilities → collaborative Enterprise Capability realisation.
- Capability Tier Progression Model: Detailed vertical derivation rules and representative composition examples.
- ICT Foundation Lenses: Technical enablement considerations guiding technology realization.

11. Business Architecture Metamodel & Modelling Rules

RAD source: `docs/markdown/03-business-architecture/metamodel/business-architecture-metamodel.md`

The following is a presentation transcription of the governed source. Source catalogue layout and retained illustrations do not allocate tiers or establish additional identities. Equations wrap as text; diagram notation is textual.

Business Architecture Metamodel & Modelling Rules

1. Overview & Metamodel Hierarchy

The Harmonia Business Architecture metamodel defines the structural and behavioural relationships governing how healthcare capabilities are operationalised into functions, exposed services, stateful processes, and information responsibilities.

RAD diagram: textual source notation.

```
graph TD
    subgraph OWNER["Owning Business / Business Enabling Capability or Feature responsibility context"]
        subgraph BC["Business Capability"]
            BC1["BC CT1"] -->|Decomposes| BC2["BC CT2"]
            BC2 -->|Decomposes| BC3["BC CT3"]
        end
        subgraph BEC["Business Enabling Capability"]
            CT1["BEC CT1"] -->|Capability decomposition| CT2["BEC CT2"]
            CT2 -->|Capability decomposition| CT3["BEC CT3"]
            CT3 -->|Frames every Harmonia Feature| FT["FT Feature<br/>(Capability Specialisation)"]
        end
        BC -.->|Enabled-by / supported-by where established| BEC
    end
    OWNER -->|Delivers| FUNC["Business Function(s)"]
    OWNER -->|Provides to clients| SERV["Business Service(s)"]
    SERV -->|May use directly| FUNC
    SERV -->|May use| PROC
    FUNC -->|May require| PROC
    PROC -->|Chains| FUNC
    OWNER -->|Encompasses where progression is material| PROC["Business Process(es)"]
    OWNER -->|Establishes| INFO["Business Information Responsibility"]
```

The diagram distinguishes two Capability hierarchies and preserves the permissible behavioural responsibility contexts. Business Capabilities describe organisational abilities; Business Enabling Capabilities describe reusable enabling abilities Harmonia provides or supports. Enablement/support is not decomposition or implementation equivalence, and the aggregate diagram establishes no individual BC↔BEC mapping. Business Capability decomposition does not terminate in Harmonia Features. Every fully reconciled Harmonia Feature requires complete **Business Enabling CT1 → CT2 → CT3** ancestry. Functions, Services and Processes may still anchor directly at their established owning Capability or Feature; they do not require an intervening Feature. Service, Function and Process form a graph rather than a mandatory hierarchy. A Function need not have either a Service or a Process.

1.1 Capability Decomposition and Terminology

The following terminology SHALL be used canonically:

Term	Meaning
Layer / Architectural Layer	Architectural layering within the TOGAF / ArchiMate architecture model.

Term	Meaning
Capability Tier	Hierarchical decomposition within a Capability Model.
Derivation / Derivation Progression	Progression between different architectural constructs or models. A Derivation Stage is a stage in that progression.
CT1	Capability Tier 1: establishes the broad Capability frame within Harmonia's architectural scope.
CT2	Capability Tier 2: decomposes the CT1 frame into constituent Capabilities.
CT3	Capability Tier 3: establishes the finest Capability frame required by Harmonia. Within the Business Enabling hierarchy, CT3 provides the context within which Harmonia Features are defined.
FT	Feature: expresses required system-enabled behaviour within its owning Business Enabling CT3 Capability context.

`Layer≠CapabilityTier≠DerivationStage`. `L1`, `L2` and `L3` are deprecated as canonical capability classifications.

Harmonia SHALL use the fixed three-tier Capability decomposition:

RAD source listing (presentation only; no inferred ancestry).

Business Capability CT1 → CT2 → CT3

enabled-by / supported-by

Business Enabling Capability CT1 → CT2 → CT3 → FT Feature

Every Harmonia Feature SHALL be immediately framed by a **Business Enabling CT3** Capability. Every established CT3 SHALL have a CT2 parent and every CT2 a CT1 parent within the same Capability category. Capability tiers SHALL NOT be skipped. Complete Business Enabling CT1 → CT2 → CT3 ancestry is required for a fully reconciled Feature; direct CT1 → FT and CT2 → FT framing SHALL NOT be used. Business Capability CT3s do not require or own Harmonia Features. An intentionally undecomposed BC branch does not indicate missing Feature framing.

> **A Feature cannot compensate for incomplete Capability decomposition. If Harmonia requires a Feature, the CT3 Capability that frames that Feature must first be identified. Failure to identify a meaningful CT3 indicates incomplete upstream Capability modelling, not permission to skip the tier.**

Complete decomposition is a modelling obligation, not authority to fabricate architectural elements. CT3 Capabilities SHALL NOT be manufactured merely to satisfy structural symmetry. Where an existing branch contains a Feature without an established CT3 frame or complete ancestry, the Feature, its established responsibility and local identifier SHALL be preserved, and its Capability ancestry/decomposition SHALL be recorded as requiring reconciliation under AX-17. It SHALL NOT be presented as fully reconciled or silently reattached to an invented owner. Established partial relationships remain valid evidence until separately adjudicated.

A Feature remains a finer-grained **Capability Specialisation** within Business Enabling responsibility and inherits Capability semantics. This retained classification does not make FT another Capability Tier, a Business Capability child or an arbitrary-depth frame. CT1 → CT2 and CT2 → CT3 denote same-category Capability decomposition; Business Enabling CT3 provides the mandatory Feature frame. BC↔BEC support relationships do not supply missing ancestry in either hierarchy.

CT3 is Harmonia's intentional Capability decomposition boundary. Harmonia's Capability Model is bounded to the level required to derive and govern Harmonia; it does not attempt to model the complete Enterprise Architecture of a hospital, health service provider, health department or other healthcare enterprise. Harmonia SHALL NOT define CT4, CT5 or deeper Capability tiers. Further detail is not authority to extend the Capability hierarchy.

Established higher-tier ancestry SHALL be represented in a lower-tier Capability's Canonical ID. Every Feature's complete structural Canonical ID SHALL include its CT1, CT2 and CT3 ancestry. An unidentified ancestor SHALL NOT permit omission of that tier; required but unresolved ancestry SHALL leave the Canonical ID unresolved, as specified in §8.

The five-stage Domain02 derivation progression SHALL NOT be reinterpreted as CT1 / CT2 / CT3 / FT. Enablement, derivation and contextual-view membership do not, by themselves, establish a Capability Tier assignment.

The approved fixed-decomposition decision record records the rationale and superseded permissive position. It changes the target metamodel, not the reconciliation status, tier allocation or identity of existing catalogue elements. K9's uncertainty and evidence rules remain controlling.

1.2 Hierarchy Naming & Uniqueness Rules

1. **Global Uniqueness**: Capability names across CT1, CT2 and CT3 are globally unique across the entire Harmonia architecture.
2. **Feature Scoping**: Feature names must be unique within their owning CT1 Capability hierarchy. An unresolved CT1 ancestor SHALL be recorded as unresolved rather than assigned speculatively.
3. **No Disconnected Elements**: Business Functions, Business Services, and Business Processes must **never** be modelled as free-standing global catalogues disconnected from an owning Capability or Feature context.

2. Capability & Feature Semantics

A Capability or Feature establishes:

- **Context & Scope**: Sets the boundary of healthcare operational relevance.
- **Architectural Responsibility**: Establishes unambiguous ownership of behaviour and state.
- **Business Functions**: Defines the internal behaviour delivered within the boundary.
- **Exposed Business Services**: Declares behaviour made available to external or cross-capability consumers.
- **Business Processes**: Encompasses progression lifecycles where activity state tracking is material.
- **Information Responsibility**: Owns the conceptual information created or governed by its functions.
- **Dependencies**: Explicitly records consumption of services provided by other capabilities.

3. Function and Service Semantics

Domain03 SHALL NOT establish a Feature identity that Strategy has not established. A Strategy Feature may be associated with Business behaviour only where that behaviour materially realises the semantic responsibility expressed by the Feature. Shared subject matter, terminology, information, Actor, Capability context or clinical purpose is insufficient. Where semantic realisation is not established, preserve the Feature, Function, Service and established owning Capability, and record **Feature association not established** without assigning a replacement Feature.

The following are **Harmonia architecture definitions**, reconciled by the 2026-10-13 B1 task decision. They apply across architectural views and need not conform to the exact semantics of ArchiMate or another modelling framework. Frameworks may represent them where useful but SHALL NOT redefine their Harmonia meaning. Domain03's owning Capability/Feature and exposure rules continue to qualify Business behaviour; they do not replace these definitions.

3.1 Business Function Rule

> A **Function** is a mechanism that moves the state of information, or the state of a capability concern as a whole, from one status to another. A Function may operate

over a small, medium or large concern. Its architectural meaning derives from what it does to or within the capability, not from what it exposes externally.

A Function answers **what change is effected?** Function accountability concerns achievement of the defined state transition. Function size does not determine whether something is a Function.

> **A Business Function represents behaviour delivered within the responsibility boundary of its owning Capability or Feature.**

A Business Function describes *what* the capability does internally to discharge its architectural mandate. A Business Function may operate upon:

- information owned by its Capability / Feature;
- information obtained through Services exposed by another Capability / Feature;
- information used under an explicit cross-capability dependency.

Therefore, **ownership of a Function does not imply ownership of every information item upon which that Function operates**.

3.2 Business Service Exposure Rule

> A **Service** is a mechanism through which a client interacts with a capability to request change, request information, or be informed of change. A Service represents the interface between the capability and its client and operates according to a Service Contract. A Service may use one or more Functions and/or Processes to fulfil that contract.

A Service answers **what can the client ask for, obtain, or be informed about?** Service accountability concerns fulfilment of the **Service Contract**. A Service may use Functions directly, Processes, or both; a Function need not be exposed through a Service.

> **When Function behaviour is exposed for consumption outside the owning Capability or Feature responsibility boundary, that behaviour is exposed through a Business Service.**

RAD source listing (presentation only; no inferred ancestry).

> Capability A (Owner) provides Service X used by Capability B (Client)
>
> fulfilled using
>
> Function(s) and/or Process(es)

3.3 Ownership and Non-Equivalence Invariants

1. **Ownership Preservation**: When Capability A exposes `FunctionX` as `BusinessServiceX`, and Capability B consumes `BusinessServiceX`, **Capability A retains sole architectural ownership of Function X and its underlying state**. Capability B is merely a consumer.
2. **No 1:1 Mechanical Generation**: A Business Service is **not** created mechanically for every Business Function. A Service is modelled *only* when an identifiable consumer exists outside the owning responsibility boundary.
3. **Consumption Scope**:
   - **Harmonia-Internal Consumption**: Services consumed by other Harmonia capabilities (e.g., Service Administration consuming Person Identifier Resolution from Entity Management).
   - **Enterprise/External Consumption**: Services consumed by external healthcare participants, client systems, or national registries (e.g., Referral Submission exposed to external GP practices).

3.4 Significance-Driven Relationships and Typed Dependencies

Role, Actor, Function, Service, Interaction, Collaboration and Process relationships SHALL be established where they materially explain responsibility, participation, exposure, collaboration or Business progression. Their absence alone does not establish missing architecture. Exhaustive Function-to-Role, Function-to-Service, Interaction-to-Service or Actor-to-Function matrices are not required.

A Service dependency requires consuming behaviour that needs behaviour actually exposed by that Service. Related information or subject matter is insufficient. Capability dependency, Service consumption, Interaction participation and Collaboration involvement SHALL retain their distinct types and meanings. Descriptive or abbreviated references do not create aliases. An unestablished element or relationship SHALL remain explicitly unresolved; no replacement dependency is inferred.

3.5 Clinical and Operational Privilege

Clinical Privilege concerns whether a practitioner is authorised/credentialed to undertake applicable clinical activity. **Operational Privilege** concerns whether an actor is authorised to undertake applicable operational activity. They are distinct responsibilities; an activity may require both.

Practitioner Role information may contextualise or contribute to a privilege determination, but Practitioner Role Resolution establishes neither Clinical Privilege nor Operational Privilege. These distinctions do not allocate new Roles, Services or policy mechanisms.

3.6 Business Responsibility and Reusable Composition

Business Architecture SHALL describe responsibility meaningful to the Business at its declared architectural layer. Shared downstream machinery does not establish a shared Business Function. In service-delivery operational contexts, **Capacity Management** concerns understanding and managing available, committed and utilised service capacity. Contexts may have materially different capacity semantics. A generalised operational-state capture mechanism is a downstream derivation concern and SHALL NOT replace those Business responsibilities. The HSO capacity responsibility boundary distinguishes context-specific capacity from resource state and higher-order consumers.

A domain-specific Business Enabling Feature MAY be realised through composition of established reusable capabilities and configured workflow/Praxis without a correspondingly specialised Business Function. The composition must sufficiently express the required responsibility; clinical purpose or setting alone does not establish another Harmonia information-management responsibility. Do not create a Feature-to-Function relationship where only a composition-level sufficiency decision is established.

3.7 Clinical Work Integration Boundary

Harmonia may integrate and coordinate around clinical work; it does not thereby manage clinical work. It may exchange work information, receive externally established requirements, route operational activity, observe progression, coordinate Harmonia-managed consequences and apply established qualification, privilege and policy constraints. It SHALL NOT thereby determine clinical work, manage clinical handover, acquire clinical allocation or decision authority, own clinical worklists or assume EMR workflow ownership. Information assembly for handover is distinct from performing or managing the handover.

Clinical facts retain their originating authority when consumed by patient-flow, capacity or discharge coordination. Clinical progression and operational patient progression SHALL remain distinct. A clinical/professional clearance may affect discharge readiness without the operational consumer acquiring authority to establish that clearance. Clinical Privilege and Operational Privilege retain §3.5's distinct meanings.

Harmonia terminology remains **Work Order = human doing**, **To Do = human review/ update/approval**, and **Task = synthetic task**. External work/task vocabulary does not redefine Harmonia Task. Coordination of a To Do supports an authorised review or approval; it does not confer clinical-work management authority.

4. Business Process Semantics

> A **Process** is an ordered chain of Functions used to realise a complex change. A Process provides a convenient mechanism for decomposing, structuring and managing change that would otherwise be difficult to understand or control as a single Function. A Function may itself require one or more Processes where the change for which that Function is responsible requires further decomposition.

A Process answers **how is a complex change decomposed and coordinated?** Process accountability concerns progression and coordination of its constituent Functions. Function and Process may be recursively composed:

RAD source listing (presentation only; no inferred ancestry).

> Function
>
> may require → Process
>
> Function
>
> Function
>
> Function
>
> may require → Process

A Business Process applies this meaning to progressing Business behaviour within an owning Capability or Feature responsibility boundary. There is no mandatory Service → Process → Function hierarchy.

4.1 Process Justification Rule

> **Model a Business Process where ordered Function decomposition is material to understanding, structuring or managing a complex Business change and its progression.**

Meaningful states, dispositions and outcomes may justify that decomposition. Not all Functions require a Process. Search, lookup, mapping, verification or presentation labels alone neither establish a Process nor exclude one where the bounded change requires decomposition. Primary Information traceability remains with the responsible Functions; Process-level aggregation does not replace it. Existing named Business Processes and responsibility boundaries are preserved; this rule creates no new catalogue entry.

4.2 Cross-Capability Process Guardrail

If a proposed Business Process appears to span multiple Capability responsibility boundaries:

1. Examine whether the process has been scoped incorrectly (conflating independent sequential activities with a single process); or
2. Examine whether an architectural responsibility is missing or misassigned.

Cross-capability processes must never be used to erase or blur Capability ownership boundaries. Cross-boundary coordination is achieved via event notifications and service invocations between bounded capabilities.

4.3 Behaviour and Process Consistency

A Business Process may elaborate progression for a defined activity within its owning Capability or Feature without reproducing the Behaviour stage list. It SHALL preserve established responsibility, ownership, authority boundaries and supported obligations. Scoped applicability, compound checkpoints and alternative dispositions SHALL remain explicit; additional Process detail SHALL NOT silently become a universal requirement or redefine owning responsibility.

Omission of a Process checkpoint from a Behaviour summary does not invalidate that checkpoint; omission of Behaviour responsibility from a Process does not remove that responsibility. Stage-count equality is not required. Greater Process detail does not override established responsibility.

4.4 Illustrative Progression and Material Outcome Uncertainty

Illustrative progression SHALL NOT imply an authoritative transition model. Only established states/transitions may be normative. Where alternatives, terminal behaviour or post-completion relationships remain unresolved, both the surrounding text and the affected diagram SHALL visibly identify the uncertainty.

Where Harmonia manages or governs an activity with a materially significant outcome and available evidence cannot establish that outcome, the uncertainty SHALL remain explicit. Absence of acknowledgement, response, observation or evidence does not by itself establish success or failure. This follows REQ-FND-004 and AX-15; it creates no universal INDETERMINATE Process state or downstream execution-state machinery.

5. Cross-Cutting Responsibility Rule

Harmonia establishes an essential architectural guardrail to maintain component cohesion and prevent boundary erosion:

> **Cross-cutting concern does not imply cross-cutting responsibility. A concern becomes cross-cutting because Capabilities depend upon Services or Functions delivered by other Capabilities whose responsibilities remain explicitly bounded.**

Concrete Guardrails:

- **Security & Control**: Security does not become partly owned by every capability that consumes security services. Authorisation evaluation and security context propagation are owned exclusively by *Health Information Control*.
- **Semantic Governance**: Terminology and canonical model definitions do not become owned by capabilities that use them. They are owned by *Information Design Governance* and *Clinical Knowledge Services*.
- **Workflow & Activity Coordination**: Coordinating tasks and to-dos does not grant ownership of the underlying clinical or operational activities. Coordination semantics are owned by *Workflow & Activity Coordination*.
- **Presentation**: Presentation capabilities do not acquire ownership of the clinical information they render.
- **Information Exchange**: Transporting or mediating messages across networks does not grant ownership of the clinical payloads transported. Exchange state is owned by *Health Information Exchange*, while clinical document ownership remains with *Clinical Record Administration*.

6. Business Information Responsibility

> **Business information ownership derives strictly from Capability responsibility and the Functions/Processes through which that responsibility is discharged.**

- **No Ownership Transfer**: Access, use, transformation, transport, search, presentation, caching, persistence, or coordination of information does not transfer architectural ownership.
- **Pure Business Semantics**: Business information concepts must retain authentic clinical/business definitions. They must **never** be prematurely translated into:
  - FHIR Resource structures (e.g., `Patient`, `Observation`, `Task`);
  - Database schemas, tables, or DDL;
  - JSON schemas, DTOs, or wire payloads;
  - Java classes or programming interfaces.

Domain04 formalises conceptual information meaning, semantic relationships and responsibility traceability. Realised representations are downstream: application logical/software representation in Domain05; exchange contracts, interoperability profiles, wire payloads and transformations in Domain06; physical platform/runtime/product/deployment realisation in Domain07. Persistence realisation may involve Domain05 and Domain07; a more precise allocation is not established. Representation does not define upstream business or conceptual meaning.

6.1 Management, Originating Authority and Historical Truth

Managing, preserving, governing access to, communicating, contextualising or maintaining the history of information SHALL NOT imply originating authority for the represented facts.

Within Harmonia's operational boundaries, Harmonia may maintain durable, immutable, append-only historical truth concerning what it received, knew, asserted, managed, decided, communicated or did. Subsequent correction, supersession or changed knowledge SHALL NOT retrospectively alter that historical record. Current authoritative state and historical operational truth remain distinct. This does not make every transient observation durable evidence or weaken the active/durable-state separation.

6.2 Business Terminology and Context

Interpret an element according to its declared architectural layer and Business meaning. Cache, index, routing, topic, session and similar terms do not prescribe a technology or product merely because those words also have technical uses. Preserve clear Business meanings; implementation structures or bindings require explicit downstream authority.

Where Healthcare Service context is material to an activity's meaning, authority, coordination, progression or accountability, that context SHALL remain identifiable through relevant Business behaviour and information responsibility. Organisation, Location, Practitioner or Role association does not imply that context. The obligation is contextual, not universal, and defines no representation, identifier structure, persistence mechanism, FHIR representation or implementation binding.

7. Qualified Architectural Reference Grammar

To unambiguously express participants, their acting capacity, and situational context without inventing transient metamodel entities, Harmonia uses the **Qualified Architectural Reference Grammar**:

RAD source listing (presentation only; no inferred ancestry).

```
<EntityType>#<Entity>-as-<Role>[#<ContextQualifier>]
```

7.1 Grammar Structure

- `<EntityType>`: A rendering of the canonical Business Actor category (`Person`, `Group`, `Organisation`, `OrganisationalUnit`, `Government/RegulatoryBody`, `System`, `Device`).
- `<Entity>`: The specific business instance identifier or name.
- `<Role>`: The canonical Business Role representing the stable business/functional capacity being fulfilled.
- `[#<ContextQualifier>]`: *(Optional)* The contextual function or situational qualifier in a specific interaction or collaboration.

Qualified references resolve to canonical architectural identities. For compact rendering, CamelCase SHOULD be used where possible: Information Supplier → `InformationSupplier`, Service Provider → `ServiceProvider`, Organisational Unit → `OrganisationalUnit`. This is syntactic rendering, not an alias or independent element. Preserve meaning and disambiguate where a compact expression could resolve to more than one identity; compact spelling alone SHALL NOT establish identity.

7.2 Canonical Examples

Qualified Expression	Actor Category	Entity Name	Fulfilled Role	Context Qualifier
`Organisation#ACTPathology-as-InformationSupplier`	`Organisation`	ACT Pathology	`InformationSupplier`	*(none)*
`Organisation#ACTHealth-as-ServiceProvider#ReferralSource`	`Organisation`	ACT Health	`ServiceProvider`	`ReferralSource`
`System#ACTPathologyLIS-as-InformationSupplier#DiagnosticResultSource`	`System`	ACT Pathology LIS	`InformationSupplier`	`DiagnosticResultSource`
`Person#Fred-as-Patient`	`Person`	Fred	`Patient`	*(none)*
`Person#DrSmith-as-Clinician#Reviewer`	`Person`	Dr Smith	`Clinician`	`Reviewer`
`OrganisationalUnit#FinanceDirectorate-as-InformationCustodian`	`OrganisationalUnit`	Finance Directorate	`InformationCustodian`	*(none)*
`GovernmentRegulatoryBody#AHPRA-as-Regulator`	`Government/RegulatoryBody`	AHPRA	`Regulator`	*(none)*

7.3 Governance Guardrails

1. The **Role** identifies stable architectural/business capacity.
2. The **Context Qualifier** identifies situational nuance; it does **not** create a new Role.
3. This notation is a **reference grammar**, not a new architectural metamodel class.

8. Architectural Element Identity and Canonical Identification

8.1 Common Identity Properties

Every governed Business Architecture element SHALL have the following identity properties:

Property	Cardinality	Meaning
Element Type	Exactly one	The element's architectural semantic type.
Canonical Name	Exactly one	The current authoritative architectural name.
Canonical ID	Exactly one current value	The Canonical Architectural Identifier expressing current architectural responsibility context.
Name Alias	Zero or more	A retained alternative or historical name for the same element.
ID Alias	Zero or more	A retained non-canonical identifier resolving directly to the same current element.

These properties SHALL preserve the semantic distinctions between Capability, Feature, Business Function, Business Service and Business Process. Existing canonical-name uniqueness and ownership rules remain applicable.

This convention establishes the identification requirement; it does not assert that the existing architecture corpus already conforms. Where required ancestry or identifier allocation is not established, the Canonical ID SHALL remain explicitly unresolved. A placeholder or speculative identifier SHALL NOT be treated as an allocated Canonical ID.

8.2 Typed Canonical Identifier Grammar

The general form is `<architectural-context>.<element-identifier>`. A root CT1 Capability has no preceding context. For Capability, Feature, Business Function, Business Service and Business Process elements, the structural grammar is:

RAD source listing (presentation only; no inferred ancestry).

canonical-id =

capability-path

| feature-path

| capability-path ”.” behaviour-token

| feature-path ”.” behaviour-token

capability-path = ct1-token [”.” ct2-token [”.” ct3-token]]

ct3-path = ct1-token ”.” ct2-token ”.” ct3-token

feature-path = ct3-path ”.” feature-token

ct1-token = ”CT1-” <approved-local-code>

ct2-token = ”CT2-” <approved-local-code>

ct3-token = ”CT3-” <approved-local-code>

feature-token = ”FEAT-” <approved-feature-code>

behaviour-token =

”FN-” <approved-local-code>

| ”SV-” <approved-local-code>

| ”PR-” <approved-local-code>

Codes SHALL be approved, nonempty tokens without the context separator `..` This grammar establishes no code allocation or numeric width. Existing established Feature identifiers SHALL retain their canonical spelling, including `FEAT-EM-01`; `FEAT-EM01` SHALL NOT be substituted or assumed equivalent.

Capability tokens SHALL explicitly carry their Capability Tier type. Untyped forms such as `CT01`, `CT02` and `CT03` SHALL NOT be used as canonical Capability tokens. The FN / SV / PR prefixes identify behaviour types and SHALL NOT constitute additional Capability Tiers.

The following are syntax examples only; they SHALL NOT assign identifiers or Capability Tiers to existing architectural elements:

RAD source listing (presentation only; no inferred ancestry).

CT1-01

CT1-01.CT2-02

CT1-01.CT2-02.CT3-03

CT1-01.CT2-02.CT3-03.FEAT-EM-01

CT1-01.FN-01

CT1-01.CT2-02.PR-01

CT1-01.CT2-02.CT3-03.SV-01

CT1-01.CT2-02.CT3-03.FEAT-EM-01.FN-01

Every ancestor prefix in a current Canonical ID SHALL identify the corresponding current Capability or Feature in the established responsibility ancestry. `CT1-01.CT3-02` SHALL NOT be generated merely because CT2 ancestry has not been identified. Unresolved ancestry SHALL remain unresolved rather than being omitted or manufactured.

The shorter `capability-path` forms identify CT1 or CT2 Capabilities and permit direct behavioural anchoring; they SHALL NOT frame a Feature token. A Feature, or behaviour anchored beneath it, requires `feature-path` with complete Business Enabling CT1 → CT2 → CT3 ancestry. A syntactically valid path under a Business Capability does not establish valid Harmonia Feature ownership. These are grammar rules only and allocate no identifier to an existing element.

The common identity properties apply to other governed Business Architecture element types, but this decision does not establish their token vocabularies or structural contexts. Those SHALL NOT be inferred from this grammar. The Qualified Architectural Reference Grammar in §7 remains distinct and unchanged.

8.3 Behavioural Anchoring and Contextual Containment

Business Functions, Business Services and Business Processes MAY be anchored directly at any Capability Tier or Feature establishing responsibility for that behaviour. CT2, CT3 or FT elements SHALL NOT be invented merely to provide behavioural anchoring.

Canonical identifier containment SHALL express architectural responsibility context. It SHALL NOT redefine semantic relationships or Element Type, and SHALL NOT be interpreted as implementation containment. A Function beneath CT2 remains a Function; a Service beneath CT1 remains a Service; a Process beneath a Feature remains a Process. Function delivery, Service exposure and justified Process progression retain the rules in §§2–4.

The complete Canonical ID SHALL preserve contextual architectural standing in flat documents, generated diagrams, models, traceability matrices, cross-document references and AI contexts without depending on Markdown nesting. A flat representation retaining that context is permitted; disconnected ownership is not.

8.4 Rename, Relocation and Alias Rules

A Canonical Name change alone SHALL NOT change architectural identity or require a Canonical ID change. A previous name MAY be retained as a Name Alias for historical or search purposes.

The approved rename `ClinicalCredentialManagement→ClinicalQualificationManagement` preserves the existing practitioner professional / clinical qualification and competency responsibility. `ClinicalCredentialManagement` MAY be retained as a Name Alias only for that same element. It SHALL remain distinct from Domain03 `CredentialManagement`, whose separately established

responsibility concerns operational badges and physical access. The rename SHALL NOT establish equivalence or aliasing between those elements.

Where an element changes architectural responsibility context, its Canonical ID SHALL change to express current structural truth. The previous Canonical ID SHALL be retained as an ID Alias. This rule also applies to descendant elements whose responsibility paths change. The following relocation is a syntax example only:

RAD source listing (presentation only; no inferred ancestry).

Previous Canonical ID: CT1-01.CT2-02.FN-01

Current Canonical ID: CT1-01.CT2-02.CT3-03.FN-01

ID Alias: CT1-01.CT2-02.FN-01

An ID Alias SHALL:

- refer to the same architectural element;
- be searchable and resolvable;
- be non-canonical;
- resolve directly to the current architectural element, without requiring an alias chain;
- never be used for a new authoritative architectural reference.

Canonical IDs and ID Aliases SHALL occupy one globally unique, non-reusable identifier namespace. Once an identifier has referred to an element, canonically or as a historical Canonical ID, it SHALL NOT subsequently identify another element. An ID Alias SHALL NOT collide with a current Canonical ID or an ID Alias assigned to another element. All retained aliases SHALL continue resolving directly after subsequent relocations. New authoritative references SHALL use the current Canonical ID.

Name Aliases are names, not members of the identifier namespace. Neither Name nor ID Aliases SHALL establish equivalence merely from similar wording or nested behaviour. The approved G1 K5 rules remain controlling: finer-grained Functions or Services SHALL NOT be promoted to Features, or assigned a broader Feature's identity, solely to obtain traceability symmetry.

8.5 Machine-Validation and Generation Intent

This convention is intended to support future automated architectural navigation, diagram generation, model generation, traceability matrices, cross-document references and AI-assisted architecture interpretation. It SHALL permit validation of at least:

- Canonical ID uniqueness;
- ID Alias uniqueness;
- Canonical ID / ID Alias collisions;
- ancestor existence and complete, correctly typed ancestry;
- valid element-type suffix or terminal token;
- valid Capability / Feature responsibility context;
- resolvable references;
- direct, unambiguous alias resolution;
- orphaned identifiers.

This decision implements no validators, schemas or classes. Generation intent does not require the exhaustive capability matrices excluded by Domain02's modelling position.

8.6 Approved K9 Derivation and Uncertainty Rules

Approved G1 K9 found no affected legacy owner assignment sufficiently validated to authorise mechanical conversion to CT1 / CT2 / CT3. Established identities and responsibilities remain valid; affected Capability Tier, complete ancestry and root status remain unresolved.

Unresolved Capability Tier or ancestry SHALL NOT prevent downstream derivation from an otherwise established owning Capability or Feature and its relevant responsibility. Downstream

traceability SHALL reference that element without manufacturing structural ancestry. Document structure, catalogue grouping, numbering, indentation and decomposition presentation SHALL NOT by themselves establish Architectural Element identity.

`ClientAdministration→PersonIdentity→IdentifierResolution[FT/FEAT-EM-01]` is an established partial responsibility chain and SHALL be preserved. Its full structural Canonical ID remains unresolved until actual Capability Tier ancestry and identifier allocation are architecturally established. The former incorrect L3 example SHALL NOT supply that ancestry.

The fixed model in §1.1 does not retrospectively validate legacy L1 / L2 / L3 assignments or convert a known owner into CT3. Existing Features with incomplete ancestry remain valid architectural evidence requiring Capability decomposition reconciliation; downstream derivation from their established responsibility does not declare them structurally reconciled. Missing CT3 frames SHALL be separately established before subsequent Feature definition, without using catalogue presentation to manufacture them.

These rules preserve AX-01/AX-04 business meaning, AX-14 distinctions, AX-15 uncertainty and AX-17 architectural authority and explicit uncertainty. They allocate no structural IDs, manufacture no ancestry and do not authorise Package2 Information Family derivation.

The subsequent R1/R2 Business Capability reconciliation explicitly establishes its stated BC tiers/parents independently of catalogue indentation. It does not resolve any affected historical BEC ancestry, convert any of the 69 enabling descriptions into Capabilities, frame the 137 Features or allocate Canonical IDs. K9 continues to govern all unestablished dimensions.

9. R1.x/R2.x Semantic Sufficiency Boundary

The human architectural adjudication of 2026-10-09 establishes semantic sufficiency rather than graph density. Domain03 must express the Business behaviour required by Strategy without mirroring Strategy's structure. Complete identifier allocation is not a prerequisite for semantic completeness; deterministic corrections preserve established identities, while unresolved ancestry, tiers and code allocations remain explicit.

Semantic completeness requires sufficient Business expression to derive Strategy's required behaviour. It requires neither one Function per Feature, one Service per Function, exhaustive Role/Function or Feature/Function mappings, identical decomposition nor structural symmetry. A composition/workflow realisation under §3.6 may suffice without a specialised Function; unestablished individual relationships remain unestablished. The residual SD-03, SD-08 and SD-14 decisions are recorded in the Service Delivery catalogue, while HSO-09 and HSO-16 are reconciled within their operational responsibilities.

The current approved assurance authority boundary and the three assurance Process purposes/progression descriptions are sufficient for this baseline. Detailed approval allocation and lifecycle transitions remain unestablished and may be revisited if later architecture demonstrates a genuine Business requirement. Evidentiary use alone establishes no Service, Interaction or dependency. The approved assurance catalogue remains two Roles, five Functions, three Processes, three Services and three Interactions; financial governance exclusion and non-recursion remain intact.

The approved G1 capability-scoped corrections and unresolved Feature/dependency relationships remain valid unless a separately authorised semantic decision establishes more. No exhaustive Value Stream derivation, participant/exposure matrix, Twin Business Actor or workflow, or organisational risk-management responsibility is required for structural completeness.

12. Harmonia Capability Decomposition & Derivation Progression

RAD source: `docs/markdown/02-strategy/capability-maps/capability-tier-model.md`

The following is a presentation transcription of the governed source. Source catalogue layout and retained illustrations do not allocate tiers or establish additional identities. Equations wrap as text; diagram notation is textual.

Harmonia Capability Decomposition & Derivation Progression

Overview

This model distinguishes hierarchical Capability decomposition from the derivation progression, composition relationships, and derivation rules connecting high-level healthcare enterprise capabilities with reusable platform capabilities and downstream architectural responsibility boundaries.

Canonical terminology SHALL distinguish:

- **Layer / Architectural Layer**: architectural layering within the TOGAF / ArchiMate architecture model.
- **Capability Tier**: hierarchical decomposition within a Capability Model.
- **Derivation / Derivation Progression**: progression between different architectural constructs or models; a Derivation Stage is a stage in that progression.

`Layer≠CapabilityTier≠DerivationStage`.

Business Capability decomposition SHALL use `CT1→CT2→CT3`; Business Enabling Capability decomposition SHALL use `CT1→CT2→CT3→FT`. BC↔BEC enabled-by/support is a separate relationship, not decomposition or implementation equivalence. In these fixed models: `CT1` means Capability Tier 1, `CT2` Capability Tier 2, `CT3` Capability Tier 3 and `FT` Feature. `L1`, `L2` and `L3` remain deprecated as canonical capability classifications.

Tier / type	Architectural role
CT1	Establishes the broad Capability frame within Harmonia's architectural scope.
CT2	Decomposes the CT1 frame into constituent Capabilities.
CT3	Establishes the finest Capability frame required by Harmonia and within the Business Enabling hierarchy, the Capability context within which Harmonia Features are defined.
FT	Expresses required system-enabled behaviour within its owning Business Enabling CT3 Capability context.

Every Harmonia Feature SHALL be immediately framed by Business Enabling CT3; Business Capabilities do not decompose into Harmonia Features; every CT3 SHALL have a CT2 parent; every CT2 SHALL have a CT1 parent. Capability tiers SHALL NOT be skipped. Complete CT1 → CT2 → CT3 ancestry is required for a fully reconciled Feature. CT1 → CT2 and CT2 → CT3 are Capability decomposition relationships.

> **A Feature cannot compensate for incomplete Capability decomposition. If Harmonia requires a Feature, the CT3 Capability that frames that Feature must first be identified. Failure to identify a meaningful CT3 indicates incomplete upstream Capability modelling, not permission to skip the tier.**

This modelling obligation SHALL NOT authorise manufactured CT3 Capabilities for structural symmetry. Existing Features without established CT3 frames SHALL be preserved with their

established responsibilities and local IDs; their Capability ancestry/decomposition requires reconciliation. Missing ancestry SHALL remain explicitly unresolved, rather than being omitted, invented or treated as direct CT1/CT2 Feature framing in the target model.

CT3 is the intentional Capability decomposition boundary. Harmonia models the level required to derive and govern Harmonia, rather than the complete Enterprise Architecture of a hospital, health service provider, health department or other healthcare enterprise. CT4, CT5 and deeper Capability tiers are outside that scope and SHALL NOT be defined. The approved decision record records the rationale and supersession; the Domain03 metamodel governs the fixed hierarchy.

The Domain03 Business Architecture metamodel governs typed Canonical IDs, identity, aliases and behavioural anchoring. The five-stage derivation progression below SHALL NOT be reinterpreted as CT1 / CT2 / CT3 / FT or used to assign Capability Tiers or identifier codes to existing elements. Approved G1 K9 establishes that affected legacy owner assignments are insufficiently evidenced; they SHALL NOT be mechanically translated to CT1 / CT2 / CT3. Capability Tier, complete ancestry, root status and structural Canonical IDs remain unresolved where not established.

Unresolved Capability Tier or ancestry SHALL NOT prevent downstream derivation where the owning Capability or Feature and its relevant responsibility are established. Traceability SHALL reference that element without manufacturing ancestry, as required by AX-17. Document structure, grouping, numbering, indentation and decomposition presentation SHALL NOT by themselves establish Architectural Element identity.

The model ensures that:

1. Enterprise healthcare practice is clearly demarcated from software enablement.
2. System-enabled capabilities reflect authentic clinical, administrative, and operational contexts rather than integration software mechanics.
3. System behaviours are decomposed into atomic, testable features.
4. Reusable capabilities (EC-01 .. EC-14) are derived systematically without creating centralized bottleneck services.
5. Technical realisations are guided by ICT Foundation lenses without allowing physical software products to define architectural capabilities.

1. The Five-Stage Derivation Progression

The framework progresses through five derivation stages relating different architectural constructs and models. Their stage numbers SHALL NOT denote Capability Tiers or Architectural Layers:

RAD source listing (presentation only; no inferred ancestry).

STAGE 1: BUSINESS CAPABILITY

What the enterprise must be capable of doing

(18 catalogued capabilities across 5 natural healthcare regions)

requires enablement by

▼

STAGE 2: BUSINESS ENABLING CAPABILITY

What systems must enable or provide in support of the business

(Structured across 5 authentic healthcare contextual views)

specifies atomic behaviour as

▼

STAGE 3: FEATURE (FT)

Smallest useful statement of required system-enabled behaviour

(Independently understandable, testable, technology-neutral)

derived into reusable

▼

STAGE 4: ENTERPRISE CAPABILITY (EC-01 .. EC-14)

Reusable ICT functionality recurring across multiple domains

(Decoupled from concurrency, message brokers, and storage DDL)

viewed through

▼

STAGE 5: ICT FOUNDATION CAPABILITY LENSES (18 Lenses)

Cross-cutting technical enablement considerations

(Evaluates technical realization via technology-substitution test)

realised by

▼

ARCHITECTURE REALISATION

Domains 03–07: Business, Information, Application, Integration, Tech

2. Derivation Stage Definitions

Stage 1: Business Capability

- **Definition**: What the healthcare enterprise must be capable of doing to deliver healthcare services, govern operations, and ensure patient safety.
- **Independence**: Exists independently of IT systems, software products, commercial organizational hierarchies, or automation.
- **Structure**: 18 CT1 Business Capability roots grouped into five natural regions, with the explicit CT2/CT3 assertions and deliberate stopping points in the canonical BC catalogue. Their tiers are established by that reconciliation, not derivation-stage position; Canonical IDs remain unallocated:
  - *Care & Health Delivery* (01–08)
  - *Health Information & Digital Health* (09–12)
  - *Research & Innovation* (13)
  - *Health Service Management* (14–16)
  - *Governance & Assurance* (17–18)
- **Harmonia Principle**: Harmonia provides core technical enablement for Health Information and Connected Health capabilities (09–12) and materially enables delivery and workforce capabilities (01, 02, 04, 06, 15) without asserting platform ownership over healthcare business practice.

Natural regions are contextual groupings, not capability tiers, organisational structures, application boundaries or ownership hierarchies, and are distinct from Business Enabling contextual views. Health Service Governance (17) retains unresolved relevance classification/enabling derivation; Health Service Assurance (18) is Harmonia-Relevant under the reconciled BC catalogue, preserving its approved independent governed-assurance participation. The approved Health Service Assurance derivation establishes only REQ-FND-005 → BC-18 → the three named Business Enabling Capabilities → collaborative Enterprise Capability realisation. Stage diagrams and existing examples do not establish additional relationships.

Stage 2: Business Enabling Capability

- **Definition**: What software systems and information infrastructure must enable or provide to support the enterprise healthcare capabilities.
- **Structuring Axis**: Organized into five contextual views reflecting genuine healthcare operating environments:

1. *Entity Management*: Governance of practitioners, organizations, locations, services, products, and devices.
2. *Service Administration*: Administrative progression of encounters, referrals, orders, scheduling, and billing.
3. *Service Delivery*: Systems enablement across clinical delivery settings (primary, acute, emergency, diagnostic, virtual).
4. *Health Service Operations*: Healthcare facility logistics, bed management, work allocation, dispatch, and discharge coordination.
5. *Intrinsic / Shared Enablement*: Longitudinal clinical records, health information exchange, clinical collaboration, and workflow coordination.

- **Independence**: Independent of commercial software product boundaries (PAS, EMR, LIS, RIS) and internal middleware engines.

The approved Assurance Design, Assurance Criteria Management and Governed Assurance additionally support BC-18. Their contextual-view placement, relevance classification, Capability Tier, complete ancestry, root status and structural Canonical IDs remain unresolved. Their direct Strategy derivation does not create intermediate Features, a sixth view or inferred hierarchy.

Stage 3: Feature (FT)

- **Definition**: The **smallest useful statement of required system-enabled behaviour** within its owning CT3 Business Enabling Capability context. Existing owners whose tier/ancestry is unresolved remain established responsibility evidence requiring reconciliation. FT is a Feature classification; derivation stage 3 SHALL NOT classify it as CT3.
- **Criteria**:
  - *Independently understandable*: Conveys a self-contained operational meaning.
  - *Testable in principle*: Allows clear verification that the behavior occurred or was enabled.
  - *Technology-independent*: Contains no product names, protocols, or storage mechanisms.
  - *Component-independent*: Does not prescribe which software module executes it.
- **Scope Demarcation**: Formally authored for Harmonia-Relevant and Harmonia-Core capabilities in the established catalogue. Reference and Adjacent capabilities are intentionally not decomposed into features. This scope statement allocates no Capability Tier to existing elements and does not waive the fixed ancestry required for a fully reconciled Feature.

Stage 4: Enterprise Capability (EC-01 .. EC-14)

- **Definition**: Reusable architectural functionality derived from recurring system-enabled features across multiple healthcare domains.
- **Derivation Logic**: Derived by analyzing features and asking: *"How is this function delivered?"* Functions that recur vertically across multiple distinct healthcare domains are abstracted into reusable Enterprise Capabilities.
- **Catalogue**: The established EC-01 Managed Entity & Relationship through EC-13 Semantic Governance & Conformance, plus approved EC-14 Service Guardian.
- **Collaborative Nature**: Enterprise Capabilities represent bounded reusable responsibilities. Their realisation may require participation, consumption or enforcement across multiple architectural elements, but this does not imply shared semantic ownership or prohibit establishment of a responsibility centre. **Distributed participation does not imply distributed responsibility. Cross-cutting concern does not imply cross-cutting responsibility.**

EC-14 supplies the assurance-specific semantics needed to bind existing reusable capabilities into governed assurance. The approved first-pass contribution matrix establishes collaborative realisation without extending EC-12's semantics or allocating EC-14 to a component. The generic progression through Features describes the existing derivation convention; it does not manufacture Feature relationships for this explicitly approved assurance derivation.

Stage 5: ICT Foundation Capability Lenses

- **Definition**: Technical enablement lenses that provide cross-cutting engineering criteria, architectural considerations, and technology-substitution testing for realizing Enterprise Capabilities.
- **Role**: They are technical lenses, NOT peers of the Business Enabling capabilities.
- **Test**: Governed by the *Technology-Substitution Test*: *"If the named product, protocol, programming language, or deployment mechanism were replaced tomorrow, would the capability still exist?"*

3. The Capability Composition Formula

Harmonia capabilities are composed according to the following foundational relationship:

Harmonia Capability = domain-specific enabling behaviour + reusable Enterprise Capabilities

Where:

- **Domain-Specific Enabling Behaviour**: The specific clinical or operational logic, context rules, and progression semantics unique to a healthcare domain (e.g., healthcare identifier system rules, diagnostic order workflows, bed turnover readiness).
- **Reusable Enterprise Capabilities**: The horizontal, platform-wide capabilities (EC-01 through EC-14) that provide state management, context propagation, policy evaluation, provenance capture, interoperable transport and the approved assurance-specific semantics.

Informing Strategic Logical Component Boundaries

In Harmonia Strategy:

- **Capabilities describe *what* the platform achieves**.
- **Strategic Logical Components describe *how responsibility is organized***.

When specific combinations of Enterprise Capabilities and domain-specific behaviours recur coherently across multiple features, they inform candidate **Strategic Logical Component responsibility boundaries**:

RAD source listing (presentation only; no inferred ancestry).

RECURRING COMPOSITION PATTERN

Domain-specific enabling behaviour

+ Reusable Enterprise Capabilities

informs clustering for

▼

STRATEGIC LOGICAL RESPONSIBILITY MODEL

- Mneme (Managed Information & Active State)
- Mnemosyne (Durable Preservation & Recovery)
- Ponos (Managed Operational Activity Execution)
- Pylai (Standards-Conformant Ingress/Egress Membrane)
- Calliope (Semantic Authority & Conformance)
- Iris (Contextual Human Interaction & Presentation)
- Digital Twin (Entity Coordination Archetype)

Architectural Principle: This composition informs component boundaries; it does **not** imply a rigid, mathematical one-to-one derivation. Components represent cohesive responsibility centres, not one-to-one wrappings of individual capabilities.

The displayed component-clustering examples remain the established EC-01 through EC-13 derivations. This convention does not allocate EC-14 or the three assurance Business Enabling Capabilities to a strategic logical component or establish a relationship to an existing implementation construct.

4. Representative Derivation Examples

To demonstrate how Enterprise Capabilities recur across distinct Business Enabling features without constructing an exhaustive matrix, three representative derivation examples are detailed below.

Example 1: Person Identifier Resolution

In healthcare integration, identifying an individual across disparate clinics, hospitals, and diagnostic centres requires resolving identifiers (MRNs, national healthcare identifiers, local practice IDs) issued by multiple independent authorities.

- **Domain-Specific Enabling Behaviour**:
  - Healthcare identifier system semantics (e.g., verifying issuing authority jurisdiction, assigning authority namespaces).
  - Scope of identifier validity (temporary vs. permanent, national vs. regional).
  - Client privacy and consent flags associated with identifier disclosure.
- **Reusable Enterprise Capability Contributions**:
  - **EC-01 Managed Entity & Relationship**: Modeling the Person entity and linking multiple scoped identifiers and alias relationships.
  - **EC-05 Search & Discovery**: Locating existing entity records matching supplied identifier sets and resolving candidate matches.
  - **EC-03 Managed State & Lifecycle**: Governing the lifecycle status (active, superseded, suspended) of identifier records.
  - **EC-07 Provenance & Traceability**: Recording the authoritative origin, verification timestamp, and issuing system attribution for each identifier.
  - **EC-06 Policy & Control**: Enforcing default-deny access policies governing which requesting systems may view or resolve specific identifier authorities.
  - **EC-12 Operational Assurance**: Ensuring deterministic matching behavior and transactional consistency during concurrent resolution requests.

Example 2: Order Closed Loop

A clinical diagnostic or procedure order requires governed progression from initial request to final diagnostic report.

- **Domain Progression Semantics**: Destination → Route → Deliver → Acknowledge → Progress → Complete → Outcome
- **Critical Healthcare Distinction**: Technical Delivery Acknowledgement ≠ Business Acknowledgement A technical acknowledgement establishes only the acceptance state defined by the applicable interaction contract. It SHALL NOT be interpreted as durable acceptance, committed business state, successful processing or successful business disposition unless that meaning is explicitly established by the governing contract. HTTP 200/202 and MLLP acknowledgements are interpreted under their applicable contracts. A handoff acknowledgement alone does not establish the eventual downstream business outcome or the receiving laboratory's clinical responsibility. REQ-FND-001 retains its explicit Harmonia durable-ingress requirement.

- **Reusable Enterprise Capability Contributions**:
  - **EC-02 Context Management**: Establishing and propagating the clinical encounter, ordering provider, and subject-of-care context across all order lifecycle events.
  - **EC-05 Search & Discovery**: Resolving the target fulfillment service endpoint and receiving facility capabilities.
  - **EC-01 Managed Entity & Relationship**: Associating the Order entity with the Subject, Ordering Practitioner, Performing Organization, and Target Service.
  - **EC-08 Interoperability & Exchange**: Standards-compliant boundary protocol adaptation, message transformation, delivery dispatch, and transport acknowledgement capture.
  - **EC-03 Managed State & Lifecycle**: Tracking the formal Order state progression (`PLACED` → `RECEIVED` → `ACCEPTED` → `IN_PROGRESS` → `COMPLETED`).
  - **EC-10 Activity & Execution**: Instantiating and executing operational units of work, tracking timeouts, managing escalations, and supervising fulfillment deadlines.
  - **EC-07 Provenance & Traceability**: Capturing complete chain-of-custody, transformation history, and delivery timestamps for clinical auditability.
  - **EC-12 Operational Assurance**: Guaranteeing dual-write safety, duplicate suppression, and exception recovery across asynchronous boundary handoffs.

Example 3: Work Allocation & Dispatch

Operational healthcare activities (e.g., patient transport, ward bed preparation, discharge coordination, emergency triage assessment) require assigning and dispatching work to qualified clinical or operational staff.

- **Domain Progression Semantics**: Event → Entity Context → Required Activity → Assignment → Dispatch → ACK → Progress → Outcome
- **Healthcare Service Context Rule**: Where operational activity occurs in the context of a Healthcare Service, that service context must be explicitly associated with the activity/state rather than inferred solely from practitioner, organization, or location.
- **Cross-Domain Recurrence**: This operational progression pattern recurs across:
  - *Discharge Progress*: Coordinating multidisciplinary discharge milestones and readiness oversight.
  - *Patient Transport*: Allocating transfer tasks to portering staff based on mobility requirements.
  - *Mobile Work*: Dispatching community health visits to roving practitioners.
  - *ED Operational Coordination*: Progressing triage queue assessments.
  - *Bed Readiness*: Dispatching environmental cleaning staff upon patient discharge.
- **Reusable Enterprise Capability Contributions**:
  - **EC-02 Context Management**: Binding the patient, physical bed/room, care team, and healthcare service context to the operational work item.
  - **EC-03 Managed State & Lifecycle**: Managing the lifecycle of the operational activity (`REQUESTED` → `ASSIGNED` → `ACCEPTED` → `IN_PROGRESS` → `COMPLETED`).
  - **EC-05 Search & Discovery**: Discovering available, rostered, and credentialed practitioners or operational personnel matching task criteria.
  - **EC-01 Managed Entity & Relationship**: Linking the assigned worker, patient, location, and equipment entities.
  - **EC-10 Activity & Execution**: Governing the assignment logic, dispatch mechanism, human notification, timeout monitoring, and escalation triggers.
  - **EC-08 Interoperability & Exchange**: Delivering work orders to mobile devices or external task management systems where external dispatch is required.
  - **EC-12 Operational Assurance**: Preventing duplicate assignment and ensuring resilient recovery if a dispatched worker does not acknowledge within defined thresholds.
  - **EC-07 Provenance & Traceability**: Recording assignment timestamps, worker acceptance, and completion evidence.

5. Architectural Modelling Position: Omission of Exhaustive N × M Matrices

Harmonia explicitly rejects the generation of exhaustive N × M traceability matrices between Business Capabilities and Business Enabling Capabilities, or between Features and Enterprise Capabilities.

Formal Rationale

> **The Business Enabling Capability model was derived with explicit consideration of the Business Capability model and is intended to provide the system-enabled capabilities necessary to support that business landscape. A formal many-to-many mapping between Business Capabilities and Business Enabling Capabilities has not been produced, as such traceability is not required for the current Harmonia architecture and strategy objectives. Such mapping may be developed subsequently where required for benefits realisation, investment analysis, business-case development or other value-traceability purposes.**

This statement records the existing derivation. BC-17 enabling derivation and relationships beyond the approved BC-18 derivation remain unresolved under AX-17. The Health Service Assurance view adds only the approved three Business Enabling Capabilities and bounded fourteen-row EC contribution matrix. That explicit matrix is not an exhaustive Business Capability-to-Business Enabling or Feature-to-Enterprise mapping, does not imply other coverage, and does not change EC-12's established operational semantics.

Utilitarian Justification

1. **Avoidance of False Precision**: In a complex regional HIE, a single Business Capability (e.g., *Care Access & Coordination*) draws upon dozens of enabling capabilities across Entity Management, Service Administration, and Shared Enablement. An exhaustive matrix suggests mechanical one-to-one connections where real systems operate through dynamic, contextual composition.
2. **Maintenance Overhead vs. Value**: Maintaining a static matrix of hundreds of capabilities across thousands of cells creates substantial documentation drift without providing actionable guidance to software engineers or architects.
3. **Clarity of Representative Derivations**: As shown in Section 4, representative derivation patterns provide clear, verifiable architectural rationale for how enterprise needs compose reusable capabilities, without the clutter of exhaustive combinatorial tables.

Topic Navigation

- Domain 02 Strategy Overview
- Capability Framework Index
- Business Capabilities
- Business Enabling Capabilities
- Enterprise Capabilities
- Health Service Assurance Strategy Derivation
- ICT Foundation Lenses

13. Health Service Assurance — Approved Strategy Derivation

RAD source: `docs/markdown/02-strategy/capability-maps/health-service-assurance-derivation.md`

The following is a presentation transcription of the governed source. Source catalogue layout and retained illustrations do not allocate tiers or establish additional identities. Equations wrap as text; diagram notation is textual.

Source qualification: The opening 2026-10-14 reconciliation qualifies the older unresolved BC-18 tier/relevance statements later in this source. Later approved Domain03 assurance behaviour is reproduced separately; earlier Strategy stop boundaries retain their own scope.

Health Service Assurance — Approved Strategy Derivation

> **BC reconciliation — 2026-10-14:** The current BC catalogue establishes BC-18 as a CT1, its stated BC CT2/CT3 decomposition and Harmonia-Relevant classification. Its macro-level confidence purpose preserves REQ-FND-005's applicable independent governed-assurance requirement. This qualifies the earlier unresolved BC-tier/relevance statements below; all existing BEC/EC support assertions remain unchanged and are not redistributed among the new BC descendants. BEC tiers, ancestry and identity allocations remain unresolved.

Purpose and Architectural Authority

This view records the approved Health Service Assurance derivation in Domain02 Strategy. It connects the existing Motivation requirement and Business Capability to three approved Business Enabling Capabilities and their collaborative Enterprise Capability realisation. Canonical definitions remain in the linked capability catalogues.

AX-17 — Architectural Authority and Explicit Uncertainty governs truthful traceability: only the relationships explicitly approved here are established. Missing relationships remain unresolved. The requested Strategy reconciliation is approved architectural input; it does not complete later architecture domains.

The materially relevant axioms also include:

- AX-14 — Semantic Distinctions Are Preserved: assurance is distinct from monitoring, evidence production, operational management, clinical adequacy, assessment and adjudication.
- AX-06 — Information Authority Is Explicit, AX-07 — Security Is Intrinsic to Managed Operations and AX-08 — Evidence Records Meaning, Not Machinery: trustworthy evidence and governed assurance preserve their existing authority, security and meaningful-evidence constraints without transferring source-information ownership.
- AX-09 — Transient Operational State Is Ephemeral by Default: contextual evidence association does not automatically make all observations or operational diagnostics durable evidence.
- AX-10 — Distribution, Load and Failure Are Normal Operating Conditions and AX-15 — Uncertainty Is Preserved Until Resolved: expected failure/recovery remains the subject activity's responsibility, and operational uncertainty remains distinct from insufficient assurance evidence.
- AX-05 — Active State and Authoritative Durable State Are Distinct: temporal/version-specific evidence does not alter the approved state-responsibility separation or derive a persistence/active-state implementation.

Motivation to Strategy Traceability

RAD diagram: textual source notation.

```
graph TD
    REQ["REQ-FND-005<br/>Independent Assurance of Governed Activity"] --> BC["BC-18<br/>Health Service
    ↪  Assurance"]
    BC --> AD["Business Enabling Capability<br/>Assurance Design"]
    BC --> ACM["Business Enabling Capability<br/>Assurance Criteria Management"]
    BC --> GA["Business Enabling Capability<br/>Governed Assurance"]
    AD --> COL["Collaborative Enterprise Capability realisation<br/>Approved contribution matrix"]
    ACM --> COL
    GA --> COL
    COL --> EXIST["Applicable existing EC-01 through EC-13<br/>Direct or Supporting as explicitly approved"]
    COL --> SG["EC-14<br/>Service Guardian"]
```

The final branches identify contributions to collaborative realisation, not ownership, execution order or component allocation. The matrix below distinguishes the contribution to each Business Enabling Capability; the aggregate diagram does not assert every existing EC contributes to each capability.

Established relationship	Authoritative Strategy reference	Boundary
REQ-FND-005 → BC-18	Approved requirement; Health Service Assurance	Independent conclusion authority and explicit evidence insufficiency; no clinical-adequacy responsibility transfer.
BC-18 → Assurance Design	Assurance Design	Defines how satisfaction is assured, not application design.
BC-18 → Assurance Criteria Management	Assurance Criteria Management	Manages criteria without replacing governing requirements.
BC-18 → Governed Assurance	Governed Assurance	Independently evaluates and concludes without managing the subject.
Three Business Enabling Capabilities → collaborative EC realisation	Contribution matrix; Enterprise catalogue	Direct does not mean sole realisation; no extra relationships implied.
EC-14 → assurance-specific contribution	Service Guardian	Reusable assurance semantics, distinct from EC-12; component allocation remains unresolved.

BC-18 refers to the existing Business Capability catalogue entry **18 Health Service Assurance**. This reference does not assign a new Capability Tier or structural Canonical ID. The three Business Enabling capability names identify the approved responsibilities without inventing CT1 / CT2 / CT3 ancestry or new Feature identifiers.

Collaborative Enterprise Capability Contribution Matrix

This is the approved **first-pass contribution model**. Existing canonical EC names are preserved. **Direct** identifies a direct contribution, not sole realisation or ownership. **Supporting** identifies a supporting contribution. **No Material Role** records the approved absence of a material contribution in this model; it is not an invitation to invent a relationship.

Enterprise Capability	Assurance Design	Assurance Criteria Management	Governed Assurance
EC-01 Managed Entity & Relationship	Supporting	No Material Role	Supporting
EC-02 Context Management	Supporting	Supporting	Direct
EC-03 Managed State & Lifecycle	Supporting	Direct	Direct
EC-04 Information Management	Direct	Direct	Direct

Enterprise Capability	Assurance Design	Assurance Criteria Management	Governed Assurance
EC-05 Search & Discovery	Supporting	Supporting	Supporting
EC-06 Policy & Control	Direct	Direct	Supporting
EC-07 Provenance & Traceability	Supporting	Supporting	Direct
EC-08 Interoperability & Exchange	No Material Role	Supporting	Supporting
EC-09 Event & Subscription	Supporting	No Material Role	Direct
EC-10 Activity & Execution	Supporting	No Material Role	Direct
EC-11 Interaction & Experience	Supporting	Supporting	Supporting
EC-12 Operational Assurance	Supporting	Supporting	Direct, limited to generic operational/ processing assurance contribution
EC-13 Semantic Governance & Conformance	Supporting	Direct	Supporting
EC-14 Service Guardian	Direct	Direct	Direct

The three Business Enabling Capabilities are collaboratively realised. EC-14 contributes the assurance-specific semantics not truthfully represented by EC-01 through EC-13. Their contributions do not transfer source-information ownership, governing-requirement authority or subject-activity management to assurance. In particular, EC-10's direct contribution does not make Governed Assurance responsible for managing the subject activity, and it does not establish a relationship to an execution component.

This bounded, approved matrix is not an exhaustive Business Capability-to-Business Enabling or Feature-to-Enterprise mapping. It is consistent with the existing representative-derivation convention and does not manufacture missing traceability.

Preserved Semantic Boundaries

- **EC-12 / EC-14**: EC-12's established operational responsibilities remain intact. Its direct Governed Assurance contribution is limited to generic operational/processing assurance. EC-14 supplies assurance disposition, criteria applicability, evidentiary association, assessment, adjudication and independent findings/conclusions; EC-12 is not expanded to absorb them.
- **Clinical assurance**: Health Service Assurance is distinct from Clinical Services Delivery Assurance. Explicitly required assurance of facts or behaviour concerning clinical activity does not make Harmonia responsible for clinical judgement, professional practice or clinical adequacy of care. Capability 06 retains its established responsibility.
- **Evidence and time**: Source-information ownership and contextual evidence association remain distinct. Evidence and criteria may each be temporal/version-specific without deriving storage, replication or persistence mechanisms.
- **Assurance and operations**: Generic processing assurance evaluates business behaviour, including expected failure/recovery; the subject activity manages its own behaviour. Operational Health ≠ Processing Assurance.
- **Assessment and conclusion**: Assessment, adjudication and explicit insufficiency preserve quantitative, qualitative, compound, temporal and confidence-bearing meaning. Insufficient evidence establishes neither satisfaction nor non-satisfaction.
- **Independence**: Assurance independence concerns control of progression and conclusion, not physical separation. Evidence dependency does not compromise assurance independence; control dependency does.

Unresolved Relationships and Downstream Boundary

This approved derivation establishes only REQ-FND-005 → BC-18 → the three named Business Enabling Capabilities → the first-pass collaborative EC contribution model, including EC-14. The five-region/eighteen Business Capability model, BC-17 Health Service Governance, capabilities 06 and 14, the existing thirteen EC semantics and approved AX-05 reconciliation remain intact.

The following remain deliberately unresolved or unestablished:

- Relevance classification for BC-17/BC-18; BC-17 enabling derivation; relationships beyond the approved BC-18 derivation.
- Contextual-view placement, relevance classification, Capability Tier, complete ancestry, root status and structural Canonical IDs for the three assurance Business Enabling Capabilities; additional Feature decomposition.
- Courses of Action, Strategic Resources, Value Stream/stage relationships and additional capability contributions not explicitly approved here.
- Downstream Business Functions, Services, Processes, actors, roles, assurance Information Concepts, outcome taxonomies, criterion catalogues and applicability/confidence models beyond the approved Strategy semantics.
- Strategic logical component, application component, execution, persistence, protocol, deployment and implementation allocation for EC-14 and the three assurance Business Enabling Capabilities.

No relationship to Dokimasia, Ponos, Praxis, Pragma, Digital Twin, Mneme, Mnemosyne, Calliope, Iris, Pylai or another implementation construct is derived. Existing Dokimasia material is not reconciled or modified. Domain03 Business Architecture and Domain04 Information Architecture are unchanged. Existing or future suggestions cannot establish an allocation through lexical similarity, familiar implementation or the conceptual behavioural sequence of Service Guardian.

Navigation

- Domain02 Strategy
- Capability Framework
- Business Capability Catalogue
- Business Enabling Capability Catalogue and Assurance Principles
- Enterprise Capability Catalogue
- Capability Derivation Progression
- Capability Maps Index

14. B2.1 — Individual Care Delivery ↔ Health Information Access

RAD source: `docs/markdown/05-information-systems-architecture/traceability/b2-1-individual-care-delivery-health-information-access.md`

The following is a presentation transcription of the governed source. Source catalogue layout and retained illustrations do not allocate tiers or establish additional identities. Equations wrap as text; diagram notation is textual.

B2.1 — Individual Care Delivery ↔ Health Information Access

B2.1 — Individual Care Delivery ↔ Health Information Access: PASS

1. Authority and bounded result

This record reconciles the explicitly agreed Domain05 Step B B2.1 decisions, 2026-10-13. The Domain02 Business Capability catalogue defines Individual Care Delivery and its six L2 capabilities. The established Health Information Access responsibility defines the enabling anchor. The Step B method governs relationship semantics and Health Information Access scope.

PASS applies only to the L1 relationship and six L2 relationships below. It is not a runtime verification, a declaration of exhaustive capability coverage or an application/component allocation. This record establishes no additional capability decomposition, Feature, Service, Function or Information mapping. **B2.2 is not commenced.** No other Business Capability or Business Enabling Capability is mapped by this reconciliation.

2. L1 relationship and rationale

`IndividualCareDeliveryenabled-byHealthInformationAccess`

Individual Care Delivery requires governed access to relevant longitudinal and current health information to support clinical assessment, diagnosis, treatment decisions, interventions and monitoring. Health Information Access enables access to that information but does not determine its clinical significance or perform the clinical decision-making of Individual Care Delivery.

3. L2 relationships and rationales

These are material capability-enablement relationships. They do not transfer clinical accountability or require each runtime exercise to retrieve information through Health Information Access. The L2 labels identify the agreed Domain02 decomposition; no additional structural Canonical ID scheme is established.

01.01 Individual Health Assessment

`IndividualHealthAssessmentenabled-byHealthInformationAccess`

Individual Health Assessment requires governed access to relevant existing and current health information so that available clinical, health and contextual information can be considered when establishing an understanding of the individual's health state. Health Information Access enables retrieval of that information but does not perform the assessment or establish the assessed findings.

Health Information Access does not own every mechanism by which assessment information is acquired. Direct examination, observation, history-taking or measurement may establish assessment information independently.

01.02 Clinical Determination

`ClinicalDeterminationenabled-byHealthInformationAccess`

Clinical Determination requires governed access to relevant current and historical health information where that information is necessary to interpret assessed findings and establish clinically meaningful conclusions about the individual's health state, condition, risk or prognosis. Health Information Access enables retrieval of that information but does not interpret its clinical significance or establish the clinical determination.

01.03 Individual Care Planning

`IndividualCarePlanningenabled-byHealthInformationAccess`

Individual Care Planning requires governed access to relevant current and historical health and care information where that information is necessary to establish an appropriate intended clinical response, including care objectives, interventions and monitoring. Health Information Access enables retrieval of that information but does not establish the care intent, select the clinical response or perform the planning.

Information concerning operational availability of services, resources, staff, facilities or appointments is not Health Information Access merely because it influences care planning.

01.04 Clinical Intervention

`ClinicalInterventionenabled-byHealthInformationAccess`

Clinical Intervention requires governed access to relevant health and care information where that information is necessary to safely and appropriately perform an intended clinical action, including information concerning the individual's current health state, established care intent and clinically relevant history. Health Information Access enables retrieval of that information but does not authorise, select or perform the clinical intervention.

Access to information describing an intended intervention does not confer authority to perform that intervention.

01.05 Individual Health Monitoring

`IndividualHealthMonitoringenabled-byHealthInformationAccess`

Individual Health Monitoring requires governed access to relevant current and historical health information where that information is necessary to establish change, stability or deviation in an individual's health state over time. Health Information Access enables access to prior, baseline and current health information required for comparison, but does not perform the monitoring or interpret the clinical significance of an identified change.

01.06 Individual Care Review

`IndividualCareReviewenabled-byHealthInformationAccess`

Individual Care Review requires governed access to relevant health and care information in order to evaluate delivered care and observed health progression against the intended care objectives. Health Information Access enables access to the care intent, relevant care history and health outcomes required for that evaluation, but does not perform the review or establish the continuing appropriateness of the care intent.

4. Preserved boundaries

The Domain02 decomposition and context rules apply: the six established children are now explicitly reconciled as BC CT2s. The current catalogue establishes the stated assessment, monitoring and review CT3s; the other branches retain their recorded uncertainty/stopping points. Under the fixed Capability model, subsequently required Harmonia Features must first have meaningful **Business Enabling CT3** frames established. BC decomposition does not terminate in Features and this B2.1 support trace establishes no ancestry or new CT3 enablement edge. None may be invented for structural completeness. Retained L1/L2 display labels alone do not allocate CT tiers. Examples are not subordinate Capabilities, and care setting, delivery mode, temporal pattern and organisational arrangement do not themselves define different business abilities. Acute, Planned/Scheduled and Longitudinal Care are not the ICD L2 decomposition.

B2-R07 distinguishes individual clinical / health / care information from operational / workforce / financial / corporate / service-resource information. No exhaustive edge-case taxonomy or allocation to other enabling capabilities is introduced.

`enabled-by` does not mean that Health Information Access interprets, judges, plans, authorises or performs clinical care. It expresses material enablement, not a mandatory runtime dependency. B2.1 PASS does not change retained candidate deferrals, declare B1 PASS or authorise subsequent Step B mappings.

15. Principal Business Processes

RAD source: `docs/markdown/03-business-architecture/processes/business-processes.md`

The following is a presentation transcription of the governed source. Source catalogue layout and retained illustrations do not allocate tiers or establish additional identities. Equations wrap as text; diagram notation is textual.

Principal Business Processes

1. Process Demarcation & Justification Rules

The named owning Capabilities retain their established responsibilities. Affected Capability Tier, complete ancestry, root status and full structural Canonical IDs remain unresolved under approved G1 K9. The explicit `ClientAdministration→PersonIdentity` relationship remains a partial chain, not a tier allocation.

Processes may elaborate scoped progression without reproducing Behaviour stage lists. Established ownership, responsibility, authority boundaries and supported obligations remain controlling. Omitted Process checkpoints are not invalidated by a Behaviour summary; omitted Behaviour responsibilities are not removed by a Process. Stage-count equality is not required, and extra detail does not establish universal applicability.

Illustrations are not complete normative transition models. Their scoped applicability and visible uncertainty annotations retain independent Business meaning; detailed execution-state machinery remains downstream. For a materially significant managed outcome that available evidence cannot establish, preserve uncertainty rather than infer success or failure from an absent acknowledgement, response or observation. No universal additional Process state is prescribed.

Where Healthcare Service context is material to the activity's meaning, authority, coordination, progression or accountability, retain that context in the relevant activity/information responsibility. Location, Organisation, Practitioner or Role cannot substitute for it; no representation or implementation binding follows.

In the Harmonia Business Architecture, a **Business Process** applies the canonical Process definition: ordered Function decomposition of complex change within the owning Capability or Feature responsibility. Existing activity-instance states, dispositions and outcomes retain their established meaning; no new transition model is inferred.

1.1 Process Inclusion Rule

> **Model a Business Process where ordered Function decomposition is material to understanding, structuring or managing a complex Business change and its progression.**

A Function may require further Processes recursively. Search, query, resolution, translation, mapping, verification, lookup and presentation do not automatically require a Process, but their labels do not exclude decomposition where justified. Service fulfilment may use Functions directly, Processes, or both. Function-level Information relationships remain primary; this reconciliation adds no named Process to the catalogue.

1.2 Canonical Business Process Catalogue

The sixteen pre-existing Business Processes remain in the current R1.x/R2.x baseline with the authorised identity/discharge reconciliations and explicit lifecycle qualifications. Three approved Health Service Assurance Processes extend this catalogue to **nineteen Business Processes**; their existing authority/progression boundaries are sufficient for this baseline while detailed lifecycle states and transitions remain unestablished.

RAD source listing (presentation only; no inferred ancestry).

Principal Business Processes

Entity & Administrative Processes

1. Governed Person Identity Correction
2. Practitioner Verification

Clinical Lifecycle & Service Administration Processes

3. Referral Progression
4. Encounter Lifecycle
5. Closed-Loop Order Progression
6. Clinical Document Lifecycle

Healthcare Facility & Logistics Operations Processes

7. Theatre Case Progression
8. Clinic / Operational Queue Progression
9. Bed Turnover
10. Operational Work Progression
11. Patient Transport
12. Specimen Transport
13. Discharge Progression

Horizontal Workflow Coordination Processes

14. Work Order Progression (Human Doing)
15. To Do Progression (Human Reviewing / Deciding)
16. Synthetic Task Progression (Automated System Work)

Health Service Assurance Processes

17. Assurance Process Design
18. Assurance Process Execution
19. Assurance Process Reporting / Communication

2. Entity & Administrative Processes

2.1 Governed Person Identity Correction Process

- **Owning Capability**: `PersonIdentity` (under `ClientAdministration`)
- **Process Purpose**: Processes externally authoritative identity corrections and merge/unlink outcomes, applies their consequences to Harmonia-managed identity information and identifier associations, and communicates resulting changes with an audit history.
- **Authority boundary**: Person correction/merge decisions originate externally. Internal triage, evidence verification and approval concern applicability and governed processing of that authoritative outcome, not person matching, golden-record determination, master-person selection or originating merge adjudication.

- **State Progression Lifecycle**: Correction Requested → HIM Triage → Evidence Verified → Correction Approved → Merge/Unlink Executed → Change Broadcasted → Closed
- **Key State Dispositions**:
  - `CorrectionRequested`: Externally authoritative correction or merge/unlink outcome received for governed application.
  - `HIMTriage`: Health Information Manager reviews the received decision/outcome and its applicability.
  - `EvidenceVerified`: Authoritative source decision and supporting provenance/documentation verified.
  - `CorrectionApproved`: Governed application of the externally authoritative outcome approved; this does not originate a person-merge decision.
  - `Merge/UnlinkExecuted`: External merge/unlink consequences applied to Harmonia's managed identifier associations and audit history.
  - `ChangeBroadcasted`: Downstream clinical systems notified of merged or rectified identity.

2.2 Practitioner Verification Process

- **Owning Capability**: `ProviderAdministration`
- **Process Purpose**: Governs the lifecycle of validating, certifying, and periodically re-verifying a healthcare practitioner's professional credentials, registration status, and clinical scope of practice.
- **State Progression Lifecycle**: Verification Initiated → Regulatory Registry Queried → Credentials Validated → Privileges Endorsed → Active Verified → Expired / Suspended
- **Key State Dispositions**:
  - `VerificationInitiated`: New practitioner registration or periodic re-credentialing triggered.
  - `RegulatoryRegistryQueried`: Real-time query executed against national registry (e.g., AHPRA).
  - `CredentialsValidated`: Primary qualifications, specialist endorsements, and sanctions checked.
  - `PrivilegesEndorsed`: Health service medical administration endorses admitting/procedural scope.
  - `ActiveVerified`: Practitioner verified for clinical ordering and electronic communication.
  - `Expired/Suspended`: Verification lapses or regulatory sanction triggers privilege suspension.

3. Clinical Lifecycle & Service Administration Processes

3.3 Referral Progression Process

- **Owning Capability**: `ReferralAdministration`
- **Process Purpose**: Governs the end-to-end operational progression of an incoming clinical referral from receipt through specialist clinical triage, booking, and final service acceptance.
- **Scope qualification**: This illustrates a specialist booking/attendance pathway; Referral does not universally require acceptance, scheduling, attendance or discharge, or establish delivery, responsibility actually assumed or Transfer of Care. Decline, rejection and redirection remain valid dispositions without invented transition paths. `Accepted/Waitlisted` is a local compound checkpoint, not universal equivalence between acceptance and waitlisting.
- **State Progression Lifecycle**: Submitted → Intake Validated → Clinically Triaged → Accepted / Waitlisted → Scheduled → Consultation Attended → Discharged / Rejected (Illustrative specialist pathway; dispositions vary)
- **Key State Dispositions**:
  - `Submitted`: Electronic referral received from GP or external facility.
  - `IntakeValidated`: Administrative validation of patient details, mandatory fields, and tests.
  - `ClinicallyTriaged`: Senior clinician assigns urgency category (e.g., Cat 1 urgent within 30 days).

- `Accepted/Waitlisted`: Referral accepted and placed onto the specialty waitlist.
- `Scheduled`: Outpatient appointment allocated and patient notified.
- `ConsultationAttended`: Patient seen; initial specialist assessment completed.
- `Discharged/Rejected`: Referral completed and discharged back to primary care, or formally rejected with clinical rationale.

3.4 Encounter Lifecycle Process

- **Owning Capability**: `Episode&EncounterAdministration`
- **Process Purpose**: Governs the clinical and administrative lifecycle of an acute, emergency, inpatient, or outpatient encounter between a patient and healthcare services.
- **State Progression Lifecycle**: Planned / Booked → Arrived → Triaged / Ingested → Active In-Progress → Discharged → Completed / Encoded (Scoped illustration; ON_LEAVE relationship unresolved)
- **Key State Dispositions**:
  - `Planned/Booked`: Elective admission or clinic appointment scheduled.
  - `Arrived`: Patient presents at facility or emergency desk.
  - `Triaged/Ingested`: Clinical triage category assigned; encounter record activated.
  - `ActiveIn-Progress`: Inpatient care, bedside monitoring, and clinical orders actively underway.
  - `Discharged`: Patient clinically discharged; care-place vacated.
  - `Completed/Encoded`: Clinical documentation finalized, ICD/DRG coding completed, and encounter legally closed.
- **Scope qualification**: These completion criteria retain their locally described applicability; they do not establish universal signing, legal closure or originating authority over clinical information. Finalisation, signing and legal qualification remain distinct. The relationship with Strategy `ON_LEAVE`, including applicability and transitions, remains unresolved.

3.5 Closed-Loop Order Progression Process

- **Owning Capability**: `OrderAdministration`
- **Process Purpose**: Governs the rigorous, closed-loop tracking of diagnostic pathology, radiology, and procedural orders from requisition to result correlation, preventing dropped or unfulfilled investigations.
- **Scope qualification**: These examples describe a diagnostic/procedural progression and do not exclude medication from the broader Order Administration responsibility. Specimen and result checkpoints are scoped. The signed requisition/report requirements remain within this illustrated scope; signing is not universally equivalent to finalisation or authority. Result consumption/binding does not define originating validity, authority, authorship or legal status.
- **State Progression Lifecycle**: Requisition Placed → Order Dispatched → Specimen Collected / Scheduled → In-Execution → Preliminary Result Bound → Final Result Bound → Closed / Verified (Scoped diagnostic/procedural illustration)
- **Key State Dispositions**:
  - `RequisitionPlaced`: Electronic order created and signed by requesting clinician.
  - `OrderDispatched`: Order routed and accepted by performing diagnostic service. This local compound checkpoint retains routing and recipient acceptance as distinct facts; dispatch alone establishes neither receipt nor acceptance. Behaviour `Received` is not equated with this checkpoint without evidence.
  - `SpecimenCollected/Scheduled`: Bio-specimen collected or imaging appointment booked.
  - `In-Execution`: Laboratory analysis or diagnostic scanning underway.
  - `PreliminaryResultBound`: Critical or interim observations published and linked to order.
  - `FinalResultBound`: Authoritative diagnostic report signed and bound to order.
  - `Closed/Verified`: Requesting clinician acknowledges result; order closed. This Order-closure checkpoint does not establish diagnostic-content verification, approval or clinical incorporation.

3.6 Clinical Document Lifecycle Process

- **Owning Capability**: `ClinicalRecordAdministration`
- **Process Purpose**: Governs the versioning, clinical sign-off, addenda, superseding, and legal status of clinical documents (discharge summaries, specialist letters, advance care directives).
- **State Progression Lifecycle**: Draft → Preliminary → Final Signed → Amended / Addended → Superseded → Entered-in-Error (Scoped illustration; no universal document lifecycle)
- **Key State Dispositions**:
  - `Draft`: Incomplete clinical document saved during consultation.
  - `Preliminary`: Document authored pending senior registrar or consultant countersignature.
  - `FinalSigned`: Authoritative clinical document legally signed and published.
  - `Amended/Addended`: Governed supplementary clinical addendum appended to signed document.
  - `Superseded`: Entire document replaced by a newer revision; prior version retained for audit.
  - `Entered-in-Error`: Document formally retracted due to wrong patient or invalid content; content struck through with retraction notice.
- **Scope qualification**: `Final≠automaticallyFinalSigned`. The described signing/countersigning and legal qualifications remain local requirements, not a universal document lifecycle. Authorship, attestation, approval, authentication, verification, signature, finalisation, authority and legal qualification remain distinct. Amendment, supersession and entered-in-error are not mandatory stages for every document, and consumer processing does not establish originating authority or legal status.

4. Healthcare Facility & Logistics Operations Processes

4.7 Theatre Case Progression Process

- **Owning Capability**: `TheatreOperations`
- **Process Purpose**: Governs the perioperative operational milestones of a surgical procedure within the operating theatre suite.
- **State Progression Lifecycle**: Case Called → In Anaesthetic Bay → Anaesthesia Commenced → Patient in Theatre → Knife-to-Skin → Procedure Finished → In PACU Recovery → Ward Handover Complete
- **Key State Dispositions**:
  - `CaseCalled`: Theatre team requests ward to send patient for surgery.
  - `InAnaestheticBay`: Patient arrives in theatre holding and identity/consent verified.
  - `AnaesthesiaCommenced`: Anaesthetic induction underway.
  - `PatientinTheatre`: Patient transferred onto operating table.
  - `Knife-to-Skin`: Surgical incision initiated (operative start).
  - `ProcedureFinished`: Wound closure completed and dressings applied.
  - `InPACURecovery`: Patient monitored in post-anaesthesia care unit.
  - `WardHandoverComplete`: Post-operative clinical handover to inpatient ward completed.

4.8 Clinic / Operational Queue Progression Process

- **Owning Capability**: `Clinic&PracticeOperations`
- **Process Purpose**: Governs the movement and state tracking of outpatients through an ambulatory specialty clinic session.
- **State Progression Lifecycle**: Scheduled → Patient Arrived → Checked-In → Roomed / Pre-Consult → Consultation In-Progress → Consultation Concluded → Departed
- **Key State Dispositions**:
  - `Scheduled`: Appointment booked on session list.
  - `PatientArrived`: Patient enters waiting area.
  - `Checked-In`: Reception verifies demographics and marks patient ready.

- `Roomed/Pre-Consult`: Nursing staff record baseline vitals and room patient.
- `ConsultationIn-Progress`: Clinician actively conducting consultation.
- `ConsultationConcluded`: Clinical discussion finished; follow-up orders placed.
- `Departed`: Patient checks out and departs clinic facility.

4.9 Bed Turnover Process

- **Owning Capability**: `Bed&Care-PlaceManagement`
- **Process Purpose**: Governs the rapid physical turnover, cleaning, sanitisation, and re-allocation of inpatient and emergency care-places.
- **State Progression Lifecycle**: Bed Vacated → Cleaning Dispatched → Sanitisation In-Progress → Terminal Cleaning Verified → Bed Ready / Available → Bed Allocated / Occupied
- **Key State Dispositions**:
  - `BedVacated`: Prior patient discharged or transferred.
  - `CleaningDispatched`: Cleaning job allocated to environmental services team.
  - `SanitisationIn-Progress`: Ward cleaning or isolation decontamination underway.
  - `TerminalCleaningVerified`: Ward coordinator or supervisor verifies care-place readiness.
  - `BedReady/Available`: Bed marked available for incoming patient allocation.
  - `BedAllocated/Occupied`: Incoming patient allocated and physically placed in bed.

4.10 Operational Work Progression Process

- **Owning Capability**: `WorkAllocation&Dispatch`
- **Process Purpose**: Governs the dispatch, assignment, execution, and completion of facility operational jobs (e.g., equipment moves, waste removal, linen supply).
- **State Progression Lifecycle**: Work Requested → Queued in Dispatch → Assigned to Worker → Job Accepted → Work In-Progress → Work Completed / Aborted
- **Key State Dispositions**:
  - `WorkRequested`: Facility support task submitted by clinical unit.
  - `QueuedinDispatch`: Job prioritized in operational queue.
  - `AssignedtoWorker`: Job allocated to specific staff member or team.
  - `JobAccepted`: Worker acknowledges dispatch on mobile device.
  - `WorkIn-Progress`: Task actively being executed.
  - `WorkCompleted/Aborted`: Job marked finished or formally cancelled with reason.

4.11 Patient Transport Process

- **Owning Capability**: `PatientTransport`
- **Process Purpose**: Governs the physical portering and transport of patients between hospital departments, wards, diagnostic suites, and external facilities.
- **State Progression Lifecycle**: Transport Requested → Porter Dispatched → Patient Collected → In-Transit → Delivered at Destination → Clinical Handover Complete
- **Key State Dispositions**:
  - `TransportRequested`: Clinical unit books wheelchair, bed, or ambulance transport.
  - `PorterDispatched`: Wardsperson assigned and en route to pickup location.
  - `PatientCollected`: Patient identity verified and transfer initiated.
  - `In-Transit`: Patient actively moving through facility corridors or ambulance transit.
  - `DeliveredatDestination`: Patient arrives at target diagnostic suite or ward.
  - `ClinicalHandoverComplete`: Safe custody and clinical briefing transferred to receiving staff.

4.12 Specimen Transport Process

- **Owning Capability**: `ClinicalLogisticsCoordination`

- **Process Purpose**: Governs the physical courier chain-of-custody, transport, and delivery of pathology bio-specimens, blood products, and surgical biopsies.
- **State Progression Lifecycle**: Specimen Packaged → Courier Dispatched → Chain-of-Custody Signed → In-Transit (Monitored) → Lab Ingress Received → Specimen Accepted
- **Key State Dispositions**:
  - `SpecimenPackaged`: Clinical unit packages bio-sample with barcode manifest.
  - `CourierDispatched`: Courier route assigned for pickup.
  - `Chain-of-CustodySigned`: Courier scans barcode and accepts legal custody.
  - `In-Transit(Monitored)`: Specimen in transit under temperature/time constraints.
  - `LabIngressReceived`: Central pathology specimen reception logs physical receipt.
  - `SpecimenAccepted`: Specimen integrity verified and passed to analytical track.

4.13 Discharge Progression Process

- **Owning Capability**: `DischargeManagement`
- **Process Purpose**: Governs the multidisciplinary coordination of patient discharge planning, pharmacy reconciliation, transport, and community handover.
- **State Progression Lifecycle**: Discharge Planning Initiated → Clinical Readiness Confirmed → Medications Reconciled → Transport & Services Booked → Discharge Summary Signed → Physically Departed (Illustrative preparation; publication, authorisation and exit distinct)
- **Key State Dispositions**:
  - `DischargePlanningInitiated`: Estimated Date of Discharge (EDD) set on admission.
  - `ClinicalReadinessConfirmed`: Attending medical team declares patient fit for discharge.
  - `MedicationsReconciled`: Hospital pharmacy reconciles and dispenses discharge medications.
  - `Transport&ServicesBooked`: Community nursing, home equipment, and patient transport booked.
  - `DischargeSummarySigned`: Applicable signed summary is available/published and may be communicated before departure where the Business behaviour requires it; signing, finalisation, publication and discharge authorisation remain distinct.
  - `PhysicallyDeparted`: Confirmed physical exit triggers departure notifications, updated bed-state communication and finalised-summary dispatch to external care providers under `FEAT-HSO-29`; care-place turnover follows its own responsibility.
- **Timing reconciliation**: Preparation, information availability/publication, applicable discharge authorisation and physical exit are distinct. Earlier preparation/communication is permitted; it does not realise a Feature triggered by confirmed exit. This illustrated local pathway establishes neither universal publication timing nor a universal discharge state machine, and adds no inferred publication event or authorisation state.
- **Preserved uncertainty**: Summary signing and finalisation are not universal equivalents. Detailed applicability, discharge-authority workflows and timing beyond the established preparation/confirmed-exit distinction remain unestablished.

5. Horizontal Workflow Coordination Processes

5.14 Work Order Progression Process

- **Owning Capability**: `Workflow&ActivityCoordination`
- **Activity Archetype**: **Human Doing** — physical or operational task executed by healthcare staff.
- **State Progression Lifecycle**: Created → Ready → Dispatched → Accepted → In-Progress → Completed / Failed / Cancelled
- **Key State Dispositions**:
  - `Created`: Work order instantiated with scope, target, and required skills.
  - `Ready`: Preconditions satisfied; available for assignment.
  - `Dispatched`: Sent to target individual or worker group queue.
  - `Accepted`: Worker claims task.

- `In-Progress`: Execution underway.
- `Completed`: Task finished with recorded completion telemetry.

5.15 To Do Progression Process

- **Owning Capability**: `Workflow&ActivityCoordination`
- **Activity Archetype**: **Human Reviewing / Updating / Deciding** — clinical review, document countersignature, or administrative authorization task.
- **State Progression Lifecycle**:

RAD diagram: textual source notation.

```
graph LR
    I["Issued"] --> B["In-Inbox"]
    B --> R["Opened / Under-Review"]
    R --> A["Actioned / Decided"]
    D["Dismissed / Delegated<br/>Disposition/position unresolved"]
    NOTE["Illustrative progression, not a normative transition model.<br/>Relationship of dismissal/delegation
    ↪  to action/decision is unestablished."]
    classDef note fill:#fff8dc,stroke:#8a6d3b,stroke-dasharray:5 5;
    class NOTE,D note;
```

- **Key State Dispositions**:
  - `Issued`: To Do generated (e.g., review abnormal potassium result).
  - `In-Inbox`: Displayed in practitioner's actionable task list.
  - `Opened/Under-Review`: Clinician inspecting relevant clinical context.
  - `Actioned/Decided`: Decision executed (e.g., signed off, order placed, acknowledged).
  - `Dismissed/Delegated`: Task reassigned to registrar or dismissed with comment.
- **Preserved uncertainty**: Whether dismissal/delegation is an alternative disposition, a subsequent action or a scoped variant remains unresolved; its disconnected diagram node establishes no transition or position in the illustrative progression.

5.16 Synthetic Task Progression Process

- **Owning Capability**: `Workflow&ActivityCoordination`
- **Activity Archetype**: **Non-Human Executable Work** — automated system workflows, batch syndications, or policy evaluation tasks.
- **State Progression Lifecycle**:

RAD diagram: textual source notation.

```
graph LR
    D["Draft / Scheduled"] --> E["Executing"]
    E --> W["Awaiting-Dependency"]
    W --> C["Completed"]
    F["Progression Stalled / Failed<br/>Outcome/position unresolved"]
    NOTE["Illustrative progression, not a normative transition model.<br/>No Completed-to-Stalled/Failed
    ↪  transition is established."]
    classDef note fill:#fff8dc,stroke:#8a6d3b,stroke-dasharray:5 5;
    class NOTE,F note;
```

- **Key State Dispositions**:
  - `Draft/Scheduled`: Automated task instantiated and scheduled for processing.
  - `Executing`: System actively processing task.
  - `Awaiting-Dependency`: Paused awaiting asynchronous external reply or event.
  - `Completed`: Automated execution succeeded and output artifact produced.
  - `ProgressionStalled/Failed`: Transient fault triggers scheduled recovery attempt; permanent failure records failure evidence.
- **Preserved uncertainty**: Whether stalled/failed is an alternative outcome, reopening/post-completion behaviour or a presentation defect remains unresolved. Its disconnected diagram node does not make success and failure equivalent or establish post-completion transitions.

6. Health Service Assurance Processes

The approved human review decisions on 2026-10-08 establish exactly the following three assurance Processes, building on the assurance Strategy and the two established assurance Roles and five Functions. **Element Type: Business Process (PR)** applies to each. Capability ownership below follows the established responsibility; **Feature association is not established** and **Canonical IDs remain unresolved** because Capability Tier, complete ancestry and local identifier allocation remain unestablished. Catalogue numbering is navigation, not identifier allocation.

6.1 Assurance Process Design

- **Canonical Name**: `AssuranceProcessDesign`.
- **Owning Capability**: Assurance Design.
- **Process Purpose**: Establishes and maintains how satisfaction of a governed requirement, constraint or expected behaviour is to be assured.
- **Principal Performing Role**: Service Assurance Modeller.
- **Participating Functions**: Design Service Assurance and Manage Assurance Criteria. Manage Assurance Criteria retains ownership within Assurance Criteria Management; its participation does not subsume that Capability's criteria lifecycle or transfer its information responsibility to Assurance Design.
- **Conceptual Output**: Assurance Definition, retaining its conceptual Business Architecture output/boundary treatment.
- **Boundary**: Governance Authority establishes what is authoritative. Design does not grant Service Assurance Modeller approval authority. Detailed Assurance Definition lifecycle, approval/publication workflows, criteria lifecycle and information modelling remain unresolved. Participation of the two Functions does not establish a cross-capability lifecycle or a new exchange/Service between their owners.

6.2 Assurance Process Execution

- **Canonical Name**: `AssuranceProcessExecution`.
- **Owning Capability**: Governed Assurance.
- **Process Purpose**: Performs defined assurance for an identifiable governed subject in accordance with an applicable Assurance Definition and establishes an Assurance Finding / Conclusion from applicable trustworthy evidence.
- **Principal Performing Role**: Service Guardian.
- **Participating Functions**: Establish Assurance Context, Assess Assurance Evidence and Adjudicate Assurance Assessment, all retaining Governed Assurance ownership.
- **Conceptual Basis and Output**: Applicable Assurance Definition plus identifiable governed subject; Assurance Finding / Conclusion established through adjudication where sufficient trustworthy evidence permits. Correct execution with insufficient evidence does not imply satisfaction or non-satisfaction or force a conclusion.
- **Boundary**: Function completion does not define Process lifecycle states. No Context Established, Evidence Assessed, Adjudicated or Concluded states are inferred. Detailed lifecycle states, transitions and initiation mechanics remain unresolved. Requesting this Process does not grant control of evidence assessment, adjudication or the resulting finding/conclusion.

RAD diagram: textual source notation.

```
graph TD
    DEF["Applicable Assurance Definition<br/>Conceptual basis"] -->|Assurance basis| EXEC["PR: Assurance Process
    ↪  Execution<br/>Owner: Governed Assurance"]
    SUBJECT["Identifiable Governed Subject"] -->|Subject of assurance| EXEC
    EXEC -->|Participating Function| CONTEXT["FN: Establish Assurance Context"]
    EXEC -->|Participating Function| ASSESS["FN: Assess Assurance Evidence"]
    EXEC -->|Participating Function| ADJ["FN: Adjudicate Assurance Assessment"]
    EXEC -->|Establishes through adjudication| FINDING["Assurance Finding / Conclusion"]
```

6.3 Assurance Process Reporting / Communication

- **Canonical Name**: `AssuranceProcessReporting/Communication`.
- **Owning Capability**: Governed Assurance.
- **Process Purpose**: Communicates applicable established Assurance Findings and Conclusions to legitimate recipients according to their responsibilities, authority and information requirements, without altering the established assurance outcome or assuming responsibility for the recipient's subsequent action.
- **Starting Basis**: An already established Assurance Finding / Conclusion. This Process does not perform assessment or adjudication, alter or reinterpret the established outcome.
- **Communication Participation**: Service Guardian participates through Service Assurance Outcome Communication, associated with Communicate Service Assurance Outcome. No additional Function is introduced to populate this Process.
- **Boundary**: Reporting / Communication is not limited to documentary report production; an appropriate Business Interaction may communicate the outcome without a report document. It assumes no operational response, remediation, escalation, policy/compliance response, process improvement, clinical management, clinical judgement or other recipient action. Detailed communication progression and completion semantics remain unresolved.

6.4 Process-Family Relationship and Preserved Boundaries

RAD diagram: textual source notation.

```
graph TD
    DESIGN["PR: Assurance Process Design<br/>Owner: Assurance Design"] -->|Conceptual output| DEF["Assurance
    ↪  Definition"]
    DEF -->|Reusable assurance basis| EXEC["PR: Assurance Process Execution<br/>Owner: Governed Assurance"]
    EXEC -->|Established through adjudication| FINDING["Assurance Finding / Conclusion"]
    FINDING -->|Already established outcome| COMM["PR: Assurance Process Reporting / Communication<br/>Owner:
    ↪  Governed Assurance"]
```

Both diagrams express semantic Business Architecture relationships, not lifecycle states, mandatory runtime topology, application orchestration, deployment sequence, component structure, message flow or a complete implementation workflow. An applicable Assurance Definition may be reused by multiple Execution instances; each assurance activity does not require a newly executed Design instance. Using the modelling output establishes a responsibility/information dependency, not a Modeller-to-Guardian Interaction or Collaboration.

> **Governed Assurance does not recursively assure its own execution. Assurance activity execution integrity is provided by the established activity execution framework.**

Managed progression of assurance activity does not make that execution another Service Assurance subject. No recursive assurance Process, Function, Service or conclusion is established; the activity execution framework reference allocates no solution component. System Steward's status observation is management/stewardship of progression, not assurance of assurance. See the non-recursion boundary.

The sixteen pre-existing Processes retain their owners, definitions and state models. Clinical review, sign-off, countersignature and clinical decisions remain with applicable clinical processes/authorities; To Do completion is not an independent assurance conclusion. Generic Work Order, To Do and Synthetic Task progression do not supply assurance lifecycle states or an assurance execution allocation.

Service Guardian may manage its own assurance progression without managing its subject. Neither assurance Role acquires the subject's progression, assignment, delegation, failure/recovery, operational escalation, remediation or improvement responsibility. Execution failure/indeterminate outcome remain distinct from correctly executed assurance with insufficient evidence. Remaining decisions include lifecycle states/transitions, initiation mechanics, detailed information semantics and response mechanisms; the three Process names and purposes are resolved.

16. Health Service Assurance — Bounded Business Architecture Derivation

RAD source: `docs/markdown/03-business-architecture/behaviours/health-service-assurance.md`

The following is a presentation transcription of the governed source. Source catalogue layout and retained illustrations do not allocate tiers or establish additional identities. Equations wrap as text; diagram notation is textual.

Health Service Assurance — Bounded Business Architecture Derivation

1. Authority and Derivation Scope

This view carries the approved Domain02 Health Service Assurance derivation into Domain03. Its upstream responsibility chain is REQ-FND-005 — Independent Assurance of Governed Activity → BC-18 Health Service Assurance → **Assurance Design**, **Assurance Criteria Management** and **Governed Assurance**, with the approved collaborative Enterprise Capability contributions including **EC-14 Service Guardian**.

AX-14 requires the responsibility distinctions below to remain explicit. AX-17 and the Domain03 metamodel require established responsibility to be distinguished from unestablished naming, decomposition, exposure, progression and authority relationships. AX-06, AX-07, AX-08 and AX-09 continue to govern information authority, governed access, meaningful evidence and contextual evidence selection. AX-15 governs operational uncertainty without making it equivalent to insufficient assurance evidence.

The approved human review decisions on 2026-10-08 extend the previous bounded derivation with **Service Assurance Modeller** and **Service Guardian** Business Roles and exactly five Business Functions: **Design Service Assurance**, **Manage Assurance Criteria**, **Establish Assurance Context**, **Assess Assurance Evidence** and **Adjudicate Assurance Assessment**. Their capability ownership and Role performance are documented in §3. The assurance Role previously named Guardian is now Service Guardian; legal/personal-welfare Guardian remains unchanged. These naming and Function-decomposition decisions are resolved; further decomposition is not approved.

The next approved human review decisions on 2026-10-08 establish exactly three Business Processes, three Business Services and three Business Interactions, with the approved consumer/participant relationships. They also establish the Financial Governance deliberate exclusion and assurance non-recursion boundary. The five approved Functions are preserved without reinterpretation or further decomposition.

The three assurance capabilities remain established responsibility contexts with unresolved contextual-view placement, Capability Tier, complete ancestry, Feature decomposition and structural Canonical IDs. This document is a capability-scoped view, not a sixth contextual view or an identifier ancestor. Specific Actors and Collaborations remain unallocated; detailed Process states/transitions, information models and implementation remain unresolved.

EC-14 Service Guardian is an Enterprise Capability. Service Guardian is also the intentional canonical name of a distinct Business Role. Shared terminology does not equate their architectural types or make either a software component. The approved human review decisions supply the Business Architecture relationships; they are not inferred from the EC contribution model or its explanatory sequence. Previous completion reports supply execution context only and are not architectural authority.

2. Governance, Management, Guardianship and Clinical Authority

RAD source listing (presentation only; no inferred ancestry).

Governance defines

↓

Management performs

↓

Guardianship assures

This expresses semantic responsibilities, not organisational structure, a compulsory execution sequence or technical topology.

Responsibility	Business meaning	Preserved boundary
Governance	Defines applicable requirements, obligations, authority, constraints, controls, reporting requirements and checkpoints.	Establishes the normative envelope without taking over the operational means or progression of the governed activity. Subject-specific governance remains intrinsic to its established subject capability.
Operational management	Performs or manages the subject activity within its constraints, including monitoring for progression, assignment, delegation, operational escalation, response, remediation and expected failure/recovery.	Monitoring for operational completion or deciding what happens next does not establish independent assurance.
Service Assurance modelling	Service Assurance Modeller determines and models how satisfaction of governed requirements, constraints and expected behaviours is to be assured within Assurance Design and Assurance Criteria Management.	Modelling does not inherently approve the governing requirement, Assurance Definition or Assurance Criteria, perform independent assurance, or manage the subject.
Guardianship / governed assurance	Service Guardian independently evaluates governed activity, information, state or outcomes against applicable criteria and establishes findings and conclusions from sufficient trustworthy evidence.	Service Guardian may manage its own assurance activity; it does not thereby perform or manage the subject, direct its operational response or confer clinical authority.
Clinical review / clinical assurance	Exercises the applicable clinical process and authority's responsibility for clinical evaluation and professional judgement.	Neither assurance Role establishes clinical adequacy, clinical correctness or professional clinical judgement. Clinical quality, safety and improvement retain their existing responsibilities.

Governance Authority establishes what is authoritative. Service Assurance Modeller models how satisfaction will be assured. Service Guardian independently establishes what the applicable evidence demonstrates. Governance Authority references the existing mandated authority Roles, including Policy Authority for policy and Regulator for statutory authority, with existing steward mandates preserved. No new Governance Authority Role or extension of those mandates is established. Modelling does not grant approval authority for the governing requirement, Assurance Definition or Assurance Criteria; exact approval allocation/workflows remain unresolved.

RAD diagram: textual source notation.

```
graph TD
    GOV["Governance Authority<br/>Existing mandated authority Roles"] -->|Establishes authoritative basis|
    ↪  MOD["Business Role<br/>Service Assurance Modeller"]
```

```
MOD -->|Performs| DESIGN["Function: Design Service Assurance<br/>Owner: Assurance Design"]
MOD -->|Performs| CRITERIA["Function: Manage Assurance Criteria<br/>Owner: Assurance Criteria Management"]
MOD -->|Models| DEF["Assurance Definition<br/>Conceptual modelling output"]
DEF -->|Modelling boundary for assurance| GUARD["Business Role<br/>Service Guardian"]
GUARD -->|Performs| CONTEXT["Function: Establish Assurance Context<br/>Owner: Governed Assurance"]
GUARD -->|Performs| ASSESS["Function: Assess Assurance Evidence<br/>Owner: Governed Assurance"]
GUARD -->|Performs| ADJ["Function: Adjudicate Assurance Assessment<br/>Owner: Governed Assurance"]
ADJ -->|Establishes| FINDING["Assurance Finding / Conclusion"]
```

This is a responsibility/semantic model, not mandatory execution topology, organisational or application structure, deployment topology or a complete Business Process. The output labels do not introduce Information Objects or a formal information model. The diagram distinguishes performance through a Role from Function ownership by a Capability and does not define process ordering or an assurance initiation/approval exchange.

A real-world Actor may fulfil different Roles in different contexts where governance permits. That possibility does not relax REQ-FND-005: assurance progression and conclusion must remain independently governed rather than controlled by the performer or manager of the subject. The subject may contribute evidence but must not solely determine, suppress, manufacture or retrospectively alter its assurance conclusion. Evidence dependency and control dependency remain distinct.

Neither Service Assurance Modeller nor Service Guardian inherently possesses clinical authority or replaces professional clinical judgement, clinical peer review or clinical governance. Information and behaviour concerning clinical activity may be subject to governed assurance where an explicit applicable requirement establishes that concern. This does not constitute **Clinical Services Delivery Assurance** or transfer clinical-care responsibility to Guardianship or Harmonia. The approved clinical boundary, Service Delivery care-enablement principle, and existing clinical review responsibilities remain controlling. No generic clinical-assurance authority Role is derived.

Neither assurance Role manages the subject merely because it designs or performs assurance. Service Guardian may manage progression of its own assurance activity without assuming progression of its subject. Findings may cause another responsible party to initiate management, escalation, remediation or other operational action; no such response responsibility is allocated to Service Guardian.

3. Direct Capability-Scoped Responsibility Consequences

The following five Functions are approved Business Architecture. Each is behaviour within its established owning Capability, performed through the stated Business Role. **Element Type: Business Function (FN)** applies to each. **Canonical IDs remain unresolved** because Capability Tier, complete ancestry and local identifier allocation are unestablished. **Feature association is not established** for any of these Functions; no Feature or structural ID is invented to anchor them. Function ownership does not transfer source-information ownership to an assurance Role or Capability.

3.1 Assurance Design — Service Assurance Modeller

- **Owning Capability**: Assurance Design.
- **Performing Business Role**: Service Assurance Modeller.

Design Service Assurance

- **Canonical Name**: `DesignServiceAssurance`.
- **Definition**: Defines how satisfaction of a governed requirement, constraint or expected behaviour is to be independently established, including the applicable assurance disposition, assurance criteria, required evidence and evaluation expectations.

- **Boundary**: Concerns assurance design. It does not establish the underlying governance requirement, grant authority to it, perform the resulting assurance, manage the subject activity or prescribe application or implementation mechanisms. Modelling the resulting Assurance Definition does not grant approval authority. The Strategy peer relationship to behaviour design and failure/recovery design creates no additional Capability or Function.

3.2 Assurance Criteria Management — Service Assurance Modeller

- **Owning Capability**: Assurance Criteria Management.
- **Performing Business Role**: Service Assurance Modeller.

Manage Assurance Criteria

- **Canonical Name**: `ManageAssuranceCriteria`.
- **Definition**: Establishes and manages reusable, governed and temporally identifiable assurance criteria through which governed subjects may be evaluated.
- **Scope**: Criteria may express governance rule boundaries, quality constraints, performance expectations, compliance criteria, temporal criteria, evidence requirements or completeness expectations. These examples are not an exhaustive taxonomy. Criteria may have lifecycle, version and temporal applicability.
- **Boundary**: Managing a criterion does not transfer authority for the governing requirement from which it may derive. Criteria may operationalise a requirement without silently altering, weakening, strengthening or replacing it; not every criterion must derive from formal Governance. Modelling/managing criteria does not inherently confer approval authority. Detailed criterion lifecycle states and approval workflows remain unresolved.

3.3 Governed Assurance — Service Guardian

- **Owning Capability**: Governed Assurance.
- **Performing Business Role**: Service Guardian.

Establish Assurance Context

- **Canonical Name**: `EstablishAssuranceContext`.
- **Definition**: Establishes the governed subject, applicable assurance purpose and criteria, relevant temporal context, and contextual association of sufficient trustworthy source information as evidence required for an assurance activity.
- **Scope**: Includes the evidence-assembly responsibility previously described as Assurance Evidence Assembly; no separate evidence-assembly Function is established. Sufficiency is required for conclusion, not assumed merely because context is established; unavailable or insufficient evidence remains explicit.
- **Information Boundary**: Source information remains owned by its responsible source Capability. Its use as evidence does not transfer ownership or originating authority. Governed Assurance owns the contextual association that particular information constitutes evidence for a particular assurance purpose. Evidence association and applicable criteria may each be temporal and version-specific; association distinguishes the information state relevant to assurance from its current state.
- **Realisation Boundary**: Uses generic EC-02 context-management capability together with EC-14 assurance-specific semantics as documented in §4.1. Establishment of Assurance Context prescribes no copying, persistence, aggregation, caching or other technical mechanism.

Assess Assurance Evidence

- **Canonical Name**: `AssessAssuranceEvidence`.
- **Definition**: Evaluates the available trustworthy evidence against the applicable assurance criteria and establishes what that evidence demonstrates for each relevant criterion or assessment concern.
- **Boundary**: Assessment remains distinct from adjudication. It may identify satisfaction, exception, insufficiency or other criterion-level results where applicable; no universal result taxonomy is established. Quantitative, qualitative, compound, temporal and confidence-bearing assessment remain possible. **Insufficient evidence is not equivalent to either satisfaction or non-satisfaction.**

Adjudicate Assurance Assessment

- **Canonical Name**: `AdjudicateAssuranceAssessment`.
- **Definition**: Determines the assurance finding or conclusion that follows from the assessed evidence and applicable assurance criteria.
- **Boundary**: Establishment of the finding/conclusion is part of adjudication. It remains distinct from evidence assessment, operational response, remediation, escalation, workflow progression and clinical judgement. No separate conclusion-establishment or reporting Function is introduced merely because a conclusion is established or may be communicated.

3.4 Preserved Assurance Obligations

The approved assurance dispositions remain explicit governed decisions. **Not Assurable** does not remove the requirement; **Not Worth Assuring** does not make it optional. Disposition does not require specific assurance of every individual activity, information item or outcome.

The approved generic processing assurance obligation constrains Harmonia-managed workflow behaviour. Evaluating business progression, completion, timeliness, pathways, failure/recovery and outcomes for assurance is distinct from monitoring to progress that work. The existing operational Processes and Functions retain their owners; neither successful completion nor an operationally healthy environment establishes an assurance conclusion. No final criterion catalogue or applicability model is supplied by these examples.

Assessment asks what the evidence demonstrates against applicable criteria; adjudication establishes what finding or conclusion follows. The approved decisions now establish these as distinct Functions performed through Service Guardian; they do not require separate Roles or process stages. Assurance is not universally reducible to PASS / FAIL / UNKNOWN. An assurance activity may execute correctly while lacking enough evidence to conclude. That insufficiency is distinct from its own execution failure or indeterminate execution outcome and must not imply satisfaction or non-satisfaction.

4. Information and Dependency Consequences

The Business Information Responsibility material records the directly supported demarcations: Assurance Design defines how satisfaction will be assured; Assurance Criteria Management manages criteria; Governed Assurance owns contextual evidentiary association and its findings/conclusions. Underlying source information remains owned and managed by its established capability. Evaluating it does not acquire its originating authority.

Evidence association must distinguish the relevant information state from its current state where temporal/version-specific inclusion matters. Evaluation must use temporally appropriate evidence and criteria. Evidence collection, provenance, audit information and operational completion are not themselves assurance conclusions. No assurance Information Object, outcome taxonomy, representation, custody, retention rule or persistence structure is defined here.

An evidence need does not identify a Business Service/provider or authorise a new consumption edge. The dependency assessment preserves that distinction, including the prohibition on

subject control of assurance progression or conclusion. The approved Enterprise Capability contribution matrix remains a Strategy contribution model, not a Business Service consumption matrix.

4.1 Context Management Contribution

EC-02 Context Management supplies generic establishment, validation, propagation and preservation of meaningful context. EC-14 Service Guardian supplies assurance-specific subject/purpose, criterion applicability and contextual evidentiary-association semantics. Their contributions together support **Establish Assurance Context**, owned by Governed Assurance and performed through the Service Guardian Business Role.

RAD diagram: textual source notation.

```
graph TD
    CONTEXT["Enterprise Capability<br/>EC-02 Context Management"] -->|Generic context semantics| FN["Business
    ↪  Function<br/>Establish Assurance Context<br/>Owner: Governed Assurance"]
    ASSURANCE["Enterprise Capability<br/>EC-14 Service Guardian"] -->|Assurance-specific semantics| FN
```

This documents the approved semantic contribution relationship. In the Domain02 collaborative contribution matrix, EC-02 and EC-14 each contribute directly to Governed Assurance. Neither is a sole realiser or Function owner; the other approved capability contributions remain intact. This view is not an exhaustive Function-to-EC allocation matrix, a new Business Service consumption edge or an application/runtime allocation. How context establishment is realised belongs to later architecture; no loader/unloader mechanism or implementation pattern is derived.

4.2 Assurance Definition — Conceptual Modelling Output

Assurance Definition describes the conceptual output/boundary between Service Assurance modelling and governed assurance: how satisfaction is to be assured, including assurance disposition, applicable criteria, required evidence and evaluation expectations. This output description follows directly from **Design Service Assurance** within **Assurance Design**, performed through **Service Assurance Modeller**; governed assurance uses that modelling boundary without transferring governance authority or establishing an exposed Service or approval exchange.

The existing Domain03 rules permit documenting this business output and its responsibility context. They do not establish its formal information structure, identity, lifecycle, representation or approval model. The term therefore remains a conceptual responsibility/output description, not a newly allocated formal Business Information element, Information Object or Data Object. Formal information modelling is deferred. Modelling an Assurance Definition does not make it authoritative or grant its modeller approval authority; applicable governance authority must establish that standing.

5. Approved Governed Assurance Business Services

Exactly three assurance Services are established below. **Element Type: Business Service (SV)** applies to each; each is owned/exposed by **Governed Assurance** with Service Guardian participating through its associated Business Interaction. Identifiable consumers/recipients outside that responsibility boundary justify the exposure under the metamodel. **Feature association is not established** and **Canonical IDs remain unresolved**; no Capability Tier, ancestry or local code is invented.

The Services expose approved request, activity-progression observation and established-outcome communication behaviour. They do not mechanically correspond one-to-one with the five Functions, nor introduce new request, status or reporting Functions. Detailed internal Service-to-Function mappings and contracts beyond the approved behaviour remain unestablished. Status observation concerns assurance's own permitted activity progression, not assessment of the subject or a new status Process.

5.1 Request Service Assurance

- **Canonical Name**: `RequestServiceAssurance`.
- **Definition**: Enables an authorised consumer to request that Service Assurance be performed for an applicable governed subject in accordance with an established Assurance Definition.
- **Approved Consumers**: `CareCoordinator`, `ServiceCoordinator`, `Regulator`, `PolicyAuthority`, `SystemSteward`.
- **Associated Interaction**: Service Assurance Request, with Service Guardian and one authorised requesting Role.
- **Process Relationship**: Requests Assurance Process Execution under an applicable established Assurance Definition. It does not require a new Assurance Process Design instance for each request.
- **Independence Boundary**: The requesting Role identifies the assurance need within its responsibility and authority. Requesting does not grant authority to define or alter the Assurance Definition, alter Assurance Criteria, direct evidence assessment, determine adjudication or control the resulting Assurance Finding / Conclusion. REQ-FND-005 control independence remains applicable, including where a requester performs or manages the subject. Detailed payloads, contracts, initiation mechanics and states remain unresolved.

5.2 Request Service Assurance Status

- **Canonical Name**: `RequestServiceAssuranceStatus`.
- **Definition**: Enables the System Steward to obtain the current business progression state of an identifiable Service Assurance activity for stewardship purposes.
- **Only Approved Consumer**: `SystemSteward`.
- **Associated Interaction**: Service Assurance Status, between System Steward and Service Guardian.
- **Status Boundary**: Concerns **activity progression state**. It does not communicate provisional findings, incomplete assessment results as conclusions, predicted outcomes or unadjudicated conclusions. Other requesting Roles do not gain status access merely because they requested assurance. No detailed activity-status taxonomy is established.

Service Assurance Status answers ”where is the assurance activity in its progression?” It does not answer ”what has the assurance established?” The latter requires an adjudicated Assurance Finding / Conclusion and subsequent outcome communication; neither status nor execution completion establishes that conclusion.

Management Monitoring evaluates activity state in order to progress or manage activity. Assurance evaluates activity against governing criteria in order to establish an assurance conclusion. System Steward’s use of this Service is stewardship / management observation of assurance progression, not itself assurance or recursive assurance. The non-recursion boundary in §7.2 applies.

5.3 Communicate Service Assurance Outcome

- **Canonical Name**: `CommunicateServiceAssuranceOutcome`.
- **Definition**: Communicates applicable established Assurance Findings and Conclusions to authorised recipients according to their responsibilities, authority and information requirements.
- **Approved Potential Recipients**: `CareCoordinator`, `ServiceCoordinator`, `Regulator`, `PolicyAuthority`, `SystemSteward`.
- **Associated Interaction**: Service Assurance Outcome Communication, with Service Guardian and one or more authorised recipient Roles.
- **Process Relationship**: Exposes the communication behaviour of Assurance Process Reporting / Communication, beginning from an already established finding/conclusion. It does not perform assessment or adjudication or alter/reinterpret the established outcome. A documentary report is not required.

- **Recipient Boundary**: No mandatory broadcast is established. A particular outcome may be communicated to one, some or all legitimate recipients according to applicability, authority, responsibility and information requirements. Requester, status consumer and outcome recipient need not be the same Role. Communication does not transfer responsibility for recipients' interpretation within their authority, operational/clinical management, remediation, escalation, policy/compliance response, process improvement or other subsequent action to Service Guardian.

5.4 Approved Consumer and Recipient Concerns

The existing Role definitions and mandates remain controlling. These are approved consumer/recipient concerns, not exhaustive Role responsibilities or new Roles. Each use requires applicable authority; Role eligibility alone does not grant unrestricted information access.

Business Role	Request Service Assurance	Request Service Assurance Status	Potential recipient of Communicate Service Assurance Outcome	Principal assurance concern
Care Coordinator	Approved within authority	Not approved	Approved where applicable/ authorised	Clinical management
Service Coordinator	Approved within authority	Not approved	Approved where applicable/ authorised	Operational management and potentially task / people process improvement
Regulator	Approved within authority	Not approved	Approved where applicable/ authorised	Service governance / compliance
Policy Authority	Approved within authority	Not approved	Approved where applicable/ authorised	Service governance / compliance and policy effectiveness / improvement
System Steward	Approved within authority	Only approved consumer	Approved where applicable/ authorised	ICT process improvement and stewardship of assurance activity progression

Care Coordinator's request or receipt may inform Clinical Management; clinical interpretation, judgement and action remain with the applicable clinical authority. Service Coordinator retains operational management/improvement responsibilities; System Steward's status access does not make Service Guardian responsible for task progression, assignment, delegation, operational escalation, remediation or process improvement. Regulator and Policy Authority retain their established governance/compliance mandates. None of these consumer relationships transfers source-information ownership, governance authority or control of assurance assessment/adjudication.

6. Additional Business Elements Not Yet Established

The two assurance Roles, five Functions, three Processes, three Services, three Interactions and approved Role relationships are established. The following further concerns remain explicitly **unresolved and non-authoritative**; their inclusion does not add them to a catalogue. Financial Governance's scope exclusion and assurance non-recursion are settled boundaries, not gaps.

Candidate concern assessed	What approved architecture establishes	Architectural decision required before adding an element
Further Function decomposition / Feature association	Exactly the five approved Functions in §3, including evidence assembly within context establishment and conclusion establishment within adjudication.	Additional Functions and Feature associations require a further decision. No separate applicability/criteria-resolution, evidence-assembly, conclusion-establishment, reporting, remediation or escalation Function is approved.
Further Business Services / contracts	Exactly the three Services in §5 with their approved consumers/ recipients, owner and behaviour.	Additional criteria/evidence Services, exact evidence/criteria providers, detailed Service contracts and internal Service-to-Function mappings are unestablished. No extra Service is inferred from reportability, reuse or evidence need.
Detailed Process progression	Assurance Process Design, Execution and Reporting / Communication have established purposes, participating Functions where approved, conceptual boundaries and ownership.	Detailed lifecycle states/transitions, initiation mechanics, completion/ disposition semantics, criteria lifecycle and approval/publication workflows remain unresolved. Function completion, EC-14's sequence and existing coordination lifecycles do not supply assurance states.
Further Business Interactions	Service Assurance Request, Status and Outcome Communication with approved Role participants and associated Services.	Evidence acquisition/exchange Interactions remain unresolved; source-owned information may be available through existing governed information access. No Evidence Contribution or Modeller-to-Guardian Interaction is created from evidence association or use of an Assurance Definition. Further exchange commitments require separate approval.
Assurance Role collaborations	Roles participate in the three approved Interactions; capability realisation is collaborative.	Establish an enduring, structured Actor/ Role association with a defined purpose and permitted control relationships. Interaction does not mechanically imply Collaboration; no existing membership or new assurance collective is established.
Further responsibility relationships / dependencies	Approved request/status/ outcome Role relationships, governance mandates and independence; source ownership, operational management and clinical authority remain separate.	Detailed Actor eligibility/mandates beyond approved Roles, coverage/applicability, initiation mechanics, exact approval allocation, evidence/criteria providers, findings-to-operational-response mechanisms and exact authorised external financial-governance recipients remain unresolved. Financial governance responsibility itself is deliberately excluded.
Further Business Information responsibilities	Responsibility demarcations and contextual/temporal evidence obligations in §4; conceptual Assurance Definition; progression status distinct from established findings/conclusions and their communication.	Detailed request/status/outcome information semantics, formal Assurance Definition modelling, further information concepts/asset identity, custody/lifecycle, criterion catalogues, outcome taxonomies, confidence models and retention remain unresolved. No formal information element or source-information reassignment is inferred.

6.1 Previous Questions Resolved by Approved Human Review

Previous question	Approved resolution
Legal Guardian / assurance Guardian naming collision	**Resolved:** legal/personal-welfare Guardian remains intact; governed assurance uses the unambiguous **Service Guardian** Business Role. EC-14 retains the intentional shared name as a distinct Enterprise Capability.
Who models how satisfaction will be assured?	**Resolved: Service Assurance Modeller** performs Design Service Assurance within Assurance Design and Manage Assurance Criteria within Assurance Criteria Management, without inherent governance/approval authority.
Function names and bounded decomposition	**Resolved for this stage:** exactly five Functions are established in §3. Evidence assembly is included in Establish Assurance Context; finding/conclusion establishment is included in Adjudicate Assurance Assessment. No separate reporting Function is established.
Generic context management / assurance-specific context relationship	**Resolved to the approved extent:** EC-02 and EC-14 contribute together to Establish Assurance Context; sole realisation, Service exposure and implementation allocation are not implied.
Assurance Process names and purposes	**Resolved:** exactly Assurance Process Design, Assurance Process Execution and Assurance Process Reporting / Communication; detailed states/transitions remain unresolved.
Assurance Business Services and consumers	**Resolved:** Request Service Assurance for the five approved Roles; Request Service Assurance Status exclusively for System Steward; Communicate Service Assurance Outcome for applicable authorised recipients from the five Roles.
Assurance Business Interactions and participants	**Resolved:** Service Assurance Request, Service Assurance Status and Service Assurance Outcome Communication, with Service Guardian and the approved requesting/status/recipient Roles.
Assurance status versus outcome	**Resolved:** status concerns business progression only and exposes no unadjudicated finding/conclusion. Established-outcome communication remains separate and is not mandatory broadcast or responsibility for recipient action.
Financial Governance scope	**Deliberately excluded:** no distinct Harmonia financial-governance Role, Function, Capability, Service, Process or Collaboration. Financial relevance may justify authorised external receipt without responsibility transfer; exact external recipients remain unresolved.
Assurance recursion	**Resolved boundary:** Governed Assurance does not recursively assure its own execution; execution integrity belongs to the established activity execution framework without solution allocation.

Capability Tier, complete ancestry, contextual-view placement, Feature placement and structural Canonical IDs remain unresolved. Assurance Definition's formal information modelling remains deferred; documenting its conceptual output does not resolve those decisions. These bounded approvals do not complete Business Architecture or freeze/refreeze Domain03.

7. Deliberate Scope Boundaries

7.1 Financial Governance Exclusion

> **Financial Governance Exclusion** — Service Assurance findings may contain information that has financial relevance or may be consumed by external financial-governance activities. Harmonia does not establish a distinct Service Governance (Financials) Role, Function, Process or responsibility because financial governance is outside the presently established Harmonia business scope. Where financially relevant assurance outcomes are produced, they may be communicated to an appropriately authorised external recipient without Harmonia assuming financial-governance responsibility.

This is a deliberate scope exclusion, not an unresolved architectural gap. No Financial Governance Role, Function, Capability, Service, Process or Collaboration is created for symmetry. The allowance for authorised external receipt establishes no additional consumer Role or allocation in the approved Service/Interaction catalogues; exact external recipients and their mandates remain unresolved. Existing business responsibilities are preserved.

7.2 Assurance Non-Recursion Boundary

> **Governed Assurance does not recursively assure its own execution. Assurance activity execution integrity is provided by the established activity execution framework.**

This approved boundary distinguishes assurance execution integrity from independent evaluation of a governed subject. Managed execution/progression of an assurance activity does not make that execution another Service Assurance subject. No Assurance of Assurance, Guardian-of-Guardian, recursive Process, Function, Service or conclusion is introduced. System Steward's progression observation through Service Assurance Status is stewardship / management monitoring, not recursive assurance. The activity execution framework reference is solely a Business Architecture boundary and establishes no application/runtime responsibility allocation or implementation pattern.

8. Bounded Outcome and Navigation

The two assurance Roles and exactly five Functions, three Processes, three Services and three Interactions are established with approved Role relationships and capability, authority, clinical, operational and information boundaries. Further Business elements and detail remain unresolved as recorded above; Financial Governance is deliberately excluded and assurance non-recursion is established. Existing metamodel abbreviations **CT1** / **CT2** / **CT3** / **FT** and **FN** / **SV** / **PR**, established Feature identifiers and the Qualified Architectural Reference Grammar remain unchanged. No unresolved tier, ancestry, Feature placement or structural identifier is filled to accommodate assurance.

This derivation stops in Business Architecture. Information Architecture and all application, runtime, execution, representation, persistence and deployment allocation remain downstream. The completion report is execution evidence and confers no architectural authority.

- Domain03 orientation
- Capability-scoped behaviour index
- Service Assurance Modeller Role
- Service Guardian Role and boundaries
- Collaboration derivation boundary
- Approved assurance Services and consumer concerns
- Approved assurance Interactions
- Approved assurance Processes
- Financial Governance scope exclusion
- Assurance non-recursion boundary

17. Entity Management — Capability-Scoped Behaviour

RAD source: `docs/markdown/03-business-architecture/behaviours/01-entity-management.md`

The following is a presentation transcription of the governed source. Source catalogue layout and retained illustrations do not allocate tiers or establish additional identities. Equations wrap as text; diagram notation is textual.

Entity Management — Capability-Scoped Behaviour

1. Contextual Scope & Architectural Intent

The named owning Capabilities and established Features retain their responsibilities. Affected Capability Tier, complete ancestry, root status and structural Canonical IDs remain unresolved under approved G1 K9; document grouping and numbering do not establish architectural identity or hierarchy.

Entity Management establishes the canonical master data governance, identity correlation, and structural definitions for all core business entities participating in healthcare delivery: Clients/Patients, Healthcare Practitioners, Healthcare Organisations, Locations, Health Services, and Clinical Devices.

2. Client Administration

Client Administration governs the identity, demographic context, relationship associations, and privacy directives of persons receiving care.

2.1 Person Identity

- **Owning Capability**: `PersonIdentity` (under `ClientAdministration`)
- **Key Architectural Semantics**:
  - **Identifier Resolution**: Asks for authoritative detail and verification regarding a specific person identifier within a single domain namespace.
  - **Identifier Correlation**: Ingests and maintains cross-authority associations between identifier namespaces where the governing relationship is established by authoritative information.
  - **Identity Alias**: Contextual, legal, or alternative identity expressions (e.g., preferred names, maiden names, emergency aliases) by which a person is known. It is conceptually distinct from cross-domain correlation.
  - **R1.x/R2.x authority boundary**: Harmonia ingests and maintains authoritative identifiers, aliases and identity relationships, processes externally authoritative corrections/merge outcomes and propagates managed-state consequences. It does not originate probabilistic matching, golden-record determination, authoritative master-person selection or person-merge decisions. REQ-FND-003 and the Strategy scope statement govern this boundary.
- **Functions & Exposed Services**:
  - **Feature: Identifier Resolution**:
    - *Function*: `ResolvePersonIdentifier` — Validates and retrieves the authoritative demographic record bound to a specific identifier.
    - *Exposed Service*: `PersonIdentifierResolution` — Exposed to Service Administration and Service Delivery to disambiguate client identities.

- **Feature: Cross-Authority Identifier Correlation**:
  - *Function*: `CorrelatePersonIdentifiers` — Correlates local identifiers (e.g., Hospital MRN, National IHI, Pathology ID) using relationships established by authoritative information, retaining their source and namespace.
  - *Exposed Service*: `PersonIdentifierCorrelation` — Exposed across the HIE to establish patient cross-reference mappings.
- **Feature: Identity Alias Association**:
  - *Function*: `MaintainPersonIdentityAliases` — Records and qualifies alias names, titles, and emergency aliases for a governed person identity.
  - *Exposed Service*: *(Internal Function — accessed via Person Identifier Resolution)*.
- **Feature: Governed Identity Correction**:
  - *Function*: `ApplyGovernedPersonIdentityCorrection` — Processes externally authoritative corrections and merge/unlink outcomes, updates governed identity associations and propagates resulting managed-state consequences with audit history. Harmonia does not adjudicate the originating person-merge decision.
  - *Exposed Service*: `PersonIdentityCorrection` — Exposed to authorized HIM/Medical Records administrators.
  - *Governed Process*: **Governed Person Identity Correction Process**.
- **Information Responsibility**: Person Identity Record, Identifier Namespace Bindings, Cross-Authority Correlation Graph, Identity Aliases, Identity Merge/Split Audit Log.

2.2 Healthcare Subject

- **Owning Capability**: `HealthcareSubject`.
- **Owner derivation**: Strategy Healthcare Subject owns Subject Context Binding and Demographic Context Governance; the Functions below discharge those same responsibilities. “Healthcare Subject Context” describes information/behaviour context, not another Capability or alias.
- **Functions & Exposed Services**:
  - **Feature: Subject Context Binding**:
    - *Function*: `EstablishHealthcareSubjectContext` — Binds demographic, cultural, and communication requirements to a patient record.
    - *Exposed Service*: `HealthcareSubjectContextResolution` — Provides comprehensive demographic, language, and indigenous status context to clinical applications.
  - **Feature: Demographic Context Governance**:
    - *Function*: `GovernSubjectDemographicContext` — Applies validation rules to address changes, date of birth verification, and death notifications.
    - *Exposed Service*: `DemographicChangeDistribution` — Emits governed subject demographic updates to subscribed clinical systems.
- **Information Responsibility**: Healthcare Subject Profile, Demographic History, Communication Preferences.

2.3 Client Relationship

- **Owning Capability**: `ClientRelationship`.
- **Owner derivation**: Strategy Client Relationship owns Next-of-Kin & Guardian Association and Relationship Validity Tracking. Support-network wording describes those responsibilities without establishing a separate Capability, alias or specialisation.
- **Functions & Exposed Services**:
  - **Feature: Next-of-Kin & Guardian Association**:
    - *Function*: `MaintainClientRelationships` — Records links between patients and carers, next of kin, nominated representatives, and legal guardians.
    - *Exposed Service*: `ClientRelationshipQuery` — Discloses verified support network members and contact details to clinical teams.

- **Feature: Relationship Validity Tracking**:
  - *Function*: `GovernClientRelationshipValidity` — Verifies power-of-attorney documents, guardianship orders, and proxy validity periods.
  - *Exposed Service*: `RepresentativeAuthorityVerification` — Confirms legal decision-making authority for substitute consent.
- **Information Responsibility**: Client Relationship Record, Representative Legal Mandate, Carer Contact Directory.

2.4 Client Privacy

- **Owning Capability**: `ClientPrivacy`.
- **Owner derivation**: Strategy Client Privacy owns Consent Directive Evaluation and Confidentiality Flag Enforcement. Consent-directive wording describes that responsibility without creating a separate Capability.
- **Functions & Exposed Services**:
  - **Feature: Consent Directive Evaluation**:
    - *Function*: `EvaluateClientConsent` — Evaluates patient opt-in, opt-out, and general information sharing preferences against proposed exchanges.
    - *Exposed Service*: `ConsentDecisionService` — Provides real-time consent evaluation decisions to Health Information Control.
  - **Feature: Confidentiality Flag Enforcement**:
    - *Function*: `EvaluateClientConfidentialityRestrictions` — Evaluates sensitive record flags (e.g., VIP masking, restricted department exclusions).
    - *Exposed Service*: `ConfidentialityRestrictionEvaluation` — Enforces record-level masking directives across the HIE.
- **Information Responsibility**: Client Consent Directive, Record Masking Rule, Disclosure Restriction Register.

3. Provider Administration

Provider Administration governs individual healthcare practitioners, their professional qualifications, clinical roles, electronic communication endpoints, and organizational affiliations.

- **Owning Capability**: `ProviderAdministration`
- **Key Architectural Semantics**:
  - **Practitioner ≠ Service Provider**: A *Practitioner* is an individual professional. A *Service Provider* is an organisation or facility offering health services. A Practitioner may act in the *Service Provider* role in private practice.
  - **Role and privilege**: Practitioner Role Resolution supplies role/capacity information; it does not establish Clinical Privilege or Operational Privilege. Clinical Privilege Verification retains its clinical scope. An activity may require both distinct privilege determinations; neither follows from Role information alone.
- **Functions & Exposed Services**:
  - **Feature: National Practitioner Verification**:
    - *Function*: `VerifyPractitionerRegistration` — Validates national registration, specialty endorsements, and disciplinary sanctions against regulatory registries (e.g., AHPRA).
    - *Exposed Service*: `PractitionerVerificationService` — Discloses verified registration and status to credentialing and clinical systems.
    - *Governed Process*: **Practitioner Verification Process**.
  - **Feature: Practitioner Profile Governance**:
    - *Function*: `GovernPractitionerProfile` — Maintains official names, professional identifiers (e.g., HPI-I, Prescriber Number), and clinical qualifications.
    - *Exposed Service*: `PractitionerDirectoryQuery` — Provides provider directory lookups across the healthcare network.

- **Feature: Role & Specialty Modeling**:
  - *Function*: `MaintainPractitionerRoles` — Assigns and maintains clinical roles (e.g., Specialist Physician, Registrar, General Practitioner) fulfilled by a practitioner.
  - *Exposed Service*: `PractitionerRoleResolution` — Resolves the specific clinical capacities of a practitioner.
- **Feature: Electronic Endpoint Binding**:
  - *Function*: `BindPractitionerElectronicEndpoints` — Associates secure messaging addresses, direct communication endpoints, and notification preferences to practitioner roles.
  - *Exposed Service*: `PractitionerEndpointLookup` — Provides secure message routing destinations for clinical communication.
- **Feature: Practitioner-Organisation Affiliation**:
  - *Function*: `MaintainPractitionerAffiliations` — Tracks employment, admitting rights, and consulting appointments at specific healthcare facilities.
  - *Exposed Service*: `PractitionerAffiliationResolution` — Discloses organizational ties and practice locations.
- **Feature: Clinical Privileging Record**:
  - *Function*: `MaintainClinicalPrivileges` — Records approved clinical scopes of practice, procedural authorizations, and prescribing privileges.
  - *Exposed Service*: `ClinicalPrivilegeVerification` — Verifies procedural and prescribing authority for clinical ordering systems.
- **Information Responsibility**: Practitioner Master Record, Professional Registration Status, Practitioner Role Bindings, Electronic Communication Endpoints, Scope of Practice Privileges.

4. Organisation Administration

Organisation Administration maintains the authoritative registry of healthcare organisations, networks, directorates, and administrative subdivisions.

- **Owning Capability**: `OrganisationAdministration`
- **Functions & Exposed Services**:
  - **Feature: National Organisation Verification**:
    - *Function*: `VerifyHealthcareOrganisation` — Validates enterprise legal identity, national facility identifiers (e.g., HPI-O), and accreditation status.
    - *Exposed Service*: `OrganisationVerificationService` — Provides verified organisation credentials to external and internal partners.
  - **Feature: Organisation Profile Management**:
    - *Function*: `GovernOrganisationProfile` — Maintains official trading names, enterprise registration numbers, and governing bodies.
    - *Exposed Service*: `OrganisationDirectoryQuery` — Discloses healthcare organisation profiles across the region.
  - **Feature: Hierarchical Structure Navigation**:
    - *Function*: `MaintainOrganisationalStructure` — Models parent-subsidiary relationships, clinical directorates, departments, and operational divisions.
    - *Exposed Service*: `OrganisationalHierarchyResolution` — Resolves department nesting and reporting hierarchies.
  - **Feature: Directory Contact Management**:
    - *Function*: `MaintainOrganisationContacts` — Manages administrative, clinical, and emergency contact details for facilities.
    - *Exposed Service*: `OrganisationContactLookup` — Provides official operational contact details.
- **Information Responsibility**: Healthcare Organisation Registry, National Facility Identifier Bindings, Department Hierarchy Graph, Organisation Contact Directory.

5. Location Administration

Location Administration defines the physical, functional, and geospatial structure of healthcare environments, including sites, buildings, floors, wards, rooms, bays, and bed care-places.

- **Owning Capability**: `LocationAdministration`
- **Key Architectural Semantics**:
  - **Definition vs. Operational State**: *Location Administration* defines what a physical location or care-place **IS** (its static attributes, type, capacity, hierarchy). *Health Service Operations* owns what is **operationally happening** to that care-place (occupancy, cleaning status, isolation locks, turnover).
- **Functions & Exposed Services**:
  - **Feature: Location Geospatial & Address Resolution**:
    - *Function*: `MaintainHealthcareLocation` — Records physical sites, campuses, outpatient clinics, and community health centres.
    - *Exposed Service*: `LocationDirectoryQuery` — Exposes location profiles and geospatial addresses.
  - **Feature: Physical Hierarchy Modeling**:
    - *Function*: `MaintainPhysicalLocationHierarchy` — Models the nested structural containment tree: `Campus` → `Building` → `Floor` → `Ward/Suite` → `Room/Bay` → `BedCare-Place`.
    - *Exposed Service*: `LocationHierarchyResolution` — Discloses physical containment and navigation paths for clinical facilities.
  - **Feature: Atomic Care-Place Modeling**:
    - *Function*: `MaintainCare-PlaceDefinition` — Defines static bed bays, treatment chairs, surgical tables, and emergency resuscitation cubicles, including equipment capabilities and oxygen/telemetry infrastructure.
    - *Exposed Service*: `Care-PlaceSpecificationLookup` — Discloses care-place physical capabilities to Bed & Care-Place Management.
- **Information Responsibility**: Location Master Registry, Physical Facility Hierarchy Tree, Care-Place Structural Definitions.

6. Health Service Administration

Health Service Administration governs the catalogue of clinical services offered across the healthcare network and resolves the critical multi-dimensional mapping between Services, Providers, Locations, and Practitioners.

- **Owning Capability**: `HealthServiceAdministration`
- **Key Architectural Semantics**:
  - **Service / Location / Provider Map**: This capability models the multi-way relationship: Healthcare Service ↔ Service Provider ↔ Healthcare Location ↔ Practitioner Role
  - **Ownership Invariant**: The mapping capability owns the **relationships and association rules**. It does **not** acquire ownership of the Service, Provider, Location, or Practitioner entities being mapped.
  - **Consumer Resolution Scenarios**: Enables clinical and administrative consumers to unambiguously resolve:
    1. *Who provides Service X?*
    2. *Where does Provider Y provide Service X?*
    3. *What Services are delivered at Location Z?*
    4. *Where can Service X be obtained?*
    5. *Which Practitioner Roles participate in Service X?*
- **Functions & Exposed Services**:
  - **Feature: Service Catalogue Publishing**:
    - *Function*: `MaintainHealthcareServiceDefinition` — Defines clinical service offerings (e.g., Cardiology Outpatient Clinic, MRI Imaging, Acute Stroke Care), specialty classifications, and clinical scope.

    - *Exposed Service*: `HealthcareServiceDirectoryQuery` — Exposes published clinical service offerings.
  - **Feature: Service-Location-Provider Binding**:
    - *Function*: `MaintainService/Location/ProviderMap` — Binds health service definitions to delivering organisations, physical clinic locations, and participating practitioner roles.
    - *Exposed Service*: `ServiceProvisionResolution` — Exposes multi-dimensional resolution queries answering who, where, and when healthcare services are delivered.
  - **Feature: Operating Schedule Specification**:
    - *Function*: `MaintainServiceAvailability` — Governs regular operating hours, emergency intake availability, and seasonal clinic schedules.
    - *Exposed Service*: `ServiceAvailabilityQuery` — Discloses operational intake hours and open status.
  - **Feature: Eligibility & Catchment Specification**:
    - *Function*: `MaintainServiceEligibilityRules` — Configures catchment criteria, age thresholds, referral requirements, and clinical indications for service access.
    - *Exposed Service*: `ServiceEligibilityEvaluation` — Evaluates referral eligibility for intake coordinators.
- **Information Responsibility**: Healthcare Service Catalogue, Service / Location / Provider Mapping Matrix, Service Availability Schedules, Service Eligibility Rule Sets.

7. Clinical Device Administration

Clinical Device Administration governs the master definition, instance identification, and electronic communication endpoints of medical equipment and point-of-care appliances.

- **Owning Capability**: `ClinicalDeviceAdministration`
- **Functions & Exposed Services**:
  - **Feature: Device Type Specification**:
    - *Function*: `MaintainDeviceDefinition` — Models medical device models, manufacturers, regulatory classifications (e.g., TGA/FDA classes), and calibration specifications.
    - *Exposed Service*: `DeviceDefinitionQuery` — Discloses approved medical device models and technical parameters.
  - **Feature: Device Serial Tracking**:
    - *Function*: `MaintainDeviceInstanceIdentity` — Tracks physical serial numbers, Unique Device Identifiers (UDI), asset tags, and firmware versions.
    - *Exposed Service*: `DeviceInstanceResolution` — Resolves physical device instances from scanned barcodes or UDIs.
  - **Feature: Device-Patient/Location Assignment**:
    - *Function*: `MaintainDeviceAssociation` — Records active associations between clinical devices, patient encounters, and specific care-places (e.g., Bed 4 Vital Signs Monitor).
    - *Exposed Service*: `DeviceAssociationQuery` — Discloses active device-to-patient and device-to-location bindings.
  - **Feature: Device Endpoint Registration**:
    - *Function*: `MaintainDeviceCommunicationEndpoints` — Manages network telemetry addresses, gateway ports, and protocol bindings for medical devices.
    - *Exposed Service*: `DeviceEndpointResolution` — Exposes network addresses for telemetry routing.
- **Information Responsibility**: Medical Device Type Registry, Physical Device Instance Directory, Device Association Ledger, Device Communication Endpoint Map.

18. Service Administration — Capability-Scoped Behaviour

RAD source: `docs/markdown/03-business-architecture/behaviours/02-service-administration.md`

The following is a presentation transcription of the governed source. Source catalogue layout and retained illustrations do not allocate tiers or establish additional identities. Equations wrap as text; diagram notation is textual.

Service Administration — Capability-Scoped Behaviour

1. Contextual Scope & Architectural Intent

The named owning Capabilities and established Features retain their responsibilities. Affected Capability Tier, complete ancestry, root status and structural Canonical IDs remain unresolved under approved G1 K9; document grouping and numbering do not establish architectural identity or hierarchy.

Service Administration governs the clinical administrative lifecycles, coordination workflows, and documentation exchanges that surround patient care episodes. It coordinates referrals, appointments, encounters, orders, diagnostic reports, medication events, care plans, and clinical documents across care boundaries.

2. Client-Centred Administration

- **Owning Capability**: `Client-CentredAdministration`
- **Key Architectural Semantics**:
  - **Administrative Coordination**: Handles operational demographic intake and administrative updates across departmental boundaries.
  - **Ownership Invariant**: Does **not** own authoritative person identity or identifier correlation (which is owned by *Client Administration / Person Identity*).
- **Functions & Exposed Services**:
  - **Feature: Demographic Update Ingestion**:
    - *Function*: `ReceiveClientDemographicChange` — Ingests administrative demographic updates from PAS/registration feeds.
    - *Exposed Service*: `DemographicIntakeService` — Exposed to departmental registration systems.
  - **Feature: Demographic Change Distribution**:
    - *Function*: `DistributeGovernedDemographicChange` — Propagates verified demographic modifications to subscribed clinical subsystems.
    - *Exposed Service*: `DemographicUpdateNotification` — Emits change notifications across the HIE.
- **Information Responsibility**: Administrative Intake Ledger, Subscribed Distribution Registry.

3. Referral Administration

- **Owning Capability**: `ReferralAdministration`
- **Key Architectural Semantics**:

  - **Referral vs. Order**: A **Referral** formally requests another care participant or service provider to assess, manage or assume some degree of clinical care responsibility. Intent, disposition, responsibility actually assumed and Transfer of Care are distinct; Referral itself establishes none of acceptance, delivery, assumption or Transfer of Care, and has no universal Appointment, Encounter or Order consequence. An **Order** concerns *bounded fulfilment of a specific diagnostic or therapeutic task*.
- **Functions & Exposed Services**:
  - **Feature: Referral Document Ingestion**:
    - *Function*: `ReceiveReferral` — Validates and ingests structured electronic referrals, clinical indications, and priority categories.
    - *Exposed Service*: `ReferralSubmissionService` — Exposed to external GP practices and referring health facilities.
  - **Feature: Referral Supporting Information**:
    - *Function*: `AssembleReferralContext` — Gathers relevant clinical history, medications, and diagnostic investigations into the referral package.
    - *Exposed Service*: `ReferralContextQuery` — Provides comprehensive referral dossiers to intake triage clinicians.
  - **Feature: Referral Status & Outcome Tracking**:
    - *Function*: `ManageReferralProgression` — Tracks referral disposition in an **illustrative specialist pathway; dispositions vary**: *Submitted* → *Triaged* → *Accepted/Waitlisted* → *Scheduled* → *Discharged*.
    - *Exposed Service*: `ReferralStatus&OutcomeService` — Exposes progression milestones to referrers and patients.
    - *Governed Process*: **Referral Progression Process**.
    - *Scope qualification*: The displayed specialist pathway is illustrative, not mandatory for every Referral; acceptance, waitlisting, decline and redirection retain their contextual meanings.
- **Information Responsibility**: Referral Master Record, Triage Decision Ledger, Referral Disposition Log.

4. Scheduling Administration

- **Owning Capability**: `SchedulingAdministration`
- **Key Architectural Semantics**:
  - **Coordination vs. Authoritative Booking**: Harmonia synchronises and communicates appointment milestones; it does **not** replace the authoritative booking/scheduling engines of host PAS or departmental scheduling platforms.
- **Functions & Exposed Services**:
  - **Feature: Appointment Notification Ingestion**:
    - *Function*: `ReceiveAppointmentNotification` — Ingests appointment creation, rescheduling, cancellation, and check-in events from host PAS engines.
    - *Exposed Service*: `AppointmentNotificationIngress` — Ingests booking events from departmental schedulers.
  - **Feature: Appointment Status Synchronization**:
    - *Function*: `SynchroniseAppointmentStatus` — Projects synchronised appointment timelines to clinical dashboards and care coordinators.
    - *Exposed Service*: `AppointmentScheduleQuery` — Exposes consolidated patient appointment schedules across facilities.
- **Information Responsibility**: Appointment Event History, Schedule Synchronisation State.

5. Episode & Encounter Administration

- **Owning Capability**: `Episode&EncounterAdministration`

- **Key Architectural Semantics**:
  - **Encounter Context**: Tracks the clinical and administrative interaction between a patient and healthcare provider during an emergency, inpatient, or outpatient episode.
- **Functions & Exposed Services**:
  - **Feature: Encounter Context Creation**:
    - *Function*: `EstablishEncounterContext` — Binds patient, attending practitioner, clinical class (e.g., Inpatient, Emergency, Ambulatory), and admitting diagnosis to a unique encounter identifier.
    - *Exposed Service*: `EncounterContextResolution` — Discloses active encounter details to clinical applications.
  - **Feature: Encounter State Progression**:
    - *Function*: `ProgressEncounterState` — Coordinates encounter progression (**scoped illustration; ON_LEAVE relationship unresolved**): *Planned → Arrived → In-Progress → Discharged → Completed*.
    - *Exposed Service*: `EncounterLifecycleEventNotification` — Emits encounter transition events across the HIE.
    - *Governed Process*: **Encounter Lifecycle Process**.
    - *Scope qualification*: Process completion qualifications remain local; their correspondence with Strategy `ON_LEAVE` is unresolved. A shorter summary neither removes established states nor creates universal signing or legal-closure requirements.
  - **Feature: Encounter Bed/Location Association**:
    - *Function*: `MaintainEncounterCare-PlaceAssociation` — Binds the encounter to physical wards, rooms, and beds over time.
    - *Exposed Service*: `EncounterLocationHistoryQuery` — Discloses patient location tracking history.
- **Information Responsibility**: Encounter Master Record, Encounter State Transition Log, Encounter Care-Place Movement Ledger.

6. Order Administration

- **Owning Capability**: `OrderAdministration`
- **Key Architectural Semantics**:
  - **Closed-Loop Progression**: Tracks clinical orders through closed-loop progression. Diagnostic, pathology, imaging and procedural examples illustrate scoped behaviour and do not exclude medication orders from Order Administration responsibility.
  - **Order ≠ Referral**: Closed-loop execution of requested investigations without transferring longitudinal care responsibility.
- **Functions & Exposed Services**:
  - **Feature: Order Request Ingestion**:
    - *Function*: `ReceiveOrderRequest` — Ingests structured orders containing clinical indication, requesting clinician, and test specifications.
    - *Exposed Service*: `OrderRequisitionIngress` — Ingests electronic orders from clinical ordering systems.
  - **Feature: Order Destination Resolution & Routing**:
    - *Function*: `ResolveOrderDestination` — Evaluates order routing rules to determine target laboratory, imaging centre, or procedural unit.
    - *Exposed Service*: `OrderDispatchService` — Transmits orders to performing diagnostic systems.
  - **Feature: Order Closed-Loop Progression Tracking**:
    - *Function*: `ManageOrderProgression` — Tracks order state (**scoped diagnostic/procedural illustration**): *Placed → Received → Specimen Collected → In-Progress → Preliminary Result → Final Result → Closed*.
    - *Exposed Service*: `OrderStatus&TrackingQuery` — Provides real-time order tracking to ordering clinicians.

    - *Governed Process*: **Closed-Loop Order Progression Process**.
    - *Scope qualification*: Specimen, investigation and result checkpoints describe the illustrated diagnostic/procedural progression. Dispatch, receipt and acceptance remain distinct; Behaviour `Received` is not equated to a Process checkpoint without evidence. Cancellation/modification responsibility remains valid even where absent from that sequence.
  - **Feature: Order Cancellation & Modification Coordination**:
    - *Function*: `CoordinateOrderModification/Cancellation` — Handles requests to amend clinical details or cancel unexecuted orders.
    - *Exposed Service*: `OrderCancellationService` — Dispatches cancel directives to performing systems.
  - **Capability-scoped behaviour: Order Result Association** (Feature identity not established):
    - *Function*: `AssociateOrderOutcome` — Correlates returning diagnostic reports and observations back to the originating order requisition.
    - *Exposed Service*: `OrderOutcomeNotification` — Emits completion notices to ordering practitioners.
- **Information Responsibility**: Clinical Order Master Record, Closed-Loop Tracking Ledger, Order-Result Correlation Matrix.

7. Diagnostic Administration

- **Owning Capability**: `DiagnosticAdministration`
- **Key Architectural Semantics**:
  - **Result Management & Distribution**: Manages the correlation, notification, and delivery of pathology, radiology, and point-of-care results.
  - **Ownership Guardrail**: Does **not** own the clinical diagnostic interpretation or specimen analysis (which is owned by the performing diagnostic service provider/LIS).
- **Functions & Exposed Services**:
  - **Feature: Diagnostic Request Correlation**:
    - *Function*: `CorrelateDiagnosticRequest` — Matches incoming lab/radiology results to active patient encounters and order requisitions.
    - *Exposed Service*: `DiagnosticCorrelationResolution` — Discloses correlated order/result linkages.
  - **Feature: Diagnostic Report Ingestion & Binding**:
    - *Function*: `BindDiagnosticReport` — Associates structured observations, reference ranges, and abnormal flags with patient health records.
    - *Exposed Service*: `DiagnosticReportIngress` — Ingests results from external LIS/RIS systems.
  - **Feature: Diagnostic Report Distribution**:
    - *Function*: `DistributeDiagnosticReport` — Delivers preliminary, final, and corrected reports to ordering and copied practitioners.
    - *Feature scope qualification*: Strategy `FEAT-SA-17–DiagnosticReportDistribution` retains its finalised-report responsibility. Preliminary, corrected and addendum/correction handling belongs to the broader Diagnostic Administration responsibility; it does not redefine that narrower Feature or confer diagnostic interpretation authority.
    - *Exposed Service*: `DiagnosticResultDeliveryService` — Emits secure result notifications.
- **Information Responsibility**: Diagnostic Request-Report Linkage Registry, Result Distribution Ledger.

8. Medication Administration

- **Owning Capability**: `MedicationAdministration`

- **Key Architectural Semantics**:
  - **Event Tracking**: Aggregates medication orders, pharmacy dispense records, and administration-event information into a coherent timeline; bedside events are one supported context.
  - **Non-Practice Guardrail**: Harmonia tracks and communicates medication events; it does **not** prescribe drugs, dispense pharmaceuticals, or perform clinical administration.
- **Functions & Exposed Services**:
  - **Feature: Medication Order Ingestion**:
    - *Function*: `ReceiveMedicationOrder` — Ingests inpatient and outpatient drug prescriptions.
    - *Exposed Service*: `MedicationOrderIngress` — Ingests prescription events.
  - **Feature: Dispense Event Tracking**:
    - *Function*: `TrackDispenseEvent` — Ingests community and hospital pharmacy dispensing records.
    - *Exposed Service*: `DispenseEventIngress` — Ingests pharmacy supply notifications.
  - **Feature: Medication Administration Record (MAR) Ingestion**:
    - *Function*: `ReceiveMedicationAdministrationEvent` — Records medication administration-event information, including nurse-administered doses, e-MAR documentation and infusion telemetry as non-exhaustive examples; scope is not limited to nursing, bedside or inpatient settings.
    - *Exposed Service*: `MedicationAdministrationIngress` — Ingests administration records.
- **Information Responsibility**: Medication Event Timeline, Prescription-Dispense-Administration Correlation Matrix.

9. Procedure Administration

- **Owning Capability**: `ProcedureAdministration`
- **Scope qualification**: Procedure information extends beyond surgery; Theatre retains its distinct surgical operational responsibility.
- **Functions & Exposed Services**:
  - **Feature: Procedure Booking Ingestion**:
    - *Function*: `ReceiveProcedureBooking/Request` — Ingests procedural and surgical requisitions.
    - *Exposed Service*: `ProcedureBookingIngress` — Receives surgical/procedural requests.
  - **Feature: Procedural Documentation Ingestion**:
    - *Function*: `ReceiveProceduralDocumentation` — Ingests operation reports, anaesthetic records, and post-procedure notes.
    - *Exposed Service*: `ProceduralDocumentationIngress` — Ingests procedural documentation, including surgical notes, into patient record streams.
- **Information Responsibility**: Procedure Request Register, Procedural Documentation Index.

10. Clinical Record Administration

- **Owning Capability**: `ClinicalRecordAdministration`
- **Key Architectural Semantics**:
  - **Document Governance**: Governs the ingestion, metadata indexing, versioning, addenda, and legal lifecycle of clinical documents (discharge summaries, specialist letters, advance care directives).
- **Functions & Exposed Services**:
  - **Feature: Clinical Document Ingestion**:
    - *Function*: `ReceiveClinicalDocument` — Receives clinical documents from information suppliers.

    - *Exposed Service*: `ClinicalDocumentIngress` — Exposes document submission endpoints across the HIE.
  - **Feature: Document Versioning & Supersession**:
    - *Function*: `GovernClinicalDocumentLifecycle` — Manages document states (**scoped illustration, not a universal transition model**): *Draft* → *Preliminary* → *Final* → *Amended* → *Superseded* → *Entered-in-Error*.
    - *Exposed Service*: `ClinicalDocumentLifecycleService` — Allows authoring clinicians to publish addenda or corrections.
    - *Governed Process*: **Clinical Document Lifecycle Process**.
    - *Scope qualification*: `Final≠automaticallyFinalSigned`. Authorship, attestation, approval, authentication, verification, signature, finalisation, authority and legal qualification remain distinct. Process signing/countersigning requirements retain their local scope, and amendment/supersession/error states are not mandatory for every document. Ingestion and consumer processing do not define originating validity, authority, authorship or legal status.
  - **Feature: Document Metadata Indexing**:
    - *Function*: `IndexClinicalDocumentMetadata` — Indexes author, specialty, encounter, date, and document type (LOINC/SNOMED).
    - *Exposed Service*: `DocumentMetadataRegistryQuery` — Discloses document registry metadata to clinical viewers.
- **Information Responsibility**: Clinical Document Registry, Document Version History, Document Lifecycle State Register.

11. Care Plan Administration

- **Owning Capability**: `CarePlanAdministration`
- **Functions & Exposed Services**:
  - **Feature: Care Plan Ingestion & Distribution**:
    - *Function*: `ReceiveCarePlan` — Ingests multidisciplinary care plans, clinical goals, and patient action steps.
    - *Exposed Service*: `CarePlanIngress` — Ingests shared care plans.
  - **Capability-scoped behaviour: Care Plan Context Association** (Feature identity not established):
    - *Function*: `AssociateCarePlanContext` — Links care plans to active diagnoses, health problems, and care team members.
    - *Exposed Service*: `CarePlanContextQuery` — Discloses active care plans to treating clinicians.
  - **Capability-scoped behaviour: Care Plan Update Distribution** (Feature identity not established):
    - *Function*: `DistributeCarePlanChange` — Broadcasts care plan updates to all enrolled care team participants.
    - *Exposed Service*: `CarePlanChangeNotification` — Emits care plan revision notices.
- **Information Responsibility**: Shared Care Plan Registry, Clinical Goal Ledger, Care Plan Revision History.

12. Clinical Communication Administration

- **Owning Capability**: `ClinicalCommunicationAdministration`
- **Key Architectural Semantics**:
  - **Technical vs. Business Acknowledgement**:
    - **Technical ACK**: May establish durable receiving-boundary acceptance into an integration queue; it does not establish business acceptance.

    - **Business Delivery ACK**: Records contextual recipient-reported facts or dispositions, which may include receipt, parsing, acceptance, rejection, filing or another explicit disposition. These are not a universal progression; business-acceptance and filing criteria remain unresolved.
- **Functions & Exposed Services**:
  - **Feature: Clinical Communication Distribution**:
    - *Function*: `DistributeClinicalCommunication` — Routes secure messages, referral letters, and critical alerts to practitioner endpoints.
    - *Exposed Service*: `SecureClinicalMessageDispatch` — Exposes message dispatch to clinical authors.
  - **Feature: Delivery Acknowledgement Tracking**:
    - *Function*: `TrackBusinessDeliveryAcknowledgement` — Tracks end-to-end delivery state and records business-level delivery/rejection acknowledgements.
    - *Receipt qualification*: Delivery/read evidence does not establish clinical review, agreement, approval or incorporation. Clinical Review itself does not automatically establish incorporation; no universal Clinical Incorporation concept is established.
    - *Exposed Service*: `CommunicationDeliveryStatusQuery` — Discloses delivery confirmation to message senders.
  - **Feature: Communication Audit Logging**:
    - *Function*: `RecordCommunicationTransaction` — Maintains an immutable audit trail of clinical message dispatch and receipt.
    - *Exposed Service*: `CommunicationAuditQuery` — Discloses transmission history for medico-legal verification.
- **Information Responsibility**: Clinical Message Dispatch Log, Business Delivery Acknowledgement Ledger, Medico-Legal Transmission Audit Register.
- **Authority boundary**: Acknowledgement, receipt, persistence, filing, indexing, assembly and review do not transfer originating authority or establish finality. Communication audit evidence concerns transmission; it does not establish the clinical content's authority or legal qualification.

19. Service Delivery — Capability-Scoped Behaviour

RAD source: `docs/markdown/03-business-architecture/behaviours/03-service-delivery.md`

The following is a presentation transcription of the governed source. Source catalogue layout and retained illustrations do not allocate tiers or establish additional identities. Equations wrap as text; diagram notation is textual.

Service Delivery — Capability-Scoped Behaviour

1. Contextual Scope & Architectural Intent

The named owning Capabilities and established Features retain their responsibilities. Affected Capability Tier, complete ancestry, root status and structural Canonical IDs remain unresolved under approved G1 K9; document grouping and numbering do not establish architectural identity or hierarchy.

Service Delivery represents the clinical operating contexts in which direct patient care is delivered across primary, acute, emergency, inpatient, diagnostic, and virtual settings.

The Fundamental Care Enablement Principle

> **Harmonia enables, informs, and coordinates healthcare delivery across care boundaries; it does not practice medicine, prescribe pharmaceuticals, make autonomous clinical decisions, or replace clinician judgment.**

Service Delivery capabilities within Harmonia define **integration enablement and clinical context correlation behaviour**. They reuse and consume core capabilities (*Patient Clinical Record, Health Information Exchange, Health Information Control, Episode & Encounter Administration, Clinical Record Administration*) rather than duplicating their responsibilities.

Residual Feature Sufficiency

The 2026-10-09 human adjudications apply the Business composition and clinical-work boundaries. General clinical-information capture, processing, preservation and distribution are expressed by Patient Clinical Record and Health Information Exchange, with applicable Service Administration responsibility. A clinical setting does not establish specialised capture or clinical-work ownership.

Strategy Feature	Adjudicated Business expression and boundary
`FEAT-SD-03–AcuteClinicalStateEventCapture`	Acute deterioration/status information is managed through general clinical-information responsibilities. No Acute-specific capture Function is required. The Strategy Feature's acute-specific framing is over-specialised if interpreted as a separate Harmonia responsibility; its existing name/definition are retained for bounded Strategy review, not silently redefined. Alert routing below remains distinct from capture.

Strategy Feature	Adjudicated Business expression and boundary
`FEAT-SD-08–InpatientClinicalProgressionTracking`	Relevant milestone information uses the same general responsibilities. Operational consequences may inform patient flow, capacity or discharge coordination; those consumers do not establish the originating clinical fact. An allied-health clearance may affect discharge readiness without becoming an operationally authored clearance. No Inpatient-specific clinical-tracking Function or EMR workflow is required. Dossier assembly below remains distinct.
`FEAT-SD-14–ScreeningRecallNotificationDistribution`	Recall is a domain-specific composition of clinical-information management, Calendar Management, Workflow & Activity Coordination and information communication/distribution, with applicable entity coordination. Configured workflow/Praxis may realise it without a screening-specific recall Function. Eligibility projection alone remains distinct from distribution. Exact workflow and provider contracts are downstream, not established by this sufficiency decision.

These are composition-level sufficiency decisions, not replacement Feature associations to the Functions below. The same reusable responsibilities can support follow-up, vaccination, diagnostic or review reminders without a new Business Function for each clinical purpose. Population/cohort identification may precede individual activity. Once activity concerns a specific managed patient, the entity-specific Digital Twin coordination rule applies; it establishes no Domain03 Twin Actor, Process or execution mechanism.

2. Clinical Care Enablement Contexts

RAD source listing (presentation only; no inferred ancestry).

Service Delivery Contexts

Primary Care

Acute Care

Emergency Care

Inpatient Care

Diagnostic Services

Medication Therapy

Preventive Care

Community Care

Outreach Care

Remote & Virtual Care

2.1 Primary Care

- **Owning Capability**: `PrimaryCare`
- **Clinical Operating Context**: General practice clinics, community medical centres, and family medicine practices providing longitudinal whole-person care.
- **Enabling Functions & Exposed Services**:
  - **Feature: General Practice Shared Record Ingestion**:
    - *Function*: `CorrelatePrimaryCareConsultationContext` — Binds GP consultation notes, vital signs, and encounter episodes to the regional Longitudinal Health Record (LHR).
    - *Exposed Service*: `PrimaryCareSummaryIngress` — Ingests GP event summaries into the HIE.
  - **Capability-scoped Functions / Services**:

  - *Feature association not established*: Strategy `FEAT-SD-02–PrimaryCareNotificationDispatch` remains valid, but semantic evidence does not establish that the following Function or Service belongs to, realises, specialises or is equivalent to it. No replacement Feature is assigned.
  - *Function*: `CoordinateMulti-AgencyChronicCarePlan` — Enables GPs to track multi-provider disease management milestones (e.g., Diabetes or Cardiovascular Care Plans).
  - *Exposed Service*: `ChronicCareTimelineQuery` — Exposes aggregated multidisciplinary care timelines.
- **Typed architectural dependencies**:
  - **Service consumption**: `PersonIdentifierResolution`; `LongitudinalClinicalRecordQuery` from Patient Clinical Record, already established in the consumption matrix.
  - **Capability dependency**: `CarePlanAdministration`.
  - **Unresolved reference**: "Referral Submission" — an exact Service identity/consuming relationship is not established here; the abbreviated wording is not an alias.

2.2 Acute Care

- **Owning Capability**: `AcuteCare`
- **Clinical Operating Context**: Hospital specialty wards, surgical suites, and intensive care units managing severe, urgent, or post-operative conditions.
- **Enabling Functions & Exposed Services**:
  - **Feature: Acute Care Clinical Context Provision**:
    - *Function*: `AssembleAcuteClinicalBaseline` — Rapidly aggregates previous discharge summaries, active medications, allergies, and diagnostic alerts upon acute hospital presentation.
    - *Exposed Service*: `AcuteAdmissionDossierQuery` — Delivers a high-priority clinical briefing to admitting teams.
  - **Capability-scoped Functions / Services**:
    - *Feature association not established*: `FEAT-SD-03–AcuteClinicalStateEventCapture` is adjudicated sufficient through general clinical-information responsibilities above. Routing a critical-result alert remains distinct from capture; no specialised capture Function or replacement association is assigned.
    - *Function*: `RouteAcuteCriticalResultAlert` — Immediately directs urgent lab/radiology alerts (e.g., critical troponin, intracranial bleed) to active acute attending clinicians.
    - *Exposed Service*: `AcuteClinicalAlertDispatch` — Dispatches urgent interruptive alerts to on-duty ward teams.
- **Typed architectural dependencies**:
  - **Service consumption**: `EncounterContextResolution`.
  - **Capability dependency**: `HealthInformationControl`; break-glass access describes the required governed context, not an independently allocated Service.
  - **Unresolved reference**: "Diagnostic Result Delivery" — intended exact Service/relationship remains unestablished; no alias or replacement edge is inferred.

2.3 Emergency Care

- **Owning Capability**: `EmergencyCare`
- **Clinical Operating Context**: Emergency Departments (ED), trauma centres, and urgent care clinics operating under time-critical, high-acuity constraints.
- **Enabling Functions & Exposed Services**:
  - **Feature: Emergency Department Arrival & Triage Ingestion**:
    - *Function*: `BindEDTriageAssessment` — Captures ATS (Australasian Triage Scale) category, presenting complaint, and ambulance paramedic handover data.
    - *Exposed Service*: `EDTriageStreamIngress` — Ingests emergency triage presentations.

  - **Feature: Emergency Care Cross-Facility Correlation**:
    - *Function*: `ProvideEmergencyLHRSummary` — Delivers rapid, override-capable access to Advance Care Directives, allergy records, and recent hospital admissions.
    - *Exposed Service*: `EmergencyFast-PathLHRQuery` — Provides low-latency emergency record summaries.
- **Typed architectural dependencies**:
  - **Service consumption**: `PersonIdentifierResolution`. "Fast-Match" does not establish a separate Service or probabilistic/master-person matching behaviour.
  - **Capability dependency**: `ClientPrivacy` for its established consent/privacy responsibility; "Break-Glass Override" does not allocate override authority or another element.
  - **Unresolved relationship to an established Feature**: `FEAT-SA-09—EncounterStateProgression`. A precise Service-consumption relationship is not established by naming that Feature.

2.4 Inpatient Care

- **Owning Capability**: `InpatientCare`
- **Clinical Operating Context**: Inpatient hospital wards, rehabilitation units, and sub-acute facilities managing multi-day hospital stays.
- **Enabling Functions & Exposed Services**:
  - **Feature: Inpatient Rounding Context Provision**:
    - *Function*: `ConsolidateInpatientClinicalRoundFeed` — Aggregates daily vital signs, overnight pathology, allied health notes, and medication administration records.
    - *Exposed Service*: `WardRoundClinicalBriefingQuery` — Discloses consolidated 24-hour clinical briefing views.
  - **Capability-scoped Functions / Services**:
    - *Feature association not established*: `FEAT-SD-08—InpatientClinicalProgressionTracking` is adjudicated sufficient through general clinical-information responsibilities above. Discharge-dossier assembly/publication remains distinct; no separate clinical-tracking Function or replacement association is assigned.
    - *Function*: `AssembleInpatientDischargeDossier` — Synthesises inpatient diagnoses, procedural summaries, reconciled discharge medications, and follow-up instructions into a canonical discharge summary package.
    - *Exposed Service*: `DischargeDossierPackagingService` — Emits canonical discharge summaries to GPs and community providers.
- **Typed architectural dependencies**:
  - **Capability dependency**: `DischargeManagement`.
  - **Unresolved references / relationships**: "Encounter Care-Place Movement," "Clinical Document Lifecycle," "Medication Administration Event Ingestion." Their intended element types and exact consumption/participation relationships are not established by these descriptions; no Function, Process or Service equivalence is inferred.

2.5 Diagnostic Services

- **Owning Capability**: `DiagnosticServices`
- **Clinical Operating Context**: Clinical pathology laboratories, diagnostic imaging networks (radiology/MRI/CT), nuclear medicine, and clinical neurophysiology departments.
- **Enabling Functions & Exposed Services**:
  - **Capability-scoped Functions / Services**:
    - *Feature association not established*: Strategy `FEAT-SD-09—Laboratory&ImagingResultDistribution` remains valid, but semantic evidence does not establish that the following Function or Service belongs to, realises, specialises or is equivalent to it. No replacement Feature is assigned.

    - *Function*: `ReceiveDiagnosticRequisitionFeed` — Normalises order requisitions from across the health network into modality worklists.
    - *Exposed Service*: `ModalityWorklistDistribution` — Supplies order worklists to diagnostic laboratory/imaging systems.
  - **Capability-scoped Functions / Services**:
    - *Feature association not established*: Strategy `FEAT-SD-10–DiagnosticHistoryConsolidation` remains valid, but semantic evidence does not establish that the following Function or Service belongs to, realises, specialises or is equivalent to it. No replacement Feature is assigned.
    - *Function*: `PublishGovernedDiagnosticReport` — Normalises and distributes structured diagnostic observations, reference ranges, and diagnostic image study links into the LHR.
    - *Exposed Service*: `DiagnosticPublicationService` — Publishes verified diagnostic reports across the enterprise.
- **Capability dependencies**: `OrderAdministration` for closed-loop order context; `InformationDesignGovernance` for governed terminology bindings. These contextual purposes establish neither an additional Feature nor a precise Service-consumption edge.

2.6 Medication Therapy

- **Owning Capability**: `MedicationTherapy`
- **Clinical Operating Context**: Hospital pharmacies, clinical pharmacotherapy review panels, community dispensing networks, and outpatient infusion suites.
- **Enabling Functions & Exposed Services**:
  - **Feature: Medication History Reconciliation Ingestion**:
    - *Function*: `ProjectLongitudinalMedicationHistory` — Aggregates community dispensing, hospital e-MAR administrations, and specialist prescriptions into a unified chronological medication profile.
    - *Exposed Service*: `LongitudinalMedicationHistoryQuery` — Discloses full medication timelines for medication reconciliation.
  - **Capability-scoped Functions / Services**:
    - *Feature association not established*: Strategy `FEAT-SD-12–MedicationAdministrationTracking` remains valid, but semantic evidence does not establish that the following Function or Service belongs to, realises, specialises or is equivalent to it. No replacement Feature is assigned.
    - *Function*: `CorrelateAdverseReaction&AllergyProfile` — Binds confirmed substance allergies, adverse drug reactions, and severity grades to the patient identity.
    - *Exposed Service*: `Allergy&AdverseReactionQuery` — Exposes active allergy profiles to clinical decision support systems.
- **Capability dependencies**: `MedicationAdministration`, `PatientClinicalRecord`, `HealthInformationControl`; no specific Service is allocated by this contextual list.

2.7 Preventive Care

- **Owning Capability**: `PreventiveCare`
- **Clinical Operating Context**: Public health screening programmes (breast/bowel/cervical), national immunisation registries, and health promotion initiatives.
- **Enabling Functions & Exposed Services**:
  - **Feature: Immunisation Event Submission**:
    - *Function*: `SyndicateImmunisationRecord` — Records vaccine encounters and syndicates records to national immunisation registers (e.g., AIR).
    - *Exposed Service*: `ImmunisationHistoryQuery` — Discloses complete vaccination history.
  - **Capability-scoped Functions / Services**:

  - *Feature association not established*: `FEAT-SD-14–ScreeningRecallNotificationDistribution` is adjudicated sufficient through reusable composition/workflow above. Projecting/querying eligibility dates remains distinct from distribution; no dedicated recall Function or replacement association is assigned.
  - *Function*: `ProjectScreeningEligibilityTimeline` — Tracks age-based and risk-based screening schedules across primary care.
  - *Exposed Service*: `PreventiveScreeningStatusQuery` — Discloses screening due dates to primary care providers.
- **Typed architectural dependencies**:
  - **Capability dependency**: `HealthInformationExchange` for syndication.
  - **Unresolved reference**: "Healthcare Subject Demographic Context" — exact element/relationship remains unestablished; descriptive context does not create a Capability alias or Service.

2.8 Community Care

- **Owning Capability**: `CommunityCare`
- **Clinical Operating Context**: District nursing, home-based palliative care, community mental health teams, and allied health community outreach.
- **Enabling Functions & Exposed Services**:
  - **Feature: Community Encounter Ingestion**:
    - *Function*: `SynchronizeCommunityVisitRecord` — Ingests clinical notes, wound assessments, and vital signs recorded in community and home settings.
    - *Exposed Service*: `CommunityCareEncounterIngress` — Ingests community care documentation.
  - **Capability-scoped Functions / Services**:
    - *Feature association not established*: Strategy `FEAT-SD-16–CommunityCarePlanSynchronization` remains valid, but semantic evidence does not establish that the following Function or Service belongs to, realises, specialises or is equivalent to it. No replacement Feature is assigned.
    - *Function*: `BindCommunitySupportNetworkContext` — Links community nursing teams with local non-clinical social care and home care packages.
    - *Exposed Service*: `CommunityCareTeamQuery` — Exposes community provider contact rosters.
- **Typed architectural dependencies**:
  - **Capability dependencies**: `ClientRelationship`, `Episode&EncounterAdministration`.
  - **Collaboration reference with unresolved involvement**: `Care-TeamCollaboration` is an established Collaboration; precise participation/involvement is not allocated by the former "Consumes" wording.

2.9 Outreach Care

- **Owning Capability**: `OutreachCare`
- **Clinical Operating Context**: Mobile specialist clinics, rural and remote fly-in fly-out (FIFO) clinical teams, and school health screening programmes.
- **Enabling Functions & Exposed Services**:
  - **Feature: Outreach Visit Context Provision**:
    - *Function*: `ProvideOutreachVisitContext` — Packages and delivers portable clinical summaries and offline-capable care summaries for remote outreach clinics.
    - *Derivation*: `FEAT-SD-17` in Strategy's Outreach Care Continuity establishes this preparation/provision responsibility. Outcome reconciliation below does not supply pre-visit context. This Function remains owned by Outreach Care, uses governed source information without acquiring its originating authority and establishes no copying, storage or offline implementation mechanism.

    - *Exposure boundary*: Remote outreach clinics are the established recipients. No separate named Service, Process, Role allocation or structural ID is introduced merely to mirror the Function.
  - **Feature: Outreach Encounter Outcome Ingestion**:
    - *Function*: `ReconcileOutreachClinicalBatch` — Reconciles and merges clinical documentation recorded during disconnected remote clinic operations.
    - *Exposed Service*: `OutreachBatchReconciliationService` — Reconciles batch clinical encounters upon network reconnection.
- **Typed architectural dependencies**:
  - **Capability dependencies**: `HealthInformationExchange`, `ClinicalRecordAdministration`.
  - **Established Service with unresolved consumption applicability**: `PersonIdentityCorrection` retains Person Identity ownership; how Outreach Care requires that Service is not established here. It cannot authorise originating person-merge decisions.

2.10 Remote & Virtual Care

- **Owning Capability**: `Remote&VirtualCare`
- **Clinical Operating Context**: Telehealth consultations, virtual hospital-in-the-home (vHIT), remote patient monitoring (RPM), and asynchronous specialist e-consults.
- **Enabling Functions & Exposed Services**:
  - **Feature: Virtual Care Session Context Binding**:
    - *Function*: `CorrelateVirtualCareConsultationContext` — Binds video consultation metadata, remote practitioner identity, and shared digital whiteboard artifacts to the encounter.
    - *Exposed Service*: `VirtualEncounterContextResolution` — Binds virtual session identifiers to patient encounters.
  - **Feature: Remote Telemetry Ingestion**:
    - *Function*: `IngestRemotePatientTelemetry` — Ingests continuous or periodic physiological measurements (e.g., blood pressure, pulse oximetry, continuous glucose) from home monitoring devices.
    - *Exposed Service*: `RemoteTelemetryIngress` — Ingests remote patient data streams into the LHR.
- **Capability dependencies**: `ClinicalDeviceAdministration`, `PatientClinicalRecord`, `ClinicalCommunicationAdministration`; no specific Service is allocated by this contextual list.

20. Health Service Operations — Capability-Scoped Behaviour

RAD source: `docs/markdown/03-business-architecture/behaviours/04-health-service-operations.md`

The following is a presentation transcription of the governed source. Source catalogue layout and retained illustrations do not allocate tiers or establish additional identities. Equations wrap as text; diagram notation is textual.

Health Service Operations — Capability-Scoped Behaviour

1. Contextual Scope & Architectural Intent

The named owning Capabilities and established Features retain their responsibilities. Affected Capability Tier, complete ancestry, root status and structural Canonical IDs remain unresolved under approved G1 K9; document grouping and numbering do not establish architectural identity or hierarchy.

Health Service Operations governs the operational logistics, physical resource management, facility capacity, staff dispatch, and operational movement required to support healthcare delivery across clinical facilities.

Operational Logistics vs. Software Plumbing

> **In Domain 03, "Operations" refers strictly to healthcare facility logistics (e.g., bed turnover, patient portering, theatre case scheduling, clinic queues, dispatch). It does not mean IT middleware monitoring, application deployment, or database administration.**

Where Healthcare Service context materially affects operational meaning, authority, coordination, progression or accountability, keep it identifiable with the activity and relevant information responsibility. Location, Organisation, Practitioner or Role does not imply that service context. This contextual obligation establishes no information representation, identifier or implementation binding.

The bounded coverage additions below derive from the established Strategy HSO Features. They reuse existing compound behaviour where its meaning supports the relationship, or add the minimum directly required behaviour. Additional Functions do not mechanically create Services, Processes, Roles or structural IDs.

Capacity Management Responsibility Boundary

The 2026-10-09 human adjudication establishes **Capacity Management** as Business responsibility for understanding and managing available, committed and utilised capacity in six service-delivery operational contexts. Clinic/practice, ward, theatre, ED, outpatient and mobile service capacity retain their distinct Business meanings. This does not establish a common capacity model, information schema or lifecycle. Generalised service/location/patient/operational-state capture belongs to downstream derivation and is not a Domain03 Function.

Context-specific responsibility establishes/manages service capacity. Resource state contributes to it: an available bed alone does not mean a ward can accept another patient; staffing, demand, acuity and operational constraints may also matter. Service Capacity Management

consumes/coordinates context-specific capacity; Work Allocation & Dispatch and Discharge Management may consume capacity/availability or capacity/progression information without becoming its owners. These are Business responsibility relationships, not newly allocated Services or consumption contracts.

HSO context	Context-specific capacity responsibility
Clinic & Practice Operations	Yes — session/queue behaviour contributes; overall clinic/ practice capacity is expressed separately in §2.1.
Ward Operations	Yes — existing operational coordination is reframed as ward capacity in §2.2.
Theatre Operations	Yes — case progression contributes; theatre capacity is expressed separately in §2.3.
Emergency Department Operations	Yes — existing departmental capacity behaviour is clarified in §2.4.
Outpatient Operations	Yes — existing session utilisation is reframed as outpatient capacity in §2.5.
Bed & Care-Place Management	No — establishes resource state contributing to ward capacity.
Clinical Resource Management	No — establishes equipment state/availability; resource readiness is distinct from service capacity.
Service Capacity Management	No — consumes/coordinates context-specific capacity.
Mobile Staff Management	Yes — presence and visit progression contribute; mobile service capacity is expressed separately in §2.9.
On-Call Management	No — resolves coverage rather than establishing context-specific capacity.
Work Allocation & Dispatch	No — may consume capacity/availability for operational work.
Credential Management	No — verifies operational access.
Patient Transport	No — coordinates transport requests/progression.
Clinical Logistics Coordination	No — coordinates logistics/custody.
Discharge Management	No — may consume capacity/progression for discharge coordination.
Clinical Qualification Management	No — verifies qualifications/competency for applicable dispatch.

Clinical work integration does not confer clinical-work management, clinical handover, clinical task determination, clinical allocation authority or EMR worklist ownership. Capacity and progression consumers retain the originating authority of clinical facts.

2. Facility Operational Units & Queue Coordination

RAD source listing (presentation only; no inferred ancestry).

Health Service Operations Contexts

1. Clinic & Practice Operations
2. Ward Operations
3. Theatre Operations
4. Emergency Department Operations
5. Outpatient Operations
6. Bed & Care-Place Management
7. Clinical Resource Management
8. Service Capacity Management
9. Mobile Staff Management
10. On-Call Management
11. Work Allocation & Dispatch
12. Credential Management

13. Patient Transport
14. Clinical Logistics Coordination
15. Discharge Management
16. Clinical Qualification Management

2.1 Clinic & Practice Operations

- **Owning Capability**: `Clinic&PracticeOperations`
- **Functions & Exposed Services**:
  - **Capability-scoped capacity behaviour**:
    - *Function*: `ManageClinic&PracticeCapacity` — Understands and manages available, committed and utilised clinic/practice capacity in relation to practitioner availability, sessions, rooms, appointments and patient demand.
    - *Responsibility boundary*: Operating-session tracking and individual queue progression contribute to this responsibility; neither alone expresses overall service capacity. Capacity coordination does not own source bookings, clinical decisions or practitioner rostering.
  - **Feature: Clinic Session Tracking**:
    - *Function*: `TrackClinicOperatingSession` — Ingests and tracks clinic operating-session start, delay and completion, distinct from an individual patient's queue progression.
    - *Derivation*: `FEAT-HSO-01` directly establishes this session-level behaviour; Manage Clinic Queue Progression and its Process concern individual patients and do not supply it.
    - *Capacity contribution*: Session start/delay/completion remains distinct observable Business behaviour contributing to clinic/practice capacity; it is not merged into capacity merely for structural neatness.
  - **Feature: Operational Queue Progression**:
    - *Function*: `ManageClinicQueueProgression` — Tracks patient arrival, rooming, consultation in-progress, and check-out states.
    - *Exposed Service*: `ClinicQueueTelemetryService` — Discloses real-time clinic waiting times and queue states.
    - *Governed Process*: **Clinic / Operational Queue Progression Process**.
- **Information Responsibility**: Clinic & Practice Capacity, Clinic Session Progression, Clinic Queue State, Patient Waiting Time Metrics.

2.2 Ward Operations

- **Owning Capability**: `WardOperations`
- **Functions & Exposed Services**:
  - **Capability-scoped capacity behaviour**:
    - *Function*: `ManageWardCapacity` — Coordinates available, committed and utilised ward capacity and its patient-flow implications, retaining acuity distribution, nurse-to-patient ratios and isolation cohorting alongside bed/care-place state, patient demand and operational constraints.
    - *Name Alias*: `CoordinateWardOperationalState` — the same established ward operational responsibility, clarified as Capacity Management; no new Function identity or identifier is allocated.
    - *Exposed Service*: `WardOperationalSummaryQuery` — Discloses ward-level operational telemetry.
    - *Existing Feature contribution*: Ward Patient Flow Coordination concerns ward occupancy, care-place allocation and patient bed status. Those patient-flow responsibilities remain within ward operational coordination; they do not define the whole capacity responsibility.

    - *Capacity boundary*: Bed & Care-Place Management contributes resource state. Ward capacity is assessed in its service context; an available bed alone does not establish capacity to accept a patient. Source staffing and clinical assertions retain their authority.
  - **Feature: Clinical Handover Context Assembly**:
    - *Function*: `AssembleWardHandoverContext` — Collates current ward patient lists, active medical concerns and pending tasks for nursing shift handover.
    - *Derivation*: `FEAT-HSO-04` directly requires this assembly. Ward acuity/cohorting telemetry alone does not provide the stated handover content. Source clinical information retains its originating authority; assembly does not perform clinical review, manage clinical handover, determine pending clinical work or acquire clinical judgement.
- **Information Responsibility**: Ward Capacity, Ward Shift Status Ledger, Acuity Balance Register, Ward Handover Context.

2.3 Theatre Operations

- **Owning Capability**: `TheatreOperations`
- **Functions & Exposed Services**:
  - **Capability-scoped capacity behaviour**:
    - *Function*: `ManageTheatreCapacity` — Understands and manages available, committed and utilised theatre capacity in relation to theatre, team and equipment availability, scheduled cases and case progression.
    - *Responsibility boundary*: Case milestones and targeted resource notifications contribute; they do not by themselves establish available or committed theatre service capacity. Clinical case determination and originating team/equipment information remain separately governed.
  - **Feature: Operating Theatre Case Progression**:
    - *Function*: `ManageTheatreCaseProgression` — Coordinates surgical case milestones: *Case Called → Patient in Anaesthetic Bay → Anaesthesia Commenced → Knife to Skin → Procedure Finished → In PACU/Recovery → Ward Handover Complete*.
    - *Exposed Service*: `TheatreCaseProgressionTelemetry` — Emits real-time surgical milestone updates.
    - *Additional Feature responsibility*: `FEAT-HSO-06–PerioperativeResourceNotification` is realised by dispatching progression-triggered alerts to portering, sterilisation and recovery teams within this same case-progression behaviour/exposure. Milestone observation alone does not establish notification; no new Function or Service is created.
    - *Governed Process*: **Theatre Case Progression Process**.
- **Information Responsibility**: Theatre Capacity, Theatre Slate Register, Perioperative Milestone Log.

2.4 Emergency Department Operations

- **Owning Capability**: `EmergencyDepartmentOperations`
- **Functions & Exposed Services**:
  - **Capability-scoped capacity behaviour**:
    - *Function*: `CoordinateEDDepartmentalCapacity` — Understands and manages available, committed and utilised ED capacity in relation to care-place availability, demand, patient presence/progression, acuity and operational constraints; retains waiting room occupancy, resuscitation bay availability and time-to-triage metrics.
    - *Exposed Service*: `EDOperationalStatusQuery` — Discloses facility ED occupancy and surge level.
    - *Existing Feature scope*: ED Length of Stay Tracking retains the existing wait/progression metrics context; capacity responsibility is broader than that Feature. This clarification does not assert a new whole-Feature realisation relationship.

    - *Additional Feature relationship*: `FEAT-HSO-07–EDBed&BayTracking` is materially realised by this capacity-tracking behaviour for occupancy/status of emergency cubicles, resuscitation bays and short-stay beds. Those care-place states are explicit within the existing Function and its operational status exposure.
    - *Capacity boundary*: Existing departmental capacity responsibility is clarified, not duplicated. Care-place state contributes to service capacity; capacity coordination neither performs clinical triage nor establishes clinical acuity assertions.
- **Information Responsibility**: ED Capacity, ED Operational Dashboard Ledger, Resuscitation Bay Occupancy Matrix.

2.5 Outpatient Operations

- **Owning Capability**: `OutpatientOperations`
- **Functions & Exposed Services**:
  - **Capability-scoped Functions / Services**:
    - *Function*: `ManageOutpatientCapacity` — Understands and manages available, committed and utilised outpatient capacity through clinic/session, practitioner and room availability, appointment demand, patient arrival/waiting and operational progression. Retains specialty room allocation and clinician arrival tracking.
    - *Name Alias*: `CoordinateOutpatientSessionUtilisation` — the existing responsibility reframed as outpatient Capacity Management; no new Function identity or identifier is allocated.
    - *Feature responsibility*: `FEAT-HSO-09–OutpatientArrivalRegistration` is realised within this capacity behaviour by ingesting patient check-in/arrival events and notifying treating clinicians. Patient arrival contributes demand/presence; it does not itself establish capacity. This bounded responsibility does not equate the whole capacity Function with the Feature or create a separate Arrival Registration Function.
    - *Exposed Service*: `AmbulatorySessionStatusQuery` — Exposes clinic room occupancy.
    - *Exposure qualification*: Query behaviour and Outpatient Operations ownership remain established; a meaningful outside consumer/mandate is not identified by the current description. That exposure relationship remains unestablished rather than populated with an inferred Actor, Role or Capability.
    - *Capacity boundary*: Operational coordination does not acquire authoritative booking/rostering or clinical-work management. Room-occupancy query remains narrower than the capacity responsibility; no enlarged exposure or new named Service is inferred.
  - **Feature: Post-Clinic Order Coordination**:
    - *Function*: `CapturePost-ClinicFollow-upRequirements` — Captures follow-up booking and diagnostic-order requirements generated at clinic completion.
    - *Derivation*: `FEAT-HSO-10` directly establishes this capture responsibility. The Clinic queue Process's consultation-concluded checkpoint describes orders placed, not Outpatient Operations' capture of follow-up requirements. Capture does not authorise clinical orders, own authoritative bookings or establish a new Service dependency.
- **Information Responsibility**: Outpatient Capacity, Outpatient Arrival/Waiting Demand, Outpatient Session Schedule, Room Allocation Register, Post-Clinic Follow-up Requirements.

2.6 Bed & Care-Place Management

- **Owning Capability**: `Bed&Care-PlaceManagement`
- **Key Architectural Semantics**:
  - **Operational State vs. Definition**: While *Location Administration* defines the physical care-place, *Bed & Care-Place Management* owns the real-time operational status (*Available*, *Occupied*, *Reserved*, *Blocked*, *Dirty*, *Cleaning In-Progress*, *Maintenance Lock*).
  - **Resource State vs. Capacity**: This state contributes to Ward Operations' capacity responsibility. Bed availability does not establish ward service capacity or authority to accept another patient.

- **Functions & Exposed Services**:
  - **Feature: Bed Availability Tracking**:
    - *Function*: `MaintainOperationalBedState` — Tracks real-time bed status and environmental locks.
    - *Exposed Service*: `BedAvailabilityQuery` — Discloses real-time bed availability across hospital directorates.
  - **Feature: Bed State Progression**:
    - *Function*: `ManageBedTurnoverProgression` — Coordinates cleaning, disinfection, and preparation: *Vacated* → *Cleaning Requested* → *Cleaning In-Progress* → *Inspected/Ready* → *Occupied*.
    - *Exposed Service*: `BedTurnoverLifecycleService` — Coordinates turnover requests with environmental services.
    - *Additional Feature responsibility*: `FEAT-HSO-13–BedReadinessNotification` requires the existing turnover behaviour/exposure to notify bed allocation managers and clinical units when a cleaned bed is ready. Readiness notification remains distinct from availability query and from physical placement; no new lifecycle state or Service is introduced.
    - *Governed Process*: **Bed Turnover Process**.
- **Information Responsibility**: Operational Bed Status Registry, Care-Place Lock Register, Bed Turnover Milestone Log.

2.7 Clinical Resource Management

- **Owning Capability**: `ClinicalResourceManagement`
- **Functions & Exposed Services**:
  - **Feature: Mobile Clinical Equipment Tracking**:
    - *Function*: `TrackMobileClinicalAssetOperationalState` — Manages the real-time location and operational readiness of mobile ventilators, infusion pumps, and telemetry transmitters.
    - *Exposed Service*: `ClinicalResourceAvailabilityQuery` — Exposes mobile equipment availability.
- **Information Responsibility**: Mobile Clinical Asset Operational Ledger, Maintenance Readiness Matrix.

2.8 Service Capacity Management

- **Owning Capability**: `ServiceCapacityManagement`
- **Functions & Exposed Services**:
  - **Feature: Operational Capacity Metric Aggregation**:
    - *Function*: `AggregateServiceCapacityTelemetry` — Consumes and coordinates context-specific service-capacity information for broader situational awareness, alongside bed occupancy, ICU surge capacity and ventilator availability across regional hospitals.
    - *Exposed Service*: `RegionalHealthSystemCapacityQuery` — Provides executive operational situational awareness.
    - *Consumer boundary*: Aggregation does not establish the underlying context-specific capacity. Resource occupancy/readiness and service capacity remain distinguishable; exact additional provider contracts are unestablished.
- **Information Responsibility**: Regional System Capacity Snapshot, Surge Level State.

2.9 Mobile Staff Management

- **Owning Capability**: `MobileStaffManagement`

- **Functions & Exposed Services**:
  - **Capability-scoped capacity behaviour**:
    - *Function*: `ManageMobileServiceCapacity` — Understands and manages available, committed and utilised mobile-service capacity in relation to workforce availability, geography, time, workload and applicable service demand.
    - *Responsibility boundary*: Staff presence and externally established assignments/visit progression contribute to capacity; they do not alone express it. Capacity coordination does not determine clinical work, own the source workforce roster or allocate underlying clinical duties.
  - **Capability-scoped Functions / Services**:
    - *Feature association not established*: `FEAT-HSO-16–MobileWorkerTaskDispatch` retains its Strategy identity with the integration boundary below. Presence/zone tracking remains distinct from mobile work-management integration; no replacement association to presence is assigned.
    - *Function*: `TrackOperationalStaffPresence` — Tracks active on-duty presence and physical zone assignment for porters, phlebotomists, and roving nurses.
    - *Exposed Service*: `StaffPresenceTelemetryQuery` — Discloses on-duty staff availability by hospital zone.
  - **Feature: Mobile Visit Status Tracking**:
    - *Function*: `ReceiveMobileVisitProgression` — Ingests arrival, visit-progression and safety check-in milestones from roving staff.
    - *Derivation*: `FEAT-HSO-17` directly requires visit-specific milestone ingestion. It remains distinct from presence, capacity assessment and work-management integration. Received progression can inform mobile capacity without determining clinical work or clinical completion.
  - **Feature: Mobile Worker Task Dispatch** (integration scope qualified in Strategy):
    - *Function*: `IntegrateMobileWorkManagement` — Exchanges externally established work/assignment, dispatch and operational progression/status information, with applicable worker/context information, between mobile work-management mechanisms and Harmonia operational coordination.
    - *Derivation and boundary*: The human adjudication of `FEAT-HSO-16` establishes integration with mobile work/task management. This responsibility is absent from presence observation and visit-specific milestone ingestion. It does not determine, allocate or manage the underlying clinical work, clinical handover or EMR worklists. Work Allocation & Dispatch retains non-clinical operational Work Order responsibility.
    - *Terminology*: External “work/task” vocabulary does not make human work a Harmonia Task. Work Order means human doing; To Do means human review/update/approval; Task means synthetic task. The preferred Strategy name `MobileWorkTaskManagementIntegration` remains a proposed naming correction pending human confirmation, not a new Feature or alias.
- **Information Responsibility**: Mobile Service Capacity, Staff Operational Presence Register, Zone Allocation Matrix, Mobile Visit Progression, Exchanged Mobile Work-Management Information. Originating work/assignment authority remains with its external/business work-management source.

2.10 On-Call Management

- **Owning Capability**: `On-CallManagement`
- **Functions & Exposed Services**:
  - **Feature: Active On-Call Provider Resolution**:
    - *Function*: `ResolveActiveOn-CallCoverage` — Resolves primary, secondary, and tertiary on-call clinical specialists for emergency escalation.
    - *Exposed Service*: `On-CallSpecialistLookup` — Discloses active on-call contacts for switchboard and emergency teams.
- **Information Responsibility**: On-Call Roster Projection, Escalation Contact Matrix.

2.11 Work Allocation & Dispatch

- **Owning Capability**: `WorkAllocation&Dispatch`
- **Key Architectural Semantics**:
  - **Ownership Guardrail**: Owns the dispatching, queuing, assignment, and operational progression of non-clinical work orders (e.g., patient transfers, cleaning, specimen pickups). It does **not** acquire ownership of Practitioners, Locations, Services, or Credentials.
  - **Capacity Consumer**: May use context-specific capacity/availability in operational allocation; it neither establishes that capacity nor acquires clinical-work allocation authority through mobile integration.
- **Functions & Exposed Services**:
  - **Feature: Work Item Instantiation**:
    - *Function*: `DispatchOperationalWorkOrder` — Allocates work requests to available wardspersons or support teams using applicable allocation policies, staff roles, credentials, location and skill, and delivers allocated work to the target worker, device or team queue.
    - *Exposed Service*: `WorkDispatchService` — Delivers allocated work and captures distinct technical and operational acknowledgements; acknowledgement alone does not establish completion.
    - *Additional Feature relationships*: `FEAT-HSO-20—WorkerMatching&Allocation` is realised by the existing allocation behaviour; `FEAT-HSO-21—WorkDispatchDelivery` by its dispatch exposure and acknowledgement capture. Applicable credential/privilege determinations remain separately governed; Role information is not itself privilege.
  - **Feature: Work Progress Oversight & Escalation**:
    - *Function*: `ManageOperationalWorkProgression` — Tracks execution states: *Queued → Assigned → Accepted → In-Progress → Completed / Aborted*.
    - *Exposed Service*: `OperationalWorkStatusQuery` — Discloses task progression to requesting wards.
    - *Governed Process*: **Operational Work Progression Process**.
- **Information Responsibility**: Operational Work Dispatch Ledger, Job Progression Milestone Ledger.

2.12 Credential Management

- **Owning Capability**: `CredentialManagement`
- **Functions & Exposed Services**:
  - **Capability-scoped behaviour: Operational Credential Check** (Feature association not established):
    - *Distinction*: Operational badge/access `CredentialManagement` is distinct from Strategy `ClinicalQualificationManagement`. Lexical similarity does not bind this behaviour to qualification/competency Feature `FEAT-HSO-23`.
    - *Function*: `VerifyOperationalAccessCredential` — Verifies physical access badges, specialty ward entry authorizations, and restricted area permissions.
    - *Exposed Service*: `OperationalCredentialCheckpointService` — Evaluates physical/operational access rights.
- **Information Responsibility**: Operational Access Authorization Ledger, Badge Credential Binding Register.

2.13 Patient Transport

- **Owning Capability**: `PatientTransport`
- **Functions & Exposed Services**:
  - **Feature: Transport Dispatch & Progress Tracking**:

    - *Function*: `ManagePatientTransportProgression` — Coordinates intra-facility and inter-facility patient movements: *Transport Requested → Porter Dispatched → Patient Collected → In-Transit → Delivered at Destination → Handover Completed*.
    - *Exposed Service*: `PatientTransportTrackingService` — Provides real-time transit telemetry.
    - *Governed Process*: **Patient Transport Process**.
    - *Additional Feature responsibility*: `FEAT-HSO-24–TransportRequestIngestion` is directly supported by the request stage of this same Function/Process. It ingests internal portering and external ambulance/inter-hospital requests with clinical mobility requirements; request ingestion is distinct from dispatch, transit and arrival.
- **Information Responsibility**: Patient Transport Request Master, Transit Milestone Log.

2.14 Clinical Logistics Coordination

- **Owning Capability**: `ClinicalLogisticsCoordination`
- **Functions & Exposed Services**:
  - **Feature: Pathology Specimen Transport Tracking**:
    - *Function*: `ManageSpecimenTransportProgression` — Tracks the physical transit of pathology bio-specimens, blood products, and surgical trays: *Collected → Courier Picked Up → In-Transit → Laboratory Ingress Received*.
    - *Exposed Service*: `SpecimenTransitTrackingService` — Exposes chain-of-custody tracking.
    - *Governed Process*: **Specimen Transport Process**.
- **Information Responsibility**: Specimen Courier Manifest, Chain-of-Custody Tracking Register.

2.15 Discharge Management

- **Owning Capability**: `DischargeManagement`
- **Capacity/Progression Consumer**: May use applicable service capacity and patient progression in discharge coordination. It does not establish the consumed capacity or originating clinical clearances; clinical progression, operational readiness, discharge authorisation and physical departure remain distinct.
- **Functions & Exposed Services**:
  - **Feature: Discharge Readiness & Coordination Oversight**:
    - *Function*: `ManageDischargeCoordinationProgression` — Synchronises multi-agency discharge readiness (**illustrative preparation; publication, authorisation and confirmed exit are distinct**): *Discharge Planning Initiated → Medications Reconciled → Transport Booked → Discharge Summary Finalised → Physically Departed*.
    - *Additional Feature relationship*: `FEAT-HSO-27–DischargeMilestoneTracking` is realised by this existing Function/Process's clinical readiness, medications, transport/services and home-care confirmation milestones.
    - *Departure-triggered responsibility*: `FEAT-HSO-29–DischargeNotificationDispatch` requires this behaviour to dispatch departure notifications, updated bed states and finalised discharge summaries to external care providers upon confirmed physical exit. Preparation/publication of applicable information may occur earlier; it does not satisfy this departure-triggered responsibility.
    - *Scope qualification*: Preparation, summary finalisation/signing, availability/publication, applicable discharge authorisation and confirmed physical departure remain distinct. The displayed preparation pathway is illustrative, not a universal state machine. Discharge readiness or available information does not establish discharge authorisation or departure.
    - *Exposed Service*: `DischargeReadinessTelemetry` — Discloses discharge barrier checklists and planned departure times.
    - *Governed Process*: **Discharge Progression Process**.

- **Information Responsibility**: Discharge Readiness Checklist Ledger, Estimated Date of Discharge (EDD) Register.

2.16 Clinical Qualification Management

- **Owning Capability**: `ClinicalQualificationManagement`.
- **Functions**:
  - **Feature: Operational Credential Check**:
    - *Function*: `VerifyClinicalQualificationforWorkDispatch` — Validates that a practitioner possesses active clinical credentials and required competency certificates before dispatch of specialised work.
    - *Derivation*: `FEAT-HS0-23` belongs to Strategy Clinical Qualification Management. Provider Administration establishes professional qualification/registration and privilege information, but the dispatch-specific qualification/competency checkpoint was not expressed by that information management alone.
    - *Boundary*: This checkpoint does not originate qualifications, confer clinical privileges, manage clinical competency training or establish a provider Service dependency. Clinical Privilege and Operational Privilege remain distinct, and an activity may require both. Physical badge/access Credential Management in §2.12 remains independently owned and is not associated with this Feature.
- **Information Responsibility**: Qualification/Competency Checkpoint Outcome and Applicable Work Context; source qualification and privilege information retains its established authority and ownership.

The section number is navigation only. No Capability Tier, ancestry, structural ID, new Role, Service or Process is allocated by this directly derived Function.

21. Intrinsic / Shared Enablement — Capability-Scoped Behaviour

RAD source: `docs/markdown/03-business-architecture/behaviours/05-intrinsic-enablement.md`

The following is a presentation transcription of the governed source. Source catalogue layout and retained illustrations do not allocate tiers or establish additional identities. Equations wrap as text; diagram notation is textual.

Intrinsic / Shared Enablement — Capability-Scoped Behaviour

1. Contextual Scope & Architectural Intent

The named owning Capabilities and established Features retain their responsibilities. Affected Capability Tier, complete ancestry, root status and structural Canonical IDs remain unresolved under approved G1 K9; document grouping and numbering do not establish architectural identity or hierarchy.

Intrinsic and Shared Enablement encompasses the horizontal platform capabilities that provide foundational security, longitudinal clinical record assembly, information exchange, workflow orchestration, collaborative spaces, and semantic governance across all healthcare contexts.

The Cross-Cutting Boundary Guardrail

> **These capabilities are bounded architectural responsibilities whose services are widely consumed across the HIE. Delivering a shared service (such as security enforcement, message exchange, workflow coordination, or UI presentation) does not grant ownership of the clinical payloads, business activities, or decisions being processed.**

2. Intrinsic Enablement Capabilities

RAD source listing (presentation only; no inferred ancestry).

Intrinsic / Shared Enablement Capabilities

1. Patient Clinical Record
2. Health Information Exchange (HIE)
3. Health Information Access
4. Health Information Communication
5. Health Information Control
6. Clinical Knowledge Services
7. Clinical Collaboration
8. General Collaboration
9. Workflow & Activity Coordination
10. Calendar Management
11. Presentation Services
12. Information Design Governance

2.1 Patient Clinical Record

- **Owning Capability**: `PatientClinicalRecord`
- **Architectural Mandate**: Governs the canonical, longitudinal, cross-institutional health record for every patient across the healthcare region.
- **Functions & Exposed Services**:
  - **Feature: Longitudinal Health Record (LHR) Assembly**:
    - *Function*: `AssembleLongitudinalClinicalRecord` — Synthesises clinical documents, encounters, diagnostic reports, and medication histories across disparate source systems into a unified chronological clinical view.
    - *Exposed Service*: `LongitudinalClinicalRecordQuery` — Discloses the comprehensive clinical timeline to authorised clinicians.
  - **Feature: Active Clinical Record Access**:
    - *Function*: `MaintainActiveClinicalRecord` — Maintains active problem lists, current medication regimens, allergies, and open clinical alerts for active care coordination.
    - *Exposed Service*: `ActiveProblem&AllergySummaryQuery` — Exposes immediate active clinical summaries.
  - **Feature: Durable Clinical Record Preservation**:
    - *Function*: `PreserveDurableClinicalRecord` — Preserves the governed longitudinal representation and durable, immutable, append-only history of what Harmonia received, knew, asserted or managed, with provenance and versioning. Corrections/supersession establish changed knowledge without retrospectively altering that history or acquiring originating clinical authority.
    - *Exposed Service*: `HistoricalClinicalRecordAccess` — Provides governed historical record retrieval for authorised clinical and legal review; preservation does not itself establish legal qualification or originating authority.
- **Information Responsibility**: Governed Longitudinal Health Record (LHR), Canonical Clinical Timeline, Active Problem List, Immutable Historical Record Store.

2.2 Health Information Exchange (HIE)

- **Owning Capability**: `HealthInformationExchange`
- **Architectural Mandate**: Manages the ingestion, transformation, routing, addressed distribution, and publish-subscribe syndication of health information across organizational and jurisdictional boundaries.
- **Ownership Invariant**: Health Information Exchange owns **exchange transaction state, delivery receipts, and routing policies**. It does **not** acquire ownership of the business information transported.
- **Functions & Exposed Services**:
  - **Feature: Direct Submission Ingestion & Delivery**:
    - *Function*: `ReceiveInformationSubmission` — Validates and ingests standards-compliant clinical messages from external health providers.
    - *Exposed Service*: `HIESubmissionGatewayService` — Ingress endpoint for regional data contributions.
  - **Feature: Clinical Record Retrieval Mediation**:
    - *Function*: `MediateInformationRetrieval` — Federates queries across participating hospital and diagnostic node repositories.
    - *Exposed Service*: `FederatedHealthInformationQuery` — Resolves distributed record queries.
  - **Feature: Addressed Clinical Distribution**:
    - *Function*: `DistributeAddressedInformation` — Directs point-to-point clinical documents to specific recipient practitioner endpoints.
    - *Exposed Service*: `AddressedDocumentDeliveryService` — Securely delivers documents to designated recipient mailboxes.
  - **Feature: Event-Driven Syndication**:

    - *Function*: `SyndicateInformationChange` — Broadcasts event notifications and record updates to subscribed regional care network systems.
    - *Exposed Service*: `ClinicalEventSyndicationStream` — Emits publish-subscribe clinical event feeds.
- **Information Responsibility**: Exchange Transaction Ledger, Route Resolution Map, Subscription & Syndication Matrix.

2.3 Health Information Access

- **Owning Capability**: `HealthInformationAccess`
- **Architectural Mandate**: Provides high-performance search, retrieval, and access-qualified filtering of clinical information across the longitudinal record.
- **Ownership Invariant**: Consumes access control and consent determinations from *Health Information Control*. Search behaviour does **not** own security policy.
- **Functions & Exposed Services**:
  - **Feature: Federated Clinical Query**:
    - *Function*: `SearchHealthInformation` — Executes structured semantic searches across diagnostic reports, clinical notes, and medication histories.
    - *Exposed Service*: `ClinicalInformationSearchService` — Discloses search capabilities to clinical user interfaces.
  - **Capability-scoped Functions / Services**:
    - *Feature association not established*: `FEAT-ISE-09–Access-ControlledInformationFiltering` requires access-controlled disclosure/filtering; retrieval of discrete information alone does not materially realise that responsibility. No replacement Feature is assigned.
    - *Function*: `RetrieveHealthInformation` — Fetches discrete clinical observations and original attachments.
    - *Exposed Service*: `ClinicalResourceRetrievalService` — Delivers structured clinical information to requesting applications.
  - **Capability-scoped behaviour: Access-Qualified Result Filtering** (Feature identity not established):
    - *Function*: `ApplyAccess-QualifiedResultFiltering` — Redacts or masks search results and clinical content based on the caller's verified security context and patient consent directives.
    - *Exposed Service*: *(Embedded within Search and Retrieval Services)*.
- **Information Responsibility**: Query Execution Context, Filtered Result Projection State.

2.4 Health Information Communication

- **Owning Capability**: `HealthInformationCommunication`
- **Architectural Mandate**: Encapsulates standards-based health information communication to mediate technical transmission between participating parties.
- **Ownership Invariant**: Does **not** own every business interaction communicated through it; provides protocol adaptation and secure transport mediation.
- **Functions & Exposed Services**:
  - **Feature: Standards-Based Boundary Exchange**:
    - *Function*: `MediateStandards-BasedHealthInformationCommunication` — Handles standards-based communication mediation, transaction coordination, and technical error handling.
    - *Exposed Service*: `StandardsCommunicationGatewayService` — Protocol interface for external client integrations.
- **Information Responsibility**: Transport Protocol Binding Map, Network Session State.

2.5 Health Information Control

- **Owning Capability**: `HealthInformationControl`
- **Architectural Mandate**: Enforces enterprise default-deny authorization, ABAC/RBAC policy evaluation, consent restriction enforcement, immutable security context propagation, and non-repudiation audit logging.
- **Functions & Exposed Services**:
  - **Feature: Health Information Access & Consent Control**:
    - *Function*: `EvaluateInformationAccessAuthority` — Evaluates user role, organisation, patient relationship, purpose of use, and break-glass overrides against default-deny policies.
    - *Exposed Service*: `PolicyEvaluation&AuthorisationService` — Evaluates access requests across the platform.
  - **Capability-scoped Functions / Services**:
    - *Feature association not established*: `FEAT-ISE-12–SecurityContextPropagation` propagates attributable security context; evaluating consent constraints does not materially realise that responsibility. No replacement Feature is assigned.
    - *Function*: `EvaluateConsentConstraint` — Cross-references access requests against client consent directives and sensitive health category restrictions.
    - *Exposed Service*: `ConsentEnforcementService` — Returns masking and redaction instructions.
  - **Capability-scoped behaviour: Security Context Binding** (Feature identity not established):
    - *Function*: `PropagateSecurityContext` — Generates and binds canonical, immutable security contexts to all intra-platform transactions.
    - *Exposed Service*: `SecurityContextValidationService` — Verifies transaction security credentials.
    - *Exposure qualification*: Validation is the declared Service behaviour. Its juxtaposition with Propagate Security Context does not establish a one-to-one Function/Service relationship; the precise internal mapping remains unestablished.
  - **Feature: Non-PHI Compliance Auditing**:
    - *Function*: `RecordSecurity-SignificantActivity` — Records tamper-evident audit evidence (who, what, when, why, patient MRN, policy outcome) without logging unmasked PHI.
    - *Exposed Service*: `SecurityAuditIngress` — Ingests audit events across all capabilities.
- **Information Responsibility**: Authorisation Policy Ruleset, Security Context Tokens, Tamper-Evident Security Audit Trail.

2.6 Clinical Knowledge Services

- **Owning Capability**: `ClinicalKnowledgeServices`
- **Architectural Mandate**: Resolves canonical medical ontologies (SNOMED-CT, LOINC, ICD-10, AMT) and executes cross-terminology concept mappings.
- **Functions & Exposed Services**:
  - **Feature: Clinical Concept Resolution**:
    - *Function*: `ResolveClinicalConcept` — Validates concept identifiers and retrieves preferred clinical rubrics and hierarchy paths.
    - *Exposed Service*: `TerminologyConceptResolutionService` — Exposes concept validation lookups.
  - **Feature: Bidirectional Terminology Mapping**:
    - *Function*: `MapClinicalConcept` — Translates local lab codes and legacy terms to canonical SNOMED/LOINC codes using governed translation maps.
    - *Exposed Service*: `TerminologyMappingService` — Translates codes across terminology systems.

- **Information Responsibility**: Canonical Clinical Ontologies, Semantic Value Sets, Cross-Terminology Mapping Tables.

2.7 Clinical Collaboration

- **Owning Capability**: `ClinicalCollaboration`
- **Architectural Mandate**: Governs real-time multidisciplinary clinical collaboration channels, patient care-team virtual workspaces, and secure clinical discussions.
- **Key Architectural Semantics**:
  - **Discourse vs. Record Guardrail**: Real-time collaborative discussion and messaging in team channels do **not** automatically constitute authoritative clinical health records or observations. Authoritative entries require explicit formal document submission.
- **Functions & Exposed Services**:
  - **Feature: Care-Team Clinical Collaboration Coordination**:
    - *Function*: `CoordinateCare-TeamCollaboration` — Manages care-team communication spaces, participant memberships, and clinical handovers.
    - *Exposed Service*: `ClinicalCollaborationSpaceService` — Manages team collaboration spaces and discussions.
  - **Feature: In-Conversation LHR Query Resolution**:
    - *Function*: `ResolveLHRQuerywithinCollaboration` — Contextually projects active patient summaries directly into collaborative clinical discussion channels.
    - *Exposed Service*: `CollaborationClinicalSummaryResolution` — Embeds clinical summaries in collaboration feeds.
    - *Established ownership*: `ClinicalCollaboration→In-ConversationLHRQueryResolution[FEAT-ISE-17]→ResolveLHRQuerywithinCollaboration→CollaborationClinicalSummaryResolution`.
    - *Unresolved dependency*: The precise source Service/provider, direct versus mediated retrieval, formal consuming Capability/Function for the exposed Service and intended referent of the withdrawn dependency row remain unestablished. Information ownership, Function ownership, Service ownership, consumption, information use and presentation are distinct; no substitute dependency is implied.
  - **Feature: Zero-PHI Collaboration Metadata Governance**:
    - *Function*: `GovernCollaborationMetadata` — Enforces privacy policies, participant access gates, and zero-PHI space metadata rules.
    - *Exposed Service*: `CollaborationSpaceGovernanceService` — Governs space lifecycles and membership.
- **Information Responsibility**: Collaboration Space Metadata, Care-Team Channel Ledger, Discussion Thread Index.

2.8 General Collaboration

- **Owning Capability**: `GeneralCollaboration`
- **Functions & Exposed Services**:
  - **Feature: Operational Team Chat Enablement**:
    - *Function*: `CoordinateOperationalCollaboration` — Provides communication spaces for facility logistics, disaster management teams, and administrative committees.
    - *Exposed Service*: `OperationalCollaborationService` — Provides non-clinical operational messaging channels.
- **Information Responsibility**: Operational Channel Index, Non-Clinical Messaging Log.

2.9 Workflow & Activity Coordination

- **Owning Capability**: `Workflow&ActivityCoordination`

- **Architectural Mandate**: Coordinates execution state, task dispatch, timers, deadlines, and escalations across three distinct activity archetypes:
  1. **Work Order**: Assigned physical/human operational tasks (*human doing* — e.g., transport a patient, clean a bay).
  2. **To Do**: Assigned clinical review or administrative decision tasks (*human reviewing/updating/deciding* — e.g., review abnormal lab result, countersign discharge summary).
  3. **Synthetic Task**: Automated system-executable activities (*non-human executable work* — e.g., generate summary, syndicate batch, evaluate rules).
- **Ownership Invariant**: Workflow & Activity Coordination owns **generic activity coordination semantics and timers**. It does **not** acquire ownership of the business meaning or clinical outcome of the activity.
- **Clinical-work boundary**: Coordination supports externally established work requirements and authorised human review/update/approval. It does not determine clinical tasks, manage clinical handover or own EMR clinical worklists. The clinical-work integration rule preserves clinical authority and the three work-unit meanings; Task remains synthetic work.
- **Outcome uncertainty**: For materially significant managed activity outcomes, missing acknowledgement, response or observation does not establish success or failure. Preserve uncertainty pending authoritative observation/reconciliation. This is a Business obligation, not a universal new lifecycle state; assurance evidence insufficiency remains distinct from execution uncertainty.
- **Functions & Exposed Services**:
  - **Feature: Work Order Progression**:
    - *Function*: `CoordinateWorkOrder` — Coordinates operational task lifecycles and dispatches.
    - *Exposed Service*: `WorkOrderCoordinationService` — Manages operational work progression.
    - *Governed Process*: **Work Order Progression Process**.
  - **Feature: To Do Decision & Approval Coordination**:
    - *Function*: `CoordinateToDo` — Coordinates human review/update/approval items, reminders and sign-off queues within Harmonia's operational responsibility; clinical requirements and decisions remain with accountable clinical authority.
    - *Exposed Service*: `ToDoManagementService` — Provides To Do inbox and sign-off coordination without acquiring underlying clinical-work management.
    - *Governed Process*: **To Do Progression Process**.
  - **Feature: Synthetic Task Orchestration**:
    - *Function*: `CoordinateSyntheticTask` — Orchestrates automated system workflows and operational progression.
    - *Exposed Service*: `SyntheticTaskExecutionService` — Coordinates automated tasks.
    - *Governed Process*: **Synthetic Task Progression Process**.
  - **Feature: Workflow Timeout & Escalation Oversight**:
    - *Function*: `SuperviseActivityTimeout/Escalation` — Monitors SLAs and triggers escalations when clinical or operational deadlines elapse.
    - *Exposed Service*: `ActivityEscalationTelemetry` — Discloses SLA breaches.
- **Information Responsibility**: Generic Activity Instance Registry, Task State Progression Graph, SLA Deadline & Timer Ledger.

2.10 Calendar Management

- **Owning Capability**: `CalendarManagement`
- **Architectural Mandate**: Projects unified chronological timelines across appointments, on-call rosters, and scheduled operational events.
- **Ownership Invariant**: Does **not** acquire authoritative ownership of rosters or appointment bookings (which remain owned by source scheduling and workforce systems).
- **Functions & Exposed Services**:
  - **Feature: Schedule Projection Integration**:

  - *Function*: `ProjectOperationalSchedule` — Aggregates and overlays multidisciplinary schedules onto a unified temporal view.
  - *Exposed Service*: `UnifiedCalendarProjectionQuery` — Discloses consolidated temporal schedules.
- **Feature: Temporal Event Correlation**:
  - *Function*: `CorrelateActivitywithTemporalContext` — Associates clinical events with shift boundaries and appointment slots.
  - *Exposed Service*: `TemporalContextResolution` — Resolves operational shifts.
- **Information Responsibility**: Projected Unified Timeline, Temporal Correlation Index.

2.11 Presentation Services

- **Owning Capability**: `PresentationServices`
- **Architectural Mandate**: Delivers user-facing portals, clinical dashboards, administrative consoles, and mobile views.
- **Ownership Invariant**: Presentation owns **rendering, user interaction, and layout concerns**. It does **not** own the underlying clinical or operational information being rendered.
- **Functions & Exposed Services**:
  - **Feature: Longitudinal Clinical & Operational Presentation**:
    - *Function*: `PresentContextualHealthandOperationalInformation` — Composes clinical summaries, work queues, and patient records into responsive role-based user interfaces.
    - *Exposed Service*: `ContextualPresentationService` — Delivers contextual health and operational information for presentation.
- **Information Responsibility**: UI View Configuration, User Preference Profile, Presentation Layout Specification.

2.12 Information Design Governance

- **Owning Capability**: `InformationDesignGovernance`
- **Architectural Mandate**: Governs the enterprise canonical information model, data definitions, constraint schemas, terminology bindings, and exchange profiles.
- **Functions & Exposed Services**:
  - **Feature: Information Standards & Semantic Governance**:
    - *Function*: `GovernInformationSemantics` — Authors and validates canonical data concepts and entity relationships.
    - *Exposed Service*: `SemanticModelDefinitionService` — Discloses canonical model definitions.
  - **Capability-scoped behaviour: Information Definition & Schema Governance** (Feature identity not established):
    - *Function*: `GovernInformationDefinition` — Maintains governed data element dictionaries, data types, and structural rules.
    - *Exposed Service*: `InformationDictionaryQuery` — Provides data dictionary specifications.
  - **Capability-scoped behaviour: Terminology Binding Governance** (Feature identity not established):
    - *Function*: `GovernTerminologyBinding` — Binds canonical data elements to authoritative terminology value sets.
    - *Exposed Service*: `TerminologyBindingSpecificationLookup` — Discloses value set bindings.
  - **Capability-scoped behaviour: Exchange Profile Governance** (Feature identity not established):
    - *Function*: `GovernExchangeProfile` — Defines standards-based interchange profiles and message constraints.

    - *Exposed Service*: `ExchangeProfileSpecificationQuery` — Discloses interoperability schemas.
  - **Feature: Semantic Conformance Verification**:
    - *Function*: `VerifySemanticConformance` — Validates candidate messages against canonical governance schemas.
    - *Exposed Service*: `SemanticConformanceVerificationService` — Validates representations for compliance.
- **Information Responsibility**: Enterprise Canonical Semantic Model, Governed Data Dictionary, Terminology Binding Matrix, Exchange Profile Catalogue, Semantic Conformance Ruleset.

22. Canonical Enterprise Capability Model (EC-01 .. EC-14)

RAD source: `docs/markdown/02-strategy/capabilities/enterprise-capabilities.md`

The following is a presentation transcription of the governed source. Source catalogue layout and retained illustrations do not allocate tiers or establish additional identities. Equations wrap as text; diagram notation is textual.

Canonical Enterprise Capability Model (EC-01 .. EC-14)

Overview & Purpose

The Enterprise Capability Tier represents **reusable ICT architectural functionality** that recurs across multiple distinct healthcare business enabling capabilities and features.

Enterprise Capabilities answer the fundamental architectural question:

> **”How is this function delivered across our systems?”**

Where Business Enabling capabilities are defined within authentic clinical, operational, and administrative healthcare contexts (e.g., Ward Census, Medication Administration, Closed-Loop Orders), Enterprise Capabilities represent the **horizontal, domain-neutral capabilities** through which those healthcare features are delivered.

RAD source listing (presentation only; no inferred ancestry).

> BUSINESS ENABLING FEATURES (Hundreds of domain-specific behaviours)
>
> Example: Patient Bed Turnover, Pathology Order, Practitioner Lookup
>
> derived across features by asking:
>
> ”How is this function delivered?”
>
> ▼
>
> ENTERPRISE CAPABILITY TIER (EC-01 .. EC-14)
>
> 14 Reusable, Technology-Neutral Architectural Capabilities
>
> EC-01 .. EC-13 inform clustering of
>
> ▼
>
> STRATEGIC LOGICAL RESPONSIBILITY MODEL
>
> Mneme, Mnemosyne, Ponos, Pylai, Calliope, Iris, Digital Twin

The component-clustering progression describes established EC-01 through EC-13 derivations only. EC-14 is added by the approved Health Service Assurance Strategy derivation; no strategic logical or application component is allocated EC-14 by this diagram or catalogue.

Derivation Methodology & Architectural Principles

The established EC-01 through EC-13 were derived by systematically analyzing the system-enabled features across all five Business Enabling views. EC-14 Service Guardian is the approved addition derived from REQ-FND-005, BC-18 and the three assurance Business Enabling Capabilities. Their first-pass contribution model records the explicitly approved relationships without inventing intermediate Features or component allocations. The derivation adheres to five strict architectural principles:

1. Vertical Recurrence Test

A functional requirement qualifies as an Enterprise Capability only if it recurs vertically across multiple, diverse healthcare domains.

- If a function appears only within laboratory orders, it remains domain-specific Business Enabling behaviour.
- If a function recurs across clinical document versioning, directory update proposals, and bed state turnover, it reflects an underlying Enterprise Capability (**EC-03 Managed State & Lifecycle**).

2. Behavioural Properties vs. Top-Level Capabilities

Not every recurring function or algorithmic mechanism warrants promotion to a standalone top-level Enterprise Capability:

- **Correlation** is an intrinsic behavioural property appearing across multiple capabilities (e.g., entity correlation in EC-01, event correlation in EC-09, context correlation in EC-02), **not** a standalone `CorrelationService`.
- **Acknowledgement** is a reusable interaction pattern within exchange semantics (EC-08) and operational activity tracking (EC-10), **not** an independent capability.
- **Assignment and Dispatch** are core execution functions within **EC-10 Activity & Execution**.
- **Indexing and Caching** are technical enabling mechanisms within **EC-04 Information Management** and **EC-05 Search & Discovery**, not top-level architectural capabilities.
- **Retry and Circuit Breaking** are resilience properties within **EC-12 Operational Assurance**.

3. Reusable Capability ≠ Centralised Service

> **An Enterprise Capability describes reusable architectural functionality; it does NOT imply a monolithic, centralized runtime service.**

Enterprise Capabilities represent bounded reusable responsibilities. Their realisation may require participation, consumption or enforcement across multiple architectural elements, but this does not imply shared semantic ownership or prohibit establishment of a responsibility centre.

Distributed participation does not imply distributed responsibility. Cross-cutting concern does not imply cross-cutting responsibility.

Examples of participation in established reusable responsibilities include:

- **EC-02 Context Management**: Context is established at ingress (Pylai), maintained during active access (Mneme), propagated across execution units (Ponos), and preserved in audit (Kleio) where required. These participation points do not require a monolithic Context Service or establish shared semantic ownership.
- **EC-06 Policy & Control**: Policy evaluation guards ingress boundaries, persistence interfaces and presentation gateways while retaining bounded policy responsibility.
- **EC-07 Provenance & Traceability**: Required attribution and meaningful evidence are preserved at significant boundaries and transformations; routine technical activity does not automatically require durable evidence.
- **EC-12 Operational Assurance**: Resilience, duplicate suppression and concurrency integrity may require enforcement across messaging, storage and execution elements without distributing semantic ownership.

Responsibility centres follow established architectural responsibility. No capability is allocated merely to complete a diagram, and distributed participation does not prohibit a responsibility centre.

4. Zero Technology Leakage

Enterprise Capabilities are technology-neutral:

- They are free of concurrency primitives (threads, worker thread pools, execution daemons).
- They are free of message broker constructs (topics, queues, partitions, JMS headers).
- They are free of storage technologies (PostgreSQL, Infinispan, JPA, DDL, table locks).
- They are free of specific software packages (HAPI FHIR, Apache Camel, Netty, Spring Boot, Vue 3).

5. Capabilities vs. Software Components

An Enterprise Capability describes *what* functionality is provided. Subsystems (such as Pylai, Ponos, Mneme, Mnemosyne, Calliope, and Iris) represent *strategic logical component responsibility centres* defined in the existing Strategic Logical Component Responsibility Model. Software components are consumers, aggregators, or providers of capabilities; **a software component name is never a capability definition**.

The 14 Canonical Enterprise Capabilities

RAD source listing (presentation only; no inferred ancestry).

EC-01 Managed Entity & Relationship

EC-02 Context Management

EC-03 Managed State & Lifecycle

EC-04 Information Management

EC-05 Search & Discovery

EC-06 Policy & Control

EC-07 Provenance & Traceability

EC-08 Interoperability & Exchange

EC-09 Event & Subscription

EC-10 Activity & Execution

EC-11 Interaction & Experience

EC-12 Operational Assurance

EC-13 Semantic Governance & Conformance

EC-14 Service Guardian

EC-01: Managed Entity & Relationship

- **Architectural Scope**: Identify, classify, relate, and maintain the structural integrity of managed healthcare and administrative entities throughout the enterprise landscape.
- **Functional Responsibilities**:
  - Uniquely identifying core domain entities (Persons, Practitioners, Roles, Organisations, Locations, Services, Devices).
  - Resolving and maintaining multi-authority identifier systems and aliases without destructive overwriting.
  - Representing and traversing multi-dimensional relationships (affiliations, organizational hierarchies, physical location trees, care team memberships).
  - Maintaining referential integrity across interconnected entity graphs.
- **Anti-Responsibilities**: Does not own physical database relational tables or foreign key constraints (Domain 04/07).

EC-02: Context Management

- **Architectural Scope**: Establish, validate, propagate, and preserve the execution, clinical, security, and administrative context within which information and activities have meaning.
- **Functional Responsibilities**:
  - Binding the subject of care, encounter, ordering provider, and healthcare service context to in-flight messages and transactions.
  - Propagating tamper-evident, attributable security context across asynchronous, multi-hop operational boundaries.
  - Ensuring context preservation during cross-protocol transformations across disparate healthcare exchange formats and protocols.
  - Detecting and rejecting requests where mandatory operational or clinical context is missing or inconsistent.
- **Collaborative Nature**: Bounded reusable context responsibility may be realised through participation across gateways, activity execution and information-access boundaries. Participation does not transfer semantic ownership.

EC-03: Managed State & Lifecycle

- **Architectural Scope**: Govern the current and historical state, effective temporal periods, state transition validations, and lifecycle progression of managed integration artifacts and entities.
- **Functional Responsibilities**:
  - Defining and enforcing valid state transition state machines (e.g., `REQUESTED` → `VALIDATED` → `IN_PROGRESS` → `COMPLETED`).
  - Maintaining temporal validity intervals (effective start and end dates) for relationships, credentials, and states.
  - Supporting non-destructive state progression, preserving historical state versions for longitudinal review.
  - Rejecting illegal or out-of-order state transitions while accommodating idempotent replay.
- **Anti-Responsibilities**: Does not define thread-level mutexes or database transaction isolation levels.

EC-04: Information Management

- **Architectural Scope**: Validate, represent, persist, retrieve, version, and manage governed clinical, administrative, and operational information assets.
- **Functional Responsibilities**:
  - Validating inbound information structures against canonical information constraints.
  - Providing authoritative durable persistence for historical health records and administrative registries.
  - Managing active distributed working memory representations for low-latency retrieval.
  - Governing document versioning, supersession, amendments, and retractions without data loss.
- **Architectural Principle**: Enforces the foundational separation between *active distributed state access* and *authoritative durable state preservation* (Axiom AX-05).

EC-05: Search & Discovery

- **Architectural Scope**: Index, search, resolve, filter, and discover information assets, entities, directory endpoints, and healthcare services across the enterprise.
- **Functional Responsibilities**:
  - Providing multi-criteria indexed search across complex entity graphs (e.g., find active cardiologists at Location X accepting new referrals).

  - Resolving service and practitioner communication endpoints for addressed delivery.
  - Supporting bounded, paginated clinical queries across longitudinal patient records.
  - Applying dynamic security and consent filters to search results prior to egress.
- **Anti-Responsibilities**: Does not dictate specific search engine indexes, inverted index implementations, or SQL query syntaxes.

EC-06: Policy & Control

- **Architectural Scope**: Evaluate and enforce security, consent, privacy, and regulatory policies including authority, access control, and information handling rules.
- **Functional Responsibilities**:
  - Executing impartial, default-deny policy evaluations for all integration transactions.
  - Enforcing Role-Based and Attribute-Based Access Control (RBAC/ABAC) policies.
  - Evaluating patient consent directives and privacy restrictions at boundary and query checkpoints.
  - Enforcing statutory information disclosure boundaries (e.g., mental health, VIP records).
- **Collaborative Nature**: Evaluated across boundary gateways, internal bus dispatch, storage interfaces, and presentation layers.

EC-07: Provenance & Traceability

- **Architectural Scope**: Preserve required origin attribution, meaningful transformation history, significant execution checkpoints, tamper-evident audit evidence and end-to-end traceability. Harmonia SHALL preserve provenance, audit and operational evidence where required by governing obligations, architectural significance, accountability, reconstruction, security or assurance needs. Routine technical activity SHALL NOT automatically require durable evidentiary recording.
- **Functional Responsibilities**:
  - Recording attributable, verifiable origin metadata for information sources, authors, and timestamps.
  - Tracking information-significant transformation history and required mapping checkpoints as information crosses system seams.
  - Capturing non-PHI operational and security audit trails for regulatory compliance (HIPAA, GDPR).
  - Supporting end-to-end transaction tracing across asynchronous processing pipelines.
- **Collaborative Nature**: Participating elements preserve required provenance and audit context according to policy. Telemetry, operational facts, provenance, audit and assurance evidence remain distinct. Being evidence is contextual, not an intrinsic property of every recorded fact; preservation does not establish an assurance conclusion.

EC-08: Interoperability & Exchange

- **Architectural Scope**: Resolve destinations, transform schemas, route, transport, deliver, acknowledge, and coordinate standards-based information exchange across enterprise boundaries.
- **Functional Responsibilities**:
  - Providing multi-protocol boundary adaptation for healthcare standards (HL7 v2, FHIR REST, DICOM, secure messaging).
  - Executing bidirectional structural and semantic transformations between legacy formats and canonical representations.
  - Ensuring reliable addressed delivery and fan-out distribution to external clinical endpoints.
  - Processing and distinguishing technical delivery acknowledgements from business acknowledgements.

- **Boundary Rule**: Egress terminates Harmonia management of the emitted representation (Axiom AX-13).

EC-09: Event & Subscription

- **Architectural Scope**: Publish, ingest, correlate, filter, and syndicate events and subscription-driven changes across the enterprise landscape.
- **Functional Responsibilities**:
  - Ingesting operational and clinical event triggers from external systems and internal state changes.
  - Managing topic- and criteria-based event subscriptions for internal and external consumers.
  - Evaluating dynamic subscription filters to match event payloads with interested subscriber criteria.
  - Delivering asynchronous event notifications reliably to registered subscribers.
- **Decoupling Note**: Defined strictly in terms of event publishing, criteria matching, and notification delivery; entirely independent of broker topics, partitions, or queues.

EC-10: Activity & Execution

- **Architectural Scope**: Define, instantiate, match, assign, dispatch, supervise, and progress coordinated operational and clinical activity units.
- **Functional Responsibilities**:
  - Instantiating operational work items (Work Orders, To Dos, synthetic platform Tasks) in response to triggers.
  - Matching and assigning work units to eligible human or automated performers and roles based on policy rules.
  - Dispatching work units to their assigned performers with acknowledgement according to the applicable interaction contract.
  - Supervising execution progress, tracking timeouts, managing escalations, and handling activity interruptions.
- **Decoupling Note**: Free of worker threads, thread pools, execution daemons, or specific workflow runtime engines.

EC-11: Interaction & Experience

- **Architectural Scope**: Provide contextual interaction between human actors, Harmonia, and enterprise presentation services, including visualization, alerting, and collaboration integration.
- **Functional Responsibilities**:
  - Mediating presentation interactions via dedicated backend-for-frontend gateways decoupled from internal databases.
  - Rendering clinical timelines, provider directory management interfaces, and operational console dashboards.
  - Providing contextual notifications and alerts to human users across web and mobile channels.
  - Projecting longitudinal record context into secure multidisciplinary collaboration spaces.
- **Boundary Note**: Does not absorb the healthcare semantics of Clinical Collaboration or medical documentation. Consumes established identity and security context; does not own authentication, consent, or clinical truth.

EC-12: Operational Assurance

- **Architectural Scope**: Ensure concurrency integrity, system resilience, duplicate detection, exception handling, automated recovery, and operational telemetry across platform operations.
- **Functional Responsibilities**:
  - Enforcing sliding-window duplicate message detection and idempotent suppression.
  - Guaranteeing ingress dual-write safety (durable acceptance before emitting positive application ACK).
  - Managing structured failure recovery, exception escalation, and dead-letter isolation.
  - Emitting platform telemetry, operational health metrics, and queue processing metrics.
- **Collaborative Nature**: Realized collaboratively across boundary gateways, messaging infrastructure, execution workers, and storage layers.

EC-12 / EC-14 Boundary

EC-12 retains the established scope and responsibilities above. It may contribute directly to generic operational/processing assurance, limited to that contribution in the approved assurance matrix. It does not own the general semantics of assurance disposition, assurance-specific criteria applicability, evidentiary association, assessment against assurance criteria, adjudication, or independent assurance findings and conclusions. Those distinguishing responsibilities are introduced through EC-14 Service Guardian. Operational health and business-level processing assurance remain distinct; EC-12 is not expanded to absorb EC-14.

EC-13: Semantic Governance & Conformance

- **Architectural Scope**: Govern canonical models, schemas, terminologies, value sets, mapping tables, constraint rules, and semantic conformance validation across the enterprise.
- **Functional Responsibilities**:
  - Defining and publishing canonical healthcare data models and profile structures.
  - Governing clinical terminology mappings, value sets, and concept lookups (SNOMED CT, LOINC).
  - Validating inbound and outbound message representations against published semantic rules.
  - Governing semantic versioning, deprecation, and backward compatibility across schema revisions.
- **Component Relationship**: Informs the Calliope strategic logical component, but Calliope is the responsibility centre, not the capability definition.

EC-14: Service Guardian

- **Architectural Scope**: Service Guardian provides the reusable capability to independently establish whether governed service behaviour, state or outcomes remain within defined assurance expectations by associating applicable criteria and trustworthy evidence with a governed subject, assessing that evidence, adjudicating the assessment, and establishing reportable assurance findings and conclusions.
- **Functional Responsibilities**:
  - Supplying assurance-specific semantics for assurance disposition and assurance criteria applicability.
  - Associating a governed subject/context with applicable criteria and sufficient trustworthy evidence for a particular assurance purpose.
  - Assessing evidence against applicable criteria and adjudicating assessed detail to establish reportable findings, exceptions and conclusions.
  - Preserving assurance independence, temporal/version-specific evidence and criteria applicability, and explicit inability to establish a conclusion where evidence is insufficient.

- **Collaborative Nature**: Supplies the assurance-specific semantics required to bind existing reusable capabilities into governed assurance. It contributes directly to Assurance Design, Assurance Criteria Management and Governed Assurance; direct contribution is not sole realisation. The approved matrix preserves the contributions of EC-01 through EC-13.
- **Anti-Responsibilities**: Does not own the underlying governed subject, authoritative governance requirements, or source information used as evidence. Does not perform, manage, remediate, assign, delegate or operationally escalate the subject activity. Does not replace System Operations or EC-12 Operational Assurance, and does not provide Clinical Services Delivery Assurance.
- **Allocation Boundary**: No strategic logical component, application component, runtime service or implementation construct is allocated EC-14. The catalogue defines a capability, not a software component or deployment boundary.

Conceptual Behavioural Sequence

RAD source listing (presentation only; no inferred ancestry).

Define Good Behaviour

↓

Gather Evidence

↓

Assess Evidence against Good Behaviour

↓

Adjudicate Assessed Detail

↓

Report

This sequence explains the capability; it is not an implementation workflow specification or a derived Business Process. “Define Good Behaviour” identifies assurance expectations and criteria; it does not transfer ownership of governing requirements to Service Guardian. Source information ownership remains with its responsible capability; assurance owns its contextual evidentiary association.

The Business Enabling assurance principles govern disposition, generic processing assurance, evidence/criteria temporality, assessment/adjudication, independence, failure/recovery and explicit evidence insufficiency. These semantics are not universally reducible to PASS / FAIL / UNKNOWN and do not imply an assurance information model or persistence mechanism.

Summary Matrix: Enterprise Capabilities

ID	Enterprise Capability Name	Architectural Focus & Essence	Key Recurring Functions
EC-01	Managed Entity & Relationship	Entity identification, graph relationships, referential integrity	Identity resolution, relationship navigation, alias linking
EC-02	Context Management	Establishing, validating, and propagating operational context	Context binding, multi-hop propagation, security tokens
EC-03	Managed State & Lifecycle	State machines, temporal validity, lifecycle progression	State validation, effective periods, non-destructive updates

ID	Enterprise Capability Name	Architectural Focus & Essence	Key Recurring Functions
EC-04	Information Management	Validation, durable persistence, active distributed state	Schema validation, durable preservation and recovery, active access
EC-05	Search & Discovery	Indexing, multi-criteria search, endpoint resolution	Indexed query, criteria filtering, endpoint discovery
EC-06	Policy & Control	Impartial default-deny security, access control, consent	RBAC/ABAC evaluation, consent enforcement, VIP gating
EC-07	Provenance & Traceability	Origin attribution, audit evidence, transformation history	Tamper-evident audit logging, source attribution, chain of custody
EC-08	Interoperability & Exchange	Multi-protocol boundary exchange, transport, delivery	Protocol mediation, transformation, routing, delivery ACK
EC-09	Event & Subscription	Event publishing, criteria filtering, notification dispatch	Subscription management, criteria matching, syndication
EC-10	Activity & Execution	Work unit instantiation, assignment, dispatch, supervision	Matching, dispatching, progress tracking, escalation
EC-11	Interaction & Experience	Decoupled presentation, user interaction, dashboards	BEFE mediation, clinical view rendering, alert display
EC-12	Operational Assurance	Resilience, concurrency integrity, duplicate suppression	Dual-write safety, deduplication, recovery, telemetry
EC-13	Semantic Governance & Conformance	Canonical schemas, terminologies, conformance rules	Schema publishing, concept mapping, constraint checking
EC-14	Service Guardian	Independent governed assurance of service behaviour, state and outcomes	Assurance disposition, criteria applicability, evidentiary association, assessment, adjudication, findings and conclusions

Multi-Capability Composition Dynamics

Enterprise Capabilities do not operate in isolation; they compose harmoniously to deliver complex healthcare features. Below are three representative multi-capability compositions demonstrating this derivation logic:

Composition 1: Person Identifier Resolution

- **Healthcare Need**: Resolving multi-authority identifiers (MRN, national IHI, clinic client IDs) for an incoming patient transaction.
- **Composed Enterprise Capabilities**:
  - `EC-01ManagedEntity&Relationship`: Resolves entity models and alias relationships.
  - `EC-05Search&Discovery`: Searches existing identifier registers and resolves matches.
  - `EC-03ManagedState&Lifecycle`: Validates identifier active/superseded status.
  - `EC-07Provenance&Traceability`: Records origin authority, timestamp, and audit trail.
  - `EC-06Policy&Control`: Evaluates requesting system access permissions.
  - `EC-12OperationalAssurance`: Guarantees deterministic matching under concurrent load.

Composition 2: Closed-Loop Order Progression

- **Healthcare Need**: Ingesting a diagnostic order, routing to a laboratory, and tracking execution through to report outcome.
- **Composed Enterprise Capabilities**:
  - `EC-02ContextManagement`: Binds clinical encounter, patient, and ordering provider context.
  - `EC-05Search&Discovery`: Resolves target laboratory endpoint and service capability.
  - `EC-01ManagedEntity&Relationship`: Links order entity to patient, provider, and service.
  - `EC-08Interoperability&Exchange`: Manages boundary protocol delivery and captures technical ACK.
  - `EC-03ManagedState&Lifecycle`: Enforces order state progression (`PLACED` → `ACCEPTED` → `IN_PROGRESS` → `COMPLETED`).
  - `EC-10Activity&Execution`: Instantiates activity units, monitors timeouts, and supervises fulfillment.
  - `EC-07Provenance&Traceability`: Captures end-to-end chain of custody and delivery timestamps.
  - `EC-12OperationalAssurance`: Enforces dual-write safety and duplicate suppression.

Composition 3: Work Allocation & Dispatch

- **Healthcare Need**: Coordinating operational facility tasks (e.g., patient transport, ward bed cleaning).
- **Composed Enterprise Capabilities**:
  - `EC-02ContextManagement`: Binds patient, physical room/bed, and service context.
  - `EC-03ManagedState&Lifecycle`: Progresses work order state (`CREATED` → `DISPATCHED` → `IN_PROGRESS` → `DONE`).
  - `EC-05Search&Discovery`: Discovers available and credentialed staff matching task criteria.
  - `EC-01ManagedEntity&Relationship`: Links assigned worker, patient, location, and equipment.
  - `EC-10Activity&Execution`: Executes allocation logic, worker dispatch, and escalation oversight.
  - `EC-08Interoperability&Exchange`: Delivers work orders to mobile devices or external systems.
  - `EC-12OperationalAssurance`: Prevents duplicate assignment and ensures resilient retry.
  - `EC-07Provenance&Traceability`: Captures worker acceptance and task completion evidence.

Downstream Progression

Enterprise Capabilities represent what reusable functions the platform delivers. The existing Strategic Logical Component Responsibility Model describes the established responsibility compositions, and the Strategic Resources catalogue records resource adjudications. Downstream progression includes:

- **Domains 05–07 (Application, Integration, Technology)**: Implements these capabilities through concrete software packages, integration protocols, and runtime technologies.
- ICT Foundation Lenses: Cross-cutting technical enablement considerations guiding technology realization.
- Capability Tier Progression Model: Detailed vertical derivation rules.

The established component derivations and examples for EC-01 through EC-13 remain unchanged. EC-14 and the three assurance Business Enabling Capabilities stop at the approved Strategy derivation boundary: no allocation to Dokimasia, Ponos, Praxis, Pragma, Digital Twin, Mneme, Mnemosyne, Calliope, Iris, Pylai or any other logical/application construct is established. No downstream Business, Information or solution architecture is derived here.

23. Domain 03 to Domain 04 Traceability Framework

RAD source: `docs/markdown/04-information-architecture/traceability/domain03-traceability.md`

The following is a presentation transcription of the governed source. Source catalogue layout and retained illustrations do not allocate tiers or establish additional identities. Equations wrap as text; diagram notation is textual.

Domain 03 to Domain 04 Traceability Framework

1. The Four-Stage Derivation Framework

The Information Architecture (Domain 04) is derived systematically from the business semantics established in the Business Architecture (Domain 03).

Information Concepts, Relationships and Assemblies trace through established business responsibility using the following four-stage derivation framework. It does not allocate Capability Tiers or require invented hierarchy, Feature associations or dependencies:

RAD source listing (presentation only; no inferred ancestry).

1. Business Capability / Feature (Domain 02 / Domain 03)

↓ delivers

2. Business Function / Process (Domain 03)

↓ establishes

3. Business Information Responsibility (Domain 03 Matrix)

↓ realised as

4. Domain 04 Information Concepts, Relationships & Assemblies

RAD diagram: textual source notation.

```
graph TD
    CAP["1. Owning Business Capability / Feature<br/>(Domain 02 / Domain 03)"]
    FUNC["2. Business Function / Stateful Process<br/>(Domain 03 Behaviour & Processes)"]
    RESP["3. Business Information Responsibility<br/>(Domain 03 Ownership Matrix)"]
    D04["4. Domain 04 Information Concepts, Relationships & Assemblies<br/>(Entities, Activities, Events,
    ↪  Assertions, Definitions, Assemblies)"]

    CAP -->|"delivers"| FUNC
    FUNC -->|"establishes & governs"| RESP
    RESP -->|"formally realised as"| D04
```

1.1 Traceability Principles

1. **Derivation, Not Redefinition**: Domain 04 does not invent new business capabilities or alter the ownership boundaries established in Domain 03.
2. **Purity of Information Responsibilities**: The Business Information Responsibility matrix (`docs/markdown/03-business-architecture/information-responsibility/information-responsibility.md`) is the authoritative source for architectural information responsibilities. Architectural information responsibility is a capability property and is distinct from the governance role of `InformationSteward`.
3. **Bounded Responsibility Derivation**: Information Concepts for which Harmonia claims architectural responsibility SHALL trace to an owning Domain 03 Information Responsibility. Referenced, consumed, externally authoritative or contextual Information Concepts may be

represented where required to discharge a traced Harmonia responsibility, but SHALL NOT thereby acquire Harmonia ownership or authority.

4. **Truthful Traceability**: Under AX-17, traceability represents established relationships and explicitly preserves unestablished ones. Unresolved Capability Tier, complete ancestry or full Canonical ID does not prevent derivation from an established owning element and responsibility. Candidates remain non-authoritative until accepted.

2. Representative Derivation Examples

The following representative examples demonstrate how Domain 03 capabilities, functions, and information responsibilities realise concrete Domain 04 Information Concepts, Relationships, and Assemblies:

2.1 Representative Example 1: Health Service Administration (Mandatory Reference)

RAD source listing (presentation only; no inferred ancestry).

Health Service Administration (Capability / Feature: Service-Location-Provider Binding)

↓ delivers

Maintain Service / Location / Provider Map (Function)

↓ establishes

Service / Location / Provider Mapping Matrix (Information Responsibility)

↓ realised as

OfferedHealthcareService (Definition)

DeliverableHealthcareService (Contextual Binding)

Service-Provider-Location Relationships & Qualified Containment

↓ references participating entities; does not own their meaning

Service Provider / Healthcare Organisation (Entity; responsibility retained by its established owner)

Healthcare Location (Entity; Location Administration responsibility retained)

RAD diagram: textual source notation.

```
graph TD
    subgraph D03 ["Domain 03: Business Architecture"]
        HSA["Capability: Health Service Administration"]
        FUNC_MAP["Function: Maintain Service / Location / Provider Map"]
        RESP_MAP["Information Responsibility: Service / Location / Provider Mapping Matrix"]
        HSA --> FUNC_MAP --> RESP_MAP
    end

    subgraph D04 ["Domain 04: Information Architecture"]
        OFFERED["OfferedHealthcareService (Definition)"]
        DELIV["DeliverableHealthcareService (Contextual Binding)"]
        ORG["Healthcare Organisation (Entity)"]
        LOC["Healthcare Location (Entity)"]
        REL_MAP["Information Relationships:<br/>• Organisation.delivers(DeliverableHealthcareService)<br/>•
        ↪  DeliverableHealthcareService.locatedAt(HealthcareLocation)<br/>•
        ↪  OfferedHealthcareService.contains(OfferedHealthcareService)"]
    end

    RESP_MAP -.->|"realised as"| OFFERED
    RESP_MAP -.->|"realised as"| DELIV
    RESP_MAP -.->|"references mapped entity; responsibility retained"| ORG
    RESP_MAP -.->|"references mapped entity; responsibility retained"| LOC
    RESP_MAP -.->|"realised as"| REL_MAP
```

Under the Business ownership matrix, Health Service Administration owns the mapping associations; it does not own the mapped Provider, Location or Practitioner entities. The example references their participation without transferring their established information responsibility or originating authority. This clarification adds no Business relationship or replacement service-binding model.

2.2 Representative Example 2: Person Identity Administration

RAD source listing (presentation only; no inferred ancestry).

Person Identity (Capability / Feature: Cross-Authority Identifier Correlation)

↓ delivers

Correlate Person Identifiers (Function)

↓ establishes

Person Identity Record, Identifier Namespace Bindings, Cross-Authority Correlation Graph (Information Responsibility)

↓ realised as

Person (Entity)

Identifier Namespace Binding (Assertion)

Cross-Authority Correlation Link (Relationship with Qualification & Authority)

2.3 Representative Example 3: Order Administration

RAD source listing (presentation only; no inferred ancestry).

Order Administration (Capability)

↓ established Features

FEAT-SA-11 — Order Request Ingestion

FEAT-SA-12 — Order Destination Resolution & Routing

FEAT-SA-13 — Order Closed-Loop Progression Tracking

FEAT-SA-14 — Order Cancellation & Modification Coordination

↓ established Functions / exposed Services

Receive Order Request / Order Requisition Ingress

Resolve Order Destination / Order Dispatch Service

Manage Order Progression / Order Status & Tracking Query

Associate Order Outcome / Order Outcome Notification (Capability-scoped; Feature association not established)

↓ governed Process

Closed-Loop Order Progression Process

↓ establishes

Clinical Order & Closed-Loop Matrix (Information Responsibility)

Supporting: Clinical Order Master Record, Closed-Loop Tracking Ledger, Order-Result Correlation Matrix

↓ realised as

Order (Associated Direction Mechanism)

Order Routing Directive (Assertion / Instruction)

Order-Result Correlation Link (Relationship with Provenance & Status)

This retained Foundation example supplies canonical terminology, not Package2 derivation. A ledger/history label does not establish comprehensive retention; correlation does not establish conflict adjudication. Each Function/Service remains in its evidenced owning context without inferring a one-to-one Feature association.

2.4 Bounded Task / Work Responsibility Basis

The Task / Work model traces to Workflow & Activity Coordination's established Functions (`CoordinateWorkOrder`, `CoordinateToDo`, `CoordinateSyntheticTask`, `SuperviseActivityTimeout/Escalation`) and generic Activity Instance Registry, Task State Progression Graph and SLA Deadline & Timer Ledger information responsibilities. Work Order, To Do and Synthetic Task retain their distinct Business meanings and progression authority.

The authorised Task information concepts distinguish archetype, particular work, undertaking and governed ActionableTask resolution without assuming ownership of business meaning or clinical outcome. This responsibility basis allocates no unestablished definition-authoring/outcome-authority owner or specialised archetype/undertaking/outcome subclasses. ActionableTask governs its orthogonal execution-concurrency, outcome-participation and output-rule controls; that information policy does not transfer originating domain authority. ReportedTask and remaining detailed TaskOutcome questions remain unresolved.

3. Scope Boundary for Detailed Model Population

This foundation establishes the canonical derivation mechanism and validates it through representative references.

> **Scope Note**: Full, detailed derivation across the 137 established Strategy Features and their evidenced owning contexts and the complete Domain 03 Business Information Responsibility matrix remains reserved for subsequent, dedicated Domain 04 modelling packages.

24. Cross-Capability Dependencies & Service Exposure Matrix

RAD source: `docs/markdown/03-business-architecture/dependencies/cross-capability-dependencies.md`

The following is a presentation transcription of the governed source. Source catalogue layout and retained illustrations do not allocate tiers or establish additional identities. Equations wrap as text; diagram notation is textual.

Cross-Capability Dependencies & Service Exposure Matrix

1. Architectural Principles of Cross-Capability Dependencies

In the Harmonia Business Architecture, capabilities do not operate as isolated silos. Healthcare workflows require coordinated collaboration across bounded capabilities.

1.1 Service Exposure and Ownership Boundary Pattern

When Capability A delivers behaviour needed by Capability B, Capability A exposes that behaviour through a formal **Business Service**. Capability B consumes the Service without acquiring ownership of the underlying Function or information:

RAD diagram: textual source notation.

```
graph LR
    subgraph CapA["Capability A (Owner)"]
        FA["Function X"] -->|Exposed as| SA["Business Service X"]
    end
    subgraph CapB["Capability B (Consumer)"]
        CB["Business Behaviour"] -->|Consumes| SA
    end
```

1.2 High-Level Cross-Capability Dependency Topology

The diagram below illustrates the major inter-capability dependency relationships across the five healthcare contextual views:

These nodes are contextual views, not typed Capabilities or identifier ancestors. The arrows summarise established responsibility needs; they do not allocate particular Services, participating Interactions or Collaborations.

RAD diagram: textual source notation.

```
graph TD
    EM["Entity Management<br/>(Identity, Providers, Orgs, Locations, Services)"]
    SA["Service Administration<br/>(Encounters, Referrals, Orders, Documents)"]
    SD["Service Delivery<br/>(Primary, Acute, Emergency, Inpatient, Virtual)"]
    HSO["Health Service Operations<br/>(Wards, Beds, Dispatch, Logistics, Discharge)"]
    ISE["Intrinsic / Shared Enablement<br/>(LHR, HIE, Access, Control, Workflow, Collab)"]

    SA -->|Resolves entities & services| EM
    SD -->|Associates encounters & orders| SA
    SD -->|Retrieves longitudinal record & exchange| ISE
    HSO -->|Coordinates care-places & staff| EM
    HSO -->|Orchestrates operational workflows| ISE
    HSO -->|Synchronises discharge & admissions| SA
    SA -->|Enforces security context & audit| ISE
```

2. Cross-Capability Service Consumption Matrix

The matrix below documents the primary inter-capability service dependencies across Harmonia:

Consuming Capability Context	Exposed Business Service Consumed	Exposing / Owning Capability	Business Purpose
Referral Administration	`PersonIdentifierResolution`	**Person Identity** *(Client Administration)*	Validates source-attributed identifiers against their authoritative issuing information; does not determine a master person.
Referral Administration	`ServiceProvisionResolution`	**Health Service Administration**	Resolves target clinical specialty clinics, receiving providers, and referral catchment rules.
Episode & Encounter Administration	`Care-PlaceSpecificationLookup`	**Location Administration**	Resolves physical ward, room, and bed structural attributes for encounter bed placement.
Episode & Encounter Administration	`BedAvailabilityQuery`	**Bed & Care-Place Management**	Checks real-time operational bed readiness before confirming patient bed moves.
Order Administration	`ServiceProvisionResolution`	**Health Service Administration**	Determines performing diagnostic laboratory or imaging centre routing destinations.
Diagnostic Administration	`TerminologyMappingService`	**Clinical Knowledge Services**	Normalises local laboratory and radiology test codes to canonical SNOMED-CT/ LOINC concepts.
Clinical Record Administration	`HealthcareSubjectContextResolution`	**Healthcare Subject**	Validates patient demographic context and indigenous/ interpreter requirements for document headers.
Clinical Record Administration	`SemanticConformanceVerificationService`	**Information Design Governance**	Verifies clinical document structural and semantic compliance against governed interchange profiles.
Primary Care	`LongitudinalClinicalRecordQuery`	**Patient Clinical Record**	Retrieves regional patient history, past discharge summaries, and medication timelines for GP consultations.
Acute Care	`ConsentEnforcementService`	**Health Information Control**	Evaluates patient consent directives and break-glass emergency access permissions during acute admissions.

Consuming Capability Context	Exposed Business Service Consumed	Exposing / Owning Capability	Business Purpose
Emergency Care	`PersonIdentifierCorrelation`	**Person Identity** *(Client Administration)*	Resolves established, source-authoritative identity associations for emergency presentations; does not originate probabilistic matching or person-merge decisions.
Inpatient Care	`DischargeReadinessTelemetry`	**Discharge Management**	Coordinates multidisciplinary ward discharge checklists and pharmacy reconciliation.
Bed & Care-Place Management	`Care-PlaceSpecificationLookup`	**Location Administration**	Resolves physical care-place capabilities (e.g., negative pressure, telemetry wiring) for isolation placement.
Work Allocation & Dispatch	`StaffPresenceTelemetryQuery`	**Mobile Staff Management**	Allocates portering and cleaning work orders to active on-duty staff in the nearest physical zone.
Patient Transport	`EncounterLocationHistoryQuery`	**Episode & Encounter Administration**	Validates current patient ward location and target diagnostic department before dispatching porters.
Clinical Logistics Coordination	`SecureClinicalMessageDispatch`	**Clinical Communication Administration**	Sends automated electronic specimen tracking notifications and urgent critical result dispatch alerts.
Workflow & Activity Coordination	`PolicyEvaluation&AuthorisationService`	**Health Information Control**	Evaluates actor task-claiming permissions before allowing work order acceptance or document countersignature.
Presentation Services	`ClinicalInformationSearchService`	**Health Information Access**	Executes search queries across longitudinal clinical records for display in clinical portals.

3. Dependency Governance Rules

The former Clinical Collaboration consumption of `CollaborationClinicalSummaryResolution` from Patient Clinical Record is withdrawn as unsupported under approved G1 K12. Clinical Collaboration retains ownership of that exposed Service through `FEAT-ISE-17` and `ResolveLHRQuerywithinCollaboration`. Its precise source Service/provider, direct or mediated retrieval, formal Service consumer and the incorrect row's intended referent remain unresolved. No replacement dependency is established.

Patient Clinical Record retains longitudinal assembly, active record maintenance, durable preservation and canonical integrated synthesis; Clinical Collaboration retains spaces, membership, discourse, in-conversation resolution and contextual clinical-summary projection;

Health Information Access retains search, retrieval, access-qualified filtering, query execution context and result projection state. Information ownership, Function ownership, Service ownership, consumption, use and presentation remain distinct.

The Order Administration → Practitioner Role Resolution row is withdrawn: the Service resolves Role/capacity information, not Clinical Privilege or Operational Privilege. The Diagnostic Administration → Order Requisition Ingress row is withdrawn: that Service ingests requisitions, not the asserted correlation of incoming reports. Both Services retain their owners and valid exposed behaviour. Required privilege/correlation behaviour remains valid, but precise consuming Service relationships remain unestablished; no replacement edge is inferred.

Clinical Privilege concerns a practitioner's authority/credentials for applicable clinical activity; Operational Privilege concerns an actor's authority for applicable operational activity. Either or both may be required. Practitioner Role information may contribute context but establishes neither.

The former Discharge Management → Clinical Document Lifecycle Service "publication upon departure" assertion is withdrawn. That Service exposes authoring addenda/corrections; the alleged discharge-publication consumption is not established. Discharge Management retains a capability dependency on Clinical Record Administration for applicable discharge information, with the precise Service/provider contract unresolved. Preparation, information availability/publication, discharge authorisation and physical departure remain distinct. Applicable information may be prepared/communicated before departure; `FEAT-HS0-29` dispatch preserves confirmed physical exit as its trigger. No universal publication sequence, additional publication event or replacement Service edge is inferred.

Typed references distinguish Capability dependency, Service consumption, Interaction participation and Collaboration involvement. Abbreviated/descriptive wording is not an alias. Where identity or relationship is not established, retain the reference as unresolved rather than convert it into an edge.

1. **Unidirectional Service Coupling**: Dependency arrows point strictly from the consuming capability to the exposed Business Service. The exposing capability remains completely agnostic of which downstream capabilities consume its services.
2. **Encapsulated Implementation**: Consuming capabilities depend exclusively upon the abstract service contract and its business semantics, never upon internal algorithms or persistence structures.
3. **Fail-Closed Security Context**: All cross-capability service invocations must carry a valid, immutable `SecurityContext` evaluated by *Health Information Control*.

4. Assurance Dependency Derivation Boundary

The approved Strategy contribution matrix establishes contributions to Assurance Design, Assurance Criteria Management and Governed Assurance. It is not a Business Service consumption matrix: neither Direct nor Supporting identifies an exposed Service, its owning Function or an identifiable Business Service consumer. No assurance row or topology edge is added to §2 or §1.2 from those contributions.

Governed Assurance needs applicable criteria and trustworthy evidence. Source-information ownership and subject management remain with their established capabilities. The subject may supply evidence, but that dependency must not give it control of assurance progression or conclusion: **evidence dependency does not compromise assurance independence; control dependency does**. Service Guardian does not acquire assignment, delegation, escalation or remediation responsibility through a finding. Service Assurance Modeller also does not manage the subject merely by designing assurance concerning it.

The approved EC-02 / EC-14 contribution relationship supports **Establish Assurance Context** through generic context-management capability and assurance-specific semantics together. It does not identify an exposed Business Service, a Service consumer, sole realisation, a technical mechanism or an implementation allocation. The five approved Functions establish internal capability-owned behaviour; they do not mechanically create exposed Services or consumption rows.

The approved human review separately establishes the following three Governed Assurance Services and their Role-level consumption/communication relationships. These are approved relationships across the owning responsibility boundary, not inferred from EC contributions or Role-name similarity. No consuming Capability is allocated solely from a Role's name; these assurance relationships do not determine §2's other Capability rows or §1.2's contextual-view topology.

Approved Business Service	Exposing / Owning Capability	Approved consumer / potential recipient Roles	Associated Business Interaction
Request Service Assurance	**Governed Assurance**	Care Coordinator, Service Coordinator, Regulator, Policy Authority, System Steward — authorised requesters	Service Assurance Request
Request Service Assurance Status	**Governed Assurance**	**System Steward only** — stewardship of activity progression	Service Assurance Status
Communicate Service Assurance Outcome	**Governed Assurance**	Care Coordinator, Service Coordinator, Regulator, Policy Authority, System Steward — applicable authorised recipients	Service Assurance Outcome Communication

Requesting does not grant control of assurance assessment/adjudication or authority to alter the applicable Assurance Definition/Criteria. Status concerns assurance activity progression and does not expose unadjudicated findings/conclusions or provide general requester tracking. Outcome communication is not mandatory broadcast; requester, status consumer and recipient need not coincide. Recipients retain their own clinical, operational, governance/compliance and improvement responsibilities; communication transfers none of their subsequent action to Service Guardian.

Assurance Definition use is a responsibility/information dependency between modelling and execution; no Modeller-to-Guardian Interaction or Service is inferred. Source information may be available through existing governed access; evidence association does not create an Evidence Contribution Interaction. Exact evidence/criteria providers, additional exposed behaviour/Services, detailed contracts, consuming Capability allocation, evidence exchange and findings-to-operational-response mechanisms remain unresolved.

Financial Governance is deliberately excluded, while exact authorised external recipients of financially relevant outcomes remain unresolved. Governed Assurance does not recursively assure its own execution; System Steward's status observation is stewardship / management monitoring. Neither boundary establishes an implementation allocation. Existing Service ownership, consumption relationships, security rules and recorded uncertainties remain intact. Role participation alone does not grant information-access authority or bypass governed access.

25. Domain04 Package 2 — G1 Architectural Review Record

RAD source: `docs/markdown/04-information-architecture/reviews/package2-g1-review.md`

The following is a presentation transcription of the governed source. Source catalogue layout and retained illustrations do not allocate tiers or establish additional identities. Equations wrap as text; diagram notation is textual.

11. K9 — Capability Metamodel Typing

Status: RESOLVED — APPROVED: CAPABILITY IDENTITIES AND RESPONSIBILITIES PRESERVED; CAPABILITY TIER / ANCESTRY UNRESOLVED

Disposition: Close K9 because the architectural investigation establishes what the current architecture does and does not determine. Preserve established architectural identities, responsibilities, behavioural ownership contexts and catalogued Feature classifications. Retain unresolved Capability Tiers, ancestry, root status and structural Canonical IDs explicitly. Completion of the full Business Enabling Capability hierarchy is not a prerequisite for Domain04 Package 2 derivation. Source reconciliation remains a later, separately authorised activity.

The approved identification prerequisite in §10, the Capability decomposition / derivation model and the Architectural Element Identification metamodel remain controlling. This final disposition supersedes the prerequisite's incomplete-K9 position without changing its identification rules or retrospectively validating legacy classifications. Approved K1–K8, including K5, remain unchanged.

11.1 Approved findings and explicit uncertainty

Architectural dimension	Established finding	Unresolved boundary
Architectural identity	Business Enabling Capability identities are established for the majority of affected elements.	The architectural-element status of every named catalogue decomposition entry is not established.
Semantic responsibility	The relevant responsibilities of those established Capabilities are preserved. Named responsibility contexts for Functions, Services and Processes are substantially established.	Established responsibility does not prove numerical Capability Tier or complete structural ancestry.
Feature type	All explicitly catalogued Features are FT; existing Feature identifiers are preserved. `IdentifierResolution` is FT / `FEAT-EM-01`.	Unsupported Domain03 Feature headings remain governed by approved K5; presentation alone creates no Feature identity.
Capability Tier	CT1 / CT2 / CT3 are the approved decomposition terminology.	No affected legacy L1 / L2 / L3 owner assignment is independently validated sufficiently to establish a CT1 / CT2 / CT3 assignment.
Capability ancestry and root status	Selected explicit parent/ responsibility relationships are established, including `ClientAdministration→PersonIdentity`.	Complete Capability ancestry and root status for the affected Business Enabling Capabilities remain unresolved. The explicit relationship does not establish either element's tier or Client Administration's root status.

Architectural dimension	Established finding	Unresolved boundary
Structural Canonical ID	The approved identification grammar and established Feature identifiers remain valid.	Complete structural Canonical IDs cannot yet be established where required tier, ancestry or identifier allocation is unresolved.

The Business Enabling Capability catalogue, Domain03 behaviour ownership, Process ownership, Information Responsibility and cross-capability dependencies establish the relevant identities and responsibility contexts within their evidenced boundaries. The explicit `PersonIdentity` parent relationship is stated in Governed Person Identity Correction, §2.1; its disputed L2 label does not establish the tier.

The historical K9 evidence baseline remains evidence of the source assertions requiring reconciliation. Established owner identity, established responsibility and established Feature type SHALL remain distinct from unresolved Capability Tier, ancestry and structural identification. A typing defect SHALL NOT invalidate an otherwise established Capability identity or change its semantic responsibility.

11.2 Legacy classification disposition

No affected legacy L1 / L2 / L3 assignment has been independently validated sufficiently to authorise mechanical translation to CT1 / CT2 / CT3. The investigation categories remain:

Existing assertion	Investigation category	Approved treatment
Affected legacy owner L1 / L2 assignments	C — unsupported / insufficiently evidenced	Preserve the established owning element and responsibility; retain its Capability Tier and complete ancestry as unresolved.
Former `IdentifierResolution` L3 example	D — established Feature incorrectly represented as a Capability Tier	Preserve FT / `FEAT-EM-01`; do not classify it as CT3 or use the former label to establish ancestry.
Former `Closed-LoopOrderProgression` L3 and `DiagnosticOrders` L2 examples	C — unsupported / insufficiently evidenced	The former examples establish no validated Capability Tier or ancestry. Preserve independently evidenced Process responsibility without manufacturing a Capability identity or parent.
Unsupported Domain03 Feature headings	Approved K5 controls	Preserve valid Functions and Services in their evidenced owning context; create no Feature, Feature ID or alias to obtain traceability symmetry.

No affected legacy assignment is established as category A — a semantically validated hierarchy requiring terminology migration alone. Unsupported hierarchy assertions SHALL NOT be treated as mere deprecated wording. `L1→CT1`, `L2→CT2` and `L3→CT3` SHALL NOT be performed as a mechanical migration.

11.3 Downstream derivation rule

> Unresolved Capability Tier or ancestry SHALL NOT prevent downstream architectural derivation where the owning Capability or Feature and its relevant responsibility are otherwise established. Downstream traceability SHALL reference the established architectural element and SHALL NOT manufacture unresolved structural ancestry.

Domain04 Package 2 may therefore derive Information Concepts from established Business Architecture responsibility without waiting for complete CT1 / CT2 / CT3 classification. This permission does not establish missing responsibility or architectural identity where those are themselves unresolved.

Where a full structural Canonical ID cannot yet be established, traceability SHALL use the established architectural type, canonical name and existing identifier where one exists, while

retaining the structural Canonical ID as unresolved. Placeholder structural IDs SHALL NOT be manufactured. An existing Feature identifier SHALL NOT be represented as proof of a complete structural Canonical ID.

11.4 Architectural Element identity and catalogue presentation

> Document structure, catalogue grouping, numbering, indentation and decomposition presentation SHALL NOT by themselves establish Architectural Element identity. Canonical Architectural IDs SHALL be assigned only to governed Architectural Elements whose identity and architectural context are established.

Named catalogue decomposition entries SHALL NOT automatically become CT2 or CT3 Capabilities because they appear beneath another Capability. Their architectural-element status may remain unresolved. Historical L1 / L2 / L3 labels and derivation-stage position likewise SHALL NOT serve as self-validating evidence of ancestry or tier.

The five contextual views — Entity Management, Service Administration, Service Delivery, Health Service Operations and Intrinsic / Shared Enablement — remain organisational views. They SHALL NOT automatically become CT1 elements or identifier ancestors. Established Capability contexts beneath a view do not by themselves prove root CT1 status.

11.5 Behavioural ownership and Feature preservation

Named responsibility contexts for affected Domain03 Functions, Services and Processes are substantially established even though their complete structural paths are not. Those behaviours may continue to trace to their established owning Capability or Feature without a complete structural Canonical ID. The identification prerequisite permits direct anchoring at an established Capability Tier or FT context; missing intermediate Capabilities or Features SHALL NOT be manufactured.

All explicitly catalogued Features SHALL remain FT, and all established Feature identifiers SHALL be preserved. The control case is:

Identifier Resolution property	Approved finding
Type	FT
Existing Feature identifier	`FEAT-EM-01`
Owning context	Person Identity
Complete structural Canonical ID	Unresolved

The explicit `ClientAdministration→PersonIdentity` relationship and Identifier Resolution's owning context establish only a partial responsibility chain. They SHALL NOT supply missing Capability Tier ancestry or a complete structural Canonical ID.

Approved K5 remains controlling for unsupported Feature headings, existing Feature IDs and canonical naming. Valid Functions and Services do not establish a new Feature merely through a heading or similarity to a broader Feature. No FN / SV / PR identifiers are allocated during K9 persistence.

11.6 Clinical Qualification / Credential distinction

`ClinicalQualificationManagement≠CredentialManagement`.

The approved rename `ClinicalCredentialManagement→ClinicalQualificationManagement` preserves professional / clinical qualification and competency responsibility. Domain03 `CredentialManagement` retains its separately evidenced operational badge / physical-access responsibility. The elements SHALL NOT be merged or aliased. Neither element's Capability Tier or ancestry is established by the rename or by lexical similarity.

The lexical similarity of Feature or behaviour names SHALL NOT establish semantic identity. In particular, the badge/access-context `OperationalCredentialCheck` behaviour SHALL NOT be

bound to `FEAT-HS0-23` solely by name. That catalogued Feature's qualification/competency responsibility remains distinct from the operational badge/access behaviour.

11.7 Later, separately authorised G1 reconciliation

The minimum later reconciliation SHALL distinguish:

- **Terminology and unsupported hierarchy assertions:** remove or qualify unsupported numerical Capability Tier assertions in the affected sources; use approved terminology only where tier is independently established. Preserve established Capability identities and responsibilities; otherwise retain tier, root status and complete ancestry as unresolved.
- **Evidenced relationship:** preserve `ClientAdministration→PersonIdentity` without inventing tiers, intermediate parents or root status. Completion of the hierarchy may occur through later Business Architecture refinement and is not required to close K9.
- **Feature correction:** preserve catalogued FT identities and existing Feature IDs; correct the former Identifier Resolution Capability Tier representation; apply approved K5 to unsupported Feature headings without manufacturing Features or identifiers.
- **Canonical naming correction:** apply approved K5 naming corrections, including Service Delivery naming and Clinical Logistics Coordination, and the approved Clinical Qualification Management rename. Preserve the distinct operational Credential Management element and its responsibility.
- **Downstream traceability correction:** reconcile affected Package 1 traceability labels against established architectural types and owning responsibilities, retaining unresolved structural context explicitly. Preserve the separately approved K3/K6 constraints; do not change Function, Service, Process, Information Responsibility or dependency semantics to repair a type label.
- **Historical evidence and identification:** preserve the evidence baseline, including its historical quotations and fingerprints; allocate no speculative structural Canonical IDs. Catalogue presentation SHALL NOT create governed Architectural Elements or substitute for established context.

Approval of this disposition and authorisation to perform source reconciliation remain separate. This persistence task changes only this durable G1 review record; frozen Domain01–03, Domain04 Foundation and Package 1 sources remain unchanged.

11.8 Closure and stop boundary

2026-10-13 reconciliation: The fixed-decomposition decision changes the target metamodel, not K9's approved evidence, closure or unresolved catalogue status. Complete CT1 → CT2 → CT3 ancestry is now required for a fully reconciled Feature. Existing Features remain valid evidence requiring ancestry reconciliation where their CT3 frame is unestablished. Legacy labels, document structure, contextual views, local Feature IDs and partial ownership SHALL NOT establish missing tiers, complete Canonical IDs or new ancestors. K9's permission to derive from established responsibility remains; it does not declare that responsibility structurally reconciled. The original closure text follows unchanged.

K9 is RESOLVED on the approved interpretation above. Unresolved Capability Tier, ancestry, root status, decomposition-entry identity and structural Canonical ID information remain explicit architectural uncertainties; they do not reopen K9 or require completion of the full Business Enabling Capability hierarchy before downstream derivation from established responsibility.

This disposition preserves AX-01/AX-04 business meaning, AX-14 semantic distinctions and AX-15 uncertainty. It neither changes the frozen Capability Model nor authorises missing identity, ancestry or identifier allocation to be invented. It preserves the approved identification prerequisite and K1–K8 unchanged, completes no G1 reconciliation, derives no Package 2 Information Concept and does not investigate K10 or approve G1 as a whole. Work stops after persistence of this approved K9 disposition and review of the resulting diff.

26. Harmonia Architectural Axioms

RAD source: `docs/markdown/governance/architectural-axioms.md`

The following is a presentation transcription of the governed source. Source catalogue layout and retained illustrations do not allocate tiers or establish additional identities. Equations wrap as text; diagram notation is textual.

AX-01 — Harmonia Is Health-Information Centric

Axiom

Harmonia is a distributed health-information management and interoperability framework.

Its primary purpose is the ingestion, validation, transformation, governance, processing, persistence, distribution and exposure of health-related information.

Harmonia SHALL support applicable health-information standards and models while retaining the ability to manage information according to Harmonia's own internal operational requirements.

Rationale

Harmonia is not intended to become a generic distributed application framework to which healthcare happens to be attached.

Its architecture exists to solve health-information problems: interoperability, information authority, provenance, security, terminology, workflow, persistence, distribution, resilience and longitudinal information management.

Consequences

Health-information semantics take precedence over generic infrastructure convenience.

Standards such as HL7 FHIR and HL7 v2 are first-class architectural concerns.

Generic infrastructure capabilities SHOULD be used where useful, but SHALL NOT dictate Harmonia's health-information semantics.

This does not mean

- Harmonia is a FHIR server.
- Harmonia is restricted to FHIR information.
- Every internal Harmonia object must correspond to a healthcare interoperability standard.

AX-04 — Harmonia Owns the Semantics; Engines Provide the Machinery

Axiom

Harmonia owns the architectural semantics governing information authority, security, lifecycle, concurrency, provenance, auditability, resilience and operational integrity.

Harmonia SHALL preferentially use capabilities supplied by underlying technology engines where those capabilities satisfy Harmonia's architectural invariants rather than reproduce equivalent functionality.

Rationale

Harmonia should own its meaning without unnecessarily owning commodity infrastructure machinery.

Consequences

HAPI FHIR MAY provide FHIR persistence, indexing, versioning and search.

Infinispan MAY provide distributed caching, entry metadata and atomic coordination primitives.

ActiveMQ Artemis MAY provide durable messaging.

PostgreSQL MAY provide transactional relational persistence.

These technologies implement Harmonia capabilities. They do not define Harmonia's architectural semantics.

Maxim

Use the machinery. Own the semantics.

This does not mean:

- Harmonia should wrap every engine capability behind a bespoke implementation.
- An engine's native behaviour automatically satisfies a Harmonia invariant.
- Using HAPI FHIR makes Harmonia a HAPI FHIR application.

AX-14 — Semantic Distinctions Are Preserved

Axiom

Harmonia SHALL preserve meaningful distinctions between states, outcomes and concepts throughout its internal processing and across subsystem boundaries.

A distinction that is significant to information meaning, authority, security, concurrency, persistence, processing outcome or operational correctness SHALL NOT be collapsed merely because an underlying technology, transport, API or implementation abstraction does not represent that distinction directly.

Where an external interface contract intentionally presents a simpler or different semantic model, Harmonia MAY project its internal semantics into that contract explicitly at the applicable boundary.

Rationale

Loss of semantic distinction causes information about what Harmonia knows, what occurred and what may safely happen next to be lost.

Two conditions may produce similar implementation behaviour while having materially different meanings. Resource absence, access denial, persistence failure and indeterminate outcome may all prevent information from being returned, but they do not describe the same state.

Likewise, persistence version, active-state generation, interoperability version and information authority describe different properties even where an implementation happens to correlate them.

Implementation convenience SHALL NOT redefine those meanings.

Consequences

Semantically distinct outcomes SHALL remain distinguishable through Harmonia's internal contracts.

Security denial SHALL NOT be represented as resource absence merely because both prevent access to information.

A failure to establish authoritative state SHALL NOT be represented as authoritative absence.

An indeterminate outcome SHALL NOT be represented as either success or failure unless subsequent processing establishes that outcome.

ActiveStateToken, AuthoritativeVersion, externally meaningful version identifiers and standards-defined version metadata SHALL remain distinct where they represent different semantic domains.

Where an underlying engine or protocol lacks a distinction required by Harmonia, Harmonia SHALL introduce an appropriate semantic representation rather than infer meaning from incidental implementation details such as exception classes, message text or correlated version values.

Boundary projection MAY deliberately reduce or transform semantic distinctions where required by an external contract or policy, but such projection SHALL be explicit and SHALL NOT redefine Harmonia's internal semantics.

Maxim

Preserve semantics internally; project deliberately at boundaries.

This does not mean: Harmonia must expose every internal distinction externally. Similar outcomes can never share implementation machinery. Every semantic distinction requires a separate Java type. External standards or interface contracts may be ignored in favour of Harmonia's internal model.

AX-17 — Architectural Authority and Explicit Uncertainty

Axiom

Documented architectural decisions, definitions, relationships, boundaries and constraints are authoritative and SHALL NOT be silently reinterpreted, replaced, bypassed or contradicted by downstream architecture or implementation.

Where the architecture does not establish a fact, relationship, responsibility or decision, that absence SHALL be preserved explicitly rather than completed through inference, convention, structural convenience, lexical similarity or anticipated implementation.

Unresolved architecture MAY be accompanied by clearly identified candidates, suggestions or recommendations, but these SHALL remain explicitly non-authoritative until explicitly accepted into the architecture.

Where evidence indicates that authoritative architecture may be incorrect, incomplete or internally inconsistent, the conflict SHALL be raised explicitly for architectural review. The existing architecture remains authoritative until an approved architectural change is made. That approved change becomes the new authoritative architecture.

Rationale

Downstream derivation must distinguish established architectural knowledge from gaps and proposed resolutions. A plausible relationship supplies no authority merely because it makes a model or implementation appear complete. Absence of an established relationship is architecturally meaningful information.

Repeated architectural review during Domain04 Package2 G1 confirmed this need across different traceability relationships:

- K9: unresolved Capability hierarchy must not be completed through invented ancestry.
- K10: Behaviour / Process traceability must not be completed through invented stage equivalence.
- K11: Function / Service traceability must not be completed through unsupported Feature association.
- K12: cross-capability dependency traceability must not be completed through invented Service ownership or consumption.
- K13: downstream representation traceability must not be completed through anticipated FHIR, application or technology mappings.

These examples justify an architecture-wide rule; its application does not depend on Domain04 or G1 terminology.

Consequences

Human architects, human developers and AI development agents SHALL apply this axiom to architecture derivation, documentation generation and implementation.

Architectural traceability SHALL represent established relationships and explicitly preserve unresolved or unestablished relationships. Completeness SHALL NOT be manufactured through:

- inferred hierarchy;
- structural convenience;
- lexical similarity;
- conventional modelling patterns;
- assumed equivalence;
- anticipated implementation;
- familiar standards;
- probable application design.

Candidates, suggestions and recommendations SHALL be distinguishable from documented, approved architecture. A proposed resolution or apparently better alternative SHALL NOT acquire authority through downstream use.

AI development agents SHOULD challenge architecture when evidence warrants architectural review. They SHALL NOT silently bypass it because they recognise a familiar destination.

This axiom applies the authority and explicit-review obligations in Purpose, Relationship to Architecture Decisions and Architectural Review Rule to downstream derivation and implementation.

AX-14 preserves semantic distinctions; this axiom preserves the distinction between established, unresolved and proposed architecture. AX-15 governs uncertainty about operational state, outcome or effect; this axiom governs uncertainty about the architecture itself. Neither form of uncertainty may be silently resolved by assumption.

AX-02 and AX-04 preserve Harmonia's internal semantics from external standards and engine assumptions. Familiar standards or technology patterns do not establish otherwise undocumented architectural relationships. AX-06 governs the authority of managed information; architectural authority remains a distinct concern.

Maxim

Traceability must be truthful, not artificially complete.

AX-18 — Integration and Coordination Do Not Confer Domain Authority

Axiom

> Harmonia MAY integrate information concerning, and coordinate operational activity associated with, an externally governed domain activity without thereby acquiring authority to determine or manage that domain activity.
>
> Authority to determine or manage a domain activity SHALL remain with the responsible actor, system or governed capability unless that authority is explicitly assigned to Harmonia by the architecture.
>
> Harmonia's observation, processing, persistence, distribution, presentation or use of information concerning a domain activity SHALL NOT, by itself, establish authority over that activity.

Rationale

Participation in a domain activity and authority over that activity are different architectural concerns. Information possession, observation, integration, coordination and processing do not, by themselves, confer domain authority. Authority requires explicit architectural assignment.

This new human-approved axiom generalises an already-established class of architectural boundary. It is established at the 2026-10-09 migration boundary; it is not a claim that AX-01 through AX-17 always contained this axiom.

Consequences and qualifications

AX-18 SHALL NOT be interpreted as preventing Harmonia from performing operational activity for which architecture has assigned Harmonia responsibility. Explicit architectural assignment may establish authority to determine or manage a domain activity.

In particular, AX-18 does not mean:

- Harmonia cannot execute Praxis/Ergo activity;
- Harmonia cannot coordinate entity-specific operational activity through a Digital Twin;
- Harmonia cannot process or transform clinical information;
- Harmonia cannot perform workflow/activity coordination;
- Harmonia cannot manage responsibilities explicitly assigned to Harmonia;
- Harmonia cannot enforce policy, security, information governance or operational controls assigned to it.

Participation, information, integration and coordination do not, by themselves, establish authority to determine or manage the underlying domain activity. Assigned Harmonia operational responsibility remains applicable.

Examples and established specialisations (non-normative)

These examples explain the boundary; they do not expand the axiom or establish Application Components, Information Concepts, Services, Processes, AI allocation or implementation mechanisms.

- **Clinical work:** Harmonia may integrate and coordinate around clinical work without thereby acquiring authority to determine clinical work, manage clinical handover, own clinical worklists, allocate clinical work, make clinical decisions or become the EMR workflow authority. The detailed Domain03 clinical-work integration boundary remains the Business specialisation.
- **Digital Twins:** A Twin may coordinate operational activity associated with its real-world entity without thereby acquiring authority over originating clinical facts or domain decisions. The Strategy entity-specific coordination rule remains controlling for that coordination.

- **Capacity:** Higher-order Service Capacity Management may consume context-specific capacity without acquiring the originating responsibility for Clinic, Ward, Theatre, ED, Outpatient or Mobile Service Capacity Management. The detailed Business capacity responsibility boundary remains in Business Architecture.
- **Assurance:** Observation, access to evidence or establishment of an assurance conclusion does not confer management authority over the activity being assured. Independent Assurance remains governed by REQ-FND-005 and its established Strategy and Business derivation.
- **Presentation / UI:** Presentation or collection of information does not, by itself, confer originating information or domain-decision authority on the presenting component. The existing Iris contextual human interaction responsibility remains distinct.
- **AI:** Assistance with interpretation, pattern recognition, criteria evaluation or decision support does not, by itself, confer the authority of the governed actor, role or capability using that assistance. This example assigns no AI responsibility or capability.

Related architectural authority

- AX-06 governs information authority and credibility.
- AX-13 governs the Harmonia management boundary at ingress/egress.
- AX-14 preserves meaningful semantic distinctions.
- AX-16 governs coordinated operational activity/entity progression.
- AX-17 governs architectural authority and explicit uncertainty.
- AX-18 governs inference of domain authority from participation, information, integration or coordination.

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